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Predicting falls in community-dwelling older adults: a systematic review of prognostic models

Journal:	*BMJ Open*
Manuscript ID	bmjopen-2020-044170
Article Type:	Original research
Date Submitted by the Author:	26-Aug-2020
Complete List of Authors:	Gade, Gustav; Aalborg University Hospital, Department of Geriatric Medicine; Aalborg University, Department of Clinical Medicine Jørgensen, Martin; Aalborg University Hospital, Department of Geriatric Medicine Ryg, Jesper; Odense University Hospital, Department of Geriatric Medicine; University of Southern Denmark, Department of Clinical Research Riis, Johannes; Aalborg University Hospital, Department of Geriatric Medicine; Aalborg University, Department of Clinical Medicine Thomsen , Katja ; Odense University Hospital, Department of Geriatric Medicine; University of Southern Denmark, Department of Clinical Research Masud, Tahir ; Nottingham University Hospitals NHS Trust, Department of Healthcare for Older People Andersen, Stig; Aalborg University Hospital, Department of Geriatric Medicine; Aalborg University, Department of Clinical Medicine
Keywords:	GERIATRIC MEDICINE, PUBLIC HEALTH, STATISTICS & RESEARCH METHODS

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Title page

Title: Predicting falls in community-dwelling older adults: a systematic review of prognostic models

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Keywords:
- Accidental falls
- Aged
- Models, Theoretical
- Multivariate analysis
- Prognosis

Word count: 4,959.

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Abstract

Objective

To systematically review and critically appraise prognostic models for falls in community-dwelling older adults.

Eligibility criteria

Prospective cohort studies with any follow-up period. Studies had to develop or validate multifactorial prognostic models for falls in community-dwelling older adults (60+ years). Models had to be applicable for screening in a general population setting.

Information source

MEDLINE, EMBASE, CINAHL, The Cochrane Library, PsycINFO, and Web of Science for studies published in English, Danish, Norwegian, or Swedish until January 2020. Sources also included trial registries, clinical guidelines, and reference lists of included papers along with contacting clinical experts to locate published studies.

Data extraction and risk of bias

Two authors performed all review stages independently. Data extraction followed the CHARMS (Critical appraisal and data extraction for systematic reviews of prediction modelling studies) checklist. Risk of bias assessments on participants, predictors, outcomes, and analysis methods followed PROBAST (Prediction model risk of bias assessment tool).

Results

After screening 11,789 studies, 30 were eligible for inclusion (n = 86,369 participants). Median age of participants ranged from 67.5-83.0 years. Falls incidences varied from 5.9-59%. Included studies reported 69 developed and three validated prediction models. Most frequent falls predictors were prior falls history, age, sex, measures of gait, balance and strength, along with vision and disability. The area under the curve was available for 40 (55.6%) models, and results ranged from 0.49 to 0.87. All models had a high risk of bias, mostly due to limitations in statistical methods, outcome assessments, and restrictive eligibility criteria.

Conclusions

An abundance of prognostic models on falls risk have been developed, but with a wide range in discriminatory performance. All models exhibited a high risk of bias rendering them unreliable for prediction in clinical practice. Future prognostic prediction models should comply with recent recommendations such as TRIPOD.

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PROSPERO registration number: CRD42019124021

Strengths and limitations of this study

- This systematic review is the first to summarise all prediction models on falls in community-dwelling older adults of the general population.
- The extensive search strategy supports identifying all available prospective cohort studies predicting falls in community-dwelling older adults (60+ years).
- Guidelines on prediction modelling reviews were strictly followed for search strings, data extraction (CHARMS), risk of bias assessment (PROBAST), along with development (PRISMA-P) and transparent reporting of the review (PRISMA).
- All review stages were performed independently and in duplicate.
- The exclusion of non-English language studies constitutes a risk of selection bias.

Background

The propensity to fall is a serious and common health issue among older adults with one-third of community-dwelling adults ≥65 years and half of those ≥80 years falling annually.[1] Consequences of falls are considerable with loss of independence, increased morbidity, and mortality.[2–4] Furthermore, the healthcare costs of falls increase substantially with age.[5] Therefore, as the prevalence of older fallers is predicted to increase with changes in demography, preventing falls is of utmost importance.[6]

Falls interventions have proven effective when aimed at older adults with a high risk of falling.[7] However, identifying these high-risk individuals is not straight-forward since falling is multifactorial. Prognostic models combine risk factors to estimate the individual's risk of a future outcome.[8] Thus, a prognostic model may be a valuable tool to discriminate between older adults at high versus low risk of falling. To prevent the consequences of falls, healthcare professionals could perform screening in the general population using prognostic models.[9] However, no systematic review has addressed prognostic models on falls for community-dwelling older adults.

This systematic review aims to provide an updated overview of available models to be used by healthcare professionals and for researchers to improve upon. The primary objective was to describe the discriminatory performance of prognostic models for falls in prospective cohort studies on community-dwelling older adults. Secondary objectives were to describe the study and model characteristics of these models.

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Methods

A protocol was preregistered at the International Register of Systematic Reviews (PROSPERO) before commencing the review process (CRD: CRD42019124021)[10] and is available in data supplements (Appendix 1). The review and its protocol followed the Preferred Reporting Items for Systematic reviews and Meta-Analyses statement (PRISMA)[11] and PRISMA-Protocols[12], respectively. A completed PRISMA Checklist is available in data supplements (Appendix 2). During the review process, we found the protocol unclear in terms of eligibility criteria for the study designs, participants, models, outcomes, and settings, for which reason we have further described these. Rationales for the changes are given in the protocol. Box 1 provides an introduction to commonly used prediction modelling terms.

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Box 1: Commonly used prediction modelling terms with examples related to falls

Prognostic factor

A prognostic factor, also called a predictor, is any measure that, among people with a given health condition, is associated with a subsequent clinical outcome such as falls.[57]

Prognostic prediction model

A prognostic prediction model is a statistical combination of multiple predictors from which risks of a longitudinal outcome, e.g. falls, can be calculated for individuals.[8]

Development and validation studies

A prediction model development study aims to develop a prediction model by combining essential predictors from a data set into a model and testing its predictive performance within the same development data set.[54] A model validation study aims to assess the predictive performance of a developed prediction model using new data not used in the development of the model.[54]

Model performance, overfitting, and internal validation

A model's predictive performance is termed model performance. This term encompasses several measures with the two most important being discrimination and calibration.[17] Estimates of model performance derived directly from a data set used for developing the model is termed the apparent performance.[54] Since the model is fitted explicitly to the development data set, predictions on new data, i.e. new older adults with different characteristics, may yield poorer model performance estimates, i.e. poor generalisability. Hence, clinicians would typically find the apparent performance optimistic in terms of predicting a fall in their population which has not been used for developing the model. In consequence, fall preventive interventions could end up being provided to those not needing it and not offered to those actually in need hereof. The optimism in apparent performance is due to the model fitting too well to its data, a term known as overfitting. In such situations, predictions would be biased when the model is used on older adults with different characteristics, i.e. frequency distributions of predictors.[17] Estimating the amount of optimism in the development study's model can be done using internal validation techniques such as bootstrap validation. However, since the population of older adults is heterogeneous, generalising a model's performance to the entire population would be more clinically relevant. Here, internal validation procedures fail, and the model should instead be tested in a validation study.

Model discrimination

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Model discrimination is a performance measure referring to the models' ability to correctly predict if an individual will experience a fall or not. Therefore, as an example, it can be used by healthcare professionals to assess how confident a model assigns individuals to a high-risk group and guides the clinician when allocating fall preventive interventions.[58] A perfectly discriminating model assigns a higher risk of falling to all older adults experiencing a fall. Likewise, a lower risk is appointed to those not suffering a fall. Usually, discrimination is reported as a concordance index (c-index) or an area under the curve (AUC), but other measures are also available. Here, a value of 1 equals perfect discrimination, and 0.5 indicates that the model discriminates no better than chance. If a model shows poor discriminative performance, it could predict low-risk older adults to fall and high-risk older adults to not fall.

Model calibration

Model calibration is a performance measure used to examine whether a model over- or underestimates the predicted risks in a sample. More specifically, it is the agreement between predictions made by the model and the frequency of the outcome to be predicted.[54] Healthcare professionals can utilise this information to assess how confident the model predicts the specific risk of having a fall for the individual. In brief, it is crucial when counselling older adults on their fall risk that the risk estimate is as accurate as possible.[58] If the model predicts a person to have a 10% risk of falling within one year, the observed frequency of people falling with such a predicted risk should be 10 out of 100 for the model to have good calibration. However, should the frequency be only 5 of 100 people, the model overestimates the risk. Calibration is typically assessed graphically using calibration plots. In development studies, models are usually calibrated well to the data from which they are developed and therefore yield limited information.[4] Thus, it is more relevant how well the model is calibrated when introduced to a new sample of older adults used to validate the model. This information would enable healthcare professionals to evaluate whether the model over- or underestimates the risk of falling when used in their population of community-dwelling older adults. If the model is not correctly calibrated, it could predict low-risk older adults to have a higher risk and vice versa, or systematically over- or underestimate all predictions.

Eligibility criteria

Participants and setting

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Participants had to be community-dwelling, 60 years of age or older, and be recruited from a general population setting. For that reason, we excluded models intended for hospitals, general practitioners, and nursing homes. Studies restricted to participants with pre-specified diseases, conditions, or symptoms such as Parkinson's disease or stroke were excluded to raise external validity. However, we included studies that excluded certain types of community-dwelling older adults, such as those with known neurological, spinal, or cognitive disorders.

Index (Model)

Studies had to present a final multifactorial prognostic model defined by the inclusion of two prognostic factors or more. This definition was chosen as causes of falls are multifactorial and coexisting.[1] Thus, prognostic factor studies investigating the association between predictors and prospective falls were excluded. Both development studies and validation studies with and without model updating were included.

Outcome

We included studies defining falls as "an unexpected event in which the participants come to rest on the ground, floor, or lower-level".[13] However, studies without an outcome definition were also included since this would not rule out the definition mentioned above. We excluded studies using fall definitions excluding certain types of falls presumed to be due to a specific cause, e.g. external forces or acute medical events. This approach was chosen since post-fall classification methods may introduce recall bias.[14] Finally, we did not include studies predicting only injurious falls in older adults since risk factors for these are different from those experiencing non-injurious falls.[15] No restrictions were made on method or timing of outcome assessment other than it had to be prospectively recorded.

Study designs

We only included prospective cohort studies since this study design allows optimal control when measuring predictors and outcomes. Thus, it is the recommended study design for prognostic modelling studies.[16] We excluded randomised controlled trials since these can have different limitations incorporated within their design. Typically, strict eligibility criteria are used that generate a highly selected sample of participants. This narrows predictors' distribution and hence reduces the discriminatory performance in the prognostic models.[17] Also, strict criteria may compromise generalisability to the target population.[18] Lastly, interventions in the study may also influence the discriminatory performance of the models.[18] Retrospective cohort studies were also

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excluded due to issues of missing data and restrictions on which predictors to apply since data is already collected.[18]

Timing

No restrictions on follow-up or predictive horizon were made since we found it clinically relevant to include models both able to predict falls within short and long periods ahead in time.

Language and publication year

Due to the composition of the study group, we only included published studies reported in English, Danish, Norwegian, or Swedish languages. No restrictions on publication year were made.

Information sources

We searched electronic databases, trial registries, and clinical guidelines. Furthermore, we consulted with additional clinical experts. Lastly, we screened conference abstracts along with reference lists of both the included studies and systematic reviews found during the search. Databases included Pubmed.gov (PubMed interface), EMBASE (Embase.com), CINAHL (EBSCOhost interface), The Cochrane Library (Wiley interface), PsycINFO (APA PsycNET interface), and Web of Science (Web of Science Core Collection). All databases were searched from inception dates to the 3rd of January 2020. Trial registries included PROSPERO, Clinicaltrials.gov, WHO International Clinical Trials Registry Platform, and Open Grey. Guidelines included Guidelines International Network (GIN), the National Institute for Health and Care Excellence (NICE), Centre for Reviews and Dissemination and to Health Technology Assessments (CRD/HTA), and Scottish Intercollegiate Guidelines Network (SIGN). Conference abstracts and studies in trial registries were used to obtain full-text papers through contact with authors. Letters to the editor were excluded.

Search

We used a validated search string for prediction models.[19] With the help of a research librarian in health science, we added the following terms to the search string: independent living, aged, and accidental falls. Details on the search string are available in data supplements (Appendix 3). No search filters were applied.

Study selection

Duplicates were removed using EndNote (EndNote X9, Clarivate Analytics, Philadelphia, USA). Two reviewers independently screened titles and abstracts (GVG and JRI) and full-text papers (GVG and KT) according to the inclusion criteria. We contacted authors for clarification when information on this review's eligibility criteria was missing. Disagreement among reviewers was

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resolved by consensus for one study by including a third reviewer (MGJ). For screening of titles and abstracts along with full-text reading, we used Covidence (Covidence systematic review software, Veritas Health Innovation, Melbourne, Australia. Available at www.covidence.org). Exclusion of studies after full-text reading was performed using a prioritised list of reasons (Appendix 4). Reviewers were not blinded to author names, institutions, or journal titles.

Data collection process

We developed a standardised data collection form using REDCap[20], a research electronic data capture software, following the Critical Appraisal and Data Extraction for Systematic Reviews of Prediction Modelling Studies (CHARMS) checklist.[18] Data extraction was performed in duplicate and independently by two reviewers (GVG and JRI). Independence between reviewers was ensured using a double data entry module in REDCap, thereby denying access to each other's responses. Disagreements among the reviewers were discussed, and the third reviewer (MGJ) was not consulted during data collection since consensus was reached in all studies. We contacted all study authors for retrieval of information on data items not reported. None of the included studies was published more than once.

Data items

We extracted data on the following items: country, publication year, authors, in- and exclusion criteria, age, outcome definition, number of falls and fallers, candidate predictors, missing data, choice of statistical analysis, C-statistic and Area-Under the receiver-operating-characteristic Curve (AUC), internal and external validation procedures, final model presentation, and sources of funding. If available, 51 data items were extracted from each paper as detailed in data supplements (Appendix 5).

Risk of bias and reporting transparency

To follow current recommendations[21], the Prediction study Risk Of Bias Assessment Tool (PROBAST)[22] was used for the risk of bias assessment in individual studies. The tool comprises 20 signalling questions in four domains: participants, predictors, outcomes, and analysis. The tool also includes an evaluation of each model's applicability for the intended population, predictors and outcome of the review. Reporting transparency was assessed using the Transparent reporting of a multivariable prediction model for individual prognosis or diagnosis (TRIPOD) adherence assessment form.[23] The bias, applicability, and reporting assessments were performed in duplicate and independently by two reviewers (GVS and JRI). Independence between reviewers was ensured using a double data entry module in REDCap, thereby denying access to each other's responses.

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Disagreements among the reviewers were discussed, and consensus was reached for all studies. Thus, a third reviewer was not consulted for a final decision. The reviewers were not blinded to study authors, institutions, or journal titles. The results of the risk of bias assessments of all included studies were incorporated into the qualitative synthesis. We sought to investigate outcome reporting bias by comparing the study papers to their pertaining protocols to examine whether outcomes were pre-specified and not differing from the published paper.

Summary measures and planned method of analysis

The principal summary measure of this systematic review was the discriminatory performance measured either in a C-index or AUC. In the pre-specified protocol, we decided not to perform meta-analyses due to the presumed heterogeneity of the prognostic models. This assumption was confirmed after the review was complete. Furthermore, we summarised the study and model characteristics using ranges and percentage proportions when appropriate. When data were available, we summarised continuous measures using medians and interquartile ranges.

Declaration of sources of funding

This work was supported by the Department of Geriatric Medicine, Aalborg University Hospital, Aalborg, Denmark and the Department of Clinical Medicine, Aalborg University, Aalborg, Denmark. No external organisation had any role in the design, execution, or interpretation of data or writing of the paper.

Patient public involvement

We did not involve patients or the public in the research.

Results

Study selection

The search yielded 19,612 publications with 11,789 remaining after removal of duplicates. Screening titles and abstracts led to the exclusion of 11,611 publications leaving 178 for full-text reading. Of these, 148 were excluded due to: wrong outcome (n = 45), wrong study design (n = 45), not being a prediction model (n = 25), no full-text paper published (n = 14), wrong population (n = 8), not multifactorial (n = 8), or wrong setting (n = 3). Thirty studies met the eligibility criteria and were included.[24–53] Figure 1 displays the PRISMA flow diagram for the study selection. Details on excluded papers can be found as data supplements (Appendix 6-8).

Characteristics of included studies

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A summary of included studies along with models' performance can be found in Supplementary Table 1 (Appendix 9). All studies were published in English from 1994 to 2019. Seventy-two prognostic models were reported, of which 69 models were developed, and three were validated.

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Table 1. Reporting, risk of bias, and applicability ratings of individual studies

Study	Reporting	Risk of bias				Applicability			Overall	
	TRIPOD	Participants	Predictors	Outcome	Analysis	Participants	Predictors	Outcome	ROB	Applicability
Yamashita et al.[48], 2011	34%	-	+	-	-	-	+	?	-	-
Weiss et al.[52], 2013	36%	+	+	-	-	-	+	+	-	-
Muhaidat et al.[27], 2014	36%	+	+	?	-	+	+	?	-	?
Brauer et al.[25], 2000	37%	-	+	?	-	-	-	+	-	-
Panzer et al.[50], 2011	37%	-	+	-	-	-	+	?	-	?
Maki et al.[24], 1994	41%	+	+	-	-	+	+	+	-	+
Boulgarides et al.[45], 2003	41%	+	+	?	-	+	+	?	-	?
Viccaro et al.[51], 2011	41%	+	+	-	-	?	+	+	-	?
Lindemann et al.[44], 2008	44%	-	+	-	-	-	+	+	-	-
Singh et al.[37], 2019	44%	-	+	?	-	-	+	+	-	-
Nandy et al.[46], 2004	45%	+	+	-	-	?	+	?	-	?
Okochi et al.[43], 2006	45%	+	+	-	-	?	+	+	-	?
Rodriguez-Molinero et al.[33], 2017	46%	-	+	-	-	+	+	+	-	+
Gadkaree et al.[29], 2015	48%	+	+	-	-	+	+	+	-	+
de Vries et al.[26], 2013	50%	+	+	-	-	+	+	+	-	+
Hnizdo et al.[53], 2013	52%	+	+	?	-	-	+	+	-	-
Lohman et al.[34], 2017	52%	+	+	-	-	+	+	-	-	-
Gillain et al.[38], 2019	54%	-	+	-	-	-	-	+	-	-
Delbaere et al.[47], 2010	55%	+	+	-	-	+	+	-	-	-
Cawthon et al.[30], 2015	56%	+	+	-	-	-	+	+	-	-
Kim et al.[35], 2017	56%	-	+	-	-	-	+	?	-	-
Tromp et al.[28], 2001	57%	+	+	-	-	+	+	+	-	+
Stel et al.[41], 2003	57%	+	+	-	-	+	+	+	-	+
Lamb et al.[36], 2008	61%	+	+	-	-	-	+	?	-	-
Yamashita et al.[49], 2012	61%	+	+	-	-	+	+	-	-	-
Stalenhoef et al.[40], 2002	62%	?	+	-	-	-	+	+	-	-
Palumbo et al.[32], 2018	68%	+	+	-	-	?	+	?	-	?
Bongue et al.[39], 2011	70%	-	+	?	-	-	+	+	-	-
Palumbo et al.[31], 2016	73%	+	+	-	-	+	+	?	-	?
Pluijm et al.[42], 2006	76%	+	+	-	-	+	+	+	-	+

Note: + indicates low risk of bias /low concern regarding applicability; − indicates high risk of bias/high concern regarding applicability; ? indicates unclear risk of bias/unclear concern regarding applicability; ROB indicates risk of bias; TRIPOD indicates the percentage proportion of reported items recommended in reporting guidelines.

Participants

Studies were conducted in Australia, Belgium, Canada, France, Germany, Israel, Italy, Japan, the Netherlands, Malaysia, Spain, the United Kingdom, and the USA. Sample sizes ranged from 65 to 23,417 participants with median ages from 67.5-83 years. Studies used primarily a probability sampling method (n = 16), followed by convenience sampling (n = 8), and consecutive sampling (n = 2). Four studies did not report their sampling methods.

Index/Model

The median (IQR) number of predictors in the final models were five (3-9) and ranged from two to 96 predictors. Figure 2 shows the number of studies including a specific predictor. The most frequently applied predictors were a history of falls, age, sex, measures of gait, balance, and strength, along with vision and disability. Predictors were measured in homes (n = 19), research centres (n = 19), or both (n = 12). Locations for measuring predictors were not reported for 22 models.

Outcomes and timing

The percentage of fallers ranged from 5.9-59%, and the number of recurrent fallers (≥ 2 falls) ranged from 6.3-54.1%. Models primarily predicted any falls (n = 34), i.e. single and recurrent falls, and recurrent falls only (n = 34). Two models predicted first-time falls[38, 51] and two predicted time to a fall.[26,39] Participants were followed for a median (IQR) time of 12 (9.75-12) months. Individual study data items extracted are available in the data supplement (Appendix 10).

Model performance

Discriminatory measures were reported for 40 (55.6%) models. AUCs were 0.49-0.87 and 0.62-0.69 for developed (n = 37) and validated (n = 3) models respectively. Corresponding confidence intervals were reported for 27 (37.5%) models. Calibration measures were available for seven (9.7%) models. For validated models (n = 1), calibration was imperfect due to over- and underestimated predicted risks of falling for high- and low-risk participants, respectively.[31] Regarding developed models (n = 6), calibration was found acceptable, but studies did not assess model calibration in new participants.[33, 37, 40, 42]

Risk of bias, reporting transparency, and applicability within studies

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Risk of bias

Table 1 summarises ratings on risk of bias, applicability, and reporting transparency for the individual studies. All studies had a high risk of bias mainly due to methods of analysis and outcome assessment along with restrictive eligibility criteria. Regarding analysis methods, missing data were excluded in 13 out of 30 studies, and no internal validation methods were applied. As to the outcome, only four studies recorded falls daily with monthly notifications.[25, 27, 47, 52] Also, the majority of studies did not report the outcome definition used or whether outcomes assessors were blinded. Eligibility criteria were found restrictive for the majority of studies due to the exclusion of individuals with falls-risk-increasing conditions. These selective criteria limit the usability of models for the target population of community-dwellers. Overall, risk of bias and applicability assessments were complicated by studies only reporting, on average, 50% of all items recommended in reporting guidelines. Furthermore, this was complicated by a low response rate with four out of 30 study authors responding when contacted for clarification on study characteristics and data extraction items. Finally, outcome reporting bias assessments were not possible due to studies not referring to a preregistered protocol for their prognostic modelling study.

Applicability

Seven (23%) studies, with a total of 21 models, had low applicability concerns for the review question. Regarding participants, 17 (56.7%) studies were rated as having high or unclear applicability concerns for the review question. This concern was primarily due to restrictive eligibility criteria impeding generalisation to the general population of community dwellers. Restrictions were made by excluding participants with specific diseases or conditions that could increase the risk of falling, such as disability or impaired mobility. Furthermore, studies rated as having unclear applicability concerns did not sufficiently report whether the participants were community-dwellers or whether the setting was the general population rather than, e.g. primary or secondary care. Regarding predictors, 28 (93.3%) studies had no concerns. The remaining two studies used specific laboratory measures which may be challenging to apply in a general population setting.[25, 38] Regarding applicability concerns for the outcome, 18 (60%) studies had no concerns since they reported using the falls definition of the review or similar.

Discussion

The current systematic review found 72 prognostic models on falls risk with the area under the curve ranging from 0.49 to 0.87. All models had a high risk of bias mostly due to limitations in

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statistical methods and outcome assessments, combined with restrictive eligibility criteria. Thus, using the models in clinical practice would entail unreliable predictions. This review provides an extensive overview of prediction models for falls and information for future study methodology.

Strengths

The current review followed guidelines on prediction modelling reviews strictly for search strings[19], data extraction[18], risk of bias assessment[22], along with development[12,18] and transparent reporting of the review.[11]

Limitations

Review level

We included "Aged" as a search term in the search string. Since this would restrict the number of search hits and thus the sensitivity of the search string, we pre-tested the search string without "Aged" in all databases before commencing the review. From this, the first 3,000 hits were screened independently and in duplicate, and we did not find studies not identified by the final search string. Thus, we believe this had a limited influence on the search string. Furthermore, potential studies without full-text (n = 14) or published in other languages (n = 7) during screening of titles and abstracts were excluded. Excluded studies are listed in the data supplement.

Study level

Limitations were found in the studies with a high risk of bias, poor quality of reporting, and finally, a low response rate when contacting authors for retrieval of missing data extraction items.

Risk of bias

We found a high risk of bias within all studies. Hence, the predictive performance may be low, and predictions unreliable when models are used in clinical practice. The bias ratings were primarily based on eligibility criteria, methods of outcome assessments, and statistical analysis. Building prediction models on selected subgroups of the target population can yield biased performance estimates when used in clinical practice on a different population.[17] Thus, study eligibility criteria should be aligned with the research questions, i.e. broad and with as few exclusion criteria as possible. In terms of outcome assessments, we found the definition of falls missing for one-third of studies along with varying falls recording methods. These findings are similar to results of a previous review on methodology in falls prevention trials, where only half of the studies provided a falls definition and recording methods varied highly.[14] The problem with not defining a fall is that the notion of falls is taken for granted. As seen in our review, the prevalence of falls differed markedly between studies, which could be due to different understandings of the fall's definition.

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Consequently, falls become harder to predict[17] while at the same time, comparing and combining studies in systematic reviews with meta-analyses becomes complicated. To address these issues, a common outcome data set on falls trials is available along with a falls definition and recommendations for falls recording methods.[13] Finally, statistical analysis methods raised concerns for risk of bias. Primarily, this was due to the handling of missing data with most of the studies applying a complete-case analysis method. Significant limitations can arise from the exclusion of participants due to missing data, e.g. on a single predictor among many, since otherwise useful predictors on each participant are lost. Consequently, this can lead to low sample sizes and biased model performances. In such cases, imputation methods have proved useful when dealing with missing data.[17] Furthermore, for the majority of developed models, no internal validation procedures were applied. This shortcoming typically causes models' predictive performance to be optimistic.[54] Finally, the critical appraisal was compromised due to incomplete reporting. We believe that future studies and systematic reviews would benefit from adhering to reporting guidelines for prediction modelling studies.[16]

Implications for clinical practice

Only seven studies could address the review question appropriately, and all of these had a high risk of bias. Consequently, the evidence available to inform healthcare professionals is limited and, as mentioned, possibly biased. Thus, no model can currently be recommended for clinical practice.

Implications for research

We recognise that most studies (n = 23/30) were conducted before the publishing of prediction modelling guidelines.[16, 22] Thus, with the benefit of hindsight, studies would be expected to have different shortcomings within their methods and reporting. On the other hand, this also supports the reason for publishing guidelines in the first place. Despite this, the included studies provide valuable information on future candidate predictors. Thus, selecting predictors for prediction models on non-statistical grounds, i.e. based on literature and clinical knowledge, is commonly used to avoid predictor selection bias.[55] Therefore, future development studies may include the most frequently applied predictors found in this review. Lastly, it is essential to test the generalisability of developed models by performing validation studies to determine which models provide stable predictions across different populations.[56]

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Conclusions

There are several studies on falls prognostic models, but only a few with no concerns regarding applicability for our review question. Thus, the evidence available to address this is limited. From all included studies, we found an abundance of falls prognostic models available. However, the discriminatory performance of these varied and was only reported for half of the models. Each model had concerns regarding risk of bias mainly due to restrictive eligibility criteria along with methods of statistical analysis and outcome assessments. Consequently, this could give rise to unreliable predictions should the models be used in clinical practice. Future prognostic prediction models should comply with TRIPOD.

Acknowledgements

The authors wish to thank health science librarian Pernille Skou Gaardsted for assisting with the development of the search strategy.

Contributors:

GVG is the guarantor. GVG drafted the manuscript for the protocol and performed preliminary searches and search strategy. GVG, JRY, MGJ, TM, KT, and SA developed selection criteria. JRI assisted GVG in screening titles, abstracts, and reference lists of papers included after full text reading along with data extraction, assessing risk of bias, presence of meta-bias along with adherence to reporting guidelines. KT assisted GVG in full-text reading, and MGJ was arbitrator if agreement could not be reached between reviewers. GVG drafted the manuscript for the paper, while MGJ, JRY, SA, TM, JRI, and KT assisted in the interpretation of results, read, provided feedback, and approved the final manuscript of the paper. The corresponding author attests that all listed authors meet authorship criteria and that no others meeting the criteria have been omitted.

Funding:

Our host institutions funded this research: Department of Geriatric Medicine, Aalborg University Hospital, Aalborg, Denmark and the Department of Clinical Medicine, Aalborg University, Aalborg, Denmark. This research received no specific grant from any funding agency in the public, commercial or not-for-profit sectors.

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Transparency statement:

The lead author affirms that the manuscript is an honest, accurate, and transparent account of the study being reported; that no important aspects of the study have been omitted; and that any discrepancies from the study as originally planned have been explained.

Competing interests:

All authors have completed the ICMJE uniform disclosure form at www.icmje.org/coi_disclosure.pdf and declare: no support from any organisation for the submitted work; no financial relationships with any organisations that might have an interest in the submitted work in the previous three years; no other relationships or activities that could appear to have influenced the submitted work.

Ethical approval:

Not required.

Dissemination to participants and related patient and public communities:

The study did not involve participants for whom the study results can be disseminated. The authors intend to distribute the results of the study by engaging with local healthcare providers for the general population setting during 2020 and 2021, along with social media to reach a broader audience.

Data availability statement:

The study protocol is available online at https://www.crd.york.ac.uk/prospero/display_record.php?RecordID=124021. All information extracted for the included studies in the review is available as a data supplement.

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Figure legends:

Figure 1 PRISMA (preferred reporting items for systematic reviews and meta-analyses) diagram of the study selection process.

Figure 2 Number of studies using a specific predictor

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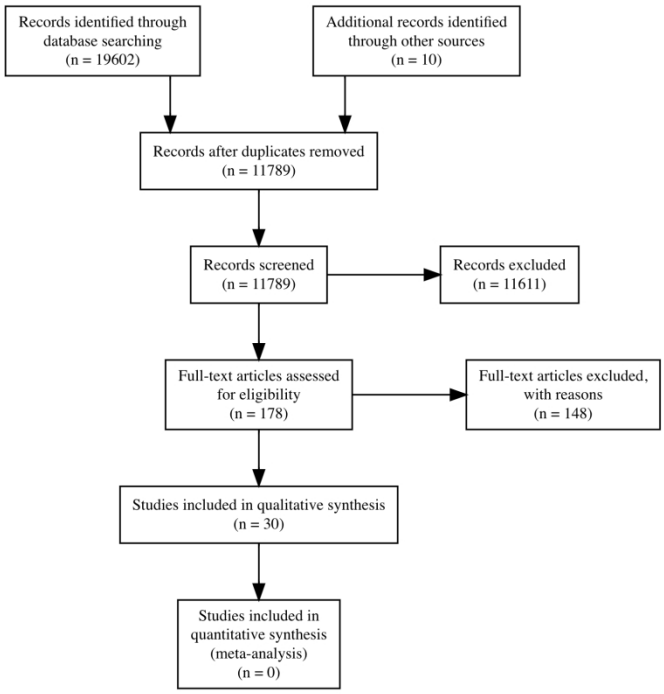

Figure 1 PRISMA (preferred reporting items for systematic reviews and meta-analyses) diagram of the study selection process.

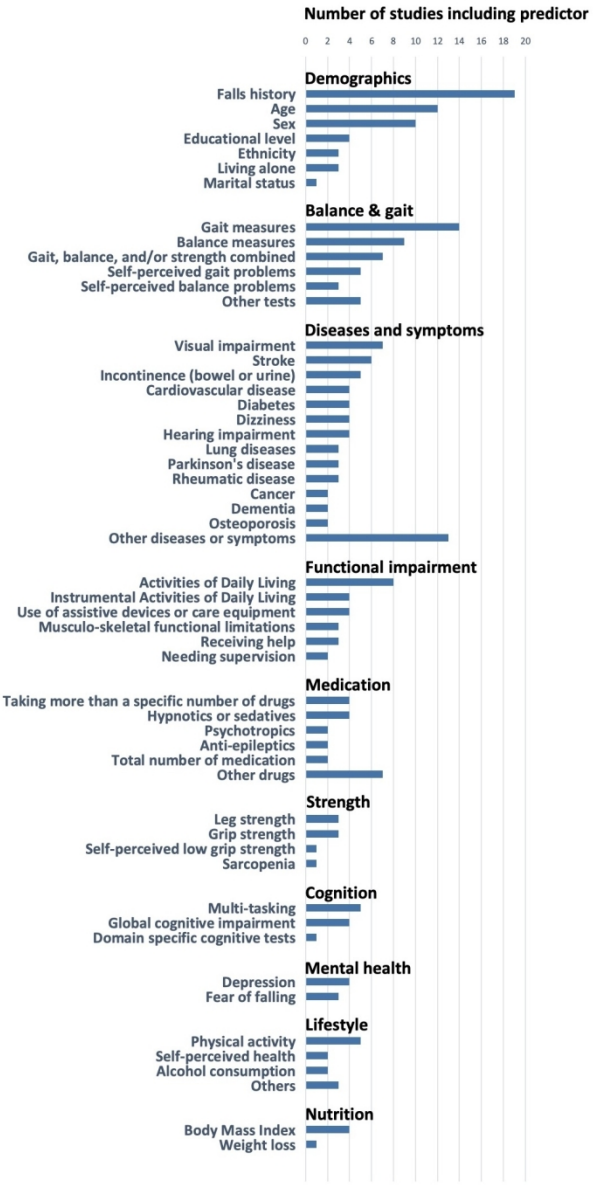

Figure 2 Number of studies using a specific predictor

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Appendix 1: protocol for systematic review

Administrative information

Title:

Systematic review of prognostic models for predicting falls in community-dwelling older adults

Registration:

This protocol was registered with the International Prospective Register of Systematic Reviews (PROSPERO) on (1st February 2019).

Registration number: CRD42019124021

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Contributions:
GS is the guarantor. GS drafted the manuscript for the protocol and performed preliminary searches and search strategy. GS, JR, MGJ and SA developed selection criteria. JRI will assist GS in screening titles, abstracts and reference lists of papers included after full-text reading along with data extraction, assessing risk of bias, presence of meta-bias along with adherence to reporting guidelines. KT will assist GS in full-text reading. MGJ will be arbitrator if agreement cannot be reached between reviewers. GS will draft the manuscript for the paper. MGJ, JRY, SA, TM, JRI and KT will assist in interpretation of results, read, provide feedback and approve the final manuscript of the paper.

Amendments:
In the event of protocol amendments, this section will describe the date, changes and rationale of each amendment. Changes will be incorporated into the protocol sections. All authors will be responsible for approving the amendments. Also, GS will be responsible for documentation and implementation of these.

Current version of the protocol: 3.

- 7th of August 2019
    - **Change #1:**
        - Setting:

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- We further specified which setting the review is, and is not, intended for. We changed the wording "community setting" to "general population setting". Also, we specified that prediction models intended for a primary care would also be excluded.

  - Rationale:
    - Change in wording: To apply the same terminology of the CHARMS checklist.
    - Primary care exclusion: To increase transparency and homogeneity in settings.

- **Change #2:**
  - Risk of bias:
    - As a supplement to the risk of bias assessments, the newly published TRIPOD adherence tool will be used.
  - Rationale
    - To assess adherence to reporting guidelines for prediction modelling studies.

- 17th of June 2019
  - **Change #1**
    - Study design:
      - We further specified which study designs would not be included. Thus, randomised controlled trials and retrospective cohort studies will not be included.
    - Rationale:
      - To increase transparency.
  - **Change #2:**
    - Participants: Age
      - We further specified the inclusion criterium regarding age. Thus, studies with total age ranges extending below 60 years will be excluded. Exclusion will also be made if mean age subtracted by two standard deviations extends below 60 years of age unless inclusion criterium in studies specifically states a lower age limit of 60 years or above.

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- Rationale:
  - To increase transparency.

- **Change #3:**
  - Participants: Community-dwelling
    - We further specified which studies would be included. Thus, we will include studies excluding certain types of community-dwelling individuals, e.g. with known neurological, spinal or cognitive disorders.
  - Rationale:
    - These samples may also contribute with relevant information about the target population.
- **Change #4:**
  - Index (Model):
    - We further specified which studies to include based on the model presented. Thus, we will also include studies with:
      - Two or more prognostic factors combined into a scale giving an individual score used to assess the predictive performance on future falls.
      - Two or more prognostic factors included in a test instrument from which a prediction model would be generated.
  - Rationale:
    - To increase transparency.
- **Change #5:**
  - Outcome (and rationale):
    - We further specified which studies to include based on the outcome. Thus, studies without an outcome definition will also be included since this will not rule out the outcome definition of this review. We will exclude studies using falls definitions excluding certain types of falls presumed to be due to a specific cause e.g. acute medical events or external forces. This post fall classification method may introduce assessor-bias.

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Support:
The Department of Geriatric Medicine, Aalborg University Hospital, Aalborg, Denmark and the Department of Clinical Medicine, Aalborg University, Aalborg, Denmark will fund and sponsor this research.

Introduction:

Rationale of the review

Falling over in community-dwelling older adults is a frequent problem with an annual prevalence of 30 % in 65+ year olds and 50 % in 80+ year olds.[1] Total number of falls are expected to increase significantly in the future due to the ageing population.[2,3] For instance, in 2017, the global population of 65+ year olds was estimated to be 962 million and is estimated to increase towards 1.4 and 2.1 billion in 2030 and 2050 respectively.[2] This frequent and escalating problem of fall accidents is a major concern globally due to their associations with elevated morbidity, mortality, poorer physical functioning and early admission to long-term care facilities which leads to elevated financial costs to society[4–6].

Fall prevention is therefore highly relevant to society, next of kin and to the individual. Unfortunately, more than 400 risk factors for falling have been identified thereby making it a complex area/problem.[7] In addition, the risk factors spread across different domains including socio-demographics, the environment, medical conditions and medications, physical performance, psychology and cognition[8]. In consequence, secondary multifactorial fall risk prevention has been recommended[3,9]. On the other hand, if individuals at high risk of falling could be identified before their first fall, primary preventive interventions could be instituted, which would be even more beneficial. Therefore, individual assessments of fall risk using multifactorial prognostic prediction models are imperative. Few reviews on the ability of prognostic prediction model studies to discriminate fallers from non-fallers in community-dwelling older adults exist[10–12]. However, in these reviews, methodologies were varied with no review protocols being reported[10–12], outcome definitions not following the current consensus definition[10–13], and search strategies being restricted[10,12] or based on search filters for diagnostic studies[11,12]. Also, risk-of-bias assessments were done using tools designed for diagnostic studies[11,12] and reporting of data extraction items and -methods were inconsistent[10,12].

Nonetheless, in recent years, prognostic research methods have evolved. Thus, new guidelines have been published to encourage researchers to transparently report prediction modelling studies[14], systematic reviews[15] and their respective protocols[16]. Also, within the field of prediction modelling reviews, literature search strategies[17] along with guidance papers on data extraction items[18] and risk

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of bias tools[19] have been developed. However, no reviews on fall prediction studies have applied the abovementioned guidelines as yet.

The scope of this review is to provide a systematic update on current model performance, and other characteristics, on developmental and validation studies within the field of fall accidents on multifactorial prognostic prediction models in community-dwelling older adults.

Objective:

The aim of this systematic review is to describe model performance along with other model characteristics (predictors along with methods of model development, -evaluation and -presentation) of existing multifactorial prognostic prediction models on falls in 60+ year old non-institutionalised, community-dwelling older adults.

Methods:

This protocol follows the guidelines of the Preferred Reporting Items for Systematic reviews and Meta-analyses Protocols (PRISMA-P)[16]. The protocol is registered in the PROSPERO database.

Eligibility Criteria

The following criteria outlined below will be used to select studies for the review.

Study designs:

We will only include prospective cohort studies since this is the preferred design for prognostic prediction modelling studies[14]. We will include both developmental and validation studies with and without model updating. Randomised controlled trials will not be included since these tend to have narrow predictor distributions resulting in poor discriminatory performance. This may also be influenced by treatment effects in the design[18]. Furthermore, generalisation to the target population may be compromised due to strict eligibility criteria[20]. Retrospective cohort studies will be excluded since the predictive performance may be substantially limited by missing data, and only predictors available in the data set collected can be applied[18].

Participants:

Only studies with all participants aged 60 years or older will be included. This cut-off was chosen in order to encompass studies using different age cut-offs for being an older adult. Thus, studies with total age ranges extending below 60 years will be excluded. Also, mean age subtracted by 2 standard deviations must not extend below 60 years unless inclusion criteria specifically stated a lower age limit of 60 years or above.

Participants should be community-dwelling and not institutionalised, i.e. living independently and not in nursing homes or short term-care where the risk of falling is substantially different from

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the general population[21]. Studies restricted to participants with pre-specified diseases, conditions or symptoms such as Parkinsonism or stroke will be excluded so that generalisation to the overall community population is not compromised. We will include studies excluding certain types of community-dwelling individuals, e.g. with known neurological, spinal or cognitive disorders since these samples may contribute with relevant information about the target population of the review.

Index (Model):

Multifactorial prognostic prediction models, i.e. including 2 prognostic factors or more due the multifactorial aetiology of falls[3,22]. Thus, explanatory studies investigating the association between a predictor and prospective falls were excluded. To broaden the search, we will include the following studies with:

- Two or more prognostic factors measuring on the same domain will be included.
- Two or more prognostic factors combined into a scale giving an individual score used to assess the predictive performance on future falls.
- Two or more prognostic factors included in a test instrument from which a prediction model would be generated.

Comparator:

None.

Outcome:

Primary outcome in the included prospective cohort studies will be falls defined by "an unexpected event in which the participants come to rest on the ground, floor, or lower level".[13] Both single and recurrent falls, i.e. >1 fall, will be included. Studies without an outcome definition were also included since this would not rule out the abovementioned definition. We excluded studies using falls definitions excluding certain types of falls presumed to be due to a specific cause e.g. acute medical events or external forces. This post fall classification method may introduce bias in the outcome assessment due to the subjective judgements involved[23].

Timing:

No restrictions on follow-up on falls will be made.

Setting:

The models should be used to screen for risk of prospective falls in a general population setting, and we will exclude models intended for primary care, hospitals and nursing homes.

Language:

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Only studies reported in an English, Danish, Norwegian or Swedish language will be included. This was chosen due to resource limitations. However, a list of possibly relevant studies in other languages found during the literature search will be included in an appendix.

Publication year:

No restrictions on publication year will be made.

Information sources

Studies will be collected from the following databases: Pubmed.gov (PubMed interface, inception date to date of search), EMBASE (Embase.com, inception date to date of search), CINAHL (EBSCOhost interface, inception date to date of search), The Cochrane Library (Wiley interface, inception date to date of search), PsycINFO (APA PsycNET interface, inception date to date of search) and Web of Science (Web of Science Core Collection, inception date to date of search). Both controlled terms (i.e. MeSH or EMTREE terms) and simple phrase terms will be used to search the databases when appropriate. Also, hand searches from the reference lists of the included studies will be performed. Conference abstracts found during the literature search will only be used for obtaining their respective full-text articles. If not found elsewhere, we will try to contact the respective authors for this. If the full-text articles are not obtainable, the study will be excluded. However, a list of these possibly relevant studies found during the literature search will be included in an appendix. Primary literature within prior systematic reviews on fall prediction models found during the literature search will be screened. Finally, two experts in the field of falls research will be consulted to enquire for knowledge on additional studies fulfilling the eligibility criteria of this systematic review.

Grey literature

PROSPERO will be searched for completed reviews with this focus. Also, Clinicaltrials.gov, WHO International Clinical Trials Registry Platform, Open Grey, GIN, NICE, CRD/HTA, SIGN will be searched for relevant studies using key-terms from the main search (falling AND elderly OR Older adults). If not found elsewhere, we will try to contact authors of these relevant studies for retrieving of the full-text.

Search strategy

The search strategy follows current Cochrane recommendations for systematic reviews on prediction models[17,24]. Also, to accommodate the search strategy to our eligibility criteria, the search string was further developed by GS in collaboration with a Health Sciences Librarian at the Medical Library of Aalborg University Hospital, Denmark. The search strategies of the selected databases are included in Appendix 1. The final search strategy will be approved by a second reviewer (KT).

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Study records:

Data management:

Duplicates will be removed using EndNote (EndNote X9, Clarivate Analytics, Philadelphia, USA). The results of the literature search will be uploaded to Covidence (Covidence systematic review software, Veritas Health Innovation, Melbourne, Australia. Available at www.covidence.org) to ease the collaboration between reviewers on titles and abstracts screening along with full-text reading. Risk of bias assessments and extraction of data will be performed using a standardised form in REDCap using a double data-entry module[25]. If several articles report results from the same trial, the "primary publication" will be prioritized; i.e. typically defined as the first full-text publication reporting on the primary outcome.

Selection process:

Screening titles and abstracts:

Two reviewers (GS and JRI) will independently screen titles and abstracts from the inclusion criteria. The screening process of titles and abstracts will undergo pilot testing. Reviewers (GS and JRI) will meet and discuss the inclusion of the first 50 articles found by the search strategy during screening of titles and abstracts. If agreement cannot be reached, a third author (MGJ) will be consulted for arbitration. If needed afterwards, refinement of inclusion criteria will be performed.

Full-text reading

Full-text reports will be obtained on eligible studies, and studies where uncertainty exists regarding eligibility based on titles and abstracts. GS and KT will independently screen the full-text reports for a final decision on eligibility. Disagreement among the reviewers will be discussed. If agreement cannot be reached, a third author (MGJ) will be consulted for arbitration. Reasons for excluding studies after full-text reading will be provided.

Data collection process:

Two reviewers (GS and JRI) will independently extract data from a pre-specified form (see Data items) in REDCap from each study found eligible for inclusion after full-text reading. If data reporting is incomplete, we will try to contact authors of the relevant studies to obtain data. A maximum of two attempts will be done to contact the authors by e-mail. If e-mails are not responded within 1 month from the first e-mail sent, the data field will be labelled as having no information. If the authors do not gain access to data, these will be extracted from figures and graphs if possible. If companion studies (multiple reports of the same study) with the same outcome of falls is found, data will be

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extracted from the primary publication of the study. Disagreement among the reviewers will be discussed. If agreement cannot be reached, a third author (MGJ) will be consulted for arbitration. The total number of times arbitration by a third author was required will be given.

Data items:

Data extraction will comply with Critical Appraisal and Data Extraction for Systematic Reviews of Prediction Modelling Studies (CHARMS) guidelines.[18] The following data will be extracted if possible:

- General study information:
  - Authors.
  - Year of publication.
  - Study design.
  - Type of prediction modelling study:
    - Developmental without external validation.
    - Developmental with external validation.
    - External model validation without model updating.
    - External model validation with model updating.
    - Others
  - Country of origin.
  - Setting where candidate predictors were measured.
  - Number of study centres.
  - Inclusion criteria.
  - Exclusion criteria.
  - Sample size.
  - Methods for participant recruitment/sampling:
    - Consecutive sampling.
    - Convenience sampling.
    - Probability sampling.
    - Others
  - Dates of participant recruitment.
  - Duration of follow-up.
  - How many participants completed follow-up percentage-wise?
- Participants:

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- Gender.
- Age.
- Fall history.

- Outcome:
  - Outcome definition.
    - Was it pre-specified?
  - Type of fall recording/method of outcome measurement.
  - Was the same outcome definition and recording method used in all participants (Yes/No/Not Available)?
  - Was the outcome assessor blinded towards predictors (Yes/No/Not Available)?
  - Number of falls.
  - Number of fallers.
  - Number of non-fallers.
  - Number of frequent fallers.
  - Fall-rate per person per year.
  - Summary of follow-up period.
- Candidate predictors:
  - Number of candidate predictors studied.
  - Definitions of candidate predictors.
  - Methods for measuring candidate predictors.
  - Number of outcomes (falls) in relation to number of candidate predictors (events per variable (EPV)).
- Missing data:
  - Number of participants with missing data (both predictors and outcomes) in total.
  - Did participants with missing data differ from those without missing data (Yes/No/Not Available)?
  - Number of participants with missing data in total for each predictor
  - Method of handling missing data.
    - Single imputation
    - Multiple imputation
    - Participants with missing data were excluded from the analysis (complete case analysis)

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- Others (comment).

- Were participants with missing data included in the model development, validation or updating (Yes/No/Not Available)?

- Model development (not relevant if the prediction modelling study does not include model development):
  - Type of model:
    - Linear regression
    - Logistic regression.
    - Survival analysis.
    - Others (comment).
  - Were assumptions for the model checked (Yes/No/Not available)?
  - Were assumptions for the model satisfied (Yes/No/Not available)?
  - Predictor selection methods for inclusion into the multivariable analysis:
    - All predictors were predetermined to be included in the analysis
    - Predictors were selected for inclusion based on univariate associations with the outcome
    - Others
  - Did any statistical transformation of candidate predictors occur (i.e. dichotomising a continuous or categorical variable) prior to inclusion in the multivariate modelling process (Yes/No/Not Available)?
    - If YES, what transformation procedure was applied?
  - Predictor selection methods during the multivariable modelling:
    - Full model approach (all predictors were predefined for the final model and no predictors were omitted).
    - Forward selection (candidate predictors were selected based on pre-specified criteria).
      - If forward selection was applied, which criteria/significance level were used?
        - P-value
        - Akaike's Information Criteria
        - R^2
        - Others

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- Backward elimination (all candidate predictors started in the model and were removed or kept based on a pre-specified criterion)
    - If backward elimination was applied, which criteria/ significance level were used?
        - P-value
        - Akaike's Information Criteria
        - R^2
        - Others
- Were shrinkage techniques applied (Yes/No/Not available)?
    - If YES, which procedure was applied?

- Model performance:
    - Overall measures of model performance
        - R^2
        - Brier Score
    - Discrimination:
        - Area Under Curve/*c*-statistic
        - D-statistic
        - Others
    - Calibration:
        - Calibration plot
        - Calibration intercept and slope
        - Calibration table
        - Hosmer-Lemeshow test
        - Observed:Expected Ratio
        - Others.
    - Classification:
        - Sensitivity
        - Specificity
        - Positive Predictive Value
        - Negative Predictive Value
        - Net reclassification index
        - Others

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- Was the cut-point:
  - Predefined/made a priori?
  - Derived from the dataset?

- Model evaluation:
  - **External validation:** Were the model performance measures based on separate external data (Y/N/NA)?
    - If YES, how was the dataset different from the developmental dataset:
      - Temporal/differed in time
      - Different geographical location
      - Different setting
      - Different investigator
      - Others
  - **Internal validation:** Were the model performance measures based on the developmental dataset (Y/N/NA)?
    - Which approach was chosen to evaluate model performance?
      - Split-sample validation
        - What was the percentage-wise allocation of participants?
        - Was the split random to model development and validation(Y/N/NA)?
      - Cross-validation
        - How many subsets were chosen?
      - Bootstrap validation
      - Jack-knife resampling
      - Others
      - None

- Model presentation:
  - What format did the study present their model in to permit calculations of individual risks?
    - Regression formula (comment)
    - Rounded scoring rules (comment)
    - Predefined risk groups (comment)

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- Were the risk-groups:
  - Predefined/made a priori?
  - Derived from the dataset?
- Nomogram
- Score chart
- Others
- None

- Sources of funding in the individual study.

Outcomes and prioritisation

Main outcomes:

The main outcome of this systematic review is to describe model performance. Secondary outcomes are to describe the following characteristics: Study setup, participants, final model predictors, outcomes together with model development, -evaluation and -presentation.

Risk of bias

The Prediction study Risk Of Bias Assessment Tool (PROBAST)[19] will be applied for risk of bias assessment. Two reviewers (GS and JRI both with no prior experience in risk of bias assessments) will independently assess for risk of bias. These will not be blinded to study titles or authors. If reporting is incomplete in order to make a complete assessment, we will try to contact authors of the concerned study. A maximum of two attempts will be done to contact the authors by e-mail. If e-mails are not responded within 1 month from the first e-mail sent, the data field will be labelled as "Unclear". Disagreement among the reviewers will be discussed. If agreement still is not reached, a third author (MGJ) will be consulted for arbitration. The total number of times arbitration by a third author was required will be given. Each domain rating will be reported instead of a summative score of all domains. Previously, one review on fall prediction models assessed reporting in included studies to be poor[12]. However, no standardised method of evaluating reporting in studies was reported. Thus, as a supplement to the risk of bias assessments, the newly published TRIPOD adherence tool[26] will be used to assess adherence to reporting guidelines for prediction modelling studies.

Data synthesis

Meta-analysis will not be considered due to this systematic review merely being descriptive. In the qualitative synthesis, information will be presented in text, figures, and tables of the included studies. Reporting of studies will be presented in tables by their publication year. Final model predictors will be presented in main categories in a figure.

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Meta-bias

Presence of outcome reporting bias will be investigated by comparing the studies with their respective protocol if available. The following aspects will be considered:

- Was publication of the protocol done before recruitment of patients?
- Was the intended outcome in the protocol the same in the published study?

Confidence in cumulative estimate

Assessment of strength of evidence will be made using the PROBAST tool.

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Appendix 2: PRISMA checklist

See tables on the next page. Fields pertaining to meta-analyses have been labelled as not available (NA) since meta-analyses was not performed.

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Section/topic	#	Checklist item	Reported on page #
TITLE			
Title	1	Identify the report as a systematic review, meta-analysis, or both.	#2
ABSTRACT			
Structured summary	2	Provide a structured summary including, as applicable: background; objectives; data sources; study eligibility criteria, participants, and interventions; study appraisal and synthesis methods; results; limitations; conclusions and implications of key findings; systematic review registration number.	#4
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of what is already known.	#6
Objectives	4	Provide an explicit statement of questions being addressed with reference to participants, interventions, comparisons, outcomes, and study design (PICOS).	#6
METHODS			
Protocol and registration	5	Indicate if a review protocol exists, if and where it can be accessed (e.g., Web address), and, if available, provide registration information including registration number.	#6
Eligibility criteria	6	Specify study characteristics (e.g., PICOS, length of follow-up) and report characteristics (e.g., years considered, language, publication status) used as criteria for eligibility, giving rationale.	#6-8
Information sources	7	Describe all information sources (e.g., databases with dates of coverage, contact with study authors to identify additional studies) in the search and date last searched.	#8
Search	8	Present full electronic search strategy for at least one database, including any limits used, such that it could be repeated.	#8 + Appendix 3
Study selection	9	State the process for selecting studies (i.e., screening, eligibility, included in systematic review, and, if applicable, included in the meta-analysis).	#8-9 + Appendix 4
Data collection process	10	Describe method of data extraction from reports (e.g., piloted forms, independently, in duplicate) and any processes for obtaining and confirming data from investigators.	#9
Data items	11	List and define all variables for which data were sought (e.g., PICOS, funding sources) and any assumptions and simplifications made.	#9 + Appendix 5
Risk of bias in individual studies	12	Describe methods used for assessing risk of bias of individual studies (including specification of whether this was done at the study or outcome level), and how this information is to be used in any data synthesis.	#9-10
Summary measures	13	State the principal summary measures (e.g., risk ratio, difference in means).	#10

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Synthesis of results	14	Describe the methods of handling data and combining results of studies, if done, including measures of consistency (e.g., I^2) for each meta-analysis.	**NA**

Section/topic	#	Checklist item	Reported on page #
Risk of bias across studies	15	Specify any assessment of risk of bias that may affect the cumulative evidence (e.g., publication bias, selective reporting within studies).	#10
Additional analyses	16	Describe methods of additional analyses (e.g., sensitivity or subgroup analyses, meta-regression), if done, indicating which were pre-specified.	**NA**
RESULTS			
Study selection	17	Give numbers of studies screened, assessed for eligibility, and included in the review, with reasons for exclusions at each stage, ideally with a flow diagram.	#10 + Fig 1 + Appendix 6-8
Study characteristics	18	For each study, present characteristics for which data were extracted (e.g., study size, PICOS, follow-up period) and provide the citations.	#10-12 + Fig 2 + Appendix 9-10
Risk of bias within studies	19	Present data on risk of bias of each study and, if available, any outcome level assessment (see item 12).	#12 + Table 1
Results of individual studies	20	For all outcomes considered (benefits or harms), present, for each study: (a) simple summary data for each intervention group (b) effect estimates and confidence intervals, ideally with a forest plot.	#12
Synthesis of results	21	Present results of each meta-analysis done, including confidence intervals and measures of consistency.	**NA**
Risk of bias across studies	22	Present results of any assessment of risk of bias across studies (see Item 15).	#12
Additional analysis	23	Give results of additional analyses, if done (e.g., sensitivity or subgroup analyses, meta-regression [see Item 16]).	**NA**
DISCUSSION			
Summary of evidence	24	Summarize the main findings including the strength of evidence for each main outcome; consider their relevance to key groups (e.g., healthcare providers, users, and policy makers).	#12-13
Limitations	25	Discuss limitations at study and outcome level (e.g., risk of bias), and at review-level (e.g., incomplete retrieval of identified research, reporting bias).	#13
Conclusions	26	Provide a general interpretation of the results in the context of other evidence, and implications for future research.	#13-15
FUNDING			

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Funding	27	Describe sources of funding for the systematic review and other support (e.g., supply of data); role of funders for the systematic review.	#10

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Appendix 3: search strategy

Pubmed.gov

((((((home-dwell*[tw] OR "Independent Living"[Mesh] OR Independent*[tw] OR community-dwell*[tw] OR home-based*[tw] OR community-living*[tw]))

AND ("Aged"[Mesh] OR aged[tw] OR senior*[tw] OR elder*[tw] OR old[tw] OR older[tw]))) AND ("Accidental Falls"[Mesh] OR fall*[Text Word])))

AND ((((Validat*[tw] OR Predict*[ti] OR Rule*[tw]) OR (Predict*[tw] AND (Outcome*[tw] OR Risk*[tw] OR Model*[tw])) OR ((History[tw] OR Variable*[tw] OR Criteria [tw]OR Scor*[tw] OR Characteristic*[tw] OR Finding*[tw] OR Factor*[tw]) AND (Predict*[tw] OR Model*[tw] OR Decision*[tw] OR Identif*[tw] OR Prognos*[tw])) OR (Decision*[tw] AND (Model*[tw] OR Clinical*[tw] OR "Logistic Models"[MESH])) OR (Prognostic[tw] AND (History[tw] OR Variable*[tw] OR Criteria[tw] OR Scor*[tw] OR Characteristic*[tw] OR Finding*[tw] OR Factor*[tw] OR Model*[tw])))) OR ((((((((((("ROC Curve"[Mesh]) OR stratificat*[tw]) OR discriminat*[tw]) OR c statistic*[tw]) OR Area under the curve*[tw]) OR AUC[tw]) OR Calibrat*[tw]) OR Indices[tw]) OR Algorithm*[tw]) OR Multivariable*[tw])) OR ((Predict*[tw] OR Predictive value of tests[mh] OR Scor*[tw] OR Observ*[tw] OR Observer variation[mh]))))

Embase:

('falling'/exp OR fall*:ti,ab,kw)

AND ('aged'/exp OR aged:ti,ab,kw OR senior*:ti,ab,kw OR elder*:ti,ab,kw OR old:ti,ab,kw OR older:ti,ab,kw)

AND (validat* OR rule* OR (predict* AND (outcome* OR risk* OR model*)) OR ((history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor*)

AND (predict* OR model* OR decision* OR identif* OR prognos*)) OR (decision* AND (model* OR clinical* OR 'statistical model'/exp)) OR (prognostic AND (history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor* OR model*)) OR 'receiver operating characteristic'/exp OR stratificat* OR discriminat* OR 'c statistic*' OR 'area under the curve*' OR auc OR calibrat* OR indices OR algorithm* OR multivariable* OR predict* OR 'predictive value'/exp OR scor* OR observ* OR 'observer variation'/exp) AND ('community living'/exp OR 'at home':ti,ab,kw OR (((community OR home OR independent*) NEAR/3 (dwell* OR based OR live OR living)):ti,ab,kw) OR 'home accident'/exp OR 'community dwelling person'/exp)

CINAHL:

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((MH "Community Living") OR (MH "Assisted Living") OR ((community OR home OR independent*) N3 (dwell* OR based OR live OR living)) OR (MH "Accidents, Home") OR at home) AND ((MH "Accidental Falls") OR fall*)

AND ((MH "Aged") OR (MH "Aged, 80 and Over") OR (MH "Frail Elderly") OR aged OR senior* OR elder* OR old OR older)

AND (validat* OR rule* OR (predict* AND (outcome* OR risk* OR model*)) OR ((history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor*) AND (predict* OR model* OR decision* OR identif* OR prognos*)) OR (decision* AND (model* OR clinical* OR (MH "Models, Statistical"))) OR (prognostic AND (history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor* OR model*)) OR (MH "ROC Curve") OR stratificat* OR discriminat* OR 'c statistic*' OR 'area under the curve*' OR auc OR calibrat* OR indices OR algorithm* OR multivariable* OR predict* OR (MH "Predictive Value of Tests") OR scor* OR observ*)

Psycinfo
http://psycnet.apa.org.auh.aub.aau.dk/permalink/19512998-9a97-e90a-722c-bd157326fa55

((Any Field: (validat*) OR Any Field: (rule*) OR (Any Field: (predict*) AND (Any Field: (outcome*) OR Any Field: (risk*) OR Any Field: (model*))) OR ((Any Field: (history) OR Any Field: (variable*) OR Any Field: (criteria) OR Any Field: (scor*) OR Any Field: (characteristic*) OR Any Field: (finding*) OR Any Field: (factor*)) AND (Any Field: (predict*) OR Any Field: (model*) OR Any Field: (decision*) OR Any Field: (identif*) OR Any Field: (prognos*))) OR (Any Field: (decision*) AND (Any Field: (model*) OR Any Field: (clinical*))) OR (Any Field: (prognostic) AND (Any Field: (history) OR Any Field: (variable*) OR Any Field: (criteria) OR Any Field: (scor*) OR Any Field: (characteristic*) OR Any Field: (finding*) OR Any Field: (factor*) OR Any Field: (model*))) OR Any Field: (stratificat*) OR Any Field: (discriminat*) OR Any Field: ('c statistic*') OR Any Field: ('area under the curve*') OR Any Field: (ROC) OR Any Field: (auc) OR Any Field: (calibrat*) OR Any Field: (indices) OR Any Field: (algorithm*) OR Any Field: (multivariable*) OR Any Field: (predict*) OR Any Field: (scor*) OR Any Field: (observ*))))

AND ((((((Any Field: ('at home'))) OR ((((Any Field: (community))) OR ((Any Field: (home))) OR ((Any Field: (independent*)))) NEAR/3 (((Any Field: (dwell*))) OR ((Any Field: (based))) OR ((Any Field: (live))) OR ((Any Field: (living)))))) OR ((((IndexTermsFilt: ("Home Accidents"))))))

AND (((Any Field: (aged))) OR ((Any Field: (elder*))) OR ((Any Field: (old))) OR ((Any Field: (older))) OR ((Any Field: (senior*))))

AND (((IndexTermsFilt: ("Falls"))) OR ((Any Field: (fall*)))))

Cochrane Library:
ID Search

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#1 MeSH descriptor: [Accidental Falls] explode all trees
#2 fall*:ti,ab,kw
#3 #1 OR #2
#4 MeSH descriptor: [Aged] explode all trees
#5 aged:ti,ab,kw
#6 senior*:ti,ab,kw
#7 elder*:ti,ab,kw
#8 old:ti,ab,kw
#9 older:ti,ab,kw
#10 #4 OR #5 OR #6 OR #7 OR #8 OR #9
#11 MeSH descriptor: [Independent Living] this term only
#12 "at home":ti,ab,kw
#13 MeSH descriptor: [Accidents, Home] explode all trees
#14 #3 or #13
#15 (((community OR home OR independent*) NEAR/3 (dwell* OR based OR live OR living)):ti,ab,kw)
#16 #11 OR #12 OR #13 OR #15
#17 #14 AND #16 AND #10
#18 MeSH descriptor: [Logistic Models] explode all trees
#19 MeSH descriptor: [ROC Curve] explode all trees
#20 MeSH descriptor: [Predictive Value of Tests] explode all trees
#21 MeSH descriptor: [Observer Variation] explode all trees
#22 ((((Validat* OR Predict*:ti OR Rule*) OR (Predict* AND (Outcome* OR Risk* OR Model*)) OR ((History OR Variable* OR Criteria OR Scor* OR Characteristic* OR Finding* OR Factor*) AND (Predict* OR Model* OR Decision* OR Identif* OR Prognos*)) OR (Decision* AND (Model* OR Clinical* OR #18)) OR (Prognostic AND (History OR Variable* OR Criteria OR Scor* OR Characteristic* OR Finding* OR Factor* OR Model*)))) OR ((((((((((#19) OR stratificat*) OR discriminat*) OR c statistic*) OR Area under the curve*) OR AUC) OR Calibrat*) OR Indices) OR Algorithm*) OR Multivariable*)) OR ((Predict* OR #20 OR Scor* OR Observ* OR #21))))
#23 #17 AND #22

Web of Science:
#1 ts=fall*
#2 ts=(aged OR senior* OR elder* OR old OR older)
#3 ts=((validat* OR rule* OR (predict* AND (outcome* OR risk* OR model*)) OR ((history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor*) AND (predict* OR model* OR decision* OR identif* OR prognos*)) OR (decision* AND (model* OR clinical* OR "statistical model")) OR (prognostic AND (history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR

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factor* OR model*)) OR ("receiver operating characteristic" OR stratificat* OR discriminat* OR "c statistic" OR "area under the curve*" OR auc OR calibrat* OR indices OR algorithm* OR multivariable* OR predict* OR "predictive value" OR scor* OR observ* OR "observer variation")))
#4 ts=("community living" OR "at home" OR ((community OR home OR independent*) NEAR/3 (dwell* OR based OR live OR living)) OR "home accident" OR "community dwelling person")
#4 AND #3 AND #2 AND #1

PROSPERO:
https://www.crd.york.ac.uk/prospero

Clinicaltrials.gov:
https://clinicaltrials.gov/ct2/results?cond=falling+and+elderly+or+older+adults&term=&cntry=&state=&city=&dist=

WHO International Clinical Trials Registry Platform:
http://apps.who.int/trialsearch/

Open Grey:
http://www.opengrey.eu/search/request?q=Falling+AND+Elderly+OR+Older+adults

GIN:
https://www.g-i-n.net/library/international-guidelines-library/international-guidelines-library/@@guideline_search_results?basic-searchable-text=falling+and+elderly+or+older+adults&type=basic&action=Search&advanced-authors=&diseases=&meshterm=&search=

NICE:
https://www.nice.org.uk/Search?q=Falling+AND+Elderly+OR+Older+adults

CRD/HTA:
https://www.crd.york.ac.uk/CRDWeb/

SIGN:
https://www.sign.ac.uk/our-guidelines.html

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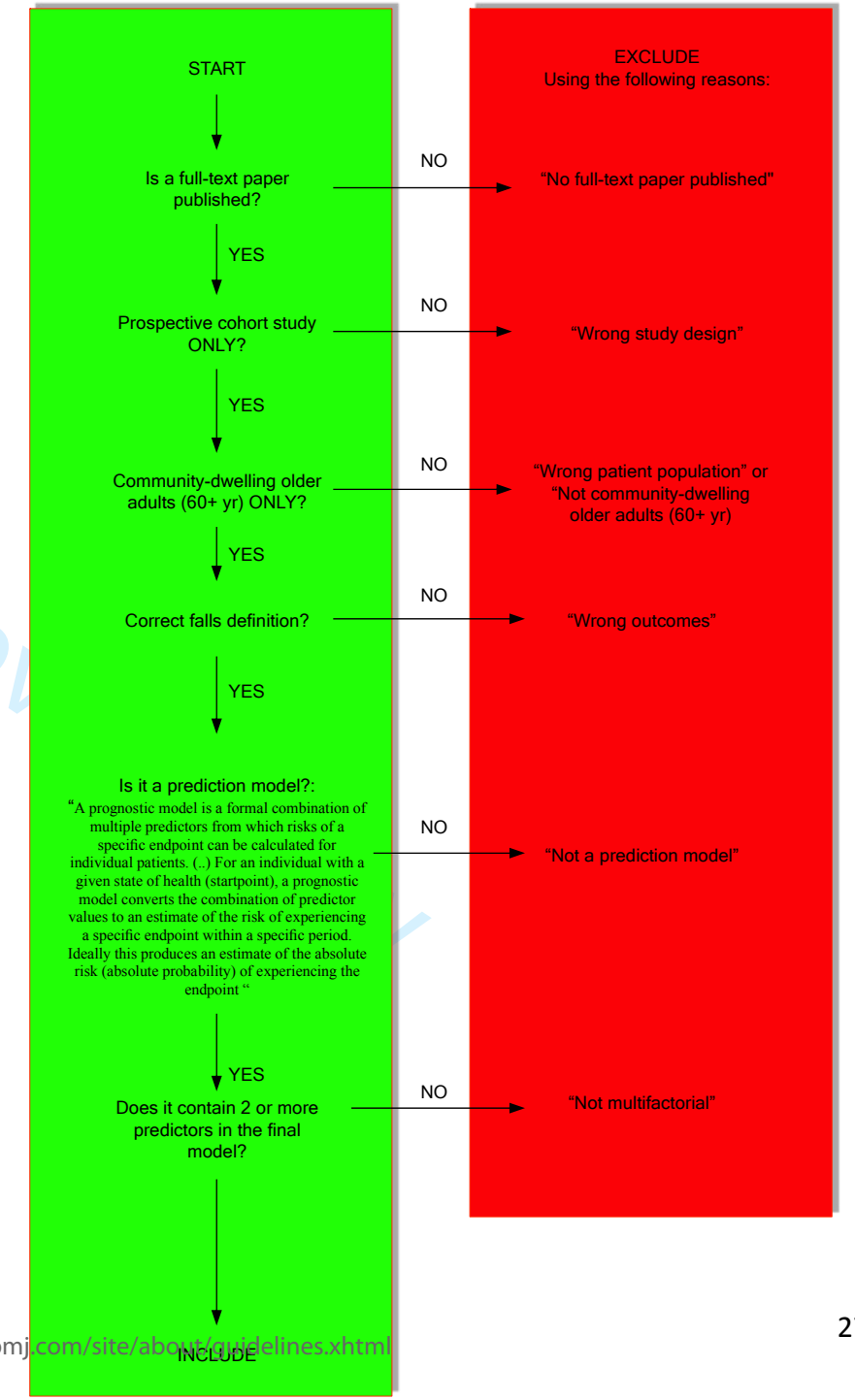
Appendix 4: prioritised list for exclusion reasons



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Appendix 5: data extraction items

- General study information:
    - Authors
    - Year of publication
    - Study design
    - Type of prediction modelling study:
        - Developmental without external validation
        - Developmental with external validation
        - External model validation without model updating
        - External model validation with model updating
        - Others
    - Country of origin
    - Setting where candidate predictors were measured
    - Number of study centres
    - Inclusion criteria
    - Exclusion criteria
    - Sample size
    - Methods for participant recruitment/sampling:
        - Consecutive sampling
        - Convenience sampling
        - Probability sampling
        - Others

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- Dates of participant recruitment
- Duration of follow-up
- How many participants completed follow-up percentage-wise?

- Participants:
  - Gender
  - Age
  - Fall history
- Outcome:
  - Outcome definition
    - Was it pre-specified?
  - Type of fall recording/method of outcome measurement
  - Was the same outcome definition and recording method used in all participants (Yes/No/Not Available)?
  - Was the outcome assessor blinded towards predictors (Yes/No/Not Available)?
  - Number of falls
  - Number of fallers
  - Number of non-fallers
  - Number of frequent fallers
  - Fall-rate per person per year
  - Summary of follow-up period
- Candidate predictors:
  - Number of candidate predictors studied
  - Definitions of candidate predictors
  - Methods for measuring candidate predictors

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- Number of outcomes (falls) in relation to number of candidate predictors (events per variable (EPV))

- Missing data:
  - Number of participants with missing data (both predictors and outcomes) in total
  - Did participants with missing data differ from those without missing data (Yes/No/Not Available)?
  - Number of participants with missing data in total for each predictor
  - Method of handling missing data
    - Single imputation
    - Multiple imputation
    - Participants with missing data were excluded from the analysis (complete case analysis)
    - Others (comment)
  - Were participants with missing data included in the model development, validation or updating (Yes/No/Not Available)?
- Model development (not relevant if the prediction modelling study does not include model development):
  - Type of model:
    - Linear regression
    - Logistic regression
    - Survival analysis
    - Others (comment)
  - Were assumptions for the model checked (Yes/No/Not available)?
  - Were assumptions for the model satisfied (Yes/No/Not available)?
  - Predictor selection methods for inclusion into the multivariable analysis:
    - All predictors were predetermined to be included in the analysis
    - Predictors were selected for inclusion based on univariate associations with the outcome
    - Others

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- Did any statistical transformation of candidate predictors occur (i.e. dichotomising a continuous or categorical variable) prior to inclusion in the multivariate modelling process (Yes/No/Not Available)?
  - If YES, what transformation procedure was applied?
- Predictor selection methods during the multivariable modelling:
  - Full model approach (all predictors were predefined for the final model and no predictors were omitted)
  - Forward selection (candidate predictors were selected based on pre-specified criteria)
    - If forward selection was applied, which criteria/significance level were used?
      - P-value
      - Akaike's Information Criteria
      - R^2
      - Others
  - Backward elimination (all candidate predictors started in the model and were removed or kept based on a pre-specified criterion)
    - If backward elimination was applied, which criteria/ significance level were used?
      - P-value
      - Akaike's Information Criteria
      - R^2
      - Others
- Were shrinkage techniques applied (Yes/No/Not available)?
  - If YES, which procedure was applied?

- Model performance:
  - Overall measures of model performance
    - R^2

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        - Brier Score
    - Discrimination:
        - Area Under Curve/*c*-statistic
        - D-statistic
        - Others
    - Calibration:
        - Calibration plot
        - Calibration intercept and slope
        - Calibration table
        - Hosmer-Lemeshow test
        - Observed:Expected Ratio
        - Others
    - Classification:
        - Sensitivity
        - Specificity
        - Positive Predictive Value
        - Negative Predictive Value
        - Net reclassification index
        - Others
        - Was the cut-point:
            - Predefined/made a priori?
            - Derived from the dataset?
- Model evaluation:

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- **External validation:** Were the model performance measures based on separate external data (Y/N/NA)?
  - If YES, how was the dataset different from the developmental dataset:
    - Temporal/differed in time
    - Different geographical location
    - Different setting
    - Different investigator
    - Others
- **Internal validation:** Were the model performance measures based on the developmental dataset (Y/N/NA)?
  - Which approach was chosen to evaluate model performance?
    - Split-sample validation
      - What was the percentage-wise allocation of participants?
      - Was the split random to model development and validation(Y/N/NA)?
    - Cross-validation
      - How many subsets were chosen?
    - Bootstrap validation
    - Jack-knife resampling
    - Others
    - None

- Model presentation:
  - What format did the study present their model in to permit calculations of individual risks?
    - Regression formula (comment)
    - Rounded scoring rules (comment)
    - Predefined risk groups (comment)

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- Were the risk-groups:
  - Predefined/made a priori?
  - Derived from the dataset?
- Nomogram
- Score chart
- Others
- None

- Sources of funding in the individual study

We used the following definitions of developmental studies and validation studies:

- Developmental study: “Model development studies aim to derive a prediction model by selecting predictors and combining them into a multivariable model”[1].
- Validation study: A fully specified existing prognostic model including both predictors and their coefficients [2].
  - Studies with prespecified predictors, but without any coefficients were classified as developmental studies.

1 Collins GS, Reitsma JB, Altman DG, *et al.* Transparent reporting of a multivariable prediction model for individual prognosis or diagnosis (TRIPOD): the TRIPOD Statement. *BMC Med* 2015;**13**:1. doi:10.1186/s12916-014-0241-z

2 Altman DG, Vergouwe Y, Royston P, *et al.* Prognosis and prognostic research: Validating a prognostic model. *BMJ* 2009;**338**:1432–5. doi:10.1136/bmj.b605

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Appendix 6: studies excluded during screening of titles and abstracts:

See separate Excel file: “Appendix 6-8+10”.

Appendix 7: possibly relevant studies in other languages excluded during screening of titles and abstract:

See separate Excel file: “Appendix 6-8+10”.

Appendix 8: studies excluded during full-text screening.

See separate Excel file: “Appendix 6-8+10”.

Appendix 9: supplementary table 1 with characteristics of included studies

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Supplementary Table 1 Characteristics of included studies

First author, year, country, study type	Study characteristics (sample size, age distribution, % female, % of sample with a prior fall history)	Outcomes (n; %) and length of follow-up	Statistical model and validation technique	Final models and their presentation (model no. (outcome): predictors with/without weights)	Model performance
Maki et al. [1], 1994, Canada Development study study	n = 100 Age, mean (SD): 83 (6) years Female: 83% Fall history: 46.8%	Any falls (59; 59%) Recurrent falls (23; 23%) Follow-up: 12 months	Logistic regression Cross-validation (n-1)	**Model** 1 (any falls): Spontaneous mediolateral sway (root-mean-square) with eyes blindfolded; induced sway in anterior-posterior direction with eyes open (mean COP location / length of base-of-support) **Model** 2 (recurrent falls): Induced sway in mediolateral direction with eyes blindfolded (mean coherence of the input-output model); induced sway in mediolateral direction with eyes blindfolded (Relative COP overshoot in the predicted transient response)	**Model** 1: AUC (SE): 0.76 (0.05) Calibration: no information SN: 78% (43/55) SP: 50% (18/36) **Model** 2: AUC (SE): 0.87 (0.05) Calibration: no information SN: 53% (9/17) SP: 89% (31/35)
Brauer et al. [2], 2000, Australia Developmental study	n = 100 Age, mean (SD): 71 (5) years Female: 100% Fall history: 35%	Any falls (35; 35%) Follow-up: 6 months	Logistic regression Cross-validation (no information on subsets)	**Model** 1 (any falls): Gluteus medius onset time; Movement time in a high preparation step task; Step time in a high preparation step task; Maximum COP excursion when moving to the right LOS; COP maximum mediolateral velocity; Total distance moved in quiet stance **Model** 2 (any falls): Movement time in a high preparation step task; step time in a high preparation step task; total time in a high preparation step task; movement time in a neutral preparation step task; step time in a neutral preparation step task; total time in a neutral preparation step task **Model** 3 (any falls): Gluteus medius in a neutral preparation step task; tensor fascia latae in a neutral preparation step task; hip adductors in a neutral preparation step task; gastrocnemius onset times in a neutral preparation step task; gluteus medius in a high preparation step task; tensor fascia latae in a high	**Model** 1: AUC: no information Calibration: no information SN: 51% SP: 91% Total predictive ability: 77% **Model** 2: AUC: no information Calibration: no information SN: 34% SP: 89% Total predictive ability: 70% **Model** 3: AUC: no information Calibration: no information SN: 23% SP: 88% Total predictive ability: 65%

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				preparation step task; hip adductors in a high preparation step task; gastrocnemius onset times in a high preparation step task **Model** 4 (any falls): COP maximum mediolateral velocity with eyes open; COP maximum mediolateral velocity with eyes closed; COP position in quiet stance with eyes open; COP position in quiet stance with eyes closed; COP total distance moved in quiet stance with eyes open; COP total distance moved in quiet stance with eyes closed **Model** 5 (any falls): COP maximum excursion when moving to the right LOS; COP maximum excursion when moving to the left LOS; COP maximum excursion when moving to the anterior LOS; COP maximum excursion when moving to the posterior LOS **Model** 6 (any falls): Left Functional Reach; Right Functional Reach; Right Lateral Reach; Left Lateral Reach; Step-Up number; Berg Balance Scale score **Model** 7 (any falls): Left Functional Reach; Right Functional Reach; Right Lateral Reach; Left Lateral Reach; Step-Up number; Berg Balance Scale score; Gluteus medius onset time; Movement time in a high preparation step task; Step time in a high preparation step task; Maximum COP excursion when moving to the right LOS; COP maximum mediolateral velocity; Total distance moved in quiet stance	**Model** 4: AUC: no information Calibration: no information SN: 29% SP: 88% Total predictive ability: 67% **Model** 5: AUC: no information Calibration: no information SN: 6% SP: 97% Total predictive ability: 65% **Model** 6: AUC: no information Calibration: no information SN: 12% SP: 95% Total predictive ability: 66% **Model** 7: AUC: no information Calibration: no information SN: 59% SP: 86% Total predictive ability: 77%
Tromp et al. [3], 2001, The Netherlands, Development study	n = 1,374 Age, mean (SD): 75.2 (6.5) years Age, range: 64.8-88.6 years	Any falls (428; 31.1%) Recurrent falls (147; 10.7%) Follow-up: 12 months	Logistic regression No information on model validation	**Model** 1 (any falls): Regression table with regression coefficients: constant: no information; previous falls: 0.90; urinary incontinence: 0.46; visual impairment: 0.44; use of benzodiazepines: 0.44 Score chart - previous falls: 5; urinary incontinence: 2; visual impairment: 2; use of benzodiazepines: 2	**Model** 1: AUC: 0.65 Calibration: no information

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	Of the 1,285 with complete follow-up, female: 51% Fall history: 31%			**Model** 2 (recurrent falls): Regression table with regression coefficients: constant: no information; previous falls: 0.99; urinary incontinence: 0.53; visual impairment: 0.82; use of benzodiazepines: 0.54 Score chart: previous falls: 5; urinary incontinence: 3; visual impairment: 4; use of benzodiazepines: 3	**Model** 2: AUC: 0.71 Calibration: no information Cut-off: 7 points SN: 54% SP: 79% PPV (CI): 24.9% (22.5-27.3%) NPV (CI): 93% (91.6-94.4%)
Stalenhoef et al. [4], 2002, The Netherlands, Development study	n = 302 Of the 287 with complete follow-up, age, mean (SD): - Female: 78.5 (5.2) years - Male: 77.2 (4.9) years Of the 287 with complete follow-up, female: 60% Fall history: 66%	Recurrent falls (46; 15.2%) Follow-up: 9 months	Logistic regression No information on model validation	**Model** 1 (recurrent falls): Regression table including regression coefficients with SE: constant: -2.28; female gender: -0.39 (0.4); age >= 80 years: 0.04 (0.39); falls >= 2 in previous year: 1.14 (0.39); depression - SCL90 >= 22: 0.78 (0.37); hand dynamometry (men <= 22 kg or women: <= 12 kg: 1.14 (0.38); postural sway abnormal: 1.36 (0.58) Rounded scoring rule - men: age >= 80 years: was not included due to low impact; falls >= 2 in previous year: 6; depression - SCL90 >= 22 4; hand dynamometry (Men <= 22 kg or Women: <= 12 kg): 6; postural sway abnormal: 7 Rounded scoring rule – women: age >= 80 years: Was not included due to low impact; falls >= 2 in previous year: 4; depression - SCL90 >= 22: 2; hand dynamometry (men <= 22 kg or women: <= 12 kg): 4; postural sway abnormal: 5	**Model** 1: AUC: 0.79 Calibration: "The comparison of the percentages predicted probability with the percentage of observed recurrent fallers showed a general agreement. The predicted values of the model, calculated according to the Hosmer Lemeshow goodness of fit, showed good fit." Cut-off: 0.30 SN: 59% SP: 87% PPV: 52% NPV: 90%
Stel et al. [5], 2003, The Netherlands, Development study	n = 1,365 Age, mean (SD): 75.3 (6.4) years Age, range: 64.8-88.6 years Female: 51% Fall history: 31%	Recurrent falls (337; 24.7%) Follow-up: - Primary length of follow-up: 3 years - Secondary length of follow-up: 1 year.	Tree-structured survival analysis No information on model validation	**Model** 1 (recurrent falls): Classification tree with a follow-up of 3 years: fall history, function limitations, dizziness, performance test score, grip strength, alcohol consumption, pain, educational level, and physical activity **Model** 2 (recurrent falls): Classification tree with a follow-up of 1 year: fall history, function limitations, and grip strength	**Model** 1: AUC: no information Calibration: no information **Model** 2: AUC: no information Calibration: no information
Boulgarides et al. [6], 2003, USA,	n = 106	Recurrent falls (20; 18.9%)	Logistic regression	**Model** 1 (recurrent falls): Regression table with coefficients and SE:	**Model** 1: AUC: no information

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Development study	Age, mean (SD): 74.02 (5.64) years Age range: 65-90 years Of 99 participants included in analysis: female: 61% Fall history: 50.5%	Follow-up: 12 months	No information on model validation	constant: no information; postural sway while standing on a firm surface with eyes closed: 1.786 (1.332); age: 0.072 (0.048); sex: 0.822 (0.540)	Calibration: no information % Correct prediction: 80.8%, though only predicted 2/20 of multiple fallers SN: 10% SP: 98.7%
Nandy et al. [7], 2004, UK, Development study	n = 510 Of the 345 with complete follow-up, age, mean (SD): 74.4 (6.4) years Of the 345 with complete follow-up, female: 55% Fall history: 25%	Any falls (no information) Follow-up: 6 months	Only SN, SP, PPV, NPV, and Youden's index were calculated No information on model validation	**Model** 1 (any falls): Three or more of the following risk factors: history of falling in the previous year, taking four or more prescribed medications, history of stroke or Parkinson's disease, and reported problems with balance and loss of proximal muscle strength	**Model** 1: AUC: no information Calibration: no information SN: 0.42 (0.32-0.54) SP: 0.92 (0.88-0.94) PPV: 0.57 (0.43-0.69) NPV: 0.86 (0.83-0.89) Youden's Index: 0.339 (0.185-0.493)
Pluijm et al. [8], 2006, The Netherlands, Development study	n = 1,365 Age, mean (SD): 75.3 (6.4) years Age, range: 64.8-88.6 years Female: 51.1% Fall history: 14.2%	Recurrent falls (457; 33.5%) Follow-up: - Primary: 3 years - Secondary: 1 year	Logistic regression No information on model validation	**Model** 1 (recurrent falls within 3 years): Regression table with regression coefficients: constant: - 2.19; two or more previous falls: 0.71; dizziness: 0.77; functional limitations: 0.53; weak grip strength: 0.55; low body weight: 0.37; fear of falling: 0.34; the presence of dogs/cats in the household: 0.40; a high education level: 0.21; drinking of 18 or more alcoholic consumptions per week: 0.11; interaction term (high education × 18 or more alcohol consumptions per week): 0.86; interaction term (two or more previous falls × fear of falling): 0.83 Score chart: two or more previous falls: 4; dizziness: 4; functional limitations: 3; weak grip strength: 3; low body weight: 2; fear of falling: 2; the presence of dogs/cats in the household: 2; a high education level: 1; drinking 18 or more alcoholic consumptions per week: 1; interaction term (high education × 18 or more alcohol consumptions per week): 4; interaction term (two or more previous falls × fear of falling): 4 **Model** 2 (recurrent falling within 1 year): Regression table with regression coefficients: constant: - 3.13; two or more previous falls: 0.64; dizziness: 0.52; functional limitations: 0.39;	**Model** 1: AUC (CI): 0.71 (0.67-0.74) Calibration: The Hosmer-Lemeshow goodness-of-fit test for the multiple logistic regression was not significant (p=0.56), indicating that the model fits the data well Cut-off: 5 points SN: 59% SP: 71.4% PPV: 38.6% NPV: 85.1% **Model** 2: AUC: (CI): 0.72 (0.67- 0.77) Calibration: The Hosmer-Lemeshow goodness-of-fit test for

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				weak grip strength: 0.65; low body weight: 0.32; fear of falling: 0.09; the presence of dogs/cats in the household: 0.81; a high educational level: 0.08; drinking of 18 or more alcoholic consumptions per week: - 0.15; interaction term (high education × 18 or more alcohol consumptions per week): 0.87; interaction term (two or more previous falls × fear of falling): 1.15	the multiple logistic regression was not significant (p=0.94), indicating that the model fits the data well
Okochi et al. [9], 2006, Japan, Development study	n = 1,734 Age, mean (SD): 75.8 (6.8) years Female: no information Fall history: 16%	Any falls (208; 12.0%) Follow-up: 6 months	Logistic regression Split-sample validation (random split, 50%/50%)	**Model** 1 (any falls): Scoring system from 0-13 based on the odds ratio at an integer level from logistic regression: history of falls - probable score 0/5; do you feel your walking speed has declined recently - probable score 0/2; do you use cane when you walk - probable score 0/2; is your back bended: probable score 0/2; do you take more than five kinds of prescription medicines - probable score 0/2	**Model** 1: AUC (95% CI): 0.74 (0.69-0.79) Calibration: no information Cut-off: 6 SN: 68% SP: 70% PPV: 27.9% Negative predictive power: 93%
Lindemann et al. [10], 2008, Germany, Development study	n = 65 Age, mean (SD): 67.7 (6.0) years Female: 57% Fall history: 45%	Any falls (30; 46.2%) Follow-up: 12 months	The cut-off values for differentiating between persons, who fell and persons who did not, were defined for each parameter as the point on the Receiver Operating Characteristic curve with the minimal Euclidian distance to the point (0/1) No information on model validation	**Model** 1 (any falls): Adjusted mean valid step length (cut-off: 64% of body height), and at least one fall in previous year **Model** 2: (any falls): Adjusted max. valid step length (cut-off: 66% of body height), and at least one fall in previous year	**Model** 1: AUC and calibration: author response: "The information in the paper is the only we can provide. New calculations are not possible" SN (CI): 93% (86.7-100) SP (CI): 54% (40.5-67.1) PPV (CI): 70% (57.8-82.2) NPV (CI): 88% (78.7-96.3) +LR: 2.0 -LR: 0.1 **Model** 2: AUC: no information Calibration: no information SN (CI): 90% (82-98) SP (CI): 58% (44.5-70.9) PPV (CI): 71% (58.9-83.2) NPV (CI): 83% (73.4-93.3) +LR: 2.1 -LR: 0.2
Lamb et al. [11], 2008, USA, Development study	n = 1,002 Age, mean (SD): 78 (8.1) years	Any falls (346; 34.5%) Follow-up:	Tree-based classification	**Model** 1 (any falls): Decision tree with self-report algorithm: how many falls have you had in the last year?; how often do you have problems balancing while	**Model** 1: AUC: no information Calibration: no information

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	Of the 885 included, female: 100% Of 830 included, fall history: 34%	12 months	Cross-validation (20 subsets)	walking?; how much difficulty do you have with activities of daily living? **Model** 2 (any falls): Decision tree with self-report and performance item algorithm: how many falls have you had in the last year?; how often do you have problems balancing while walking?; knee extensor strength test; 4-metre usual gait speed; Body Mass Index	Fall probability threshold: >= 0.34; SN: 0.59; SP: 0.64; +LR: 1.64; -LR: 0.64; Diagnostic Odds Ratio: 2.56 Fall probability threshold: >= 0.44; SN: 0.46; SP: 0.77; +LR: 2.00; -LR: 0.70; Diagnostic Odds Ratio: 2.85 Fall probability threshold: >= 0.55; SN: 0.32; SP: 0.87; +LR: 2.46; -LR: 0.78; Diagnostic Odds Ratio: 3.15 Fall probability threshold: >= 0.62; SN: 0.09; SP: 0.96; +LR: 2.25; -LR: 0.95; Diagnostic Odds Ratio: 2.37 **Model** 2: AUC: no information Calibration: no information Fall probability threshold: >= 0.25; SN: 0.78; SP: 0.46; +LR: 1.44; -LR: 0.48; Diagnostic Odds Ratio: 3.02 Fall probability threshold: >= 0.33; SN: 0.73; SP: 0.56; +LR: 1.66; -LR: 0.48; Diagnostic Odds Ratio: 3.44 Fall probability threshold: >= 0.42; SN: 0.54; SP: 0.74 +LR: 2.08; -LR: 0.62; Diagnostic Odds Ratio: 3.34 Fall probability threshold: >= 0.46; SN: 0.47; SP: 0.80; +LR: 2.35; -LR: 0.66; Diagnostic Odds Ratio: 3.54 Fall probability threshold: >= 0.56; SN: 0.33; SP: 0.90

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					+LR: 3.30; -LR: 0.74; Diagnostic Odds Ratio: 4.43 Fall probability threshold: >= 0.69; SN: 0.16; SP: 0.97 +LR: 5.33; -LR: 0.87; Diagnostic Odds Ratio: 6.15
Delbaere et al. [12], 2010, Australia, Development study	n = 500 Age, mean (SD): 77.9 (4.1) years Female: 54% Fall history: 29.6%	Recurrent falls (94; 18.8%) Follow-up: 12 months	Classification and Regression Tree (CRT) No information on model validation	**Model** 1 (recurrent falls): Risk groups: - Low risk: Physiological fall risk (Physiological Profile Assessment) <0.60. Subgroups were made from the Disability score >0. If Disability score >0, further subdivision was made using Incidental and Planned Exercise Questionnaire < 4hrs/week - High risk group: Physiological fall risk (Physiological Profile Assessment) >=0.60. Subgroups were made from Trail-Making-Test time <50. If Trail-Making-Test > 50, further subdivision was made using poor coordinated stability (error score >= 15). If score >= 15, further subdivision was done using Incidental and Planned Exercise Questionnaire > 0	**Model** 1: AUC and calibration: "We didn't calculate an AUC or related measure for our tree and our sample was not big enough to run a calibration analysis as well. No other classification measures were calculated " (author response)
Yamashita et al. [13], 2011, USA, Development study	n = 23,417 Age, mean (SD): 76.1 (8.94) years Female: 79% Fall history: 3%	Any falls (approx. 1,400 (5.9%) Follow-up: between one day and 18 months due to the study using an open cohort design	Logistic regression No information on model validation	**Model** 1 (any falls): Regression table with coefficients: constant: no information; fall history: 0.997; female: 0.133; age: 0.013; blacks: -0.334; others: -0.363; married: -0.137; Alzheimer's disease: 0.055; cancer: -0.307; cataracts: -0.21; dementia: -0.135; depression: 0.334; diabetes: -0.034; emphysema: -0.171; glaucoma: 0.118; incontinence: 0.189; Parkinson's disease: 0.261; stroke: 0.103; vertigo: 0.085; total number of medications: 0.029; activities of daily living score: -0.07; instrumental activities of daily living score: 0.01; gait-shuffling: 0.027; gait-unsteady: 0.178; grasp-tremors: 0.426; grasp-weakness: -0.451; joint pain: 0.129; recent weight loss: 0.332; medication administration: 0.119; need for supervision: -0.265	**Model** 1: AUC: 0.61 Calibration: no information
Panzer et al. [14], 2011, USA, Development study	n = 74 Non-fallers: - Age, range: 65-87 years - Age, mean (SD): 75.1 (6.5) years	Recurrent falls (40; 54.1%) Follow-up: 12 months	Only sensitivity and specificity were calculated	**Model** 1 (recurrent falls): Multiple falls, gait velocity, turn time, turn number of steps, down 3 stairs, and step in tub **Model** 2 (recurrent falls):	**Model** 1: AUC: no information Calibration: no information Extracted from figure 2: SN: 52%; SP: 55%

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	Fallers: - Age, range: 70-94 years - Age, mean (SD): 80.1 (6.2) years Female: no information Fall history: 63.5%		No information on model validation	Multiple falls, gait velocity, turn time, turn number of steps, and down 3 stairs **Model** 3 (recurrent falls): Multiple falls, gait velocity, turn time, and turn number of steps **Model** 4 (recurrent falls): Multiple falls, quiet standing, maximal leaning, sway area, and medial-lateral excursion **Model** 5 (recurrent falls): Multiple falls, quiet standing, maximal leaning	**Model** 2: AUC: no information Calibration: no information Extracted from figure 2: SN: 55%; SP: 55% **Model** 3: AUC: no information Calibration: no information Extracted from figure 2: SN: 55%; SP: 52% **Model** 4: AUC: no information Calibration: no information Extracted from figure 2: SN: 78%; SP: 55% **Model** 5: AUC: no information Calibration: no information Extracted from figure 2: SN: 68%; SP: 55%
Bongue et al. [15], 2011, France, Development study	n = 1,759 Age, mean (SD): 70.7 (4.6) years Female: 51% Fall history: 26%	Any falls (563; 32%) Follow-up: 12 months	Cox regression No information on model validation	**Model** 1 (time to any falls): Regression table (coefficient): baseline hazard: no information; women 0.506; living alone: in couple: 0, family: 0.438, alone: 0.315; self-reported osteoarthritis: 0.22; history of falls - 1 year: 0 falls: 0, 1 fall: 0.616, 2 falls: 0.907, 3 or more falls: 1.42; psychoactive drug use: 0.217; balance impairment: 0.270 Scoring rule (points): women (2); living alone (1); self-reported osteoarthritis (1); history of falls - 1 year: 1 fall (2), 2 falls (4), 3 or more falls (6); psychoactive drug use (1); balance impairment (1) Cut-off: 7 Low risk: Score: 0-3; frequencies: 55.3; OR: 1 Moderate risk: Score: 4-6; frequencies: 34.9; OR 2.4 (2.2-3.4) High risk: Score: 7-12; frequencies: 9.8; OR: 7.8 (5.5-11.1)	**Model** 1: AUC (CI): 0.70 (0.67-0.73) Calibration: no information Youden index = 3, for this: SN: 70.2% SP: 60.3% PPV: 45.5% NPV: 81.1% Evolution of PPV and NPV cut-off = 7, for this: SN: 19.2% SP: 96.5% PPV: 72% NPV: 72.7%
Viccaro et al. [16], 2011, USA,	n = 492	First time falls (83; 19.5%)	Logistic regression	**Model** 1 (first time fall):	**Model** 1: AUC: 0.60

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Development study	Age, mean (SD): 74 (5.7) years Of the 457 with complete follow-up (except n = 18 who died during follow-up): female: 43.5% Fall history: 29.7%	Any falls (155; 36.5%) Recurrent falls (58; 13.9%) Follow-up: 12 months	No information on model validation	Timed Up & Go test, 4 m gait speed test, age, and fall history **Model** 2 (any fall): Timed Up & Go test, 4 m gait speed test, age, and fall history **Model** 3 (recurrent falls): Timed Up & Go test, 4 m gait speed test, age, and fall history	Calibration: no information **Model** 2: AUC: 0.729 Calibration: no information **Model** 3: AUC: 0.786 Calibration: no information
Yamashita et al. [17], 2012, USA, Development study	n = 9,661 Age, mean (SD): 74.2 (7.16) years Of the 9,592 included in the analyses: female: 57.8% Fall history: 31%	Any falls (3,299; 34%) Follow-up: 24 months	Logistic Tree with Unbiased Selection (LOTUS)/ Logistic Regression Tree Method Cross-validation (10 subsets)	**Model** 1 (any falls): Fall history, age, difficulty with knees, activities of daily living, cognitive impairment, self-rated health, instrumental activities of daily living, prescription drugs, and stroke	**Model** 1: AUC: 0.71 Calibration: no information
Weiss et al. [18], 2013, Israel, Development study	n = 71 Age, mean (SD): 78.36 (4.71) years Female: 65% Fall history: 45%	Recurrent falls (12; 16.9%) Follow-up: 6 months	Logistic regression No information on model validation	**Model** 1 (recurrent falls): Four Square Step Test, total activity duration (Accelerometer), anterior-posterior acceleration range (Accelerometer), anterior-posterior width (Accelerometer), and age	**Model** 1: AUC: no information Calibration: no information SN: 75% SP: 100% "94.7% of the subjects were successfully identified as future fallers and non-fallers"
Hnizdo et al. [19], 2013, USA, Validation	n = 113 Age, mean: 79.8 years Female: 34.6% Fall history: 49.5%	Any falls (33; 29.2%) Follow-up: participants were followed until discharged from home health services	Validation study	**Model** 1 (any falls): Age, fall history, elimination problems, high risk medications, use of patient care equipment, limited mobility, and altered cognition	**Model** 1: AUC (95% CI): 0.66 (0.55-0.78) Calibration: no information Cut-off: 14 points SN: 72.5% SP: 52.2% PPV: 39.6% NPV: 81.4%
de Vries et al. [20], 2013, The Netherlands, Development study	n = 1,509 Age, median (range): 75.6 (64.8-88.8) years Female: 51.8% Fall history: 32.2%	Recurrent falls, ≥ 2 falls (174; 11.5%) Recurrent falls, ≥ 3 falls (91; 6%) Any falls	Cox regression Logistic regression No information on model validation	**Model** 1 (time to second fall): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24 **Model** 2 (any falls): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low	**Model** 1: AUC:0.58 (0.53-0.62) Calibration: no information **Model** 2: AUC: 0.51 (0.346-0.56) Calibration: no information **Model** 3:

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		(468; 31.0%) Follow-up: 12 months		visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24 **Model** 3 (recurrent falls, ≥ 2 falls): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24 **Model** 4 (recurrent falls, ≥ 3 falls): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24	AUC: 0.50 (0.42-0.57) Calibration: no information **Model** 4: AUC: 0.49 (0.39-0.59) Calibration: no information
Muhaidat et al. [21], 2014, United Kingdom, Development study	n = 66 Non-fallers: Age, mean (SD): 75 (11.5) years Fallers: Age, mean (SD): 82 (12) years Female: 66% Fall history: 45%	Any fall (13; 19.7%) Follow-up: 6 months	Random Forrest Classification Training set 67% of the sample Test set: 33% of the sample	**Model** 1 (any falls): Table of predictors with corresponding Mean Decrease in Accuracy, and Mean Decrease in Gini: time required to complete triple task, time required avoiding a moving obstacle and cup, time required for TUG and cup, time required for single-task avoiding a moving obstacle, absolute difference in time between single-task TUG and dual-task TUG	**Model** 1: AUC: no information Calibration: no information Out-of-bag error rate: 27.4% Correct classification: 72.6%
Gadkaree et al. [22], 2015, USA, Development study	n = 7,609 Age groups, % (CI): - 65-69: 27.9% (27.0-29.0) - 70-74: 25.0% (24.1-25.8) - 75-79: 19.1% (18.2-19.9) - 80-84: 14.7% (14.0-15.4) - 85-89: 9.1% (8.5-9.8) - 90+: 4.3% (3.8-4.7) Female: 56.6% Fall history: 30.5%	Any falls (2,028; 26.7%) Recurrent falls (957; 12.6%) Follow-up: 12 months	Logistic regression Split-sample validation (random split; 66.6%/33.3%) Cross-validation (no information on subsets)	**Model** 1 (any falls): Age, gender, and race **Model** 2 (recurrent falls): Age, gender, and race **Model** 3 (any falls): y = -1.44 + ((Age 70-74 years)* -0.33) + ((Age 75-59 years)*0.07) + ((Age 80-84 years)*0.17) + ((Age 85-89 years)*0.37) + ((Age 90+ years)*0.26) + Female*0.12 + (Black ethnicity * -0.27) + (Other ethnicity * -0.52) + (Hispanic ethnicity * 0.07) + Self-reported balance problems*0.69 + Fall history*1.15 **Model** 4 (recurrent falls)	**Model** 1: AUC (95% CI): 0.57 (0.54-0.60) Calibration: no information **Model** 2: AUC (95% CI): 0.59 (0.56-0.61) Calibration: no information **Model** 3: AUC (95% CI): 0.69 (0.67-0.71) Calibration: no information Performance in validation set: AUC (95% CI): 0.70 (0.67-0.73) **Model** 4: AUC (95% CI): 0.77 (0.74-0.79)

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				y = 2.67 + ((Age 70-74 years)* -0.66) + ((Age 75-59 years)*-0.08) + ((Age 80-84 years)*0.11) + ((Age 85-89 years)*0.49) + ((Age 90+ years)*0.47) + Female*-0.22 + (Black ethnicity * -0.27) + (Other ethnicity * -0.99) + (Hispanic ethnicity * 0.02) + Self-reported balance problems*1.11 + Fall history*1.46 **Model** 5 (any falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, and hearing impairment **Model** 6 (recurrent falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, and hearing impairment **Model** 7 (any falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, hearing impairment, and Short Physical Performance Battery **Model** 8 (recurrent falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, hearing impairment, and Short Physical Performance Battery	Calibration: no information Performance in validation set: AUC (95% CI): 0.76 (0.73-0.80) **Model** 5: AUC (95% CI): 0.71 (0.69-0.73) Calibration: no information **Model** 6: AUC (95% CI): 0.78 (0.76-0.81) Calibration: no information **Model** 7: AUC (95% CI): 0.72 (0.70-0.73) Calibration: no information **Model** 8: AUC (95% CI): 0.79 (0.76-0.81) Calibration: no information
Cawthon et al. [23], 2015, USA, Development study	n = 5,994 Age, mean: 74 years (based on other studies on the same cohort)	Recurrent falls (694; 11.6%) Follow-up: 12 months	Logistic regression No information on model validation	**Model** 1 (recurrent falls): Age and Baumgartner Sarcopenia Definition **Model** 2 (recurrent falls): Age and Newman Sarcopenia Definition	**Model** 1: Change in AUC compared to age alone (AUC: 0.577): 0.000 (-0.002; 0.003) Calibration: "We did not generate calibration plots for these

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	Female: 0% Fall history: 21% (based on other studies on the same cohort)			**Model** 3 (recurrent falls): Age and IWG Sarcopenia Definition **Model** 4 (recurrent falls): Age and EWGSOP Sarcopenia Definition **Model** 5 (recurrent falls): Age and FNIH Definition#1 **Model** 6 (recurrent falls): Age and FNIH Definition#2	analyses, just the discrimination and the C statistic information" (author response) NRI events: 0.12 (0.08, 0.16) NRI non-events: -0.12 (-0.14, -0.11) **Model** 2: Change in AUC compared to age alone (AUC: 0.577): 0.001 (-0.002; 0.003) Calibration: no information NRI events: 0.07 (0.04, 0.11) NRI non-events: -0.08 (-0.09, -0.06) **Model** 3: Change in AUC compared to age alone (AUC: 0.577): 0.010 (0.002; 0.018) Calibration: no information NRI events: -0.33 (-0.38, -0.28) NRI non-events: 0.34 (0.32, 0.35) **Model** 4: Change in AUC compared to age alone (AUC: 0.577): 0.009 (0.002; 0.015) Calibration: no information NRI events: -0.33 (-0.38, -0.28) NRI non-events: 0.35 (0.34, 0.36) **Model** 5: Change in AUC compared to age alone (AUC: 0.577): 0.004 (-0.001; 0.008) Calibration: no information NRI events: -0.11 (-0.14, -0.08) NRI non-events: 0.07 (0.06, 0.08) **Model** 6:

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					Change in AUC compared to age alone (AUC: 0.577): 0.001 (-0.001; 0.003) Calibration: no information NRI events: -0.05(-0.06, -0.03) NRI non-events: 0.03(0.02, 0.03)
Palumbo et al. [24], 2016, Italy, Germany, Ireland, and England, Validation	ActiFE: n = 1,416 ELSA: n = 3,303 InCHIANTI: n = 892 TILDA: n = 2,101 Age, mean (SD): ActiFE: 75.7 (6.76) years ELSA: 74.56 (7.31) years InCHIANTI: 73.78 (6.62) years TILDA: 72.79 (5.22) years Female: ActiFE: 56.8% ELSA: 56.7% InCHIANTI: 56.2% TILDA: 53.5% Fall history: ActiFE: 36.1% ELSA: 22.7% InCHIANTI: 20.8% TILDA: 22.8%	Any falls ActiFE (466; 32.9%) ELSA (730; 22.1% 1 years adjusted) InCHIANTI (203; 22.8%) TILDA (569; 27.1% 2 years adjusted) Follow-up: ActiFE: 12 months ELSA: 2 years InCHIANTI: 1 year TILDA: 2 years	Validation study	**Model** 1 (any falls): Age, cognitive impairment, depression, diabetes, comorbidity, dizziness and vertigo, fear of falling, female sex, gait problems, hearing impairment, history of falls, history of stroke, instrumental disability, living alone, number of medications, pain, Parkinson's disease, physical activity limitation, physical disability, poor self-perceived health status, rheumatic disease, urinary incontinence, use of antiepileptics, use of antihypertensives, use of sedatives, vision impairment, and walking aid use	**Model** 1: ActiFE: AUC (95% CI): 0.562 (0.530 - 0.594) ELSA: AUC (95% CI): 0.699 (0.680 - 0.718) InCHIANTI: AUC (95% CI): 0.636 (0.594 - 0.681) TILDA: AUC (95% CI): 0.685 (0.660 - 0.709) Calibration: calibration plots were displayed for all four cohorts. For ActiFE and InCHIANTI, participants with low (high) risk scores, experienced more (respectively, less) falls than expected. For ELSA and TILDA, the model overestimated the risk consistently across strata
Rodriguez-Molinero et al. [25], 2017, Spain, Development study	n = 772 Of participants completing the first follow-up period, age, median (SD): 80.7 (0.1) years Of participants completing the first follow-up period, female: 62.5% Fall history: 26.4%	Recurrent falls (43; 9.9%) Follow-up: 12 months	No regression analyses were performed. Sensitivity, specificity and area under the ROC curve (AUC) were calculated as well as the Odds Ratio (OR) and Relative Risk (RR) associated to positive responses	**Model** 1(recurrent falls): Score chart (range 1-6) on the questions: - Have you ever fallen in the last 6 months? - What is the probability that you fall in the next few months?	**Model** 1: AUC (95% CI): 0.74 (0.66-0.82) Calibration: after contacting study authors, a calibration plot was provided showing acceptable calibration. Calibration slopes were not considered. Cut-off: 3 points SN (95% CI): 70% (56%-84%) SP (95% CI): 72% (68%-76%)

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			No information on model validation		
Lohman et al. [26], 2017, USA, Development study	n = 7,609 Age groups, n (%): 65-69 years: 2,099 (28.4%) 70-74 years: 1,863 (25.2%) 75-79 years: 1,427 (19.3%) 80-84 years: 1,079 (14.6%) 85-89 years: 636 (8.6%) 90+ years: 288 (3.9%) Female: 56.4% Fall history: no information	Any falls (3,903; 51.3%) Recurrent falls (2,181; 28.7%) Follow-up: 48 months	Logistic regression No information on model validation	**Model** 1 (any falls): STEADI algorithm: - Low risk, all present: no falls in past year, no worrying about falling, no unsafe/unsteady feeling while walking - Moderate risk, all present: yes, to one of the above-mentioned questions the low risk group, >4 chair stands in 30 sec., completion of all balance stages in 4 stage balance test. If NO to one of tests then both succeeding questions need to be answered as follows: no multiple falls in the past year, and no previous hip fracture since the age of 50 - High risk, all present: the same as in the moderate risk group except one of the succeeding questions are answered as follows: yes, to multiple falls in the past year or yes to previous hip fracture since the age of 50 **Model** 2 (any falls): Covariates: age, race, gender, education, living alone, smokings status, body mass index, vision impairment, hearing impairment, medical burden, functional Impairment, and frailty **Model** 3 (any falls): Model 1 and 2 combined	**Model** 1: AUC: 0.641 ("No CI was calculated" – author response) Calibration: no information SN: 65% SP: 65% PPV: 62% NPV 68% **Model** 2: AUC: 0.575 Calibration: no information **Model** 3: AUC: Calibration: no information
Kim et al. [27], 2017, USA, Development study + Validation study	Development: n = 5,593 (3,960 at follow-up) Validation: n = 4,424 (3,273 at follow-up) Age, median (IQR): Development: 77 (71, 83) Validation: 78 (71, 83) Female: Development: 58.4% Validation: 57.4% Fall history: no information	Recurrent falls - Development: (834; 14.9%) - Validation: (514; 11.6%) Follow-up: No exact length of follow-up was reported. However, they reported that they wanted to predict the outcomes in the following year from baseline	Lasso regression model Bootstrap validation	**Model** 1 (recurrent falls - development): Age, sex, combined comorbidity index, 52 International Classification of Diseases 9-codes, 25 Current Procedural Terminology codes, and 16 Healthcare Common Procedure Coding System level II codes **Model** 2 (recurrent falls - validation): Age, sex, combined comorbidity index, 52 International Classification of Diseases 9-codes, 25 Current Procedural Terminology codes, and 16 Healthcare Common Procedure Coding System level II codes	**Model** 1: C-statistic: 0.62-0.66 Calibration: no information **Model** 2: C-statistic: consistent with C-statistic in Development study sample Calibration: no information

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Palumbo et al. [28], 2018, Italy, Development study	n = 541 Age, mean (SD): 82.4 (6.5) years Of the 438 participants with a complete data set, female: 60.7% Fall history: 27%	Recurrent falls (34; 6.3%) Any falls (87; 16.1%) Follow-up: 12 months	No regression analysis was performed. The predictive accuracy was quantified from TP, TN, FP and FN in terms of SN, SP, PPV, NPV, and accuracy No information on model validation	**Model** 1 (any fall): A table with classification measures for the model with 3 different cut-off values for the TUG-test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Timed up and go test (cut-off: >12 s, >13.5s, and >15s) **Model** 2 (any fall): A table with classification measures for the model with 2 different cut-off values for the SPPB. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Short Physical Performance Battery (cut-off: < 9, and < 11 point) **Model** 3 (any falls): A table with classification measures for the model with 2 different cut-off values for the 7m gait speed test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and 7m gait speed test (cut-off: <0.8 m/s, and < 1m/s) **Model** 4 (recurrent falls): A table with classification measures for the model with 3 different cut-off values for the TUG-test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Timed up and go test (cut-off: >12 s, >13.5s, and >15s) **Model** 5 (recurrent falls): A table with classification measures for the model with 2 different cut-off values for the SPPB. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Short Physical Performance Battery (cut-off: < 9, and < 11 point)	**Model** 1: AUC: no information Calibration: no information TUG > 12s: SN: 36.5% (22.3%-54.5%) SP: 82.5% (76.9%-87.1%) PPV: 25.5% (16.8%-37.6%) NPV: 88.8% (83%-93.6%) Accuracy: 76% (70.2%-81.4%) TUG > 13.5s: SN: 35.8% (23.2%-52.7%) SP: 84% (79.3%-88.4%) PPV: 26.9% (17.3%-38.8%) NPV: 88.8% (83.9%-93.7%) Accuracy: 77.2% (72.4%-82.3%) TUG > 15s: SN: 35.1% (22.7%-52.6%) SP: 84.1% (79.3%-88.4%) PPV: 26.7% (17.5%-38.7%) NPV: 88.7% (83.2%-93.1%) Accuracy: 77.2% (71.7%-82.4%) **Model** 2: AUC: no information Calibration: no information SPPB < 9: SN: 37.2% (24.1%-54.1%) SP: 83.4% (78.7%-87.7%) PPV: 27% (17.8%-37.6%) NPV: 89% (83.3%-94%) Accuracy: 76.9% (71%-81.8%) SPPB < 11: SN: 43.3% (28.4%-62.7%) SP: 79% (72.7%-84.5%) PPV: 25.4% (16.4%-35.8%) NPV: 89.4% (83.9%-94.3%) Accuracy: 74% (67.8%-79.9%) **Model** 3: AUC: no information Calibration: no information

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				Model 6 (recurrent falls): A table with classification measures for the model with 2 different cut-off values for the 7m gait speed test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and 7m gait speed test (cut-off: <0.8 m/s, and < 1m/s)	Gait speed < 0.8 m/s: SN: 35.1% (22.6%-52.5%) SP: 84.3% (78.8%-88.6%) PPV: 26.9% (17.7%-39.1%) NPV: 88.8% (82.9%-93.4%) Accuracy: 77.4% (71%-82.5%) Gait speed < 1 m/s: SN: 35.8% (22.4%-54.4%) SP: 82.4% (76.9%-87.3%) PPV: 25.1% (15.9%-36.5%) NPV: 88.6% (83%-93.3%) Accuracy: 75.8% (69.8%-81.5%) **Model** 4: AUC: no information Calibration: no information TUG > 12 s: SN: 56.2% (32.2%-92.8%) SP: 82.1% (76.9%-86.6%) PPV: 16.8% (8.9%-27.8%) NPV: 96.7% (92.9%-99.7%) Accuracy: 80.5% (75%-85.3%) TUG > 13.5s SN 56.2% (27.6%-89.8%) SP: 83.6% (79.4%-87.6%) PPV: 18.1% (9.7%-29.2%) NPV: 96.7% (92.3%-99.6%) Accuracy: 81.9% (76.9%-86.4%) TUG > 15 s SN: 56.2% (30.3%-92.6%) SP: 83.8% (79.2%-87.7%) PPV: 18.3% (10%-28.6%) NPV: 96.7% (93.1%-99.7%) Accuracy: 82.1% (76.9%-86.8%) **Model** 5: AUC: no information Calibration: no information SPPB < 9: SN: 56.2% (32.1%-93.2%)

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					SP: 82.9% (78.1%-87%) PPV: 17.5% (9.3%-28.4%) NPV: 96.7% (93%-99.7%) Accuracy: 81.3% (76%-85.6%) SPPB < 11: SN: 59% (32.3%-97.4%) SP: 78.1% (72.4%-83.3%) PPV: 14.8% (8.1%-24.3%) NPV: 96.7% (92.7%-99.9%) Accuracy: 76.9% (71%-82.6%) **Model** 6: AUC: no information Calibration: no information Gait speed < 0.8 m/s: SN: 56.2% (30.6%-91.7%) SP: 84% (79.3%-88.2%) PPV: 18.4% (9.9%-29.9%) NPV: 96.7% (93.1%-99.7%) Accuracy: 82.3% (77.2%-87.2%) Gait speed < 1 m/s: SN: 56.2% (30.8%-92%) SP: 82.1% (77.2%-86.5%) PPV: 16.8% (8.9%-27.4%) NPV: 96.7% (92.9%-99.6%) Accuracy: 80.5% (74.9%-85.3%)
Singh et al. [29], 2019, Malaysia, Development study	n = 325 Age, mean (SD): 67.55 (5.5) years Of the n = 305 analysed, female: 56.1% Fall history: 16.7%	Any falls (81; 24.9%) Follow-up: 6 months	Logistic regression No information on model validation	**Model** 1 (any falls): constant: -5.03, age: -0.003, gender: 0.19, medication: -0.24, primary education: -0.27, secondary education: -0.85, history of falls: 0.67, Walk While Talking Test: 0.68, gait speed: 0.25, instrumental activities of daily living: -0.01, Timed up and go test: 0.14, and Physiological Profile Assessment: 1.16 **Model** 2 (any falls): constant: -5.06, age: 0.05, gender: 0.46, medication: -0.14, primary education: 0.07, secondary education: 0.85, history of falls: 0.12, Walk While Talking Test: 0.23, gait speed: -	**Model** 1: Cox-Snell R^2: 0.21 Nagelkerke R^2: 0.31 AUC: no information Calibration: "Hosmer-Lemeshow test result confirmed that the model was a good fit for the data $\chi 2(df = 8, N = 305) = 10.80, P = .21$" Accuracy: 76.6% **Model** 2: Cox-Snell R^2: 0.07 Nagelkerke R^2: 0.95 AUC: no information Calibration: "Hosmer-Lemeshow test results confirmed that the

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				0.07, instrumental activities of daily living: -0.03, and Timed up and go test: 0.16	model was a good fit for the data $\chi 2$ (df = 8, N =305) = 4.77, P = 0.78" Accuracy: 74.1%
Gillain et al. [30], 2019, Belgium, Development study	n = 105 Age, mean (SD): 71.3 (5.4) years Age, range: 65-89 years Of the 96 analysed, female: 50% Fall history: 0%	First time falls (35; 33.3%) Follow-up: 24 months	Classification tree Split-sample validation (no information on allocation) Cross-validation (10 subsets)	**Model** 1 (first time falls): Classification tree: symmetry dual task walking condition cost, fast walking condition stride length, stiffness, comfortable walking condition mean minimum toe clearance, dual task walking condition coefficient of variation of minimum toe clearance cost, fast walking condition variance of minimum toe clearance values, fast walking condition mean minimum toe clearance value, dual task walking condition delta1 minimum toe clearance, and gender	**Model** 1: AUC: 0.84 Calibration: no information SN: 80% SP: 87% PPV: 78% NPV: 88%

Note: AUC = Area Under the Curve; Any falls = both single a CI = confidence interval; COP = centre of pressure; FN = False negative; FP = False positive; kg = kilograms; LOS = limits of stability; +LR = positive likelihood ratio; -LR = negative likelihood ratio; PPV = positive predictive value; N = no; NPV = negative predictive value; n = number; SD = standard deviation; SE = standard error; SN = sensitivity; SP = specificity; TN = True negative; TP = True positive; Y = yes; % = percentage proportion

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Appendix 10: extracted data

See separate Excel file: “Appendix 6-8+10”.

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REFERENCES EXCLUDED DURING SCREENING OF T

Title

Fall Risk Classification in Community-Dwelling Older Adults Using a Smart Wrist-Worn Device and the
UNOBTRUSIVE, SENSOR-BASED MEASUREMENT OF IN-HOME WALKING SPEED AND ITS DAY-TO-DAY V
Predicting First-Time Injurious Falls in Older Men and Women Living in the Community: Development
Using random forest plots to examine predictive and discriminative ability of mobility measures for fu
A Home-Based Older Person Upstreaming Prevention Physical Therapy (HOP-UP-PT) Program Utilizing
Geriatric Assessment Predicts Hospitalization Frequency and Long-Term Care Use in Older Adult Canc
Progressive central vision impairment and concern about falling: A longitudinal study
Use of non-benzodiazepine hypnotics is associated with falls in nursing home residents: a longitudina
Investigating multisite pain as a predictor of self-reported falls and falls requiring health care use in a
Is the SPPB a useful tool for stratifying fall risk in primary care?
IMPACT OF MALNUTRITION ON QUALITY OF LIFE AND CLINICAL OUTCOMES IN OLDER PEOPLE IN THE
Overexpression of ICAM-1 Predicts Poor Survival in High-Grade Serous Ovarian Carcinoma: A Study Ba
[Detection of the largest population susceptible to prescription of a program of exercises in Primary C
Cardiovascular autonomic neuropathy and falls in Parkinson disease: a prospective cohort study
Factors associated with medication related problems in an ambulatory medicare population and the
The spatial parameters of gait and their association with falls, functional decline and death in older ad
Evaluation of the Tinetti score and fall risk assessment via accelerometry-based movement analysis
Effects of benzodiazepines on orthostatic blood pressure in older people
Predictors of Incident Fear of Falling in Community-Dwelling Older Adults
RAMCIP: An advanced social robot for providing assistance at home to older people in need
Using the Child Behavior Checklist (CBCL) with transgender/gender nonconforming children and adole
Perceptions of Fall Circumstances, Recovery Methods, and Community Participation in Manual Whee
Validation of an individualized reduction of falls intervention program among wheelchair and scooter
Effectiveness of a matter of balance program within an assisted living community
Recent history of abuse and fear of falling in Colombian elders
Lower Skeletal Muscle Mass at Admission Independently Predicts Falls and Mortality 3 Months Post-d
Inhibition and decision-processing speed are associated with performance on dynamic posturography
PS-08-001 Symptomatic androgen deficiency develops only when both total and free testosterone de
Predictors of Falls per Step and Falls per Year At and Away From Home in Glaucoma
Outcomes in Critically Ill Patients With Traumatic Brain Injury: Ethnicity, Documentation, and Insuran
Potentially Inappropriate Prescribing and Related Hospital Admissions in Geriatric Patients: A Compar
Falls and Balance/Mobility Outcomes in Racially-Diverse Community-Dwelling Older Adults
Symptoms of anxiety and depression among Chinese women transitioning through menopause: findi
The difference between self-perceived and chronological age in the elderly may correlate with genera
Occurrence of significant long PR intervals in patients implanted for sinus node dysfunction and moni
Risk factors for falls in Iranian older adults: a case-control study
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ASSESSING THE ASSOCIATION OF FEAR OF FALLING AND DEPRESSION IN OLDER ADULTS
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Cross-cultural Instrument Translation and Adaptation: Challenges and Strategies
Characteristics of indoor injuries in hotels compared to home among young children
Dissemination of the CAPABLE Model of Care in a Medicaid Waiver Program to Improve Physical Func

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Incidence and predictive factors of falls in community-dwelling elderly: a longitudinal study
Development, feasibility, acceptability, and adjustment of a portable, multifactorial falls risk test batt
Physical Activity Level and Risk of Falling in Community-Dwelling Older Adults: Systematic Review anc
Fear of falling and its association with anxiety and depression disorders among community-dwelling c
Older women's patterns of home and community care use and transition to residential aged care: An
Effects of an exercise protocol in standing over sedentary lifestyle of frail elderly
Risk factors for falls in the elderly: effects of different programs on Perception, Cognition, Clinical and
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Risk Factors and Number of Falls as Determinants of Quality of Life of Community-Dwelling Older Adu
Incidence, risk factors and economic burden of fall-related injuries in older Chinese people: a systema
Fear of falling and risk of falling: a systematic review and meta-analysis
Objetivo: verifi car si el miedo de caer es factor de riesgo en personas mayores que viven en la comur
Objetivo: Verifi car se o medo de cair √© fator de risco em pessoas idosas que vivem na comunidade.
Prevalence and Impact of Functional and Psychosocial Problems in Hospitalized Adults: A Prospective
Prevalence and Associated Factors of Sarcopenia and Frailty in Parkinson's Disease: A Cross-Sectional
Nursing students' experience with fall risk assessment in older adults
Psychometric properties of home accessibility assessment tools: A systematic review
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Association Between Symptom Burden and Physical Function in Older Patients with Cancer
Comparison of Mobility Indices for Predicting Early Death in Older Patients With Cancer: The Physical
Simulating the effects of a clinical guidelines screening algorithm for fall risk in community dwelling o

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Assessing feasibility of a focused geriatric assessment in older adults with sickle cell disease to addres
Mobility performance among healthy older adults eligible for long-term care in Japan: a prospective c
The Relationships Between Physical Performance, Activity Levels, and Falls in Older Men
The Prevalence and Determinants of Vitamin D Inadequacy among U.S. Older Adults: National Health
Reliability of the Functional Reach Test Using a Mobile Pole Versus the Traditional Fixed Ruler
Evaluation of Implementing a Home-Based Fall Prevention Program among Community-Dwelling Olde
Factors Associated with Participation of Community-Dwelling Older Adults in a Home-Based Falls Prev
Reconstruction of body motion during self-reported losses of balance in community-dwelling older ad
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Physical Function and Frailty for Predicting Adverse Outcomes in Older Primary Care Patients
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Use it or lose it: a qualitative study of the maintenance of physical activity in older adults
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What are the factors associated with the implementation of a peer-led health promotion program? Ir
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Admissions criteria as predictors of first-term success at a chiropractic institution
Comparing Estimates of Fall-Related Mortality Incidence Among Older Adults in the United States
Predicting motor, cognitive & functional impairment in Parkinson's
Validation of a Motor-Cognitive Assessment for a Stepping Exergame in Older Adults: Use of Game-Sp
Body balance analysis of children and youth with intellectual disabilities
Association between visual problems, insufficient emotional support and urinary incontinence with d
Dyskinesia and FAB score predict future falling in Parkinson's disease
Self-rated health, lifestyle habits and risk assessment in 75-year-old persons attending preventive clir
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Development and validation of SIRT3-related nomogram predictive of overall survival in patients with
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Exercise interveNtion outdoor proJect in the cOmmunitY for older people - the ENJOY Senior Exercise
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Influence of functional, psychological, and environmental factors on falls among community-dwelling
Addressing safety in integrated care programs for older people living at home: A systematic review
Older Adults-Potential Users of Technologies
Epilepsy in the elderly
Quantitative mobility metrics from a wearable sensor predict incident parkinsonism in older adults
Reliability, Validity, and Ability to Identity Fall Status of the Berg Balance Scale, Balance Evaluation Sys
Fear of falling among Brazilian and Portuguese older adults
Referral pathway of patients aged 75 years and older after a telephone triage by the French emergen
The role of concern about falling on stepping performance during complex activities
Dietary Protein Intake and Falls in Older People: Longitudinal Analyses From the Osteoarthritis Initiati
The Multidimensional Prognostic Index Predicts Falls in Older People: An 8-Year Longitudinal Cohort S
Perspectives on the preventability of emergency department visits by older patients
Feature based fall detection system for elders using compressed sensing in WVSN
A community-academic partnership to understand the correlates of successful aging in place

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"I Feel Worn Out, as if I Neglected Myself": Older Patients' Perspectives on Post-hospital Symptoms A
Similarity of Repeated Falls in Older Long-Term Care Residents: Do the Circumstances of Past Falls Pre
Concern About Falling Is Associated With Gait Speed, Independently From Physical and Cognitive Fun
Catch the ruler: concurrent validity and test-retest reliability of the ReacStick measures of reaction tir
Identification of thoracic injuries by emergency medical services providers among trauma patients
Prevalence and incidence of nocturia in a hospital setting: An exploratory study
Life-space mobility in older persons with cognitive impairment after discharge from geriatric rehabilit
Association of bioimpedance phase angle and prospective falls in older adults
Psychological and educational interventions for preventing falls in older people living in the communi
Pragmatic development of an evidence-based intensive care unit-specific falls risk assessment tool: Tl
The effects of toe grip training on physical performance and cognitive function of nursing home resid
The physical Housing Environment and Subjective Well-being Among Older People Using Long-Term C
Judgment Capacity, Fear of Falling and the Risk of Falls in Community-Dwelling Older Adults: the Pro.'
Gait Abnormalities and the Risk of Falls in CKD
Analysis of acute respiratory infections due to influenza virus A, B and RSV during an influenza epiden
Psychological difficulties among custodial grandchildren
Lack of consistent sex differences in D-amphetamine-induced dopamine release measured with [(18)I
A comparison of falls between wheelchair users and ambulators with spinal cord injury
Diabetes, arthritis, urinary incontinence, poor self-rated health, higher body mass index and lower ha
Common balance measures and fall risk scores among older adults in Pakistan: Normative values and
To what extent do older adult community exercise programs in Winnipeg, Canada address balance ar
Relationships between orthostatic hypotension, frailty, falling and mortality in elderly care home resi
Defining frailty for healthcare practice and research: A qualitative systematic review with thematic ar
Effects of active video game training on postural control of older adults
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CBOs' Role in Addressing Malnutrition in Community-Dwelling Older Adults: Community-based organ
Associated risk factors of restraint use in older adults with home care: a cross-sectional study
Health outcome of older hospitalized patients in internal medicine environments evaluated by Identif
Remote timed up and go evaluation from activities of daily living reveals changing mobility after surg
Serum adiponectin is associated with body composition and cognitive and psychological status in olde
How costly is voting? Explaining individual differences in the costs of voting
Protein Intake and Risk of Falls: A Prospective Analysis in Older Adults
The association of fatigue and social participation in multiple sclerosis as assessed using two different
An Epidemiological Study of the Risk Factors of Bicycle-Related Falls Among Japanese Older Adults
A Mixed-Methods Evaluation of a Nurse-Led Community-Based Health Home for Ethnically Diverse O
Identifying Barriers in the Home Environment among Urban Community-Dwelling Older Adults with F
Practices for the prevention of accidents in the home in caregivers of children from 1 to 5 years in a d
Muscle exercises for weak elderly
Center of pressure characteristics from quiet standing measures to predict the risk of falling in older a
Resident presentation the art of physical examination: Alcohol abuse masking progressive supranucle
The Association Between Social Engagement, Mild Cognitive Impairment, and Falls Among Older Prim
Body Sensor Network-Based Gait Quality Assessment for Clinical Decision-Support via Multi-Sensor Fu
Physical Performance Predictor Measures in Older Adults With Falls-Related Emergency Department
Effectiveness of training on postural stablity in mild to moderate diabetic neuropathy patients
Falling Again? Falls in Geriatric Adults-Risk Factors and Outcomes Associated With Recidivism

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Contribution of hip abductor-adductor muscles on static and dynamic balance of community-dwellinç
Multifactorial exercise and dance-based interventions are effective in reducing falls risk in communit
Inner-City Asthma in Children
Functional trajectories after traumatic injury among older trauma intensive care unit patients
Physical performance after prolonged mechanical ventilation
"Pisando Fuerte": an evidence-based falls prevention program for Hispanic/Latinos older adults: resu
Biopsychosocial factors associated with the frailty and pre-frailty among older adults
Mapping Associations Between Gait Decline and Fall Risk in Mild Cognitive Impairment
The five W's of falls - weekly online health survey of community-dwelling older adults: analysis of fou
Exposure to anticholinergic and sedative medications using the Drug Burden Index and its association
Identifying community-dwelling older adults' vision loss during mobility assessments: A scoping revie
Relationship between Perceived Indoor Temperature and Self-Reported Risk for Frailty among Comm
Health Smart Homes: User Perspectives...Health Informatics Conference, August 12-14, 2019, Melbou
Home-literacy environment of low-income rural families: Association with child- and caregiver-level c
An integrated approach to fear of falling
Prevalence and determinants of Instrumental Activities of Daily Living (IADL) disability among commu
Tumor infiltrating lymphocytes and homologous recombination deficiency are independently associa
Association between speed of sound of calcaneal bone assessed by quantitative ultrasound and sarcc
How does balance during functional tasks change across older adulthood?
Factors Determining the Increased Risk of Falls in Individuals With Knee Pain in the Malaysian Elders L
Neuroimaging correlates of lateral postural control in older ambulatory adults
Incorporating wheelchair users in people detection
Balance Evaluation of Prefrail and Frail Community-Dwelling Older Adults
Fall determinants in the adult Portuguese: do chronic conditions change the risk of falling?
The validity of an oculus rift to assess postural changes during balance tasks
An Investigation of the Predictors of Comfortable and Fast Gait Speed in Community-Dwelling Older A
One hundred and counting: Centenarian use of emergency departments in New South Wales
Early elementary grade dual language learners from Spanish-speaking homes struggling with English r
An efficient vision based elderly care monitoring framework using fall detection
The role of attentional focus on walking efficiency among older fallers and non-fallers
Effects of two physical activity programs on sleep quality in MCI individuals: a pilot study
QUEDAS EM IDOSOS PRATICANTES DE ATIVIDADE F√çSICA
A atividade f√≠sica √© uma pr√°tica comum entre idosos
Fitness band accuracy in older community dwelling adults
Fall prevention in primary care using chronic disease management plans: A process evaluation of prov
Risk Factors for Low Muscle Mass in a Population-based Prospective Cohort of Brazilian Community-d
Dance as Prevention of Late Life Functional Decline Among Nursing Home Residents
Adherence of older emergency department patients to community-based specialized geriatric service
Detection of alcohol abuse in older patients with cancer: The integration of alcohol screening questio
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Does handwriting the name of a potential trial participant on an invitation letter improve recruitmen
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Feasibility of a Home-Based Balance Intervention in Middle-Aged Women Using Wii Fit Plus√Ç¬Æ
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The timed "up & go" test is a useful predictor of falls in community-dwelling older people
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Risk factors for hip fracture in men. Hip Fracture Study Group
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A falls education initiative on a trauma orthopedic ward: pilot and evaluation...College of Occupation
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Methodology and baseline characteristics for the Sarcopenia and Hip Fracture study: a 5-year prospe
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Femur strength index predicts hip fracture independent of bone density and hip axis length
Risk factors for falls in the Chinese elderly population
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Effect of fish diets and weight loss on serum leptin concentration in overweight, treated-hypertensive
Low-intensity exercise and reduction of the risk for falls among at-risk elders

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Muscle weakness and falls in older adults: A systematic review and meta-analysis
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Does baseline depression increase the risk of unexplained and accidental falls in a cohort of communi
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An interdisciplinary intervention to prevent falls in community-dwelling elderly persons: protocol of a
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The CAREFALL Triage instrument identifying risk factors for recurrent falls in elderly patients
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Profile differences of purchasers, non-purchasers, and users and non-users of Personal Emergency Re
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Do ED staffs have a role to play in the prevention of repeat falls in elderly patients?
Prevalence and treatment of osteoporosis in older Australian men: Findings from the CHAMP study
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A novel successful concept of geriatrics
Effect of high-dosage cholecalciferol and extended physiotherapy on complications after hip fracture
A Prospective Study of Outcomes of Nursing Home Residents with Chronic Kidney Disease with and w
Profile of elderly patients who suffered trauma in Londrina- Paran√°
Old women with a recent fall history show improved muscle strength and function sustained for six m
Diagnosing what's not there
Complex interventions to improve physical function and maintain independent living in elderly peopl
Maintaining independence in older people
A pilot study investigating the effect of a patient-held pain assessment tool in palliative care outpatie
Survival of aged nursing home residents with hip fracture
Prehospital airway management using the laryngeal tube. An emergency department point of view
Impact of palliative care and clinical features of patients with terminal diseases in areas of Internal M
A multi-institutional, hospital-based assessment of clinical, functional, sociofamilial and health-care c
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Fall risk factors in community-dwelling Elderly People
Review: evidence from single studies shows that a few fall risk assessment tools can predict falls in el
Stand by me! Reducing the risk of injurious falls in older adults
Reduced human transitional B cell T1/T2 ratio is associated with subsequent deterioration in renal all
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Characteristics associated with recurrent falls among the elderly within aged-care wards in a tertiary
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Characteristics associated with falls among the elderly within aged care wards in a tertiary hospital: a
Comprehensive geriatric functional analysis of elderly populations in four categories of the long-term
Predictors of falls among community-dwelling older adults with cancer: results from the health and re
Response to letter to the editor, "predictors of falls among community-dwelling older adults with can
Response to letter to the editor, ‚ÄúPredictors of falls among community-dwelling older adults with c
Assessment of impact of medication use and dementia on fall risk in clients receiving home health ca
Assessment of fall risk in community-dwelling older persons
Psychotropic medications and fall risk in community-dwelling older persons using walking-aids
Daytime sleepiness is independently associated with falls in older adults with dementia
Future perspective on active ageing from rapidly ageing Asian countries
Risk factors for fracture following a fall among older people in residential care facilities in Australia
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Factors associated with nursing home placement of all patients admitted for inpatient rehabilitation i
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Trends in length of stay, functional outcomes, and discharge destination stratified by disease type for
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Major trauma in older persons

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Quality of life and physical components linked to sarcopenia: The SarcoPhAge study
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Erratum: Change in arm position during one-leg balance test: A predictor of recurrent falls in commu
Change in arm position during one-leg balance test: A predictor of recurrent falls in community-dwell
Change in arm position during one-leg balance test: a predictor of recurrent falls in community-dwell
Change in Arm Position During One-leg Balance Test: A Predictor of Recurrent Falls in Community-Dw
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The ankle-foot orthosis improves balance and reduces fall risk of chronic spastic hemiparetic patients
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Predictors of postoperative outcome after general surgical procedures in patients with congenital hea

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Tetralogy of Fallot revealed by autopsy in an elderly subject
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INVESTIGATING THE RELATIONSHIPS BETWEEN FULL SPINAL CURVATURES AND FALLS IN THE AGEING

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Transfusion utilization during adnexal or peritoneal cancer surgery: effects on symptomatic venous th
The relationship between frailty, functional dependence, and healthcare needs among community,Äé
Skeletal muscle troponin as a novel biomarker to enhance assessment of the impact of strength train
Factors associated with recurrent falls in a cohort of older adults
Effect of balance exercises on ankle motion during normal and dual-task gait in older adults
Implementation of a Multicomponent intervention to Prevent Physical Restraints In Nursing home re
The impact of impaired kidney function and HIV infection on the risk of anemia
Prevalence of sarcopenia and osteopenia/osteoporosis in older adults with parkinson's disease: A cro
The effect of osteopenia/osteoporosis and sarcopaenia on frailty in older adults: A cross-sectional an
Epidemiology and clinical characteristics of traumatic brain injury in Lebanon: A systematic review
Incidence and characteristics of falls leading to hip fracture in Iranian population
The effectiveness of nonsteroidal anti-inflammatory agents in the treatment of pelvic inflammatory d
Randomized trial of two intravenous schedules of the topoisomerase I inhibitor liposomal lurtotecan
Insights into the field carcinogenesis of ovarian cancer based on the nanocytology of endocervical an
Population-based study of survival for women with serous cancer of the ovary, fallopian tube, peritor
A phase II trial of Sunitinib malate in recurrent and refractory ovarian, fallopian tube and peritoneal c
Use of hematologic biomarkers during chemotherapy predicts survival in ovarian cancer patients
Role of histological type on surgical outcome and survival following radical primary tumour debulking
Trajectory of physical activity after hip fracture: An analysis of community-dwelling individuals from t
Predicting frailty in older people
Developing capacities in aging studies in the Middle East: Implementation of an Arabic version of the
Profile of trauma patients in the emergency department of a tertiary care hospital in South India
Family caregivers,Äô decision process to institutionalize persons with Parkinson's disease: A grounde
Gait speed at usual pace as a predictor of adverse outcomes in community-dwelling older people an I
Distal-proximal skin temperature gradient prior to sleep onset in infants for clinical use
Risk factors associated with neonatal deaths: a matched case-control study in Indonesia
Home-related falls: An underestimated mechanism of injury
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Visual factors should be assessed in older people presenting with falls or hip fracture
Long-term morbidity and mortality related to falls in hemodialysis patients: role of age and gender - a
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Street Youth Mortality: Leaning With Intent to Fall
Community multidimensional fall risk screening
Multifactorial assessment and targeted intervention to prevent recurrent falls in community dwelling
Early derangements in oxygen and glucose metabolism following head injury: the ischemic penumbra
Discovery and Validation of Agonistic Angiotensin Receptor Autoantibodies as Biomarkers of Adverse
Relationship between myocardial performance index and severity of coronary artery disease in patie
Predictors of outcome in acute traumatic central cord syndrome due to spinal stenosis
Engagement of general practitioners in falls prevention assessment and referral to occupational thera

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'ITLES AND ABSTRACTS:

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predisponi √©ndolos a un mayorriesgo de ca√≠das. La funci√≥n muscular de los abductores y de los
predispondo-o a um risco aumentado de cair. A fun√ß√£o muscular dos abdutores e adutores de qua
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Olsen, Christine; Pedersen, Ingeborg; Bergland, Astrid; Enders-Slegers, Marie-Jos√©; Ihleb√¶k, Camill
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=IRCT2016011426008N1
http://www.who.int/trialsearch/Trial2.aspx?TrialID=IRCT20150721023277N6
http://www.who.int/trialsearch/Trial2.aspx?TrialID=IRCT2014012016285N1
http://www.who.int/trialsearch/Trial2.aspx?TrialID=IRCT20131117015426N7
Experimental Gerontology
Prehosp Emerg Care
Neuromodulation
Scientific reports
JBMR Plus
Topics in Geriatric Rehabilitation
European Journal of Health Economics
Asia-Pacific Journal of Public Health
Open Med (Wars)
Rev Esp Geriatr Gerontol
2019 13th Ieee International Conference on Semantic Computing
Gerontechnology
Sports
Age and Ageing
Australasian Emergency Care
JAMA Network Open
Journal of Intellectual Disability Research
J Gerontol Nurs
International Journal of Integrated Care (IJIC)
MEDICC Review
Hepatology Research
Cardiopulmonary Physical Therapy Journal
European Review of Aging and Physical Activity
J Am Med Dir Assoc
Health & Social Care in the Community
Journal of the American Geriatrics Society
Journal of physical activity & health
JAMA Network Open
International Journal of Lower Extremity Wounds

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Archives of Gerontology and Geriatrics
Journal of the American Geriatrics Society
J Geriatr Phys Ther
Clin J Sport Med
Geriatrics & Gerontology International
European Stroke Journal
JBMR Plus
Topics in Geriatric Rehabilitation
PLoS ONE
International Journal of Medical Informatics

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Int J Geriatr Psychiatry
Journal of the American Pharmacists Association
Journal of the American Geriatrics Society
European Journal of Hospital Pharmacy
Psychosomatic Medicine
npj Digital Medicine
Osteoporosis International
Rehabilitation
Clinical Nutrition
Oncologist
Geriatrics & Gerontology International
Bone
Disabil Rehabil
Zeitschrift f√ºr Gerontologie und Geriatrie
BMJ Open
BMJ Open
Neurourology and Urodynamics
Journal of the American Geriatrics Society
J Adv Nurs
Age Ageing
BMJ Open
Journal of the American Geriatrics Society
Journal of Geriatric Physical Therapy
Journal of Thoracic Oncology
http://www.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2013-002722-23-NL
http://www.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2006-006295-37-GB
http://www.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2011-004890-10-GB
http://www.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2012-001249-41-DE
Clinical Infectious Diseases
)m√°n, Daniel; Gonzalo, Montserrat; G√≥mez Candela, Carmen; Mart√≠nez Olmos, Miguel √Ångel; Pin
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Pan African Medical Journal
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J Craniofac Surg
J Pediatr Ophthalmol Strabismus
International Journal of Circumpolar Health
Genet Med
Archives of Disease in Childhood
World J Pediatr Congenit Heart Surg
J Thorac Cardiovasc Surg
Thorac Cardiovasc Surg
Shock
J Am Heart Assoc
J Am Heart Assoc
Congenit Heart Dis
https://clinicaltrials.gov/show/NCT04144283
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Journal of the American Geriatrics Society
Journal of Cachexia Sarcopenia and Muscle
Pilot Feasibility Stud
Journal of the American Geriatrics Society
Exp Gerontol
Journal of the American Geriatrics Society
Journal of Geriatric Physical Therapy
Age and Ageing
Int Orthop
Orthopaedic Surgery
J Nutr Health Aging

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Journal of the American Medical Directors Association
Bone
J Sports Med Phys Fitness
Working with Older People: Community Care Policy & Practice
Jmir Research Protocols
Aging Clinical and Experimental Research
Giornale italiano di cardiologia
Psychiatry Res
http://www.who.int/trialsearch/Trial2.aspx?TrialID=RBR-3s9m65
http://www.who.int/trialsearch/Trial2.aspx?TrialID=RBR-9p8whf
Experimental Gerontology
Clinical Trials
Journal of Alzheimer's Disease
American Journal of Alzheimers Disease and Other Dementias
Hemasphere
Osteoporosis International
Pain
BioMed Research International

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Fall Prevention Guidelines for Patients in Wheelchairs or Patients with Delirium: A Review of Evidence
JBMR Plus
Asian Spine J
BMJ Open
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN14852234
http://www.who.int/trialsearch/Trial2.aspx?TrialID=IRCT20190727044347N1
http://www.who.int/trialsearch/Trial2.aspx?TrialID=IRCT20190223042807N1
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Archives of Physical Medicine and Rehabilitation
International Journal of Medical Informatics
Clinical Rehabilitation
Journal of Public Health (Germany)
Cyberpsychol Behav Soc Netw
Physiotherapy Research International
PLoS ONE
JAMA Neurol
Journal of back and musculoskeletal rehabilitation
Isr Med Assoc J
http://www.who.int/trialsearch/Trial2.aspx?TrialID=JPRN-UMIN000015078
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Danish medical bulletin
BMC Geriatrics
PLoS ONE
Nicotine and Tobacco Research
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN70130279
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN17697307
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN05434601
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN69957256
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN13461584
BMC Geriatr
JAMA: Journal of the American Medical Association
PLoS Medicine
Journal of the American Geriatrics Society
Perceptual & Motor Skills

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Journal of advanced nursing
Clinical Rehabilitation
Public Health
JAMA: Journal of the American Medical Association
J Am Geriatr Soc
Journal of Geriatric Physical Therapy
JAMA Network Open
Int J Geriatr Psychiatry
Clinical Rehabilitation
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN52788952

JBI Database System Rev Implement Rep
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of the American Medical Directors Association
Archives of Gerontology and Geriatrics
Journal of hospital medicine
Journal of the American Geriatrics Society
Academic Emergency Medicine
Pilot Feasibility Stud
Journal of the American Geriatrics Society
Cochrane Database of Systematic Reviews
Osteoporosis International
Salmand-Iranian Journal of Ageing
International journal of yoga therapy
J Gerontol A Biol Sci Med Sci
Neurourology and Urodynamics
J Geriatr Phys Ther
Physiological Reports
Journal of General Internal Medicine
Cad Saude Publica
Cochrane Database of Systematic Reviews
Osteoporosis International
BMC medical research methodology
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Journal of physiotherapy
Age and Ageing
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BMC Geriatrics
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Health Technology Assessment
BMC Medicine
BMJ Open
Drugs & Aging
Cochrane Database of Systematic Reviews
Journal of Science and Medicine in Sport
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN34353836
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN97503221
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN00695885
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN16478561
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN48015966
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN17157277
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN13567538
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN74345449
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN52388040
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN50080330
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN27374299
Pilot Feasibility Stud
Age & Ageing
Br J Sports Med
Cochrane Database Syst Rev
Health Technology Assessment
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Journal of physiotherapy
http://www.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2013-003798-82-GB
http://www.who.int/trialsearch/Trial2.aspx?TrialID=EUCTR2006-005345-11-GB
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Int J Qual Health Care
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Ieee Access
BMC Geriatrics
Canadian Journal of Emergency Medicine
Indian Journal of Public Health
Arch Gerontol Geriatr
Human Movement Science
Journal of Nursing Research
World Neurosurg
Current Geriatrics Reports
Clinical Nutrition
The Journal of nutrition
Trauma Case Reports
Aging Clinical and Experimental Research
Clinical Interventions in Aging
Journal of Bone and Mineral Research
International Journal for Quality in Health Care
Journal of the American Medical Directors Association
Maturitas
Int J Older People Nurs
Journal of Geriatric Physical Therapy
Injury Prevention
Journal of Intellectual Disability Research
J Allergy Clin Immunol Pract
Animals
Am J Case Rep
Sensors
Israel Journal of Occupational Therapy
Sleep
Isr Med Assoc J
Curr Gerontol Geriatr Res
Journal of Applied Gerontology
European Journal of Public Health
BMC Palliative Care
Frontiers in Psychology
Aging Clin Exp Res
Trials
Geriatrics & Gerontology International
Journal of Racial and Ethnic Health Disparities
2019 Ieee Sensors Applications Symposium
Archives of Physical Medicine and Rehabilitation
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN86057119
Women Health
BMC Geriatrics
Rheumatology (United Kingdom)

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BMJ Case Rep
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Professional Psychology: Research and Practice
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BMC Geriatrics
Int J Environ Res Public Health
J Am Geriatr Soc
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Appl Nurs Res
J Neuroeng Rehabil
Journal of Frailty & Aging
Ciencia & saude coletiva
Archives of Osteoporosis
Internet Journal of Allied Health Sciences & Practice
BMC Public Health
J Acquir Immune Defic Syndr
JBI Database System Rev Implement Rep
AIDS and Behavior
Singapore Medical Journal
Journal of the American Geriatrics Society
Journal of aging and health
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Arthritis Research and Therapy
Journal of Urology
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Stroke
Circulation
Journal of the American Medical Directors Association
Pharmacotherapy
Implement Sci
Journal of the American Geriatrics Society
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https://clinicaltrials.gov/show/NCT03996083
https://clinicaltrials.gov/show/NCT03994172
https://clinicaltrials.gov/show/NCT04078217
https://clinicaltrials.gov/show/NCT04181983
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Experimental Aging Research
Neurologia
Aging Clin Exp Res
BMC Geriatrics

Mov Disord Clin Pract
Clinical Interventions in Aging
BMJ Open
Neurourology and Urodynamics
J Environ Radioact
Journal of Geriatric Physical Therapy
BMC Public Health
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International Journal of Therapy and Rehabilitation
Arch Orthop Trauma Surg
Endocrinol Diabetes Metab Case Rep
J Gerontol A Biol Sci Med Sci
Presse Med
Aging & Mental Health
Rehabilitation Psychology
Archives of Physical Medicine and Rehabilitation
International Journal of Older People Nursing
BMC Geriatrics
Journal of Substance Abuse Treatment
Journal of Medical Internet Research
BMJ Open
Gait and Posture
Wounds : a compendium of clinical research and practice
Abreu, Daniela Cristina Carvalho
Nueva York
Nova York
Brain Imaging and Behavior
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PLoS ONE
PLoS Medicine

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Age & Ageing
Quality of Life Research
European Journal of Public Health
Bioresearch Open Access
Int J Cardiovasc Imaging
Journal of bone and joint surgery. American volume
Aging Clinical and Experimental Research
Osteoporosis International
Anesthesiology
Trials
Journal of the American Medical Directors Association
Canadian Journal of Respiratory, Critical Care, and Sleep Medicine
Journal of Geriatric Physical Therapy
EGEMS (Wash DC)
Exp Gerontol
Journal of the American Medical Directors Association
Global Public Health
Annals of Long Term Care
Neurology
Journal of Aging & Physical Activity
Age and Ageing
Journal of developmental and behavioral pediatrics : JDBP
PLoS Medicine
Image Processing on Line
Arthritis Care and Research
Arch Iran Med
Calcif Tissue Int
Journal of Neurotrauma
Archives of Gerontology and Geriatrics
Clinical Nutrition
Aging Clinical and Experimental Research
Journal of prevention of alzheimer's disease
J Biomech
Journal of Medical Systems
Eur Radiol
Giornale di gerontologia
ScientificWorldJournal
Neurology
Journal of the American Geriatrics Society
Echocardiography
Clinical Interventions in Aging
Human Movement Science
OTJR: Occupation, Participation & Health
J Am Geriatr Soc
Front Public Health
Contemporary Clinical Trials

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Archives of Physical Medicine & Rehabilitation
Journal of Geriatric Physical Therapy
J Nutr Sci Vitaminol (Tokyo)
Pasqual; Gomes Bertolini, Sonia Maria Marques
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Br J Clin Pharmacol
Experimental Brain Research
World Neurosurg
J Geriatr Phys Ther
Cluster Computing-the Journal of Networks Software Tools and Applications
European Journal of Heart Failure
Orthopaedics and Traumatology: Surgery and Research
Disability & Rehabilitation: Assistive Technology
Clin Rehabil
African Journal of Emergency Medicine
Sensors (Basel)
Annals of Saudi Medicine
Ieee Access
J Geriatr Phys Ther
Archives of Disease in Childhood
Icc 2019 - 2019 Ieee International Conference on Communications
Sustainable Computing-Informatics & Systems
Transplant Proc
American journal of men's health
Pan Afr Med J
International Journal of Environmental Research and Public Health
BMC Pediatr
Int J Cardiol
European Journal of Sport Science
Ann Rehabil Med
American Journal of Emergency Medicine
Journal of Trauma and Acute Care Surgery
Indian Journal of Physiotherapy & Occupational Therapy
Trials
Frontiers in Aging Neuroscience
Visually Impaired OLder people's Exercise programme for falls prevenTion (VIOLET): a feasibility study
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000143123
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12617000895381
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000293268
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12608000303347
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619001114134
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12605000056695
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12612001265864
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619001014145
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000908235
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000318280
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12614000096651
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12606000025538
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000851268
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000034134
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000936123
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000600246
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12612000224820
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12616001186448
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12616000783426
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12608000065392
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619001183178
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12611001164987
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12605000667617
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12617000564358

http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000540112
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618001827224
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000430134
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000811202
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618002029279
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000823279
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12607000625471
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12611000963921
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12617001186347
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618000903280

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Injury
Neurourol Urodyn
Aging Clin Exp Res
Int J Rehabil Res
International Psychogeriatrics
Neurourology and Urodynamics
Journal of Geriatric Physical Therapy
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ISRCTN13790881
Merec bulletin
European Journal of Physiotherapy
Cochrane Database of Systematic Reviews
BMJ Open
Gut

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Lancet Neurology
American Journal of Geriatric Psychiatry
BMJ Open
J Assoc Res Otolaryngol
Sao Paulo Med J
Archives of Physical Medicine & Rehabilitation
Journal of Hand Therapy
Age & Ageing
Age and Ageing
J Gerontol A Biol Sci Med Sci
Geriatrics
Ent-Ear Nose & Throat Journal
Canadian Journal of Emergency Medicine
JAMA Intern Med
Trials

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J Aging Res
Health & Social Care in the Community
Frontiers in Neurology
Pediatr Cardiol
Physiotherapy
Journal of the American Geriatrics Society
BMC Geriatrics
International Journal of Environmental Research and Public Health
American Surgeon
Age & Ageing
Age and Ageing
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of Cachexia Sarcopenia and Muscle
Disabil Rehabil Assist Technol
Journal of aging and health
HPA Resource
Alzheimer's and Dementia
Maturitas
BMC Geriatrics
BMC Geriatrics
BMC Geriatrics
Revue du Rhumatisme Monographies
Journal of Applied Gerontology
J Allergy Clin Immunol Pract
Br J Clin Pharmacol

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Nutrients
Cochrane Database of Systematic Reviews
Cochrane Clinical Answers
Plast Reconstr Surg
Journal of Urology
Neurourology and Urodynamics
Int J Surg
Journal of General Internal Medicine
J Thromb Thrombolysis
Health & Social Care in the Community
BMC Geriatrics
Int J Environ Res Public Health
Journal of Applied Gerontology
Ciencia & saude coletiva
Med Intensiva
Journal of the American Geriatrics Society
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ChiCTR-ONRC-13005199
Data Technologies and Applications
Res Social Adm Pharm
Experimental Gerontology
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000415101
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12619000349145
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12610001049066
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12615000703505
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12618001097235
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Forensic Sci Int
Journal of Geriatric Oncology
Health & Social Care in the Community
International Journal of Nursing Studies
BMC Geriatrics
International Journal of Environmental Research and Public Health
Advances in Physiotherapy
Age and Ageing
Parkinsonism and Related Disorders
Zeitschrift fur Gerontologie und Geriatrie
Harefuah
Irish Journal of Medical Science

Revista de Saude Publica
Japanese Journal of Geriatrics
Tijdschrift voor Gerontologie en Geriatrie
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Aging and Physical Activity
Osteoporosis International

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Family Practice
Age & Ageing
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Medical Journal of Malaysia
Physiotherapy (United Kingdom)
Journal of Neurology
Journal of the American Geriatrics Society
Gerontology
Kinesitherapie Revue
Homecare Direction
Homecare Quality Management
Journal of Gerontological Nursing
Southern Online Journal of Nursing Research
Parkinsonism and Related Disorders
Consultant Pharmacist
Journal of Rehabilitation Research & Development
Journal of the American Geriatrics Society
Korean Journal of Adult Nursing
Journal of Korean Gerontological Nursing
Arak Medical University Journal
Bone
Respiratory Medicine and Science
Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
Revista Espanola de Geriatria y Gerontologia
The Clinical neuropsychologist
Annals of the Rheumatic Diseases
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Sleep and Breathing
Clinical journal of the American Society of Nephrology : CJASN
Drugs & Aging
Harefuah

Journal of Nutrition, Health and Aging
Age and Ageing
Public Health Reports
IEEE Transactions on Biomedical Engineering
BMC Complementary & Alternative Medicine
Maturitas
Postgraduate medicine
Geriatrics & gerontology international
European Journal of Clinical Investigation
2013 Ieee Pacific Rim Conference on Communications, Computers and Signal Processing
Clinical interventions in aging
Clinical interventions in aging
Archives of Gerontology and Geriatrics

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Singapore medical journal
Journal of the American Geriatrics Society
Ieee Access
Archives of internal medicine
Scientific reports
Zhonghua liu xing bing xue za zhi = Zhonghua liuxingbingxue zazhi
Journal of the American Geriatrics Society
International Journal of Nursing Practice
Gerontology
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Neurobiology of Aging
PloS one
Asian nursing research
Annals of Nutrition and Metabolism
Journal of Surgical Research
Quality of Life Research
Soil Biology & Biochemistry
Journal of Diabetes
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PeerJ
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Journal of Geriatric Physical Therapy

Gerontologist
Gerontologist
Geriatrics & gerontology international
Eastern Mediterranean Health Journal
Gait & posture
Recenti progressi in medicina
Assistenza Infermieristica e Ricerca
Educational Gerontology
Journal of hypertension
Age and Ageing
Clinical Biomechanics
Journal of Orthopaedics Trauma and Rehabilitation
Consultant Pharmacist
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Ambient Assistive Health and Wellness Management in the Heart of the City, Proceeding
Taehan Kanho Hakhoe chi
BMC geriatrics
Home healthcare now
Journal of Aging & Health
Clinical Biomechanics

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The American surgeon
Japanese Journal of Geriatrics
Sensors (Basel, Switzerland)
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Journal of the neurological sciences
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Journal of Gerontological Nursing
Journal of physiological anthropology
Environmental Health and Preventive Medicine
Journal of Physiological Anthropology
Nippon ronen igakkai zasshi japanese journal of geriatrics
European Journal of Trauma & Emergency Surgery
Journal of cardiology
Diabetes
American heart journal
Developmental medicine and child neurology
Infectious Diseases of Poverty
Journal of hypertension
Journal of Headache & Pain
Journal of General Internal Medicine
Consultant Pharmacist
Zhonghua yu fang yi xue za zhi [Chinese journal of preventive medicine]
European Geriatric Medicine
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http://www.who.int/trialsearch/Trial2.aspx?TrialID=DRKS00013644
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International Gambling Studies
Topics in Geriatric Rehabilitation
Am J Geriatr Psychiatry
Progress in Neuro-Psychopharmacology & Biological Psychiatry
Am J Med
Medicine and science in sports and exercise
Arch Osteoporos
http://www.who.int/trialsearch/Trial2.aspx?TrialID=ACTRN12617001352392
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Clinical calcium
Computers
Gerontologist
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Bone
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Preventive Medicine
Archives of Gerontology and Geriatrics
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Perceptual and Motor Skills
Telemedicine Journal and e-Health
Osteoporosis International
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
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Gait and Posture
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Osteoporosis International
Community dentistry and oral epidemiology
Current Orthopaedic Practice
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Accident; analysis and prevention
Disability & Rehabilitation
Journal of Aging Research

International Psychogeriatrics
Menopause-the Journal of the North American Menopause Society
Canadian Journal of Occupational Therapy
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Geriatric Orthopaedic Surgery and Rehabilitation
Journal of the American Medical Directors Association
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Journal of Nutrition Health & Aging
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Journal of Rehabilitation Research and Development
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International Psychogeriatrics
BMC Health Services Research
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Preventive medicine
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International Journal of Environmental Research and Public Health
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BMC psychiatry
Injury
Proceedings of the 20th Annual International Conference of the Ieee Engineering in Medicine and Bic
International Journal of Nursing Studies
Gesundheitswesen
Ageing Research Reviews
International journal of cardiology
Journal of Epidemiology & Community Health
Sleep
Preventive medicine
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Clinical Rehabilitation
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Journal of Applied Biomechanics
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Journal of Nutrition Health & Aging
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Physiological Measurement
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Aging & mental health
Aging and Disease
Evaluation and Program Planning
BMC geriatrics
Experimental Aging Research
Nutrition & Dietetics
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Southern Medical Journal
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Journal of the American Geriatrics Society
Quality of Life Research
Canadian Journal on Aging
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Otology & Neurotology
Scientific reports
Nursing Older People
Nursing & Residential Care
Age
PloS one
British journal of haematology

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Academic Emergency Medicine
Revista Brasileira de Enfermagem
Journal of the American Geriatrics Society
Age & Ageing
Age and ageing
Journal of the American Geriatrics Society
Blood Pressure Monitoring
Archives of General Psychiatry
International Journal of Human-Computer Interaction
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International journal of clinical practice
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Acta Paulista de Enfermagem

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Archives des maladies du coeur et des vaisseaux
Blood Pressure Monitoring
Human-Centric Computing and Information Sciences
Physical Therapy
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Archives of Physical Medicine and Rehabilitation
Journal of the American Geriatrics Society
Journal of sleep research
Journal of the American Geriatrics Society
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QJM : monthly journal of the Association of Physicians
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Journal of clinical epidemiology
Gerontology
European Journal of Endocrinology
Tijdschrift voor Gerontologie en Geriatrie
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Clinical Nutrition
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Proceedings of Future Generation Communication and Networking, Workshop Papers, Vol 2
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Journal of the American Geriatrics Society
Acta Paulista de Enfermagem
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Circulation
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SSM - Population Health
Gerontologist
APA handbook of testing and assessment in psychology, Vol. 2: Testing and assessment in clinical and
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Dementia and Geriatric Cognitive Disorders
Gait & posture
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Disability and rehabilitation
PloS one
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Auris Nasus Larynx
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Social Science & Medicine

Scandinavian journal of clinical and laboratory investigation
NPG Neurologie - Psychiatrie - G√©riatrie
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Archives of Physical Medicine & Rehabilitation
The Journal of sports medicine and physical fitness
Journal of Transport & Health
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Nihon Naibunpi Gakkai zasshi
Patient Preference and Adherence
Current Orthopaedic Practice
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Blood pressure
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Geriatric Nursing
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Journal of Nutrition, Health and Aging
The Journal of frailty & aging
Current gerontology and geriatrics research
American Heart Journal
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American Journal of Respiratory and Critical Care Medicine
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Journal of Sport & Exercise Psychology
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Revista Da Escola De Enfermagem Da Usp
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Indian Journal of Physiotherapy & Occupational Therapy
Disability and Rehabilitation
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Gerontology
Blood
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Age and ageing

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Journal of cardiovascular pharmacology
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Disability and rehabilitation
Revista da Rede de Enfermagem do Nordeste
Journal of Nursing UFPE / Revista de Enfermagem UFPE
2015 10th Iberian Conference on Information Systems and Technologies
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Osteoporosis International
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Quality of Life Research
JAMA Internal Medicine

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International Journal of Qualitative Studies on Health and Well-Being
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Journal of neurology, neurosurgery, and psychiatry
Western journal of nursing research
Journal of Geriatric Physical Therapy
Archives of Gerontology and Geriatrics
Journal of clinical microbiology
Age and Ageing
The Gerontologist
RoFo : Fortschritte auf dem Gebiete der Rontgenstrahlen und der Nuklearmedizin
Social Science & Medicine
Aging & mental health
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Biomarkers : biochemical indicators of exposure, response, and susceptibility to chemicals
Parkinsonism and Related Disorders
Journal of Clinical Gerontology and Geriatrics
Age and ageing
Journal of Neurophysiology
Cardiology
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Journal of Clinical Forensic Medicine
Journal of physiological anthropology and applied human science
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Psychogeriatrics
Environmental Health Perspectives
Journal of health communication
Malaria journal
Malaria journal
Journal of abnormal psychology
American Journal of Kidney Diseases

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Paediatric and Perinatal Epidemiology
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Nutricion Hospitalaria
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2016 Ieee 18th International Conference on E-Health Networking, Applications and Services
2014 Ieee-Embs International Conference on Biomedical and Health Informatics
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Biomed Research International
The American journal of geriatric psychiatry : official journal of the American Association for Geriatric
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Investigative ophthalmology & visual science
Biomedicine (India)
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Geriatrics & Gerontology International
Haemophilia
Physiotherapy (United Kingdom)
Parasites and Vectors
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Controlo'2014 - Proceedings of the 11th Portuguese Conference on Automatic Control
European Review of Aging and Physical Activity
Archives of Gerontology and Geriatrics
Public health nursing (Boston, Mass.)
Clinical Interventions in Aging
The Israel Medical Association journal : IMAJ
Geriatrics & Gerontology International
Journal of Nutrition, Health & Aging
Age
Journal of Epidemiology
Journal of epidemiology
Frontiers in Aging Neuroscience
Archives of Gerontology and Geriatrics
Japanese Journal of Geriatrics
Journal of physical therapy science
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Archives of Physical Medicine and Rehabilitation
Geriatrics & Gerontology International
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Journal of Musculoskeletal Neuronal Interactions
Bulletin of emergency and trauma
Archives of Gerontology & Geriatrics

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Journal of Telemedicine & Telecare
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Zeitschrift fur Gerontologie
Statistics in medicine
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Biomedica
Neurobiology of aging
The Journal of Clinical Psychiatry
Comprehensive psychiatry
Aging clinical and experimental research
Research in Nursing & Health
Journal of Health Psychology
Nursing older people
Actas Urologicas Espanolas

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International Journal of Environmental Research and Public Health
Archives of Gerontology & Geriatrics
Salud Publica de Mexico
American Journal of Physical Medicine & Rehabilitation
Journal of Aging and Health
Journal of safety research
Aging - Clinical and Experimental Research
Clinics in Geriatric Medicine
Menopause (New York, N.Y.)
21st International Conference on Advanced Networking and Applications Workshops/Symposia, Vol 2
PloS one
Sleep
Archives of Physical Medicine & Rehabilitation
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The journals of gerontology. Series A, Biological sciences and medical sciences
Archives of clinical neuropsychology : the official journal of the National Academy of Neuropsycholog
Journal of Nutrition, Health and Aging
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Journal of the American Geriatrics Society
Medicine
PloS one
The American journal of occupational therapy : official publication of the American Occupational The
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Journal of the American Geriatrics Society
Plos One
Spine
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Mental Health, Religion & Culture
World neurosurgery
Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Canc
Journal of the American Geriatrics Society
Clinical Interventions in Aging
Frontiers in endocrinology
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Annals of Internal Medicine
Journal of the American College of Surgeons
Journal of Child & Adolescent Substance Abuse
Physical & Occupational Therapy in Geriatrics
Activities Adaptation & Aging
Medicine
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New England Journal of Medicine
Journals of Gerontology
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American Journal of Epidemiology
American journal of epidemiology
Annals of Internal Medicine
Jama
JAMA Internal Medicine
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PloS one
Gerontology
Public health research & practice
Journal of Science and Medicine in Sport
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of orthopaedic trauma
Geriatrics
Geriatrics
Journal of clinical nursing
Acta Paediatrica
Disability & Rehabilitation
Physiotherapy
Physiological measurement
International Journal of Environmental Research and Public Health
Physical Therapy
Journal of Hand Therapy

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Internal Medicine Journal
Aging clinical and experimental research
British heart journal
Journal of Applied Gerontology
Australasian Journal on Ageing
Archives of Physical Medicine and Rehabilitation
Australian Journal of Physiotherapy
Journal of Physiological Anthropology
Journal of Geriatric Physical Therapy
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Physical Therapy
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PM & R: Journal of Injury, Function & Rehabilitation
Geriatrics & Gerontology International
Acta Medica Nagasakiensia
Japanese Journal of Geriatrics
Japanese Journal of Geriatrics
Archives of Gerontology and Geriatrics
Advances in skin & wound care
Medical Journal of Osaka University
Vascular health and risk management
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Physical therapy
Geriatrics & Gerontology International
Maturitas
Applied Nursing Research
Journal of thrombosis and thrombolysis
Home Health Care Management & Practice
Hypertension Research
Brain Impairment
Age and ageing
Gait and Posture
Frontiers in Human Neuroscience
Archives of Physical Medicine & Rehabilitation

Progress in Community Health Partnerships-Research Education and Action
Journal of the American Geriatrics Society

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Addictive Behaviors
European Journal of Epidemiology
American Journal of Recreation Therapy
Journal of Aging and Health
Endocrine Reviews
Bmc Geriatrics
Archives of Gerontology and Geriatrics
BMC Musculoskeletal Disorders
Physiotherapy (United Kingdom)
The New Zealand medical journal
Technology & Disability
Journal of clinical medicine research
Nihon Ronen Igakkai zasshi. Japanese journal of geriatrics
Clinical calcium
Journal of Gerontological Nursing
Journal of Obstetrics and Gynaecology Research
PloS one
Journal of foot and ankle research
Fungal Ecology
Sensors and Actuators a-Physical
The Journal of clinical psychiatry
Physical Therapy
Journal of the American Geriatrics Society
2013 International Conference on Recent Trends in Information Technology
European Geriatric Medicine
American journal of hypertension
Clinical and experimental hypertension (New York, N.Y. : 1993)
Sleep and Biological Rhythms
BMC public health
Archives of Gerontology and Geriatrics
Information Visualization
Age and Ageing
Journal of Gerontological Nursing
Gender, Work and Organization
Chest
2016 Joint Imeko Tc1-Tc7-Tc13 Symposium: Metrology across the Sciences: Wishful Thinking?
American journal of physical medicine & rehabilitation
Sensors (Basel, Switzerland)
Sensors (Basel, Switzerland)
American Journal of Public Health
Workshop Proceedings of the 9th International Conference on Intelligent Environments
Australian & New Zealand Journal of Psychiatry
Journal of Rheumatology
Gait & posture
PLoS ONE
Physical Therapy

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Pain Medicine
Disability and rehabilitation
Pain practice : the official journal of World Institute of Pain
Pain medicine (Malden, Mass.)
Geriatrics & gerontology international
Physical therapy

Sleep medicine
Biological psychiatry
European journal of clinical nutrition
Clinical Rehabilitation
JBI Database of Systematic Reviews and Implementation Reports
Thorax
Human factors
Age and Ageing
Journal of the American Geriatrics Society
Gait & Posture
Proceedings of the 2013 7th International Conference on Pervasive Computing Technologies for Heal
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
IEEE transactions on bio-medical engineering
Nutrition & Dietetics
European Journal of Cardiovascular Prevention and Rehabilitation
Physiotherapy Canada
The Canadian journal of nursing research = Revue canadienne de recherche en sciences infirmieres
Preventive Medicine
Australian Occupational Therapy Journal
Journal of Nutrition, Health & Aging
Clinical interventions in aging
Age & Ageing
Journal of the American Geriatrics Society
Journal of Aging & Health
Epilepsy Research
Clinical nutrition (Edinburgh, Scotland)
Annals of internal medicine
New Zealand Journal of Physiotherapy
BMJ Open
Physical & Occupational Therapy in Geriatrics
Maturitas
Arthritis Care & Research
Journal of the American Geriatrics Society
Journal of cardiovascular risk
Topics in Clinical Nutrition
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
American Journal of Gastroenterology
Pflegewissenschaft
The Journal of bone and joint surgery. British volume

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International SportMed Journal
Journal of the American Geriatrics Society
Age & Ageing
Archives of Physical Medicine & Rehabilitation
Journal of Aging and Physical Activity
The journals of gerontology. Series A, Biological sciences and medical sciences
Research in Social & Administrative Pharmacy
The Journal of the Acoustical Society of America
Journal of the American Geriatrics Society
Journal of Rehabilitation Research & Development
Journal of Aging & Physical Activity
Current Sports Medicine Reports
Atencion primaria
Archives of Gerontology and Geriatrics
Journal of biomechanics
Games for health journal
Evaluation & the Health Professions
Journal of Evaluation in Clinical Practice
Geriatrics & Gerontology International
BMJ Open
Journal of geriatric physical therapy (2001)
Journal of rehabilitation research and development
Journal of Clinical Nursing
Journal of Clinical Nursing
Clinical neurophysiology : official journal of the International Federation of Clinical Neurophysiology
Accident; analysis and prevention
Journal of community health
Manual Therapy
Journal of the American Geriatrics Society
Archives of physical medicine and rehabilitation
Obesity Facts
International Journal of Medical Informatics
The Journal of experimental biology
Applied Vegetation Science
Proceedings of the Nutrition Society
Disability and rehabilitation

Orthopaedic Nursing
Physiotherapy Theory and Practice

Lancet
Geriatrics & Gerontology International
Critical care (London, England)
Family Practice
PloS one
Journal of the American Geriatrics Society

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British Journal of Community Nursing
Journal of Gerontological Nursing
Disability and rehabilitation
Artificial Intelligence in Medicine
Gait and Posture
Gait and Posture
Journal of the American Geriatrics Society
Neurorehabilitation and Neural Repair
American journal of human genetics
Physical & Occupational Therapy in Geriatrics
PLoS ONE
American Journal of Health Promotion
Archives of Gerontology & Geriatrics
Age and Ageing
BMC Cardiovascular Disorders
American journal of men's health
Saude E Sociedade
Archives of Gerontology and Geriatrics
Metabolism: clinical and experimental
Aging & mental health
Revista Panamericana de Salud Publica
Journal of Hypertension
Alzheimer's and Dementia
Annales de cardiologie et d'angeiologie
American Journal of Medicine
Western Journal of Nursing Research
Long-Term Care Interface
Nursing research
Nursing Research
Orthopedic nursing
Applied Nursing Research
Geriatric Nursing
Journal of the American Medical Directors Association
Journal of animal science
Journal of Geriatric Physical Therapy
International Journal of Sensor Networks
International Journal of Chronobiology
Clinical transplantation
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of geriatric physical therapy (2001)
World journal of surgery
The American Journal of Drug and Alcohol Abuse
The Journal of the Acoustical Society of America
Methods of Information in Medicine
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Gait & Posture

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Gait and Posture
Kidney international
Psychology and psychotherapy
European Journal of Clinical Nutrition
Sensors
Wound Practice & Research
Archives of Physical Medicine and Rehabilitation
Journal of rehabilitation research and development
Clinical endocrinology
Innovation Management and Sustainable Economic Competitive Advantage: From Regional Developm
Journal of environmental and public health
American Journal of Transplantation
Thorax
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Bmc Geriatrics
Nephrology (Carlton, Vic.)
The Gerontologist
Lancet (london, england)
Journal of Gerontological Nursing
Fertility and Sterility
Ceylon Medical Journal
Physical & Occupational Therapy in Geriatrics
Physiotherapy (united kingdom)
Jornal Brasileiro de Psiquiatria
Jornal Brasileiro de Psiquiatria
Arthritis care & research
Revista panamericana de salud publica = Pan American journal of public health
Physiotherapy research international : the journal for researchers and clinicians in physical therapy
The American journal of clinical nutrition
Seizure
The American journal of the medical sciences
Journal of the Medical Association of Thailand
International journal of injury control and safety promotion
Journal of human hypertension
Sleep
Journal of Nutrition, Health & Aging
The Journal of thoracic and cardiovascular surgery
Gait & Posture

Journals of Gerontology Series A: Biological Sciences & Medical Sciences

Archives of Physical Medicine and Rehabilitation
Journal of the American Geriatrics Society
International Journal of Industrial Ergonomics
Gait & posture

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Clinics in Geriatric Medicine
BMJ open
Medical science monitor : international medical journal of experimental and clinical research
British medical journal
JACC. Heart failure
American Journal of Physical Medicine & Rehabilitation
Physiological Reviews
Medicine
Aging & mental health
Journal of the American Geriatrics Society
Disability & Rehabilitation
Value in Health
International Journal of Clinical Pharmacy
Ceylon Medical Journal
Indian heart journal
Bmc Geriatrics
International journal of public health
Hypertension research : official journal of the Japanese Society of Hypertension
Journal of the American Geriatrics Society
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Physical Therapy Case Reports
Archives of Neurology
Epilepsy Research
International journal of pediatric otorhinolaryngology
Brazilian journal of physical therapy
Fisioterapia em Movimento
Respiration; international review of thoracic diseases
Dementia and Geriatric Cognitive Disorders
Age and Ageing
Gait and Posture
International Psychogeriatrics
The journals of gerontology. Series A, Biological sciences and medical sciences
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Physiotherapy (United Kingdom)
Nutricion hospitalaria
European Journal of Preventive Cardiology

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Sensors (Switzerland)
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Journal of clinical sleep medicine : JCSM : official publication of the American Academy of Sleep Medi
Alcohol (Fayetteville, N.Y.)
Gerontologist

Dysphagia
Emerging infectious diseases
CHEST
Lung
Therapeutic Research
Fiziologicheskii zhurnal
Pediatrics
Journal of the American Geriatrics Society
Geriatrics and Gerontology International
Japanese Journal of Physical Fitness and Sports Medicine
Archives of Gerontology and Geriatrics
NMR in biomedicine
American journal of preventive medicine
Clinical infectious diseases : an official publication of the Infectious Diseases Society of America
Aging Clinical & Experimental Research
International Journal of Medical Research & Health Sciences
Journals of Gerontology
Aging clinical and experimental research
International journal of cardiology
Age and ageing
BMJ (Clinical research ed.)
Obesity
Osteoporosis International
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Arthritis Care and Research
QJM
Zeitschrift fur Gerontologie und Geriatrie
Gesundheitswesen (Bundesverband der Arzte des Offentlichen Gesundheitsdienstes (Germany))
Journal of the American Geriatrics Society
Annals of epidemiology
Journal of the American Geriatrics Society
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Gait & posture
Archives of Physical Medicine and Rehabilitation

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Earth and Planetary Science Letters
Journal of neuroengineering and rehabilitation
Journal of Pediatric Nursing
Gait & Posture
Health promotion practice
Zeitschrift Fur Gerontologie Und Geriatrie
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Psychiatry research
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Sao Paulo Medical Journal
Enfermeria Clinica
Indian Journal of Physiotherapy & Occupational Therapy
BMC public health
Nursing & health sciences
Physiotherapy Practice & Research
European Review of Aging and Physical Activity
The Journal of heart and lung transplantation : the official publication of the International Society for
Aging clinical and experimental research
Circulation. Cardiovascular interventions
International journal of chronic obstructive pulmonary disease
Journal of Geriatric Physical Therapy

Preventing chronic disease
British Journal of Clinical Pharmacology
Wiener Medizinische Wochenschrift
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Journal of applied physiology (Bethesda, Md. : 1985)
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Mycoses
European Journal of Preventive Cardiology
American Journal of Physical Medicine & Rehabilitation
Disability and rehabilitation
Cadernos de Saude Publica
Geriatric Nursing

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Rejuvenation Research
Jama-Journal of the American Medical Association
Journal of the American Geriatrics Society
Current gerontology and geriatrics research
Revista Rol de Enfermer√≠a
Aging clinical and experimental research
Disability and rehabilitation. Assistive technology
Osteoporosis International
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Australasian Journal on Ageing
Health promot j aust
Hematologic pathology
Journal of Trauma
Methods of information in medicine
British journal of clinical pharmacology
Journal of Clinical Microbiology
Ciencia & Saude Coletiva
The international journal of behavioral nutrition and physical activity
Archives of Physical Medicine and Rehabilitation
BMC ecology
The European respiratory journal
Journal of Biomechanics
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Journals of Gerontology - Series A Biological Sciences and Medical Sciences
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Journal of geriatric physical therapy (2001)
Aging & Mental Health
Gerontology & Geriatrics Education
Topics in Geriatric Rehabilitation
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Gait and Posture
Journal of hypertension
Consultant Pharmacist
Age and Ageing
Journal of clinical epidemiology
PLoS ONE
Perspectives (Gerontological Nursing Association (Canada))
Revista Mineira de Enfermagem
Physiotherapy (United Kingdom)
2016 Computing in Cardiology Conference
Gerontology
Neuropadiatrie
European heart journal
Postgraduate medical journal
Age & Ageing
Second Joint 3dim/3dpvt Conference: 3d Imaging, Modeling, Processing, Visualization & Transmissior
Biomedical Engineering Letters
Drugs - real world outcomes
Value in Health
The Journal of frailty & aging
Journal of preventive medicine and public health = Yebang «ìihakhoe chi
Journal of physical therapy science
Psychiatry Investigation
Experimental Gerontology
American journal of physical medicine & rehabilitation / Association of Academic Physiatrists
Journal of Korean Academy of Nursing
Sleep medicine
The European respiratory journal
Archives of Physical Medicine & Rehabilitation
Physical Therapy

Archives of Physical Medicine and Rehabilitation
Physical Therapy
Archives of Physical Medicine and Rehabilitation
Current Opinion in Cardiology
Gerontology
The European respiratory journal
Neurourology and Urodynamics
Patient Safety in Surgery
Revista Terapia Manual
Journal of Nepal Health Research Council
PLoS One
American journal of primatology
Journal of the American Society of Hypertension
Medicine

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Journal of geriatric physical therapy (2001)
Cleveland Clinic Journal of Medicine
Fisioterapia
Applied Psycholinguistics
Ieee Access
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Turkish Journal of Geriatrics-Turk Geriatri Dergisi
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Journal of the American Geriatrics Society
Osteoarthritis and Cartilage
American Journal of Occupational Therapy
Sleep

Journal of Surgical Research
Japanese Journal of Geriatrics
Biochemical and biophysical research communications
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of biomechanics
The American journal of clinical nutrition
American Journal of Tropical Medicine and Hygiene
Archives of Gerontology and Geriatrics
Canadian Journal of Occupational Therapy
Clinical science (London, England : 1979)
Nephron. Clinical practice
Geriatrics & gerontology international
Sleep medicine
Journal of Rheumatology
Clinical Biomechanics
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Frontiers in public health
Journal of Aging and Physical Activity
Journal of Geriatric Physical Therapy
Journal of Urology
European Urology
Health and Quality of Life Outcomes
Journal of Geriatric Physical Therapy
Disability and Rehabilitation-Assistive Technology
Archives of Physical Medicine and Rehabilitation
BMC geriatrics
PloS one
Issues on Aging
Journal of Geriatric Physical Therapy
Physiotherapy Canada. Physiotherapie Canada
Geriatric Nursing

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Journal of aging and health
Journal of the American Geriatrics Society
The international journal of behavioral nutrition and physical activity
British Journal of Occupational Therapy
European Geriatric Medicine
Ieee Transactions on Human-Machine Systems
The journals of gerontology. Series A, Biological sciences and medical sciences
Games for health journal
European Journal of General Practice
Journal of Hypertension
Journal of investigative medicine : the official publication of the American Federation for Clinical Rese
Current gerontology and geriatrics research
Current gerontology and geriatrics research
Europace : European pacing, arrhythmias, and cardiac electrophysiology : journal of the working grou
BMC geriatrics
Evaluation and Program Planning
Journal of the American Medical Directors Association
Sleep and Breathing
Journal of Neuroradiology
Neuroscience Letters
Drugs & aging
Anadolu kardiyoloji dergisi : AKD = the Anatolian journal of cardiology
Journal of abnormal psychology
Vector borne and zoonotic diseases (Larchmont, N.Y.)
Journal of Physical Therapy Science
Journal of the American Geriatrics Society
The journals of gerontology. Series A, Biological sciences and medical sciences
Colombia Medica
Journal of the American Geriatrics Society
European Heart Journal
Sage Open
Topics in Geriatric Rehabilitation
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Archives of gerontology and geriatrics
American journal of epidemiology
Gait & posture
European Geriatric Medicine
BMC geriatrics
Irish Journal of Medical Science
European Geriatric Medicine
International Journal of Integrated Care (IJIC)
Journal of Gerontological Nursing
Economic and Social Review
Experimental Brain Research
Home Health Care Consultant
Physiotherapy Canada

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Age and Ageing
Blood pressure monitoring
Journal of the American Medical Directors Association
Ageing & Society
Gerontology
JPMA. The Journal of the Pakistan Medical Association
Annals of the Rheumatic Diseases
Archives of disease in childhood

Geriatrics and Gerontology International
American Journal of Public Health
Journal of nuclear medicine : official publication, Society of Nuclear Medicine
Aging Clinical and Experimental Research
Archives of Gerontology and Geriatrics
Nippon eiseigaku zasshi. Japanese journal of hygiene
Blood Pressure Monitoring
Blood Pressure Monitoring
Journal of Nutrition for the Elderly
Revista Da Escola De Enfermagem Da Usp
Medical journal of the Islamic Republic of Iran
Maturitas
Journal of geriatric physical therapy (2001)
Journal of the American Geriatrics Society
Revue Medicale de Bruxelles
International Journal of Integrated Engineering
Climacteric
Climacteric
Pharmacology

American journal of kidney diseases : the official journal of the National Kidney Foundation
Tohoku Journal of Experimental Medicine
8th World Congress of the International Rehabilitation Medicine Association
Aging Clinical and Experimental Research
Acta Medica Nagasakiensia
Kidney International
Thorax
Postgraduate medical journal
Zeitschrift f√ºr Gerontologie und Geriatrie
Zeitschrift fur Gerontologie und Geriatrie

Jmir Mhealth and Uhealth
European Geriatric Medicine
Journal of Aging & Health
International Journal of Environmental Research and Public Health
Journal of Aging and Physical Activity

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Issues on Aging
Aging Clinical and Experimental Research
Social Science & Medicine
European journal of heart failure
Physiotherapy (United Kingdom)
Journal of Geriatric Physical Therapy
Annals of Long Term Care
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Journal of Geriatric Physical Therapy
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
PloS one
Journal of Bodywork & Movement Therapies
The Journal of infectious diseases
The journal of nutrition, health & aging
Journal of Geriatric Physical Therapy
Issues on Aging
European Journal of Physiotherapy
Journal of geriatric physical therapy (2001)

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Https://clinicaltrials.gov/show/nct02554838
Https://clinicaltrials.gov/show/nct02828826
Https://clinicaltrials.gov/show/nct02017067
Https://clinicaltrials.gov/show/nct03676452
Nurse Prescribing

Preventive Medicine
American journal of kidney diseases : the official journal of the National Kidney Foundation
BMC health services research
Tencon 2009 - 2009 Ieee Region 10 Conference, Vols 1-4
Journal of women & aging
Hypertension research : official journal of the Japanese Society of Hypertension
Archives of physical medicine and rehabilitation
Revista Portuguesa De Investigacao Comportamental E Social
Drugs and Aging
Journal of Japan Academy of Nursing Science
Journal of critical care
Clinical and experimental hypertension (New York, N.Y. : 1993)
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Physical & Occupational Therapy in Geriatrics
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Journal of Aging and Health
Gait and Posture
Journal of Physical Therapy Science
Neuropsychologia
Experimental aging research
Frontiers in Aging Neuroscience
Geriatrics & Gerontology International
Aging Clinical & Experimental Research
Aging clinical and experimental research
The Journal of frailty & aging
Physiotherapy
Mechanisms for Low-Light Therapy V
International Journal of Therapy & Rehabilitation
Pediatrics
Circulation
Journal of the Medical Association of Thailand
Diabetes care
Revista portuguesa de cardiologia : orgao oficial da Sociedade Portuguesa de Cardiologia = Portugues
Applied Animal Behaviour Science
Journal of Palliative Care
Assaults: Interventions, preventive strategies and societal implications.
PloS one
Disability and rehabilitation
2010 Annual International Conference of the Ieee Engineering in Medicine and Biology Society
Journal of Neurology

Arthritis care & research
Arthritis and Rheumatology
Pedobiologia
Studies in health technology and informatics
Nephron. Clinical practice
Cadernos de sa√ſde p√ſblica
Age and Ageing
Clinical and Experimental Allergy
Informatics in primary care
Journal of geriatric physical therapy (2001)
Experimental gerontology
Nursing Science Quarterly
Ieee Sensors Journal
Gait & posture
Sleep

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Accident; analysis and prevention
Western Journal of Nursing Research

Sleep

Annals of Internal Medicine
Frontiers in public health
Osteoporosis International

Disability & Rehabilitation
Gerokomos
Journal of Aging & Health

Sleep
Age and Ageing
Journal of hypertension
American journal of hypertension
Annals of Nutrition and Metabolism
International urology and nephrology
Age and Ageing
PloS one
Clinical endocrinology
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journals of Gerontology Series A: Biological Sciences & Medical Sciences

Physiotherapy Theory and Practice
BMC Public Health
European Geriatric Medicine
European Journal of Pain
Physiotherapy (United Kingdom)
Physiotherapy (United Kingdom)
Archives of Gerontology and Geriatrics
Journal of orthopaedic science : official journal of the Japanese Orthopaedic Association
Arthritis care & research
Schweizerische medizinische Wochenschrift
Experimental Aging Research
Australian Journal of Physiotherapy
BMC geriatrics
BMC geriatrics

Gerodontology
Gerodontology

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International Dental Journal
Journal of clinical epidemiology
Developmental Neurorehabilitation
Rehabilitation Research and Practice
Health and quality of life outcomes
NPG Neurologie - Psychiatrie - G√©riatrie
Journal of the American Geriatrics Society
Medicine & Science in Sports & Exercise
Journal of the American Geriatrics Society
Revista Espanola de Geriatria y Gerontologia
Physiotherapy research international : the journal for researchers and clinicians in physical therapy
Revista latino-americana de enfermagem
Experimental gerontology
Bmc Geriatrics
Journal of the American Medical Directors Association
Human Reproduction
BMC Geriatrics

Research Quarterly for Exercise and Sport
European Geriatric Medicine
Scientific Reports
BMC Geriatrics
Journal of the American Geriatrics Society
European journal of clinical pharmacology
Current opinion in ophthalmology
Healthcare Papers
Medical Science Monitor
Journal of biomechanics
Australian Family Physician
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BMC Health Services Research
Human Vaccines and Immunotherapeutics

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Clinical science (London, England : 1979)
Anthropological Science
Journal of Physical Therapy Science
Gerontology
Geriatrics and Gerontology International
Biogerontology

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Journal of Trauma Nursing
Journal of Safety Research
Archives of Gerontology and Geriatrics
Australasian Journal on Ageing
Geriatrics & Gerontology International
European journal of clinical pharmacology
Rheumatology (Oxford, England)
European Geriatric Medicine
Sleep medicine
Therapeutic apheresis and dialysis : official peer-reviewed journal of the International Society for Apl
Journal of aging research
Journal of Applied Gerontology
Drugs & aging
Ieee Access
Clinical nephrology
Human movement science
Journal of vascular surgery
Physical Therapy
Archives of Physical Medicine & Rehabilitation
Disability & Rehabilitation
Evaluation & the Health Professions
Physiotherapy (United Kingdom)
Topics in Geriatric Rehabilitation
Journal of wildlife diseases
Nutrition & Dietetics
Journal of Geriatric Physical Therapy

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Trials
Clinics in Geriatric Medicine
Theory and Practice of Logic Programming
International Journal on Disability and Human Development
Journal of the American Geriatrics Society
Physical & Occupational Therapy in Geriatrics
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Revista Brasileira De Reumatologia
Journal of the National Society of Allied Health
Topics in Geriatric Rehabilitation
Journal of the American Geriatrics Society
Family & Community Health
American journal of physical medicine & rehabilitation
Journal of the American Geriatrics Society
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Gait and Posture
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European Geriatric Medicine
Australasian Journal on Ageing
American Journal of Clinical Nutrition
Epitheorese Klinikes Farmakologias kai Farmakokinetikes
Journal of the American Geriatrics Society
Journal of sleep research
Diabetes/metabolism research and reviews
Behaviour & Information Technology
Journal of back and musculoskeletal rehabilitation
Medicine and science in sports and exercise
Journal of the American Medical Directors Association
Maturitas
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Journal of the American Podiatric Medical Association
The journals of gerontology. Series A, Biological sciences and medical sciences
Gait and Posture
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Age & Ageing
Journal of the American Podiatric Medical Association
Journal of the American Geriatrics Society
Gerontology

Medicine
Revista Eletronica de Enfermagem
Reviews in Clinical Gerontology
Respirology
COPD: Journal of Chronic Obstructive Pulmonary Disease
Journal of the American Geriatrics Society
European Review of Aging and Physical Activity
Australasian Journal on Ageing
Spinal cord
Chronobiology international
Archives of Gerontology and Geriatrics
Journal of the American Geriatrics Society
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Medicina cl√≠nica
Gaceta Sanitaria
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Gerontology
Gerontology
Osteoporosis International
Gerontology
Disability & Rehabilitation
Age & Ageing
Physiotherapy (united kingdom).
Zeitschrift fur Gerontologie und Geriatrie
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Medical Biometrics, Proceedings
Fs'09: Proceedings of the 10th Wseas International Conference on Fuzzy Systems
Dementia and geriatric cognitive disorders
Violence against women
Physical Therapy
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Journal of Orthopaedic & Sports Physical Therapy
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International Journal of Therapy & Rehabilitation

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OTJR: Occupation, Participation & Health
Disability & Rehabilitation
Clinical and experimental pharmacology & physiology
American Journal of Preventive Medicine
Nutrients
Arthritis and Rheumatology
Journal of medical screening
Global Environmental Change-Human and Policy Dimensions
Clinical endocrinology
Bmc Geriatrics
Science in China Series D-Earth Sciences
Journal of the neurological sciences
Journal of community health

Topics in Advanced Practice Nursing
Expert Opinion on Therapeutic Patents
Scandinavian Journal of Primary Health Care
Journal of Geriatric Physical Therapy
Alzheimer's and Dementia
20th European Conference on Artificial Intelligence
Journal of Geriatric Physical Therapy
Knowledge-Based Systems
Physiotherapy (United Kingdom)
Games for health journal
Consultant Pharmacist
Health Promotion Journal of Australia
Brazilian Journal of Physical Therapy / Revista Brasileira de Fisioterapia
Brazilian Journal of Physical Therapy
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PLoS ONE
BMC geriatrics
Applied clinical informatics
International Journal of Eating Disorders
Sleep Health
Journal of the American Geriatrics Society
Clinical Interventions in Aging
2009 Ieee International Conference on Robotics and Biomimetics
Journal of safety research
Movement Disorders
Appetite
The British journal of ophthalmology
Journal of Science and Medicine in Sport
Journal of the American Geriatrics Society

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Archives of Physical Medicine and Rehabilitation
Gerontology
Age & Ageing
Journal of the American Geriatrics Society
Diabetic Medicine
Archives of Physical Medicine and Rehabilitation
The journal of prevention of Alzheimer's disease
Diabetes care
American journal of industrial medicine
Journal of the American Geriatrics Society
JBI Database of Systematic Reviews and Implementation Reports
Iranian Journal of Public Health
American Journal of Geriatric Psychiatry
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Pediatric Psychology
2013 Asee Annual Conference
Signal Processing, Sensor/Information Fusion, and Target Recognition Xxiii
Public Health Nursing
Clinical Journal of the American Society of Nephrology
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Gerontologist
Gait & Posture
Perceptual and motor skills
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Chinese Nursing Research
Journal of the American Geriatrics Society
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Clinical Biomechanics
Stress and Health
Geroscience
Acta oto-laryngologica
Bmc Geriatrics
Physical & Occupational Therapy in Geriatrics
Physical Therapy
The journals of gerontology. Series A, Biological sciences and medical sciences
Archives of Physical Medicine & Rehabilitation
Journal of Aging & Health
BMJ Open
GM: Midlife & Beyond
International journal of cardiology
Hypertension
BMC geriatrics
Clinical journal of the American Society of Nephrology : CJASN
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Journal of aging and physical activity
Clinical Interventions in Aging
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Diabetologia

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Annals of Long Term Care
Nursing older people

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Neuropsychology
Atencion primaria
Injury prevention
Journal of Pain & Palliative Care Pharmacotherapy
Pharmacy Practice
Circulation
International Journal of Behavioral Development
Research on Aging
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Brazilian journal of otorhinolaryngology
Physical Therapy
Physiotherapy Canada
Physiotherapy
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Sports medicine (Auckland, N.Z.)
Gait & posture
Journal of the American Geriatrics Society
Luts-Lower Urinary Tract Symptoms
Annals of Internal Medicine
Nephrology (Carlton, Vic.)
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PloS one
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Disability & Rehabilitation
European Journal of Physical and Rehabilitation Medicine
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Kidney international
Ontario health technology assessment series
Danish medical bulletin
Journal of Applied Ecology
Journal of Trauma
International Psychogeriatrics
Spine
Clinical transplantation
BMC Health Services Research
Journal of Gerontological Nursing
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Metas de Enfermer√≠a
Blood pressure monitoring
PloS one
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Journal of the American Geriatrics Society
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Drugs & Aging
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Innovation in aging
Aging clinical and experimental research
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Journal of the American College of Surgeons
Enfermeria Clinica
Clinical Interventions in Aging
Journal of the Royal Society of Medicine
Gerontology
Geriatrics & gerontology international
Alzheimer's and Dementia
Critical care medicine
Activities, Adaptation & Aging
Archives of physical medicine and rehabilitation
NeuroRehabilitation
Physical Therapy
Neurorehabilitation and neural repair
Journal of the American Geriatrics Society
American journal of respiratory and critical care medicine
Injury
Acta Ophthalmologica
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International Journal of Medical Informatics
The Journals of Gerontology: Series A: Biological Sciences and Medical Sciences
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Journal of Clinical Endocrinology & Metabolism
Disability and rehabilitation
Journal of Rehabilitation Medicine (Stiftelsen Rehabiliteringsinformation)
Distributed Computing, Artificial Intelligence, Bioinformatics, Soft Computing, and Ambient Assisted L
Ambient Intelligence Perspectives
Disability & Rehabilitation
Psychotherapie, Psychosomatik, medizinische Psychologie
Cerebrovascular Diseases
Journal of Stroke & Cerebrovascular Diseases
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American Family Physician
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Clinical Rehabilitation
Gerontechnology
Journal of the American Geriatrics Society
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Physical therapy
Plos One
Disability & Rehabilitation
Journal of the american medical directors association
Critical care and resuscitation : journal of the Australasian Academy of Critical Care Medicine
Annals of the Rheumatic Diseases
Journal of the American Medical Directors Association
Alimentary Pharmacology & Therapeutics
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Interciencia
International Journal of Geriatric Psychiatry
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Eating behaviors
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Revue d'epidemiologie et de sante publique
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Health Services Research
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European Geriatric Medicine
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Academic emergency medicine : official journal of the Society for Academic Emergency Medicine
Disability and rehabilitation
Journal of Clinical Nursing
American journal of physiology. Heart and circulatory physiology
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The Journal of trauma
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Journal of Head Trauma Rehabilitation

Parkinsonism and Related Disorders
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Gait and Posture

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BMC Pediatrics
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Clinical Orthopaedics and Related Research
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Age & Ageing
Transplantation
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Palaeogeography Palaeoclimatology Palaeoecology
Neurocritical care
Systematic Reviews
Value in Health
Frontiers in Public Health
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Bmc Geriatrics
Parkinsonism & related disorders
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European Geriatric Medicine
Zhonghua xin xue guan bing za zhi
Journal of human hypertension
Canadian Medical Association Journal
Aging Male
Drugs & Aging
International psychogeriatrics
Physical Therapy
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of the American Medical Directors Association
Health & Social Care in the Community
Journal of the Pakistan Medical Association
American journal of cardiovascular drugs
Osteoporosis International
Neurocomputing
Preventive Medicine
European Journal of Preventive Cardiology
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Neurology
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Scandinavian journal of caring sciences
Alzheimers Research & Therapy
Pharmacoepidemiology and Drug Safety
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Osteoporosis International
Journal of the American Medical Directors Association
Archives of physical medicine and rehabilitation
Aging Clinical & Experimental Research
Advances in therapy
Journal of Neurology
Journal of General Internal Medicine
Disability & Rehabilitation
Aging Clinical & Experimental Research
International Psychogeriatrics
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Maturitas
Stroke
Pacific Rim International Journal of Nursing Research
Journal of Neurologic Physical Therapy
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of Emergency Medicine
International Journal of Integrated Care (IJIC)
Clinical calcium
American journal of physical medicine & rehabilitation / Association of Academic Physiatrists
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Journal of Nutrition, Health & Aging
Journal of Bone and Mineral Research
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Osteoporosis International
Journal of vision
Bone
Australian health review : a publication of the Australian Hospital Association
Health Promotion Practice
MMWR: Morbidity & Mortality Weekly Report
American Journal of Preventive Medicine
International Psychogeriatrics
Gait & Posture
International Psychogeriatrics
Archives of Physical Medicine & Rehabilitation
Physiotherapy
Archives of gerontology and geriatrics
Osteoporosis International
The journals of gerontology. Series A, Biological sciences and medical sciences
Irish Journal of Medical Science
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Parkinson's Disease
Parkinsonism and Related Disorders
Disability & Rehabilitation
Physiotherapy Research International
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Age and Ageing

American Journal of Geriatric Psychiatry

Research in Gerontological Nursing
Journal of the Neurological Sciences
Stroke
Neurology
Archives of Internal Medicine
Journal of Bone and Mineral Research
Journal of wound, ostomy, and continence nursing : official publication of The Wound, Ostomy and C
Applied Nursing Research
Academic radiology
Age and Ageing
Clinical biochemistry
BMC infectious diseases
Journal of the American Geriatrics Society
Archives of Internal Medicine
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Journal of the American Geriatrics Society
Tidsskrift for den Norske laegeforening
PLoS ONE
European Geriatric Medicine
Injury
Schweizerische Medizinische Wochenschrift
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Alzheimer's and Dementia
American Journal of Occupational Therapy
OTJR Occupation, Participation and Health
Neurology
The Lancet
Journal of epidemiology / Japan Epidemiological Association
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Drugs & aging
Journal of the American Geriatrics Society
PharmacoEconomics

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Clinical Nursing Research
Sleep
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Minerva anestesiologica
The American journal of gastroenterology
Anaesthesia
European Journal of Geriatrics
Medical Care
Movement Disorders
Disability and rehabilitation
Medecine du Sommeil
American Journal of Emergency Medicine
Scandinavian Journal of Caring Sciences
The Journal of hand surgery
BMJ Open
Pharmacy in Practice
Journal of physical therapy science
Health Services Research
Aging & Mental Health
Journal of Clinical Nursing
Journal of Korean Academy of Nursing
International Journal of Clinical Pharmacy
Stroke (00392499)
Journal of hepatology
Journal of the Medical Association of Thailand
Annals of Emergency Medicine
Academic Emergency Medicine
Journal of the American Medical Directors Association
Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
Archives of physical medicine and rehabilitation
Multiple Sclerosis
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Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Bone
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Archives of Physical Medicine & Rehabilitation
Multiple Sclerosis Journal
Clinical Neurology and Neurosurgery
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Journal of Bone and Joint Surgery - Series A
Journal of Palliative Medicine
Archives of Gerontology and Geriatrics
International journal of surgery (London, England)
Accident; analysis and prevention
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Ieee Journal of Biomedical and Health Informatics
IEEE Journal of Biomedical and Health Informatics
IEEE journal of biomedical and health informatics
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
AJR. American journal of roentgenology
Journal of Gerontological Nursing
No shinkei geka. Neurological surgery
Aging Clinical and Experimental Research
Journal of Korean Academy of Nursing
Neurorehabilitation and neural repair
Journal of Korean Academy of Nursing
Journal of the American Geriatrics Society
International journal of nursing practice
Medicine
Osteoporosis International

Home Healthcare Nurse

Journal of Rehabilitation Research & Development
Preventive Medicine
Ieee Transactions on Mobile Computing
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Journal of Clinical Gerontology and Geriatrics
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Geriatrics & Gerontology International
Journal of physical therapy science
Quality of Life Research
Journal of the American Medical Directors Association
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Endocrine Journal
Acta neurochirurgica
Journal of physical therapy science
Journal of the American Geriatrics Society
Digital Human Modeling: Applications in Health, Safety, Ergonomics and Risk Management
Archives of physical medicine and rehabilitation
Injury Prevention
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Journal of Research in Nursing
Annals of emergency medicine
Journal of the American Medical Directors Association
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Proceedings of the Sixth Ieee Global Humanitarian Technology Conference Ghtc 2016

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The American journal of geriatric pharmacotherapy
Medicina (Brazil)
Journal of neurology, neurosurgery, and psychiatry
Journal of the American Medical Directors Association
Maturitas
Clinical gastroenterology and hepatology : the official clinical practice journal of the American Gastro
Archives of Physical Medicine & Rehabilitation
Journal of Stroke & Cerebrovascular Diseases
Journal of rehabilitation medicine
Malaysian Orthopaedic Journal
The Journals of Gerontology: Series A: Biological Sciences and Medical Sciences
The British journal of psychiatry : the journal of mental science
Medical care
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PrzeglaÃßd lekarski
Zeitschrift Fur Gerontologie
Parkinsonism & related disorders
Neurosurgical focus
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British Journal of Nursing

Neurology
British Journal of Nursing
Journal of Consumer Research
Journal of Trauma Nursing
Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Canc
Journal of Clinical Oncology
Osteoporosis International
Physiotherapy (United Kingdom)
Preventive Medicine
European Respiratory Journal
Journal of cancer research and clinical oncology
Radiotherapy and oncology : journal of the European Society for Therapeutic Radiology and Oncology
West Indian Medical Journal
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Journal of the American Geriatrics Society
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Journal of Aging and Health
Topics in Geriatric Rehabilitation

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Journal of the American Geriatrics Society
Bone
Journal of cachexia, sarcopenia and muscle
Journal of neurosurgery
Annu. Rev. Gerontol. Geriatr.
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The journals of gerontology. Series A, Biological sciences and medical sciences
Annals of epidemiology
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The Journal of arthroplasty
Cerebrovascular diseases (Basel, Switzerland)
Kai Tiaki Nursing New Zealand
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Animal reproduction science
Clinical Rehabilitation
Developmental Psychology
Journal of the American Medical Directors Association
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American Journal of Health Education
Transplant international : official journal of the European Society for Organ Transplantation
Amia ... Annual symposium proceedings. AMIA symposium
Scandinavian Journal of Caring Sciences
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Journal of Alzheimer's disease : JAD
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Emergency Medicine Journal
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Journal of gastrointestinal surgery : official journal of the Society for Surgery of the Alimentary Tract
Archives of Physical Medicine & Rehabilitation
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European Stroke Journal
Circulation

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Journal of Musculoskeletal Pain
BMC geriatrics
Journal of the American Geriatrics Society
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Journals of Gerontology Series A: Biological Sciences & Medical Sciences
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Journal of Geriatric Physical Therapy
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BMC geriatrics
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Journal of Tissue Viability
The Journal of the American Academy of Orthopaedic Surgeons
Current opinion in neurology

Geriatric Nursing
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Hepatology (Baltimore, Md.)
Drugs & Aging
Journal of Investigative Medicine
Osteoporosis International
Nephrology, dialysis, transplantation : official publication of the European Dialysis and Transplant Ass
The Israel Medical Association journal : IMAJ
Polish Nursing / Pielegniarstwo Polskie
Investigative Ophthalmology and Visual Science
Sensors (Basel, Switzerland)
Nephrology Dialysis Transplantation
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International journal of geriatric psychiatry
Journal of Applied Biomechanics
Quality & Safety in Health Care
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Journal of Oral & Maxillofacial Surgery (02782391)
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Disability and rehabilitation
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Journal of Clinical Psychopharmacology
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Clinical Rehabilitation

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Journal of palliative medicine
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American Journal of Emergency Medicine
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Australasian Emergency Nursing Journal
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Deutsche medizinische Wochenschrift (1946)
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Journal of aging research
Prehospital emergency care : official journal of the National Association of EMS Physicians and the Na
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Mayo Clinic proceedings
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Journal of orthopaedic trauma
Journal of the American Geriatrics Society
Gait and Posture
Smart Health
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
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Bmc Cardiovascular Disorders
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Gait & posture
Geriatric Nursing
Archives of Gerontology & Geriatrics

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Academic Emergency Medicine
Journal of Trauma - Injury, Infection and Critical Care
Spine Journal
PM & R : the journal of injury, function, and rehabilitation
Nutrition Research
European Journal of Clinical Nutrition
Current osteoporosis reports
International Psychogeriatrics
MMW Fortschritte der Medizin
Zeitschrift f√ºr Gerontologie und Geriatrie
Respiration; international review of thoracic diseases
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Journal of Cognitive Rehabilitation
European Geriatric Medicine
American Journal of Epidemiology
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Zeitschrift fur arztliche Fortbildung und Qualitatssicherung
Academic Emergency Medicine
Journal of the American Medical Directors Association
Pain medicine (Malden, Mass.)
International journal for quality in health care : journal of the International Society for Quality in Heal
Journal of Neurology Neurosurgery and Psychiatry
Psychological Medicine
Journal of Aging & Physical Activity
American Journal of Occupational Therapy
Movement Disorders
Journal of Rehabilitation Research and Development
Journal of rehabilitation research and development
Age and Ageing
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Journal of cell science
Parkinsonism and Related Disorders
Topics in Stroke Rehabilitation
Neuroepidemiology
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Physiotherapy theory and practice
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Medicine
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Stroke
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Journal of Bone and Mineral Research
Age and Ageing
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Journal of Evidence-Informed Social Work
Neurorehabilitation and Neural Repair
Aging Clinical & Experimental Research
Journal of internal medicine
Nutrition Research
JAMA otolaryngology-- head & neck surgery
Journal of the American Geriatrics Society
Neurourology and Urodynamics
International journal of clinical practice
Archives of physical medicine and rehabilitation
Journal of Stroke & Cerebrovascular Diseases
Journal of Biomechanics
International Journal of Risk and Safety in Medicine
International Journal of Nursing Studies
Value in Health
Journal of clinical densitometry : the official journal of the International Society for Clinical Densitome
Journal of the neurological sciences
Healthy Aging Research
European Journal of Public Health
The Journal of trauma
Clinics (Sao Paulo, Brazil)
European Journal of Physiotherapy
Journal of Clinical Epidemiology
Journal of Clinical Epidemiology
Stroke
Geriatric Medicine
Pacing and clinical electrophysiology : PACE
International Journal of Endocrinology
Osteoporosis International
Journal of the American Geriatrics Society

For peer review only

Journal of the American Geriatrics Society
Clinical interventions in aging
Emergency medicine journal : EMJ
Annals of Nutrition and Metabolism
Aging Clinical & Experimental Research
Aging Clinical and Experimental Research
Hypertension (Dallas, Tex. : 1979)
European Geriatric Medicine
Disability & Rehabilitation
Journal of Bone and Mineral Research
Pan African Medical Journal
The Journal of neuroscience nursing : journal of the American Association of Neuroscience Nurses
Clinical Rehabilitation
Stroke
Alzheimer's and Dementia
Neurourology and Urodynamics
Injury
Public Health
Public Health
Journal of General Internal Medicine
Clinical Geriatrics
The Journal of rheumatology
Journal of Geriatric Physical Therapy
Advances in long-term care, Vol. 1.
Clinics in Geriatric Medicine
Health and Quality of Life Outcomes
Asia Pacific Journal of Clinical Nutrition
Journal of Clinical Sleep Medicine
Cerebrovascular diseases (Basel, Switzerland)
Bone
Intensive Care Medicine Experimental
Journal of the American Geriatrics Society
Journal of Urology
Annals of Emergency Medicine
Acta Ophthalmologica
European journal of ageing
Journal of Bone and Mineral Research
Revista de Ortopedia y Traumatologia
European journal of clinical investigation
Alzheimer's and Dementia
Australasian Journal on Ageing
Osteoporosis International
Age and Ageing
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
The International journal of neuroscience
Clinical interventions in aging

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BMC public health
Health Care for Women International
Neurologia i neurochirurgia polska
Neurologia i neurochirurgia polska
Journal of the American Geriatrics Society
Journal of Clinical Outcomes Management
Canadian Medical Association Journal
Journal of the American Animal Hospital Association
Nephrology, dialysis, transplantation : official publication of the European Dialysis and Transplant Ass
European neurology
Medicine
Complementary Therapies in Medicine

Journal of the American Geriatrics Society
Disability and rehabilitation
Current medical research and opinion

Brazilian journal of physical therapy
Huisarts en Wetenschap
Nihon Ronen Igakkai zasshi. Japanese journal of geriatrics
Journal of the American Geriatrics Society
Clinical Biomechanics
Journal of the American Geriatrics Society
Gerontologist
Geriatrics & gerontology international
Gerontologist
Gerontology and Geriatric Medicine
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Geriatric Physical Therapy
Current Geriatrics Reports
Journal of Rehabilitation Research & Development
Investigative radiology
Archives of Gerontology & Geriatrics
The American journal of gastroenterology
European journal of gastroenterology & hepatology
BMC medical informatics and decision making
Academic Emergency Medicine
Brain Injury
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Neuroepidemiology
Journal of the American Medical Directors Association
Alzheimers Research & Therapy
Age & Ageing
Movement Disorders
Neurology
Age and ageing

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Journal of Gerontological Nursing
Geriatrics & gerontology international
Journal of Gerontological Nursing
European Geriatric Medicine
Physical & Occupational Therapy in Geriatrics
Archives of medical science : AMS
The spine journal : official journal of the North American Spine Society
Bone reports
Annals of Emergency Medicine
Archives of Physical Medicine & Rehabilitation
Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
Revista espanola de geriatria y gerontologia

Physical therapy
Gerontologist
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Medicine
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Journal of Epidemiology & Community Health
Ciencia & Saude Coletiva
Clinics in Geriatric Medicine
Journal of the American Geriatrics Society
Frontiers in Public Health
European Geriatric Medicine

Archives of gerontology and geriatrics
Circulation
Osteoporosis International
Nursing Standard
Age and ageing
Archives of Physical Medicine and Rehabilitation
Bmc Health Services Research
Current medical research and opinion
Osteoporosis International
Medical care
Melanoma research
Journal of internal medicine
American Journal of Physical Medicine & Rehabilitation
Surgery
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society

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Journal of the American Geriatrics Society
Collegian
International Journal of Nursing Studies
International Journal of Nursing Studies
Health Education & Behavior
Journal of the American Academy of Nurse Practitioners
Diabetes
Clinical interventions in aging
European journal of vascular and endovascular surgery : the official journal of the European Society f
Osteoporosis international : a journal established as result of cooperation between the European Fou
Asia-Pacific Journal of Public Health
Asia-Pacific Journal of Public Health
Movement disorders : official journal of the Movement Disorder Society
Journal of Arthroplasty
British journal of neurosurgery
Journal of the American Medical Directors Association
Journal of the American Medical Directors Association
Circulation
JBI library of systematic reviews

European Geriatric Medicine
Atencion Primaria
Journal of the American Geriatrics Society
Gait and Posture
PloS one
Osteoporosis international : a journal established as result of cooperation between the European Fou
Stroke
Geriatrie Et Psychologie Neuropsychiatrie De Vieillissement
Annals of Physical and Rehabilitation Medicine
Zdravstveno varstvo
CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne
Anesthesia and analgesia
The Journal of clinical investigation
The Journal of head trauma rehabilitation
Research in Nursing & Health
The journals of gerontology. Series A, Biological sciences and medical sciences
Revue m√©dicale de Bruxelles
Best Practice and Research: Clinical Rheumatology
Aging clinical and experimental research
Kinesitherapie
Osteoporosis international : a journal established as result of cooperation between the European Fou
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of geriatric physical therapy (2001)
JAMA Ophthalmology
Bone
Journal of Rehabilitation Research and Development

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Nursing & Residential Care
Journal of the American Medical Directors Association
Investigacion y educacion en enfermeria
European journal of physical and rehabilitation medicine
Journal of clinical and diagnostic research : JCDR
The Cochrane database of systematic reviews
Geriatrics & Gerontology International

Bmc Geriatrics
Journal of Ambient Intelligence and Smart Environments
Multimedia Tools and Applications
Clinical journal of the American Society of Nephrology : CJASN
Wiener Klinische Wochenschrift
Journal of stroke and cerebrovascular diseases : the official journal of National Stroke Association
Topics in stroke rehabilitation
Wiener klinische Wochenschrift
Gerontology
American journal of preventive medicine
Diabetes care
Clinical rehabilitation
Clinical and experimental allergy
Cerebrovascular Diseases
Revista de saude publica
Journal of Nutrition, Health and Aging
Polskie Archiwum Medycyny Wewnetrznej
Journal of the neurological sciences
The Journal of trauma
Journal of Nutrition Health & Aging
Journal of the neurological sciences
IEEE transactions on bio-medical engineering
Frontiers in neurology
Frontiers in Neurology
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
The Scientific World Journal
The West Virginia medical journal
Epilepsia
Circulation
Journal of Trauma-Injury Infection and Critical Care
Journal of Nutrition, Health and Aging
Medicine
Movement Disorders
Dementia and Geriatric Cognitive Disorders Extra
Maturitas

The Cochrane database of systematic reviews
Gerontology

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The journals of gerontology. Series A, Biological sciences and medical sciences
European Journal of Clinical Investigation
Parkinsonism & related disorders
Osteoporosis International
Journal of Neuroimaging
Geriatrics & Gerontology International
Journal of Clinical Nursing
BMJ open
Frontiers in aging neuroscience
Italian Journal of Physiotherapy
International psychogeriatrics
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Clinical Epidemiology
BMC Geriatrics
Journal of evaluation in clinical practice
Age and Ageing
Age and Ageing
Ann. New York Acad. Sci.
European Journal of Neurology
Injury
JBI library of systematic reviews
Journal of the American Geriatrics Society

The Journal of bone and joint surgery. British volume

Issues on Aging
European journal of clinical pharmacology
Clinical and experimental allergy
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Revista brasileira de epidemiologia = Brazilian journal of epidemiology
Archives of physical medicine and rehabilitation
Neurorehabilitation and Neural Repair
Neurorehabilitation and Neural Repair
European journal of neurology
Movement disorders : official journal of the Movement Disorder Society
Examining Variation in Access to Long-term Home Care Services for Ontario Seniors
Gerontology
Journal of vascular surgery
Rheumatology
Calcified Tissue International
Otology & neurotology : official publication of the American Otological Society, American Neurotolog
BMC Geriatrics
Journal of internal medicine
Ethnicity and Disease
Journal of the American Geriatrics Society

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International journal of geriatric psychiatry
Drugs & aging
Drugs & Aging
Biomed Research International
BJU International
Osteoporosis International
Health Technology Assessment
Clinical endocrinology
Health Technology Assessment
Archives of osteoporosis
Cochrane database of systematic reviews (Online)
Cochrane database of systematic reviews (Online)
The Journal of clinical endocrinology and metabolism
Medicina Clinica
Osteoporosis International
Current medical research and opinion
Scandinavian journal of trauma, resuscitation and emergency medicine
Osteoporosis International
Nihon rinsho. Japanese journal of clinical medicine
Acta Ophthalmologica
American Journal of Geriatric Psychiatry
Journal of the American Geriatrics Society
World neurosurgery
Italian Journal of Medicine
Circulation
Alimentary pharmacology & therapeutics

Sensors (Basel, Switzerland)
Osteoporosis international : a journal established as result of cooperation between the European Fou
Osteoporosis international : a journal established as result of cooperation between the European Fou
Bone
Regional Anesthesia and Pain Medicine

Research & Theory for Nursing Practice
Clinica chimica acta; international journal of clinical chemistry
Geriatrics & gerontology international
Investigative ophthalmology & visual science
The journals of gerontology. Series A, Biological sciences and medical sciences
American journal of orthopedics (Belle Mead, N.J.)
Clinical science (London, England : 1979)
Journal of the American Geriatrics Society
BMJ open
Neurology
Geriatrika
Journal of Education & Research in Nursing / Hemsirelikte Egitim ve Arastirma Dergisi
Annals of the rheumatic diseases

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Journal of Neuroengineering and Rehabilitation
Drugs & Aging
PloS one
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Rehabilitation Nursing
Geriatrics & Gerontology International
Physical Therapy
Gerontology
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
2008 Ieee 25th Convention of Electrical and Electronics Engineers in Israel, Vols 1 and 2
European Geriatric Medicine
Age and Ageing
BMC geriatrics
Disability and rehabilitation
Physiotherapy (United Kingdom)
Translational medicine @ UniSa
Public Health
Journal of Nursing Scholarship
Journal of Housing for the Elderly
Journal of Healthcare Engineering

Dental Traumatology
Physiotherapy (United Kingdom)
Geriatrics & gerontology international
Journal of Applied Gerontology
PLoS ONE
Journal of Medical and Biological Engineering
Revista brasileira de epidemiologia = Brazilian journal of epidemiology
Physiotherapy
International journal of nursing practice
Applied Nursing Research
Alzheimer's and Dementia
British heart journal
Annals of Geriatric Medicine and Research
Journal of epidemiology and community health
The Lancet
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Journal of Internal Medicine of Taiwan
Ciencia & saude coletiva
Journal of General Internal Medicine
Health and Social Care in the Community
International Journal of Therapy and Rehabilitation
Journal of Aging and Physical Activity
Physiotherapy Singapore
Age & Ageing

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AJNR. American journal of neuroradiology
Clinical nursing research
Plos One
PLoS ONE
Journal of Gerontological Nursing
Journals of Gerontology
Journal of Clinical Geropsychology
Clinical and experimental obstetrics & gynecology
Journal of neurosurgery
Bmc Geriatrics
Nutrients
American journal of respiratory and critical care medicine
Journals of Gerontology Series B-Psychological Sciences and Social Sciences
Clinical Neurophysiology
Dental Traumatology
Neuropsychopharmacology : official publication of the American College of Neuropsychopharmacolo
BMC Geriatrics
JAMA - Journal of the American Medical Association
Pacing and clinical electrophysiology : PACE
BioMed Research International
International Journal of Geriatric Psychiatry
Psychogeriatrics
Journal of Aging & Health
Polish Annals of Medicine
The Journal of frailty & aging
Fortschritte der Neurologie, Psychiatrie
Harnessing the Power of Technology to Improve Lives
Korean Journal of Adult Nursing
Catheterization and cardiovascular interventions : official journal of the Society for Cardiac Angiograp

International journal of environmental research and public health
Journal of the American Geriatrics Society
Medicine
Age and ageing
The Journal of cardiovascular nursing
Bmc Family Practice
Journal of aging and physical activity
Age & Ageing
Journal of Korean Academy of Nursing
European Geriatric Medicine
Sensors (Basel, Switzerland)
Journal of the American Geriatrics Society
Age & Ageing
British Journal of Occupational Therapy
Spine
Aging clinical and experimental research

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Clinical interventions in aging
Journal of Nutrition, Health and Aging
Psychologie et NeuroPsychiatrie du Vieillissement
Everyday Technology for Independence and Care
Plos One
American Family Physician
Journal of the American Geriatrics Society
The journal of nutrition, health & aging
Journal of clinical nursing
Age & Ageing
Journal of the American Geriatrics Society
Health education research
International Psychogeriatrics
Health Education Journal
Entropy
Growth hormone & IGF research : official journal of the Growth Hormone Research Society and the Ir
2015 15th International Conference on Control, Automation and Systems
Care Management Journals
Joint Commission Journal on Quality & Patient Safety
Sage Open
Brain Injury
Clinical Nursing Research
Revue d'Epidemiologie et de Sante Publique
European Journal of Clinical Pharmacology
Sleep health

Journal of hypertension
Age & Ageing
Pharmacy practice
Journal of Aging and Health
BMJ (Clinical research ed.)
Australian Occupational Therapy Journal
European journal of clinical pharmacology
Texto & Contexto Enfermagem
Forensic science international
Public Health
Journal of the American Geriatrics Society
Journal of Bone and Mineral Research
American Family Physician
American Family Physician
The Annals of thoracic surgery
Nursing Outlook
Journal of the American Medical Directors Association
Nursing Outlook
BMC geriatrics
International psychogeriatrics

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Canadian Journal of Cardiology
Osteoporosis International

European Geriatric Medicine
Annals of vascular surgery
Annals of surgical oncology
Topics in Geriatric Rehabilitation
Journal of the American Geriatrics Society
Neurorehabilitation and Neural Repair
AJN American Journal of Nursing
Journal of the Academy of Nutrition and Dietetics
Journal of the American Geriatrics Society
Gerontology
The Journal of endocrinology
Journal of the American Geriatrics Society
Aging - Clinical and Experimental Research
Journal of Medical Economics
Journal of rehabilitation medicine
Journal of the American Geriatrics Society
Primary Health Care Research & Development (Cambridge University Press / UK)
Osteoarthritis and Cartilage
Bmc Musculoskeletal Disorders
Annals of the Rheumatic Diseases
Archives de Pediatrie
International Journal of Medical Informatics
Journal of Enabling Technologies
Hematological oncology
Journal of the American Geriatrics Society
The Journal of surgical research
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Plos One
Physical Therapy
Physical & Occupational Therapy in Geriatrics
The Journal of neuroscience : the official journal of the Society for Neuroscience
Bmj Open
Journal of Korean Academy of Nursing
Pain Medicine
Ergonomics
Physioscience
Age and ageing
Physiotherapy
Journal of General Internal Medicine
Journal of the American Medical Informatics Association
Annales francaises d'anesthesie et de reanimation
Journals of Gerontology Series B-Psychological Sciences and Social Sciences

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Canadian Journal of Occupational Therapy-Revue Canadienne D Ergotherapie
Frontiers in pharmacology
Experimental Gerontology
Archives of Physical Medicine and Rehabilitation
European Geriatric Medicine
Journal of Surgical Research
Circulation
Archives of Gerontology and Geriatrics
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Journal of the American Geriatrics Society
Graefe's archive for clinical and experimental ophthalmology = Albrecht von Graefes Archiv fur klinisc
European Geriatric Medicine
Revue d'Epidemiologie et de Sante Publique
Journal of Physical Therapy Science
Aging Clinical and Experimental Research
Sensors (Basel, Switzerland)
Journal of aging and health
Asian nursing research
Age and ageing
Journal of Science and Medicine in Sport
European Journal of Geriatrics
Journal of the American Medical Directors Association
Trials
JBI Database of Systematic Reviews and Implementation Reports
Australian Dental Journal
Healthcare (Basel, Switzerland)

The journal of nutrition, health & aging
Aging - Clinical and Experimental Research
European Journal of Epidemiology
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of hypertension
Journal of Aging & Health
Gait & Posture
Human Movement Science

Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
The Journals of Gerontology: Series A: Biological Sciences and Medical Sciences
Journal of gerontological nursing

International Journal of Clinical Pharmacy
Archives of neurology
Journal of rehabilitation medicine : official journal of the UEMS European Board of Physical and Rehal
Japanese Journal of Physical Fitness and Sports Medicine
Age and Ageing
Journal of the American Geriatrics Society

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Clinical Interventions in Aging
2013 International Conference on Advanced Robotics and Intelligent Systems
Journal of Evaluation in Clinical Practice
Gerontology
Age & Ageing
Gerontology
Archives of Physical Medicine & Rehabilitation
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Archives of Gerontology and Geriatrics
Gerontology
BMJ Open
Journal of the American Medical Directors Association
Plos One
Hong Kong Physiotherapy Journal
Hong Kong Physiotherapy Journal
The journals of gerontology. Series A, Biological sciences and medical sciences
Pathogens and global health
Physiotherapy (United Kingdom)
Journal of Gerontological Nursing
Medicina cl√≠nica
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
OTJR Occupation, Participation and Health
Hellenic journal of cardiology : HJC = Hellenike kardiologike epitheorese
Zeitschrift fur Gerontologie und Geriatrie
Journal of the American Geriatrics Society
Movement Disorders
European Geriatric Medicine
Impact of Nursing Staff Ratios on Falls Rates In Skilled Nursing Facilities
Rinsho shinkeigaku = Clinical neurology
Medical Engineering and Physics
BMC Geriatrics
PloS one
Gerontologist
Journal of Urology
Archives of physical medicine and rehabilitation
Revista da Rede de Enfermagem do Nordeste
Aging Male
Bmc Geriatrics
Clinical therapeutics
Endocrine journal
PM and R
PM and R
Clinical and Translational Science
Scientific Reports
Journal of the American Medical Directors Association
Parkinsonism & Related Disorders

For peer review only

Clinical rheumatology
Brain : a journal of neurology
International Journal of Integrated Care (IJIC)
International Journal of Integrated Care (IJIC)
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Academic Emergency Medicine
International Journal of Nursing Studies
International Journal of Nursing Studies
Age and ageing
International Journal of Integrated Care (IJIC)
Age and Ageing
Drugs & Aging
Revue De Medecine Interne
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Journal of the American Geriatrics Society
Osteoporosis International
PloS one
International Journal of Therapy & Rehabilitation

Aging Male
Australasian Journal on Ageing
Neurourology and Urodynamics
Alzheimer Disease and Associated Disorders
Age & Ageing
BMC Neurology
Journal of Neurology, Neurosurgery & Psychiatry
Nihon Ronen Igakkai zasshi. Japanese journal of geriatrics
PloS one
Annu. Rev. Gerontol. Geriatr.
Inflammation & allergy drug targets
The Journal of clinical endocrinology and metabolism
American journal of epidemiology
American journal of epidemiology
Journal of Neurology, Neurosurgery and Psychiatry
Journal of Neurology, Neurosurgery & Psychiatry
Clinical interventions in aging
Age & Ageing
Aging Clinical & Experimental Research
Accident; analysis and prevention
Menopause (New York, N.Y.)
Leukemia
Advances in Renal Replacement Therapy
Female Pelvic Medicine and Reconstructive Surgery
Clinical interventions in aging
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of General Internal Medicine

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International psychogeriatrics
Journal of the American Geriatrics Society
Nederlands tijdschrift voor geneeskunde
The journal of nutrition, health & aging
PLoS ONE
Osteoporosis International
Osteoporosis and Sarcopenia
Osteoporosis International
American Journal of Tropical Medicine and Hygiene
American Journal of Tropical Medicine and Hygiene
International journal of cardiology
Kango kenkyu. The Japanese journal of nursing research
European Review of Aging and Physical Activity
Clinical otolaryngology and allied sciences
European Journal of Physical and Rehabilitation Medicine
Osteoporosis International
BMJ Open
European journal of cardiovascular prevention and rehabilitation : official journal of the European So
The American journal of medicine
Journal of Obstetrics and Gynaecology Research
Aging Clinical and Experimental Research
BMJ (Clinical research ed.)
Neoplasma
Turkiye Fiziksel Tip Ve Rehabilitasyon Dergisi-Turkish Journal of Physical Medicine and Rehabilitation
Journal of Bone and Mineral Research
Journal of Bone and Mineral Research
Osteoporosis International
Osteoporosis International
The Journal of clinical endocrinology and metabolism
Archives of Physical Medicine & Rehabilitation
Journal of the American Geriatrics Society
Journal of the American Medical Association
Journal of gerontology
Geriatrie Et Psychologie Neuropsychiatrie De Vieillissement
G√©riatrie et Psychologie Neuropsychiatrie du Vieillissement

International Journal of Geriatric Psychiatry
Journal of plastic surgery and hand surgery
Archives of physical medicine and rehabilitation
Bmc Geriatrics
Journal of burn care & research : official publication of the American Burn Association
Journal of Primary Health Care
Https://clinicaltrials.gov/show/nct02696668
Https://clinicaltrials.gov/show/nct00289458
Https://clinicaltrials.gov/show/nct01539200
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Age and Ageing
BMC neurology
Journal of the American Geriatrics Society

Gait & Posture
American Journal of Epidemiology
Kansenshogaku zasshi. The Journal of the Japanese Association for Infectious Diseases
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of the American Geriatrics Society
Osteoporosis International
European Journal of Physiotherapy
Developmental Medicine and Child Neurology
Archives of Physical Medicine & Rehabilitation
Revista Gaucha de Enfermagem
Cadernos de Terapia Ocupacional da UFSCar
Journal of gastrointestinal surgery : official journal of the Society for Surgery of the Alimentary Tract
Przeglad lekarski
2016 Ieee International Conference on Recent Trends in Electronics, Information & Communication T
Journal of cardiovascular electrophysiology

Age & Ageing
Age and Ageing
Aging clinical and experimental research
World journal of surgery
Aging & Mental Health
Journal of Physical Therapy Science
Journal of Physical Therapy Science
Drug Safety
Journal of Housing for the Elderly
BMC family practice
JACC. Cardiovascular imaging
Canadian Journal of Public Health
Journal of the American Geriatrics Society
Preventive Medicine
Journal of the American Geriatrics Society
Journal of neurotrauma
Gait and Posture
Psychology and Aging
Nursing Administration Quarterly
Journal of Aging and Physical Activity
Journal of aging and physical activity
Archives of Gerontology and Geriatrics
Orvosi Hetilap
Orvosi hetilap
Diabetes Research & Clinical Practice
European Geriatric Medicine

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European Geriatric Medicine
Investigative ophthalmology & visual science
Aging Health
Revue Neurologique
Revue Neurologique
Advances in Conceptual Modeling, Er 2016 Workshops
Environmental research
Przeglad epidemiologiczny
OTJR: Occupation, Participation and Health
Archives of physical medicine and rehabilitation
Clinical journal of the American Society of Nephrology : CJASN
Proceedings of Knowledge Management International Conference
Proceedings of the 6th International Conference on Computing and Informatics: Embracing Eco-Frien
Alcohol
Australian & New Zealand Journal of Public Health
Archives of neurology
Journal of Human Hypertension

[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Safety Science
Journal of Prosthodontic Research
Primary care
International Journal of Occupational Safety & Ergonomics
Agriculture Ecosystems & Environment
Geriatrics & gerontology international
Journal of Physical Therapy Science
Disability and Rehabilitation
Journal of the American Medical Directors Association
Journal of Epidemiology and Community Health
Molecular psychiatry
Rheumatology international
Journal of Trauma Nursing
Collegian (Royal College of Nursing, Australia)
Annals of the rheumatic diseases
Modern Rheumatology
Diabetes Care
European journal of trauma and emergency surgery : official publication of the European Trauma Soc
Trials
Annals of epidemiology
Activities, Adaptation & Aging
Journal of Rehabilitation Research & Development
Journal of clinical epidemiology

Journal of Aging and Physical Activity
Journal of forensic sciences
Journal of epidemiology and community health

For peer review only

PloS one
Physiotherapy Canada. Physioth√©rapie Canada
BMC geriatrics
The Journal of heart and lung transplantation : the official publication of the International Society for
Pacific Rim International Journal of Nursing Research
Expert review of medical devices
Geriatrics & Gerontology International
Bmc Geriatrics
Pakistan Journal of Biotechnology
Drugs & Aging
Physical therapy
The British Journal of Psychiatry
Diabetes Technology & Therapeutics
British Journal of Occupational Therapy
Journal of Rheumatology
Journal of the American Geriatrics Society
Journal of Physical Therapy Science
International Nursing Review
International Urogynecology Journal and Pelvic Floor Dysfunction
Journal of Urmia Nursing & Midwifery Faculty
Clinical Rehabilitation
Journals of Gerontology
Age and Ageing
Rehab Management: The Interdisciplinary Journal of Rehabilitation

Geriatrics & Gerontology International
Journal of Physical Therapy Science
The Korean journal of internal medicine
Journal of sport and health science
Journal of Physical Therapy Science
The Journal of clinical endocrinology and metabolism
European Geriatric Medicine
Experimental Gerontology
Quality in ageing : policy, practice and research
Quality in Ageing
Gait and Posture
Journal of Investigative Medicine
Annals of Geriatric Medicine and Research
Journal of Clinical Endocrinology & Metabolism
Bio-medical materials and engineering
Archives of Physical Medicine and Rehabilitation
Taehan Kanho Hakhoe chi
Annals of rehabilitation medicine
Geriatrics & Gerontology International
Neurourology and Urodynamics
Japanese Journal of Geriatrics

For peer review only

Japanese Journal of Geriatrics
Journal of the American Geriatrics Society
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Brain Injury
Health Promotion International
Plos One
Nordisk Fysioterapi
Journal of Physical Therapy Science
Journal of Physical Therapy Science
Journal of Physical Therapy Science
Early Education and Development
Health Environments Research & Design Journal (HERD) (Sage Publications, Ltd.)
Journal of Housing for the Elderly
Teachers College Record
Journal of Housing for the Elderly
Annals of the Academy of Medicine Singapore
Aging Clinical and Experimental Research
Journal of the American Geriatrics Society
American Journal Geriatric Pharmacotherapy
African Health Sciences
Current medical research and opinion
Clinical Ophthalmology
Scandinavian journal of caring sciences
Journal of applied physiology (Bethesda, Md. : 1985)
Polski merkuriusz lekarski : organ Polskiego Towarzystwa Lekarskiego
Bulletin of St. Luke's College of Nursing
Radiation and environmental biophysics
Physiotherapy (United Kingdom)
Journal of Bodywork & Movement Therapies
Journal of Nutrition, Health & Aging
PloS one
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of the American Medical Directors Association
The journal of nutrition, health & aging
Jurnal Teknologi
Journal of Gerontological Nursing
Journal of Gerontological Nursing
International Nursing Review
Gerokomos
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
The journals of gerontology. Series A, Biological sciences and medical sciences
American journal of preventive medicine
Artificial Intelligence in Medicine
Journal of Medical Systems
Journal of geriatric physical therapy (2001)

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Asian Nursing Research
Ethnicity & Health
Journal of Epidemiology & Community Health
Journal of neurophysiology
Wiadomosci lekarskie (Warsaw, Poland : 1960)
Journal of the American Geriatrics Society
The journal of trauma and acute care surgery
Australian & New Zealand Journal of Public Health
Journal of Aging and Physical Activity
Danish medical journal
Archives of Physical Medicine & Rehabilitation
Journal of Physical Therapy Science
Physiological measurement
CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne

Journal of Rheumatology
Research Quarterly for Exercise and Sport
Health & Quality of Life Outcomes
Journal of Geriatric Cardiology
The American naturalist
Radar Sensor Technology Xx
Physical therapy

Educational Gerontology
Public Health Nursing
The Journal of frailty & aging
International Review of Victimology
Thorax
Thorax
Circulation research

Rheumatology
Nihon Ronen Igakkai zasshi. Japanese journal of geriatrics
The Laryngoscope
Journal of geriatric physical therapy (2001)
Journal of physical therapy science
Journal of Ethnopharmacology
Canadian journal on aging = La revue canadienne du vieillissement
Clinical journal of the American Society of Nephrology : CJASN
Journal of hypertension
Osteoporosis International
BMC Geriatrics
Acta otorhinolaryngologica Italica : organo ufficiale della Societa italiana di otorinolaringologia e chiru
Disability & Rehabilitation
Aging clinical and experimental research
Ecology

For peer review only

Orvosi hetilap
Clinical Rehabilitation
Journal of the American Geriatrics Society
The Korean journal of internal medicine
Proceedings of the Estonian Academy of Sciences
Journal de Th√©rapie Comportementale et Cognitive
Drugs & Aging
Age and ageing
Srpski Arhiv Za Celokupno Lekarstvo
PLoS ONE

Journal of the American College of Cardiology
Journal of Clinical Gerontology and Geriatrics
Current aging science
Journal of clinical virology : the official publication of the Pan American Society for Clinical Virology
Journal of the American Academy of Audiology
Gerontology
Journal of the American Geriatrics Society
European Respiratory Journal

British Journal of Nutrition
Archives of Gerontology and Geriatrics
Aging - Clinical and Experimental Research
Journal of Physiological Anthropology
Chronobiology International
Stroke (00392499)
Journal of Orthopaedic Science
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Journal of the American Geriatrics Society
The Journal of clinical endocrinology and metabolism
Archives of Gerontology and Geriatrics
Gait & posture
Physiotherapy (United Kingdom)
Physiotherapy (United Kingdom)
Bmc Geriatrics
Therapeutic apheresis and dialysis : official peer-reviewed journal of the International Society for Apl
Anthropologist
Sleep Medicine
Chest
Aging Clinical and Experimental Research
Geriatrics & gerontology international
Clinical Kinesiology: Journal of the American Kinesiotherapy Association
Quality of Life Research
BMC health services research
[Nihon k≈çsh≈´ eisei zasshi] Japanese journal of public health

For peer review only

Neurourology and Urodynamics
The National medical journal of India
Journal of Geriatric Physical Therapy
Tzu Chi Medical Journal
Tzu Chi Medical Journal
Osteoporosis International
The journal of trauma and acute care surgery
The nine degrees of autism: A developmental model for the alignment and reconciliation of hidden n
Neurology
Geriatric Nursing
Https://clinicaltrials.gov/show/nct03705364
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British Journal of Healthcare Assistants
British Journal of Community Nursing
Nursing Standard
Advances in Artificial Intelligence and Its Applications, Micai 2015, Pt Ii
Journal of Women's Health (15409996)
Heart (British Cardiac Society)
Social Behavior and Personality: An International Journal
Functional neurology
European Journal of Physical and Rehabilitation Medicine
The Journal of clinical endocrinology and metabolism
Journal of Applied Gerontology
Autonomic neuroscience : basic & clinical
Neurourology and Urodynamics
Journal of Urology
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
The Journal of frailty & aging
Disability & Rehabilitation
European Journal of Pain Supplements
Age and Ageing
Physiotherapy (United Kingdom)
Age & Ageing
Menopause
Australian Journal of Physiotherapy
Bone
American Journal of Epidemiology
The Journal of nutritional biochemistry
Hepatology (Baltimore, Md.)
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Hospital Medicine
Plastic and reconstructive surgery
Age and ageing

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Geriatrics & Gerontology International
Geriatrics & Gerontology International
Journal of Nutrition, Health and Aging
Physiotherapy theory and practice
Journal of the american geriatrics society
Journal of Bone and Mineral Research
Journal of the American Podiatric Medical Association
Physiotherapy Theory & Practice
Journal of Population Therapeutics and Clinical Pharmacology
Osteoporosis International
Journal of Neurologic Physical Therapy
Revue d'epidemiologie et de sante publique
Magnetic resonance imaging
Journal of the American Geriatrics Society
The Journal of trauma
Johns Hopkins Advanced Studies in Nursing
Aging clinical and experimental research
The Journals of Gerontology: Series A: Biological Sciences and Medical Sciences
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of the American Geriatrics Society
Journal of the American Medical Directors Association
The journals of gerontology. Series A, Biological sciences and medical sciences
Revista da Escola de Enfermagem da USP
Medical Journal of Australia
Journal of General Internal Medicine
American Family Physician
Gerontology
European Journal of Internal Medicine
BMC Geriatrics
European Geriatric Medicine
Internal Medicine Journal
Alzheimer's and Dementia
Social Work in Health Care
The Israel Medical Association journal : IMAJ
Journal of Clinical Gerontology & Geriatrics
Australasian Emergency Nursing Journal
Annals of the Academy of Medicine, Singapore
Physiotherapy
Arthritis and Rheumatism
Topics in stroke rehabilitation
Journal of Geriatric Physical Therapy
2009 Ieee 11th International Conference on Rehabilitation Robotics, Vols 1 and 2
Salmand-Iranian Journal of Ageing
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Joint commission journal on quality and patient safety
Value in Health

Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of General Internal Medicine
Physiotherapy Canada
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of Geriatric Physical Therapy
Journal of Nutrition, Health and Aging
International Journal of Nursing Studies
Journal of the American Medical Directors Association
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Physical Activity Review
Assistive Technology
Scandinavian Journal of Rheumatology, Supplement
Evidence Based Nursing
Journal of Advanced Nursing
Scandinavian journal of primary health care
2013 Ieee International Conference on Body Sensor Networks
Journal of Parkinsons Disease
Age and Ageing

Metas de Enfermer√≠a
Journal of Alzheimer's disease : JAD
Health and Quality of Life Outcomes
Translational Behavioral Medicine
European Geriatric Medicine
Arthritis Care & Research
Journal of General Internal Medicine
Rehabilitation nursing : the official journal of the Association of Rehabilitation Nurses
Journal of the American Geriatrics Society
Research in Nursing & Health
PloS one
Neurology
Drugs & Aging
Value in Health
Journal of Urmia Nursing & Midwifery Faculty
Brain Injury
European journal of physical and rehabilitation medicine
Archives of Physical Medicine & Rehabilitation
Applied Nursing Research
Journal of the American Medical Directors Association
Geriatric Nursing
BMC health services research

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Alzheimer's and Dementia
Consultant Pharmacist
Consultant Pharmacist
Journal of the American Geriatrics Society
Australian Journal of Primary Health
Movement Disorders
Alcohol and alcoholism (Oxford, Oxfordshire)
Journal of Nutrition Health & Aging
Anesthesiology
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Osteoporosis international : a journal established as result of cooperation between the European Fou
Movement disorders : official journal of the Movement Disorder Society
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Journal of Nutrition, Health and Aging
Clinical cardiology

Age
Worldviews on evidence-based nursing / Sigma Theta Tau International, Honor Society of Nursing
Diabetologia
The European respiratory journal
Journal of Clinical Rehabilitative Tissue Engineering Research
Motor Control
Gait & Posture
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Bmc Geriatrics
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JAMA internal medicine
Archives of gerontology and geriatrics
British journal of community nursing
Chest
Journal of Physical Therapy Science
American Journal of Geriatric Psychiatry
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Internal medicine journal
Archives of Physical Medicine & Rehabilitation
Sleep
Issues on Aging
International Psychogeriatrics
Home Healthcare Nurse
Journal of the American Geriatrics Society
Stroke
Journal of public health management and practice : JPHMP
Journal of Neurology
European journal of clinical pharmacology
The Medical journal of Australia

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Journal of Geriatric Physical Therapy
Physical Therapy
Alzheimer's and Dementia
International Journal of Nursing Studies
Neurology
Journal of Nursing Research (Taiwan Nurses Association)
Taiwan Journal of Public Health
Journal of the American College of Cardiology
Physical Therapy
Clinical interventions in aging
Journal of Nursing Research (Taiwan Nurses Association)
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Alzheimer's and Dementia
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Gait & Posture
Journal of Applied Biomechanics
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Journal of Intellectual Disability Research
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International journal of environmental research and public health
PLoS ONE
Injury Epidemiology
PLoS ONE
Physical & Occupational Therapy in Geriatrics
Physical & Occupational Therapy in Geriatrics
Journal of the American Geriatrics Society
Annals of Geriatric Medicine and Research
Diabetes Research & Clinical Practice
Journal of Geriatric Physical Therapy
Australian Health Review
Rhode Island medical journal
Clinical transplantation
The Journal of thoracic and cardiovascular surgery
ARYA Atherosclerosis
Ageing & Society
BMC geriatrics
The journal of primary prevention
Journal of injury & violence research
Osteoporosis International

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Journal of Women's Health
Annals of biomedical engineering
Ieee Transactions on Consumer Electronics
Pediatric nephrology (Berlin, Germany)
Journal of rehabilitation medicine
Studies in health technology and informatics
The European respiratory journal
Archives of Physical Medicine & Rehabilitation
International Journal of Environmental Research and Public Health
Diabetic medicine : a journal of the British Diabetic Association
Geriatrics & Gerontology International
International journal of chronic obstructive pulmonary disease
Disability and Health Journal
Age and Ageing
European Geriatric Medicine
Irish Journal of Medical Science
Journal of Bone and Mineral Research
Journal of the American College of Surgeons
Scandinavian Journal of Public Health
Accident; analysis and prevention
BMC Public Health
Disability & Rehabilitation
The Journal of clinical endocrinology and metabolism
Irish Journal of Medical Science
Journal of alternative and complementary medicine (New York, N.Y.)
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Epilepsy and Behavior
Proceedings of the 2013 7th International Conference on Pervasive Computing Technologies for Heal
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Journal of Physical Therapy Science
Archives of Physical Medicine and Rehabilitation
International Journal of Integrated Care (IJIC)
Journal of Transcultural Nursing

BMC musculoskeletal disorders
Journal of rural medicine : JRM
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Archives of Gerontology and Geriatrics
Geriatric Nursing

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Nihon Ronen Igakkai zasshi. Japanese journal of geriatrics
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Gait & Posture
Brazilian Journal of Physical Therapy / Revista Brasileira de Fisioterapia
Scandinavian Journal of Caring Sciences
Journal of internal medicine
Archives of Gerontology and Geriatrics
International Psychogeriatrics
Home Healthcare Nurse
Journal of Geriatric Physical Therapy
Journal of geriatric physical therapy (2001)
Cerebrovascular diseases (Basel, Switzerland)
Wwai '08: Proceedings of the 2nd International Conference on Waste Management, Water Pollution,
Aging clinical and experimental research
Bone
Journal of the American College of Cardiology (JACC)
Gerontology
Turkish Journal of Physiotherapy Rehabilitation
Klinik Psikofarmakoloji Bulteni
Archives of Gerontology and Geriatrics
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Gait & Posture
Gerontology
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine anc
Gait and Posture
Journal of geriatric cardiology : JGC
Journal of neurophysiology
Journal of biomechanics
Gait & posture
Osong Public Health and Research Perspectives
American Journal of Hypertension
Journal of the American Geriatrics Society
Indian Journal of Psychiatry
Journal of Aging & Health
Journal of the American Medical Directors Association
International Journal of Environmental Research and Public Health
Physiotherapy (United Kingdom)
Bulletin of St. Luke's College of Nursing
Cahiers de l'Annee Gerontologique
Physical therapy
PloS one
American Journal of Occupational Therapy
Ageing & Society
Journal of Rehabilitation Research & Development

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Occupational therapy in health care
Archives of Physical Medicine and Rehabilitation
Archives of physical medicine and rehabilitation
Journal of the American Geriatrics Society
Clinical Nursing Research
Journal of Adolescence
British Journal of Occupational Therapy
Journal of Geriatric Physical Therapy
Clinical research in cardiology : official journal of the German Cardiac Society
Aging Clinical & Experimental Research
Geriatric nursing (New York, N.Y.)
The European respiratory journal
Clinical Rehabilitation
Journal of Pharmacy Practice
Medical Care
European Geriatric Medicine

Journal of Bone and Mineral Research
Journal of cystic fibrosis : official journal of the European Cystic Fibrosis Society
Current osteoporosis reports
Annals of Pharmacotherapy
Journal of the American Geriatrics Society
Canadian Journal on Aging
The journal of trauma and acute care surgery
Canadian Pharmacists Journal (Sage Publications Inc.)
Canadian pharmacists journal : CPJ = Revue des pharmaciens du Canada : RPC
Nicotine & Tobacco Research
Journal of the American Medical Directors Association
Archives of Gerontology and Geriatrics
Singapore Nursing Journal
European journal of internal medicine
Age and Ageing
Canadian Journal on Aging
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
Archives of Physical Medicine and Rehabilitation
BMC nephrology
Neuromuscular Disorders
Clinical interventions in aging
Australian Journal of Primary Health
The American journal of geriatric psychiatry : official journal of the American Association for Geriatric
Pharmacoepidemiology and Drug Safety
Archives of Orthopaedic and Trauma Surgery
The British journal of radiology
Journal of neurology
Journal of biomechanics
Arthritis and Rheumatism

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Osteoarthritis and Cartilage
Blood
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Medical Directors Association
Acta neurologica Scandinavica
Movement Disorders
European journal of neurology
Digestive Diseases & Sciences
Emergency medicine journal : EMJ
Atencion Primaria
Journal of physical therapy science
Bone
Movement Disorders
Medicina
Journal of Pharmacy Practice
Bone
JMIR research protocols
Journal of the American Geriatrics Society
Medical Care
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
The journals of gerontology. Series A, Biological sciences and medical sciences
The Journal of urology
Journal of Urology
Journal of the American Medical Directors Association
International Journal of Stroke
Bone
Digestive Diseases
American Journal of Physical Medicine & Rehabilitation
Hypertension (Dallas, Tex. : 1979)
The Ochsner journal
European Geriatric Medicine
BioMed research international
European Geriatric Medicine
Osteoporosis International
Journal of Biomechanics
Age and ageing
Journal of Neuroscience Nursing
Frontiers in Physiology
International Journal of Clinical Pharmacy
Sleep
Clinical Gerontologist
Bmc Geriatrics
Parkinsonism and Related Disorders
Journal of Motor Behavior
Disability and rehabilitation

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Clinical schizophrenia & related psychoses

Gait & Posture
Journal of Manipulative and Physiological Therapeutics
The American journal of geriatric psychiatry : official journal of the American Association for Geriatric
Postgraduate Medical Journal

Journal of the American Geriatrics Society
Age & Ageing
JAMA surgery
International journal of clinical pharmacology, therapy, and toxicology
Technology & Health Care

Journal of the American Geriatrics Society
Drugs & Aging
Annals of Pharmacotherapy
Journal of Bone and Mineral Research

PLoS ONE
Journal of geriatric psychiatry and neurology
Age and Ageing
Physiotherapy
Annals of epidemiology
Age and Ageing
Physical Therapy
Physical therapy
Parkinson's Disease
Bmc Geriatrics
Clinical Interventions in Aging
Medicine
Journal of the American Geriatrics Society
Journal of Epidemiology & Community Health
Archives of Physical Medicine & Rehabilitation
Physiotherapy Canada
Clinical Interventions in Aging
BMJ open

Journal of the American Geriatrics Society
BMC geriatrics
Clinical and experimental immunology
Health and quality of life outcomes
Journal of the Medical Association of Thailand
Journal of the American Geriatrics Society
Arthritis Care and Research
Canadian journal of diabetes
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and

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Aging & Mental Health
Brain Topography
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
American Journal of Managed Care
American journal of epidemiology
International Journal of Nursing Studies
The Journal of bone and joint surgery. American volume
Arthroscopy : the journal of arthroscopic & related surgery : official publication of the Arthroscopy As
Journal of the American Medical Directors Association
WMJ : official publication of the State Medical Society of Wisconsin
Journal of Gerontological Nursing
Clinical interventions in aging
Revue Neurologique
Revue Neurologique
Journal of Geriatric Physical Therapy
Journal of the american geriatrics society
Archives of Physical Medicine and Rehabilitation
Journal of Bone and Mineral Research
BMC Geriatrics
Irish Journal of Medical Science
Archives of Psychiatric Nursing
Integrated Computer-Aided Engineering
Journal of the American Geriatrics Society
Clinical Rehabilitation
Aging - Clinical and Experimental Research
Nephrology, dialysis, transplantation : official publication of the European Dialysis and Transplant Ass
Journal of the American Medical Association
Osteoporosis International
Arthritis and Rheumatology
Postgraduate medical journal
Physiological and biochemical zoology : PBZ
Alzheimer's and Dementia
Schizophrenia research
Public Health
Journal of Bone and Mineral Research
Osteoporosis international : a journal established as result of cooperation between the European Fou

Aging Clinical and Experimental Research
Disability & Rehabilitation
Ageing & Society
Journal of Allied Health
Clinical Rheumatology
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of the American Medical Directors Association
Journal of the American Geriatrics Society
Physiotherapy Research International

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British Journal of Nutrition
Archives of gerontology and geriatrics
Hong Kong medical journal = Xianggang yi xue za zhi
The journal of nutrition, health & aging
Medical Journal of Australia
Gerontology
Journal of the American Geriatrics Society
Journal of Physiology-London
Age & Ageing
Journal of Neurology
Dementia & Geriatric Cognitive Disorders
Interciencia
Journal of the American Geriatrics Society
Neurology
Public Health
Circulation
PloS one
BMC health services research
Stroke
Journal of Bone and Mineral Research
Annals of the Rheumatic Diseases
British journal of haematology

Physiotherapy (United Kingdom)
Journal of Advanced Nursing
Australian journal of public health
International Journal of Environmental Research and Public Health
Osteoporosis International
Journal of Clinical Densitometry
Journal of Bone and Mineral Research
Aging Health

Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Bone
Virology
Annals of Emergency Medicine
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
MEDICC review
American Journal of Alzheimer's Disease and Other Dementias
Journal of Trauma
Chinese Nursing Research
Clinical Rehabilitation
International psychogeriatrics
Acta Neurologica Scandinavica
Journal of Trauma-Injury Infection and Critical Care
Osteoporosis International

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Osteoporosis International
British Journal of Occupational Therapy
Journal of Rehabilitation Medicine
Injury
Journal of the American Geriatrics Society
Cerebrovascular diseases (Basel, Switzerland)
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
Journal of the American Geriatrics Society
Plos One
Journal of clinical nursing
PloS one
PloS one
Frontiers in public health
International Journal of Geriatric Psychiatry
Journal of geriatric physical therapy (2001)
Journal of Clinical Nursing
Hong Kong medical journal = Xianggang yi xue za zhi
Geriatrics & Gerontology International
Scientific Reports
Eurasip Journal on Advances in Signal Processing
Age and Ageing
Journal of Neurology, Neurosurgery & Psychiatry
BMC geriatrics
Diabetic medicine : a journal of the British Diabetic Association
British journal of cancer
Journal of Clinical Gerontology and Geriatrics
Osteoporosis International
Annals of Rehabilitation Medicine-Arm
Korean Journal of Adult Nursing
Geriatric Nursing
Anaesthesia and intensive care
American Journal of Audiology
Journal of Vestibular Research: Equilibrium and Orientation
Otology & neurotology : official publication of the American Otological Society, American Neurotolog

Journal of applied gerontology : the official journal of the Southern Gerontological Society
Journal of Alternative & Complementary Medicine
BMC geriatrics
Kidney international
Journal of Manipulative & Physiological Therapeutics
The Australian and New Zealand journal of psychiatry
Archives of Physical Medicine & Rehabilitation
Physical Therapy
International Journal of Mental Health Nursing
Anales del Sistema Sanitario de Navarra

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Obesity research & clinical practice
Physical Therapy
American journal of physical medicine & rehabilitation / Association of Academic Physiatrists
Western Journal of Nursing Research
Annals of Pharmacotherapy

BMC neurology
Critical care (London, England)
Journal of the American Medical Directors Association
Acta chirurgiae orthopaedicae et traumatologiae Cechoslovaca
Journal of the American Geriatrics Society
Journal of Rehabilitation Research & Development
The New England Journal of Medicine
PLoS ONE
Journal of the American Geriatrics Society
Annals of human genetics
Geriatrics & Gerontology International
Geriatrics & gerontology international
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of the American Medical Association
Movement Disorders
Journal of Trauma
PLoS Medicine
Journal of interpersonal violence
Journal of Gerontological Nursing
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of the American Geriatrics Society
Consultant Pharmacist
The Journal of frailty & aging
American Journal of Geriatric Psychiatry
Maturitas
Journal of Pain
Hong Kong medical journal = Xianggang yi xue za zhi / Hong Kong Academy of Medicine
Clinical Interventions in Aging
Brain injury. Conference: 11th world congress on brain injury of the international brain injury associa
European journal of clinical pharmacology
International Journal of Geriatric Psychiatry
Osteoporosis International
American journal of therapeutics
Herd
Frontiers in Public Health
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Medical Directors Association

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The Gerontologist
Medical Care
Journal of Aging & Health
Archives of Gerontology and Geriatrics
The American Journal of dermatopathology
Aging clinical and experimental research
Journal of the American Geriatrics Society
PLoS medicine
The journal of trauma and acute care surgery
Journal of the American Geriatrics Society
The Journal of clinical endocrinology and metabolism
International journal of surgery (London, England)
PM and R
Archives of neurology
Aging Clinical & Experimental Research
PM & R : the journal of injury, function, and rehabilitation
Journal of ultrasound in medicine : official journal of the American Institute of Ultrasound in Medicine
Disability and Rehabilitation
Health expectations : an international journal of public participation in health care and health policy
Journal of the American Geriatrics Society
Osteoporosis International
Movement Disorders
Scientific Reports
Gerontology
Aging clinical and experimental research
Journal of aging and physical activity
BMJ quality & safety

Alzheimer Disease & Associated Disorders
Movement disorders : official journal of the Movement Disorder Society
Prevention Science
Journal of the American Medical Directors Association
Safety Science
European Geriatric Medicine
Calcified tissue international
Annals of Internal Medicine
Current aging science
Neurourology and urodynamics
Journal of Bone and Mineral Research
Journal of Neuroengineering and Rehabilitation

Singapore medical journal
International Journal of Geriatric Psychiatry
Physical Therapy
International Journal of Stroke
American journal of health-system pharmacy : AJHP : official journal of the American Society of Healt

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Revista De Nefrologia Dialisis Y Trasplante
Revista de Nefrologia, Dialisis y Trasplante
The journal of nutrition, health & aging
Injury
Maturitas
Journal of Nutrition, Health and Aging
Gait & posture
The Laryngoscope
International Journal of Social Psychiatry
Diabetes Research and Clinical Practice
Journal of Clinical Nursing
Health & Social Care in the Community
Journal of the american medical directors association
Journal of Surgical Research
Journal of Surgical Research
Gait & Posture
BMC Health Services Research
Stroke (00392499)
Technology and health care : official journal of the European Society for Engineering and Medicine
Journal of Bone and Mineral Research
CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne
The American journal of clinical nutrition
Disability and rehabilitation
Journal of Bone and Mineral Research
Journal of the American Medical Directors Association
Age and ageing
Journal of Bone and Mineral Research
Clinical Nutrition
Proteomics
Journal of the American Medical Directors Association
American Journal of Managed Care
Internal medicine journal
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
International Nursing Review
Asian Journal of Nursing Studies
Arthritis and Rheumatology
International Journal of Nursing Studies
Pharmacoepidemiology and Drug Safety
Journal of applied physiology (Bethesda, Md. : 1985)
Plos one
Archives of Gerontology and Geriatrics
Physiotherapy (United Kingdom)
Physical Therapy
[Nihon koshu eisei zasshi] Japanese journal of public health
Journal of the American Geriatrics Society
Public Health Nursing

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Archives of Physical Medicine & Rehabilitation
Journal of Community Health Nursing
Journal of Aging and Physical Activity
Journal of Clinical Epidemiology
European Geriatric Medicine
Zeitschrift fur Gerontologie und Geriatrie
Gerontology
Healthy Aging Research
Gerontologist
Osteoporosis International
Geriatrics Today: Journal of the Canadian Geriatrics Society
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
Journal of neurology
Journal of neurology
Journal of rehabilitation medicine
Annals of internal medicine
The Journal of clinical endocrinology and metabolism
Ear, nose, & throat journal
Journal of clinical nursing
Journal of the American Geriatrics Society
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Aging and Physical Activity
Menopause (New York, N.Y.)
Menopause (New York, N.Y.)
Schweizerische medizinische Wochenschrift

European Geriatric Medicine
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Journal of Geriatric Physical Therapy
Journal of Orthopaedic Research
Methods of information in medicine
Medical Care Research and Review
Physiology & behavior
Journal of General Internal Medicine
Bmc Health Services Research
Journal of geriatric physical therapy (2001)
Journal of the American Medical Directors Association
Journal of Clinical Outcomes Management
Journal of the American Medical Informatics Association
International journal of geriatric psychiatry
Sleep: Journal of Sleep and Sleep Disorders Research
NeuroRehabilitation
European Geriatric Medicine
Aging - Clinical and Experimental Research
Internist

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BMC geriatrics
Age and Ageing
BMJ (Clinical research ed.)
Journal of Nutrition, Health & Aging
Medicine
Journal of Clinical Epidemiology
Journal of the American Geriatrics Society
Archives of Physical Medicine & Rehabilitation

Clinical rehabilitation
Gait & posture
Farmacia
Neurorehabilitation & Neural Repair
Journal of Epidemiology & Community Health
Experimental Gerontology
Kai Tiaki Nursing Research
Journal of the american geriatrics society

Journal of Nutrition, Health and Aging
The journals of gerontology. Series A, Biological sciences and medical sciences
Hong Kong Journal of Occupational Therapy
Australian and New Zealand journal of public health
Archives of family medicine
Age and ageing
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Journal of geriatric physical therapy (2001)
BMC Pulmonary Medicine
Journal for Nurse Practitioners
Rural & Remote Health
Frontiers in Public Health
Journal of Investigative Medicine
The Lancet
Current aging science
Thorax
International Journal of Therapy & Rehabilitation
Journal of Asthma
Journal of Geriatric Physical Therapy
Yonago Acta Medica
International Journal of Therapy & Rehabilitation
Jmir Mhealth and Uhealth
Work
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Progress in Community Health Partnerships-Research Education and Action
Clinical Interventions in Aging
Facilitating End-of-Life Care Discussions with Older Chinese Americans & Their

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Complementary therapies in medicine
Nursing and midwifery studies
Journal of the American Geriatrics Society
American journal of hypertension
Journal of the American Medical Directors Association
British Journal of Clinical Pharmacology
Osteoporosis international : a journal established as result of cooperation between the European Fou
Disability and rehabilitation
Alternative therapies in health and medicine

Physical Therapy
Journal of Aging and Physical Activity
The Journal of family practice
Medical Principles & Practice
European Journal of Physiotherapy
Physiotherapy theory and practice
Journal of Developmental and Physical Disabilities
Zeitschrift f√ºr Gerontologie und Geriatrie
Archives of gerontology and geriatrics
Archives of gerontology and geriatrics
Archives of Physical Medicine and Rehabilitation
PLoS ONE
Archives of Physical Medicine & Rehabilitation
Journal of Foot and Ankle Research
Arthritis and Rheumatism
European Geriatric Medicine
Gerontology & geriatric medicine
Archives of Physical Medicine & Rehabilitation
Restoration Ecology
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Annals of Pharmacotherapy
Physical Therapy
Journal of Trauma & Acute Care Surgery

Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
Archives of gerontology and geriatrics
Gerontology
Journal of Aging & Health
Accident; analysis and prevention
New England Journal of Medicine
BMC gastroenterology
International journal of clinical practice
Journal of Geriatric Physical Therapy
Preventive medicine reports
Journal of Aging and Physical Activity

For peer review only

Journal of Engineering Science and Technology
The Annals of thoracic surgery
Assistive technology
Journal of the American Geriatrics Society
Revista espanola de anestesiologia y reanimacion
International journal of cardiology
European Geriatric Medicine
Aging Clinical and Experimental Research

Journal of psychiatric research
Topics in Geriatric Rehabilitation
Brazilian Journal of Physical Therapy
Clinical rehabilitation
European review of aging and physical activity : official journal of the European Group for Research ir
American Journal of Health Education

Scandinavian journal of caring sciences
Current gerontology and geriatrics research
BMC Geriatrics
Diabetes Technology and Therapeutics
2011 Ieee International Conference on Systems, Man, and Cybernetics

Anaesthesia
Journal of the American Academy of Child & Adolescent Psychiatry
BMC health services research
Maturitas
Annals of Global Health
Journal of Studies on Alcohol and Drugs
Gut
Australian Economic Review
Rehabilitation nursing : the official journal of the Association of Rehabilitation Nurses
Wireless Sensor Networks, Ewsn 2014
European Journal of Applied Physiology
Occupational Therapy in Health Care
European Geriatric Medicine
Physiological Measurement
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Age and ageing
Physiological measurement
Geriatric Nursing
Clinical and experimental pharmacology & physiology
Acta Psychiatrica Scandinavica
Age and Ageing
Journal of nursing management
Experimental aging research
Journal of the American Geriatrics Society

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Geriatric Nursing

Annals of epidemiology
Nursing Quebec
Physical & Occupational Therapy in Geriatrics
Gerontology
Journal of Bodywork & Movement Therapies
Journal of Biomechanics
Journal of the American Geriatrics Society
Medicine and Science in Sports and Exercise
Quality of Life Research
Canadian journal on aging = La revue canadienne du vieillissement

International Urology and Nephrology
Home Health Care Management & Practice
Nephrology Dialysis Transplantation
Journal of Aging and Physical Activity
Asia-Pacific journal of ophthalmology (Philadelphia, Pa.)
Military medicine
Journal of the American Medical Directors Association
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
The Journal of clinical endocrinology and metabolism
Medical Engineering and Physics
Archives of Gerontology and Geriatrics
Journal of Pain and Symptom Management
Heart failure reviews
Zeitschrift fur Gerontologie und Geriatrie
Physical & Occupational Therapy in Geriatrics
Gerokomos
Colombia Medica
European Geriatric Medicine
American journal of hypertension
Revista espanola de cardiologia
Revista Rol de Enfermer√≠a
Revista Rol De Enfermeria
Journal of the American Geriatrics Society
Brazilian Journal of Physical Therapy / Revista Brasileira de Fisioterapia
Journal of nutritional science and vitaminology
Preventive medicine
Arthritis and Rheumatism
Arthritis and Rheumatism
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Current Cardiovascular Risk Reports
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Archives of Gerontology & Geriatrics
Physiotherapy theory and practice

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Physiotherapy theory and practice
Aging Clinical and Experimental Research
Zeitschrift Fur Gerontologie Und Geriatrie
Allergy and Asthma Proceedings
Maturitas
Physical Therapy
Geriatrics & Gerontology International
Tijdschrift voor Gerontologie en Geriatrie
Tijdschrift voor Gerontologie en Geriatrie
International Journal of Cardiology
Disability and rehabilitation. Assistive technology
Revista Terapia Manual
Lancet (London, England)
12th International Conference on Intelligent Environments - Ie 2016
Bmc Geriatrics
Disability and Rehabilitation
Acta diabetologica
Archives of gerontology and geriatrics
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Journal of Geriatric Physical Therapy
Journal of the American Medical Directors Association
Cochrane Database of Systematic Reviews
American journal of industrial medicine
Journal of the American Geriatrics Society
American Journal of Public Health
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Journal of Clinical Epidemiology
Journal of the American Geriatrics Society
Journal of the American Medical Association
JAMA - Journal of the American Medical Association
Gerontology
Neurology Report
Journal of Public Health
Zeitschrift fur Gerontologie und Geriatrie
Journal of Sleep Research
Physical & Occupational Therapy in Geriatrics
Journal of Gerontological Social Work
Clinics in Geriatric Medicine
Annual review of medicine
Atherosclerosis
Injury
The Journal of clinical endocrinology and metabolism
Journal of the American College of Nutrition
Journal of Patient Safety
Journal of Geriatric Physical Therapy
Journal of the American Geriatrics Society

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Journal of affective disorders
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
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Wiadomo≈õci lekarskie (Warsaw, Poland : 1960)
Waste management (New York, N.Y.)
Psychogeriatrics : the official journal of the Japanese Psychogeriatric Society
Journal of Nursing Care Quality
American Family Physician
Journal of physical therapy science
Osteoporosis International
Ict Innovations 2015: Emerging Technologies for Better Living
Salud Mental
Environmental health perspectives
Age and Ageing
International Journal of Older People Nursing
Osteoporosis International
Topics in Geriatric Rehabilitation
Osteoporosis International
Revista clinica espanola
Psychiatry Research
European Geriatric Medicine
BMC geriatrics
European Geriatric Medicine
The international journal of tuberculosis and lung disease : the official journal of the International Un
Journal of the American Geriatrics Society
Arthritis Care and Research
Le Journal medical libanais. The Lebanese medical journal
Journal of the American Geriatrics Society
Sleep and Biological Rhythms
Rural and Remote Health
Research on Aging
Research on Aging
Journal of acupuncture and meridian studies
Journal of Geriatric Physical Therapy
Journal of the American Medical Directors Association
Enfermeria Clinica
International Review of Victimology
Age and ageing
European Geriatric Medicine
Journal of Research on Adolescence
Revista da Associacao Medica Brasileira (1992)
The American journal of geriatric psychiatry : official journal of the American Association for Geriatric
Experimental brain research
Archives of Gerontology & Geriatrics

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Experimental Aging Research
Occupational and environmental medicine
Hearing research
Archives of Gerontology and Geriatrics
International journal of geriatric psychiatry
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
European Review of Aging and Physical Activity
Health, Risk & Society
Archives of Gerontology & Geriatrics
Home Health Care Management & Practice
Home Healthcare Nurse
Home Health Care Services Quarterly
BMC Geriatrics
Maturitas
Journal of Cachexia, Sarcopenia and Muscle
Canadian Journal of Occupational Therapy
Gerontologist
Journal of Geriatric Physical Therapy

Age and ageing
Research on Aging
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Current Opinion in Clinical Nutrition & Metabolic Care
Age & Ageing
Australasian Journal on Ageing
Complementary Therapies in Clinical Practice
Journal of Gerontological Nursing
Journal of bone and mineral metabolism
The American journal of medicine
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Japanese Journal of Geriatrics
Psychiatry and clinical neurosciences
European Journal of Epidemiology
PloS one
Rejuvenation research
Quality in Ageing & Older Adults
Age and Ageing
Journal of Housing for the Elderly
Digestive diseases and sciences
Journal of hepatology
QJM : monthly journal of the Association of Physicians
Pharmacological Research
Sleep medicine
Journal of Clinical Nursing
International journal of older people nursing
Internet Journal of Allied Health Sciences and Practice

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Art Therapy
Journal of the American Pharmacists Association
Aging Clinical & Experimental Research
Topics in Geriatric Rehabilitation
Osteoporosis International
Clinical physiology (Oxford, England)

Journal of Head Trauma Rehabilitation
Journal of hypertension
BMC geriatrics
Nagoya Journal of Medical Science
Emergency Medicine Journal
Brain Injury
Journal of manipulative and physiological therapeutics
Chiropractic and Osteopathy
Informatics for Health & Social Care
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Journal of Geriatric Physical Therapy
Japanese Journal of Physical Fitness and Sports Medicine
Nicotine & tobacco research : official journal of the Society for Research on Nicotine and Tobacco
Journal of Clinical Epidemiology
Modern Rheumatology
Archives of Gerontology & Geriatrics
Journal of Aging & Physical Activity
Injury
BJOG : an international journal of obstetrics and gynaecology
Archives of gerontology and geriatrics
Reproduction, nutrition, development
Gerontologist
Neurology
International Disability Studies
Familiendynamik:Interdisziplinaere Zeitschrift fuer systemorientierte Praxis und Forschung
British journal of sports medicine
Medicine
Disability & Rehabilitation: Assistive Technology
Geriatrics & Gerontology International
Gerontology
The Annals of pharmacotherapy
European Geriatric Medicine
Bmc Geriatrics
International Journal of Nursing Studies
Psychology at the turn of the millennium, Vol. 1: Cognitive, biological, and health perspectives.
Clinical Nutrition
Applied radiation and isotopes : including data, instrumentation and methods for use in agriculture, i
The journal of nutrition, health & aging
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Technology and health care : official journal of the European Society for Engineering and Medicine
Pharmacoepidemiology and drug safety
The Journal of rural health : official journal of the American Rural Health Association and the Nationa
Pediatric Physical Therapy
JBI database of systematic reviews and implementation reports
Journal of the American Heart Association
Human mutation
Journal of the American Medical Directors Association
Health, Risk and Society
Annals of Long Term Care
Clinical Interventions in Aging
Scottish Medical Journal
Rejuvenation Research
Journal of Nutrition, Health & Aging
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American Journal of Alzheimer's Disease and other Dementias
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The New England journal of medicine
Archives of Physical Medicine and Rehabilitation
European Geriatric Medicine
Cerebellum (London, England)
Journal of hypertension
Revista brasileira de enfermagem
Evidence-based Complementary and Alternative Medicine
Osteoarthritis and cartilage
Journal of the American Society of Nephrology : JASN
Journal of Urban Health-Bulletin of the New York Academy of Medicine
Diabetes research (Edinburgh, Scotland)
Canadian Journal on Aging
Journal of Patient Safety
Journal of patient safety
Annales de cardiologie et d'angeiologie

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Australian Occupational Therapy Journal
Fisioterapia e Pesquisa
Journal of the American Geriatrics Society
Age & Ageing
Journal of Intellectual Disability Research
Medical Care
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention

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Archives of Physical Medicine and Rehabilitation
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Journal of Aging & Health
Disability and rehabilitation
The American journal of cardiology
Archives of Gerontology & Geriatrics
Archives of Gerontology & Geriatrics
Journal of nursing management
European Journal of Clinical Pharmacology
Revista De Saude Publica
Disability and rehabilitation
Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
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Community dentistry and oral epidemiology
Revista de saude publica
JAMA ophthalmology
Annals of internal medicine
Acta Physiologica
Journal of Assistive Technologies
Zeitschrift f√ºr Gerontologie und Geriatrie
Gait and Posture
Annals of Long-Term Care
Bmj-British Medical Journal
Health psychology : official journal of the Division of Health Psychology, American Psychological Asso
European heart journal
Preventing Chronic Disease
Science and Sports
Personal and Ubiquitous Computing
PeerJ
Journal of clinical oncology : official journal of the American Society of Clinical Oncology
Insurance Regulation and Supervision in Economies in Transition
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Journal of Nervous and Mental Disease
Birth: Issues in Perinatal Care

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Physiotherapy
Surgical neurology
Journal of human hypertension
Hypertension (Dallas, Tex. : 1979)
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Scandinavian Journal of Public Health

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Consultant Pharmacist
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European Geriatric Medicine
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Minerva anestesiologica
Educational Gerontology

Female Pelvic Medicine and Reconstructive Surgery
Iet Communications
Blood pressure monitoring
Blood pressure monitoring
Advances in reading/language research: A research annual, Vol. 5: Literacy through family, communit
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Gait & posture
Journal of the American Geriatrics Society
Annals of the Academy of Medicine Singapore
Canadian Journal of Emergency Medicine
Giornale di Gerontologia
Neurourology and Urodynamics
Cardiopulmonary Physical Therapy Journal

Journal of Correctional Health Care
Swiss Journal of Psychology
International journal of psychiatry in medicine
Bmc Geriatrics

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Saudi Medical Journal
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Praxis
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Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Gerontology
Family Practice
Circulation
Annals of the rheumatic diseases

Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Diabetes, obesity & metabolism
Sozial-Und Praventivmedizin
The Journal of neuroscience : the official journal of the Society for Neuroscience
Evaluation and Program Planning

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Family & Community Health: The Journal of Health Promotion & Maintenance
AONE's Leadership Prospectives
Jama
Drugs & aging
Scandinavian journal of caring sciences
Journal of clinical nursing
Psychological assessment
Pm&R
Terapevticheskii arkhiv
Sleep: Journal of Sleep and Sleep Disorders Research
Annales de Toxicologie Analytique
Depression research and treatment
Jornal brasileiro de nefrologia : 'orgao oficial de Sociedades Brasileira e Latino-Americana de Nefrolog
Age
The Journal of allergy and clinical immunology
Turkish journal of medical sciences
Journal of the American Geriatrics Society
American Journal of Respiratory and Critical Care Medicine
Journal of the American Geriatrics Society
Biomed Research International
American journal of public health
Journals of Gerontology
European heart journal
Irish Journal of Medical Science
Neurourology and Urodynamics
Osteoarthritis and Cartilage
BMC geriatrics

Springerplus
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
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PloS one
Bmc Geriatrics
Journal of Clinical Geropsychology
Injury Prevention (1353-8047)
Scandinavian journal of gastroenterology
2016 Progress in Electromagnetics Research Symposium
International psychogeriatrics
Geriatrics & Gerontology International
Archives of Physical Medicine and Rehabilitation
The Journal of trauma
International Journal of Geriatric Psychiatry
Journal of Physiotherapy
Australian Health Review
Foundation Years

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Journal of telemedicine and telecare
Clinical science (London, England : 1979)
Age and ageing
Arthritis Care & Research
Journal of the American Medical Directors Association
Irish Journal of Medical Science
Irish Journal of Medical Science
Irish Journal of Medical Science
Geriatrics and Gerontology International
Gait and Posture
American journal of hypertension
Sports medicine (Auckland, N.Z.)
Journal of Aging and Physical Activity
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Respiratory physiology & neurobiology
Osteoporosis International
Collegium antropologicum
Family Practice

Geriatrics & gerontology international
Pacing and clinical electrophysiology : PACE
Journal of hypertension
The Gerontologist
Turkish Journal of Physical Medicine & Rehabilitation / Turkiye Fiziksel Tip ve Rehabilitasyon Dergisi
Turkiye Fiziksel Tip Ve Rehabilitasyon Dergisi-Turkish Journal of Physical Medicine and Rehabilitation
The British journal of nutrition
Journal of Applied Psychology

Archives of Gerontology and Geriatrics
Annals of rehabilitation medicine
Annals of the Royal College of Surgeons of England
Gait & Posture
Journal of Population Therapeutics and Clinical Pharmacology
Archives of Physical Medicine & Rehabilitation
Annals of Physical and Rehabilitation Medicine

The training of children: A book for young teachers.
Perceptual and Motor Skills
Ambulatory Pediatrics
Physical and Occupational Therapy in Pediatrics
Public Health Nursing
Prehospital Emergency Care
Chest
Experimental nephrology
The Journal of surgical research
Transfusion Medicine

For peer review only

Journal of the American Geriatrics Society
Academic Emergency Medicine
Revista de Saude Publica
Experimental gerontology
Toxicology and industrial health
EMA - Emergency Medicine Australasia
Signal Processing, Sensor/Information Fusion, and Target Recognition Xxiii
Annals of Emergency Medicine

Informatics for Health & Social Care
2016 Second International Conference on Science Technology Engineering and Management
The journal of the American Academy of Psychoanalysis and Dynamic Psychiatry
Behavioral medicine (Washington, D.C.)
The Journal of the Acoustical Society of America
Artificial Intelligence Applications and Innovations
Ieee Journal of Biomedical and Health Informatics
Nutricion Hospitalaria
Disability & Rehabilitation: Assistive Technology
International Journal on Artificial Intelligence Tools
Aging clinical and experimental research
European Geriatric Medicine
Psychology & health
2013 Ieee International Conference on Visual Communications and Image Processing

Journal of Hospital Medicine
Australian and New Zealand Journal of Medicine
Current Medical Research & Opinion
Prehospital and disaster medicine
Aging Male
Archives of gerontology and geriatrics
Journal of General Internal Medicine
Journal of Community Health
Nutrients
Academic Emergency Medicine
Journal of Epidemiology & Community Health
Academic Emergency Medicine
World journal of surgery
Bmj Open
Geriatrics & Gerontology International
International Emergency Nursing
Journal of interpersonal violence

Journal of Prosthetics & Orthotics (JPO)
Californian Journal of Health Promotion
Robotics and Autonomous Systems
Journal of the American Geriatrics Society

For peer review only

2016 Ieee International Conference on Communication Systems
Archives of gerontology and geriatrics
Accident Analysis and Prevention
Brain Injury
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
International journal of injury control and safety promotion
International Journal of Qualitative Studies on Health and Well-Being
Neurourology and Urodynamics
Disability & Rehabilitation: Assistive Technology
Cjem
Bmc Geriatrics
Addictive behaviors
American Journal of Preventive Medicine
Irish Journal of Medical Science
Physiotherapy Theory and Practice
Australian Health Review
Sensors
Neurodegenerative disease management
Annales de readaptation et de medecine physique : revue scientifique de la Societe francaise de reed
European neurology
Journal of Rehabilitation Research and Development
BMC Neurology
Journal of Parkinson's disease
Movement disorders : official journal of the Movement Disorder Society
Ieee Journal of Biomedical and Health Informatics
Movement disorders : official journal of the Movement Disorder Society
Neurobiology of disease
Journal of neurology
Healthcare Technology Letters
Behavioural Brain Research
European journal of neurology
Archives of physical medicine and rehabilitation
Parkinsonism & related disorders
Physical Therapy
Australasian Emergency Nursing Journal
Studies in health technology and informatics
Neurology
Geriatrics & gerontology international
Clinical autonomic research : official journal of the Clinical Autonomic Research Society
Journal of rehabilitation research and development
Archives of physical medicine and rehabilitation
Physical Therapy
Physical therapy
Journal of clinical neuroscience : official journal of the Neurosurgical Society of Australasia
Journal of Aquatic Physical Therapy
Archives of Physical Medicine & Rehabilitation

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Frontiers in Aging Neuroscience
The American journal of geriatric psychiatry : official journal of the American Association for Geriatric
Annals of Physical and Rehabilitation Medicine
Central European Journal of Nursing & Midwifery

Journal of biomechanics
Physiology & behavior
Peritoneal dialysis international : journal of the International Society for Peritoneal Dialysis
Journal of Hypertension
Value in Health
Archives of Physical Medicine & Rehabilitation
Archives of Gerontology & Geriatrics
Archives of gerontology and geriatrics
Archives of Gerontology and Geriatrics
Archives of Gerontology and Geriatrics
Gazzetta Medica Italiana Archivio per le Scienze Mediche
Gazzetta Medica Italiana Archivio per le Scienze Mediche
International Journal of Gerontology
Quality of Life Research
Journal of the American Geriatrics Society
Hospital Physician
European Psychiatry
Journal of the American Medical Directors Association
Injury
New Zealand Medical Journal
Clinical calcium
Der Unfallchirurg
Clinics in Geriatric Medicine
Archives of Physical Medicine and Rehabilitation
Journal of psychosomatic research
Clinical Nursing Research
Journal of the American Medical Directors Association
Western Journal of Nursing Research
Arquivos brasileiros de cardiologia
Journal of Nursing Law
Circulation
Physiotherapy research international : the journal for researchers and clinicians in physical therapy
Archives of Physical Medicine and Rehabilitation
Osteoporosis International
Journal of Bone and Mineral Metabolism
Journal of Bone and Mineral Research
Rehabilitation nursing : the official journal of the Association of Rehabilitation Nurses

European Journal of Hospital Pharmacy
International Journal of Geriatric Psychiatry
Geriatrics & gerontology international

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Consultant (00107069)
The Journal of heart valve disease
Age & Ageing
Age and Ageing
American Sociological Review
BMJ Open
The western journal of emergency medicine
Journal of Geriatric Physical Therapy

Zeitschrift f√ºr Gerontologie und Geriatrie : Organ der Deutschen Gesellschaft f√ºr Gerontologie und
Gerontologist
JMIR human factors
Clinical Interventions in Aging
European neurology
Molecular phylogenetics and evolution
Acta Orthopaedica Scandinavica
Journal of Bone and Mineral Research
International Journal of Stroke
Cerebrovascular Diseases
BMC Geriatrics
Dementia & Geriatric Cognitive Disorders
Praxis
Experimental and clinical endocrinology & diabetes : official journal, German Society of Endocrinolog
International Journal of Gerontology
Experimental Gerontology
Saudi Medical Journal
Bone
International Journal of Nursing Studies
BMC Geriatrics
International Journal of Geriatric Psychiatry
Journal of rehabilitation medicine
International journal of colorectal disease
Research in Gerontological Nursing
Health Services Research
Journal of bone and mineral metabolism
European Journal of Trauma & Emergency Surgery
Clinical neuropharmacology
Journal of hospital medicine
Archives of psychiatric nursing
Journal of Geriatric Physical Therapy
Medical & biological engineering & computing
Geriatrics & Gerontology International
Journal of the American Geriatrics Society
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-t
Journal of Clinical Nursing
Japan Journal of Nursing Science

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Nursing & Health Sciences
Journal of Physical Therapy Science
The spine journal : official journal of the North American Spine Society
Journal of Clinical Nursing
Journal of neurology, neurosurgery, and psychiatry
Journal of the American Geriatrics Society
Bone marrow transplantation
Osteoporosis International
International journal of oral and maxillofacial surgery
Zeitschrift Fur Gerontologie Und Geriatrie
Journal of Aging & Physical Activity
Przeglad lekarski
Health & Social Work
Archives of gerontology and geriatrics
Surgical infections
Archives of Gerontology and Geriatrics
Journal of neurosurgery
Injury
Clinical journal of the American Society of Nephrology : CJASN
The Lancet. Public health
Perceptual and motor skills
Journal of evaluation in clinical practice
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Neurology
Physiotherapy (United Kingdom)
Age and ageing

Journal of the American Geriatrics Society

Circulation
Bandolier
Annals of the Academy of Medicine Singapore
Ontario health technology assessment series
Medical Care

Research in gerontological nursing
British Journal of Nursing
Bone
Medicine
Neurourology and Urodynamics
Scandinavian Journal of Caring Sciences
Journal of the American Geriatrics Society
Geriatrics & Gerontology International
Journal of neurology

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Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Brain and behavior
American journal of respiratory and critical care medicine

BMC Geriatrics
BMC infectious diseases
Geriatric Nursing
Archives of Physical Medicine & Rehabilitation
American journal of physical medicine & rehabilitation
BMC geriatrics
Journal of the American Geriatrics Society
Alzheimer disease and associated disorders
Journal of the American Geriatrics Society
Examining Factors Associated With Early, Post-Discharge, Acute Healthcare Utilization In Older Adults

Dementia and geriatric cognitive disorders
Journal of Gerontological Nursing
Alzheimer's and Dementia
American journal of physiology. Regulatory, integrative and comparative physiology
Osteoporosis international : a journal established as result of cooperation between the European Fou
Osteoporosis International
Internal medicine (Tokyo, Japan)
International journal of older people nursing
Journal of Clinical Nursing
American Journal of Geriatric Pharmacotherapy
Bone
Journal of the American Geriatrics Society
QJM : monthly journal of the Association of Physicians
Journal of the American Geriatrics Society
Maturitas

International Journal of Audiology
Journal of Musculoskeletal & Neuronal Interactions
American journal of orthopedics (Belle Mead, N.J.)
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Neurology
Neurology
Osteoporosis International
Reading and Writing: An Interdisciplinary Journal
Southern medical journal
The American journal of cardiology
Aging Clinical and Experimental Research
Journal of Aging & Physical Activity
Medicina clinica
Aging clinical and experimental research

For peer review only

Archivos Espanoles de Urologia
Brazilian Journal of Physical Therapy / Revista Brasileira de Fisioterapia
Physiotherapy (United Kingdom)
Studies in health technology and informatics
Zeitschrift f√ºr Gerontologie und Geriatrie
Zeitschrift fur Gerontologie und Geriatrie
Gerontology
Journal of the American Geriatrics Society
Physical Therapy
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Journal of the American Geriatrics Society
Age and ageing
Maturitas
Journal of the American Medical Directors Association
Gastrointestinal endoscopy
Journal of Clinical Gerontology & Geriatrics
Sleep
British Journal of Learning Disabilities
Calcified tissue international
Journal of Investigative Medicine
Studies in Health Technology & Informatics
Journal of Trauma
International Journal for Quality in Health Care
Age and ageing
Journal of the American Geriatrics Society
Journal of Geriatric Physical Therapy
American journal of physical medicine & rehabilitation
Journal of the American Geriatrics Society
Physical Therapy
Frontiers in public health
Journal of Geriatric Physical Therapy
American Journal of Geriatric Psychiatry
The European journal of general practice
Anesthesiology
Aging Clinical and Experimental Research
Drugs & Aging
Physiotherapy research international : the journal for researchers and clinicians in physical therapy
Physical Therapy
Aging Clinical & Experimental Research
Journal of General Internal Medicine
Journal of general internal medicine
Nutrition reviews

Clinical calcium
Disability and rehabilitation
Clinical Nurse Specialist: The Journal for Advanced Nursing Practice

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PloS one
Journal of Hypertension
American Journal of Hospice & Palliative Medicine
Ann. New York Acad. Sci.
Journal of thrombosis and thrombolysis
Journal of Physical Therapy Science
American Journal of Geriatric Pharmacotherapy
Pflege
Osteoporosis International
Neurology
European Journal of Medical Research
Journal of Advanced Nursing
Chronobiology international
Cost Effectiveness and Resource Allocation
Journal of the American Medical Directors Association
Nordic Journal of Nursing Research & Clinical Studies / V√•rd i Norden
Enfermeria Clinica
European Journal of Neurology
Journal of Aging Studies
International Psychogeriatrics
Seminars in nuclear medicine
Nano Energy
Scandinavian Journal of Public Health
Journal of the American Geriatrics Society
The American journal of managed care
Disability and rehabilitation
The British Journal of Psychiatry
American journal of surgery
Archives of Internal Medicine
Panminerva medica
Arthritis and Rheumatism
The Gerontologist
Alzheimer's and Dementia
American Journal of Gastroenterology
Injury
Experimental aging research
Journal of the American Geriatrics Society
Annales Medico-Psychologiques
Scandinavian Journal of Surgery
Nursing & Health Sciences
Journal of the American Geriatrics Society
Journal of occupational and environmental medicine
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
The Journal of frailty & aging
Journal of the American Geriatrics Society

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Journal of Nutrition, Health and Aging

Hypertension Research
International Journal of Health Planning and Management
European Geriatric Medicine
European Geriatric Medicine
Aging & mental health
Journal of Orthopaedic Science
Journal of Biomechanics
International Journal of Surgery Case Reports
Nephron. Clinical practice
Journal of multidisciplinary healthcare
Archives of gynecology and obstetrics
Annals of Pharmacotherapy
Drugs and Aging
European Geriatric Medicine
Comptes Rendus - Biologies
Annals of Epidemiology
The British journal of surgery
Fundamental and Clinical Pharmacology
Journals of Gerontology Series B-Psychological Sciences and Social Sciences
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Clinical rehabilitation
Disability & Rehabilitation
Age and Ageing
Journal of nephrology
Aging Clinical & Experimental Research
Medicine (Spain)
The journals of gerontology. Series A, Biological sciences and medical sciences
Sao Paulo Medical Journal
Revista Brasileira de Enfermagem
Cochlear implants international
Clinical Interventions in Aging
Journal for Healthcare Quality: Promoting Excellence in Healthcare
Die Rehabilitation
Journal of Aging & Physical Activity
Journal of pineal research
Journal of Ambient Intelligence and Smart Environments
Age and Ageing
Minerva endocrinologica
BMC geriatrics
Psychologie & NeuroPsychiatrie du Vieillissement
Journal of the American Geriatrics Society
Topics in Geriatric Rehabilitation
Texto & Contexto Enfermagem

For peer review only

Australasian Journal on Ageing
Brazilian journal of otorhinolaryngology
Brazilian Journal of Otorhinolaryngology
Sensors (Basel, Switzerland)
Revista Eletronica de Enfermagem
Health Economics
Gait & Posture
Therapeutic Advances in Drug Safety
Revista Brasileira de Enfermagem
Revista Brasileira de Enfermagem
The journal of ECT
Disability and Rehabilitation
Revue neurologique
Deutsche Zeitschrift Fur Sportmedizin
Osteoporosis International
American journal of epidemiology
European heart journal
The journal of nutrition, health & aging
Gerontechnology
Journal of the American Medical Directors Association
European Geriatric Medicine
Journal of the American Geriatrics Society
Behavioral and Brain Functions
Journal of neurotrauma
Diabetes care
Methods of Information in Medicine
Alzheimer's and Dementia
Archives of Gerontology and Geriatrics
Journal of Aging & Physical Activity
Diabetic Medicine
Medical Journal of Australia
2016 19th International Conference on Information Fusion
Journal of the American Geriatrics Society
Age and Ageing
BMJ open sport & exercise medicine
Bmc Geriatrics
Alzheimer's and Dementia
Gait & Posture
Journal of the American Podiatric Medical Association
Kidney international
Medical Journal of Australia
Archives of General Psychiatry
Nephrology, dialysis, transplantation : official publication of the European Dialysis and Transplant Ass
Injury Prevention (1353-8047)
Journal of geriatric physical therapy (2001)
Implementation science : IS

For peer review only

Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
European Geriatric Medicine
Wisconsin Medical Journal

Archives des maladies du coeur et des vaisseaux
Archives of physical medicine and rehabilitation
The New England journal of medicine
Arthritis and rheumatism
Archives of gerontology and geriatrics
Population Health Management
American heart journal
Southern Medical Journal
Journal of Vestibular Research: Equilibrium and Orientation
Age and Ageing
European Geriatric Medicine
International Archives of Otorhinolaryngology
European Geriatric Medicine
Experimental Gerontology
Emergency Medicine Journal
British Journal of Occupational Therapy
Australian Occupational Therapy Journal
Occupational Therapy Journal of Research
Disability & Rehabilitation
Disability & Rehabilitation
Journal of Urban Health
Journals of Gerontology Series B-Psychological Sciences and Social Sciences
Drugs & Aging
Clinical journal of sport medicine : official journal of the Canadian Academy of Sport Medicine
Hua xi kou qiang yi xue za zhi = Huaxi kouqiang yixue zazhi = West China journal of stomatology
Neurology India
Age and ageing
Archives of gerontology and geriatrics
Pm&R
Journal of the American Geriatrics Society
Geriatrics & gerontology international
Journal of Craniofacial Surgery
The journal of nutrition, health & aging
Gait & posture
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of General Internal Medicine
The American journal of physiology
Geriatric Nursing
Journal of Bone and Mineral Research
International Journal of Geriatric Psychiatry
Journal of Advanced Nursing

For peer review only

Health Technology Assessment
Health Technology Assessment
Intensive care medicine
International journal of nursing studies
BMC Geriatrics
Physiotherapy (United Kingdom)
PloS one
BMC Geriatrics
BMJ Open
Https://clinicaltrials.gov/show/nct01218373
Journal of Epidemiology and Community Health
Annals of Nutrition & Metabolism
European Journal of Ageing
Journal of the american geriatrics society.
BMC geriatrics
The Cochrane database of systematic reviews
JBI library of systematic reviews
British journal of clinical pharmacology
Complementary Therapies in Medicine
BMJ (Online)
Age and Ageing
Journal of Physiotherapy (Elsevier)
Evidence Based Nursing
BMC Geriatrics
Climacteric
Cochrane Database of Systematic Reviews
Danish medical bulletin
BMC Health Services Research
The Lancet Neurology
Health Technology Assessment
Australian journal of physiotherapy
Age and ageing
F1000Research
Dental Traumatology
Aging Clinical and Experimental Research
Physiotherapy (United Kingdom)
Health Technology Assessment
JBI library of systematic reviews
Journal of Science and Medicine in Sport
International rehabilitation medicine
Age and ageing
Bmc Medical Research Methodology
Trials
Trials
Gait & Posture
Disability and rehabilitation

For peer review only

Cochrane Database of Systematic Reviews
Australian and new zealand journal of public health
British Medical Journal
Bmj-British Medical Journal
Value in Health
Age and Ageing
Age and ageing
Trials
BMC Geriatrics
Thorax
Thorax
Journal of Epidemiology & Community Health
Implementation science
The Cochrane database of systematic reviews
Cochrane Database of Systematic Reviews
Health & Social Care in the Community
The journal of nutrition, health & aging
Injury Prevention (1353-8047)
EMA - Emergency Medicine Australasia
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
Cochrane Database of Systematic Reviews
Journal of Science and Medicine in Sport
BMC health services research
Australasian journal on ageing
Age and ageing
BMC geriatrics
BMJ (Online)
Nederlands Tijdschrift voor Geneeskunde
European Geriatric Medicine
BMC geriatrics
Evidence Based Nursing
Evidence Based Nursing
Journal of Physiotherapy (Elsevier)
Health Technology Assessment
BMC geriatrics
Drugs & aging
Heart (British Cardiac Society)
Western journal of nursing research

Diabetes, obesity & metabolism
Nursing & Health Sciences
Bone
Journal of Bone and Mineral Research
JAMA: Journal of the American Medical Association
Archives of Disease in Childhood-Fetal and Neonatal Edition
Gerontology

For peer review only

European Geriatric Medicine
BMC geriatrics
Gerontology
JAMA surgery
Journal of Aging & Physical Activity
Topics in Geriatric Rehabilitation
Journal of the American Geriatrics Society

Praxis
International Journal of Integrated Care (IJIC)
Cochrane Database of Systematic Reviews
Journal of Science and Medicine in Sport
Proceedings of Singapore Healthcare
Age and ageing
Physiotherapy (United Kingdom)
BMC Geriatrics
BMC geriatrics
Age & Ageing
Osteoporosis international
The British journal of general practice : the journal of the Royal College of General Practitioners
Archives of Gerontology and Geriatrics
Public Health
Chiropractic & Manual Therapies
Injury Prevention
European Geriatric Medicine
The Cochrane database of systematic reviews
Scandinavian Journal of Medicine and Science in Sports
Age and ageing
Archives of gerontology and geriatrics
Journal of clinical nursing
Movement disorders. Conference: 22nd international congress of parkinson's disease and movement
Clinical Biomechanics
2016 International Conference on Medical Engineering, Health Informatics and Technology
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of Applied Gerontology
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of Physical Therapy Science
International Journal of Circumpolar Health
Scandinavian Journal of Primary Health Care
Behavioral Sleep Medicine
Western Journal of Emergency Medicine: Integrating Emergency Care with Population Health
Journal of Nursing
Journal of Nursing Research (Lippincott Williams & Wilkins)
The gerontology nurse's guide to the community-based health network.
American Journal of Geriatric Psychiatry
Clinical orthopaedics and related research

For peer review only

Topics in Geriatric Rehabilitation
Evidence Based Nursing
Physiotherapy (United Kingdom)
Journal of Bone and Mineral Research
Journal of Bone and Mineral Research
PloS one
Trauma surgery & acute care open
Drugs & Aging
Australasian Journal on Ageing
Journal of Orthopaedic & Sports Physical Therapy
American Journal of Epidemiology

Acta Orthopaedica
American journal of epidemiology
The journal of nutrition, health & aging
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Journal of Affective Disorders
Social Science and Medicine
Aging Clinical and Experimental Research
Preventive Medicine Reports
Osteoporosis international : a journal established as result of cooperation between the European Fou
Zeitschrift Fur Gerontologie Und Geriatrie
International journal of clinical practice
Aging clinical and experimental research
The Annals of pharmacotherapy
Osteoporosis International
Archives of Physical Medicine and Rehabilitation
Revista Clinica Espanola
Revista Clinica Espanola
Journal of Nutrition, Health and Aging
Hong Kong Journal of Occupational Therapy

British Journal of Occupational Therapy
Irish journal of medical science
Journal of Bone and Mineral Research
Journal of Bone and Mineral Research
Osteoporosis International
Journal of the American Geriatrics Society
BMC health services research
Archives of Osteoporosis
Journal of Bone and Mineral Research
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
BMC public health
Osteoporosis international : a journal established as result of cooperation between the European Fou
The journals of gerontology. Series A, Biological sciences and medical sciences
Bone

For peer review only

Hong Kong medical journal = Xianggang yi xue za zhi
Climacteric : the journal of the International Menopause Society
BMJ (Clinical research ed.)
Aging - Clinical and Experimental Research
Journal of the American Geriatrics Society
Calcified tissue international
Archives of internal medicine
Investigative Ophthalmology and Visual Science
2007 Virtual Rehabilitation
Australasian Journal on Ageing
JAMA internal medicine
Recenti progressi in medicina
Journal of the British Menopause Society
Gerontology
Geriatric nursing (New York, N.Y.)
Orthopedics
Clinics
Brazilian Journal of Physical Therapy
Journal of the American Geriatrics Society
Home Care Provider
Archives of internal medicine
Revista Medica de Chile
Dental traumatology : official publication of International Association for Dental Traumatology
Implementation science : IS
Diabetes research and clinical practice
JBMR Plus
Pesquisa Brasileira Em Odontopediatria E Clinica Integrada
QJM
British journal of clinical pharmacology
Journal of Clinical Pharmacology
Journal of clinical densitometry : the official journal of the International Society for Clinical Densitome
Journal of the American Geriatrics Society
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Archives of osteoporosis
Arthritis and Rheumatology
Drugs & Aging
Gender Medicine
Aging clinical and experimental research
Journal of Bone and Mineral Research
Journal of Bone and Mineral Research
British Journal of Nursing
Journal of Clinical Gerontology & Geriatrics
Journal of gerontology & geriatric research
Gait & Posture
Archives of Gerontology and Geriatrics
Bone

For peer review only

Journal of nutrition for the elderly
Journal of Hand Therapy
Osteoporosis International
American journal of kidney diseases : the official journal of the National Kidney Foundation
Injury Epidemiology
Journal of Geriatric Physical Therapy
Zhonghua liu xing bing xue za zhi = Zhonghua liuxingbingxue zazhi
Age and Ageing

Health Services Research
The journal of nutrition, health & aging
Irish journal of medical science
Emergency medicine journal : EMJ
Medical care
Neurology
Osteoporosis international : a journal established as result of cooperation between the European Fou
Jama
Sleep
Journal of the American Geriatrics Society
Neuropsychology
Annals of Internal Medicine
Journal of renal nutrition : the official journal of the Council on Renal Nutrition of the National Kidney
Archives of osteoporosis
Journal of Bone and Mineral Research
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of gerontological social work
Physical Therapy
American Journal of Clinical Nutrition

International Journal of Epidemiology
Journal of Neurotrauma
Swiss Medical Weekly
Clinical and Experimental Ophthalmology
Salmand-Iranian Journal of Ageing
European journal of epidemiology
Diabetology and Metabolic Syndrome
Children & Society
BMC oral health
Topics in Geriatric Rehabilitation
Ciencia & Saude Coletiva
Revista Latino-Americana de Enfermagem (RLAE)
The neuroradiology journal
European Geriatric Medicine
Disability and Rehabilitation
Annals of Long Term Care
Hypertension (Dallas, Tex. : 1979)

For peer review only

Experimental Brain Research
National vital statistics reports : from the Centers for Disease Control and Prevention, National Cente
Australasian Journal on Ageing
Journal of Applied Gerontology
American Journal of Occupational Therapy
American journal of epidemiology
Hypertension Research
Progress in clinical and biological research
European journal of clinical chemistry and clinical biochemistry : journal of the Forum of European Cli
American family physician
British Journal of Preventive & Social Medicine
Journal of Electromyography and Kinesiology
Age and Ageing

Journal of Applied Research in Intellectual Disabilities

International Journal of Audiology
International journal of audiology
Journal of the American Academy of Audiology
Journal of geriatric physical therapy (2001)
Respiration; international review of thoracic diseases
European review for medical and pharmacological sciences
Journal of the American Geriatrics Society
Gait & Posture
Gait & Posture
Journal of American college health : J of ACH
Journal of the American College of Cardiology
Medical Care
Journal of geriatric physical therapy (2001)
Ethnicity & disease
The British journal of dermatology
QJM : monthly journal of the Association of Physicians
The Journal of infectious diseases
Quality in Ageing & Older Adults
European archives of paediatric dentistry : official journal of the European Academy of Paediatric Der
European heart journal
Journal of the American Geriatrics Society
Research in Nursing & Health
Salud publica de Mexico
Gerontology
Physical and Occupational Therapy in Geriatrics
Physical & Occupational Therapy in Geriatrics
Physical & Occupational Therapy in Geriatrics
Clinical journal of the American Society of Nephrology : CJASN
Journal of the neurological sciences
Parkinson's disease

For peer review only

Kidney international
Archives of physical medicine and rehabilitation
Archives of Physical Medicine & Rehabilitation
Epidemiologia e servicos de saude : revista do Sistema Unico de Saude do Brasil
Ciencia & saude coletiva
The American journal of medicine
American Journal of Geriatric Psychiatry
Movement disorders
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
BMC Health Services Research
Age & Ageing
International Journal of Nursing Studies
European journal of physiotherapy
BMC geriatrics
British medical journal (Clinical research ed.)
Neurodegenerative Diseases
Health Technology Assessment
The Cochrane database of systematic reviews
The Cochrane database of systematic reviews
Bmc Geriatrics
BMC public health
Bmc Geriatrics
Gerontology
Trials
Evidence Based Mental Health
British Journal of Ophthalmology
Age and Ageing
Huisarts en Wetenschap
JBI library of systematic reviews
Journal of Epidemiology & Community Health
The Cochrane database of systematic reviews
BMC Geriatrics
BMC geriatrics
Clinical Rehabilitation
Age and Ageing
BMJ Open
Health Technology Assessment
Cochrane Database of Systematic Reviews
BMC health services research
Trials
Cochrane Database of Systematic Reviews
BMJ Open
Aging & Mental Health
International Musculoskeletal Medicine
Journal of the Pakistan Medical Association
Age and ageing

For peer review only

The Journal of steroid biochemistry and molecular biology
Zeitschrift fur Gerontologie und Geriatrie
Cochrane Database of Systematic Reviews
British medical journal
British Medical Journal
International Journal of Nursing Studies
International Journal of Nursing Studies
International Journal of Nursing Studies
BMJ open
BMJ Open
European Journal of Hospital Pharmacy
BMJ: British Medical Journal
Trials
BMJ Open
Health & Social Care in the Community
The Cochrane database of systematic reviews
Journal of Science and Medicine in Sport
BMJ Open
BMC public health
Quality of life research : an international journal of quality of life aspects of treatment, care and rehal
Journal of Clinical Nursing
PloS one
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Archives of Internal Medicine
Journal of the American Geriatrics Society
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
BMC Public Health
Eastern Mediterranean Health Journal
Clinical Science
European Geriatric Medicine
Current osteoporosis reports
Jama
Journal of the American Medical Directors Association
Minerva medica
Osteoporosis International
Journal of Community Health
Revista Brasileira de Enfermagem
Osteoporosis International
Journal of Bone and Mineral Research
Lancet (London, England)
Revue d'Epidemiologie et de Sante Publique
Emergency medicine journal : EMJ
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of the American Geriatrics Society
Revista de Saude Publica
Journal of Bone and Mineral Research

For peer review only

Archives of Gerontology & Geriatrics
Bone
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Journal of Bone and Mineral Research
Journal of intellectual disability research : JIDR
Journal of Bone and Mineral Research
Archives of internal medicine
American Journal of Physical Medicine & Rehabilitation
Age & Ageing
Clinical endocrinology
The American journal of medicine
Journal of the American Geriatrics Society
Geriatrics and Gerontology International
Journal of Geriatric Physical Therapy
Orthopedics
Bone & Joint Journal
Age and ageing
Journal of neurology

Transplant international : official journal of the European Society for Organ Transplantation
Journal of the American Geriatrics Society

Physiotherapy (United Kingdom)
American Journal of Tropical Medicine and Hygiene
Archives of Physical Medicine & Rehabilitation
Disability and Rehabilitation
Journal of Neurology, Neurosurgery and Psychiatry
International Journal of Rehabilitation Research

Stroke (00392499)
Archives of Physical Medicine and Rehabilitation

Stroke
Aging Clinical & Experimental Research
PLoS ONE
Physical Therapy
South African Journal of Physiotherapy
Biomedical instrumentation & technology
Physiotherapy theory and practice
New England Journal of Medicine
Diabetes Care
Annals of internal medicine
Physical & Occupational Therapy in Geriatrics
Geriatrics & gerontology international
[Nippon k≈çsh≈´ eisei zasshi] Japanese journal of public health
Stroke

For peer review only

Circulation
Stroke
Archives of internal medicine
Age and ageing
Stroke
2017 3rd International Conference on Pattern Recognition and Image Analysis
Disability & Rehabilitation
Journal of stroke and cerebrovascular diseases : the official journal of National Stroke Association
Stroke
Journal of stroke and cerebrovascular diseases : the official journal of National Stroke Association
Kardiologia polska
Revista de Investigacion Clinica
The American journal of medicine
Neurourology and Urodynamics
CHEST
Annales de Readaptation et de Medecine Physique
American journal of nephrology
British Medical Journal
Topics in stroke rehabilitation
BMC neurology
Age & Ageing
American journal of physical medicine & rehabilitation
Medicine & Science in Sports & Exercise
European Heart Journal
Irish Journal of Medical Science
Australian & New Zealand Journal of Public Health
International Journal of Stroke
International Journal of Rehabilitation Research

ASAIO journal (American Society for Artificial Internal Organs : 1992)
Revista da Rede de Enfermagem do Nordeste
Journal of the American Geriatrics Society
International Journal of Stroke
Circulation
Archives of physical medicine and rehabilitation
Disability & Rehabilitation
Gait & Posture
Https://clinicaltrials.gov/show/nct02314585
Systematic Reviews
Archives of Physical Medicine and Rehabilitation
Pm&R
Archives of physical medicine and rehabilitation
Journal of interferon research
American journal of physical medicine & rehabilitation
Physiotherapy
Archives of Physical Medicine & Rehabilitation

For peer review only

Neurorehabilitation and Neural Repair
Bmc Geriatrics
PM & R: Journal of Injury, Function & Rehabilitation
NeuroRehabilitation
Archives of Physical Medicine & Rehabilitation
JBI Database of Systematic Reviews and Implementation Reports
Journal of Neurologic Physical Therapy
Journal of the neurological sciences
Multiple Sclerosis
American Journal of Occupational Therapy
Nefrologia : publicacion oficial de la Sociedad Espanola Nefrologia
Collegium Antropologicum
The European journal of medicine
Archives of Gerontology and Geriatrics
Magnesium research
Journal of the American Geriatrics Society
Geriatrics & gerontology international
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
BioMed Research International
PM and R
International Journal of Environmental Research and Public Health

Journal of the American Geriatrics Society
Language and speech
Physical Therapy

Accident; analysis and prevention
Disability & Rehabilitation
Aging & Mental Health
American Journal of Occupational Therapy
Physical & Occupational Therapy in Geriatrics
International journal of rehabilitation research. Internationale Zeitschrift f√ºr Rehabilitationsforschur

Experimental physiology
Archives of emergency medicine
Cases Journal
Journal of the American Geriatrics Society
Otology & Neurotology

Physical Therapy
Journal of the Medical Association of Thailand = Chotmaihet thangphaet

Age and ageing
British Journal of General Practice
British Journal of General Practice
Trials

For peer review only

Health Technology Assessment
The Cochrane database of systematic reviews
Journal of Clinical Nursing
Geriatrics & gerontology international
Alzheimer's and Dementia
Alzheimer's and Dementia
Geriatrics & Gerontology International
Osteoporosis International
Journal of Orthopaedics, Trauma and Rehabilitation
Journal of General Internal Medicine
Medicine
Emergency medicine Australasia : EMA
Cochrane Database of Systematic Reviews
Cochrane Database of Systematic Reviews
Age & Ageing
Injury
Journal of Rehabilitation Medicine (Stiftelsen Rehabiliteringsinformation)
Journal of Rehabilitation Medicine
Pflege
Journal of the American Geriatrics Society
Neurourology and Urodynamics
Medical Education
Singapore Medical Journal
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Journal of Paediatrics and Child Health
Journal of the American Geriatrics Society
Investigative ophthalmology & visual science
Endocrine Reviews
Endocrine Reviews
Archives of osteoporosis
Calcified Tissue International
Oral health & preventive dentistry
Eurorehab
European Geriatric Medicine
Journal of Epidemiology & Community Health
Physiotherapy Theory & Practice
Journal of Chiropractic Medicine
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
European Geriatric Medicine
Annals of emergency medicine
Nephrology Dialysis Transplantation
Gait and Posture
Diabete & metabolisme
Journal of the American Pharmacists Association
Calcified Tissue International
Atencion Primaria

For peer review only

International Design Engineering Technical Conferences and Computers and Information in Engineeri
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
BMJ (Online)
Annals of the Academy of Medicine, Singapore
Archives of Physical Medicine & Rehabilitation
BMC geriatrics
Evidence Based Nursing
International Journal of Geriatric Psychiatry
Journal of Orthopaedic Research
Zeitschrift fur Gerontologie und Geriatrie
Journal of the american college of cardiology
The American journal of managed care
Osteoporosis International
Geriatric Nursing
Archives of Gerontology & Geriatrics
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of the American Geriatrics Society
Disability & Rehabilitation
Archives of gerontology and geriatrics
VA Evidence-based Synthesis Program Evidence Briefs
American journal of respiratory and critical care medicine
Healthy Aging and Longevity
Australasian Journal on Ageing
Journal of Advanced Nursing
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
International Journal of Adolescent Medicine and Health
Medico-Legal Update
Aging & Mental Health
Acta Neurologica Scandinavica
Annals of Rehabilitation Medicine-Arm
Medicine
Medicine (united states)
NPJ Parkinson's disease
Journal of Korean Academy of Nursing
Journal of aging and health
International journal of environmental research and public health
Journal of the American Medical Directors Association
Osteoporosis international
Journal of Korean medical science
Aging Clinical and Experimental Research
International Journal of Consumer Studies
Journal of Physical Therapy Science
Physical & Occupational Therapy in Geriatrics
PloS one
American journal on intellectual and developmental disabilities
Journal of diabetes and its complications

For peer review only

Geriatrics & Gerontology International
Journal of Gerontological Nursing
Australian Journal of Physiotherapy
Therapeutic Recreation Journal
Journal of Transcultural Nursing
Age (Dordrecht, Netherlands)
Age
Osteoporosis International
Journal of gerontological nursing
Journal of Clinical Nursing
Journal of Physical Therapy Science
International Journal of Environmental Research and Public Health
American Journal of Epidemiology
Parkinsonism & related disorders
Medical science monitor : international medical journal of experimental and clinical research
2016 13th International Conference on Electrical Engineering/Electronics, Computer, Telecommunica
Journal of the Chinese Medical Association : JCMA
Rheumatology (Oxford, England)
Journal of aging and physical activity
2015 17th International Conference on E-Health Networking, Application & Services
Journals of Gerontology Series B: Psychological Sciences & Social Sciences
Journal of the American College of Nutrition
Issues on Aging
Diabetic medicine : a journal of the British Diabetic Association
Sleep
Journal of Nursing UFPE / Revista de Enfermagem UFPE
British Journal of Nursing
Journal of Aging and Physical Activity
BMC geriatrics
Age and Ageing
Gerontology
Age and ageing
Salmand-Iranian Journal of Ageing
Pilot and Feasibility Studies
Journal of the American Geriatrics Society
Archives of Gerontology & Geriatrics
The Cochrane database of systematic reviews
Disability and Rehabilitation
Alzheimer's and Dementia
Age & Ageing
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Age & Ageing
Journal of neurotrauma
Bmc Palliative Care
Journal of advanced nursing
BMC medical informatics and decision making

For peer review only

Chest
Clinical interventions in aging
Journal of neurologic physical therapy
Journal of the American Academy of Audiology
Journal of medical screening
British journal of surgery
Clinical Rehabilitation
Journal of the American Geriatrics Society
International Journal of Nursing Education
British Journal of Ophthalmology
Journal of Psychiatric & Mental Health Nursing
BMC public health
Journal of the american geriatrics society
Archives of cardiovascular diseases
Cerebrovascular Diseases
Deutsches Arzteblatt international
Archives of Physical Medicine & Rehabilitation
Archives of physical medicine and rehabilitation
Journal of the american medical directors association
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society

Journal of aging and physical activity
Preventing chronic disease
Bmc Geriatrics
Pakistan Journal of Statistics
Journals of Gerontology - Series B Psychological Sciences and Social Sciences
2015 International Conference on Control, Automation and Robotics Iccar 2015
Worldviews on evidence-based nursing

Calcified tissue international
Journal of clinical epidemiology
Journal of Bodywork and Movement Therapies
Aging Clinical and Experimental Research
European review of aging and physical activity : official journal of the European Group for Research ir
International Journal of Clinical Practice
Journal of Clinical and Diagnostic Research
Journal of applied gerontology
Annals of physical and rehabilitation medicine.
Journal of aging and health
Journal of the american geriatrics society
Archives of Physical Medicine and Rehabilitation
Drugs & Aging
Journal of Clinical Psychopharmacology
Climacteric : the journal of the International Menopause Society
Physical therapy

For peer review only

Clinical Rehabilitation
Geriatrics & Gerontology International
Journal of Bone and Mineral Metabolism
Physical Therapy
Clinical rehabilitation
Archives of Physical Medicine and Rehabilitation
Journal of the American Geriatrics Society
Journal of geriatric physical therapy (2001)
Journal of Clinical and Diagnostic Research
Journal of the american geriatrics society
Medicine
Archives of Physical Medicine and Rehabilitation
Clinical Rehabilitation
PloS one
Journal of clinical pharmacy and therapeutics
Physical & Occupational Therapy in Geriatrics
The journals of gerontology. Series A, Biological sciences and medical sciences
Aging Clinical & Experimental Research
International Journal of Geriatric Psychiatry
Aging & Mental Health
Archives of internal medicine
Aging Friendly Technology for Health and Independence
Journal of the American Geriatrics Society
Clinical rehabilitation
Zeitschrift fur Gerontologie und Geriatrie
Journal of geriatric physical therapy (2001)
BMC psychology
Preventing Chronic Disease
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Gait & posture
Journal of the American Medical Directors Association
Journal of hypertension. Supplement : official journal of the International Society of Hypertension
Brain and behavior
Journal of the American Geriatrics Society
American Journal of Respiratory and Critical Care Medicine
Stroke
American journal of public health
Stroke
PLoS ONE
Disability and rehabilitation
Clinical Rehabilitation
Pediatrics
Journal of Applied Gerontology
Journal of the american medical directors association
Japanese Journal of Allergology
Stroke

For peer review only

Physiotherapy (United Kingdom)
American journal of health behavior
Scandinavian Journal of Medicine and Science in Sports
Journal of Clinical Gerontology and Geriatrics
American journal of physical medicine & rehabilitation
Age & Ageing
Gerontology
Quality in Ageing
Journal of evaluation in clinical practice
Epidemiologia e prevenzione
Archives of gerontology and geriatrics
Geriatric Nursing
Journal of Science and Medicine in Sport
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Stroke
Preventive medicine
Movement disorders
American journal of geriatric psychiatry.
Journal of the American Geriatrics Society
Journal of the International Society of Sports Nutrition
Health & Social Care in the Community
The Gerontologist
Trials
Gait & Posture
Journal of Safety Research
Journal of aging and physical activity
Physiotherapy theory and practice
Archives of Physical Medicine and Rehabilitation
Journal of the American Medical Directors Association
Journal of the American Geriatrics Society
Journal of the American Medical Directors Association
Experimental Gerontology
The British journal of general practice : the journal of the Royal College of General Practitioners
Aging Clinical & Experimental Research
Deutsches arzteblatt international
Deutsches Arzteblatt International
Age and ageing
Rawal Medical Journal
Journal of the American Geriatrics Society
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
International Nursing Review
Aging Clinical & Experimental Research
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Physiotherapy (United Kingdom)
Journal of Psychosomatic Research
Journal of psychosomatic research

For peer review only

Frontiers in Aging Neuroscience

Disability and Rehabilitation
Disability & Rehabilitation
Journal of Bone and Mineral Research
Atencion Primaria
Journal of Rural Health
Journal of Bone and Mineral Research
The Journal of trauma
The Journal of trauma
Journal of Bone and Mineral Research
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
International journal of geriatric psychiatry
Journal of Alzheimers Disease
Trials
Public Health
Infection Control and Hospital Epidemiology
BMC Musculoskeletal Disorders
Infection Control & Hospital Epidemiology
Journal of the American Geriatrics Society
Psychiatry research
Psychiatry research
Journal of the American Medical Directors Association
Movement Disorders
Journal of the American Geriatrics Society
JAMA - Journal of the American Medical Association
JAMA
Alzheimer Disease and Associated Disorders
Alzheimer Disease and Associated Disorders
Revista espanola de geriatria y gerontologia
The Journal of sports medicine and physical fitness
Blood pressure monitoring
Diabetes care
Calcified tissue international
Journal of Manual and Manipulative Therapy
Neurorehabilitation and Neural Repair
Archives of Internal Medicine
Archives of Physical Medicine & Rehabilitation
Physiotherapy (United Kingdom)
Journal of the American Geriatrics Society
Osteoporosis International
Journal of foot and ankle research
Journal of Krishna Institute of Medical Sciences University
Journal of the American Geriatrics Society
Gerontology

For peer review only

Gaceta Sanitaria
American journal of physical medicine & rehabilitation
Annals of oncology : official journal of the European Society for Medical Oncology
American journal of obstetrics and gynecology. Conference: 42nd annual meeting of the society of gy
Female Pelvic Medicine and Reconstructive Surgery
JMIR research protocols
Journal of aging research
International Journal of Geriatric Psychiatry
Journal of the American Geriatrics Society
Neurology
Prevention science : the official journal of the Society for Prevention Research
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Scientific Reports
Atencion Primaria
Statistics in medicine
Hypertension (Dallas, Tex. : 1979)
Clinical and experimental pharmacology & physiology
Jama-Journal of the American Medical Association
Archives of Internal Medicine
Archives of Internal Medicine
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Geriatric Nursing
Osteoporosis International
Https://clinicaltrials.gov/show/nct02631330
Https://clinicaltrials.gov/show/nct02425267
Https://clinicaltrials.gov/show/nct02615899
Https://clinicaltrials.gov/show/nct00013507
Https://clinicaltrials.gov/show/nct03118414
Https://clinicaltrials.gov/show/nct03614182
Https://clinicaltrials.gov/show/nct02419105
Https://clinicaltrials.gov/show/nct01407185
Https://clinicaltrials.gov/show/nct03320668
Https://clinicaltrials.gov/show/nct03649698
Https://clinicaltrials.gov/show/nct03585972
Https://clinicaltrials.gov/show/nct02941497
Https://clinicaltrials.gov/show/nct01697553
International psychogeriatrics
Clinical Rehabilitation
BMC musculoskeletal disorders
Geriatrics & Gerontology International
Biological research for nursing
BMJ (Clinical research ed.)
Le Travail Humain: A Bilingual and Multi-Disciplinary Journal in Human Factors
Travail Humain
Journal of the American Geriatrics Society
Emergency Medicine Journal

For peer review only

Hypertension (dallas, tex. : 1979)
Of wisdom: The second and third books, 2nd ed.
Disability & Rehabilitation
PloS one
Activities, Adaptation & Aging
Quality of Life Research
Journal of the Medical Association of Thailand
Asia Pacific Journal of Speech, Language & Hearing
American journal of epidemiology
Archives of physical medicine and rehabilitation
Journal of Nursing Scholarship
Journal of Clinical Nursing
Journal of Community Health Nursing
Journal of neuroimmunology
Geriatrics & Gerontology International
PloS one
Frontiers in Aging Neuroscience
European journal of endocrinology
American Journal of Geriatric Psychiatry
Transplantation
Plos One
Annals of the Academy of Medicine, Singapore
American journal of epidemiology
Geriatrics & Gerontology International
Gait and Posture
Physiotherapy (united kingdom).
Archives of Physical Medicine & Rehabilitation
Journal of Bone and Mineral Research
Osteoporosis International
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Journal of Bone and Mineral Research
Archives of neurology
Systematic reviews
Https://clinicaltrials.gov/show/nct02652988
Https://clinicaltrials.gov/show/nct02618746
Https://clinicaltrials.gov/show/nct00330720
Https://clinicaltrials.gov/show/nct02296957
Https://clinicaltrials.gov/show/nct02374463
Https://clinicaltrials.gov/show/nct00000436
Https://clinicaltrials.gov/show/nct02292511
BMC geriatrics
Journal of NeuroEngineering and Rehabilitation
Activities, Adaptation & Aging
Journal of Nursing Scholarship
Archives of Cardiovascular Diseases Supplements

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Journal of Maternal-Fetal and Neonatal Medicine
Journal of Midwifery & Women's Health
International Journal of Older People Nursing
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Injury Prevention (1353-8047)
Injury Prevention (1353-8047)
The Journal of nursing education
Journal of Clinical Nursing
Journal of gerontological nursing
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Sports Medicine
Nutrition & Dietetics
Plos One

Journal of the American Geriatrics Society
Professional Case Management
Archives of physical medicine and rehabilitation
Journal of the american geriatrics society
Journal of the American Geriatrics Society
Journal of Nutrition, Health and Aging
Journal of multidisciplinary healthcare
Nursing Research
European Geriatric Medicine
Research in Gerontological Nursing
Personal and Ubiquitous Computing
Educational Gerontology
Educational Gerontology
Journal of Nursing UFPE / Revista de Enfermagem UFPE
Journal of the American Medical Directors Association
Behavioral and Brain Functions
Geriatrics & gerontology international
Journal of Gerontological Nursing
Journal of the American Geriatrics Society
Archives of gerontology and geriatrics
Drugs and Aging
Journal of Advanced Nursing
Psychiatria Danubina
BMC Geriatrics
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Drugs & Aging
Wiener Klinische Wochenschrift
Statistics in Medicine

For peer review only

The Journal of clinical endocrinology and metabolism
JAMA Internal Medicine
JAMA: Journal of the American Medical Association
Topics in Geriatric Rehabilitation
Topics in Geriatric Rehabilitation
Journal of Clinical Epidemiology
Journal of Life Care Planning
Journal of Telemedicine and Telecare
Primary Health Care Research and Development
Archives of Gerontology and Geriatrics
Injury
The european respiratory journal
Addiction (Abingdon, England)
Neurorehabilitation and neural repair
Journal of neuroengineering and rehabilitation
Clinical Rehabilitation
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Gerontology
American Journal of Health Promotion
Jmir Mhealth and Uhealth
Annals of behavioral medicine : a publication of the Society of Behavioral Medicine
Anesthesiology
BMJ Evidence-Based Medicine
Europace : European pacing, arrhythmias, and cardiac electrophysiology : journal of the working grou
American Journal of Respiratory and Critical Care Medicine

Spine Journal
PLoS ONE
Langenbeck's archives of surgery
Heart and vessels
Atencion primaria / Sociedad Espa√±ola de Medicina de Familia y Comunitaria
Archives of physical medicine and rehabilitation
Archives of physical medicine and rehabilitation
PM & R: Journal of Injury, Function & Rehabilitation
Osteoarthritis and Cartilage
Preventive Medicine Reports
Journal of neurologic physical therapy
Journal of Aging and Physical Activity
JBI database of systematic reviews and implementation reports
The Cochrane database of systematic reviews
BMC geriatrics
Internal Medicine Journal
Preventive Medicine: An International Journal Devoted to Practice and Theory
Journal of Bone and Mineral Research
Drugs and Aging

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Preventing Chronic Disease
Journal of geriatric physical therapy (2001)
Pharmacoeconomics
Archives of internal medicine
BMC musculoskeletal disorders
Der Anaesthesist
The Journal of thoracic and cardiovascular surgery
Revue de chirurgie orthopedique et reparatrice de l'appareil moteur
Injury
BMC geriatrics
The Cochrane database of systematic reviews
Cochrane Database of Systematic Reviews
Progres en urologie : journal de l'Association francaise d'urologie et de la Societe francaise d'urologie
Journal of the American Geriatrics Society
The American journal of medicine
Alcoholism: Clinical and Experimental Research
Bone
PloS one
European Geriatric Medicine
Neurocritical care
Diabetologia
British heart journal
Biomed Research International
Journal of the Royal Society of Medicine
Journal of the American Podiatric Medical Association
Annals of thoracic and cardiovascular surgery : official journal of the Association of Thoracic and Card
Clinical Nursing Research
BMC Geriatrics
American journal of surgery
Journal of the American College of Surgeons
The Journal of bone and joint surgery. British volume
The Cochrane database of systematic reviews
The Journal of rheumatology
Zeitschrift fur Gerontologie und Geriatrie
Revista de calidad asistencial : organo de la Sociedad Espanola de Calidad Asistencial
Journal of the American Medical Directors Association
Bmc Geriatrics
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Care Management Journals
International Journal of Integrated Care (IJIC)
Journal of Clinical Endocrinology and Metabolism
International journal of sports medicine
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
Aging Clinical and Experimental Research
Journal of the American Geriatrics Society

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Journal of the american geriatrics society
Rheumatology international
International Psychogeriatrics
Physiotherapy (United Kingdom)
Journal of the American Geriatrics Society
Plos One
Osteoporosis International
Journal of the American Society of Nephrology : JASN
JAMA
Activities, Adaptation & Aging
African journal of medicine and medical sciences
Jama
Https://clinicaltrials.gov/show/nct02305433
Https://clinicaltrials.gov/show/nct00130624
Https://clinicaltrials.gov/show/nct02645006
Https://clinicaltrials.gov/show/nct00599807
Injury
AJR. American journal of roentgenology
British journal of anaesthesia
Journal of the American College of Cardiology
Internal Medicine Journal
Journal of craniovertebral junction & spine
JAMA surgery
Journal of the American Geriatrics Society
Archives of Gerontology & Geriatrics
The Journal of arthroplasty
Archives of neurology
Osteoporosis International
Recenti Progressi in Medicina
Southern medical journal
The Journal of rheumatology
International Urogynecology Journal and Pelvic Floor Dysfunction
Psychology, health & medicine
The Gerontologist
Canadian journal of anaesthesia = Journal canadien d'anesthesie
Journal of Association of Physicians of India
Injury Prevention (1353-8047)
Knee surgery, sports traumatology, arthroscopy : official journal of the ESSKA
Journal of postgraduate medicine
Bone
International Journal of Surgery Case Reports
Clinical orthopaedics and related research
Annals of vascular surgery
Anesthesiology
Age & Ageing
Journal of Evaluation in Clinical Practice

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British journal of anaesthesia
The European journal of health economics : HEPAC : health economics in prevention and care
Age & Ageing
Ugeskrift for laeger
Injury
The spine journal : official journal of the North American Spine Society
Journal of Safety Research
Journal of anesthesia
Cirugia y cirujanos
Regional Anesthesia and Pain Medicine
European Journal of Anaesthesiology
American Journal of Geriatric Psychiatry
JAMA: Journal of the American Medical Association
Gait & Posture
Disability and rehabilitation
Ortopedia, traumatologia, rehabilitacja
BMC Musculoskeletal Disorders
Journal of Aging & Health
BMC geriatrics
BMC Geriatrics
Revue de chirurgie orthopedique et reparatrice de l'appareil moteur
Archives of orthopaedic and trauma surgery
Disability & Rehabilitation
International Journal of Geriatric Psychiatry
Age & Ageing
Clinical rehabilitation
Journal of Clinical Anesthesia
Cochrane Database of Systematic Reviews
Cochrane Database of Systematic Reviews
Cochrane Database of Systematic Reviews
Telemedicine journal and e-health : the official journal of the American Telemedicine Association
Geriatrics & Gerontology International
European Review of Aging and Physical Activity
JAGS
Journal of the American Geriatrics Society
Der Orthopade
Archives of Physical Medicine and Rehabilitation
Osteoporosis International
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Age and Ageing
Journal of the American Geriatrics Society
Archives of physical medicine and rehabilitation
Physiotherapy (United Kingdom)
Geriatrics & gerontology international
Age & Ageing
PLoS ONE

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Plos one
Pharmatherapeutica
Current diabetes reports
Annales de readaptation et de medecine physique : revue scientifique de la Societe francaise de reed
Disability & Rehabilitation
FASEB Journal
Journal of Aging & Health
Trials
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Osteoporosis international : a journal established as result of cooperation between the European Fou
JAMA internal medicine
Ostomy Wound Management
Journal of hypertension
Journal of the American Geriatrics Society
BMJ (Clinical research ed.)
Clinical Rehabilitation
Diabetes Technology & Therapeutics
Jama-Journal of the American Medical Association
International Journal of Nursing Studies
Journal of Gerontological Nursing
Australian Journal of Physiotherapy
Contemporary clinical trials
Plos One
Physiotherapy (United Kingdom)
American journal of epidemiology
Dementia and Geriatric Cognitive Disorders
Archives of Gerontology and Geriatrics

Archives of Physical Medicine & Rehabilitation
Circulation
Clinical Rehabilitation
Neuromodulation : journal of the International Neuromodulation Society
Journal of the American Geriatrics Society
Gerontology
American journal of geriatric psychiatry
Trials
Neurourology and Urodynamics
Journal of the american geriatrics society.
Behavior, Health, & Aging
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Annals of Epidemiology
The Cochrane database of systematic reviews
International journal of cardiology
Archives of physical medicine and rehabilitation
Clinical rehabilitation

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BMC geriatrics

Clinical rehabilitation
The Journal of rheumatology
Journal of Geriatric Drug Therapy
Contemporary Clinical Trials
Annals of Emergency Medicine
BMC Emergency Medicine
BMC Emergency Medicine
Arthritis and Rheumatism
Health Education & Behavior
Cochrane Database of Systematic Reviews
Diabetes care
Trials
Rural and remote health
Neurorehabilitation and neural repair
Journal of the American Geriatrics Society
Canadian Geriatrics Journal
Journal of the American Geriatrics Society
Liver transplantation : official publication of the American Association for the Study of Liver Diseases
Healthcare quarterly (Toronto, Ont.)
International Journal for Quality in Health Care
Nursing in critical care
Journal of the American Geriatrics Society
Journal of Neuroengineering and Rehabilitation
Zeitschrift Fur Sportpsychologie
Geriatric nursing (New York, N.Y.)
Preventive Medicine
Implementation of an Evidence-Based Fall Prevention Toolkit in Convalescent Homes
Journal of the American Geriatrics Society
Revista De Saude Publica
Journal of Advanced Nursing
Journal of Nursing Scholarship
Sleep
Cerebrovascular Diseases
Archives of Internal Medicine

Gerontology
Children's Nurses: Italian Journal of Pediatric Nursing Science / Infermieri dei Bambini: Giornale Italia
Journal of Clinical Nursing
American Journal of Medicine
International Journal of Nursing Studies
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
The American journal of medicine
Clinical Nursing Research

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Journal of Nursing Care Quality
Clinical Nursing Research
BMC health services research
Aging Clinical & Experimental Research
Health Affairs
Maturitas
Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
Revista Espanola de Geriatria y Gerontologia
Annals of Pharmacotherapy
Therapeutische Umschau. Revue th√©rapeutique
Journal of the American Medical Directors Association
American Journal of Epidemiology
Medical care
Journal of Alzheimer's Disease
Gait & Posture
Https://clinicaltrials.gov/show/nct03636672
Https://clinicaltrials.gov/show/nct03501108
Https://clinicaltrials.gov/show/nct00729521
Https://clinicaltrials.gov/show/nct01745263
Https://clinicaltrials.gov/show/nct02976506
Https://clinicaltrials.gov/show/nct02066441
Annals of Internal Medicine
Https://clinicaltrials.gov/show/nct00173693
The New England journal of medicine
Https://clinicaltrials.gov/show/nct02443038
American Journal of Preventive Medicine
American Journal of Geriatric Psychiatry
QJM - Monthly Journal of the Association of Physicians
Diabetes care
Journal of the American Geriatrics Society
Challenges in Synthesizing and Interpreting the Evidence From a Systematic Review of Multifactorial I
BMC Geriatrics
Archives of physical medicine and rehabilitation
Age & Ageing
Journal of Geriatric Physical Therapy
Annals of Physical and Rehabilitation Medicine
Physiotherapy (United Kingdom)
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Implementation science : IS
Aging clinical and experimental research
British Journal of Occupational Therapy
Nursing Research
Investigative Ophthalmology and Visual Science
PLoS ONE
BMC Geriatrics
Journal of the American College of Cardiology

For peer review only

Aging clinical and experimental research
American Journal of Geriatric Psychiatry
Physiotherapy (United Kingdom)
Nephrology
BMC geriatrics
Ceska a Slovenska Neurologie a Neurochirurgie
European journal of clinical pharmacology
Clinical Rehabilitation
Brazilian journal of physical therapy
Assistive technology : the official journal of RESNA
Archives of Physical Medicine & Rehabilitation
PLoS ONE
PLoS ONE
Journal of the American Geriatrics Society
Stroke
Journal of the american medical directors association
Medical Education
JAMA Neurology
European Geriatric Medicine
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of the American Geriatrics Society
JAMA neurology
Topics in Geriatric Rehabilitation
European archives of oto-rhino-laryngology
Accident Analysis and Prevention
Physical therapy
Aging Clinical & Experimental Research
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
American journal of hypertension
Journal of the American Geriatrics Society
Western Journal of Nursing Research
Journal of orthopaedic trauma
Journals of gerontology. Series A, Biological sciences and medical sciences
Journal of Aging Research

Revista portuguesa de cardiologia : orgao oficial da Sociedade Portuguesa de Cardiologia = Portugues
Archives of physical medicine and rehabilitation
Journal of the American Geriatrics Society
Public Health Nursing
Journal of clinical sport psychology
Education and Treatment of Children
Zeitschrift fur Gerontologie und Geriatrie
Chest
American Journal of Clinical Nutrition
International journal of geriatric psychiatry
BMC geriatrics

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Nihon Ronen Igakkai zasshi [Japanese journal of geriatrics]
Clinical Rehabilitation
Rheumatology (oxford, england)
Journal of Sleep Research
Journal of the American Geriatrics Society
European journal of nutrition

Journal of the American Geriatrics Society
Aging Clinical and Experimental Research
Pm&R
Journal of the American Geriatrics Society
Clinical Interventions in Aging
Journal of Cross-Cultural Gerontology
American journal of medicine
Medical Journal of Australia
Journal of cardiothoracic and vascular anesthesia
Https://clinicaltrials.gov/show/nct02566785
Https://clinicaltrials.gov/show/nct02074488
Https://clinicaltrials.gov/show/nct00389844
Https://clinicaltrials.gov/show/nct02687048
Https://clinicaltrials.gov/show/nct03384602
Https://clinicaltrials.gov/show/nct03328793
Https://clinicaltrials.gov/show/nct01986647
Https://clinicaltrials.gov/show/nct03478423
Https://clinicaltrials.gov/show/nct01030874
Https://clinicaltrials.gov/show/nct02483286
Https://clinicaltrials.gov/show/nct03592420
Https://clinicaltrials.gov/show/nct03040011
Https://clinicaltrials.gov/show/nct00618800
Https://clinicaltrials.gov/show/nct02350127
Https://clinicaltrials.gov/show/nct01837017
Https://clinicaltrials.gov/show/nct00217360
Https://clinicaltrials.gov/show/nct00606606
Https://clinicaltrials.gov/show/nct03417531
Https://clinicaltrials.gov/show/nct00817193
Https://clinicaltrials.gov/show/nct02475850
Https://clinicaltrials.gov/show/nct02166333
Https://clinicaltrials.gov/show/nct02335177
Https://clinicaltrials.gov/show/nct01940055
Https://clinicaltrials.gov/show/nct01833182
Https://clinicaltrials.gov/show/nct02167737
Https://clinicaltrials.gov/show/nct03522454
Https://clinicaltrials.gov/show/nct03046771
Https://clinicaltrials.gov/show/nct03263585
Journal of hypertension
Journals of gerontology. Series A, Biological sciences and medical sciences

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Journal of the American Geriatrics Society
Annals of internal medicine
BMC neurology
BMC Geriatrics
Fisioterapia e Pesquisa
Evidence Based Nursing
Journal of the American Medical Directors Association
Movement Disorders
Physiotherapy (United Kingdom)
American journal of geriatric psychiatry
Osteoporosis international
Contemporary clinical trials
Nutrition in Clinical Practice
Clinical biomechanics (bristol, avon)
Contemporary Clinical Trials
Interventions to Prevent Falls in Older Adults: An Updated Systematic Review
Topics in Stroke Rehabilitation
PLoS medicine
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Physical & Occupational Therapy in Geriatrics
Ciencia & Saude Coletiva
Journal of Rehabilitation Research & Development
Gerodontology
Physical therapy
Journal of Trauma & Acute Care Surgery
Journal of internal medicine
Igiene e sanit√† pubblica
Journal of hypertension
Journal of hypertension

Manual Therapy
Journal of the American Geriatrics Society
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
Endocrine practice : official journal of the American College of Endocrinology and the American Assoc
Multiple Sclerosis
Journal of the American Geriatrics Society
Osteoporosis International
Arthritis and Rheumatism

Australian Occupational Therapy Journal
International Journal of Geriatric Psychiatry
Clinical interventions in aging
Physiotherapy
Journal of clinical epidemiology
Australian Health Review
Neurourology and Urodynamics

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International Psychogeriatrics
Journal of neurosurgery
Clinical Rehabilitation
Journal of the American Geriatrics Society
Tropical Medicine and International Health
Scandinavian Journal of Primary Health Care
Archives of Internal Medicine
Health and quality of life outcomes
Proceedings of the Eighteenth International Symposium on Artificial Life and Robotics
The Canadian journal of nursing research = Revue canadienne de recherche en sciences infirmieres
Journal of shoulder and elbow surgery
Zeitschrift f√ºr Gerontologie und Geriatrie
Journal of the American Geriatrics Society
Zeitschrift fur gerontologie und geriatrie
Physical therapy
Journal of rehabilitation research and development
European Review of Aging and Physical Activity
Clinical Rehabilitation
Clinical rehabilitation
Archives of Gerontology and Geriatrics
Journal of the American Geriatrics Society
Journal of the american geriatrics society. Conference: american geriatrics society 2016 annual scient
Archives of Physical Medicine & Rehabilitation
Journal of the American Geriatrics Society
Zhonghua liu xing bing xue za zhi
Bmc Geriatrics
BMC Geriatrics
The Journal of frailty & aging
Blood pressure. Supplement
Wei sheng yan jiu = Journal of hygiene research
Ieee Sensors Journal
Journal of the American Geriatrics Society
Age and ageing
Quality of life research. Conference: 25th annual conference of the international society for quality o
European Urology, Supplements
Asian Biomedicine
Osteoporosis International
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of Bone and Mineral Research
Journal of Bone and Mineral Research
Aging & mental health
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Health Technology Assessment
The Cochrane database of systematic reviews

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Clinical Rehabilitation
Annals of Internal Medicine
Annals of Internal Medicine
Archives of Internal Medicine
Academic Emergency Medicine
PloS one
Gerontology
The Cochrane database of systematic reviews
Journal of General Internal Medicine
Internal Medicine Journal
BMC Complementary and Alternative Medicine
Journal of Bone and Mineral Research
Zeitschrift fur gerontologie und geriatrie
Journal of the American Geriatrics Society
Journal of rehabilitation medicine
Clinical Rehabilitation
Ieee Transactions on Mobile Computing

Nursing Research

Journal of Applied Gerontology
BMC geriatrics
JAMA: Journal of the American Medical Association
Evidence-based Complementary and Alternative Medicine
Journal of hypertension
BMC Geriatrics
International Journal of Technology Assessment in Health Care
Journal of the American Geriatrics Society
Archives of physical medicine and rehabilitation
Bmc Geriatrics
Neurourology and Urodynamics
Journal of psychopharmacology (Oxford, England)
Injury Epidemiology
International Journal of Integrated Care (IJIC)
Journal of the American Geriatrics Society
BMC Geriatrics
American Journal of Occupational Therapy
British Journal of Occupational Therapy
Journal of the American Geriatrics Society
Pilot and feasibility studies
Telemedicine journal and e-health : the official journal of the American Telemedicine Association
Journal of Urology
Annals of Family Medicine
Journal of the American Geriatrics Society
Journal of the american geriatrics society

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Australian & New Zealand Journal of Public Health
The European respiratory journal
Implementation Science
Frontiers in physiology
The journal of nutrition, health & aging
Scandinavian Journal of Primary Health Care
Age and Ageing
BMC research notes
PloS one
Sleep
Journal of the American Geriatrics Society
American Journal of Emergency Medicine
Multiple sclerosis (Houndmills, Basingstoke, England)
American Journal of Preventive Medicine
Journal of the American Geriatrics Society
Journal of Clinical Pharmacy & Therapeutics
Academic Emergency Medicine
Maturitas
Journal of Burn Care & Research
Acta Paulista de Enfermagem
Journal of the american geriatrics society
Journal of the American Geriatrics Society
Drugs & aging
The American journal of emergency medicine
Age & Ageing
Revista RoI de Enfermer√≠a
Medical Care
Journal of Applied Gerontology
Journal of the American Geriatrics Society
Clinical Nursing Research
Applied Nursing Research
Journal of the American Geriatrics Society
Physiotherapy (United Kingdom)
Journal of the American Geriatrics Society
Bmc Geriatrics
BMC infectious diseases
Journal of the American Geriatrics Society
Maturitas
International Journal of Medical Informatics
Clinical Rehabilitation
Journal of the American Medical Directors Association
Journal of the American Geriatrics Society
BMC geriatrics
Alzheimer's and Dementia
Clinical rehabilitation
Journal of physical therapy science

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Journal of General Internal Medicine
Journal of General Internal Medicine
Clinical Rehabilitation
Gerontology
Sports Medicine
Journal of the American Geriatrics Society
Injury Prevention
Physiotherapy Research International
Physiotherapy (United Kingdom)
Journal of Clinical Nursing
BMC Geriatrics
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
European Geriatric Medicine
European Geriatric Medicine
Cyberpsychology, behavior and social networking
Physical Therapy
Sleep
Archives of Internal Medicine
JAMA
Bone
Journal of the American Medical Directors Association
Journal of the American Medical Directors Association
Archives of Internal Medicine
Physician and Sportsmedicine
Archives of physical medicine and rehabilitation
Journal of the American Medical Directors Association
The Journal of Head Trauma Rehabilitation
The Journal of Head Trauma Rehabilitation
The Lancet
[Nihon koshu eisei zasshi] Japanese journal of public health
Archives of Gerontology & Geriatrics
Pediatric research
Osteoporosis International
Journal of the American Geriatrics Society
Journal of the American Medical Directors Association

The Journal of clinical endocrinology and metabolism
British journal of nutrition
Aging & Mental Health
International Journal of Nursing Studies
Journal of Advanced Nursing
Public Health Nursing
Japanese Journal of Physical Fitness and Sports Medicine
JAMA internal medicine
Journal of aging and health
Physical therapy

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Journal of the American Geriatrics Society
Journal of exercise rehabilitation
Hypertension Research
International Journal of Geriatric Psychiatry
Cochrane Database of Systematic Reviews

Jmir Mhealth and Uhealth
Dementia and Geriatric Cognitive Disorders
Journal of manipulative and physiological therapeutics
Trials
Clinical physiology and functional imaging
CMAJ : Canadian Medical Association journal
Journal of Clinical Nursing
Zeitschrift f√ºr Gerontologie und Geriatrie
BMC geriatrics
Drug Safety
Nederlands Tijdschrift Voor Fysiotherapie
European journal of clinical pharmacology
Age & Ageing
Journal of Applied Gerontology
Experimental and clinical endocrinology & diabetes : official journal, German Society of Endocrinology
Optometry and vision science : official publication of the American Academy of Optometry
Journal of the American Geriatrics Society
Gerontology
Journal of the American Geriatrics Society
Archives of Physical Medicine & Rehabilitation
American journal of physical medicine & rehabilitation
Allergy
Academic Emergency Medicine
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Calcified tissue international
Journal of physical activity & health
PLoS neglected tropical diseases
Neurology
Journal of the American Geriatrics Society
International Journal of Care Coordination
Archives of physical medicine and rehabilitation
Injury Prevention (1353-8047)
Trials
Journal of Physiotherapy
WMJ : official publication of the State Medical Society of Wisconsin
Drugs & Aging
Clinical Rehabilitation
Archives of gerontology and geriatrics
British Medical Journal
The Journal of surgical research

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Bmc Geriatrics
Lancet (london, england)
Journal of Aging & Health
Osteoporosis International
Journal of Sensor and Actuator Networks
Rejuvenation research
Cochrane Database of Systematic Reviews
BMC infectious diseases
BMC geriatrics
BMC Geriatrics
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the american geriatrics society
Journal of the American Geriatrics Society
Ontario health technology assessment series
Ontario health technology assessment series

The Gerontologist
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Bmc Public Health
Gerontologist
Clinical Rehabilitation
Plos One
Journal of the American Geriatrics Society
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
Current alzheimer research
Journal of Aging and Physical Activity
BMC Health Services Research
Ubiquitous Intelligence and Computing
Bmc Neurology
Physiotherapy (United Kingdom)
Archives of physical medicine and rehabilitation
BMC Musculoskeletal Disorders
BMC Musculoskeletal Disorders
Iranian Journal of Medical Sciences
JAMA Ophthalmology
Porto Biomedical Journal
Chest
Physiotherapy (United Kingdom)
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Parkinsonism and Related Disorders

Archives of Physical Medicine & Rehabilitation

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BMC public health
Osteoporosis International
Plos One
Clinical rehabilitation
Aging clinical and experimental research
Aging Clinical and Experimental Research
Archives of Gerontology and Geriatrics
Journal of aging and physical activity
Https://clinicaltrials.gov/show/nct02688777
Https://clinicaltrials.gov/show/nct03582137
Https://clinicaltrials.gov/show/nct02522403
Https://clinicaltrials.gov/show/nct02923843
European review of aging and physical activity : official journal of the European Group for Research ir
Journal of Clinical Nursing
Journal of Trauma
Trials
Journal of the American Geriatrics Society
Archives of Gerontology & Geriatrics
Journal of physiological anthropology
Journal of the American Heart Association
International journal of behavioral medicine
Pathologie-biologie
Sleep
Thorax
Journal of Forensic & Legal Medicine
Revista espanola de geriatria y gerontologia
Journal of Clinical Epidemiology
The journals of gerontology. Series A, Biological sciences and medical sciences
Zeitschrift fur Allgemeinmedizin
Nursing Outlook
Interventions to Prevent Falls in Community-Dwelling Older Adults: A Systematic Review for the U.S.
JAMA - journal of the american medical association
Patient related outcome measures
The Journal of Head Trauma Rehabilitation

Pharmacoepidemiology and Drug Safety
Australasian Journal on Ageing
Research in Gerontological Nursing
Bmc Geriatrics
Archives of Internal Medicine
Age & Ageing
Australasian Journal on Ageing
Journal of Neuroengineering and Rehabilitation
Journal of sports sciences
Nursing & Residential Care
The American review of respiratory disease

For peer review only

Journal of Bone and Mineral Research
Osteoporosis International
Journal of Bone and Mineral Research
Academic radiology
Age and Ageing
Osteoporosis International
Age (Dordrecht, Netherlands)
Journal of the American Medical Directors Association
Technology & Health Care
Journal of geriatric physical therapy (2001)
Thrombosis research
Psycho-oncology
Physiotherapy (United Kingdom)
Cadernos de Saude Publica
Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Canc
European journal of cancer (Oxford, England : 1990)
Cochrane Database of Systematic Reviews
European journal of cancer (Oxford, England : 1990)
Microsurgery
Journal of Epidemiology and Community Health
Journal of the American Geriatrics Society
Journal of the National Comprehensive Cancer Network : JNCCN
Journal of Clinical Oncology
Bone
Human pathology
Pneumonologia i alergologia polska
Obesity
Cancer Research
Health expectations : an international journal of public participation in health care and health policy
Gait and Posture
Journal of the American Medical Directors Association
The Journal of pathology
Patologicheskaia fiziologiia i eksperimental'naia terapiia
Pharmacy world & science : PWS
Oncology Nursing Forum
Journal of the National Medical Association
Gan to kagaku ryoho. Cancer & chemotherapy
Journal of Health Communication

Journal of Neurological Surgery Reports
Journal of the American Geriatrics Society
The oncologist
Acta chirurgica Scandinavica
Research on Aging

PloS one

For peer review only

Https://clinicaltrials.gov/show/nct02885337
Journal of the American Geriatrics Society
European journal of public health
Journal of epidemiology
Preventive medicine
The Prostate
British journal of cancer
Case Reports in Medicine
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
The oncologist
European Journal of Cancer
Journal of the American Medical Directors Association
International Journal of Urology
Hepato-gastroenterology
Journal of the American Board of Family Medicine : JABFM
PloS one
Medicina (Brazil)
Technology in cancer research & treatment
European journal of oncology nursing : the official journal of European Oncology Nursing Society

British journal of cancer
Breast cancer research and treatment
Lancet (London, England)
Rehabilitation Oncology
BMC geriatrics
International journal of radiation oncology, biology, physics
Psycho-Oncology
Pacific health dialog : a publication of the Pacific Basin Officers Training Program and the Fiji School o
BMC geriatrics
Cancer Epidemiology Biomarkers and Prevention
Revista Enfermagem UERJ
British journal of cancer
Journal of Nutrition, Health and Aging
Basic & clinical pharmacology & toxicology
Bmc Medicine
European Geriatric Medicine
Bulletin De L Academie Nationale De Medecine
Journal of the American Geriatrics Society
Geriatrics & gerontology international
British Journal of Occupational Therapy
Carcinogenesis
Preventive Medicine
Zeitschrift f√ºr Gerontologie und Geriatrie
Diabetologia
European Journal of Oncology Nursing
Health Communication

For peer review only

Lancet (London, England)
American journal of hypertension
The oncologist
Preventive Medicine
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
Radiation research
Journal of Alzheimers Disease
BMJ (Clinical research ed.)
Digestive diseases and sciences
Gynecologic oncology
Drugs & Aging
Journal of Clinical Oncology
BMC Geriatrics
Journal of the American Geriatrics Society
Rehabilitation Oncology
Clinical Interventions in Aging
Rehabilitation Oncology
Journal of geriatric physical therapy (2001)
Gait and Posture
PLoS ONE
Anticancer research
Breast
British journal of cancer
Journal of surgical oncology
ANZ journal of surgery
Value in Health
Annals of surgery
BMJ (Clinical research ed.)
Bmc Geriatrics
BMC geriatrics
The journals of gerontology. Series A, Biological sciences and medical sciences
Archives des Maladies Professionnelles et de Medecine du Travail
Clinical interventions in aging
European Journal of Hospital Pharmacy
The Gerontologist
International Journal of Clinical Pharmacology and Therapeutics
Journal of the American Geriatrics Society
BMC Geriatrics
Journal of Epidemiology & Community Health
Journal of the American Society of Nephrology : JASN
Aging & Mental Health
Research in Nursing & Health
International Psychogeriatrics
Worldviews on Evidence-Based Nursing
Atencion primaria
Journal of the American Geriatrics Society

For peer review only

Journal of the American Academy of Nurse Practitioners
Diabetes technology & therapeutics
American Journal of Forensic Medicine and Pathology
European Geriatric Medicine
Journal of the American Medical Directors Association
Physical & Occupational Therapy in Geriatrics
Journal of Medicinal Food
Archives of Gerontology and Geriatrics
Journal of the American Geriatrics Society
Journal of Geriatric Physical Therapy
Journal of Geriatric Physical Therapy
Journal of Clinical Outcomes Management
European Geriatric Medicine
Annals of Internal Medicine

Aging & Mental Health
Journal of the American Geriatrics Society
Revista Brasileira de Enfermagem
Journal of Clinical Nursing

Age and ageing
Journal of the American Medical Directors Association
Clinical Rehabilitation
British Journal of Community Nursing
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
Archives of Gerontology and Geriatrics
Trials
International Journal of Geriatric Psychiatry
Human movement science
Journal of bone and mineral research
Journal of Bone and Mineral Research
Journal of the american medical directors association
Journal of aging and physical activity
Osteoporosis International
Hypertension (Dallas, Tex. : 1979)
International journal of nursing studies
Korean j health promot
Journal of Clinical Gerontology and Geriatrics
Physiotherapy (United Kingdom)
Japan journal of nursing science : JJNS
Journal of the American Geriatrics Society
Western Journal of Nursing Research
Archives of Gerontology and Geriatrics
Geriatric Nursing
Topics in Geriatric Rehabilitation

For peer review only

BMC medicine
Lancet (London, England)
Aging Clinical and Experimental Research
Journal of the American Geriatrics Society
BMJ (Online)
The journals of gerontology. Series A, Biological sciences and medical sciences
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
Trials
Cochrane Database of Systematic Reviews
Journal of Evidence-based Care
BMC geriatrics
Journal of the American College of Cardiology
Journal of the American Geriatrics Society
Hypertension (Dallas, Tex. : 1979)
Circulation. Heart failure
Geriatrics and Gerontology International
American Journal of Geriatric Psychiatry
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
BMC Geriatrics
Journal of Clinical Oncology
Journal of Clinical Oncology
Trials
International Journal of Aging & Human Development
Cochrane database of systematic reviews (Online)
Cochrane Database of Systematic Reviews
Cochrane Database of Systematic Reviews
Maturitas
Bmc Public Health
Maturitas
Journal of Cardiovascular Medicine
Archives of Internal Medicine
World neurosurgery
New Zealand Journal of Physiotherapy
Bmj Open
Journal of the American Geriatrics Society
CNS Drugs
Neurogastroenterology and Motility
Journal of Nutrition, Health and Aging
The journal of nutrition, health & aging
Clinical rehabilitation
Romanian Journal of Physical Therapy / Revista Romana de Kinetoterapie
Journal of the American Geriatrics Society

For peer review only

Journal of the American Geriatrics Society
Biomed Research International
Frontiers in neuroscience
Journal of Urology
Archives of Internal Medicine
Archives of Gerontology and Geriatrics
Bmc Geriatrics
Sports medicine (Auckland, N.Z.)
Maturitas
Clinical Rehabilitation
Clinical and Experimental Pharmacology and Physiology
International Psychogeriatrics
Medicina clinica
Evidence-Based Complementary and Alternative Medicine
Journal of the American Medical Directors Association
Journal of the american geriatrics society
JAMA internal medicine
Zeitschrift fur Gerontologie und Geriatrie
Journal of the Academy of Nutrition & Dietetics
Patient preference and adherence
Age & Ageing
Informatics for Health & Social Care
Archives of Physical Medicine & Rehabilitation
Archives of Physical Medicine & Rehabilitation
International Psychogeriatrics
Clinical rehabilitation
Canadian geriatrics journal : CGJ
Journal of assisted reproduction and genetics
Annals of Internal Medicine
Journal of the American Medical Directors Association
Journal of Bone and Mineral Research
Journal of Science and Medicine in Sport
Archives of physical medicine and rehabilitation
American Journal of Respiratory & Critical Care Medicine

Emergency Medicine Journal
Emergency Medicine Journal
Archives of Gerontology & Geriatrics
Arteriosclerosis (dallas, tex.)
Journal of the american geriatrics society
Gerontology
JBI database of systematic reviews and implementation reports
Journal of the American Geriatrics Society
The journals of gerontology. Series A, Biological sciences and medical sciences
European Respiratory Journal
Physical therapy

For peer review only

Journal of Bone and Joint Surgery-American Volume
Journal of Women's Health (15409996)
Journal of Aging & Health
American Journal of Geriatric Pharmacotherapy
Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
Clinical Interventions in Aging
Revista Espanola de Geriatria y Gerontologia
BMC health services research
Carcinogenesis
Journal of the American Medical Directors Association
International Journal of Sports Medicine
Chinese journal of cancer
Sports Medicine

Journal of Geriatric Physical Therapy
Journal of Rehabilitation Medicine (Stiftelsen Rehabiliteringsinformation)
Gerontology
Rheumatology international
Frontiers in Medicine
European Geriatric Medicine
Epidemiology
Neurorehabilitation and neural repair
Physiotherapy (United Kingdom)
JAMA internal medicine
Bmc Geriatrics
Osteoporosis International
Archives of Physical Medicine and Rehabilitation
Global Health Promotion
European Geriatric Medicine
Archives of Physical Medicine and Rehabilitation
Clinical rehabilitation
Rehabilitation Nursing
Journal of endocrinological investigation
New England journal of medicine
Gerontology
Age & Ageing
Journal of Cachexia, Sarcopenia and Muscle
Journal of the American Geriatrics Society
Journal of applied gerontology : the official journal of the Southern Gerontological Society
Rheumatology international
Journal of Nursing Scholarship
American Journal of Public Health
International Journal of Nursing Studies
Hong Kong Journal of Occupational Therapy

Clinical Rehabilitation

For peer review only

Annales de Readaptation et de Medecine Physique
Gerontologist
Journal of Palliative Medicine
Injury Prevention
Singapore medical journal
European journal of clinical investigation
Journal of the Formosan Medical Association

Global Spine Journal
Nefrologia : publicacion oficial de la Sociedad Espanola Nefrologia
Journal of hypertension
American journal of preventive medicine
BMJ: British Medical Journal (International Edition)
Neuropsychologia
Clinical biomechanics (Bristol, Avon)
Haemophilia
Journal of the American Geriatrics Society
Preventing Chronic Disease: Public Health Research, Practice, and Policy
American Journal of Medicine
JAMA internal medicine
Jama-Journal of the American Medical Association
Journal of Bone and Mineral Research
IEEE Transactions on Biomedical Engineering
Ieee Transactions on Biomedical Engineering
European Geriatric Medicine
Clinical pharmacokinetics
Journal of atrial fibrillation
Cochrane Database of Systematic Reviews
Tokyo Jikeikai Medical Journal
Cochrane Database of Systematic Reviews
Archives of Internal Medicine
Journal of hypertension. Supplement : official journal of the International Society of Hypertension
Journal of General Internal Medicine
Emergency Medicine Journal
Journal of the American Medical Directors Association
BMJ Open
Stroke (00392499)
Clinical rehabilitation
Physiotherapy (United Kingdom)
European Neurology
Revue Neurologique
Revue Neurologique
Aging Clinical & Experimental Research
Physical & Occupational Therapy in Geriatrics

Proceedings of the 19th Annual International Conference of the Ieee Engineering in Medicine and Bic

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Journal of Korean Academy of Nursing
Clinical Journal of Oncology Nursing
Journal of Nutrition
Journal of Intellectual Disability Research
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
The Journal of physiology
Biomed Research International
The Journal of neuroscience nursing : journal of the American Association of Neuroscience Nurses
Circulation
Oncology Nursing Forum
The American review of respiratory disease
Journal of Geriatric Oncology
European journal of nuclear medicine
Osteoporosis International

BJU international
Psycho-oncology
The Annals of thoracic surgery
European journal of gynaecological oncology
Sleep
European Geriatric Medicine
Journal of clinical oncology : official journal of the American Society of Clinical Oncology
Aging - Clinical and Experimental Research

Oncology Nursing Forum
Journal of Gerontological Nursing
Cancer Nursing
The British journal of surgery

Annals of surgical oncology
Journal of Psychosocial Oncology
Journal of clinical oncology : official journal of the American Society of Clinical Oncology
International journal of epidemiology
Irish Medical Journal
Journal of Supportive Oncology
Cancer
Journal of Parkinson's disease
Supportive Care in Cancer
Current Medical Research & Opinion
Journal of Geriatric Oncology
Clinical nutrition (Edinburgh, Scotland)
Sozial- und Praventivmedizin
Schweizerische medizinische Wochenschrift
Schweizerische Medizinische Wochenschrift
Archives of Orthopaedic and Trauma Surgery
Zeitschrift f√ºr Gerontologie und Geriatrie

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Zeitschrift fur Gerontologie und Geriatrie
Journal of the American Society of Hypertension
Arteriosclerosis (Dallas, Tex.)
BMC Geriatrics
Parkinsonism and Related Disorders
Journal of neurology, neurosurgery, and psychiatry
Journal of the American Geriatrics Society
The Prostate
European Geriatric Medicine
Journal of Nursing UFPE / Revista de Enfermagem UFPE
Brazilian Journal of Physical Therapy
The Cochrane database of systematic reviews
European Geriatric Medicine
European Geriatric Medicine
PloS one
The Journal of frailty & aging
Pilot and feasibility studies
Journal of the American Geriatrics Society
BMC cancer
Annals of Long Term Care
Journal of Geriatric Physical Therapy
Acta chirurgica Belgica
Archives of Gerontology and Geriatrics
BMC Geriatrics
Clinical rehabilitation
Journal of physiotherapy
Cochrane database of systematic reviews (Online)
Cochrane database of systematic reviews (Online)
Cochrane database of systematic reviews (Online)
Annals of Physical and Rehabilitation Medicine
Clinical pediatrics
Journal of trauma
Child's nervous system : chns : official journal of the international society for pediatric neurosurgery
Topics in Geriatric Rehabilitation
Https://clinicaltrials.gov/show/nct01229969
Injury prevention
The Cochrane database of systematic reviews
Acta orthopaedica Scandinavica
Scandinavian journal of medicine & science in sports
Nicotine & Tobacco Research
Medical journal of the Islamic Republic of Iran
Journal of the American Medical Directors Association
Archives of Physical Medicine & Rehabilitation
Journal of Managed Care Pharmacy

BMC musculoskeletal disorders

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Chiropractic and Manual Therapies
Journal of Clinical Psychiatry
Journal of the American Geriatrics Society
Endocrine practice : official journal of the American College of Endocrinology and the American Assoc
Clinical Rehabilitation
Canadian Journal on Aging
Journal of Clinical Nursing
Journal of Athletic Training
Circulation
Osteoporosis International
Osteoporosis International
Journal of the American Geriatrics Society
Journal of Investigative Medicine
Pediatrics
Journal of Epidemiology & Community Health
Pediatrics
Gerontology
The Journal of the Acoustical Society of America
Clinical interventions in aging
Journal of animal science
World Journal of Orthopedics
Comprehensive gerontology. Section B, Behavioural, social, and applied sciences
AINS: An√§sthesiologie, Intensivmedizin, Notfallmedizin, Schmerztherapie
Anasthesiologie Intensivmedizin Notfallmedizin Schmerztherapie
Current Medical Research & Opinion
Neurourology and Urodynamics
Clinical Interventions in Aging
Aging clinical and experimental research
Osteoporosis International
European geriatric medicine.
Archives of Physical Medicine & Rehabilitation
European Geriatric Medicine
Current Therapeutic Research
Journal of the American Medical Directors Association
Clinical Therapeutics
Trials
Journal of foot and ankle research
Age & Ageing
Scandinavian Journal of Caring Sciences
Scandinavian Journal of Caring Sciences
European journal of heart failure. Conference: heart failure 2016 and the 3rd world congress on acute
BMC Public Health
BMJ (Online)
BMJ (Online)
BMC Geriatrics
Thorax

For peer review only

Bmc Health Services Research
Journals of Gerontology Series B: Psychological Sciences & Social Sciences
Bmc Health Services Research
Australian Journal of Physiotherapy
Trials
BMJ open
BMC geriatrics
BMJ: British Medical Journal (International Edition)
British Journal of Sports Medicine
British Journal of Sports Medicine
British journal of sports medicine
Cochrane Database of Systematic Reviews
Journal of Bone and Mineral Research
BMJ open
Health technology assessment (Winchester, England)
The Cochrane database of systematic reviews
The Cochrane database of systematic reviews
Injury prevention
Cochrane Database of Systematic Reviews
Physiotherapy (United Kingdom)
Physiotherapy (United Kingdom)
Trials
Journal of Physiotherapy
Academic Emergency Medicine
Journal of hospital medicine
Canadian Medical Association Journal
Journal of the American Medical Association
Physical Therapy
Archives of physical medicine and rehabilitation
Topics in Geriatric Rehabilitation
Health Status of Older Adults Discharged Home from an Acute Care Hospital: A Descriptive Study
Injury Prevention
Ieee Sensors Journal
Central European Journal of Public Health
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Obesity Facts
Clinical Biomechanics
Journal of neurotrauma
Journal of neurotrauma
Neurological Sciences
Respiratory medicine
BMC Pulmonary Medicine
Clinical Interventions in Aging
NPG Neurologie - Psychiatrie - G√©riatrie
Journal of the American Geriatrics Society
Journal of Clinical Endocrinology & Metabolism

For peer review only

International journal of clinical pharmacology and therapeutics

Journal of Geriatric Physical Therapy
Physical Therapy
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Annals of Internal Medicine
Menopause
Clinical rehabilitation
Journal of General Internal Medicine
BMC Emergency Medicine
Clinics
Aging Clinical & Experimental Research
Research in Nursing & Health
Surgery, gynecology & obstetrics

Osteoporosis International
Age and ageing
Journal of cardiovascular magnetic resonance : official journal of the Society for Cardiovascular Magn
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
Academic Emergency Medicine
Journal of Alternative & Complementary Medicine
Revista brasileira de enfermagem
Revista portuguesa de cardiologia : orgao oficial da Sociedade Portuguesa de Cardiologia = Portugues
International journal of radiation oncology, biology, physics
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Drugs & Aging
Neurotherapeutics
Annals of Physical and Rehabilitation Medicine
Neurophysiologie Clinique
Neurophysiologie Clinique-Clinical Neurophysiology
Neurophysiologie Clinique
Drugs & aging

Journal of the American Geriatrics Society
PloS one

Trials
Journal of occupational and environmental medicine
Diabetes Research & Clinical Practice
Health Economics
Journal of psychosomatic research
American journal of hematology
European Geriatric Medicine
Evidence Based Nursing
Evidence Based Nursing
Journal of the American Medical Directors Association

For peer review only

Aging Clinical and Experimental Research
British Medical Bulletin
International journal of orthopaedic and trauma nursing
Journal of geriatric physical therapy (2001)
Journal of the American Geriatrics Society
Australian Journal of Primary Health
Diabetes research and clinical practice
The Journal of clinical endocrinology and metabolism
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Frontiers in Ecology and Evolution
Age & Ageing
Journal of the American Geriatrics Society
Aging clinical and experimental research
Journal of animal science
Aging & Mental Health
The American journal of medicine
Journal of Palliative Medicine
Neurorehabilitation & Neural Repair
Journal of hypertension

Journal of the American Geriatrics Society
Osteoporosis International
Journal of the American Geriatrics Society
Archives of Physical Medicine & Rehabilitation
Neurology
Geriaction
Brazilian Journal of Physical Therapy / Revista Brasileira de Fisioterapia
Epilepsy & behavior : E&B
Physiotherapy research international : the journal for researchers and clinicians in physical therapy
Home Health Care Management & Practice
The bankruptcy of marriage.
European journal of physical and rehabilitation medicine
Journal of Emergency Medicine (0736-4679)
Annals of emergency medicine.
Home Healthcare Now
Drugs & Aging
European Geriatric Medicine
American heart journal
International Journal of Geriatric Psychiatry
Stroke
Journal of Parkinson's Disease
The Iowa orthopaedic journal
European Journal of Cancer
Research in social & administrative pharmacy : RSAP
BMC geriatrics

For peer review only

Journal of the American Geriatrics Society
Supportive care in cancer : official journal of the Multinational Association of Supportive Care in Canc
Haematologica
JBI Database of Systematic Reviews & Implementation Reports
Clinical Proteomics
European Geriatric Medicine
Journal of steroid biochemistry and molecular biology
European Journal of Cancer Care
Journal of Cancer Education

Supportive Care in Cancer
Journal of Pain & Symptom Management
Journal of the American Geriatrics Society
BMJ Case Reports
Cahiers de l'Annee Gerontologique
Hormone Research in Paediatrics
Journal of Urology
Arthritis and Rheumatology
Supportive Care in Cancer
Health Technology Assessment
Health Technology Assessment
La Revue du praticien
Oncology Nursing Forum
Journal of Cancer Education
Acta obstetricia et gynecologica Scandinavica
European urology
BMJ supportive & palliative care
The British journal of ophthalmology
Endocrine Reviews
Journal of the American College of Surgeons
Neuro-oncology
World journal of surgery
Journal of gastroenterology and hepatology
Journal of the American Geriatrics Society
Revista de saude publica
Gynecologic oncology
Public Health Research & Practice
Journal of the American Geriatrics Society
Cancer research
Psychological medicine
Revue medicale de Bruxelles
Medical oncology (Northwood, London, England)
American Journal of Gastroenterology
American journal of health behavior
The Journal of trauma
Trials

For peer review only

Journal of geriatric oncology
Infection Control & Hospital Epidemiology
Experimental Gerontology
Avian pathology : journal of the W.V.P.A
BMC Nursing
International journal of nursing studies
The oncologist
Journal of the american geriatrics society. Conference: american geriatrics society 2016 annual scient
Journal of gerontological nursing
Archives of physical medicine and rehabilitation
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of the American Geriatrics Society
Contemporary Clinical Trials
Journal of the American Geriatrics Society
Neuropsychology
Clinical Interventions in Aging
Neurourology and Urodynamics
PM & R: Journal of Injury, Function & Rehabilitation
Cerebrovascular Diseases
Journal of the American Geriatrics Society
JACC. Cardiovascular interventions
Age and Ageing
Age and Ageing
Gait and Posture
Cardiology in the Elderly
The journals of gerontology. Series A, Biological sciences and medical sciences
The Journal of clinical endocrinology and metabolism
Physiotherapy Canada
Archives of Internal Medicine
Gait & posture
International Psychogeriatrics
PloS one
BMC Public Health
Age and ageing
Journal of the American Geriatrics Society
Proceedings of the Ieee
American journal of physical medicine & rehabilitation / Association of Academic Physiatrists
Gait and Posture
Therapeutische Umschau. Revue therapeutique
International journal of laboratory hematology
Cardiopulmonary Physical Therapy Journal
Zeitschrift fur Gerontologie und Geriatrie
Psychology and aging
Quarterly Journal of Medicine
Osteoporosis International
Journal of Geriatric Physical Therapy

For peer review only

The British Journal of Occupational Therapy
Orthopaedic nursing / National Association of Orthopaedic Nurses
Hydrological Processes
Weather and Forecasting
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of Sleep Research
Gerontechnology
Pharmacoepidemiology and Drug Safety
Praxis
Likars'ka sprava
Journal of cardiovascular pharmacology
Rheumatology international
Medicina clinica
Journal of geriatric physical therapy (2001)
BMC evolutionary biology
The lancet. Diabetes & endocrinology
American journal of physical medicine & rehabilitation
Experimental aging research
Journal of the American Geriatrics Society
PM & R : the journal of injury, function, and rehabilitation
Journal of the American Geriatrics Society
Sleep
Gerontology
Journal of the American Geriatrics Society
Journal of Geriatric Physical Therapy
Journal of advanced nursing
International Surgery
Acta chirurgica Belgica
Sleep health
Journal of Long-Term Care Administration
Journal of the American Medical Directors Association
Circulation
Journal of Clinical Oncology
International Journal of Evidence-Based Healthcare
Journal of Geriatric Physical Therapy
Seminars in Dialysis
Journal of the American Medical Directors Association
Journal of Evaluation in Clinical Practice
Journal of Community Health Nursing
Revista espanola de anestesiologia y reanimacion
Revue des maladies respiratoires
American Journal of Alzheimers Disease and Other Dementias
Journal of geriatric oncology
Spinal cord
Hepatology (Baltimore, Md.)
International journal of immunopharmacology

For peer review only

Archives of gerontology and geriatrics
Aging clinical and experimental research
Archives of Gerontology & Geriatrics
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Heart Rhythm
Cochrane Database of Systematic Reviews
Atherosclerosis
Clinical nephrology
Canadian Journal of Emergency Medicine
Cochrane Database of Systematic Reviews
Journal of the American Geriatrics Society
Anticancer research
Safety and Health at Work
Journal of the American Geriatrics Society
European radiology
Journal of the American Medical Directors Association
Respiratory medicine
Journal of the American Geriatrics Society
Evidence Based Nursing
Bone
Supportive Care in Cancer
Gait and Posture
Journal of Gerontological Nursing
Journal of orthopaedic trauma
Nigerian medical journal : journal of the Nigeria Medical Association
Osteoporosis International
Practical radiation oncology
American Journal of Emergency Medicine
International Journal of Integrated Care (IJIC)
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
Neurourology and Urodynamics
Neurourology and Urodynamics
Journal of Nutrition, Health and Aging
Journal of Alzheimer's Disease
Home Health Care Management & Practice
The European respiratory journal
Academic Emergency Medicine
Age and ageing
Age and Ageing
Inflammation research : official journal of the European Histamine Research Society ... [et al.]
BMC geriatrics
Age & Ageing
Surgery
Neurourology and Urodynamics
Journal of the American Medical Directors Association
Medical Journal of Australia

For peer review only

Osteoporosis international : a journal established as result of cooperation between the European Fou
International Journal of Industrial Ergonomics
Circulation
The Consultant pharmacist : the journal of the American Society of Consultant Pharmacists
Health Services & Outcomes Research Methodology
International Gambling Studies
Clinical cancer research : an official journal of the American Association for Cancer Research
Journal of the American Medical Directors Association
Atencion Primaria
Stroke
Http://www.drks.de/drks00008863
Evidence Based Nursing
Annals of Family Medicine
The Annals of thoracic surgery
Ciencia & saude coletiva
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
Chronobiology International
Cochrane Database of Systematic Reviews
International Journal of Geriatric Psychiatry
Age and ageing
Australian Journal of Advanced Nursing
Australian health review : a publication of the Australian Hospital Association
Anatolian Journal of Cardiology / Anadolu Kardiyoloji Dergisi
Pharmacoeconomics
Nursing Economics
International Journal of Geriatric Psychiatry
Age & Ageing
Bmc Public Health
Sleep.
Osteoporosis International
British Medical Journal
Journal of Nursing Management
International Journal of Pharmacy Practice
The Cochrane database of systematic reviews
Age and Ageing
Bmj Open
Trials
The Cochrane database of systematic reviews
Iranian Red Crescent Medical Journal
The Cochrane database of systematic reviews
BMC health services research
Thorax
Age & Ageing
Disability & Rehabilitation
British medical journal (Clinical research ed.)
Health Technology Assessment

For peer review only

Lancet
Bmc Geriatrics
Bmj-British Medical Journal
Age and Ageing
Ageing & Society
British Medical Journal
British journal of sports medicine
Sports medicine (Auckland, N.Z.)
BMC Neurology
Physiotherapy (United Kingdom)
British Medical Journal
Movement Disorders
Bmc Geriatrics
Trials
Zeitschrift fur Gerontologie und Geriatrie
Proceedings of Singapore Healthcare
Osteoporosis international.
The American journal of medicine
The American journal of medicine
Journal of the American College of Cardiology
BMC public health
The Journal of frailty & aging
Australasian journal on ageing
The New Zealand medical journal
Gerontologist
European Geriatric Medicine
Osteoporosis International
Journal of the American Geriatrics Society
Geriatrics & Gerontology International
Journal of the American Medical Directors Association
Age and Ageing
Disability and Rehabilitation
Revista de Pesquisa: Cuidado e Fundamental
Archives of Physical Medicine & Rehabilitation
Aging - Clinical and Experimental Research
Archives of Physical Medicine & Rehabilitation
Journal of clinical epidemiology
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Acta Neurologica Scandinavica
Journal of the American Medical Directors Association
The International journal of artificial organs
Geriatrics and Gerontology International
Obesity surgery
Archives of internal medicine
G√©riatrie et Psychologie Neuropsychiatrie du Vieillissement
Journal of Affective Disorders

For peer review only

G√©riatrie et Psychologie Neuropsychiatrie du Vieillissement
Geriatrie Et Psychologie Neuropsychiatrie De Vieillissement
European Journal of Physical and Rehabilitation Medicine
Osteoporosis International
Bone
European Journal of Physical Medicine and Rehabilitation
Journal of the American Medical Directors Association
Journal of the American Medical Directors Association
Parkinsonism and Related Disorders
Acta Gastro-Enterologica Belgica
European Geriatric Medicine
Journal of Trauma Nursing
Gerontology
American Journal of Alzheimer's Disease & Other Dementias
American Journal of Alzheimer's Disease & Other Dementias
BMC geriatrics
Fizyoterapi Rehabilitasyon
Journal of the American Geriatrics Society
Journal of the American College of Cardiology
Journal of cancer survivorship : research and practice
American journal of obstetrics and gynecology
Journal of the American Geriatrics Society
Effectiveness of an Advance Care Planning Program for Frail Older People in Nursing Homes in Mainla
Recherche en Soins Infirmiers
American heart journal
Irish Journal of Medical Science
Journal of the American Medical Directors Association
Osteoporosis International
Health and quality of life outcomes
Journal of the American Geriatrics Society
Archives of Physical Medicine and Rehabilitation
The Journal of trauma
Archives of Gerontology and Geriatrics
Thorax
British Journal of Clinical Pharmacology
Journal of Emergency Medicine (0736-4679)
JAMA: Journal of the American Medical Association
BMC geriatrics
Cancer
European Geriatric Medicine
Physiotherapy (United Kingdom)
SAGE open medicine
Social Science & Medicine
BMC Public Health
International Journal of Clinical Pharmacy
Drugs & aging

For peer review only

International Journal of Clinical Pharmacy
European Geriatric Medicine
Journal of the American Medical Directors Association
Advances in experimental medicine and biology
Physiotherapy (United Kingdom)
Alzheimer's and Dementia
Journal of the American Medical Directors Association
Bone marrow transplantation
Journal of human hypertension
British Journal of Occupational Therapy
Australasian journal on ageing
The Medical journal of Australia
Journal of the American Geriatrics Society
Physiological measurement
Medical Hypotheses
IEEE Transactions on Biomedical Engineering
Journal of Parkinson's disease
The European respiratory journal
Bmj Open
BMC geriatrics
Public Health Nursing
Home healthcare nurse
Home Healthcare Nurse
Archives of Physical Medicine & Rehabilitation
International Journal of Geriatric Psychiatry
International Journal of Geriatric Psychiatry
American Journal of Geriatric Psychiatry
Gerontology
International journal of cardiology
Female Pelvic Medicine and Reconstructive Surgery
Arthritis care & research
BMC musculoskeletal disorders
Seminars in arthritis and rheumatism
Osteoporosis international : a journal established as result of cooperation between the European Fou
Arthritis and Rheumatism
BMC Public Health
Clinics (Sao Paulo, Brazil)
BMC public health

Archives of Gerontology and Geriatrics
Physical Therapy
Physiotherapy Research International
European Geriatric Medicine
Thorax
The American journal of clinical nutrition
The American surgeon

For peer review only

Vaccine
Consultant Pharmacist
Neurology
International journal of environmental research and public health
Human Factors and Ergonomics in Manufacturing & Service Industries
International Journal of Environmental Research and Public Health
Physical Therapy
Gait & Posture
Tijdschrift voor gerontologie en geriatrie
International Journal of Geriatric Psychiatry

Turkish Journal of Geriatrics-Turk Geriatri Dergisi
Journal of epidemiology and community health
Psychology of Women Quarterly
Diabetes care
Age and ageing
Geriatrics & Gerontology International
The American journal of geriatric pharmacotherapy

Clinical journal of the American Society of Nephrology : CJASN
Injury Prevention (1353-8047)
Nursing Standard
Nursing Standard
Nursing standard (royal college of nursing (great britain) : 1987)
International Psychogeriatrics
Journals of Gerontology Series B-Psychological Sciences and Social Sciences
Journal of clinical epidemiology
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Calcified tissue international
Presse Medicale
Clinical Nutrition
Journal of Biomechanics
2010 Annual International Conference of the Ieee Engineering in Medicine and Biology Society
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
2016 38th Annual International Conference of the Ieee Engineering in Medicine and Biology Society
Copd
Journal of Biomedical Informatics
Journal of Bone and Mineral Research
Osteoporosis International
Osteoporosis International
BMC geriatrics
Journal of the American Geriatrics Society
BMJ open
Injury Prevention (1353-8047)
Journal of the American Geriatrics Society

For peer review only

Gait & posture
Journal of Telemedicine and Telecare
Journal of telemedicine and telecare
Journal of Telemedicine & Telecare
BMJ (Clinical research ed.)
Journal of Advanced Nursing
Annals of physical and rehabilitation medicine
Journal of Sport Rehabilitation
PLoS ONE
Smart Homes and Beyond
Cochrane Database of Systematic Reviews
Clinical endocrinology
Irbm
Archives of Gerontology and Geriatrics

Orthopaedic Physical Therapy Clinics of North America
Giornale di Gerontologia
Sleep medicine
Distributed Computing, Artificial Intelligence, Bioinformatics, Soft Computing, and Ambient Assisted L
European Journal of Social Work
International Journal of Environmental Research and Public Health
Thorax
Journal of Physical Therapy Science
Journal of wound, ostomy, and continence nursing : official publication of The Wound, Ostomy and C
Central European Journal of Nursing & Midwifery
Systematic reviews
Archives of Physical Medicine and Rehabilitation
Fisioterapia em Movimento
The European respiratory journal
International journal of clinical pharmacy
The European respiratory journal
Trials
Clinical research in cardiology : official journal of the German Cardiac Society
Journal of neurophysiology
Geriatrics & gerontology international
Journal of the American Geriatrics Society
Diabetes
Gerontologist
Geriatric Nursing
Journal of Nutrition Health & Aging
Bone
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
Journal of Aging & Physical Activity
BMJ open
Revue neurologique
Population health management

For peer review only

Menopause (New York, N.Y.)
Stroke
Telemedicine and e-Health
Journal of Rehabilitation Research & Development
Neurogastroenterology and motility : the official journal of the European Gastrointestinal Motility So
Clinical Gerontologist
Journal of Geriatric Physical Therapy
Cochrane Database of Systematic Reviews
Journal of Acute Care Physical Therapy (Acute Care Section - APTA, Inc.)
Journal of Trauma & Acute Care Surgery
Leukemia research
Osteoporosis international : a journal established as result of cooperation between the European Fou
Neurorehabilitation
Family Practice
The Australian journal of rural health
Human reproduction (Oxford, England)
Presse medicale (Paris, France : 1983)
Aging Clinical and Experimental Research
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
BMC Geriatrics
Bone
Journal of Bone and Mineral Research
Isme Journal
Online Brazilian Journal of Nursing
BMC Geriatrics
BMC geriatrics
Frontiers in Neurology
Basic and clinical pharmacology and toxicology. Conference: 37th SEF national meeting with guest so
BMC Geriatrics
Archives of Physical Medicine & Rehabilitation
Journal of the American Geriatrics Society
American journal of geriatric pharmacotherapy
Journal of the American Dietetic Association
Age & Ageing
Osteoporosis international : a journal established as result of cooperation between the European Fou
BMJ Quality and Safety
Developmental Psychology
Journal of neurology, neurosurgery, and psychiatry
PM & R : the journal of injury, function, and rehabilitation
Journal of Geriatric Physical Therapy

Journal of nutrition, health & aging
Alzheimer's and Dementia

American journal of otology

For peer review only

Optometry and Vision Science
Optometry and vision science : official publication of the American Academy of Optometry
Ophthalmic & physiological optics : the journal of the British College of Ophthalmic Opticians (Optom
Trials
BMC Health Services Research
BMC geriatrics
Calcified Tissue International
Archives of Physical Medicine and Rehabilitation
Physical Therapy
Age and Ageing
Jama
Aging (Milan, Italy)
Archives of gerontology and geriatrics
Arquivos brasileiros de cardiologia
The Laryngoscope
Journal of the American Medical Directors Association
PloS one
Journal of the American Geriatrics Society
Clinical nursing research
Macedonian Journal of Medical Sciences
The British journal of psychiatry : the journal of mental science
Brain : a journal of neurology
Osteoporosis International
Journal of the American Medical Directors Association
Irish Journal of Medical Science
BMC Geriatrics
JAMA internal medicine
Bone
Osteoporosis International
Rheumatic diseases clinics of North America
American Journal of Epidemiology

Journal of hypertension
Injury Prevention
Osteoporosis international
Age and ageing
Journal of aging research
Journal of aging and physical activity
Gait and Posture
Journal of Aging & Physical Activity
Behavior research methods, instruments, & computers : a journal of the Psychonomic Society, Inc
Genes Brain and Behavior
Physiotherapy (United Kingdom)
Journal of the American Geriatrics Society
Experimental Aging Research
Health & Quality of Life Outcomes

For peer review only

Disability and rehabilitation
Pharmaceutical Biology
Stroke
Journal of Geriatric Physical Therapy
Topics in Geriatric Rehabilitation
Topics in Geriatric Rehabilitation
Irish Journal of Medical Science
The Annals of thoracic surgery

Osteoporosis International
Journal of Aging & Physical Activity
Gait and Posture
Journal of the American Medical Directors Association
Environmental health: Home, school and community.
11th Euromicro Conference on Digital System Design - Architectures, Methods and Tools : Dsd 2008,
IEEE journal of biomedical and health informatics
Ortopedia, traumatologia, rehabilitacja
Injury
Osteoporosis International
Journal of the american geriatrics society
Gait & posture
International journal of cardiology
Journal of the American Medical Directors Association
Journal of Immigrant & Minority Health
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Journal of the Association for Research in Otolaryngology : JARO
Brazilian Journal of Occupational Therapy / Cadernos Brasileiros de Terapia Ocupacional
PM and R
American Journal of Geriatric Psychiatry
Aging & Mental Health
The journals of gerontology. Series A, Biological sciences and medical sciences
Physical & Occupational Therapy in Geriatrics
Archives of Gerontology and Geriatrics
Annals of the Royal College of Surgeons of England
Journal of General Internal Medicine
Cochrane Database of Systematic Reviews
Australian Journal of Physiotherapy
Revista de Saude Publica
Journal of the American Medical Directors Association
American Journal of Geriatric Psychiatry
Journal of intensive care medicine
Revista de Enfermagem Refer√™ncia
Maturitas
Journal of Clinical Psychopharmacology
Canadian Journal of Ophthalmology-Journal Canadien D Ophtalmologie

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International journal of geriatric psychiatry
PLoS ONE
Aging Clinical and Experimental Research
European Geriatric Medicine
BMC Geriatrics
Key aspects of elder care: Managing falls, incontinence, and cognitive impairment.
International journal of surgery (London, England)
Injury Prevention (1353-8047)
Australian and New Zealand journal of public health
PloS one
Osteoporosis International
General Hospital Psychiatry
Osteoporosis International
Current Psychiatry Reports
British journal of cancer
Physical Therapy
The Journal of emergency medicine
ASAIO journal (American Society for Artificial Internal Organs : 1992)
The journals of gerontology. Series A, Biological sciences and medical sciences
Heart (British Cardiac Society)
PloS one
2017 Ieee 6th Global Conference on Consumer Electronics
Acta Paulista de Enfermagem
2014 Ieee International Symposium on Innovations in Intelligent Systems and Applications
Sleep
Sleep
Health & Social Work

JAMA Internal Medicine
Aging Clinical and Experimental Research
Issues in Mental Health Nursing
Proceedings of the Iadis International Conference E-Health 2012
Journal of the American Society of Hypertension
Neurology
Journal of Behavioral Health Services & Research
Osteoporosis International
British Journal of Therapy & Rehabilitation
Journal of the American Medical Directors Association
Nephrology Dialysis Transplantation
The American surgeon
JAMA internal medicine
Cancer
Geriatrics & gerontology international
Trials
Current gerontology and geriatrics research
Journal of Nutrition, Health and Aging

For peer review only

American Journal of Emergency Medicine
Journal of Geriatric Physical Therapy
Disability and health journal
European Journal of Epidemiology
American Journal of Emergency Medicine
Medical Journal of Australia
Journal of Nutrition, Health and Aging
Annals of Physical and Rehabilitation Medicine
European Geriatric Medicine
Archives of Internal Medicine
Journal of the American Geriatrics Society
Revista da Escola de Enfermagem da USP
Aging - Clinical and Experimental Research
Journal of the American Geriatrics Society
Lancet
Reviews in Clinical Gerontology
Palliative Medicine
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Der Anaesthesist
Revista Clinica Espanola
Archives of Gerontology and Geriatrics
Gerokomos
Journal of general internal medicine
Norsk Epidemiologi
Evidence Based Medicine
Cleveland Clinic journal of medicine
Kidney international

Aging & Mental Health
Archives of gerontology and geriatrics
Chinese Medical Journal
Chinese medical journal
Geriatrics and Gerontology International
Supportive Care in Cancer
Supportive Care in Cancer
Supportive Care in Cancer
Home Health Care Management & Practice

Physiotherapy (United Kingdom)
Geriatrics & gerontology international
European Geriatric Medicine
Journal of the American Geriatrics Society
The Journal of heart valve disease
FASEB Journal
PLoS ONE
Journal of Nursing

For peer review only

Archives of Physical Medicine and Rehabilitation
Progres en Urologie
Transplantation
Journal of the American Geriatrics Society
American journal of preventive medicine
BMC Geriatrics
Perspectives in Health Information Management
The British journal of surgery
Clinical Rehabilitation
Journal of women's health (2002)
Health Care for Women International
Journal of applied gerontology : the official journal of the Southern Gerontological Society
Revue de Geriatrie
The Journal of clinical endocrinology and metabolism
Journal of gerontological social work
Annales de Genetique
BMC geriatrics
Texto & Contexto Enfermagem
Training & Development
Calcified tissue international
Injury Prevention (1353-8047)
Journal of Public Health Dentistry
Physiotherapy
Fundamental and Clinical Pharmacology
Journal of Applied Gerontology
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine anc
International journal of immunopathology and pharmacology
Revista de Investigacion Clinica
Sleep
Archives of Physical Medicine and Rehabilitation
Journal of oral and maxillofacial surgery : official journal of the American Association of Oral and Max
Journal of the American Medical Directors Association
Journal of Geriatric Physical Therapy
Archives of Physical Medicine & Rehabilitation
Bmc Public Health

British Journal of Clinical Pharmacology
Journal of geriatric physical therapy (2001)
British Journal of Occupational Therapy

Journal of Trauma
Cochrane Database of Systematic Reviews
The Journal of asthma : official journal of the Association for the Care of Asthma
Zeitschrift f√ºr Gerontologie und Geriatrie
Journal of the american geriatrics society
Bjs Open

For peer review only

Nutrition journal
Nutrition (burbank, los angeles county, calif.)
Earth-Science Reviews
Osteoporosis international : a journal established as result of cooperation between the European Fou
Clinical Gerontologist
Osteoporosis International
European Geriatric Medicine
Experimental Gerontology
BMC geriatrics
Osteoporosis International
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Geroscience
The Journal of emergency medicine
Plos One
Neurology
Journal of the American Geriatrics Society
Journal of Neural Transmission
Gerontology
Journal of geriatric physical therapy (2001)
Public Health
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Menopause (New York, N.Y.)
American Journal of Geriatric Psychiatry
International Urogynecology Journal and Pelvic Floor Dysfunction
Disability & Rehabilitation
Journal of hypertension
Zeitschrift fur Gerontologie und Geriatrie
Archives of Gerontology and Geriatrics

Complementary Health Practice Review
Activities, Adaptation & Aging

Preventing chronic disease
Revista Latino-Americana De Enfermagem
Journal of physiotherapy
Health Promotion Journal of Australia
Journal of Aging and Physical Activity
Journal of Hospice & Palliative Nursing
Stroke
Sleep and Biological Rhythms
Archives of physical medicine and rehabilitation

For peer review only

BMC neurology
Australasian journal on ageing
Canadian Journal of Rehabilitation
Sleep medicine
Nursing Research
Healthmed
Osteoporosis international : a journal established as result of cooperation between the European Fou
Journal of Environmental Health
Aging Successfully
Journal of Bone and Mineral Research
Arthritis care & research
Journal of Bone and Mineral Research
Arthritis and Rheumatology
Journal of Bone and Mineral Research
Age
Clinical interventions in aging
Harefuah
BMC Health Services Research
Injury Prevention
The American journal of medicine
International journal of cardiology
10th World Congress on Human Reproduction
GMS health technology assessment
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Journal of the American Geriatrics Society
Experimental Gerontology
Journal of Nutrition, Health and Aging
American Journal of Kidney Diseases
Journal of Advanced Nursing
Health Care Management Review
PloS one
European Journal of Epidemiology
Journal of Bone and Mineral Research
American Journal of Recreation Therapy
BJOG : an international journal of obstetrics and gynaecology
Annals of emergency medicine
Archives of Physical Medicine & Rehabilitation
Journal of Bone and Mineral Research
Osteoporosis International
Psychologie & Neuropsychiatrie Du Vieillissement
IRBM
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Nutrition (Burbank, Los Angeles County, Calif.)
Pharmacy Education
Bulletin de l'Academie nationale de medecine

For peer review only

International journal of microcirculation, clinical and experimental
Drug and Alcohol Dependence
Nephron
Journal of Alzheimer's Disease
International Journal of Geriatric Psychiatry
Hypertension (Dallas, Tex. : 1979)
European Geriatric Medicine
Archives of Physical Medicine & Rehabilitation
Nicotine & tobacco research : official journal of the Society for Research on Nicotine and Tobacco
PLoS ONE
Gait & posture
BMC geriatrics
Alzheimer's and Dementia
Brain Imaging and Behavior
Age and ageing
The spine journal : official journal of the North American Spine Society
European Geriatric Medicine
Alzheimer's and Dementia
Parkinsonism & Related Disorders
Neurology
Osteoporosis international : a journal established as result of cooperation between the European Fou
FASEB Journal
Archives of Gerontology and Geriatrics
British journal of clinical pharmacology
American Journal of Geriatric Psychiatry
International Journal of Geriatric Psychiatry
Early Childhood Research Quarterly
Journal of Occupational Medicine and Toxicology
Journal of orthopaedic trauma
Otolaryngology-Head & Neck Surgery
Alzheimer Disease and Associated Disorders
Social Psychiatry and Psychiatric Epidemiology: The International Journal for Research in Social and G
Journal of the American Geriatrics Society
PloS one
Journals of Gerontology Series a-Biological Sciences and Medical Sciences
PloS one
Plos One
Clinical science and molecular medicine
The Journal of steroid biochemistry and molecular biology
BMC geriatrics
Aging Clinical & Experimental Research
European Geriatric Medicine
Journal of the American Geriatrics Society
Journal of the American Dietetic Association
Clinical Rehabilitation
Medical Journal of Australia

For peer review only

PloS one
Zeitschrift fur Gerontologie und Geriatrie
Annals of Physical and Rehabilitation Medicine
Physical Therapy
PM and R
The Journal of clinical endocrinology and metabolism

Journal of hospital medicine
Journal of the American Geriatrics Society
Revue de Geriatrie
Orthopedics
PloS one
Journal of orthopaedic trauma
Le Mali medical
Archives of Orthopaedic and Trauma Surgery
European Geriatric Medicine
La Tunisie medicale
Anesthesia and analgesia
Unfallchirurg
Regional Anesthesia and Pain Medicine
Journal of orthopaedic research : official publication of the Orthopaedic Research Society
The spine journal : official journal of the North American Spine Society
Archives of Physical Medicine & Rehabilitation
The Journal of head trauma rehabilitation
Spinal Cord
The journal of spinal cord medicine
Journal of neurosurgery. Spine
Journal of Trauma
Hawaii medical journal
Journal of rehabilitation medicine
Spine
Archives of Physical Medicine and Rehabilitation
Ulusal travma ve acil cerrahi dergisi = Turkish journal of trauma & emergency surgery : TJTES
Archives of Physical Medicine & Rehabilitation
Journal of the American College of Surgeons
Journal of Neurotrauma
Spinal Cord
Neurourology and urodynamics
Topics in Spinal Cord Injury Rehabilitation
Gait and Posture
Spinal cord
The journal of spinal cord medicine
The Journal of pharmacology and experimental therapeutics
Puerto Rico Health Sciences Journal
Physiotherapy Theory & Practice
Neuromodulation : journal of the International Neuromodulation Society

For peer review only

Nursing & Health Sciences
Journal of the American Geriatrics Society
Injury Prevention (1353-8047)
Revista de Saude Publica
Influenza and other respiratory viruses
HPB : the official journal of the International Hepato Pancreato Biliary Association
Journal of Nutrition Health & Aging
The Cochrane database of systematic reviews
Maturitas
Cochrane Database of Systematic Reviews
Medical care
Paleobiology
New South Wales Public Health Bulletin
Australian and New Zealand journal of public health
Journal of medical case reports
Annals of Internal Medicine
Journal of Aging & Physical Activity
Physiotherapy Research International
Physiotherapy Research International
Physical & Occupational Therapy in Geriatrics
Aging - Clinical and Experimental Research
Trials
Journal of telemedicine and telecare
Physiotherapy (United Kingdom)
Journal of the American Geriatrics Society
Journals of Gerontology - Series A Biological Sciences and Medical Sciences
Journal of affective disorders
Journal of the American Geriatrics Society
Alzheimer's and Dementia
Screening for Visual Impairment in Older Adults: Systematic Review to Update the 1996 U.S. Preventi
Pediatrics
Investigative Ophthalmology and Visual Science
Singapore medical journal
Injury epidemiology
PLoS ONE
Journal of the Formosan Medical Association = Taiwan yi zhi
European Geriatric Medicine
International journal for quality in health care : journal of the International Society for Quality in Heal
Archives of gerontology and geriatrics
Archives of physical medicine and rehabilitation
International journal of biometeorology
The Southeast Asian journal of tropical medicine and public health
The British journal of ophthalmology
American Journal of Geriatric Pharmacotherapy
Value in Health
Canadian Journal of Geriatrics

For peer review only

Kidney and Blood Pressure Research
First International Symposium on Bioengineering

Aging Clinical & Experimental Research
Journal of the Formosan Medical Association = Taiwan yi zhi
Osteoporosis International
Western journal of nursing research
Journal of Neuroengineering and Rehabilitation
American Journal of Alzheimer's Disease
European heart journal
The journal of nutrition, health & aging
Injury
Osteoporosis International
Osteoporosis International
International journal of injury control and safety promotion
Age & Ageing
Age & Ageing
Journal of the American Medical Directors Association
Injury Prevention (1353-8047)
European journal of physical and rehabilitation medicine
Yonsei medical journal
The Lancet
Journal of the American Geriatrics Society
AJR. American journal of roentgenology
Joint Bone Spine
Annals of Physical and Rehabilitation Medicine
Osteoporosis International
Rejuvenation research
International journal of environmental research and public health
Aging clinical and experimental research
Osteoporosis international
European Journal of Ageing
Injury Prevention (1353-8047)
Harefuah
European Respiratory Journal
Blood Pressure
Applied ergonomics
Ergonomics
Ergonomics
Revue Medicale de Bruxelles
Osteoporosis International
Physiotherapy theory and practice
Physical Therapy
The Journal of thoracic and cardiovascular surgery
IEEE transactions on medical imaging
Yonsei medical journal

For peer review only

International journal of cardiology
International journal of cardiology
Journal of Exotic Pet Medicine
The Canadian journal of cardiology
Heart, lung & circulation
Circulation
The Annals of thoracic surgery
The Annals of thoracic surgery
Heart (British Cardiac Society)
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
The Journal of heart valve disease
International journal of cardiology
Revista portuguesa de cardiologia : orgao oficial da Sociedade Portuguesa de Cardiologia = Portugues
Journal of Cardiovascular Medicine
European journal of radiology
The American journal of cardiology
The American journal of cardiology
International journal of cardiology
Circulation
Revista portuguesa de cardiologia : orgao oficial da Sociedade Portuguesa de Cardiologia = Portugues
World journal for pediatric & congenital heart surgery
Mymensingh medical journal : MMJ
Pediatric cardiology
Autonomic neuroscience : basic & clinical
Journal of Investigative Medicine
The Annals of thoracic surgery
The American journal of cardiology
Clinical autonomic research : official journal of the Clinical Autonomic Research Society
Radiology
The New England journal of medicine
International journal of cardiology
International journal of cardiology
Circulation
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
American heart journal
Molecular genetics & genomic medicine
Clinical anatomy (New York, N.Y.)
Pacing and clinical electrophysiology : PACE
American heart journal
Circulation
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
European Journal of Heart Failure
The Annals of thoracic surgery
Scandinavian journal of thoracic and cardiovascular surgery
Journal of pediatric surgery

For peer review only

Pediatric cardiology
Kokyu to junkan. Respiration & circulation
Heart (British Cardiac Society)
The American journal of cardiology
Journal of the American College of Cardiology
The American journal of cardiology
PloS one
Pediatric cardiology
Circulation
Journal of the American College of Cardiology
British heart journal
Journal of the American College of Cardiology
Circulation
The Annals of thoracic surgery
Revista portuguesa de cardiologia : orgao oficial da Sociedade Portuguesa de Cardiologia = Portugues
International journal of cardiology
JACC. Clinical electrophysiology
International journal of cardiology

Monographs of the Society for Research in Child Development
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
Heart (British Cardiac Society)
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
The international journal of cardiovascular imaging
Canadian Journal of Surgery
Journal of Trauma
Epilepsy and Behavior
Cancer
Spinal Cord
International Journal of Geriatric Psychiatry
American Journal of Geriatric Psychiatry
The International journal of social psychiatry
Journal of injury & violence research
World Psychiatry
Praxis der Psychotherapie und Psychosomatik
Cardiology in the young
Journal of the American Geriatrics Society
International Journal of Geriatric Psychiatry
Journal of the American Geriatrics Society
American Journal of Geriatric Psychiatry
Journal of the American Geriatrics Society
The Journal of clinical psychiatry
Journal of Investigative Medicine
Gerontologist
Psychosomatics: Journal of Consultation and Liaison Psychiatry
PLoS ONE

For peer review only

Journal of the American Geriatrics Society
Psychological Medicine
Journal of Affective Disorders
Journal of homosexuality
Crisis: The Journal of Crisis Intervention and Suicide Prevention
Psychiatry Research
Journal of General Internal Medicine
Psychological Medicine
Acta Chirurgica Austriaca
Journal of Psychosomatic Research

Cochrane Database of Systematic Reviews
The Journal of trauma
BJPsych Bulletin
International Journal of Geriatric Psychiatry
Deutsches √Ñrzteblatt International
JAMA Psychiatry
JAMA Psychiatry
The American journal of medicine
Gut
Age & Ageing
Journal of foot and ankle research
European journal of cardio-thoracic surgery : official journal of the European Association for Cardio-tl
Archives of Gerontology and Geriatrics
Acta chirurgica Scandinavica
American journal of epidemiology
Annals of oncology : official journal of the European Society for Medical Oncology
European Geriatric Medicine
Presse medicale (Paris, France : 1983)
Journal of internal medicine
American Journal of Clinical Nutrition
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Cochrane Database of Systematic Reviews
Oncology nursing forum

Asian Pacific journal of cancer prevention : APJCP
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American Journal of Geriatric Psychiatry
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PloS one
Addiction
Jama
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Revista portuguesa de pneumologia
The New England Journal of Medicine
The Journal of infection

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The Journal of the Association of Nurses in AIDS Care : JANAC
Antiviral therapy
Antiviral therapy
Journal of the american geriatrics society.
Menopause (New York, N.Y.)
Journal of the International AIDS Society
Hispanic Health Care International
Aids Patient Care and Stds
Progress in Palliative Care
Journal of clinical immunology
Https://clinicaltrials.gov/show/nct01823250
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Archives of Gerontology and Geriatrics
Age & Ageing
Physical Therapy
World Council of Enterostomal Therapists Journal
American Journal of Alzheimer's Disease and Other Dementias
Geriatric Nursing
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Archives of Physical Medicine & Rehabilitation
BMJ (Clinical research ed.)
Drugs & Aging
The Gerontologist
American Journal of Preventive Medicine
Journals of Gerontology Series A: Biological Sciences & Medical Sciences
Journal of General Internal Medicine
Endocrine Reviews
European Geriatric Medicine
JBI Database of Systematic Reviews and Implementation Reports
Clinical Gerontologist
Journal of General Internal Medicine
The Journal of Head Trauma Rehabilitation
Surgery for obesity and related diseases : official journal of the American Society for Bariatric Surgery
International Journal of Integrated Care (IJIC)
Enfermeria clinica
American Journal of Geriatric Psychiatry
Age
Journal of the American Geriatrics Society
Journal of Epidemiology & Community Health
Optometry and Vision Science
Alzheimer disease and associated disorders
Journal of Trauma - Injury, Infection and Critical Care
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Cochrane Database of Systematic Reviews
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NT Research
Brain Injury
Clinical Rehabilitation
Injury Prevention (1353-8047)
Archives of disease in childhood

PloS one
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Archives of Gerontology & Geriatrics
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The Gerontologist
Clinical interventions in aging
Journal of bone and mineral research : the official journal of the American Society for Bone and Mine
Alzheimer's and Dementia

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Health technology assessment (Winchester, England)
Age & Ageing
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Otolaryngology--head and neck surgery : official journal of American Academy of Otolaryngology-Hea
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Neurocritical care
European journal of epidemiology
Journal of Bone and Mineral Research
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Journal of Communication Disorders
American Journal of Kidney Diseases
Proceedings of the 2nd World Congress of the International Society of Physical and Rehabilitation Me
Arthritis & rheumatology (Hoboken, N.J.)
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Journal of the American Geriatrics Society
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Journal of Clinical Nursing
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Journal of Geriatric Physical Therapy
Journal of General Internal Medicine
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Oral diseases
Genes, chromosomes & cancer
The European respiratory journal
Archives of Biochemistry and Biophysics
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Neuropeptides
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The Korean journal of gastroenterology = Taehan Sohwagi Hakhoe chi
Journal of viral hepatitis
Virology
International journal of clinical practice
Journal of mammalian evolution
Multiple sclerosis (Houndmills, Basingstoke, England)
Injury
Topics in Spinal Cord Injury Rehabilitation
The journal of spinal cord medicine
Journal of Spinal Cord Medicine
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Archives of Physical Medicine and Rehabilitation
Spinal cord
Physiotherapy (United Kingdom)
PM and R
Archives of Physical Medicine and Rehabilitation
The journal of trauma and acute care surgery
Physical therapy
Disability and rehabilitation
International Archives of Medicine
The Journal of trauma
Archives of Physical Medicine & Rehabilitation
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European Spine Journal

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Pediatric neurology
Spinal cord
Pediatrics
Journal of Physiotherapy (Elsevier)
Archives of physical medicine and rehabilitation
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Annals of Emergency Medicine
AIDS and Behavior
Thrombosis research
Occupational Therapy in Health Care
Blood
National vital statistics reports : from the Centers for Disease Control and Prevention, National Cente
Archives of Physical Medicine and Rehabilitation

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CMAJ : Canadian Medical Association journal = journal de l'Association medicale canadienne
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The Journal of the Association of Nurses in AIDS Care : JANAC
Journal of the American Geriatrics Society
HIV medicine
Journal of acquired immune deficiency syndromes (1999)
BMC geriatrics
Journal of Head Trauma Rehabilitation
Respiratory Medicine
Accident; analysis and prevention
Circulation
Burns (03054179)
Archives of Disease in Childhood
Academic emergency medicine : official journal of the Society for Academic Emergency Medicine
Respirology

Https://clinicaltrials.gov/show/nct03002311
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Medical science monitor : international medical journal of experimental and clinical research
Journal of paediatrics and child health
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Clinical Endocrinology
Current treatment options in neurology
Cochrane database of systematic reviews (Online)

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The Journal of thoracic and cardiovascular surgery

Irish Medical Journal
Neurosurgical focus
Circulation. Cardiovascular imaging
Pediatrics
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Foundations of Augmented Cognition, Vol 11
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International Journal of Caring Sciences
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British journal of clinical pharmacology
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Acta Stomatologica Croatica
Klinische Padiatrie
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Jornal de Pediatria
Childs Nervous System
The journal of trauma and acute care surgery
Journal of Trauma
Turkish Journal of Pediatrics
The Journal of clinical endocrinology and metabolism
Journal of pediatric surgery
Endocrine Reviews
Rhinology
AIDS care
Maternal & Child Health Journal
Injury
European journal of epidemiology
Anaesthesia and intensive care
American Journal of Obstetrics and Gynecology
Pediatric Emergency Care
Injury
Evidence report/technology assessment
Archives de Pediatrie
Clinical infectious diseases : an official publication of the Infectious Diseases Society of America
Childs Nervous System
Eurasian Journal of Emergency Medicine
Journal of Trauma
Journal of Neurosurgery: Pediatrics
Journal of Orthopaedic Trauma
European review of aging and physical activity : official journal of the European Group for Research ir

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Cirugia Y Cirujanos
Contemporary Clinical Trials
Journal of aging and physical activity
American Journal of Gastroenterology
Tropical medicine & international health : TM & IH

AIDS Research & Therapy
European Neuropsychopharmacology
Jama Internal Medicine
Blood pressure monitoring
Archives of gynecology and obstetrics
Https://clinicaltrials.gov/show/nct03644680

Case Reports in Perinatal Medicine
Journal of biomechanics
Plos One
Circulation
American Journal of Hypertension
Journal of Substance Abuse Treatment
Experimental physiology
Seminars in thoracic and cardiovascular surgery
Journal of General Internal Medicine
PloS one
Human reproduction (Oxford, England)
Cochrane Database of Systematic Reviews
Australian & New Zealand Continence Journal
Asia Pacific journal of clinical nutrition

Physiotherapy (United Kingdom)
International orthopaedics
Zhongguo xiu fu chong jian wai ke za zhi = Zhongguo xiufu chongjian waike zazhi = Chinese journal of
Chinese journal of traumatology = Zhonghua chuang shang za zhi
Surgical Neurology
Journal of Epidemiology & Community Health
Disaster medicine and public health preparedness
Minnesota medicine
Gait & Posture
International Journal of Injury Control and Safety Promotion
Thorax
Journal of Clinical Neuroscience
Ethnicity & Health
Public Health
Canadian Journal of Surgery
Revue d'Epidemiologie et de Sante Publique
Journal of trauma management & outcomes
Chinese Journal of Traumatology

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Ulusal travma ve acil cerrahi dergisi = Turkish journal of trauma & emergency surgery : TJTES
Injury
International Journal of Injury Control & Safety Promotion
Promet-Traffic & Transportation
Revue de chirurgie orthopedique et reparatrice de l'appareil moteur
BMC Ophthalmology
Pediatrics
Archives of Physical Medicine & Rehabilitation
Children-Basel
Macedonian Journal of Medical Sciences
New Zealand Medical Journal
European Journal of Pediatric Surgery
Cirug√≠a pedi√°trica : organo oficial de la Sociedad Espa√±ola de Cirug√≠a Pedi√°trica
Acta Neurochirurgica
Archives of Disease in Childhood
Injury Prevention (1353-8047)
The Tohoku journal of experimental medicine
Preventive Medicine Reports
Archives de Pediatrie
Injury
Central European journal of public health
Journal of Neurotrauma
Indian Journal of Pediatrics
Foreign Affairs
Mobile Networks for Biometric Data Analysis
Atherosclerosis
The picture of health: Medical ethics and the movies.
Osteoporosis International
Prehospital emergency care : official journal of the National Association of EMS Physicians and the Na
Pharmacoepidemiology and Drug Safety
International journal of gynecological pathology : official journal of the International Society of Gynec
Evidence-Based Practice
PLoS biology
Sensors (Basel, Switzerland)
Journal of biomechanics
Multisensory Research
AJR. American journal of roentgenology
PLoS ONE
Gerontologist
African journal of medicine and medical sciences
Sleep Medicine Reviews
Scandinavian journal of trauma, resuscitation and emergency medicine
Injury Prevention (1353-8047)
Pharmacoepidemiology and Drug Safety (PDS)
American Journal of Forensic Medicine and Pathology
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Injury
World journal of surgery
Osteoporosis International
Acta anaesthesiologica Scandinavica
British journal of neurosurgery
Tropical Doctor
Bulletin of the World Health Organization
Respirology (Carlton, Vic.)
Zhongguo xiu fu chong jian wai ke za zhi = Zhongguo xiufu chongjian waike zazhi = Chinese journal of
Scandinavian Journal of Public Health
Medicinski arhiv

Journal of tropical pediatrics
Annals of surgery
Sleep
European journal of trauma and emergency surgery : official publication of the European Trauma Soc
Zhongguo xiu fu chong jian wai ke za zhi = Zhongguo xiufu chongjian waike zazhi = Chinese journal of
Archives of Pediatrics and Adolescent Medicine
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BMJ open
International Orthopaedics
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Tanzania Journal of Health Research
Injury-International Journal of the Care of the Injured
Surgery
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Child Abuse & Neglect

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Diagnostics
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Medicine
Archives of Physical Medicine and Rehabilitation
Parkinsonism and Related Disorders
Journal of medical Internet research
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Journal of Research in Childhood Education
Journal of the American College of Nutrition

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Einstein (S√£o Paulo, Brazil)
Pediatric blood & cancer
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Neurological Surgery

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BMC Public Health

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Early Childhood Research Quarterly
European Child and Adolescent Psychiatry
Journal of Pain
Medicine
Cochrane Database of Systematic Reviews
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Journal of General Internal Medicine
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Journal of Sleep Research
Child Abuse & Neglect
Archives of Pediatrics and Adolescent Medicine
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Journal of Epidemiology & Community Health
Journal of Nutrition
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Pediatric Surgery International
Emergency (Tehran, Iran)

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Nephrology, dialysis, transplantation : official publication of the European Dialysis and Transplant Ass
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Journal of the American Geriatrics Society
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Human-Computer Interaction - Interact 2013, Pt Iv
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Revista Panamericana de Salud Publica
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Ciencia e Saude Coletiva
BMC geriatrics
Journal of Neuroscience Nursing
Revista Eletronica de Enfermagem
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Maturitas
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Revista Brasileira de Enfermagem
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Archives of surgery (Chicago, Ill. : 1960)
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Anales de pediatria (Barcelona, Spain : 2003)
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Klinische Padiatrie
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Zhonghua yi xue za zhi
Psychological Science
Journal of Trauma & Acute Care Surgery
Paediatr. Respir. Rev.
Acta Clinica Croatica
Journal of Clinical Sleep Medicine

Israel Medical Association Journal
Acta Medica Iranica
Annals of the Royal College of Surgeons of England
Burns Open
Arthroscopy - Journal of Arthroscopic and Related Surgery
Preventive medicine
BMC cancer
Journal of Trauma
Indian Journal of Pediatrics
Academic Emergency Medicine
Diabetologia
BioMed research international
JAMA: Journal of the American Medical Association

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Canadian Journal of Public Health-Revue Canadienne De Sante Publique
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Nitric oxide : biology and chemistry
International journal of cardiology
Acta Odontologica Latinoamericana: AOL
HIV Medicine
Journal of clinical virology : the official publication of the Pan American Society for Clinical Virology
AIDS research and therapy
European Geriatric Medicine
Tohoku journal of experimental medicine
Journal of Korean Academy of Nursing
BMC Health Services Research
Stroke
Blood pressure monitoring

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Journal of neurology, neurosurgery, and psychiatry
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Journal of Aging & Physical Activity
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Science of the Total Environment
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Physiotherapy (United Kingdom)
Age & Ageing
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Movement Disorders
Clinical Interventions in Aging
Journal of the American Geriatrics Society

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Journal of the American Geriatrics Society
BMC geriatrics
International Journal of Geriatric Psychiatry
Age & Ageing
Seishin shinkeigaku zasshi = Psychiatria et neurologia Japonica
Seminars in hematology
Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and
OTJR Occupation, Participation and Health
Journal of the American Geriatrics Society
Journal of Investigative Medicine
Angiology
Maturitas

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BMC Geriatrics
Journal of Orthopaedic & Sports Physical Therapy
The Gerontologist
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Physiotherapy (United Kingdom)
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Kidney international
European journal of applied physiology
Journal of the American Geriatrics Society
Japanese Journal of Geriatrics
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American Journal of Physical Medicine & Rehabilitation
Geriatrics & gerontology international
Journal of the American College of Cardiology
2014 Ieee International Conference on Fuzzy Systems
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Revista Enfermagem UERJ
Calcified Tissue International
Journal of Bone and Mineral Research

Revista Da Rede De Enfermagem Do Nordeste
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Experimental Aging Research
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Journal of Geriatric Physical Therapy
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Journal of Biosocial Science
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Annals of Emergency Medicine
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Zeitschrift f√ºr Gerontologie und Geriatrie
Zeitschrift fur Gerontologie und Geriatrie
Sleep

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Brain Injury
Physical Therapy
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Maturitas
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Turkish Journal of Geriatrics-Turk Geriatri Dergisi
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HIV medicine
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Journal of the American Academy of Nurse Practitioners
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Journal of Affective Disorders
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Pediatric Surgery International
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Cephalalgia
Annals of Neurology
Archives De Pediatrie
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Journal of Family Studies
Journal of Bone & Joint Surgery, American Volume
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Cochrane Database of Systematic Reviews
Early Childhood Research Quarterly
The Lancet
American journal of epidemiology
The Cochrane database of systematic reviews
Journal of Intellectual Disability Research
Journal of Developmental and Behavioral Pediatrics
Bulletin of the World Health Organization
Injury Prevention (1353-8047)
Gesundheitswesen
Cochrane Database of Systematic Reviews
Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention
JAMA pediatrics
Journal of Medical Screening
Pediatrics
Physiology & Behavior
Journal of Medical Internet Research
Erciyes Medical Journal
Paediatric and Perinatal Epidemiology
Pediatric emergency care
Appetite
European Respiratory Journal
Journal of Urban Health
General hospital psychiatry
Journal of Women's Health
Journal of the american dietetic association
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Ambulatory pediatrics
Vaccine
Social science & medicine (1982)
Pediatrics
Journal of experimental child psychology
Journal of epidemiology and community health

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Academic Emergency Medicine
Archives of Disease in Childhood

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Social Science & Medicine
Childs Nervous System
Journal of Asthma
Clinical Radiology
Transactions of the American Clinical and Climatological Association

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Journal of public health policy
Journal of the American Academy of Child and Adolescent Psychiatry
European Psychiatry
International Journal of Epidemiology
Injury Prevention (1353-8047)
Legal Medicine
Epidemiology (Cambridge, Mass.)
Children and Youth Services Review
Injury
Developmental Medicine and Child Neurology
BMC Geriatrics
Preventing Chronic Disease: Public Health Research, Practice, and Policy
Injury

Recent patents on drug delivery & formulation
Regional Anesthesia and Pain Medicine
Pediatrics
Early Childhood Research Quarterly
Pediatrics
Salud p√ʃblica de M√©xico
Pediatric Rehabilitation

Child: Care, Health and Development
Journal of epidemiology and community health

BMC Public Health
South African Journal of Psychology
PsyCh Journal
Annals of surgical oncology
Gynecologic Oncology
European journal of surgical oncology : the journal of the European Society of Surgical Oncology and
The Journal of clinical endocrinology and metabolism

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Menopause
Cancer
Oman Medical Journal
Pediatric critical care medicine : a journal of the Society of Critical Care Medicine and the World Fede
Pediatric cardiology
Journal of Urban Health-Bulletin of the New York Academy of Medicine
Chinese Journal of Traumatology
Cochrane Database of Systematic Reviews
Gynecologic oncology
Gynecologic oncology
Journal of the Chinese Medical Association : JCMA
Revista brasileira de fisioterapia (Sao Carlos (Sao Paulo, Brazil))
PsyCh Journal
Asia-Pacific Journal of Public Health
Maternal and Child Health Journal

Early Childhood Research Quarterly
American Educational Research Journal
British journal of obstetrics and gynaecology

Journal of Agromedicine
Injury Prevention
Archives of pediatrics & adolescent medicine
Contemporary clinical dentistry
JAMA ophthalmology
Paediatric and Perinatal Epidemiology
School Psychology Review

Adolescence in the 21st century: Constants and challenges.
Child Abuse & Neglect
Annals of Allergy, Asthma & Immunology

Presse medicale (Paris, France : 1983)

Prevention Science
Archives de Pediatrie
Psychology, Health & Medicine
Smart Health, Icsh 2015
Psychiatry research
Child Maltreatment
Advanced Topics in Optoelectronics, Microelectronics, and Nanotechnologies Viii
Child Abuse & Neglect
PLoS ONE
Topics in Early Childhood Special Education

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Child: Care, Health & Development
British Journal of General Practice
British Journal of General Practice

Emergency Medicine Journal
Journal of the American Academy of Child & Adolescent Psychiatry

Environmental Health Perspectives
Teachers College Record
What social workers need to know: A psychoanalytic approach.
Journal of Speech & Hearing Research

Neurotoxicology and teratology
Thorax
Thorax

Archives of disease in childhood
Developmental Medicine and Child Neurology
Educational and Child Psychology
European Geriatric Medicine
Injury Prevention (1353-8047)
Thorax
Journal of Reading Behavior
JAMA Pediatrics
Journal of neurology, neurosurgery, and psychiatry
Implementation science : IS
The Journal of school health

International Journal of Injury Control & Safety Promotion
Australian occupational therapy journal
Demography
Israel Medical Association Journal
Bmc Public Health

Allergy
Early Education and Development
Frontiers in Psychology
Acta paediatrica (Oslo, Norway : 1992)
Annals of Global Health
Journal of the Academy of Nutrition and Dietetics

Gynecologic oncology
Scientific reports
International journal of experimental pathology

For peer review only

Cancer
The Cochrane database of systematic reviews
Medicine
Gynecologic oncology
Gynecologic oncology
The New England journal of medicine
International journal of gynecological cancer : official journal of the International Gynecological Canc
American journal of obstetrics and gynecology
Gynecologic oncology
Clinical cancer research : an official journal of the American Association for Cancer Research
Anticancer research
Journal of experimental & clinical cancer research : CR
Gynecologic oncology
Journal of assisted reproduction and genetics
Histopathology
Modern pathology : an official journal of the United States and Canadian Academy of Pathology, Inc
Gynecologic oncology
Journal of Minimally Invasive Gynecology
European journal of gynaecological oncology
Applied immunohistochemistry & molecular morphology : AIMM
Diagnosis
Journal of the Medical Association of Thailand = Chotmaihet thangphaet
Journal of the Medical Association of Thailand = Chotmaihet thangphaet
Journal of the Medical Association of Thailand
Archives of gerontology and geriatrics
European Geriatric Medicine
Geriatrics & Gerontology International
Https://clinicaltrials.gov/show/nct01698047
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The journal of obstetrics and gynaecology research
Familial cancer
Korean journal of radiology
The journal of obstetrics and gynaecology research
American Family Physician
International journal of gynecological pathology : official journal of the International Society of Gynec
Cancer
Folia morphologica
Archives of gynecology and obstetrics
International journal of gynecological cancer : official journal of the International Gynecological Canc
BJOG : an international journal of obstetrics and gynaecology
Oncogene
European Journal of Obstetrics & Gynecology and Reproductive Biology
Gynecologic oncology
The American journal of surgical pathology

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American journal of obstetrics and gynecology
Gegenbaurs morphologisches Jahrbuch
Modern pathology : an official journal of the United States and Canadian Academy of Pathology, Inc
Japanese journal of clinical oncology
International journal of gynecological cancer : official journal of the International Gynecological Cance
Gynecologic and obstetric investigation
European journal of cancer (Oxford, England : 1990)
Modern pathology : an official journal of the United States and Canadian Academy of Pathology, Inc
Tumour biology : the journal of the International Society for Oncodevelopmental Biology and Medicir
Clinical endocrinology
BioMed research international
International journal of gynecological cancer : official journal of the International Gynecological Cance
PloS one
Gynecologic oncology
The journal of obstetrics and gynaecology research
Fertility and Sterility
BMC cancer
Journal of cancer research and clinical oncology
Diagnostic cytopathology
Pathology
American journal of obstetrics and gynecology
International journal of cancer
Annals of surgical oncology
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Gynecologic oncology
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APMIS : acta pathologica, microbiologica, et immunologica Scandinavica
Fertility and Sterility
Gynecologic Oncology
Cancer chemotherapy and pharmacology
Gynecologic oncology
The Central African journal of medicine
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Physiotherapy Theory & Practice
Biomedical Research (India)
Gerontology
Disability & Rehabilitation
Parkinsonism & related disorders
Age and ageing
Movement Disorders
International journal of epidemiology
Arkhiv patologii
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Geriatrics and Gerontology International
Aging clinical and experimental research
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Obesity surgery
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Revista Espa√±ola de Geriatr√≠a y Gerontolog√≠a
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Nutrients
Journal of the American Medical Directors Association
European Geriatric Medicine
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Brain : a journal of neurology
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Physiotherapy
Bmc Health Services Research
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Alzheimer's and Dementia
Topics in Geriatric Rehabilitation
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Nose2014: 4th International Conference on Environmental Odour Monitoring and Control
Zentralblatt fur Chirurgie
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Nursing Research
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Gerontology
Journal of Alzheimer's disease : JAD
Spinal cord
European Journal of Orthopaedic Surgery and Traumatology
European Geriatric Medicine
Kidney international
BMC Health Services Research
Journal of Neurologic Physical Therapy
BMC Psychiatry
Urologic oncology
Harvard review of psychiatry

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Journal of aging research
Revista de neurologia
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Journal of Geriatric Physical Therapy
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Journal of diabetes and its complications
Archives of disease in childhood
Injury prevention
Injury
American Journal of Geriatric Psychiatry
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Gynecologic oncology
International journal of gynecological cancer : official journal of the International Gynecological Canc
International journal of gynecological cancer : official journal of the International Gynecological Canc
Clinical cancer research : an official journal of the American Association for Cancer Research
Psychosomatic Medicine
The Lancet. Oncology
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BMC musculoskeletal disorders
Journal of obstetrics and gynaecology Canada : JOGC = Journal d'obstetrique et gynecologie du Canac
Familial cancer
International journal of gynaecology and obstetrics: the official organ of the International Federation
JAMA Neurology
Gynecologic oncology
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Journal of gynecologic oncology
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The Cochrane database of systematic reviews
International journal of clinical oncology
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The international journal of behavioral nutrition and physical activity
Journal of Paediatrics and Child Health
Pediatrics
Movement Disorders
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Journal of neurophysiology
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Consultant Pharmacist

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Plos one
Malawi medical journal : the journal of Medical Association of Malawi
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Irish Journal of Medical Science
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Consultant Pharmacist
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Colombia medica (Cali, Colombia)
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Pm&R
Metabolism: clinical and experimental
Medical Decision Making
Archives of physical medicine and rehabilitation
Menopause-the Journal of the North American Menopause Society
PM and R
Global spine journal

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European Journal of Human Movement
Experimental Gerontology
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Influences on the Decision to Place a Son Or Daughter with Learning Disability & Challenging Behavio
Upsala Journal of Medical Sciences
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Prevention Science
Osteoporosis International
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Occupational and environmental medicine
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Turkiye Klinikleri Journal of Medical Sciences
Calcified Tissue International
Gynecologic oncology
Archives of gynecology and obstetrics

TAF Preventive Medicine Bulletin
ESC heart failure
Experimental aging research
Pediatric pulmonology
Journal of the American Geriatrics Society
Osteoporosis International
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The Journal of physiology
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Archives of surgery (Chicago, Ill. : 1960)
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BMJ Open
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Clinical Rehabilitation
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Dental Traumatology
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Drug, healthcare and patient safety
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International Journal of Nursing Studies
Journal of Nutrition, Health and Aging
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Injury prevention : journal of the International Society for Child and Adolescent Injury Prevention

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Neurocritical care
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British Journal of Occupational Therapy

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14	9	e0221875
21	8	1113-1121
19	1	52
322	10	946-956
16	6	e1002843
67		S301
126	3	389-409

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75		11	3078-3087
33		4	681-692
175			90-100
321		21	2092-2100
67		9	1812-1819
42		3	123-129
2		2	e188280-e188280
34		12	1765-1775
33		3	439-449
80			No Pagination Specified-No Pagination Specified
17		3	390-413
74		9	1511-1517
19		4	361,Äê369
82			259-265
13		4	
67		6	1211-1217
26			S125,Äê
5			106
		7	76
29		11	2545,Äê2556
13		5	652-665
38			S228-S230
42		4	230-242
7		9	
34		2	S471
35		8	e00115718
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15	Suppl 1		S144,Äê5
18		1	173
63		2	94,Äê100
48		4	506-512
8			
8			
19		1	1-Oct
7			19

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18	1	183
48	4	526-532
13	45	ix,Äê158
17	1	N.PAG-N.PAG
9	7	
36	3	247-267
	3	
22		S101
5		89
48	2	185-195
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23	27	1-+
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	3	
63	1	40,Äê44
9	9	e029682
30	6	985,Äê986
31	5	325-330
38	6	791-804
13	2	129-133
74	2	269-273
42	4	275-280

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30		S72
7		38670-38687
19	1	1-Aug
21	2	249-252
63	1	51-+
85		103913
69		
27	3	10
125		e665-e670
8	3	232-238
23		
		13
14		721-726
34	3	464-474
31	2	133-139
20	10	1287-1293
128		43-48
		e12294
42	4	E85-E90
25	5	453-458
63	6	624-629
9	6	24
20		1587-1591
19	3	9
28	2	E35-E36
42		A402,Äê
21	4	269-274
2019		2370796
38	7	983-998
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18	1	
9		
20	1	N.PAG-N.PAG
19	9	854-860
6	6	1144-1156
100	10	1894-1906
23	12	1684-1690
60	2	140-155
19	1	1-Oct
58		iii105-iii106

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39		11	2541-2559
12		11	
24		1	67-75
15		7	P1480
50		6	419-426
20		1	
10		2	303-312
19		1	238
16		8	
67		2	246-253
17	Suppl 1		S122,Äê3
48			1-Jul
16		1	71
			5
24		10	3981-3990
14		1	
17		2	1-Sep
19		1	792
81		1	110-117
17		6	1154-1228
23		2	427-432
44		2	65,Äê73
67		10	2102-2107
31		3	379-396
			1-Sep
			1,Äê9
21		1	
184		5	2030,Äê2031
67		8	1656-1661
9		10	
10		6	853-862
19			S63,ÄêS64
32			S98,Äê
62			145-159
20		5	569-582.e7
20		7	835-835
52		1	45-60
50			
139			
20		5	530-+
39		8	816-826
14		1	60
67		8	1571-1581

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45	4	357-371
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14		1045-1064
9	2	9
38		S347,ÄêS348
212		106122
42	4	209-215
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42	3	E87-E93
26	10	14
139	1	107-112
2019		
48	2	120-126
23	8	942-951
64	3	307-319
100	12	e213
14	4	N.PAG-N.PAG
19	1	249,Äê
99		124-133
21	10	12
9	6	
76		44-50
31	10	269-271

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EUA). A compa ajustado pel sexo

13	2	446-460
26		S237-S238
14	5	e0215764
16	10	15

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48	2	213-219
28	12	3187-3195
8	1	65-73
35	1	143-151
31	4	511-518
30	7	1363-1370
129	3	536,Äê543
20	1	317
20	10	1268-1273
3		36
42	4	224-229
7	1	12
130		110785
20	3	377-381
14	9	1288-1301
27	8	16-20
92	15	
27	2	166-183
48		
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16	5	Jan-18
9		143-157
71	7	865-874
22	2	59-64
105	4	392-402
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39	1	276-281
5	1	S34,ÄêS35
		109574
43	8	N.PAG-N.PAG
29	7	3772-3781
52	1	15,Äê23
2019		9716919
92	15	
67		S141
36	3	512-520
14		1751-1760
66		157-163
39	4	247-256
67	6	1145-1151
7		225
86		13

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	100	5	859-864
	42	3	136-147
	65	1	1-Jul
2¬±60	2 y 263		2¬±116
totalizando 16	2¬±60		2 e 263
	85	9	2134-2142
	237	3	769-776
	123		e740-e746
	42	3	E116-e121
	22		13583-13590
	21		249
	105	5	985-990
	14	6	566-573
	33	1	120-127
	9		S32-S37
	19	11	
	39	3	197-204
	7		152399-152411
	104		A158-A159
	22		96-106
	51	7	2330-2333
	13	3	1.55799E+15
	32	Suppl 1	7
	16	12	15
	19	1	177
	283		107-111
	19	10	1386-1394
	43	2	195-203
	87	3	672-677
	13	3	31-34
	20		11
	11		17

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38	7	1932-1943
31	8	1057-1067
42	3	234-239
31	2	277-286
38		S289-S291
42	3	148-152
12	2	5,Äê8
21	3	124-133
	4	
9	6	
69	1	103-111

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	18	11	998-1008
	27	3	S32-S33
	9	3	e022927
	20	1	73-88
	137	4	369-378
	100	4	695-702
	32	4	497-506
	48		iii1-iii16
	48		
	74	4	500-506
	4	2	10
	98	1	37-43
	21		S5-S6
	179	3	394-405
	20	1	89

en las dimensic001). La depe 3% de los ancianos
nas dimens√µe nas duas ava 001). A depend√™ncia reduziu de 97

en las dimensic001). La depe 3% de los ancianos
nas dimens√µe nas duas ava 001). A depend√™ncia reduziu de 97

	2019		1959486
			No Pagination Specified-No Pagination Specified
	10		13
	40	5	1009-1016
	83	7	352
	67		S21-S22
	19		16
	16	3	12
	85	7	752-756
	48		iii17-iii65
	48		
	75	1	181-188
	10	1	177-187
			1-May
	31	7	1235-1258
	19	1	22-22
	15	7	P1245
	120		7-Nov
	19	1	1-Oct
	19		10
	19	1	188,Äê
	86	3	164-171
	38	6	805-824
	84	7	1598,Äê1610

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11	7	15	
	2		
144	2	252e-263e	
201	4	e141	
38	5	1399-1408	
66		48-52	
34	2	S441-S442	
47	2	331-335	
		No Pagination Specified-No Pagination Specified	
19	1	10	
16	3		
38	6	775-790	
24	8	2845-2858	
67		S331-S332	
53	4	528-545	
122		53-59	
296		80-84	
27	3	642-653	
96		27-34	
19	1	Jan-13	
16	20		
4	2	65-73	
43			
16		S8	
40	4	255-267	
138	3	189-194, 271	
182		S269	
79		No Pagination Specified-No Pagination Specified	
36	6	709-716	
42	3	346-352	
33	1	21-25	
64	8	896-901	
23	2	200-204	
13		S67-S67	

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17		6	490-496
37		6	634-639
2017			3810-3813
72			53
101			eS958
257		9	1543-1551
39		12	1194-200
42		4	199-203
		111	45-48
22		8	10-Dec
4		3	40-52
42		11	7-Jul
8		4	10-Oct
16			
27		10	
49		9	1411-1420
59		1	148-157
27		6	684-694
19		2	154-163
20		8	39-48
81			122-130
910			63-70
45		5	259-266
45		5	259-266
29		5	678-88
63		5	549-554
57		10	M691-M694
12		4	317-322
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29		7	565-576
157		1	5-Oct
76			No Pagination Specified-No Pagination Specified
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131		2	331-339
56		12	2858-2867
12	Suppl 1		1-Jan
84			32-7
127		2	186-93
17		3	463,Äê470
45		10	1069-1076
			330-335
9			131,Äê140
9			131-40
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57	8	444-51
62		S60
5		11364-11373
171	18	1655-61
7	1	2924
34	8	778-781
64	6	1281-6
22	3	275-283
58	5	398-406
67	10	1085-1091
33	7	1177-1185
8	9	e75513
10	1	51-5
71		293
186	2	557
26	1	25-34
104		208-217
11	2	107-114
18	1	84-85
19	5	415-429
6		e4326
38	2	145-52
34	2	88-94
68		5177-5177
46	3	367-376
49	5	685-696
17	6	898-904
11	1-Feb	235-239
55		116-120
95	12	570-4
37	2	62-67
43	11	582-588
17	10	1387-93
46		iii13
27	2	151-157
25		37-48
33	10	587
35	4	e276-297
25	1	Mar-16
24	2	103-111
5597		209-+
33	2	200-209
17	1	140
34	7	369-375
13	4	511-526
28	8	927-932

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81		9	879-83
44		2	231-237
15		9	22451-22472
65		2	98-103
356	1-Feb		113-7
34		1	111-122
37		9	28-24
27		1	25-32
12		1	25-32
28		2	55-62
40		1	47,Äê52
44		4	567-571
60		5	377-82
45		12	1729-33
147		5	931-8
53		1	40-7
4		1	
32		10	1999-2004; discussion 2004
11		3	187-195
27			S301-S302
24		10	742
52		5	498-510
6			S184
19		1	148-166
35		2	141-148
93			284-290
132		12	e817-e826
51		6	1303-1313
14		1	53

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5		2.05436E+15
13	8	1010-1014
5	4	
42	1	17-23
46	5	650-660
34	6	614-619
23	3	158-163
		1087-1092
30	1	93-8
13	1	40-40
19	2	130-+
110		386-391
31		
19	3	277.e1-277.e8
24	1	33-42
80	4	359-69
39	6	776-780
47	16	3876-3881
46	14	2463-2468
45	7	1259-1264
6		45021-45033
27	2	329-332
9	3	129-133
37	5	702-707
42	5	38-46
39	3	110-6
6	3	202-211
16	3	746-54
42	7	320-327
57	5	511-517
57	2	143-150
26	4	751-766
124	5	912-931
12	5	561-570
27		S766-S767
73	1	102-108
121	4	597-602

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	36		3	541-545
53A			4	M264-74
	29		4	859-870
	19		3	325-327
	19		4	465-467
	43		4	1480-1487
	32		16	1354-1363
				1-Oct
	73			No Pagination Specified-No Pagination Specified
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	70		3	174-182
	43	suppl_1		i9-i9
	73			No Pagination Specified-No Pagination Specified
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	8		4	256-262
	16		5	412-419
	15		9	630-634
	123		8	549-556
	14		2	91-95
	12		6	e0179895
	9		1	1-Jul
	20			93
	39		11	S280
	22		4	410-414
	16			S16
	28		3	137-141
	55		3	653-9
	102		2	204-209
	28		1	30-Jun
	47		1	80-86
	51		9	1353-1364
	62		8	1534-9
	61		5	776-81
	6			S7
	39		2	337-343
64A			2	249-255
	45		2	172-179
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	107		2	247-53
	24		10	752
	62		6	545-557; discussion 558

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31	6	295-302
19	2	49-70
154		19-27
9	8	865-72
25	2	215-227
17		Jan-13
118	7	1277-1290.e4
95	2	253-61
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96	7	1210-1218
105		43
3	1	Sep-15
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226	3	288-92; discussion 292-3
73		No Pagination Specified-No Pagination Specified
91	1	106-114
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20		1151-1154
82		13-19
67		S180-S186
12	2	552-560
168	4	4103-10
35	3	200-204
16	6	539-44
44	3	260-4
50	3	143-147
26	8	2101-9
304	6822	282-283
44	7	796-803
28	4	295-304
70		1751-1751
48		632-632
20	6	585-592
15	2	Jan-13
50	8	1089-98
55	3	690-5
34	6	567-71
38	1	33-41
17	8	899-906
17	8	899-906
92	6	730-735
18	3	271-277
39	2	170-177

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19		10	26-28
18		10	905-911
66			S6
50		1	136-145
130		3	353-6
7		3	187-197
161	19-20		469-76
2			29-36
110		10	614-9
126			116-146
28		2	539-547
38		2	198-202
29		2	271-83
8		8	754-60
24		6	1899-908
128		11	541-4
6		3	55-60
32		12	2003-2018
1			S13
44		4	217-221
62			475-479
217			857-864
18		5	628-641
7		5	585-592
67			200-206
18		1	266
20		3	177-188
67		2	84-89
23		1	Sep-13
51		1	59-64
2		2	47-54
18	3 Suppl		179s-183s
9		3	306-314
102		4	397-404
102		3	325-8
63		2	315-20
23		4	307-318
80		12	735-744
31		1	168-172
6			25420
15		10	14-16
8		5	209-211
37		3	
10		11	e0142123
120		6	990-9

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16		3	189-193
43		6	851-6
65			S219
3		2	107-114
41		12	1169-1177
34		2	99-116
52		3	335-339
59		1	17-20
32		3	338-42
54		10	654-7
20		3	306-15
20		3	326-332
73			No Pagination Specified-No Pagination Specified
19		2	145-159
89		2	203-9
11		1	17-20
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76		9	994-1002
53		6	M457-M464
90		6	1018-1025
52		4	S43-S43
27		6	e12714
58		7	1398-1399
106		3	295-299
59			S203
98		3	191-8
99		10	673-82
55		9	855-62
58		5	407-412
179		2	73-84
36		4	146-154
36		4	146-154
16		6	e159
62		603	e696-e702
320		7237	754-758
5	MAR		
44			161-167
10		7	e0134350
47		12	2925-2932
10		1	
6			S179
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	7		S217-S218
	32	6	1007-1011
	4	5	1182-92
	57		e152
			60-65
	67		A394
	63		S157-S158
	24	6	828-833
	137	11	1348-59
	35	5	969
	3		S120
	137		
	18	3	212-220
	2		468-475
	55		696-696
l counseling psychology.			555-568
	11		4587-4591
	24	8	1627-32
	12	1 Pt 1	35-9
	96	6	442-5
	85	12	2002-2010
	30	1	93-101
	27		
	40	9	973-977
	247	2-Mar	253-261
	13	1	1-Aug
	106	2	192-195
	24	3	459-64
	27	4	950-3
	8	5	214-220
	8	6	256-262
	17	12	1191-5
	21		216
	54	3	429-33
	16	3	260-265
	130	12	1533-8
	70		981-981
	16	1	97-105
	23	5-Jun	400-405
	48	12	1626-1632
	80	6	686-91
	24	2	152-156
	40	1-Feb	13-21
	35	2	282-6
	24	4	361-4

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	78	1	62-6
	16	12	
	113		26-31
		Ph.D.	249 p-249 p
	24	1	42-49
	30	2	142-9
	9	11	e111949
	65	5	1019-1025
	41	1	Oct-16
	23	5-Jun	364-371
	43	6	744-747
	71	12	761-769
	43	9	2874-81
	40	5	865-77
	206		Jan-13
	68		4599-4599
	48	2	123-30
	15	90	337-345
	71	1	138-144
	91	5	750-758
	58	9	1325-1330
	2	1	44-49
	58	6	612-621
	63	1	26-33
	9		1637-1645
	28	6	580-585
	39	12	1207-15
	22	7	501-8
	12	3	200-204
	14		
	39	23	2420-2427
	30	7	957-965
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	22	1	19-26
	33	12	1633-1640
	17	1	26-33
	33	2	152-162
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	70		1751-1751
	12	4	680-687
	4	4	563-571

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20	2	326-336
33	1	150-5
2	4	
16	3	219-25
12	4	e0174710
63		S14
94	7	1312-9
32	3	188-194
14	12	911-5
40	4	516-9
		1176-1182
183	11	1037-1044
50	3	457-462
58	4	759-767
95	6	1060-6
27	2	463-71
26	1	S257
185		
7	1	Jan-14
29	5	623-634
26		330
22	3	204-210
27	5	423-9
68		1121-1121
1	4	317-323
26	3	3-Aug
15	4	523-541
10	2	145-9
29	4	669-687
40	4	385-387
19	12	1357-1365
26	8	2157-64
36	9	1982-1997
		933-936
30	3	223-229
26	2	285-95
232	2	423-434
55	2	162-8
90	5	786-792
40	1	175-80
11	7	492-7
9	5	739-753
32	1	1-Sep

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17		3	334-40
24		1-Feb	13-28
31		6	1226-33
255		1	59-67
24		3	261-274
24		3	261-274
32		3	378-83
162	1 Pt 1		Sep-14
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DOI

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del √°rea de la salud a fi n de establecer medidas preventivas.

15 vezes maior para o grupo de idosos com medo de cair. Conclus√£o: O medo de cair foi identifi cad

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onde o evento foi mais relatado nas resid√™ncias e relacionou-se com v√°rios aspectos de sa√∫de."

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POSSIBLY RELEVANT STUDIES IN OTHER LANGUAG

Title

Fall Prediction Model for Community-dwelling Elders based on Gender

Fall risk and fracture. Aging and fall/fracture

Fall risk assessment in regular exercising elderly women

Risk profiles and preventive measures of falls in elderly persons

Fall-related factors in a cohort of elderly community residents

Identifying the elderly at risk for falling and accompanying protocols

Impact of fall risk and fear of falling on mobility of independently living senior citizens transitioning to

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)F TITLES AND ABSTRACTS:

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Rodriguez Perracini, M.; Ramos, L. R.

Galinsky, D.; Fried, V.; Biderman, A.; Cwikel, J.; Ben Moshe, Y.

Anders, J.; Dapp, U.; Laub, S.; Von Renteln-Kruse, W.

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2000	Hebrew	Harefuah. 2000;138(3):189-271.
2007	German	Z Gerontol Geriatr. 2007;40(4):255-267. doi:10.1007/s00391-007-0473

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Title

Accuracy of clinical-functional tools to identify risk of falls among community-dwelling older adults
Do voluntary step reactions in dual task conditions have an added value over single task for fall prediction? A prospective s
Use of the BESTest and the Mini-BESTest for Fall Risk Prediction in Community-Dwelling Older Adults Between 60 and 102
Psychometric comparisons of the timed up and go, one-leg stand, functional reach, and Tinetti balance measures in comm
Comparison of 2 Frailty Indexes for Prediction of Falls, Disability, Fractures, and Death in Older Women
A comparison of frailty indexes for the prediction of falls, disability, fractures, and mortality in older men
Predicting falls in community dwelling older adults using the Activities-specific Balance Confidence Scale
Predicting falls in older adults using the four square step test
Fusion of Clinical, Self-Reported, and Multisensor Data for Predicting Falls
Using Machine Learning on Home Health Care Assessments to Predict Fall Risk...The 17th World Congress of Medical and H
Agreement and predictive power of six fall risk assessment methods in community-dwelling older adults
The Clinical Potential of Frailty Indicators on Identifying Recurrent Fallers in the Community: The Mr. Os and Ms. OS Cohor
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Reliability and validity of a steadiness score
Comparison of Fallers and Nonfallers on Four Physical Performance Tests: A Prospective Cohort Study of Community-Dwel
Special feature: the Home Assessment Profile -- a reliable and valid assessment tool
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22	5-Jun	360-366	
			10.1519/jpt.0000000000000236
52	8	1343-1348	10.1111/j.1532-5415.2004.52366.x
168	4	382-389	10.1001/archinternmed.2007.113
57	3	492-498	10.1111/j.1532-5415.2009.02137.x
72		142-145	10.1016/j.archger.2017.06.007
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			10.1109/jbhi.2019.2951230
264		684-688	10.3233/SHTI190310
87		103975	10.1016/j.archger.2019.103975
20	12	1605-1610	10.1016/j.jamda.2019.06.019
19	1		10.1186/s12877-019-1300-2
48		i16	10.1093/ageing/afy209.01
80			
81		51-5	10.1016/j.exger.2016.04.016
83		185-194	10.1016/j.archger.2019.02.018
2	1	e12114	10.2196/12114
			10.1016/j.rehab.2019.08.003
			10.1093/gerona/glz128
		1-Aug	10.1080/09638288.2019.1582721
66		117-123	10.1016/j.humov.2019.04.004
44	3	237-245	10.1080/17538157.2018.1496089
44	3	487-491	10.1007/s10900-019-00664-3
13	2	33-37	10.5958/0973-5674.2019.00041.8
14	11		10.1371/journal.pone.0225118
44	2	294-299	10.1093/ageing/afu151
18	9	774-779	10.1016/j.jamda.2017.03.021
58	5	472-80	10.1159/000337259
12		645-651	10.2147/CIA.S133523
6	FEB		10.3389/fnagi.2014.00022
10			10.3389/fnagi.2018.00044
10		7	10.1186/1743-0003-10-7
56	4	M253-M259	10.1093/gerona/56.4.M253
90	5	761-773	10.2522/ptj.20090069
18	7	1038-1048	10.1111/j.1365-2702.2008.02591.x
24	1		
13	12	1932-1939	
29	5	914-920	10.1589/jpts.29.914
10	12	e0146247	10.1371/journal.pone.0146247
17	10	1463-1469	10.1111/ggi.12893
2014		4531-4	10.1109/embc.2014.6944631
17	2	e41	10.2196/jmir.4064

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56	8	631-636	
13	5		10.1371/journal.pone.0197091
56	3	M146-M156	
57	12	2369-2371	10.1111/j.1532-5415.2009.02591.x
356	9234	1001-1002	
75	4	447-453	
37	4	159-165	10.1519/JPT.0b013e3182abe7cb
29	1	S502	10.1007/s00198-018-4465-1
32		S118-S119	
21		S80-S81	10.1016/j.jsams.2018.09.184
20	5	161-167	
30		189-201	10.3109/02703181.2012.694586
1040		63-71	10.1007/5584_2017_100
59		S183	10.1111/j.1532-5415.2011.03416.x
44	6	967-972	10.1093/ageing/afv144
11	7		10.1371/journal.pone.0158623
91	12	1823-1832	
43	suppl_2	ii18-ii18	
14	3	417-22	
68	2	92-8	
33	4	165-172	10.1097/JPT.0b013e3181ff23cc
62	4	358-367	
28	1	59-70	10.1007/s00198-016-3691-7
97		eS720-eS721	10.1016/j.physio.2011.04.002
6			10.1186/1477-5751-6-2
28	3	119-119	
57	9	1532-1539	10.1111/j.1532-5415.2009.02394.x
42	2	151-160	10.1080/0361073X.2016.1132881
143	11	1129-1136	
58	3	226-32	10.1016/j.jclinepi.2004.06.013
59		S58	10.1111/j.1532-5415.2011.03416.x
13			10.1186/s11556-016-0162-2
20	1	83-90	
33	4	368-373	
53	9	1582-1586	10.1111/j.1532-5415.2005.53485.x
12	1	22-26	10.1016/j.ijge.2017.04.006
16	3	77-88	
44	1	109-115	10.1093/ageing/afu054
58	1	205-206	10.1111/j.1532-5415.2009.02657.x
76	6	576-83	
26	12	2793-2802	10.1007/s00198-015-3194-y
126	2	176-178	
29	1	P 5-16	
55	1	M17-21	
30			10.1002/jbmr.2763
69		15-20	10.1016/j.archger.2016.09.008

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60	3	389-393	10.1016/j.archger.2015.02.009	
39	2	117-23	10.1159/000213521	
63	2	171-178	10.1093/gerona/63.2.171	
32	1	92-97	10.1016/j.gaitpost.2010.03.012	
98	10	e16-e17	10.1016/j.apmr.2017.08.050	
88	4	449-459	10.2522/ptj.20070251	
82	3	545-554		
59		15-18	10.1016/j.clinbiomech.2018.08.006	
57	8	M504-M510		
50	8	1329-1335	10.1046/j.1532-5415.2002.50352.x	
6	3	160-162	10.1016/j.ijge.2011.08.001	
25	4	427-432	10.1007/s40520-013-0061-6	
52	4	237-242		
8		S26-S28	10.1111/j.1447-0594.2007.00401.x	
37	4	430-435		
13	4		10.1177/1550147717703257	
55	5	532-8	10.1159/000236364	
72		4532-4532		
58		183-187	10.1016/j.gaitpost.2017.07.115	
56	10	M627-32		
49	5	508-515	10.1046/j.1532-5415.2001.49107.x	
28	4	111-120		
18	11	921-927	10.1016/j.jamda.2017.08.001	
16		S49	10.1016/S1353-8020(10)70170-X	
16		51	10.1186/s12877-016-0212-7	
25	6	645-650	10.1007/s40520-013-0159-x	
23	1	41-48		
11	1	28-39		
21	3	725-731	10.1109/JBHI.2016.2539098	
19	8	523-530	10.1016/j.annepidem.2009.03.012	
39	9	669-683	10.1080/03601277.2012.661338	
69		4457-4457		
97	10	e129		
18	5	402-408	10.1016/j.jamda.2016.11.003	
15	1	65-73	10.1023/A:1007531101765	
54	8	1169-1176	10.1111/j.1532-5415.2006.00813.x	
47	11	2769-2776	10.17576/jsm-2018-4711-19	
34	2	119-126		
15	6	386-391	10.1007/s12199-010-0154-1	
65A	8	896-903	gerona/glq067	
31	3	317-321	10.1016/j.gaitpost.2009.11.013	
42	3	297-302		
16	2	259-265	10.1111/ggi.12463	
2016		1520932	10.1155/2016/1520932	
25	3	311-6	10.1007/s40520-013-0042-9	
11	1	66-80		

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19	1	61-69	10.1590/bjpt-rbf.2014.0067
76			
42	10	1110-1117	
100	4	263-7	
65	1	31-9	10.3138/ptc.2011-17
12		2009-2016	10.2147/CIA.S138287
21	5	416-422	10.1089/rej.2017.2005
27		48-56	10.1016/j.ejim.2015.11.019
29	2	232-234	10.1589/jpts.29.232
89	9	1708-1713	
16	9	624-629	
90	4	550-560	10.2522/ptj.20090158
62	11	2595-2601	10.1109/tbme.2015.2464689

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Notes

Exclusion reason: Not multifactorial (i.e. 2 factors or more)
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Exclusion reason: Wrong patient population
Exclusion reason: Wrong outcomes
Exclusion reason: Wrong study design
Exclusion reason: Wrong study design
Exclusion reason: Wrong study design
Exclusion reason: No full-text paper published
Exclusion reason: Wrong study design
Exclusion reason: Wrong outcomes
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Exclusion reason: Wrong outcomes
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Exclusion reason: Wrong outcomes
Exclusion reason: Not a prediction model
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Exclusion reason: Wrong outcomes
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EXTRACTED DATA ON I

Publication year:	Author:
1994	Maki et al.
2000	Brauer et al.
2001	Tromp et al.
2002	Stalenhoef et al.
2003	Stel et al.
2003	Boulgarides et al.
2004	Nandy et al.
2006	Pluijm et al.
2006	Okochi et al.
2008	Lindemann et al.
2008	Lamb et al.
2010	Delbaere et al.
2011	Yamashita et al.
2011	Panzer et al.
2011	Bongue et al.
2011	Viccaro et al.
2012	Yamashita et al.
2013	Weiss et al.
2013	Hnizdo et al.
2013	de Vries et al.
2014	Muhaidat et al.
2015	Gadkaree et al
2015	Cawthon et al.
2016	Palumbo et al.
2017	Rodriguez-Molinero et al.
2017	Lohman et al.
2017	Kim et al.
2018	Palumbo et al.
2019	Singh et al.
2019	Gillain et al.

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NCLUDED STUDIES

Title:

A prospective study of postural balance and risk of falling in an ambulatory and independent elderly p
A Prospective Study of Laboratory and Clinical. Measures of Postural Stability to Predict Community-D
Fall-risk screening test: A prospective study on predictors for falls in community-dwelling elderly
A risk model for the prediction of recurrent falls in community-dwelling elderly: A prospective cohort
A classification tree for predicting recurrent falling in community-dwelling older persons
Use of Clinical and Impairment- Based Tests to Predict Falls by Community-Dwelling Older Adults
Development and preliminary examination of the predictive validity of the Falls Risk Assessment Tool
A risk profile for identifying community-dwelling elderly with a high risk of recurrent falling: results of
Simple screening test for risk of falls in the elderly
Maximum step length as a potential screening tool for falls in non-disabled older adults living in the c
The Optimal Sequence and Selection of Screening Test Items to Predict Fall Risk in Older Disabled Wo
A Multifactorial Approach to Understanding Fall Risk in Older People
Fall Risk Factors in Community-Dwelling Elderly Who Receive Medicaid-Supported Home- and Commu
Mobility Assessment: Sensitivity and Specificity of Measurement Sets in Older Adults
A screening tool with five risk factors was developed for fall-risk prediction in community-dwelling eld
Is timed up and go better than gait speed in predicting health, function, and falls in older adults?
Risk factors of falls in community-dwelling older adults: logistic regression tree analysis
Does the evaluation of gait quality during daily life provide insight into fall risk? A novel approach usir
Validity and reliability of the modified John Hopkins Fall Risk Assessment Tool for elderly patients in h
Does frailty predict increased risk of falls and fractures? A prospective population-based study
Validity of Simple Gait-Related Dual-Task Tests in Predicting Falls in Community-Dwelling Older Adults
Comparison of Simple Versus Performance-Based Fall Prediction Models: Data From the National Hea
An evaluation of the usefulness of consensus definitions of sarcopenia in older men: results from the
Predictive Performance of a Fall Risk Assessment Tool for Community-Dwelling Older People (FRAT-up
A two-question tool to assess the risk of repeated falls in the elderly
Operationalisation and validation of the Stopping Elderly Accidents, Deaths, and Injuries (STEADI) fall
Measuring Frailty in Medicare Data: Development and Validation of a Claims-Based Frailty Index
Simulating the effects of a clinical guidelines screening algorithm for fall risk in community dwelling o
Sociodemographic Characteristics as Determinants and Physical Performance Measures as Correlates
Using supervised learning machine algorithm to identify future fallers based T on gait patterns: A two

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Protocol registry:

No information.
No information
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Based on author response: "No protocol was preregistered)
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Based on author response, no protocol was preregistered. (Answer: "No, we didn't - wasn't a custom
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Author response: "No, we did not [register a protocol]. It is not mandatory for observational studies
No protocol was preregistered.
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No information. The paper refers to the protocol of the InCHIANTI follow-up study and not the specifi
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Registry number:
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Based on author response, no protocol was preregistered. (Answer: "No, we didn't - wasn't a custom
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Author response: "No, we did not [register a protocol]. It is not mandatory for observational studies
No protocol was preregistered.
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No information. The paper refers to the protocol of the InCHIANTI follow-up study and not the specifi
No information
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Type of prediction modelling study:	Describe the study type:
Developmental without external validation.	
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External model validation without model updating.	
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Developmental with external validation.	
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Developmental without external validation.	
Developmental without external validation.	

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Development	Study design:	Validation:	Study design:
	Prospective Cohort study		
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Development: Describe the study design: **Validation: Describe the study design:**

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Country of origin
Canada
Australia
The Netherlands
The Netherlands
The Netherlands
USA
UK
The Netherlands
Japan
Germany
USA
Australia
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USA
France
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USA
Israel
USA
The Netherlands
United Kingdom
USA
USA
Italy, Germany, Ireland, and England.
Spain
USA
USA
Italy
Malaysia
Belgium

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Development: Setting where candidate predictors were measured: (choice=Homes)

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For peer review only

Development: Setting where candidate predictors were measured: (choice=General Practitioner)

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For peer review only

Development: Setting where candidate predictors were measured: (choice=Study centre)

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For peer review only

Development: Setting where candidate predictors were measured: (choice=Others)

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For peer review only

Development: Setting where candidate predictors were measured: (choice=Not specified)

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For peer review only

Which other settings were chosen?:

Health examination center

Face-to-face or telephone interview

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Validation: Setting where predictors were measured: (choice=Homes)

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Validation: Setting where predictors were measured: (choice=General Practitioner)

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Validation: Setting where predictors were measured: (choice=Study centre)

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For peer review only

Validation: Setting where predictors were measured: (choice=Others)

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Validation: Setting where predictors were measured: (choice=Not specified)

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Which other settings were chosen?:

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Development: Number of study centres:	
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At least 2 (After a home interview, participants were invited to the VU University Medical Center (VU	
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1 (Nationally representative cohort)	
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No information	
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Validation: Number of study centres:

mc) or a health care centre)

1

4

1

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Which in-/exclusion criteria were used?

Inclusion: - Living independently in private or shared apartments - Able to stand unaided for 90 sec. -
Inclusion: - Community-dwelling - 65+ yr - female sex Exclusion: - History of surgery on either lower li
Inclusion: - Age 65 years or over - Community-dwelling - Had participated in the previous data collect
Inclusion: - Community-dwelling - Age 70 years or over Exclusion: - Wheelchair dependency - Serious
Inclusion: - Community-dwelling - Age 65 years or older
Inclusion: - Community-dwelling - 65 yr or older - Able to stand upright for 5 minutes without assistar
Inclusion: - 65 years or older - Living in one primary care group area
Inclusion: - Living in the community - Age 65 years or older
Inclusion: - Age 65 years or older - Living in urban or rural communities
Inclusion: - Community-dwelling - 65+ yr Exclusion: - Use of walking aid - Self-reported neurological d
Inclusion: - Female - Medicare beneficiary - Reported at least mild disability in 2 or more of the follow
Inclusion: - Community-dwelling - 70+ yr Exclusion: - Neurological, cardiovascular, or major musculosl
Inclusion: - Age 60 years or older - Certain income/resource level - Nursing home intermediate level c
Inclusion: - Community-dwelling Exclusion: - Cognitive impairment (MMSE < 24) - Legally blind - Obes
Inclusion: - Community-dwelling - 65+ yr Exclusion: - Living i nursing home - Neurological disease such
Inclusion: - Age 65 years or older - Lived in the community - Able to walk 4 m - Have a gait speed betv
Inclusion: - Age 65 years or above
Inclusion: - Community-dwelling older adult - Not previously clinically diagnosed with any gait or bala
Inclusion: - Ambulatory - 65 years of age and older - Admitted to home health services requiring at le
Inclusion: - Community-dwelling - 65+ yr
Inclusion: - 65 years or older - Community dwelling - Able to speak and understand English - Able to t
Inclusion: - Community-dwelling - Age 65+ yr. Exclusion: - Not able to perform balance tests - Use of v
Inclusion: - 65+ yr of age - Able to consent - Able to walk without assistance of another person - No bi
ActiFE: - Living in the area greater Ulm or Neu-Ulm, located in the South of Germany. - Being 65-90 yr
Inclusion: - Non-institutionalised - Older than 64 years - Living in Spain
Inclusion - 65+ yr of age Exclusion: - Living in a nursing home or unspecified residential facility at base
Inclusion: - Community-dwelling - 65+ yr - Available claims data for the entire year from the Medicare
Inclusion: - Age 65 years or older - Must reside in Greve in Chianti or Bagno A Ripoli, Italy - Subjects w
Inclusion: - Age 60 years or above - Living in the community - Ambulating independently with or with
Inclusion: - Living independently at home - Age 65 years or above - Able to understand French - Able t

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Development: Method for participant recruitment:
Convenience sampling.
Convenience sampling.
Probability sampling.
Probability sampling.
Probability sampling.
Convenience sampling.
Probability sampling.
Probability sampling.
No information
Convenience sampling.
Probability sampling.
Probability sampling.
Consecutive sampling.
Convenience sampling.
Consecutive sampling.
No information
Probability sampling.
No information
ast two visits. Exclusion: - Bed-bound participants
Probability sampling.
Convenience sampling.
Probability sampling.
Convenience sampling.
old - Not being institutionalised - Being able to walk inc
Probability sampling.
Probability sampling.
Probability sampling.
Probability sampling.
Probability sampling.
Convenience sampling.

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Development: What other methods of participant recruitment were used?:

dependently through their own room - Not having serious difficulties in German la

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Validation: Method for participant recruitment:

No information

Probability sampling.

Probability sampling.

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Development: What other methods of participant recruitment were used?:

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Development: Date of First Enrollment:	Development: Enrollment Closure Date:
1/1/1995	
1/1/1995	
1/1/1995	12/31/1996
8/1/2004	10/1/2004
9/1/1992	
1/1/2006	10/1/2007
7/1/2005	6/30/2006
1/1/2007	5/1/2007
1/1/2004	
1/1/1995	12/31/1996
1/1/2011	
3/1/2000	4/1/2002
1/19/2008	1/30/2009
1/1/2011	
1/1/2006	12/31/2006
6/1/2013	7/1/2014
5/1/2012	2/1/2013
7/1/2014	10/1/2015

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Validation: Date of First Enrollment:	Validation: Enrollment Closure Date:
9/1/2011	12/1/2011
1/1/1999	1/1/2013
1/1/2011	12/31/2011

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What was the length of follow-up chosen?

12 months
6 months
12 months
9 months
Primary length of follow-up: 3 years Secondary length of follow-up: 1 year.
12 months
6 months
Primary length of follow-up: 3 years Secondary length of follow-up: 1 year.
6 months
12 months
12 months
12 months
Between 1 day and 18 months due to the study using an open cohort design.
12 months
12 months
12 months
24 months
6 months
Participants were followed until discharged from home health services.
12 months
6 months
12 months
12 months
ActiFE: 12 months ELSA: 2 years InCHIANTI: 1 year (After 3 years participants were asked about falls i
12 months
48 months
No length of follow-up was stated. However, they reported that they wanted to predict the outcome
12 months
6 months
24 months

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Did the validation study differ in other ways from the development study (comment):

n the past 12 months) TILDA: 2 years

s in the following year from baseline.

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Outcome definition:

"An occasion on which the participants found themselves unintentionally on the floor, ground or othe
Any event that resulted in coming to rest inadvertently on the ground or another lower level
A fall was defined as an unintentional change in position resulting in coming to rest on the ground or
A "fall" was defined as the subject unintentionally coming down on the floor or to a lower level.
A fall was defined as "an unintentional change in position resulting in coming to rest at a lower level
"Any disturbance of balance during routine activities that resulted in a person's trunk, knee, or hand
No outcome definition was reported. However, respondents were asked if they had 'fallen in the last
An unintentional change in position resulting in coming to rest at a lower level or on the ground.
"A fall was defined as an unintentional change in position resulting in coming to rest on the ground o
An unexpected event in which the subject comes to rest on the ground, floor, or lower level
Falling was defined as falling on the ground or at some other level such as chair level
"An unexpected event in which the person comes to rest on the ground, floor, or lower level" Faller
No information
No information.
"Unintentionally coming to rest on the ground, floor, or other lower level"
A fall is defined as unintentionally coming to rest on the ground or other surface.
No definition. Participants were asked: "Have you fallen down in the last two years?".
A fall was defined as unintentionally coming to rest on a lower surface
"An unexpected event in which the participant comes to rest on the ground, floor or lower level"
"An unintentional change in position resulting in coming to rest at a lower level or on the ground"
No information. However, the fall recording method recommended by the PROFANE group was refer
"Any fall, slip, or trip in which you lose your balance and land on the floor or ground or at a lower leve
Falls: "An unexpected event in which the participants come to rest on the ground, floor, or lower leve
ActiFE: PROFANE definition (found in previous studies) ELSA: No information. InCHIANTI: No informat
"Any event, where the subject unintentionally came to rest on the ground or a lower level"
No definition was applied since outcome reporting was based on self-report.
No information
No outcome definition was reported. However, participants were asked: "Have you ever fallen to the
"An event which resulted in a person coming to rest inadvertently on the ground or floor or other lov
"A fall was defined as an unexpected event in which the participant comes to rest on the ground, floo

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Was the outcome prespecified?:	Type of fall recording: (choice=Fall calendars)
No information	Checked
Yes	Checked
Yes	Checked
Yes	Unchecked
Yes	Checked
Yes	Checked
No information	Unchecked
Yes	Checked
Yes	Unchecked
Yes	Checked
Yes	Unchecked
No information	Checked
No information	Unchecked
No information	Unchecked
Yes	Unchecked
Yes	Unchecked
No	Unchecked
Yes	Checked
Yes	Checked
Yes	Checked
No information	Checked
Yes	Unchecked
No information	Unchecked
No information	Checked
Yes	Unchecked
No	Unchecked
No information	Unchecked
No	Unchecked
Yes	Checked
Yes	Checked

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Type of fall recording: (choice=Telephone calls)

Checked
Checked
Unchecked
Checked
Unchecked
Checked
Unchecked
Unchecked
Unchecked
Unchecked
Unchecked
Checked
Unchecked
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Unchecked
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Unchecked
Unchecked
Checked
Checked
Checked

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Type of fall recording: (choice=Follow-up visits)	Type of fall recording: (choice=Others)
Unchecked	Unchecked
Unchecked	Unchecked
Unchecked	Unchecked
Unchecked	Unchecked
Unchecked	Unchecked
Unchecked	Checked
Unchecked	Checked
Unchecked	Unchecked
Unchecked	Checked
Unchecked	Unchecked
Checked	Unchecked
Unchecked	Unchecked
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Unchecked	Unchecked
Checked	Unchecked
Unchecked	Checked
Checked	Checked
Unchecked	Unchecked
Checked	Unchecked
Unchecked	Checked
Unchecked	Unchecked
Unchecked	Unchecked
Unchecked	Unchecked

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What type of fall recording was used?:

E-mail and telephone
Postal questionnaire

The outcome of fall occurrence and the number of falls were confirmed by social workers, health visi

Data on falls for program participants were not part of the initial assessment, rather the case manage
Postcards with validation by telephone afterwards

Follow-up interviews

Mailed questionnaire
ActiFE: Fall calendars ELSA: Follow-up visits InCHIANTI: Follow-up visits TILDA: Follow-up visits

Survey

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How often was the outcome recorded? (e.g. every month)

Weekly recording with notifications upon incidence.
Daily recording in fall calendars returned monthly and with telephone validation upon fall incidence
Weekly recording with notifications every three month.
Every 6 weeks
Weekly recording with notifications every three month.
Daily recording with notifications every 2-4 weeks by telephone or email.
After 6 months
Weekly recording with notifications every three month.
After 6 months
Daily recording with reminders to fill out the calendar every second month.
Every 6 months
Daily recording with monthly notifications.
Upon incidence.
Weekly recording with notification upon a fall incidence.
Monthly
Every 3 months
After 2 years
Daily recording with monthly notifications
Daily recording with notification (adequate surveillance of documentation and ascertainment of deta
Weekly recording in fall calendars, mailed in every three months. Notifications in case of non-return c
Daily recording with monthly notifications.
Reinterview after 12 months
Three times per year
ActiFE: Weekly fall calendars with notifications every three months. ELSA: Re-interviewed after 2 yea
Every three months
Reinterview every 12 months
Three times per year
Every month for the first 6 months after baseline, and a final call after 12 months.
Daily recording witout monthly telephone reminders
Fall diary with recording upon incidence and notification every three months.

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Was the same outcome definition and recording method used in all participants?:

Yes
Yes
Yes
Yes
Yes
Yes
No information
Yes
Yes
Yes
Yes
Yes
No information
No information
Yes
Yes
Yes
Yes
Yes
Yes
No information
Yes
Yes
No information
Yes
Yes
No information
No information
Yes
Yes

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Development: Was the outcome assessor blinded towards predictors?:

No information
No information
No information
No information
No information
No information
No information
No information
No information
No
No information
Yes
No information
No information
Yes
No information
No information
No information

No information
No information
No information
Yes

Yes
Yes
No information
No information
No information
No information

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Validation: Was the outcome assessor blinded towards predictors?:

No information

Yes

No information

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Candidate predictor #1

Balance - Spontaneous sway with eyes open and closed (movable force platform) (24 measures)
Step movement speed reaction time during high preparation (Force platform - MEUQ, The University
Age (Questionnaire)
Physical, mental, and social functioning (Short version of the Sickness Impact Profile (SIP68))
Age (Questionnaire)
Visual problems (questionnaire)
History of falling in the previous year (Questionnaire)
Age ≥ 80 years (vs. <80 years) (Questionnaire)
History of fall within one year (Questionnaire)
Maximum valid step length adjusted to body height (Modified Maximum Step Length Test)
Activities of Daily Living (Self-rated with subsequent scoring)
Number of medical conditions defined a scardiovascular, respiratory, mus- culoskeletal, endocrine, u
Fall history (Comprehensive assessment tool was developed by the Ohio Department of Aging to mee
Quiet standing (Clinical Romberg test with eyes open compared to eyes closed in a comfortable stanc
Age
Physical performance (4 m gait speed test)
Fall history (Self-reported)
Balance (Dynamic gait index)
Age (Self-report)
Body Mass Index (Calibrated bathroom scale)
Dual tasking (Straight walking and visuospatial clock task)
History of myocardial infarction (Self-report)
Age
Age
Have you fallen in the last 6 months? (Questionnaire)
Experienced a fall in the previous year (Y/N) (Self-report)
ICD-9 diagnoses (166 variables)
Two or more falls in prior 12 months (Questionnaire)
Age (Questionnaire)
Mean Minimal Toe Clearance at self-selected comfortable walking speed (CodaMotion®, Charnwood

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Candidate predictor #2

Balance - Induced sway with eyes open and closed (movable force platform) (44 measures)
Step movement speed weight shift time during high preparation (Force platform - MEUQ, The Univer
Gender (Questionnaire)
Activities of daily living (Barthel Index)
Gender (Questionnaire)
Dizziness in the past year (questionnaire)
Taking four or more prescribed medication (Questionnaire)
Women (vs. men) (Questionnaire)
History of stumbling within one year (Questionnaire)
Mean length of valid steps adjusted to body height (Modified Maximum Step Length Test)
Age (No method of measurement reported)
No. of Medications (Interview)
Age (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet Ohio'
Quiet standing (Clinical Romberg test with eyes open with feet together compared to eyes open with
Sex
Physical performance (Timed up and go test)
Age (Self-reported)
Balance (Berg Balance Scale)
Fall history defined by one fall in past 6 months (Self-report)
Peak Expiratory flow (Mini-Wright peak flow meter)
Dual tasking (Walking with turns and naming animals)
History of coronary artery disease (Self-report)
Baumgartner Sarcopenia Definition
Cognitive impairment
Do you think you may fall in the next few months? (Questionnaire)
Worried about falling (Y/N)? (Self-report)
ICD-9 procedures (100 variables)
Present with acute fall (Questionnaire)
Cognition (Mini-Mental State Examination)
Median Minimal Toe Clearance at self-selected comfortable walking speed (CodaMotion®, Charnwoo

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Candidate predictor #3

Balance - Anticipatory postural response during arm raise (movable force platform)
Step movement speed step time during high preparation (Force platform - MEUQ, The University of C
Educational level (Questionnaire)
Cogition (Mini-mental state examination)
Educational length (Questionnaire)
Number of medication (questionnaire)
History of stroke or Parkinson's disease (Questionnaire)
Education ≥11 years (vs. <11 years) (Questionnaire)
Can you climb stairs without help? (Questionnaire)
Number of missteps (Modified Maximum Step Length Test)
Body Mass Index (Stadiometer and calibrated bathroom type digital scale)
Fall history (Interview)
Gender (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet O
Quiet standing (Clinical Romberg test with eyes open compared to eyes closed with feet together usi
Living alone
Age (Method of measurement not reported)
Years of education (Self-reported)
Functional performance (Timed Up & Go)
Presence of elimination problems (bowel and urine) (Self-report)
Cognitive function (Mini-mental State Examination, MMSE)
Dual tasking (Walking with turns and counting backwards in 3s)
History of stroke (Self-report)
Newman Sarcopenia Definition
Depression
What is the probability that you fall in the next few months? (Questionnaire)
Feeling unsafe or unsteady while walking (Y/N) (Self-report)
CPT-4 Services (169 variables)
Difficulty with walking or balance (Questionnaire)
Depressive symptoms (Geriatric Depression Score)
Minimum Minimal Toe Clearance at self-selected comfortable walking speed (CodaMotion®, Charnw

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Candidate predictor #4

Balance - Timed one-leg stance with eyes open and closed (movable force platform)
Step movement speed movement time during high preparation (Force platform - MEUQ, The Univers
Urbanisation level (Questionnaire)
State of mind (Depression sub scale of the Symptom Checklist (SCL90)
Number of chronic diseases (Questionnaire)
Cardiac problems (questionnaire)
Reported problems with balance (Questionnaire)
Living in an highly urbanised area (Questionnaire)
Do you feel your walking speed declined recently? (Questionnaire)
Number of falls in previous year (Self-Report)
Depressive symptoms (Geriatric Depression Scale 15-item)
Physical performance (Physiological Profile Assessment)
Race (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet Ohic
Maximal Leaning in forward direction (Centre of pressure excursion using a forceplate)
Socioprofessional categories
Gender (Method of measurement not reported)
Household income and assets (Self-reported)
Stability (Four Square Step Test)
Use of high risk medications defined by opiates
Visual acuity (Self-report)
Dual tasking (Avoiding stationary obstacles and naming animals)
History of osteoporosis (Self-report)
IWG Sarcopenia Definition
Diabetes

Balance (4 stage balance test)
HCPCS Level II supplies (145 variables)
One fall in prior 12 months (Questionnaire)
Gender (Questionnaire)
Standard Deviation of Minimal Toe Clearance values at self-selected comfortable walking speed (Cod

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Candidate predictor #5

Balance (Clinical balance performance assessment)
Step movement speed total movement time during high preparation (Force platform - MEUQ, The Ur
Urinary incontinence (Questionnaire)
Body weight (Method of measurement not reported)
Osteoarthritis (Questionnaire)
Pulmonary problems (questionnaire)
Loss of proximal muscle strength (Inability to rise from a chair without using arms)
≥ One chronic disease (vs. < one disease) (Questionnaire)
Can you cross the road within the green signal interval? (Questionnaire)

History of diabetes (Self-reported)
Executive function (Trail-making-test, A, B, A-B)
Marital status (Comprehensive assessment tool was developed by the Ohio Department of Aging to n
Maximal Leaning in right direction (Centre of pressure excursion using a forceplate)
Urinary incontinence (self-reported)
Education (Method of measurement not reported)
Race (Self-reported)
Balance confidence (Activities-specific Balance Confidence)
Use of patient care equipment (Self-report)
Auditory acuity (Self-report)
Dual tasking (Avoiding a moving obstacle and carrying a cup)
History of diabetes mellitus (Self-report)
EWGSOP Sarcopenia Definition
Comorbidities

Muscle strength (5 times chair stand test)

Gait and balance (Timed up and go)
Race (Questionnaire)
Variance of Minimal Toe Clearance values at self-selected comfortable walking speed (CodaMotion®,

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Candidate predictor #6

Step movement speed reaction time during neutral preparation (Force platform - MEUQ, The Univers
Joint disorders (Questionnaire)
Body height (Method of measurement not reported)
Number of medications used (Questionnaire)
Use of an assistive device (questionnaire)

Osteoarthritis (yes/no) (Questionnaire)
Can you walk 1 km continuously? (Questionnaire)

History of osteoporosis (Self-reported)
Language skills (Boston Naming Test)
Alzheimer's disease (Comprehensive assessment tool was developed by the Ohio Department of Agir
Maximal Leaning in left direction (Centre of pressure excursion using a forceplate)
Osteoarthritis (self-reported)
Previous falls in past year (Method of measurement not reported)
Gender (Self-reported)
Mobility (4 m Gait speed test)
Limited mobility (Assessor based)
Urinary incontinence (Self-report)
Dual tasking (Timed Up & Go (TUG) and carrying a cup)
History of hypertension (Self-report)
FNIH #1
Dizziness and vertigo

Multiple falls in past year (Y/N)? (Self-report)

Gait and balance (Short Physical Performance Battery)
Formal education (Questionnaire)
Coefficient of Variation of Minimal Toe Clearance values at self-selected comfortable walking speed (

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Candidate predictor #7

Step movement speed weight shift time during neutral preparation (Force platform - MEUQ, The Univ
Number of chronic diseases (Questionnaire)
Body Mass Index
Involuntary loss of urine (Questionnaire)
Cancer (questionnaire)

Medication use ≥ four drugs (vs. < four drugs) (Questionnaire)
Can you stand on one foot for about five seconds? (Questionnaire)

History of stroke (Self-reported)
Memory performance (Logical memory subtest - Story A from Wechsler Memory Scale)
Cancer (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet Ol
Maximal Leaning in backward direction (Centre of pressure excursion using a forceplate)
Medication

Marital status (Self-reported)
Three day acceleration derived measures - Total number of walking bouts (DynaPort Hybrid, McRobe
Presence of altered cognition (Assessor based)
Sense of Mastery (Short version Pearlin and Schooler Mastery scale)
Dual tasking (Stair descent and naming animals)
History of vision impairment (Self-report)
FNIH #2
Fear of falling

Previous hip fracture (Y/N)? (Self-report)

Gait speed (7m gait speed test)
Hypertension (Questionnaire)
Delta1 Minimal Toe Clearance at self-selected comfortable walking speed (CodaMotion®, Charnwood

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Candidate predictor #8

Step movement speed step time during neutral preparation (Force platform - MEUQ, The University
Number of medications used (Questionnaire)
Blood pressure (Method of measurement not reported)
Dizziness (Questionnaire)
Number of falls from the previous year (questionnaire)

Involuntary loss of urine (yes/no) (Questionnaire)
Do you use cane when you walk? (Questionnaire)

History of Parkinson's disease (Self-reported)
Visuoconstructional ability (Block design subtest - WAIS)
Cataract (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet
Sit-to-stand time (Forceplate)
Psychoactive drug use

Insured (Self-reported)
Three day acceleration derived measures - Total walking duration (DynaPort Hybrid, McRoberts, The

Depressive symptoms (Center for Epidemiological Studies Depression scale, CES-D)
Dual tasking (Walking while talking complex)
History of hearing impairment (Self-report)

Female sex

Age (self-report)

Diabetes (Questionnaire)
Delta2 Minimal Toe Clearance at self-selected comfortable walking speed (CodaMotion®, Charnwood

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Candidate predictor #9

Step movement speed movement time during neutral preparation (Force platform - MEUQ, The Univ
Use of benzodiazepines (Questionnaire)
Pulse rhythm (Method of measurement not reported)
Systolic blood pressure (Omron HEM 706)
Physically active defined as exercising at least one time per week (questionnaire)

Dizziness (yes/no) (Questionnaire)
Can you squeeze the towel tightly? (Questionnaire)

History of arthritis (Self-reported)
Fear of falling (FES-I)
Dementia (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet
Sit-to-stand medial-lateral excursion (Forceplate)
Fall history (1 yr)

Self-rated health (Self-reported)
Three day acceleration derived measures - Total number of steps (DynaPort Hybrid, McRoberts, The I

Physical activity (LASA Physical Activity Questionnaire)
Triple tasking (Straight walking, visuospatial clock task, and carrying a cup)
Self-perceived balance and coordination problems (Self-report)

Gait problems

Race (self-report)

Heart diseases (Questionnaire)
Mean Minimal Toe Clearance at self-selected fast walking speed (CodaMotion®, Charnwood Dynamic

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Candidate predictor #10

Step movement speed total movement time during neutral preparation (Force platform - MEUQ, The
Use of anti epileptics drugs (Questionnaire)
Pulse frequency (Method of measurement not reported)
Orthostatic hypotension (Method of measurement not reported)
Balance (Berg Balance Scale)

Systolic blood pressure ≤ 133 mmHg (vs. >133 mmHg) (Omron HEM 706)
Do you feel dizzy? (Questionnaire)

History of myocardial infarction (Self-reported)
Depression (Geriatric Depression Scale - 15 item)
Depression (Comprehensive assessment tool was developed by the Ohio Department of Aging to mee
Sit-to-stand anterior-posterior excursion (Forceplate)
ADL (Katz index)

High blood pressure (Self-reported)
Three day acceleration derived measures - Median walking bout duration (DynaPort Hybrid, McRobe

Falls in previous years (Method of measurement not reported)
History of falls (Self-report)

Hearing impairment

Gender (self-report)

Arthritis (Questionnaire)
Median Minimal Toe Clearance at self-selected fast walking speed (CodaMotion®, Charnwood Dynam

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Candidate predictor #11

EMG onset time on right gluteus medius during high preparation (Force platform - MEUQ, The Univer
Depressive symptoms (Center for Epidemiologic Studies - Depression scale)
Visual acuity (Snellen Chart)
Visual imparirment (Method of measurement not reported)
Balance (Dynamic Gait Index)

Orthostatic hypotension (yes/no) (Questionnaire)
Is your back bended? (Questionnaire)

History of angina (Self-reported)
Symptoms of anxiety (Goldberg Anxiety Scale - 9 item)
Diabetes (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet
Sit-to-stand force impulse (Forceplate)
Depression (Geriatric Depression Scale - 4 item)

Takes at least one prescription drug (Self-reported)
Three day acceleration derived measures - Median number of steps for bout (DynaPort Hybrid, McRo

Age (Method of measurement not reported)
Mobility (Short Physical Performance Battery)

History of falls

Education (self-report)

History of falls (Questionnaire)
Minimum Minimal Toe Clearance at self-selected fast walking speed (CodaMotion®, Charnwood Dyna

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Candidate predictor #12

EMG onset time on right tensor fascia latae during high preparation (Force platform - MEUQ, The Uni
Visual impairment (Questionnaire)
Hearing of speech (assessed at a distance of 2 meters with or without hearing aid)
Hearing impairment (Self-reported hearing ability (LASA*031))
Balance (Timed Up and Go Test)

Visual impairment (yes/no) (Questionnaire)
Do you have knee pain? (Questionnaire)

History of chronic heart failure (Self-reported)
Positive affect (Positive and Negative Affect Scale)
Emphysema (Comprehensive assessment tool was developed by the Ohio Department of Aging to me
Self-selected gait speed (8.1 metres - speed m/s)
Near and distance visual acuity (Parinaud scale and Monoyer scale)

Body Mass Index (Calculated on self-reported weight and height)
Three day acceleration derived measures - Amplitude of dominant frequency (DynaPort Hybrid, McR

Sex (Method of measurement not reported)
Age (Self-report)

History of stroke

Living alone (self-report)

Instrumental Activities of Daily Living (Lawton IADL)
Standard Deviation of Minimal Toe Clearance values at self-selected fast walking speed (CodaMotion

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Candidate predictor #13

EMG onset time on left hip adductor muscle group during high preparation (Force platform - MEUQ,
Hearing impairment (Questionnaire)
Inspection for skin lesions, deformities and callus in feet (Clinical examination)
Foot problems (Clinical examination)
Postural sway standing on a firm surface with eyes open (Modified Clinical Test for Sensory Interactio

Hearing impairment (yes/no) (Questionnaire)
Do you have a vision problem? (Questionnaire)

History of high blood pressure (Self-reported)
Personality (NEO Personality Inventory: Neuroticism, openness, conscientiousness)
Glaucoma (Comprehensive assessment tool was developed by the Ohio Department of Aging to mee
Self-selected gait speed (8.1 metres - Centre-of-Pressure excursion during initiation
Alcohol (self-reported)

Self-rated vision (Self-reported)
Three day acceleration derived measures - Width of dominant frequency (DynaPort Hybrid, McRober

Fear of falling (FES-I)
Gender (Self-report)

Instrumental disability

Smoking (self-report)

Mobility and balance (Timed Up and Go)
Variance of Minimal Toe Clearance values at self-selected fast walking speed (CodaMotion®, Charnwo

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Candidate predictor #14

EMG onset time on right gluteus medius during neutral preparation (Force platform - MEUQ, The Uni
Functional limitations (OECD indicator of chronic functional limitations)
Strength in feet (Method of measurement not reported)
Poor self-perceived health (Questionnaire with two questions, taken from the Netherlands Health Int
Postural sway standing on a firm surface with eyes closed (Modified Clinical Test for Sensory Interacti

Foot problems (yes/no) (Questionnaire)
Do you have a hearing problem? (Questionnaire)

History of any other heart problem (Self-reported)
Disability (WHODAS II - 12 item)
Incontinence (Comprehensive assessment tool was developed by the Ohio Department of Aging to m
Gait speed "as fast as possible" (8.1 metres - speed m/s)
Mobility (Timed Up and Go test)

Limitations in Activities of Daily Living (ADL5 - Self-reported)
Three day acceleration derived measures - Slope of dominant frequency (DynaPort Hybrid, McRobert

Executive function (Executive Interview -15 item)
Ethnicity (Self-report)

Living alone

Body Mass Index (self-report)

Gait speed (10 meter gait speed test)
Coefficient of Variation of Minimal Toe Clearance values at self-selected fast walking speed (CodaMo

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Candidate predictor #15

EMG onset time on right tensor fascia latae during neutral preparation (Force platform - MEUQ, The
Dizziness (Questionnaire)
Propriocepsis in feet (Method of measurement not reported)
Hospital admission past 6 months (Method of measurement not reported)
Postural sway standing on a foam surface with eyes open (Modified Clinical Test for Sensory Interacti

Poor or very poor self-perceived health (vs. fair/ good/excellent) (Questionnaire)
Do you think you are forgetful? (Questionnaire)

History of cancer (Self-reported)
Quality of life (AQoL II)
Parkinson's disease (Comprehensive assessment tool was developed by the Ohio Department of Agin
Gait speed "as fast as possible" (8.1 metres - Centre-of-Pressure excursion during initiation)
Balance impairment (One-leg balance test with change in arm position during first 5 sec.)

Limitations in Instrumental Activities of Daily Living (IADL5 - Self-reported)
Three day acceleration derived measures - Average step duration (DynaPort Hybrid, McRoberts, The

Anxiety and depression (Hospital Anxiety and Depression Scale)
Gait speed (Short Physical Performance Battery)

Number of medications

Vision impairment (self-report)

Dual-tasking (Walk While Talking test)
Delta1 Minimal Toe Clearance at self-selected fast walking speed (CodaMotion®, Charnwood Dynami

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Candidate predictor #16

EMG onset time on left hip adductor muscle group during neutral preparation (Force platform - MEU
Feet problems (Questionnaire)
Sensibility in feet (Method of measurement not reported)
Two falls or more in the past year (Questionnaire)
Postural sway standing on a foam surface with eyes closed (Modified Clinical Test for Sensory Interac

Hospital admission in the past 6 months (yes/no) (Questionnaire)
Do you feel anxious to fall when you walk? (Questionnaire)

History of lung disease (Self-reported)
Disability (Disability scaled derived from WHODAS II, AQoL II)
Stroke (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet Oh
Walk to chair and sit - Standing 2 strides away from a chair - Time from the first step until the subject
Type of residence (self-reported)

Incontinence (Self-reported)
Three day acceleration derived measures - Average stride duration (DynaPort Hybrid, McRoberts, The

Mobility (Performance Oriented Mobility Assessment)

Pain

Hearing impairment (self-report)

Risk of falls (Physiological Profile Assessment)
Delta2 Minimal Toe Clearance at self-selected fast walking speed (CodaMotion®, Charnwood Dynami

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Candidate predictor #17

Average COP position relative to center of plates (mm) in mediolateral position during quiet stance w
Physical activity (LASA Physical Activity Questionnaire (LAPAQ))
Grip strength in dominant hand (My Griper dynamometer)
Pain (Questionnaire)
Limits of stability reaction time (100 % Limits of Stability Test)

Falls in the previous year ≥ two (vs. < two) (Questionnaire)
Do you take more than five kinds of prescribed medicines? (Questionnaire)

History of hip fracture (Self-reported)
Exercise (Incidental and Planned Exercise Questionnaire)
Vertigo (Comprehensive assessment tool was developed by the Ohio Department of Aging to meet O
Walk to chair and sit - Standing 2 strides away from a chair - Number of steps taken towards the chai
Retirement (self-reported)

Back problems (Self-reported)
Three day acceleration derived measures - Step regularity (DynaPort Hybrid, McRoberts, The Hague,

Note: Primary task and secondary task performance in ST, DT, and the propor- tionate and absolute c

Parkinson

No. of comorbidities/ Medical burden (self-report)

Number of medications (Questionnaire)
Mean Minimal Toe Clearance during a dual-task walking condition (CodaMotion®, Charnwood Dynam

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Candidate predictor #18

Average velocity of COP displacement (mm/s) in mediolateral position during quiet stance with eyes
Physical performance (Performance tests of walking, chair stands, and tanden stand)
Balance and gait (Get-up and go test)
Body weight (Calibrated bathroom balance scale)
Limits of stability end time excursion (100 % Limits of Stability Test)

Pain (yes/no) (Questionnaire)
Do you feel dark walking within your home? (Questionnaire)

Difficulties with control of urine or any wetting (Self-reported)
Physical Assessment (Coordinated Stability Test)
Total number of medications (Comprehensive assessment tool was developed by the Ohio Departme
Walk to chair and sit - The turning moment about the vertical axis (Forceplate)
BMI (GP)

Difficulty with kneeling (Self-reported)
Three day acceleration derived measures - Stride regularity (DynaPort Hybrid, McRoberts, The Hague

ifferences were used to predict falls.

Physical activity limitation

Functional impairment (self-report)

Median Minimal Toe Clearance during a dual-task walking condition (CodaMotion®, Charnwood Dyna

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Candidate predictor #19

Root mean-square COP displacement (mm) of COP displacement (mm/s) in mediolateral position dur
Grip strength (Strain-gauged dynamometer - sum of each hand (Takei TKK 5001, Takei Scientific Instru
Proprioception (Romberg Test)
Body height (Stadiometer)
Limits of stability movement speed (100 % Limits of Stability Test)

Body weight: women ≤ 62 kg (vs. >62 kg); men ≤ 70 kg (vs. >70 kg) (Calibrated bathroom balance scal
Are there any obstacles within the house? (Questionnaire)

Multisite pain (Self-reported)
Physical Assessment (One-leg Balance Test)
Function limitations in six Activities of Daily Living (Questionnaire)
Reaching in a cabinet - Maximum excursion forward from the initial Centre-of-Pressure position (Fore
One Leg Balance dominant side test time

Difficulty with extending arms above the shoulders (Self-reported)
Three day acceleration derived measures - Step symmetry (DynaPort Hybrid, McRoberts, The Hague,

Physical disability

Frailty (Adapted Fried Criteria: objective measurement of grip strength and gait speed, self-report we

Minimum Minimal Toe Clearance during a dual-task walking condition (CodaMotion®, Charnwood Dy

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Candidate predictor #20

Total COP displacement (mm) during quiet stance with eyes open (Force platform - MEUQ, The Unive
Fall history (Questionnaire)
Spontaneous and anterior-posterior sway (Postural Sway Test)
Number of functional limitations (OECD indicator of chronic functional limitations)
Limits of stability maximum excursion (100 % Limits of Stability Test)

Body height: women ≤156 cm (vs. >156 cm); men≤169 cm (vs. >169 cm) (Stadiometer)
Is there any level difference within your home? (Questionnaire)

Frequency of experiencing fainting in last year (Self-reported)
Age (Interview)
Function limitation in five Instrumental Activities of Daily Living (Questionnaire)
Reaching in a cabinet - Sway area during forward reach (Forceplate)
One Leg Balance non-dominant side test time

Cognitive impairment (Telephone Interview for Cognitive Status (TICS))
Three day acceleration derived measures - Harmonic ratio (DynaPort Hybrid, McRoberts, The Hague,

Poor self-perceived health status

eight loss, exhaustion and low physical activity)

Standard Deviation of Minimal Toe Clearance values during a dual-task walking condition (CodaMotic

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Candidate predictor #21

Average COP position relative to center of plates (mm) in mediolateral position during quiet stance w
Fear of falling (Falls Efficacy Scale)
Balance (Bending down test)
Physical performance (Performance tests of walking, chair stands, and tanden stand)
Limits of stability directional control (100 % Limits of Stability Test)

Body Mass Index ≤24 kg/m2 (vs. >24)
Do you have to use stairs in daily living? (Questionnaire)

Frequency of experiencing dizziness in last year (Self-reported)
Sex (Interview)
Shuffling gait when walking (Questionnaire)
Reaching in a cabinet - Velocity of the movement (Forceplate)
Orthostatic hypotension (GP)

Smoking status (Self-reported)
Three day acceleration derived measures - Acceleration range (DynaPort Hybrid, McRoberts, The Hag

Rheumatic disease

Variance of Minimal Toe Clearance values during a dual-task walking condition (CodaMotion®, Charn

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Candidate predictor #22

Average velocity of COP displacement (mm/s) in mediolateral position during quiet stance with eyes
Living alone (Questionnaire)
Mobility (Trendelenburg Test)
Using walking aid (Questionnaire)
Age (self-reported)

Functional limitations ≥ three (vs. < three) (OECD indicator of chronic functional limitations.)
Do you walk steep slope around the house? (Questionnaire)

Frequency of experiencing spinning in last year (Self-reported)
Physical assessment (6-m Walking Test)
Unsteady gait when walking (Questionnaire)
Pulling open a door - Maximum excursion backward of initial centre-of-pressure position (Forceplate)
Tobacco use (self-reported)

Drinking alcoholic beverages (Self-reported)
Number of falls (self-report)

Urinary incontinence

Coefficient of Variation of Minimal Toe Clearance values during a dual-task walking condition (CodaM

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Candidate predictor #23

Root mean-square COP displacement (mm) of COP displacement (mm/s) in mediolateral position dur
Body weight (Calibrated bathroom balance scale)
Balance (Functional Reach Test)
Grip strength (Strain-gauged dynamometer (Takei TKK 5001, Takei Scientific Instruments Co. Ltd, Tok
Sex (self-reported)

Performance test score ≤4(vs. >4) (range: 0-12) (Performance tests: walking + chair stands + put on a

Polypharmacy defined as receiving 4 or more medications (Self-reported)
Diabetes mellitus (Interview)
Tremors while grasping (Questionnaire)
Pulling open a door - Sway area during the pull (Forceplate)

Frequency of participation in light physical activities (Self-reported)
Number of missteps (self-report)

Use of antiepileptics

Delta1 Minimal Toe Clearance during a dual-task walking condition (CodaMotion®, Charnwood Dynar

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Candidate predictor #24

Total COP displacement (mm) during quiet stance with eyes closed (Force platform - MEUQ, The Univ
Body height (Stadiometer)
Cognitive behaviour (Double Task Test)
Number of physical activities (LASA Physical Activity Questionnaire (LAPAQ))

Grip strength: women ≤32 kg (vs. >32 kg); men≤56 kg (vs. >56 kg) (strain-gauged dynamometer (Take

Use of sedative/hypnotic medications (Self-reported)
Dizziness (Interview)
Weakness in grasping (Questionnaire)
Pulling open a door - Movement velocity (Forceplate)

Frequency of participation in moderate physical activities (Self-reported)

Use of antihypertensives

Delta2 Minimal Toe Clearance during a dual-task walking condition (CodaMotion®, Charnwood Dynar

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Candidate predictor #25

Peak COP position as a percentage of foot length or width (%) in right direction (Force platform - MEL
Body Mass Index
Indoor fall risk factors (Home-safety checklist of indoor fall risk)
Cognitive impairment (Mini-mental state examination)

Physical activity ≥ three activities in the last 2 weeks (vs. < three activities in the last 2 weeks) (range:

Visual acuity (Snellen chart)
Psychoactive Drugs
Joint pain (Questionnaire)
Stepping into a bathtub - Anticipatory weight adjustment - time

Frequency of participation in vigorous physical activities (Self-reported)

Use of sedatives

Dual-task walking time cost (CodaMotion®, Charnwood Dynamics Ltd, Rothley UK)

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Candidate predictor #26

Peak COP position as a percentage of foot length or width (%) in left direction (Force platform - MEUC
Cognition (Mini-mental state examination)
Age
Depression (Center for Epidemiologic StudiesFDepression scale)

Cognitive impairment (MMSE score <24 vs. ≥24) (range: 0-30)

Difficulty recognizing a face across a small room (Self-reported)
Arthritis
Recent weight loss (Questionnaire)
Stepping into a bathtub - Anticipatory weight adjustment medial-lateral excursion (Forceplate)

Diabetes (Self-reported)

Vision impairment

Fast walking improvement (CodaMotion®, Charnwood Dynamics Ltd, Rothley UK)

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Candidate predictor #27

Peak COP position as a percentage of foot length or width (%) in anterior direction (Force platform - M
Perception impairments (Questionnaire)
Sex
Fear of falling (Falls Efficacy Scale)

Depression score (CES-D ≥16 vs. <16) (range: 0-60)

Difficulty in hearing to converse in a small room (Self-reported)

Needing assistance with medication administration (Comprehensive assessment tool was developed
Stepping into a bathtub - Anticipatory weight adjustment anterior-posterior excursion (Forceplate)

Lung disease (Self-reported)

Walking aid use

Age (Interview)

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Candidate predictor #28

Peak COP position as a percentage of foot length or width (%) in posterior direction (Force platform -
Blood pressure (Omron HEM 706)
Living alone
Loneliness (11-item scale De Jong Gierveld)

Fear of falling score (FES ≥1 vs. 0) (range: 0-30)

Gait speed (Fastest walking speed over 4 metres)

Level of supervision needed (Comprehensive assessment tool was developed by the Ohio Departmen
Stepping into a bathtub - Transfer of body weight to the standing leg time

Cancer (Self-reported)

Gender (Interview)

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Candidate predictor #29

Balance (Berg balance scale)
Heart rate (Omron HEM 706)
Fall history
Living alone (Questionnaire)

Loneliness score ≥5 (vs. <5) (range: 0-11) (11-item scale De Jong Gierveld)

Gait speed (Usual walking speed over 4 metres)

ıt of Aging to meet Ohio's Administrative Code)
Stepping into a bathtub - Transfer of body weight to the standing leg medial-lateral excursion (Forcep

Heart disease (Self-reported)

Leg length (Clinical assessment)

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Candidate predictor #30

Balance (Functional reach test)
Orthostatic hypotension (Omron HEM 706)

Alcohol use (Questionnaire)

Living alone (yes/no) (Questionnaire)

Balance (5 times chair stand test)

Stepping into a bathtub - Transfer of body weight to the standing leg anterior-posterior (Forceplate)

Stroke (Self-reported)

Instrumental Activities of Daily Living (Lawton IADL)

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Candidate predictor #31

Balance (Lateral reach test)
Expiratory peak flow (Mini-Wright peak flow meter)

Dogs and cats in the household (Questionnaire)

Alcohol use ≥18 consumptions per week (vs. <18 consumptions per week) (Questionnaire)

Balance (Tinetti Balance protocol)

Stepping into a bathtub - Position maintenance time

Arthritis (Self-reported)

Extrapyramidal stiffness (Unified Parkinson's disease rating scale criteria)

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Candidate predictor #32

Balance (Step-up test)
Alcohol consumption (Questionnaire)

Special adjustments in house (Questionnaire)

Current smoker (yes/no) (Questionnaire)

Frequency of difficulty balancing while walking and dressing (Self-reported)

Stepping into a bathtub - Position maintenance medial-lateral excursion (Forceplate)

Mobility and Balance (Short Physical Performance Battery Score)

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Candidate predictor #33

Smoking status (Questionnaire)

Sex Hormone Binding Globulin ≥44.5 nmol/l (vs. <44.5 nmol/l) (Blood sample)

Poor balance (Self-reported)

Stepping into a bathtub - Position maintenance anterior-posterior excursion (Forceplate)

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Candidate predictor #34

Loneliness (11-item scale De Jong Gierveld)

Insulin-like growth factor-1 ≤10.3 nmol/l (vs. >10.3 nmol/l) (Blood sample)

Difficulty walking 2-3 blocks (Self-reported)

Walk down stairs - Time from the initial unweighting of the lead leg until completion of the stair desc

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Candidate predictor #35

25-hydroxy vitamin D ≤25 nmol/l (>25 nmol/l) (Blood sample)

Cognition (MMSE)

Walk down stairs - Amount of pause in contact with the forceplate at the bottom before walking forw

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Candidate predictor #36

Albumin ≤42 g/l (>42 g/l) (Blood sample)

Physical activity defined as number of city blocks walked in the last week (Self-reported)

Walk down stairs - Centre-of-pressure excursion (Forceplate)

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Candidate predictor #37

Dogs or cats in household (yes/no) (Questionnaire)

Alcohol consumption per week (Self-reported)

Walk down stairs - Anterior-posterior moment normalised by foot length during gait initiation (Force

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Candidate predictor #38

Special adjustments in house (yes/no) (Questionnaire)

Maximum knee extensor strength (Muscle dynamometer)

plate)

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Candidate predictor #39

Presence of "fear of falling" within the previous year (Self-reported)

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Candidate predictor #40

Frequency of "fear of falling" within the previous year (Self-reported)

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Candidate predictor #41	**Candidate predictor #42**
Falling in the previous year (Self-reported)	Number of falls in the previous year (Self-reported)

For peer review only

Candidate predictor #43

For peer review only

Which statistical modeling technique was used for model development?: (choice=Linear regressior

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For peer review only

Which statistical modeling technique was used for model development?: (choice=Logistic regressic

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For peer review only

Which statistical modeling technique was used for model development?: (choice=Cox regression)

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For peer review only

Which statistical modeling technique was used for model development?: (choice=Others)

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For peer review only

Which other statistical modeling technique was used?

Tree-structured survival analysis

Only sensitivity, specificity, positive predictive value, negative predictive value, and Youden's index w

The cut-off values for differentiating between persons who fell and persons who did not were define
Tree-based classification
Classification and Regression Tree (CRT)

Only sensitivity and specificity were calculated

Logistic Tree with Unbiased Selection (LOTUS)/Logistic Regression Tree Method

Random Forrest Classification

Lasso regression model
The predictive accuracy was quantified from TP, TN, FP and FN in terms of sensitivity (Se), specificity (

Classification tree

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Were assumptions for the model checked?	Were assumptions for the model satisfied?
No information	
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Yes	Yes
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Yes	Yes
No information	
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No information	
No information	
Yes	Yes
Yes	Yes
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For peer review only

Why were assumptions for the model not satisfied?

For peer review only

Which predictor selection methods for inclusion into the multivariable analysis were used?: (choice

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For peer review only

Which predictor selection methods for inclusion into the multivariable analysis were used?: (choice

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For peer review only

Which predictor selection methods for inclusion into the multivariable analysis were used?: (choice

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For peer review only

Which other predictor selection strategy was used?:

Only potential predictors with less than 15% missing values were included. To avoid a power problem

Predictors were chosen from previous models based on expert opinion of the authors. However, univ
Predictors with a prevalence of less than 10% or predictors of which the number of missing values ex

Variables showing moderate reliability (ICC

Timed Up & Go test and gait speed were predetermined to be included in the analysis. Age and fall hi

2 out of 3 predictors were chosen for the final model. Author response: "Question 1 and 2 had redun

Because variables with low prevalence may not add meaningfully to the estimation, the study applied

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Did any statistical transformation of candidate predictors occur (i.e. dichotomising a continuous or

No
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What transformation procedure was applied?

All predictors were dichotomised
Continuous variables were dichotomised
All predictors were dichotomised

All categorical and continuous variables were dichotomised (yes/no)

Dichotomization around clinically relevant cut-points.

Falls history was dichotomised.
"Variables were evaluated for normality and normalized if necessary."

All continuous variables were split.

All continuous predictors were categorized

Categorization of continuos variables (gait speed)

Four-stage balance tests was dichotmosied based on whether participants held each stage for >10s. C

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Which predictor selection methods DURING the multivariable modelling was used? (choice=Full mc

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For peer review only

Which predictor selection methods DURING the multivariable modelling was used? (choice=Forwar

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For peer review only

Which predictor selection methods DURING the multivariable modelling was used? (choice=Backw

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For peer review only

Which predictor selection methods DURING the multivariable modelling was used? (choice=Others

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For peer review only

Which other predictor selection method was used DURING the multivariable modelling?

Both direct and sequential logistic regression were used.

Kaplan-Meier survival curves and log rank test to perform splitting and a pruning algorithm to find op

Not specified on what terms predictors were selected.
The algorithms were built by first selecting the variable most closely associated to the outcome; it the
Classification and Regression Tree (CRT)
No information

LOTUS chooses the predictor variable from the collection of numeric predictor variables that results i

Random Forrest Classification
A tiered approach was used to generate a series of prediction models by sequentially adding variable

Lasso regression
A full model approach was performed. However, three models were run with different assessments c
No information

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Which criteria/significance level were/was used for for forward selection?
Alpha = 0.01

p < 0.05
No information
)timal tree size.
P-to-enter: 0.10

P < 0.05

en identifies cut points that distinguish risk groups maximally (using chi-square st

P-to-enter = 0.05 P-to-remove = 0.10

n the model with minimum logistic deviance. This predictor variable and associat

groupings that progressed in clinical complexity, which allowed for evaluation of

)f gait/balance (TUG, SPPB, 7m gait speed test).

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Which criteria/significance level were/was used for for backward selection?

p < 0.05

tatistics).

ted logistic regression are only used if this node ends up being a terminal node in tl
No information

f the relative contribution of each set of variables to the overall predictive power o

Accuracy and Sensitivity. No criteria were reported.

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Was shrinkage of predictor weights or regression coefficients applied?

No information
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No information
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For peer review only

Which shrinkage procedure was applied? (choice=Uniform shrinkage)

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For peer review only

Which shrinkage procedure was applied? (choice=Penalized estimation)

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For peer review only

Which shrinkage procedure was applied? (choice=Others)

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For peer review only

Which shrinkage procedure was applied?

LASSO

For peer review only

Which approach was chosen to evaluate model performance? (choice=None)

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For peer review only

Which approach was chosen to evaluate model performance? (choice=Split-sample validation)

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For peer review only

Which approach was chosen to evaluate model performance? (choice=Cross-validation)

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For peer review only

Which approach was chosen to evaluate model performance? (choice=Bootstrap validation)

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For peer review only

Which approach was chosen to evaluate model performance? (choice=Jack-knife resampling)

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For peer review only

Which approach was chosen to evaluate model performance? (choice=Others)

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For peer review only

What was the percentage-wise allocation of participants to the training set?

50%

66.6%

No information

For peer review only

Was the split of the data set done randomly?

Yes

Yes

No information

For peer review only

What was the percentage-wise allocation of participants to the testing set?

50%

33.3%

No information

For peer review only

Which other approach was chosen to evaluate model performance?:

Random Forrest Classification: Training set 67% of the sample. Test set: 33% of the sample.

For peer review only

How many subsets were chosen?

N-1

No information

20

10

No information

10

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Development: Number of participants with missing data (both predictors and outcomes) in total:
No information

1

89 (lost to follow-up) + 115 (predictors)
15 (Outcome)
273 (lost to follow-up)

7

174 (165 missing responders, 6 with missing information on outcomes, 3 with missing information on
1106 (Predictors) + 273 (outcome)
Of 1734 elderly, 1378 (79%) completed the questionnaire both at the baseline study and its six month
8 (57 complete fall calendars + 1 with outlying MSL)

169

6 (Missing in table 1)

235

No information
31 (outcome)
75 (35 lost to follow-up + 40 multiple falls (hereof 32 with missing on any falls))
69 (outcome+fall history)
34 on predictors (12 non-fallers + 22 fallers did not wear accelerometer for all 3 days)

No information (at least 58 with missing data on outcome, which has to be calculated from table 3)
4 lost to follow-up
1553 (outcome)
166 (106 on outcome, 60 on predictors)

312 (during follow-up) + 4 (predictors)

217

1.633

20 (predictors) and 103 (outcome)
20 (outcome)
9 (Outcome)

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Validation: Number of participants with missing data (both predictors and outcomes) in total:

ı balance problems,)

h follow-up

18. Outcome = 6, predictors = 13 (calculated from table 2)

Predictors: ActiFE: 1,099, ELSA: 3,303, InCHIANTI: 892, TILDA: 2,101. Outcomes: ActiFE: 90, ELSA: 753

1.151

For peer review only

Development: Did participants with missing data differ from those without missing data?:
No information
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Yes
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, InCHIANTI: 263, TILDA: 271.
Yes
Yes
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No information
No information
No information

For peer review only

Validation: Did participants with missing data differ from those without missing data?:

No information

No information

No information

For peer review only

Development: How did they differ?:

Participants lost to follow-up either died, refused further participation for personal reasons, were phy

One of the participants with missing data showed extremely outlying results in step length > 2 SD. No
Women who died in the follow-up period were older and more frail. Women lost to follow-up were n

The 35 participants (7.1%) who did not complete the study were not significantly different in baseline

Twenty subjects were excluded from the analysis due to severe cognitive deterioration (Pfeiffer test >
Response from study authors: "I don't think we conducted an analysis regarding these differences bu

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Validation: How did they differ?:

ysically unable, cognitively unable, ac

information was reported for the re
nore depressed than those retained i

characteristics from those who did.

> 7 errors). Furthermore, 292 subject
t it can likely be assumed that these

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Development: Which predictors had missing data? (choice=None)

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For peer review only

Development: Which predictors had missing data? (choice=No information)

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For peer review only

Development: Which predictors had missing data? (choice=[canpre1])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre2])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre3])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre4])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre5])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre6])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre7])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre8])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre9])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre10])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre11])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre12])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre13])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre14])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre15])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre16])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre17])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre18])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre19])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre20])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre21])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre22])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre23])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre24])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre25])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre26])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre27])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre28])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre29])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre30])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre31])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre32])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre33])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre34])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre35])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre36])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre37])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre38])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre39])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre40])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre41])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre42])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre43])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre44])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre45])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre46])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre47])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre48])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre49])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre50])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre51])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre52])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre53])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre54])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre55])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre56])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre57])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre58])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre59])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre60])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre61])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre62])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre63])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre64])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre65])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre66])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre67])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre68])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre69])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre70])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre71])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre72])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre73])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre74])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre75])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre76])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre77])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre78])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre79])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre80])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre81])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre82])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre83])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre84])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre85])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre86])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre87])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre88])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre89])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre90])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre91])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre92])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre93])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre94])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre95])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre96])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre97])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre98])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre99])

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For peer review only

Development: Which predictors had missing data? (choice=[canpre100])

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For peer review only

Development: How many participants had missing data for the predictor: [canpre1]
0 (Eyes open) / 4 (Eyes closed)

For peer review only

5 had missing data on previous falls

Development: How many participants had missing data for the predictor: [canpre2]
3 (Eyes open) / 11 (Eyes closed)

For peer review only

60

1

Development: How many participants had missing data for the predictor: [canpre3]

12

1

60

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For peer review only

Development: How many participants had missing data for the predictor: [canpre4]

11 (Eyes open) / 19 (Eyes closed)

3

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5 had missing data on previous falls

For peer review only

Development: How many participants had missing data for the predictor: [canpre5]

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60

15 did not perform the TUG, the SPPB, or the walking test

For peer review only

Development: How many participants had missing data for the predictor: [canpre6]

60

15 did not perform the TUG, the SPPB, or the walking test

For peer review only

Development: How many participants had missing data for the predictor: [canpre7]

1

34

60

15 did not perform the TUG, the SPPB, or the walking test

For peer review only

Development: How many participants had missing data for the predictor: [canpre8]

34

For peer review only

Development: How many participants had missing data for the predictor: [canpre9]

3

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For peer review only

Development: How many participants had missing data for the predictor: [canpre10]

37

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For peer review only

Development: How many participants had missing data for the predictor: [canpre11]

23

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For peer review only

Development: How many participants had missing data for the predictor: [canpre12]

4

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For peer review only

Development: How many participants had missing data for the predictor: [canpre13]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre14]

15

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For peer review only

Development: How many participants had missing data for the predictor: [canpre15]

3

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For peer review only

Development: How many participants had missing data for the predictor: [canpre16]

1

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For peer review only

Development: How many participants had missing data for the predictor: [canpre17]

5

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For peer review only

Development: How many participants had missing data for the predictor: [canpre18]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre19]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre20]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre21]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre22]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre23]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre24]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre25]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre26]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre27]

28

For peer review only

Development: How many participants had missing data for the predictor: [canpre28]

119

For peer review only

Development: How many participants had missing data for the predictor: [canpre29]

1

For peer review only

Development: How many participants had missing data for the predictor: [canpre30]

3

For peer review only

Development: How many participants had missing data for the predictor: [canpre31]

2

For peer review only

Development: How many participants had missing data for the predictor: [canpre32]

1

For peer review only

Development: How many participants had missing data for the predictor: [canpre33]

121

For peer review only

Development: How many participants had missing data for the predictor: [canpre34]

123

For peer review only

Development: How many participants had missing data for the predictor: [canpre35]

122

For peer review only

Development: How many participants had missing data for the predictor: [canpre36]

117

For peer review only

Development: How many participants had missing data for the predictor: [canpre37]

For peer review only

Development: How many participants had missing data for the predictor: [canpre38]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre39]

For peer review only

Development: How many participants had missing data for the predictor: [canpre40]

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Development: How many participants had missing data for the predictor: [canpre41]

2

For peer review only

Development: How many participants had missing data for the predictor: [canpre42]

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For peer review only

Development: How many participants had missing data for the predictor: [canpre43]

For peer review only

Validation: Which predictors had missing data? (choice=None)

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For peer review only

Validation: Which predictors had missing data? (choice=No information)

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre1])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre2])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre3])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre4])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre5])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre6])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre7])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre8])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre9])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre10])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre11])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre12])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre13])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre14])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre15])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre16])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre17])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre18])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre19])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre20])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre21])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre22])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre23])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre24])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre25])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre26])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre27])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre28])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre29])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre30])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre31])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre32])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre33])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre34])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre35])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre36])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre37])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre38])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre39])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre40])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre41])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre42])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre43])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre44])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre45])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre46])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre47])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre48])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre49])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre50])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre51])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre52])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre53])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre54])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre55])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre56])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre57])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre58])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre59])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre60])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre61])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre62])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre63])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre64])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre65])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre66])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre67])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre68])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre69])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre70])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre71])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre72])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre73])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre74])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre75])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre76])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre77])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre78])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre79])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre80])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre81])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre82])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre83])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre84])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre85])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre86])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre87])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre88])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre89])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre90])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre91])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre92])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre93])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre94])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre95])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre96])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre97])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre98])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre99])

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For peer review only

Validation: Which predictors had missing data? (choice=[canpre100])

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For peer review only

Validation: How many participants had missing data for the predictor: [canpre1]

13

For peer review only

Validation: How many participants had missing data for the predictor: [canpre2]

ActiFE: 115/1416, ELSA: None, InCHIANTI: None, TILDA: 4/2101

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Validation: How many participants had missing data for the predictor: [canpre3]

ActiFE: 74/1416, ELSA: 1/3303, InCHIANTI: 22/892, TILDA: 32/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre4]

ActiFE: 4/1416, ELSA: None, InCHIANTI: None, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre5]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre6]

ActiFE: 14/1416, ELSA: 56/3303, InCHIANTI: 59, TILDA: 4/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre7]

ActiFE: 20/1416, ELSA: 1/3303, InCHIANTI: 1/892, TILDA: 2/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre8]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre9]

ActiFE: 42/1416, ELSA: 268/3303, InCHIANTI: 71/892, TILDA: 387/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre10]

ActiFE: 2/1416, ELSA: None, InCHIANTI: 52/892, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre11]

ActiFE: 18/1416, ELSA: 13/3303, InCHIANTI: None, TILDA: 1/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre12]

ActiFE: 6/1416, ELSA: None, InCHIANTI: 2/892, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre13]

ActiFE: 27/1416, ELSA: 1/3303, InCHIANTI: None, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre14]

ActiFE: 23/1416, ELSA: None, InCHIANTI: None, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre15]

ActiFE: None, ELSA: 3303/3303, InCHIANTI: None, TILDA: 17/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre16]

ActiFE: 9/1416, ELSA: 13/3303, InCHIANTI: 4/892, TILDA: 2/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre17]

ActiFE: None, ELSA: None, InCHIANTI: 7/892, TILDA: 2101/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre18]

ActiFE: 202/1416, ELSA: 3/3303, InCHIANTI:3/892 , TILDA: 11/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre19]

ActiFE: 14/1416, ELSA: 1/3303, InCHIANTI: None, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre20]

ActiFE: 7/1416, ELSA: 3303/3303, InCHIANTI: 23/892, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre21]

ActiFE: 7/1416, ELSA: None, InCHIANTI: None, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre22]

ActiFE: 20/1416, ELSA: 3/3303, InCHIANTI: None, TILDA: 11/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre23]

ActiFE: None, ELSA: 3303/3303, InCHIANTI: None, TILDA: 2101/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre24]

ActiFE: None, ELSA: 3303/3303, InCHIANTI: None, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre25]

ActiFE: None, ELSA: 3303/3303, InCHIANTI: None, TILDA: 2101/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre26]

ActiFE: 24/1416, ELSA: None, InCHIANTI: 130/892, TILDA: 487/2101

For peer review only

Validation: How many participants had missing data for the predictor: [canpre27]

ActiFE: 120/1416, ELSA: 33/3303, InCHIANTI: 57/892, TILDA: None

For peer review only

Validation: How many participants had missing data for the predictor: [canpre28]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre29]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre30]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre31]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre32]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre33]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre34]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre35]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre36]

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Validation: How many participants had missing data for the predictor: [canpre37]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre38]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre39]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre40]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre41]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre42]

For peer review only

Validation: How many participants had missing data for the predictor: [canpre43]

For peer review only

Method of handling missing data: (choice=Single imputation)

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For peer review only

Method of handling missing data: (choice=Multiple imputation)

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For peer review only

Method of handling missing data: (choice=Participants with missing data were excluded from the a

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For peer review only

Method of handling missing data: (choice=Others)

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For peer review only

Method of handling missing data: (choice=None)

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For peer review only

Method of handling missing data: (choice=No information)

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What method for handling missing data was used?:

The one participant with missing data, had missing data for the outcome in the last month of follow-u
Participants lost to follow-up were excluded from the analyses. It was not reported how missing data

Predictors of which the number of missing values exceeded 10% were excluded. It was not reported h

Missing predictor values were minimal (< 1%), and were incorporated. However, 169 participants wer
Based on author response: "I believe this is described in the methods section, but in general we used

Data from participants who died or had another adverse event were included in analysis unless the p

Missing values on frailty markers were not imputed by multiple imputation but counted as "not prese

Participants with missing outcomes were excluded from the analysis.

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Were participants with missing data included in the model development, validation or updating?:

No
No information
No information
No
Yes
No information
No information
Yes
No
No
Yes
No information
No
No information
No
No
No
No information
No
No information
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No information
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Yes
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No
No
No
No

For peer review only

Sample size:

100
100
1.374
302
1365
106
510
1.365
1.734
65
1002
500
23.417
74
1759
492
9.661
71
113
1509
66
7.609
5.994

ActiFE: 1,416, ELSA: 3,303, InCHIANTI: 892, TILDA: 2,101

772
7.609

Development: 5,593 (3,960 at follow-up); validation: 4,424 (3,273 at follow-up)

541
325
105

For peer review only

Specificy the gender distribution including percentage proportion of the sample: Example: Femal

Female: 83 (83%) Male: 17 (17%)
Female: 100 (100 %)
Of the 1,285 with complete follow-up: Female: 656 (51%) Male: 629 (49%)
Of the 287 with complete follow-up Female: 172 (60%) Male: 115 (40%)
Female: 698 (51%) Male: 667 (49%)
Of 99 participants included in analysis: Female: 60 (61%) Male: 39 (39 %)
Of the 345 included: Female: 189 (55%) Male: 156 (45%)
Female: 698 (51.1%) Male: 667 (48.9%)

Female: 32 (57%) Male: 24 (43%)
n = 885 Female: 885 (100%)
Female: 270 (54 %) Male: 230 (46 %)
Female: 18,499 (79%) Male: 4,918 (21%)
No information
Female: 897 (51 %) Male: 862 (49 %)
Of the 457 with complete follow-up (except n = 18 who died during follow-up) Female: 199 (43.5%) M
Of the 9,592 included in the analyses: Female: 5,545 (57.8%) Male: 4,047 (42.3%)
Female: 46 (65%) Male: 25 (35%)
Female: 37 (34.6 %) Male: 70 (65.4 %)
Female: 781 (51.8%) Male: 728 (48.2%)
Female: 41 (66%) Male: 21 (33%)
Female: 4,307 (56.6 %) Male: 3,302 (43.4 %)
Male: 5,994 (100%)
ActiFE: Female: 804 (56.8%) Male: 613 (43.2%) ELSA: Female: 1,873 (56.7%) Male: 1,430 (43.4%) InCH
Gender of participants completing the first follow-up period: Female: 376 (62.5%) Male: 226 (37.5%)
Female: 4,170 (56.4%) Male: 3,439 (43.6%)
Development: Female: 3,267 (58.4 %) Male: 2,326 (41.6 %) Validation: Female: 2,540 (57.4 %) Male: 1
Of the 438 participants with a complete data set: Female: 266 (60.7%) Male: 172 (39,3%)
Of the 305 analysed: Female: 171 (56.1%) Male: 134 (43.9%)
Of the 96 analysed: Female: 48 (50%) Male: 48 (50%)

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What was the age of the sample participants? Examples of reporting depending on units: Mean (

Mean (SD): 83 (6) yr

Mean (SD): 71 (5) yr

Mean (SD): 75.2 (6.5) years Age range: 64.8-88.6 years

Of the 287 with complete follow-up Female mean age (SD): 78.5 (5.2) years Male mean age (SD): 77.2

Mean (SD): 75.3 (6.4) years Range: 64.8-88.6 years

Mean (SD): 74.02 (5.64) yr Range: 65-90 yr

Of the 345 included: Mean (SD): 74.4 (6.4) yr

Mean (SD): 75.3 (6.4) years Age range: 64.8-88.6 years

Mean (SD): 75.8 (6.8) yr

Mean (SD): 67.7 (6.0) yr

Mean (SD): 78 (8.1)

Mean (SD): 77.9 (4.1) yr

Mean (SD): 76.1 (8.94) years

Non-fallers: Range: 65-87 yr Mean (SD): 75.1 (6.5) yr Fallers: Range: 70-94 yr Mean (SD): 80.1 (6.2) yr

Mean (SD): 70.7 yr (4.6)

Mean (SD): 74 (5.7)

Mean (SD): 74.2 (7.16) years

Mean (SD): 78.36 (4.71) years

Mean: 79.8 years

Median (Range): 75.6 yr (64.8-88.8)

Non-fallers: Mean (SD): 75 yr(11.5) Fallers: Mean (SD): 82 yr (12)

Age groups: Proportion (confidence interval) - 65-69: 27.9 % [27.0, 29.0] - 70-74: 25.0 % [24.1, 25.8] -

Mean: 74 yr. (Based on other studies on the same cohort)

ActiFE: Mean (SD): 75.7 (6.76) yr ELSA: Mean (SD): 74.56 (7.31) yr InCHIANTI: Mean (SD): 73.78 (6.62)

Age of participants completing the first follow-up period: Median (SD): 80.7 (0.1) years

Proportions: 65-69 yr: 2,099 (28.4 %) 70-74 yr: 1,863 (25.2 %) 75-79 yr: 1,427 (19.3 %) 80-84 yr: 1,079

Development: Median (IQR): 77 (71, 83) Validation: Median (IQR): 78 (71, 83)

Mean (SD): 82.4 (6.5) yr

Mean (SD): 67.55 (5.5) yr

Mean (SD): 71.3 (5.4) yr Range: 65-89 yr.

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What was the number of participants with prior falls?:

45
35
398
199
424 (231 single fallers + 193 recurrent fallers)
50
86
2 or more falls in the past year: 194
107
25
279
148
703
47
467
135
2.974
32
53
486
28
2.321
1,268 (Based on other studies on the same cohort)
ActiFE: 511, ELSA: 750, InCHIANTI: 186, TILDA: 479
159
No information
No information
118
51
0

For peer review only

What was the percentage proportion of participants with prior falls?:

46.8%

35%

31%

66%

31%

50.5%

25%

14.2%

16%

45%

34% (of n = 830)

29.6%

3%

63.5%

26%

29.7%

31%

45%

49.5%

32.2%

45%

30.5%

21% (Based on other studies on the same cohort)

ActiFE: 36.1%, ELSA: 22.7%, InCHIANTI: 20.8%, TILDA: 22.8%

26.4%

No information

No information

27%

16.7%

0%

For peer review only

Number of falls:

120

82

No information

197

No information

90

No information

2.57

No information

Based on author response: "Data are not available any more to summarise this".

No information

478

No information

No information

No information

No information

No information

No information

No information

No information

25

No information

Based on author response, this was not possible to report.

No information

No information

Based on author response: "As reported in the manuscript - "The primary analytical outcome was self

No information

No information

No information

No information

For peer review only

Number of fallers?:
59 (59%)
35 (35%)
428 (31.1%)
95 (31.5%)
No information
42 (39.6%)
No information
755 (55.3%)
208 (12.0%)
30 (46.2%) (54% of 56)
346 (34.5%) (39% of n = 885)
214 (42.8%)
Approximately 1,400 (5.9%)
At least 49 (66.2% - missing information on single fallers)
563 (32%)
187 (38%)
3,299 (34%)
No information
33 (29.2%)
468 (31.0%)
13 (19.7%)
2,028 (26.7%)
Based on author response, this was not possible to report.
ActiFE: 466 (32.9%) ELSA: 730 (22.1% 1 yr adjusted) InCHIANTI: 203 (22.8%) TILDA: 569 (27.1% 2 yr ac
114 (original data) / 106,94 (weighted sample) (Answer from study author)
3,903 (51.3%)
No information
87 (16.1%)
81 (24.9%)
35 (33.3%)

For peer review only

Number of non-fallers?:

37
65
857
192
1.028
57

No information

590
1.17
26
539
328

Approximately 22,000

25
1.196

305 (62%)

6.293
59
74
983
49
5.581

Based on author response, this was not possible to report.

ActiFE: 950 (67.1%) ELSA: 2,573 (77,9% 1 yr adjusted) InCHIANTI: 689 (77.2%) TILDA: 1,532 (72.9% 2 y

438 (original data) / 502,76 (weighted sample) (Answer from study author)

3.488

No information

351
224
61

For peer review only

Number of frequent fallers?:
23 (23%)
16 (16%)
147 (10.7%)
46 (15.2%)
337 (24.7%)
20 (18.9%)
No information
457 (33.5%)
103 (5.9%)
No information
No information
94 (18.8%)
No information
40 (54.1%)
222 (12.6%)
74 (15%)
No information
12 (16.9%)
14 (12.4%)
174 (11.5%)
No information
957 (12.6%)
694 (11.6%)
No information
43 (9.9%)
2,181 (28.7%)
Development: 834 (14.9%) Validation: 514 (11.6%)
34 (6.3%)
No information
No information

For peer review only

Fall-rate per person per year?:
No information
No information
No information
No information
No information
0.91
No information
No information
No information
Based on author response: "This was not calculated".
No information
Based on author response, this was not calculated.
No information
No information
No information
No information
No information
No information
No information
No information
No information
No information
Based on author response, this was not possible to report. Answer: "No because we collect falls as 0-
No information
Author response: "We had 53,5 falls per 100 person-year".
Based on author response: "No fall-rate was calculated".
No information
No information
No information
No information

For peer review only

Number of outcomes (falls) in relation to number of candidate predictors (events per variable - EPV

59 / 71 = 0.8

35/32 = 1.1

Any fall: 12.6 (428 outcomes / 34 predictors) Recurrent falls: 4.3 (147 outcomes /34 predictors)

1.3 (46/35)

10.5 (337 outcomes/32 predictors)

Recurrent falls: 42/23 = 1.8

No information

EPV: 457/38 = 12

Fallers: 208/22 = 9.5

30/4 = 7.5

Assuming no categorical variables with 3 or more levels: 346/42 = 8.2

166/28 = 5.9

Approximately 50 (1,400 events/28 predictors)

Scores are calculated before modelling, however these are not reported. Based on number of candid

563/30 = 18.8

Any fall: 29 (174/6) Multiple falls: 11.5 (69/6)

3,299/39 = 84.6

0.52 (12 events/21 predictors)

Validation: at least 100 events needed. Number of events = 33.

Any falls: 468/9 = 52 Recurrent falls: 174/9 = 19

0.26

Fallers: 2028/26 = 78 Recurrent falls: 957/26 = 36.8

Age including Baumgartner, Newman, or IWG (2 parameters): 347. Age including EWGSOP, FNIH#1, c

Not relevant for validation studies

43/4 = 10.8

3,903/23 = 169.7 (STEADI is a 3 level variable constructed from 7 dichotomous variables)

834/580 = 1.44, assuming all variables are dichotomous, otherwise no information available.

At least 12.4 (87/7)

81/19 = 4.26

35/255 = 0.14 Continuous predictors (29) Lawton (7) Stiffness score (198) SPPB (11)

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How many participants completed follow-up percentage-wise?:

96%
99%
93.5%
95%
80%
93.4%
67.6%
80%
79%
95%
88.3 %
98.8% Based on author response: "Only 6 people were lost to follow up, all others had full data for
No information
77%
98.26 %
92.9%
99.3%
No information
94.6% returned fall calendars based on elegible sample size of 113. 125 were included in the study, b
100%
94%
79.6%
5828/5934 = 98.2%
ActiFE: 90 people (6%) on whom follow-up information about falls was missing were excluded. ELSA:
61.17%
Based on author response: "Not all participants completed follow-up".
Development: 3,960/5,593 = 70.8% Validation: 3,273/4,424 = 74.0%
80.9%
93.8%
91.5%

For peer review only

Summary of follow-up period?: Mean (SD): 10 (0.1) months Median (IQR) 10 (9.5-11.5) months
No information
No information
No information
No information
No information
12 Months (7 excluded due to incomplete follow-up)
6 months
No information. "Of the 273 persons with one or more 3-month periods missing from the 'fall' calend
No information
12 months.
12 months
Based on author response: "Only 6 people were lost to follow up, all others had full data for 12 mont
No information
No information
No information
No information
No information on the 0.7% not completing follow-up. All others had 2 years of follow-up
No information
Mean: 24.4 days.
12 Months
No information
No information
12 months
No information
Median: 10.66 months (SD 2.62)
No information
No information
No information
Of the 305 analysed: 6 months
For the remaining 96: 2 years.

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How many models were developed/validated?

2
7
2
1
2
1
1
2
1
2
2
1
1
7
1
3
1
2
1
4
1
8
6
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3
1
6
2
1

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Model 1: What format did the study present the model in to permit calculations of individual risks

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Model 1: What format did the study present the model in to permit calculations of individual risks

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Model 1: Please write the regression formula:

Any fall: Regression table with regression coefficients - Intercept: Not reported - Previous falls: 0.90 -
Regression table: Regression coefficients with standard error Intercept: -2.28 Female gender: -0.39 (C

Regression table: Coefficient (SE) - Intercept: No information - Postural sway while standing on a firm

Recurrent falling within 3 years: Regression coefficient - Constant: - 2.19 - Two or more previous falls:

Regression table with coefficients: Constant: None Fall history: 0.997 Female: 0.133 Age: 0.013 Blacks

Regression table (coefficient): - Intercept/baseline hazard: No information - Women 0.506 - Living alc

Age, sex, Combined comorbidity index, 52 ICD9-Codes, 25 CPT Codes and 16 HCPCS codes

Constant: -5.03 Age: -0.003 Gender: 0.19 Medication: -0.24 Primary education: -0.27 Secondary educ

For peer review only

Model 1: Please write the rounded scoring rule:

Any fall: Score chart - Previous falls: 5 - Urinary incontinence: 2 - VIsual impairment: 2 - Use of benzoc
Rounded scoring rule: Men Age >= 80 years: Was not included due to low impact. Falls >= 2 in previou

ı surface with eyes closed: 1.786 (1.332) - Age: 0.072 (0.048) - Sex: 0.822 (0.540)

Recurrent falling within 3 years: Score - Two or more previous falls: 4 - Dizziness: 4 - Functional limitat
They made a score from 0-13 based on the odds ratio at an integer level from logistic regression, Thu

s: -0.334 Others: -0.363 Married: -0.137 Alzheimer's disease: 0.055 Cancer: -0.307 Cataracts: -0.21 Der

Scoring rule (point): - Women (2) - Living alone (1) - Self-reported osteoarthritis (1) - History of falls - 1

ation: -0.85 History of falls: 0.67 Walk While Talking Test: 0.68 Gait speed: 0.25 IADL: -0.01 TUG: 0.14

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Model 1: Please specify the defined risk groups:

diazepines: 2

us year: 6 Depression - SCL90 >= 22: 4 Hand dynamometry (Men <= 22 kg or Women: <= 12 kg: 6 Postu

Length of follow-up: 3 years - Predictors: - Fall history - Function limitations - Dizziness - Performance

Three or more of the following risk factors: - history of falling in the previous year, taking four or mor

tions: 3 - Weak grip strength: 3 - Low body weight: 2 - Fear of falling: 2 - The presence of dogs/cats in

s: - History of falls: probable score 0/5 - Do you feel your walking speed has declined recently: probab

Low risk: - Physiological fall risk (PPA) <0.60 - Subgroups were made from the Disability score >0 - If D

mentia: -0.135 Depression: 0.334 Diabetes: -0.034 Emphysema: -0.171 Glaucoma: 0.118 Incontinence

Low risk: - Score: 0-3 - Frequencies: 55.3 - OR: 1 Moderate risk: - Score: 4-6 - Frequencies: 34.9 - OR 2

LOTUS/Logistic regression tree diagram: - No fall history + Age <= 77 yr + Difficulty with knees: 21.2%

STEADI algorithm: Low risk: All present - No falls in past year - No worrying about falling, - No unsafe/

PPA: 1.16

For peer review only

Model 1: Were the risk groups predefined rather than derived from the dataset used for developm

ural sway abnormal: 7 Rounded scoring rule: Women Age >= 80 years: Was not included due to low in

No

No

the household: 2 - A high education level: 1 - The drinking of 18 or more alcoholic consumptions per v

ole score 0/2 - Do you use cane when you walk: probable score 0/2 - Is your back bended: probable sc

No

: 0.189 Parkinson's disease: 0.261 Stroke: 0.103 Vertigo: 0.085 Total number of medications: 0.029 AD

No

No

Yes

For peer review only

Model 1: Please specify what other format the model was presented in:

npact. Falls >= 2 in previous year: 4 Depression - SCL90 >= 22: 2 Hand dynamometry (Men <= 22 kg or

week: 1 - Interaction term (high education × 18 or more alcohol consumptions per week): 4 - Interacti
ore 0/2 - Do you take more than five kinds of prescription medicines: probable score 0/2

Decision tree

DL score: -0.07 IADL score: 0.01 Gait-shuffling: 0.027 Gait-unsteady: 0.178 Grasp-tremors: 0.426 Grasp

Table of predictors with corresponding Mean Decrease in Accuracy, and Mean Decrease in Gini

A table with classification measures for the model with 3 different cut-off values for the TUG-test

Classification tree

For peer review only

Model 1: Please specify measures of overall performance (e.g. R2, Brier score etc.)

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No information
Cox-Snell R2: 0.21 Nagelkerke R2: 0.31
No information

For peer review only

Model 1: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

AUC (SE): 0.76 (0.05)
No information
AUC: 0.65
AUC: 0.79
No information
No information
No information
AUC (CI): 0.71 (0.67-0.74)
AUC (95% CI): 0.74 (0.69-0.79)
Based on author response: "The information in the paper is the only we can provide. New calculation
No information
Based on author response: "We didn't calculate an AUC or related measure for our tree and our samp
AUC: 0.61
No information
AUC (CI): 0.70 (0.67-0.73)
AUC: 0.60
AUC: 0.71
No information
AUC (95% CI): 0.66 (0.55-0.78)
AUC for time to second fall (recurrent falls) in Cox regression = 0.58 (0.53-0.62).
No information
Any fall: AUC (95% CI): 0.57 (0.54-0.60)
Change in AUC compared to age alone (AUC: 0.577): 0.000 (-0.002; 0.003)
ActiFE: AUC (95% CI): 0.562 (0.530 - 0.594) ELSA: AUC (95% CI): 0.699 (0.680 - 0.718) InCHIANTI: AUC
AUC (95% CI): 0.74 (0.66-0.82)
AUC: 0.641 (Based on author response: "No confidence interval was calculated")
Development: C-statistic: 0.62-0.66 Validation C-statistic: Consistent with C-statistic in developmenta
No information
No information
AUC: 0.84

For peer review only

Model 1: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte
No information
No information
No information
The comparison of the percentages predicted probability with the percentage of observed recurrent
No information
No information
No information
The Hosmer-Lemeshow goodness-of-fit test for the multiple logistic regression was not significant (p=
No information
Based on author response: "The information in the paper is the only we can provide. New calculation
No information
Based on author response: "We didn't calculate an AUC or related measure for our tree and our samp
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Based on author response: "We did not generate calibration plots for these analyses, just the discrim
Calibration plots were displayed for all four cohorts. ActiFE and InCHIANTI: Participants with low (high
After contacting study authors, a calibration plot was provided showing acceptable calibration. Autho
No information
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Hosmer and Lemeshow test result confirmed that the model was a good fit for the data $\chi 2(df = 8, N =$
No information

For peer review only

Model 1: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 78 % (43/55) SP: 50 % (18/36)

SN: 51% SP: 91% Total predictive ability: 77%

No information

Cut-off: 0.30 SN: 59% SP: 87% PPV: 52% NPV: 90%

No information

% Correct prediction: 80.8%, though only predicted 2/20 of multiple fallers. SN: 10% SP: 98.7%.

SN: 0.42 (0.32-0.54) SP: 0.92 (0.88-0.94) PPV: 0.57 (0.43-0.69) NPV: 0.86 (0.83-0.89) Youden's Index: (

Cut-off: 5 points SN: 59% SP: 71.4% PPV: 38.6% NPV: 85.1%

Cut-off: 6 SN: 68% SP: 70% PPV: 27.9% Negative predictive power: 93%

SN (CI): 93 % (86.7-100) SP (CI): 54 % (40.5-67.1) PPV (CI): 70 % (57.8-82.2) NPV (CI): 88 % (78.7-96.3)

Fall probability threshold: >= 0.34 SN: 0.59 SP: 0.64 PLR: 1.64 NLR: 0.64 Diagnostic Odds Ratio: 2.56 Fa

Based on author response, no other classification measures were calculated.

No information

Extracted from figure: SN: 35% SP: 65%

Youden index = 3, for this: SN: 70.2%, SP: 60.3 % PPV: 45.5% NPV: 81.1%. Evolution of PPV and NPV c

No information

No information

SN: 37.6% SP: 89.5% "84.2% of the subjects were successfully identified as future fallers and non-falle

Cut-off: 14 points SN: 72.5 % SP: 52.2 % PPV: 39.6 % NPV: 81.4 %

No information

Out-of-bag error rate: 27.4% Correct classification: 72.6%

No information

NRI events: 0.12 (0.08, 0.16) NRI non-events: -0.12 (-0.14, -0.11)

No information.

Cut-off: 3 points SN (95% CI): 70% (56%-84%) SP (95% CI): 72% (68%-76%)

SN: 65 % SP: 65 % PPV: 62 % NPV 68 %

No information

TUG > 12s: SN: 36.5% (22.3%-54.5%) SP: 82.5% (76.9%-87.1%) PPV: 25.5% (16.8%-37.6%) NPV: 88.8%

Accuracy: 76.6%

SN: 80% SP: 87% PPV: 78% NPV: 88%

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Model 2: What format did the study present the model in to permit calculations of individual risks

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Model 2: Please write the regression formula:

Recurrent falls: Regression table with regression coefficients - Intercept: Not reported - Previous falls

Recurrent falling within 1 year: Regression coefficient - Constant: - 3.13 - Two or more previous falls: (

Any fall: Tier 2 Model y = -1.44 + ((Age 70-74 yr)* -0.33) + ((Age 75-59 yr)*0.07) + ((Age 80-84 yr)*0.17

Constant: -5.06 Age: 0.05 Gender: 0.46 Medication: -0.14 Primary education: 0.07 Secondary educati

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Model 2: Please write the rounded scoring rule:

Recurrent falls: Score chart - Previous falls: 5 - Urinary incontinence: 3 - VIsual impairment: 4 - Use of

0.64 - Dizziness: 0.52 - Functional limitations: 0.39 - Weak grip strength: 0.65 - Low body weight: 0.32

7) + ((Age 85-89 yr)*0.37) + ((Age 90+ yr)*0.26) + Female*0.12 + (Black ethnicity * -0.27) + (Other ethr

on: 0.85 History of falls: 0.12 Walk While Talking Test: 0.23 Gait speed: -0.07 IADL: -0.03 TUG: 0.16

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Model 2: Please specify the defined risk groups:

benzodiazepines: 3

Length of follow-up: 1 years - Fall history - Function limitations - Grip strength Classification tree Leng

- Fear of falling: 0.09 - The presence of dogs/cats in the household: 0.81 - A high education level: 0.08

nicity * -0.52) + (Hispanic ethnicity * 0.07) + Self-reported balance problems*0.69 + Fall history*1.15

For peer review only

Model 2: Were the risk groups predefined rather than derived from the dataset used for developm

No

3 - The drinking of 18 or more alcoholic consumptions per week: - 0.15 - Interaction term (high educati

Recurrent fall: Tier 2 Model y = 2.67 + ((Age 70-74 yr)* -0.66) + ((Age 75-59 yr)*-0.08) + ((Age 80-84 y

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Model 2: Please specify what other format the model was presented in:

ion × 18 or more alcohol consumptions per week): 0.87 - Interaction term (two or more previous falls

Decision tree

yr)*0.11) + ((Age 85-89 yr)*0.49) + ((Age 90+ yr)*0.47) + Female*-0.22 + (Black ethnicity * -0.27) + (Ot

A table with classification measures for the model with 2 different cut-off values for the SPPB.

For peer review only

Model 2: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

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Cox-Snell R2: 0.07 Nagelkerke R2: 0.95

For peer review only

Model 2: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

AUC (SE): 0.87 (0.05)

No information

AUC 0.71

No information

AUC (CI): 0.72 (0.67- 0.77)

No information

No information

No information

AUC: 0.729

No information

AUC for any falls in logistic regression: 0 vs >=1 falls: 0.51 (0.346-0.56)

Any falls: Developmental data set: AUC (95% CI): 0.69 (0.67-0.71) Validation data set: AUC (95% CI): Change in AUC compared to age alone (AUC: 0.577): 0.001 (-0.002; 0.003)

AUC: 0.575 (Based on author response: "No confidence interval was calculated")

No information

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For peer review only

Model 2: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

No information
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The Hosmer- Lemeshow goodness-of-fit test for the multiple logistic regression was not significant (p

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Based on author response: "We did not generate calibration plots for these analyses, just the discrim

No information

No information

Hosmer and Lemeshow test result confirmed that the model was a good fit for the data $\chi 2$ (df = 8, N =

For peer review only

Model 2: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 53 % (9/17) SP: 89 % (31/35)

SN: 34% SP: 89% Total predictive ability: 70%

Cut-off: 7 points SN: 54% SP: 79% PPV (CI): 24.9% (22.5-27.3%) NPV (CI): 93% (91.6-94.4%)

No information

No information

SN (CI): 90 % (82-98) SP (CI): 58 % (44.5-70.9) PPV (CI): 71 % (58.9-83.2) NPV (CI): 83 % (73.4-93.3) LR+

Fall probability threshold: >= 0.25 SN: 0.78 SP: 0.46 PLR: 1.44 NLR: 0.48 Diagnostic Odds Ratio: 3.02 Fa

Extracted from figure 2: SN: 52% SP: 55%

No information

SN: 75% SP: 100% "94.7% of the subjects were successfully identified as future fallers and non-fallers'

No information

No information

NRI events: 0.07 (0.04, 0.11) NRI non-events: -0.08 (-0.09, -0.06)

No information

SPPB < 9: SN: 37.2% (24.1%-54.1%) SP: 83.4% (78.7%-87.7%) PPV: 27% (17.8%-37.6%) NPV: 89% (83.3

Accuracy: 74.1%

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Model 3: What format did the study present the model in to permit calculations of individual risks

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Model 3: What format did the study present the model in to permit calculations of individual risks

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Model 3: What format did the study present the model in to permit calculations of individual risks

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Model 3: What format did the study present the model in to permit calculations of individual risks

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Model 3: What format did the study present the model in to permit calculations of individual risks

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Model 3: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Model 3: Please write the regression formula:

For peer review only

Model 3: Please write the rounded scoring rule:

For peer review only

Model 3: Please specify the defined risk groups:

For peer review only

Model 3: Were the risk groups predefined rather than derived from the dataset used for developm

For peer review only

Model 3: Please specify what other format the model was presented in:

A table with classification measures for the model with 2 different cut-off values for the 7m gait spee

For peer review only

Model 3: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

No information

No information

No information

No information

No information

No information

No information

No information

For peer review only

Model 3: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

No information

No information

AUC: 0.786

AUC for recurrent falls in logistic regression: 0-1 vs >=2 falls: 0.50 (0.42-0.57)

Any falls: AUC (95% CI): 0.71 (0.69-0.73)
Change in AUC compared to age alone (AUC: 0.577): 0.010 (0.002; 0.018)

AUC: 0.668 (Based on author response: "No confidence interval was calculated")

No information

For peer review only

Model 3: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

No information

No information

No information

No information

No information

Based on author response: "We did not generate calibration plots for these analyses, just the discrim

No information

No information

For peer review only

Model 3: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 23% SP: 88% Total predictive ability: 65 %

Extracted from figure 2: SN: 55% SP: 55%

No information

No information

No information
NRI events: -0.33 (-0.38, -0.28) NRI non-events: 0.34 (0.32, 0.35)

No information

Any fall: Gait speed < 0.8 m/s: SN: 35.1% (22.6%-52.5%) SP: 84.3% (78.8%-88.6%) PPV: 26.9% (17.7%-

For peer review only

Model 4: What format did the study present the model in to permit calculations of individual risks

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Model 4: What format did the study present the model in to permit calculations of individual risks

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Model 4: What format did the study present the model in to permit calculations of individual risks

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Model 4: What format did the study present the model in to permit calculations of individual risks

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Model 4: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Model 4: Please write the regression formula:

For peer review only

Model 4: Please write the rounded scoring rule:

For peer review only

Model 4: Please specify the defined risk groups:

For peer review only

Model 4: Were the risk groups predefined rather than derived from the dataset used for developm

For peer review only

Model 4: Please specify what other format the model was presented in:

A table with classification measures for the model with 3 different cut-off values for the TUG-test

For peer review only

Model 4: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

No information

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For peer review only

Model 4: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

No information

No information

AUC for recurrent falls in logistic regression: 0-2 vs >=3 falls: 0.49 (0.39-0.59)

Any falls: AUC (95% CI): 0.72 (0.70-0.73)
Change in AUC compared to age alone (AUC: 0.577): 0.009 (0.002; 0.015)

No information

For peer review only

Model 4: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

No information

No information

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No information

Based on author response: "We did not generate calibration plots for these analyses, just the discrim

No information

For peer review only

Model 4: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 29% SP: 88% Total predictive ability: 67 %

No information

No information

No information
NRI events: -0.33 (-0.38, -0.28) NRI non-events: 0.35 (0.34, 0.36)

Multiple falls: TUG > 12 s: SN: 56.2% (32.2%-92.8%) SP: 82.1% (76.9%-86.6%) PPV: 16.8% (8.9%-27.8%

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Model 5: What format did the study present the model in to permit calculations of individual risks

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Model 5: What format did the study present the model in to permit calculations of individual risks

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Model 5: What format did the study present the model in to permit calculations of individual risks

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Model 5: Please write the regression formula:

For peer review only

Model 5: Please write the rounded scoring rule:

For peer review only

Model 5: Please specify the defined risk groups:

For peer review only

Model 5: Were the risk groups predefined rather than derived from the dataset used for developm

For peer review only

Model 5: Please specify what other format the model was presented in:

A table with classification measures for the model with 2 different cut-off values for the SPPB.

For peer review only

Model 5: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

No information

No information

No information

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For peer review only

Model 5: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

No information

No information

Recurrent falls: AUC (95% CI): 0.59 (0.56-0.61)
Change in AUC compared to age alone (AUC: 0.577): 0.004 (-0.001; 0.008)

No information

For peer review only

Model 5: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

No information

No information

No information

Based on author response: "We did not generate calibration plots for these analyses, just the discrim

No information

For peer review only

Model 5: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 6% SP: 97% Total predictive ability: 65%

Extracted from figure 2: SN: 78% SP: 55%

No information
NRI events: -0.11 (-0.14, -0.08) NRI non-events: 0.07 (0.06, 0.08)

SPPB < 9 SN: 56.2% (32.1%-93.2%) SP: 82.9% (78.1%-87%) PPV: 17.5% (9.3%-28.4%) NPV: 96.7% (93%

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Model 6: What format did the study present the model in to permit calculations of individual risks

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Model 6: What format did the study present the model in to permit calculations of individual risks

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Model 6: What format did the study present the model in to permit calculations of individual risks

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Model 6: Please write the regression formula:

Any fall: Tier 2 Model y = -1.44 + ((Age 70-74 yr)* -0.33) + ((Age 75-59 yr)*0.07) + ((Age 80-84 yr)*0.17

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Model 6: Please write the rounded scoring rule:

7) + ((Age 85-89 yr)*0.37) + ((Age 90+ yr)*0.26) + Fer

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Model 6: Please specify the defined risk groups:

male*0.12 + (Black ethnicity * -0.27) + (Other ethnici

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Model 6: Were the risk groups predefined rather than derived from the dataset used for developm

ity * -0.52) + (Hispanic ethnicity * 0.07) + Self-reported balance problems*0.69 + Fall history*1.15 R

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Model 6: Please specify what other format the model was presented in:

ecurrent fall: Tier 2 Model y = 2.67 + ((Age 70-74 yr)* -0.66) + ((Age 75-59 yr)*-0.08) + ((Age 80-84 yr)

A table with classification measures for the model with 2 different cut-off values for the 7m gait spee

For peer review only

Model 6: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

No information

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For peer review only

Model 6: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

No information

No information

Recurrent falls: Developmental data set: AUC (95% CI): 0.77 (0.74-0.79) Validation data set: AUC (9
Change in AUC compared to age alone (AUC: 0.577): 0.001 (-0.001; 0.003)

No information

For peer review only

Model 6: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

No information

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No information

Based on author response: "We did not generate calibration plots for these analyses, just the discrim

No information

For peer review only

Model 6: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 12% SP: 95% Total predictive ability: 66%

Extracted from figure 2: SN: 68% SP: 55%

No information
NRI events: -0.05(-0.06, -0.03) NRI non-events: 0.03(0.02, 0.03)

Gait speed < 0.8 m/s: SN: 56.2% (30.6%-91.7%) SP: 84% (79.3%-88.2%) PPV: 18.4% (9.9%-29.9%) NPV

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Model 7: What format did the study present the model in to permit calculations of individual risks

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Model 7: What format did the study present the model in to permit calculations of individual risks

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Model 7: Please write the regression formula:

For peer review only

Model 7: Please write the rounded scoring rule:

For peer review only

Model 7: Please specify the defined risk groups:

For peer review only

Model 7: Were the risk groups predefined rather than derived from the dataset used for developm

For peer review only

Model 7: Please specify what other format the model was presented in:

For peer review only

Model 7: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

No information

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For peer review only

Model 7: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

No information

No information

Recurrent falls: AUC (95% CI): 0.78 (0.76-0.81)

For peer review only

Model 7: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

No information

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No information

For peer review only

Model 7: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

SN: 59% SP: 86% Total predictive ability: 77%

Extracted from figure 2: SN: 35% SP: 70%

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For peer review only

Model 8: What format did the study present the model in to permit calculations of individual risks

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Model 8: What format did the study present the model in to permit calculations of individual risks

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Model 8: Please write the regression formula:

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Model 8: Please specify the defined risk groups:

For peer review only

Model 8: Were the risk groups predefined rather than derived from the dataset used for developm

For peer review only

Model 8: Please specify what other format the model was presented in:

For peer review only

Model 8: Please specify measures of overall performance (e.g. R2, Brier score etc.)

No information

For peer review only

Model 8: Please specify measures of discrimination in the model (e.g. AUC, C-index, D-statistic etc.

Recurrent falls: AUC (95% CI): 0.79 (0.76-0.81)

For peer review only

Model 8: Please specify measures of calibration in the model (e.g. Calibration plot, calibration inte

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For peer review only

Model 8: Please specify measures of classification in the model (e.g. Sensitivity, specificity, PPV, NI

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Model 9: What format did the study present the model in to permit calculations of individual risks

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Model 9: What format did the study present the model in to permit calculations of individual risks

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Model 9: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Model 9: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Model 9: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Model 9: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Model 9: What format did the study present the model in to permit calculations of individual risks

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For peer review only

Classify the type of prediction model evaluation	Outcome of interest: (choice=Single falls)
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Validation only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked
Validation only	Unchecked
Development only	Unchecked
Development only	Unchecked
Development and validation	Unchecked
Development only	Unchecked
Development only	Unchecked
Development only	Unchecked

For peer review only

Outcome of interest: (choice=Recurrent falls)	Outcome of interest: (choice=First time falls)
Checked	Unchecked
Unchecked	Unchecked
Checked	Unchecked
Checked	Unchecked
Checked	Unchecked
Checked	Unchecked
Unchecked	Unchecked
Checked	Unchecked
Unchecked	Unchecked
Unchecked	Unchecked
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For peer review only

Outcome of interest: (choice=Any fall)	Outcome of interest: (choice=Number of falls)
Checked	Unchecked
Checked	Unchecked
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Unchecked	Unchecked
Unchecked	Unchecked
Unchecked	Unchecked
Checked	Unchecked
Unchecked	Unchecked
Checked	Unchecked
Checked	Unchecked
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Unchecked	Unchecked

For peer review only

1.1 Development: Were appropriate data sources used, e.g. cohort, RCT or nested case-control stu

Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Probably yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes

Yes
Yes
Yes
Yes

Yes
Yes
Yes
Yes
Yes
Yes

For peer review only

1.1 Validation: Were appropriate data sources used, e.g. cohort, RCT or nested case-control study (

Yes

Yes

Yes

For peer review only

1.2 Development: Were all inclusions and exclusions of participants appropriate?

Probably yes
Probably no
Yes
No information
Yes
Yes
Probably yes
Yes
Yes
No
Probably yes
Yes
Probably no
No
No
Yes
Yes
Yes

Yes
Yes
Probably yes
Probably yes

No
Yes
Probably no
Yes
No
No

For peer review only

1.2 Validation: Were all inclusions and exclusions of participants appropriate?

Yes

Yes

Probably no

For peer review only

Development: Risk of bias introduced by selection of participants:

Low risk of bias
High risk of bias
Low risk of bias
Unclear
Low risk of bias
Low risk of bias
Low risk of bias
Low risk of bias
Low risk of bias
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High risk of bias
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High risk of bias

For peer review only

Development: Rationale of bias rating:

A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
The exclusion criteria excluded potentially high risk individuals of the target population of community
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was performed. Exclusion criteria were not clearly specified as "severe ps
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
Community-dwelling older adults 65+ yr who are mobile. Disease did not exclude the participants unl
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was performed. However, the sampling procedure was not described. The
A prospective cohort study was performed. The study excluded participants with self-reported neurol
A prospective cohort study was performed. The study was performed on female Medicare beneficiari
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study design was performed. The study used inclusion criteria that would result
A prospective cohort study was performed. The study used broad inclusion criteria, but excluded part
Participants with neurological disease were excluded. Thus the prediction model would not be able to
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was performed. No inclusion criterion age was defined. However, the fina

A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
Participants were community-dwelling older adults (65+ yr). Participants who were not able to compl
A prospective cohort study design was used. The study only investigated older (65p yr) men with no p

Participants were excluded if they changed their risk profile by having severe cognitive deterioration (
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
First, participants need to be part of the Medicare system in order to be in the study. Thus the predic
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger
A prospective cohort study was applied. Participants with medication that could potentially affect phy
A prospective cohort study was performed. The study used relatively strict exclusion criteria that may

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Validation: Risk of bias introduced by selection of participants:

neralizability to the target population.

y-dwelling older adults. Thus, the discriminatory ability of the model

neralizability to the target population.

sychiatric or somatic illness", which complicates assessment of appro

neralizability to the target population.

less it would compromise the inclusion criteria. This is acceptable.

neralizability to the target population.

neralizability to the target population.

e study used broad inclusion criteria which enables generalizability to

logical disorders. Thus, the prediction model would not be able to ge

ies with some degree of functional disability. This may have introduc

neralizability to the target population. Exclusion criteria were fair sin

in a selected sample of community-dwelling individuals with a great

ticipants with known pathological states that may impede mobility. T

o generalise to this population which may have an increased risk of f

neralizability to the target population. Exclusion criteria were fair sin

neralizability to the target population.

al age range showed a minimum age of 65 years. Thus, the study use

Low risk of bias

neralizability to the target population.

neralizability to the target population. Exclusion criteria were fair sin

lete tests were excluded. This could potentially reduce the discrimina

prior hip fracture, which may be a healthier population than the one

Low risk of bias

(Pfeiffer test > 7 errors), had changes in medication (which increased

neralizability to the target population.

High risk of bias

neralizability to the target population.

ysical function and balance, or had impaired physical function due to

y underestimate the model's performance in the target population o

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Validation: Rationale of bias rating:

may be reduced by excluding this part of the population.

opriateness and risk of bias is therefore rated as unclear.

o the target population.
eneralise to this population, due to underestimation of falls risk, which may have an increased risk of f
ed selection bias and optimism in the models since participants are more ill compared with the target
ce participants fulfilling these could yield a poor quality of data.
ter need for assistance, which compromises generalisability to the target population. Thus, model per
This may have introduced selection bias since this group of participants is also represented in the hete
falling. This may produce a biased estimate of the discriminatory performance of the model for the tar
ce participants fulfilling these could yield a poor quality of data.

d broad inclusion criteria which enables generalizability to the target population.
A prospective cohort study was performed. The study used broad inclusion criteria which enables ger

ce participants fulfilling these could yield a poor quality of data.
atory power of the model.
intended in the review question which allows for a more non-homogenous population than found in
A prospective cohort study was performed in all four cohorts. The studies used broad inclusion criteri
d the risk of fall), initiation or termination of rehabilitation therapy (which modified the risk of fall), hc

First, participants need to be part of the Medicare system in order to be in the study. Thus the predic

o neurological or musculoskeletal disorders were excluded, which may compromise generalizability of
f community-dwelling older adults in a general population setting.

For peer review only

Development: Concern that the included participants and setting do not match the review questio

Low concern
High concern
Low concern
High concern
Low concern
Low concern
Unclear
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Unclear
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Unclear
Low concern
High concern

neralizability to the target population.

Low concern
Low concern
Low concern
High concern

ia which enables generalizability to the target population.

Low concern
Low concern
High concern
Unclear
High concern
High concern

For peer review only

Development: Rationale of applicability rating:

The setting and participants of the study matched the intended setting of the review.
The exclusion criteria excluded potentially high risk individuals of the target population of community
The setting and participants of the study matched the review requirements.
The setting matched the intended setting of the review. Regarding participants, exclusion was done c
The setting and participants of the study matched the intended setting of the review.
Heterogenous population of community-dwelling older adults for which reason generalisability to the
The setting of the study matched the intended setting of the review. However, it was not clearly state
The setting and participants of the study matched the review requirements.
The setting of the study matched the intended setting of the review. However, it was not reported wl
The setting matched the intended setting of the review. Regarding participants, exclusion was done d
The study population consisted of community-dwelling female Medicare beneficiaries. Thus the mod
The setting and participants of the study matched the intended setting of the review.
Participants are in principal community dwelling. However, they are requiring af nursing home level c
Study participants were community-dwelling. However the study had extensive exclusion criteria, wh
Participants with neurological disease were excluded. Thus the prediction model would not be able t
Unclear exactly how participants were recruited from primary care and VA health clinics. However, th
The setting and participants of the study matched the intended setting of the review.
Participants with know gait or balance disorders were excluded. This may affect generalisability to th

The setting of the study matched the intended setting of the review.
The setting and participants of the study matched the requirements of the review.
Participants and the setting matched the review question.
The study only investigated older (65p yr) men with no prior hip fracture, which may be a healthier p

The participants of the study matched the review requirements
The setting and participants of the study matched the intended setting of the review.
Participants need to be part of the Medicare system in order to be in the study. Thus, generalisation t
It was not reported whether participants were community-dwellers, institutionalised or both.
Participants with medication that could potentially affect physical function and balance, or had impai
The setting matched the requirement of the review. The study used relatively strict exclusion criteria

For peer review only

Validation: Concern that the included participants and setting do not match the review question

/-dwelling older adults. When predicting risk of falls using this model, generalisation may only be done

due to "severe somatic or psychiatric disease" or "Not suitable for other reasons", which may compro

e target population of review may be possible.
ed whether participants were community-dwellers, but only that they resided in a primary care group

hether any participants were institutionalised or only community-dwelling.
due to presence of self-reported neurological disorders, or spinal or lower extremity joint pain interact
els may not be applicable to the entire community-dwelling population.

of care. The authors state that they are dealing with the frailest subsample of the older population.
nich may affect generalisability.
o generalise to this population which may have an increased risk of falling.
ne paper does not report whether participants were recruited when patients attended the clinics or if

e general population.
High concern

opulation than the one intended in the review question which allows for a more non-homogenous po
Low concern

High concern

ired physical function due to neurological or musculoskeletal disorders were excluded, which may con
that may not be generalisable to the target population of community-dwelling older adults in a gener

For peer review only

Validation: Rationale of applicability rating:

e to a specific subgroup of community-dwelling older adults who do not have properties increasing th

mise generalisability to the target population of the review.

) area.

ting with stepping performance. This may compromise generalisability to the target population of the

recruitment was done by contacting every patient registered at the clinics, thereby mimicking a gene

Community dwelling, but all requiring home health care, so may not be entirely applicable for the ge

pulation than found in this study. Thus these prediction models cannot be used in populations with p
The setting and participants of the study matched the intended setting of the review.

Participants need to be part of the Medicare system in order to be in the study. Thus, generalisation t

npromise generalizability of the study to the target population.
ral population setting.

For peer review only

2.1 Development: Were predictors defined and assessed in a similar way for all participants?

Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Probably yes
Yes
Yes
Probably yes
Probably yes
Yes
neral population.
Yes
Yes
Yes
Yes

Yes
Yes
Yes
Yes
Yes
Yes

For peer review only

2.1 Validation: Were predictors defined and assessed in a similar way for all participants?

Yes

Yes

Yes

For peer review only

2.2 Development: Were predictor assessments made without knowledge of outcome data?

Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes

Yes
Yes
Yes
Yes

Yes
Yes
Yes
Yes
Yes
Yes

For peer review only

2.2 Validation: Were predictor assessments made without knowledge of outcome data?

Yes

Yes

Yes

For peer review only

2.3 Development: Are all predictors available at the time the model is intended to be used?

Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Yes
Probably yes
Yes
Yes
Yes
Yes
Probably yes

Yes
Yes
Yes
Yes

Yes
Yes
Yes
Probably yes
Yes
Yes

For peer review only

2.3 Validation: Are all predictors available at the time the model is intended to be used?

Yes

Yes

Yes

For peer review only

Development: Risk of bias introduced by predictors or their assessment

Low risk of bias
Low risk of bias
Low risk of bias
Low risk of bias
Low risk of bias
Low risk of bias
Low risk of bias
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For peer review only

Development: Rationale of bias rating:

All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined using both laboratory and clinical tests that were described in detail. Due
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
The order in which predictors were measured were different for few participants. However, all receiv
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
The predictor was defined and described in detail. Due to the longitudinal design of the study, the pr
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
It was not clearly reported how all information on predictors was collected. However, it is likely that t
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
A standardized questionnaire was used for predictors. Also, physical measurements are standardised.
It was not reported how information on age, gender, education and fall history was ascertained. How
All predictors were defined, but it was not completely clear whether data collection was done throug
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal

All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were measured in a standardized manner according to previous studies defining sarcop

The predictor was defined and described in detail. Due to the longitudinal design of the study, the pr
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined. Due to the longitudinal design of the study, the presence of the outcome
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal

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Validation: Risk of bias introduced by predictors or their assessment

design of the study, the presence of the outcome at baseline measuremer
to the longitudinal design of the study, the presence of the outcome at ba
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
ved the same test battery. This was a prospective cohort study for which re
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
esence of the outcome at baseline measurement would not be possible. Th
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
this was done using questionnaires and administrative information.
design of the study, the presence of the outcome at baseline measuremer
. This was a prospective cohort study for which reason predictors assessme
vever, since data collection was performed during a visit, it is most likely tha
h phone interviews or face-to-face interviews. Due to the longitudinal desi
design of the study, the presence of the outcome at baseline measuremer
Low risk of bias
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
penia using these measures. All predictors would be available at the time o
Low risk of bias
esence of the outcome at baseline measurement would not be possible. Th
design of the study, the presence of the outcome at baseline measuremer
Low risk of bias
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer
design of the study, the presence of the outcome at baseline measuremer

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Validation: Rationale of bias rating:

nt would not be possible. All predictors would be available at the time the model is intended to be use
aseline measurement would not be possible. All predictors would be available at the time the model is
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use
eason the outcome could not have been known at predictor assessment. All predictors are available at
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use
he predictor would be available at the time the model is intended to be used.
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use

nt would not be possible. All predictors would be available at the time the model is intended to be use
ent measurement could only be done without knowledge of the outcome. All predictors were availabl
at a questionnaire or interview form was used. Thus, the study is not judged to be at high risk of bias
ign of the study, the presence of the outcome at baseline measurement would not be possible. All pre
nt would not be possible. All predictors would be available at the time the model is intended to be use
All predictors were defined and described in detail in references of the study. Due to the longitudinal
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use
f prediction.
All predictors were defined and corresponding tests were described in detail. Due to the longitudinal
he predictor would be available at the time the model is intended to be used.
nt would not be possible. All predictors would be available at the time the model is intended to be use
All predictors were defined. Due to the longitudinal design of the study, the presence of the outcome
nt would not be possible. The questionnaires and clinical tests were not performed on the same time,
nt would not be possible. All predictors would be available at the time the model is intended to be use
nt would not be possible. All predictors would be available at the time the model is intended to be use

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Development: Concern that the definition, assessment or timing of predictors in the model do not
Low concern
High concern
Low concern
Low concern
Low concern
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Low concern
design of the study, the presence of the outcome at baseline measurement would not be possible. Al
Low concern
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design of the study, the presence of the outcome at baseline measurement would not be possible. Al
Low concern
Low concern
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Low concern
Low concern
High concern

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Development: Rationale of applicability rating:

Two or more predictors were collected for which reason this matches the requirements of the review
The predictors include laboratory measures using a force platform and EMG measures. It may be deb
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Visual, cardiac and pulmonary problems were not clearly defined. However, predictors included in th
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Multiple predictors with a clear definition were used. Also, assessments were made in a community s
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
ll predictors would be available at the time the model is intended to be used.
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
The review question has no other requirements than the prediction model should be multifactorial w
ll predictors would be available at the time the model is intended to be used.
Two predictors were collected for which reason this matches the requirements of the review protocc
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review
Two or more predictors were collected for which reason this matches the requirements of the review

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Validation: Concern that the definition, assessment or timing of predictors in the model do not ma
v protocol. Predictors are judged to be applicable for the community setting.
atable whether measurements using these tests in a community setting may be applicable since they
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
e final model was clearly defined for which reason these are applicable.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors were collected in a research laboratory, but are judged to be applicable for the c
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
setting and the timing of predictor measurement is the same for all predictors at baseline.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
Low concern
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
which is the case in this study.
Low concern
ol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
Low concern
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors are judged to be applicable for the community setting.
v protocol. Predictors were assessed in a laboratory for which reason they may not be applicable for a

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Validation: Rationale of applicability rating:

are used in a laboratory setting.

community setting.

Two or more predictors were collected for which reason this matches the requirements of the review

Two or more predictors were collected for which reason this matches the requirements of the review

Two or more predictors were collected for which reason this matches the requirements of the review

community setting.

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3.1 Development: Was the outcome determined appropriately?

No
Yes
No
No
No
Yes
No
No
No
Probably no
No
Yes
No
No
Yes
No
No
Yes
/ protocol. Predictors are judged to be applicable for the community s
No
Yes
No
No
/ protocol. Predictors are judged to be applicable for the community s
No
No
No
No
Yes
No

For peer review only

3.1 Validation: Was the outcome determined appropriately?

Probably yes

No

No

For peer review only

3.2 Development: Was a pre-specified or standard outcome definition used?

Yes
Probably yes
Probably yes
No
No
Yes
No information
Yes
Yes
Yes
Yes
Probably no
No information
No information
No information
Yes
No
No

Yes
No information
Yes
Probably yes

No
No
No
No
Probably yes
Yes

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3.2 Validation: Was a pre-specified or standard outcome definition used?

Probably yes

No

No

For peer review only

3.3 Development: Were predictors excluded from the outcome definition?

Yes
Yes
Yes
Yes
Yes
Yes
Probably yes
Yes
Yes
Yes
Yes
Yes
No information
No information
Yes
Yes
Probably yes
Yes

Yes
No information
Yes
Yes

Yes
Yes
Yes
No information
Yes
Yes

For peer review only

3.3 Validation: Were predictors excluded from the outcome definition?

Yes

Probably yes

Yes

For peer review only

3.4 Development: Was the outcome defined and determined in a similar way for all participants?

Yes
Yes
Yes
Yes
Yes
Probably yes
No information
Yes
Probably yes
Yes
Yes
Yes
No information
No
Yes
Yes
No information
Yes

No
No information
Yes
Probably yes

Yes
Probably no
No information
No information
Yes
Yes

For peer review only

3.4 Validation: Was the outcome defined and determined in a similar way for all participants?

Yes

Probably yes

No information

For peer review only

3.5 Development: Was the outcome determined without knowledge of predictor information?

No information
No information
No information
No information
No information
No information
No information
No information
No information
No
No information
Yes
No information
No information
Yes
No information
No information
No information

No information
No information
No information
Yes

Yes
Yes
No information
No information
No information
No information

For peer review only

3.5 Validation: Was the outcome determined without knowledge of predictor information?

No information

Yes

No information

For peer review only

3.6 Development: Was the time interval between predictor assessment and outcome determinatic

Yes
Yes
Yes
Yes
Yes
Yes
Probably yes
Yes
No
Yes
Yes
Yes
No information
Yes
Yes
Yes
No
Yes

Yes
Yes
Yes
Yes

Yes
Yes
No
Yes
Yes
Yes

For peer review only

3.6 Validation: Was the time interval between predictor assessment and outcome determination a

Probably yes

Yes

No

For peer review only

Development: Risk of bias introduced by the outcome or its determination

High risk of bias
Unclear
High risk of bias
High risk of bias
High risk of bias
Unclear
High risk of bias
High risk of bias
High risk of bias
High risk of bias
High risk of bias
High risk of bias
High risk of bias
High risk of bias
Unclear
High risk of bias
High risk of bias
High risk of bias

High risk of bias
Unclear
High risk of bias
High risk of bias

High risk of bias
High risk of bias
High risk of bias
High risk of bias
Unclear
High risk of bias

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Development: Rationale of bias rating:

The outcome was defined. Participants sent postcards weekly, rather than daily recording, with infor
It is unknown whether the outcome assessors were blinded towards baseline predictors. A standard
According to the PROFANE guidelines: "Falls should be recorded using prospective daily recording and
The outcome was determined every 6 weeks which is not in line with PROFANE recommendations. Th
According to the PROFANE guidelines: "Falls should be recorded using prospective daily recording and
The participants were asked to keep record of their falls using calendars. Participants were contacted
No outcome definition was reported. Outcomes were ascertained by mailing a questionnaire after 6 r
The PROFANE definition was used and outcomes were determined by weekly, rather than daily, fall c
The outcome was first ascertained after 6 months which could have introduced recall bias. It was not
The outcome was determined using self-report daily fall calendars that were returned after one year.
The PROFANE definiton of falls was not applied. Also, falls were only determined every 6 months for
Outcome was a composite of injurious and recurrent, which was not prespecified and unclearly defin
No outcome definition was reported. The outcome was only assessed if the data collector was contac
No outcome definition was reported. The outcome was reported weekly, rather than daily with no m
No protocol was preregistered for the study for which reason it is not completely certain that the out
The outcome was ascertained every 3 months which could potentially have introduced recall bias am
No outcome definition was used, and it was not properly reported how the data on the outcome wer
It was not reported whether the outcome assessors were blinded. Recurrent falls were chosen as the

The outcome was assessed using weekly fall calendars. If participants were unable to complete the fa
No falls definition was reported. Also, it was not reported whether outcome assessors were blinded t
The outcome enquired after one year which could potentially lead to recall bias. It was not reported
A questionnaire was mailed three times per year which is too long according to current consensus sta

The outcome was determined every 3 months by telephone calls. This could have introduced recall b
The study had no outcome definition. Also, the outcome was measured only once a year which may h
No outcome definition was reported. Outcomes were assessed three times annually which may have
No outcome definition was reported. In the InCHIANTI cohort, participants were asked: "Have you ev
The outcome was determined using dialy fall calendars with monthly phone calls to remind the partic
The outcome was determined using fall calendars checked every three months which potentially coul

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Validation: Risk of bias introduced by the outcome or its determination

mation on whether they had fallen or not. Afterwards, falls were validated thr
outcome definition was used, however no reporting of an available pre-regist
d a notification system with a minimum of monthly reporting". Falls were recc
nus, there is a potential for recall bias and hence high risk of bias. Also, it was
d a notification system with a minimum of monthly reporting". Falls were recc
l either by phone or e-mail in order to ascertain whether they had fallen. How
months which could have introduced recall bias among participants. Since par
alendars. It was not reported whether the outcome assessors were blinded to
reported whether outcome assessors were blinded towards baseline predict
. Thus, the outcome assessment depended on the participants understanding
1 year. This may have introduced recall bias. It was not reported whether the
ed.
cted and not by daily recording and notification systems. Also, it was not repo
onthly notification system. This is judged to be essential in terms of risk of bia
come was prespecified. However, outcome definition was in line with the PR
ong participants. It was not reported whether the outcome assessors were bl
e collected. Outcomes were recorded after 2 years, which potentially could le
outcome, this was not prespecified and results for single falls were not repor
Unclear
ll calendars, if they were not returned or if they were completed incorrectly,
owards baseline predictors.
whether the outcome was assessment without knowledge of predictor measu
ndards. According to the authors, the consensus definition of a fall was used.
High risk of bias
ias among participants. The choice of "recurrent falls" as the outcome rather
ave introduced recall bias. The outcome was determined by self report, and i
High risk of bias
er fallen to the floor in the last 12 months?". It was not reported whether out
cipants to fill out the calendars. However, it was not reported whether falls we
ld introduce recall bias. It was not reported whether outcome assessors were

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Validation: Rationale of bias rating:

rough interviews. It was not reported whether the outcome assessors were blinded.
rered protocol was done.
orded weekly and reported back every three months in this study for which reason there is a potentia
not reported whether the outcome assessors were blinded . The choice of recurrent falls as outcome
orded weekly and reported back every three months in this study for which reason there is a potentia
vever, it was not reported who assessed the outcome and whether the assessor was blinded towards
rticipants were the ones to report their baseline characteristics and their following outcome, blinding
owards baseline predictors, however fall calendars were used, which limits impact on risk of bias.
ors.
the definition of falls and remembered to fill out the fall calendars. Since notification (adequate surve
outcome assessors were blinded to baseline assessments.

rted how long time each participants was followed, but only that it happened within given time perio
as for which reason this may have introduced a high risk of bias. It was not reported whether outcome
OFANE definition. It is not clear whether the outcome was recurrent og any fall. Outcome was detern
linded towards predictors.
ead to recall bias. It was not reported whether the outcome assessors were blinded toward the baseli
rted.
No pre-registered protocol was reported. It was not reported whether the outcome assessors were b
participants were contacted. It was not reported whether outcome assessors were blinded towards p

urements.
. The outcome was determined through contact to the participants or next of kin, if the participant ha
Outcome was determined differently in different cohorts, some of which were inappropriate. Howeve
than falls was not prespecified or justified.
it is not clear whether participants had a uniform understanding of the outcome definition. No prereg
No outcome definition was reported. Outcomes were assessed three times annually which may have
tcome assessors were blinded. The outcome was assessed monthly for 6 months after which only a fir
ere reported monthly or whether outcome assessors were blinded.
blinded to the baseline predictors.

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Development: Concern that the outcome, its definition, timing or determination do not match the
Low concern
Low concern
Low concern
Low concern
Low concern
Unclear
Unclear
Low concern
Low concern
Low concern
Unclear
High concern
Unclear
Unclear
Low concern
Low concern
High concern
Low concern
linded towards baseline predictor measurements. Follow-up was short which may have reduced num
Low concern
Unclear
Low concern
Low concern
er, there are also the ActiFe cohorts with appropriate determination in which all models were validate
Low concern
High concern
Unclear
Unclear
Low concern
Low concern

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Development: Rationale of applicability rating:

The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition is in line with definition stated in the review protocol. Monthly fall calendars
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition differs slightly compared to the outcome definition of the review. It defines a
No outcome definition was reported.
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The PROFANE definition of falls was not applied. Although close, the authors applied a definition that
The falls definition matches that of the review protocol. The outcome was a composite of injurious ar
No outcome definition was reported.
No outcome definition was reported.
The outcome definition is in line with the review question. No restrictions regarding assessment metl
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
Participants were asked: "Have you fallen down in the last two years?". This does not rule out the PR
The outcome definition matches that of the review question. No restrictions were otherwise specifiec
ber of events. Since the frequency of notification (adequate surveillance of documentation and ascer
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
No falls definition was reported. However, the fall recording method recommended by the PROFANE
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The review question only required the outcome to follow current consensus defintion which is the ca
ed. Therefore, as all model are validated in a setting with appropriate methods of outcome determina
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome was based on self-report. This does not rule out the PROFANE definition, but also make
No outcome definition was reported.
No outcome definition was reported.
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec
The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec

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Validation: Concern that the definition, assessment or timing of predictors in the model do not ma
d.
were used which is in line with current recommendations.
d.
d.
d.
a fall as an event that has to happen during routine activities, which is not necessary in the consensus

d.
d.
d.
t did not explicitly define what falling means. This was done by stating that "Falling was defined as falli
nd recurrent falls, which is not directly comparable to the review question. No restrictions were other

hod for the outcome has been specified for the review question. This study used monthly phone calls
d.
OFANE definition, but also makes the definition a fall open for interpretation, thereby giving high con
d.
Low concern
d.
group was referred to. Thus, it is likely that the fall definition may be according to PROFANE group als
d.
se in this study.
Unclear
d.
s the definition a fall open for interpretation, thereby giving high concerns regarding the applicability
Unclear

d.
d.

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Validation: Rationale of applicability rating:

definition of a fall in the review. Also, it is not clear how a "routine activity" is defined.

ing on the ground or at some other level such as chair level". Would this also include slipping or trippi
wise specified.

with a clear and standardised definition which is acceptable.

cerns regarding the applicability of the study for the review.

The outcome definition matches that of the review protocol. No restrictions were otherwise specifiec

so.

ActiFE uses the profane defintion and has low concern for applicability. The other are of unclear conc

of the study for the review.
No outcome definition was reported.

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Development: Describe any participants who were excluded from the analysis:

ng and thereby coming to rest on the ground?
No information

d.
No information

From the community-dwelling Round 1 population, participants who were alive at Round 2 were adm

cern as there are no falls definitions.

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Validation: Describe any participants who were excluded from the analysis:

Five did not have the fall calendars collected and one refused to complete the calendar.

ıinistered an identical SP questionnaire regardless of interval changes in residential status (N =

No information.

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4.1 Development: Were there a reasonable number of participants with the outcome?

No
No
Probably no
No
Probably yes
No
No information
Yes
Probably no
No
No information
No
Yes
No information
Yes
Yes
Yes
No

Yes
No
Yes
Yes

Probably yes
Yes
No
Probably yes
No
No

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4.1 Validation: Were there a reasonable number of participants with the outcome?

No

Yes

Yes

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4.2 Development: Were continuous and categorical predictors handled appropriately?

Yes
Yes
No
No
No
Probably yes
Yes
No
Yes
No
Yes
Yes
No
No information
Yes
Yes
No
Yes

No
No information
No information
Probably yes

Yes
No
No information
Yes
No information
Yes

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4.2 Validation: Were continuous and categorical predictors handled appropriately?

Yes

Yes

No information

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4.3 Development: Were all enrolled participants included in the analysis?

No
Probably yes
No
No
No
No
No
No
Probably no
No
No
Probably no
No
No
No
No
No
No information

Probably no
No
Probably no
No

No
No
No
No
No
No

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4.3 Validation: Were all enrolled participants included in the analysis?

No

No

No

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4.4 Development: Were participants with missing data handled appropriately?

No
Yes
No information
No
No
No
No information
No information
No
No
No information
No information
No
No information
No
No information
No
No information

No information
No information
No information
No

No
No
No
No
No
No

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4.4 Validation: Were participants with missing data handled appropriately?

No

No

No

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4.5 Development: Was selection of predictors based on univariable analysis avoided?

Yes
Yes
No
No
Yes
Yes
Yes
No
No
No information
Yes
No
Yes
Yes
No
No
Yes
Yes

Yes
Yes
Probably yes
Yes

Yes
Yes
Yes
Yes
No information
Yes

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4.6 Development: Were complexities in the data (e.g. censoring, competing risks, sampling of cont

No
Yes
No
No
No
No
No
No
No
No
No
Probably no
No information
No information
No
No
No
No information

No information
No
Probably no
No

No
Probably no
No
No
No
No

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4.6 Validation: Were complexities in the data (e.g. censoring, competing risks, sampling of controls

No

No

No

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4.7 Development: Were relevant model performance measures evaluated appropriately?

No information
No information
No information
Yes
No
No
No
Yes
No information
No
No
No
No information
No
No information
No information
No information
No information

No information
No
No information
No information

Yes
No information
No
No
No information
No information

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4.7 Validation: Were relevant model performance measures evaluated appropriately?

No information

Yes

No information

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4.8 Development: Were model overfitting and optimism in model performance accounted for?

Yes
Probably yes
No information
No information
No information
No
No
No
No
No
Yes
No
No information
No information
No
No information
Yes
No information

No
Yes
Yes
Yes

No
No
Yes
No information
No information
Yes

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4.9 Development: Do predictors and their assigned weights in the final model correspond to the re

No information
No information
Yes
No
Yes
No information
No information
Yes
No information
No information
Yes
No information
No information
No information
Yes
No information
No information
No information

No information
No information
Yes
No information

No information
No information
Yes
No information
Yes
No information

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Development: Risk of bias introduced by the analysis

High risk of bias
High risk of bias
High risk of bias
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Development: Rationale of bias rating:

EPV was low. Participants lost to follow-up were excluded from the analyses. No calibration measures
The study had a low EPV for the outcome. No calibration measures were reported. Also, the study rep
EPV was low for the recurrent falls model. All predictors were dichotomised which may have led to lo
EPV was low, all continuous predictors were dichotomised, participants lost to follow-up were exclud
For the tree-based analyses, it is a prerequisite to dichotomise continuous predictors which may lead
The outcome was multiple falls defined as 2 or more falls. This event happened 42 times. With 21 pre
Only 67% of participants were used for the analysis due to missing data on outcome (non-responders
All predictors were dichotomised. It was not reported sufficiently how participants with missing data
The total number of falls was not reported. Only participants with complete baseline and follow-up d
No regression analyses were performed. EPV was low. Number of falls was dichotomized instead of k
EPV was at least 8.2 and the total number of falls was not reported. Thus, EPV may have been higher.
EPV was low. Unclear which participants completed follow-up and if there was missing data. Selection
Participants with missing data were excluded from the analyses. Number of previous falls was dichoto
EPV was probably low. It was not reported whether missing data were present and how this was hand
It is not clear whether the outcome is recurrent falls, single falls or any falls. No definition exist for rec
Partipants lost to follow-up were excluded from the analyses. Selection of predictors other than TUG
EPV was high, all continuous variables were dichotomised. Also, 0.7% of partipants were excluded fro
EPV was low. It was not reported how participants with missing data was handled. It was reported tha

Continuous predictors were categorized. It is unclear if any of the participants had missing data and h
EPV was low. The paper did not account for how participants lost to follow-up were handled in the ar
It is not clearly reported how participants lost to follow up were handled statistically. Logistic regressi
EPV was acceptable. Missing data was present for 166 participants who were not included in the ana

Around 40 per cent of participants were excluded from the analysis due to changes in their baseline r
The results of the 30 sec chair stand was dichotomized, but this may be a known cut-point for the tes
EPV was low and participants with missing data were excluded. No handling of competing risks. No ca
Participants with missing data were excluded from the analyses. No measures of discrimination or cal
EPV was low. Participants lost to follow-up were excluded from the analyses. Predictor selection tec
EPV was low due to a high number of parameter levels. Participants lost to follow-up were excluded f

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Validation: Risk of bias introduced by the analysis

s were reported. No regression formulas were presente
ports a "predictive accuracy" for each model. It is not c
ss of information. It was not reported how participants
led from the analyses, selection of predictors was perfo
to loss of information in the model. Violation of indepe
edictors this would give an EPV of 2 which may result in
with no exact date on when they were lost to follow-u
were handled. Selection of predictors were based on ir
ata were included in the analysis. It was not reported h
eeping it continuous. Participant with missing data wer
. On the other hand, cross-validation procedures were a
n of predictors for the CRT analysis was done using univ
omised. Also, loss to follow-up among participants, cali
dled. Thus, a complete-case analysis was performed. N
current falls either. No information exist for the numbe
and gait speed were selected based on their univariate
m the analysis due to missing data, including participar
at the models "successfully identified participants as fa
High risk of bias
ow this was handled. No internal validation procedure
nalyses. No discrimination or calibration measures were
ion was used and thus it must be assumed that analysis
lysis. The analysis did not take lost to follow up/censor
High risk of bias
risk profile.
t. However, since the predictor is in nature a discrete v
High risk of bias
libration were reported. No method of internal validati
chniques were not reported. No interval validation proc
from the analyses. It was not reported whether any par

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Validation: Rationale of bias rating:

ed.

ompletely clear whether this is equivalent to an AUC. A cross-validation procedures was used to estim
s with missing data on predictors were handled. Participants lost to follow-up were excluded from the
ormed using univariate associations, and no internal validation procedures were reported. Finally, a si
endent censoring assumption in Kaplan-Meier analysis which overestimates predicted risks. Neither w
overfitting. Internal validation procedures were not applied in order to account for this. Regarding m
up). Since imputations methods become irrelevant in terms of the outcome, this seems fair. Also, ther
nvariable analyses. Twenty percent of participants were lost to follow-up, but it was not sufficiently re
ow many had complete baseline data only. Predictors were selected using univariable associations. N
re excluded. It was not reported which baseline measures were candidate predictor for the model, or
applied to correct for any optimism. Participants lost to follow up were excluded. It seem from Table 1
variate associations. No performance measures were reported. No internal validation procedures wer
bration measures, internal validation procedures, and a constant for the prediction model was not re
o measures of model performance, methods for handling model optimism/overfitting, or predictor w
r of any falls. EPV may therefore be miscalculated. Number of psychotropics may have been dichoton
association with the outcome. No measures of model calibration or regression coefficients were rep
nts lost to follow-up. However, this may not have impacted the results of the analysis due to the smal
llers and non-fallers" with a certain percentage. However, it was not reported whether this was the A
EPV was low and only 33 outcome events were present. Participants with missing data on outcomes v
was performed. No regression coefficients were reported.
e reported. It was not possible to calculate individual fall risk based on the results presented.
s was complete case analysis. Unclear how continuous variables were handled. Furthermore, selectior
ing into account. Only change in AUC and NRI was reported. Not specific AUC-measure along with cal
Participants with missing data on the outcome were excluded in all four cohorts.

ariable with counts. Using the number of chair stands performed may have given a more precise prec
No calibration measures were reported. No handling of competing risks. Participants with missing da
ion was reported. No regression analysis was performed.
cedures were reported. No discrimination measures were reported.
rticipant had missing data for the baseline predictors. No calibration measures were reported. No regr

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Development - Overall judgement of risk of bias: Low risk of bias: - If all domains were rated low
High risk of bias
High risk of bias
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High risk of bias
were excluded from the analyses. It was not reported specifically which predictors had missing data o
High risk of bias
High risk of bias
High risk of bias
High risk of bias

High risk of bias
High risk of bias
High risk of bias
High risk of bias
High risk of bias
High risk of bias

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Validation - Overall judgement of risk of bias: Low risk of bias: - If all domains were rated low risk

High risk of bias

High risk of bias

High risk of bias

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Development - Overall judgement of applicability: Low concerns regarding applicability: If low co

Low concern
High concern
Low concern
High concern
Low concern
Unclear concern
Unclear concern
Low concern
Unclear concern
High concern
High concern
High concern
High concern
Unclear concern
High concern
Unclear concern
High concern
High concern

Low concern
Unclear concern
Low concern
High concern

Low concern
High concern
High concern
Unclear concern
High concern
High concern

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Validation - Overall judgement of risk of bias: Low concerns regarding applicability: If low concer

High concern

Unclear concern

High concern

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Development - Summary of sources of potential bias:

It was not reported whether outcome assessors were blinded. Participants sent postcards weekly, rat
High risk individuals may have been excluded from the sample. Blinding of the outcome assessor was
According to the PROFANE guidelines: "Falls should be recorded using prospective daily recording and
The outcome was determined every 6 weeks. According to PROFANE guidelines "Falls should be reco
Falls were reported back every three months in this study for which reason there is a potential for rec
It is not reported whether outcome was determined unblinded. The outcome was multiple falls defin
No outcome definition was reported. Outcomes were ascertained by mailing a questionnaire after 6 r
It was not reported whether the outcome assessors were blinded towards baseline predictors. Falls w
A prospective cohort study was performed. However, the sampling procedure was not described. The
All calendars were mailed after one year which may have influenced the reporting of outcome events
The study was performed on female Medicare beneficiaries with some degree of functional disability
The outcome was a composite of injurious and recurrent falls, which was not prespecified. EPV was lo
A prospective cohort study design was performed. The study used inclusion criteria that would result
No outcome definition was reported. This is judged to be essential in terms of risk of bias for which re
Participants with neurological disease were excluded. Thus, the prediction model would not be able t
The outcome was ascertained every 3 months which could potentially have introduced recall bias am
It was not completely clear whether data collection was done through phone interviews or face-to-fa
It was not reported whether the outcome assessors were blinded. EPV was low. It was not reported h

No pre-registred protocol was reported. It was not clear whether the outcome assessor was blinded t
No falls definition was reported. Also, it was not reported whether outcome assessors were blinded t
Outcome assessment was done after 1 year by using participant recall. It was not reported whether t
The outcome was determined three times per year. The study had missing data for 166 participants. T

The outcome was determined every 3 months by telephone calls. This could have introduced recall bi
The study had no outcome definition. No pre-registrered protocol was reported. Discrete and continu
Participants need to be part of the Medicare system in order to be in the study. Thus the predictive p
No outcome definition was reported. In the InCHIANTI cohort, participants were asked: "Have you ev
A prospective cohort study was applied. Participants with medication that could potentially affect phy
A prospective cohort study was performed. The study used relatively strict exclusion criteria that may

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Development - Summary of applicability concerns:

Participants, predictors and outcomes matched the review protocol's requirements.
The population excluded high risk individual for which reason a generalisation to the target populatio
Participants, predictors and outcomes matched the review protocol's requirements.
The setting matched the intended setting of the review. Regarding participants, exclusion was done d
Participants, predictors and outcomes matched the review protocol's requirements.
The outcome definition differed slightly from the review's outcome definition.
The setting of the study matched the intended setting of the review. However, it was not clearly state
The setting and participants of the study matched the review requirements. Two or more predictors v
The setting of the study matched the intended setting of the review. However, it was not reported wh
The setting matched the intended setting of the review. Regarding participants, exclusion was done d
The study population consisted of community-dwelling female Medicare beneficiaries. Thus the mode
The outcome was a composite of injurious and recurrent falls, which is not directly comparable to the
Participants are in principal community dwelling. However, they are requiring af nursing home level c
The participants of the study matched the review requirements. However, it was not specified in whi
The study predictors and outcome matches the review question. Participants with neurological diseas
Unclear exactly how participants were recruited from primary care and VA health clinics. However, th
No outcome definition was used in the study.
Participants with know gait or balance disorders were excluded. This may affect generalisability to th

Participants, predictors and outcomes matched the review protocol's requirements.
No falls definition was reported. However, the fall recording method recommended by the PROFANE
Participants, predictors and outcomes matched the review protocol's requirements.
The population was consisting of male older adults (65+ yr) with no prior hip fracture. The prediction

The participants of the study matched the review requirements. Two or more predictors were collect
The study had no outcome definition for which reason it may not match that of the review protocol.
Participants need to be part of the Medicare system in order to be in the study. Thus, generalisation t
It was not reported whether participants were community-dwellers, institutionalised or both. No out
Participants with medication that could potentially affect physical function and balance, or had impai
The setting matched the requirement of the review. The study used relatively strict exclusion criteria

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Validation - Summary of sources of potential bias:

ɔn of community-dwelling older adults may be hindered. Predictor measurements were applicable for

due to "severe somatic or psychiatric disease" or "Not suitable for other reasons", which may compror

ed whether participants were community-dwellers, but only that they resided in a primary care group
were collected for which reason this matches the requirements of the review protocol. Predictors are
hether any participants were institutionalised or only community-dwelling.
due to presence of self-reported neurological disorders, or spinal or lower extremity joint pain interact
els may not be applicable to the entire community-dwelling population. The PROFANE definition of fa
e review question.
of care. The authors state that they are dealing with the frailest subsample of the older population. No
ch setting the study was performed. No outcome definition was reported.
se were excluded. Thus the prediction model would not be able to generalise to this population which
ne paper does not report whether participants were recruited when patients attended the clinics or if

e general population.
No pre-registrered protocol was reported. It is not reported whether the outcome assessor was blind

group was referred to. Thus, it is likely that the fall definition may be according to PROFANE group als

models would thus not be used for a more heterogenous population consisting of female participants
Only ActiFE used the PROFANE definition and weekly fall calendars whereas the three other cohorts
ted and the outcome matches that of the review.

Participants need to be part of the Medicare system in order to be in the study. Thus the predictive p
come definition was reported.
red physical function due to neurological or musculoskeletal disorders were excluded, which may com
that may not be generalisable to the target population of community-dwelling older adults in a gener

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Validation - Summary of applicability concerns:

a laboratory setting, but it may be difficult to transfer this to a community-setting.

mise generalisability to the target population of the review. Predictors and outcomes matched the rev

area. No outcome definition was reported.
judged to be applicable for the community setting. The outcome definition matches that of the revie

ting with stepping performance. This may compromise generalisability to the target population of the
lls was not applied. Although close, the authors applied a definition that did not explicitly define what

outcome definition was reported.

may have an increased risk of falling. However, it is not clear whether the prediction model predict s
recruitment was done by contacting every patient registered at the clinics, thereby mimicking a gene

Participants all received home health care, which may have affected generalizability to the general po

so.

s or participants with prior hip fractures.
No outcome was defined for three of the cohorts.

Participants need to be part of the Medicare system in order to be in the study. Thus, generalisation t

mpromise generalizability of the study to the target population. Two or more predictors were collecte
al population setting. Two or more predictors were collected for which reason this matches the requi

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Was publication of the protocol done before recruitment of patients?

No information
No information
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For peer review only

Which date was the protocol published?:

For peer review only

Was the intended outcome in the protocol the same in the published study?

No information
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For peer review only

Please specify the sources of funding for the study:

The study was supported by an operating grant (AG-06357) from the National Institute of Aging.
This research was supported by the Public Health Research and Development Committee (Grant 96/
The study is based on data collected in the context of the Longitudinal Aging Study Amsterdam (LASA
This study was funded by a grant from Zorg Onderzoek Nederland (ZON) (Care Research in the Nethe
The study is based on data collected in the context of the Longitudinal Aging Study Amsterdam (LASA
This study was supported by a grant from the California State University, Sacramento, Research and C
Funding by PPP Healthcare Medical Trust.
With respect to funding: this study is based on data collected in the context of the Longitudinal Aging
This study was financially supported by a grant from the Ministry of Health, Labour and Welfare in Jap
The study was supported by the Robert Bosch Foundation.
This work was supported by the National Institute on Aging (NIA; contract NO1-AG12112 and the NIA
This research was conducted as part of a study on Understanding Fear of Falling and Risk- taking in Ol
The authors disclosed receipt of the following financial support for the research and/ or authorship o
Supported by the National Institute on Aging, National Institutes of Health (grant nos. AG022092, AG
No funding source was reported.
National Institutes of Health T32 Training Grant (AG 021885), Merck Research Laboratories, Pittsburg
The authors received no financial support for the research and/or authorship of this article.
The author(s) disclosed receipt of the following financial support for the research, authorship, and/or
No information
No information
"Supported by the University of Jordan. No commercial party having a direct financial interest in the r
The authors disclosed receipt of the following financial support for the research, authorship, and/or p
The Osteoporotic Fractures in Men (MrOS) Study is supported by National Institutes of Health fundin
The ActiFE study was supported by grants from the Ministry of Science, Research and Arts, state of Ba
This work was supported by the European Commission within the 6th Framework Programme (projec
This work was supported by a grant from the National Institute of Mental Health at the National Insti
This research was funded by the Paul B. Beeson Clinical Scientist Development Award in Aging (K08AG
This work was supported by the European Commission (FARSEEING, Grant number 288940, and Preve
This study was funded by the Ministry of Education Malaysia, Long-term Research Grant Scheme (LRG
This study was supported by a grant from the Belgian National Fund for Scientific Research (F.N.R.S.).

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The words developing/development, validation/validating, incremental/added value (or synonyms

D - No
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D+V - Yes
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For peer review only

The words prediction, risk prediction, prediction model, risk models, prognostic models, prognostic

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V - Yes
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D+V - No
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For peer review only

The target population is reported in the title

D - Yes
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V - Yes
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V - Yes
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D+V - No
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For peer review only

The outcome to be predicted is reported in the title

D - Yes
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D - Yes
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V - Yes
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V - Yes
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D - Yes
D+V - No
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For peer review only

1 - Title - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y'

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For peer review only

The objectives are reported in the abstract

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V - Yes
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V - Yes
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D+V - Yes
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For peer review only

Sources of data are reported in the abstract E.g. Prospective cohort, registry data, RCT data.

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V - Yes
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V - Yes
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D+V - Yes
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For peer review only

The setting is reported in the abstract E.g. Primary care, secondary care, general population, adult c

D - No
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V - No
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D+V - No
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For peer review only

A general definition of the study participants is reported in the abstract E.g. patients with suspicion

D - Yes
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V - Yes
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D+V - No
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For peer review only

The overall sample size is reported in the abstract

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V - No
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D+V - Yes
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For peer review only

Predictors included in the final model are reported in the abstract. For validation studies of well-kn

D - Yes
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V - Yes
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V - Yes
D - Yes
D - Yes
D+V - No
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For peer review only

The outcome is reported in the abstract

D - Yes
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V - Yes
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D+V - Yes
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For peer review only

Statistical methods are described in the abstract For model development, at least the type of statis

D - No
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D - No
D - Yes
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D - Yes
D - No
V - Yes
D - No
D - Yes
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V - Yes
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D - Yes
D+V - No
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For peer review only

Results for model discrimination are reported in the abstract This should be reported separately fo

D - No
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V - Yes
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D+V - Yes
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For peer review only

Results for model calibration are reported in the abstract This should be reported separately for de

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D+V - No
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For peer review only

Conclusions are reported in the abstract In publications addressing both model development and v

D - Yes
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V - Yes
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V - Yes
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D+V - Yes
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For peer review only

The number of events (or % outcome together with overall sample size) is reported in the abstract.

D - No
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D+V - No
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For peer review only

2 - Abstract - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y' or 'NA'

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The background and rationale are presented

D - Yes
D - Yes
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D - Yes
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V - Yes
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D+V - Yes
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For peer review only

Reference to existing models is included (or stated that there are no existing models)

D - Yes
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D - Yes
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D - Yes
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D - No
V - Yes
D - Yes
D - Yes
D+V - Yes
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For peer review only

3a - Background and Objectives - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y'

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For peer review only

It is stated whether the study describes development and/or validation and/or incremental (added

D - No
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V - Yes
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V - Yes
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D+V - Yes
D - Yes
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For peer review only

3b - Background and objectives - Score: D, V, IV, D+V: Score 1 if element is scores as 'Y'

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For peer review only

The study design/source of data is described. E.g. Prospectively designed, existing cohort, existing I

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V - Yes
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V - Yes
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D+V - Yes
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For peer review only

4a - Source of Data - Score: D, V, IV, D+V: Score 1 if element is scored as 'Y'

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For peer review only

The starting date of accrual is reported	The end date of accrual is reported
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D - Yes	D - Yes
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D - Yes	D - Yes
D - R	D - No
D - Yes	D - Yes
D - Yes	D - Yes
D - No	D - No
D - Yes	D - Yes
D - No	D - No
D - No	D - No
D - No	D - No
V - Yes	V - Yes
D - R	D - R
D - No	D - No
D - No	D - No
D - Yes	D - Yes
V - R	V - R
D - No	D - No
D - No	D - No
D+V - No	D+V - No
D - Yes	D - Yes
D - R	D - R
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For peer review only

The length of follow-up and prediction horizon/time frame are reported, if applicable. E.g. 'Patients

D - Yes
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D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - No
D - Yes
D - Yes
D - Yes

For peer review only

4b - Source of Data - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y', 'NA', or 'R'

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For peer review only

The study setting is reported (e.g. primary care, secondary care, general population). E.g.: 'surgery

D - Yes
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D - R
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D - Yes
D - R
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D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - R
D - Yes
D - Yes
D - R

For peer review only

The number of centres involved is reported If the number is not reported explicitly, but can be con

D - Yes
D - No
D - R
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D - R
D - Yes
D - Yes
D - Yes
D - Yes
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V - Yes
D - No
D - No
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D - Yes
V - R
D - No
D - No
D+V - R
D - Yes
D - Yes
D - R

For peer review only

The geographical location (at least country) of centres involved is reported If no geographical locati

D - Yes
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D - Yes
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V - Yes
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D - Yes
D - R
V - Yes
D - Yes
D - Yes
D+V - Yes
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For peer review only

5a - Participants - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y' or 'R'

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For peer review only

In-/exclusion criteria are stated. These should explicitly be stated. Reasons for exclusion only descr

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D - Yes
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V - Yes
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V - Yes
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D+V - Yes
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For peer review only

5b - Participants - Score: D, V, IV, D+V: Score 1 if element is scored as 'Y'

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Details of any treatments received are described. This item is notably for prognostic modelling stud

D - NA
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D - NA
D - No
D - No
D - NA
D - NA
V - No
D - NA
D - No
D - NA
D - NA
V - NA
D - NA
D - NA
D+V - NA
D - NA
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For peer review only

5c - Participants - Score: D, V, IV, D+V: Score 1 if element is scored as 'Y'; score Not applicable if ele

Not applicable
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For peer review only

The outcome definition is clearly presented This should be reported separately for development an

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D - Yes
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D - Yes
V - Yes
D - Yes
D - No
D - Yes
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V - No
D - Yes
D - No
D+V - No
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For peer review only

It is described how outcome was assessed (including all elements of any composite, for example CV

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D - Yes
D - Yes
D - Yes
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V - Yes
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D - Yes
D - Yes
D - Yes
V - R
D - Yes
D - Yes
D+V - No
D - Yes
D - Yes
D - Yes

For peer review only

It is described when the outcome was assessed (time point(s) since T0)

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - No
D - Yes
D - Yes
D - Yes

For peer review only

6a - Outcome - Score: D, V, IV, D+V: Score 1 if all elements are scores 'Y' or 'R'

1
1
1
1
1
1
0
1
1
1
1
1
0
0
1
1
0
1
1
1
0
1
0
0
1
0
0
0
1
1

For peer review only

Actions to blind assessment of outcome to be predicted are reported If it is clearly a non-issue (e.g.

D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
V - No
D - No
D - No
D - No
D - No
V - No
D - Yes
D - No
D+V - No
D - No
D - No
D - No

For peer review only

6b - Outcome - Score: D, V, I+V, D+V: Score 1 if element is scores as 'Y'.

0
0
0
0
0
0
0
0
0
0
0
0
0
0
1
0
0
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0
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0
1
0
0
0
0
0

For peer review only

All predictors are reported For development, 'all predictors' refers to all predictors that potentially

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - No
D - Yes
D - Yes
D - No

For peer review only

Predictor definitions are clearly presented

D - Yes
D - Yes
D - Yes
D - Yes
D - R
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - R
D - Yes
D - Yes
V - R
D - Yes
D - Yes
D+V - No
D - R
D - R
D - R

For peer review only

It is clearly described how the predictors were measured

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - R
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - No
D - R
D - R
D - Yes

For peer review only

It is clearly described when the predictors were measured

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - No
D - Yes
D - Yes
D - Yes

For peer review only

7a - Predictors - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y' or 'R'

1
1
1
1
1
1
1
1
1
1
1
1
1
0
1
1
0
1
1
1
1
1
1
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1
1
1
0
1
1
1

For peer review only

It is clearly described whether predictor assessments were blinded for outcome For predictors for v

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

It is clearly described whether predictor assessments were blinded for the other predictors

D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
V - No
D - No
D - No
D - No
D - No
D - No
V - No
D - No
D - No
D+V - No
D - No
D - No
D - No

For peer review only

7b - Predictors - Score: D, V, IV, D+V: Score 1 if both elements are scored as 'Y'

0
0
0

It is explained how the study size was arrived at Is there any mention of sample size, e.g. whether t

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - No
D - Yes
D - No
D - No
D - Yes
D - No
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
V - No
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

8 - Sample size - Score: D, V, IV, D+V: Score 1 if element is scored as 'Y'

1
1
1
1
1
0
0
1
0
0
1
0
1
0
1
1
1
0
0
1
1
1
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1
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1
1
1
1
1

For peer review only

The method for handling missing data (predictors and outcome) is mentioned E.g. Complete case (e

D - No
D - Yes
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - Yes
D - No
D - Yes
D - No
D - Yes
D - No
V - Yes
D - Yes
D - No
D - No
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - No
D - No

For peer review only

If missing data were imputed, details of the software used are given. When under 9i explicit mentic

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - No
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - NA
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - NA
D - NA
D - NA
D - NA

For peer review only

If missing data were imputed, a description of which variables were included in the imputation pro

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - No
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - NA
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - NA
D - NA
D - NA
D - NA

For peer review only

If multiple imputation was used, the number of imputations is reported When under 9i explicit me

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - No
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - NA
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - NA
D - NA
D - NA
D - NA

For peer review only

9 - Missing data - Score: D, V, IV, D+V: Score 1 if all elements are scored as 'Y' or 'NA'

0
1
0
0
0
0
0
0
1
1
0
0
1
0
1
0
1
0
1
1
0
0
1
1
1
1
1
1
0
0

For peer review only

For continuous predictors it is described whether they were modelled as linear, nonlinear (type of t

D - No
D - No
D - Yes
D - Yes
D - Yes
D - No
D - NA
D - Yes
D - NA
D - No
D - Yes
D - Yes
D - Yes
D - No
D - No
D - Yes
D - Yes
D - No
V - NA
D - Yes
D - No
D - No
D - Yes
V - NA
D - NA
D - Yes
D+V - No
D - Yes
D - No
D - No

For peer review only

For categorical or categorized predictors, the cut-points were reported If no categorical or categori:

D - NA
D - NA
D - Yes
D - Yes
D - Yes
D - NA
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
V - NA
D - Yes
D - NA
D - Yes
D - Yes
V - NA
D - Yes
D - Yes
D+V - No
D - Yes
D - No
D - No

For peer review only

For categorized predictors the method to choose the cut-points was clearly described. If no categor

D - NA
D - NA
D - Yes
D - Yes
D - Yes
D - NA
D - Yes
D - Yes
D - NA
D - No
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - NA
V - NA
D - Yes
D - NA
D - Yes
D - Yes
V - NA
D - NA
D - Yes
D+V - No
D - Yes
D - No
D - NA

For peer review only

10a - Statistical analysis methods - Score: D, IV, D+V: Score 1 if all elements are scored as 'Y' or 'NA

0
0
1
1
1
0
1
1
1
0
1
1
1
0
0
0
1
0

Not applicable

1
0
1
1

Not applicable

1
1
0
1
0
0

For peer review only

The type of statistical model is reported E.g. Logistic, Cox, other regression model (e.g. Weibull, ord

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - NA
D - Yes
D - Yes
D - Yes
D - Yes
V - NA
D - No
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

Testing of interaction terms is described If it is explicitly mentioned that interaction terms were not

D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - No
D - No
D - Yes
D - No
D - Yes
D - No
D - No
D - Yes
D - Yes
D - No
V - NA
D - Yes
D - No
D - No
D - No
V - NA
D - No
D - No
D+V - No
D - No
D - No
D - No

For peer review only

Internal validation is reported E.g. Bootstrapping, cross validation, split sample. If the use of intern

D - Yes
D - Yes
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - No
D - Yes
D - No
D - Yes
D - No
D - No
D - No
D - Yes
D - No
V - NA
D - No
D - Yes
D - Yes
D - No
V - NA
D - No
D - No
D+V - Yes
D - No
D - No
D - Yes

For peer review only

The approach used for predictor selection BEFORE modelling is described 'Before modelling' means

D - No
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - No
D - Yes
D - NA
D - No
V - NA
D - Yes
D - NA
D - Yes
D - Yes
V - NA
D - No
D - Yes
D+V - Yes
D - NA
D - No
D - No

For peer review only

The approach used for predictor selection DURING modelling is described E.g. Univariable analysis,

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - NA
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - No
D - NA
D - Yes
D - Yes
D - Yes
D - Yes
V - NA
D - NA
D - Yes
D - Yes
D - NA
V - NA
D - No
D - Yes
D+V - Yes
D - NA
D - No
D - Yes

For peer review only

Testing of the proportionality of hazards in survival models is described. If no proportional hazard r

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - No
D - NA
D - NA
D - NA
V - NA
D - No
D - NA
D - NA
D - NA
V - NA
D - NA
D - NA
D+V - NA
D - NA
D - NA
D - NA

For peer review only

10b - Statistical analysis methods: Score D, IV, D+V: Score 1 if all elements are scored as 'Y' or 'NA'

0
0
0
1
0
1
1
1
1
0
1
0
0
1
0
0
1
0
Not applicable
0
0
1
0
Not applicable
0
0
0
Not applicable
0
0

For peer review only

It is described how predictions for individuals (in the validation set) were obtained from the model

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - Yes
D - NA
D - NA
D - NA

For peer review only

10c - Statistical analysis methods: Score V, IV, D+V: Score 1 if extraction item is scored as 'Y' D: Sco

Not applicable
Not applicable
Not applicable

Measures for model discrimination are described. E.g. C-index / area under the ROC curve

D - Yes
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
V - Yes
D - Yes
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - No
D - No
D - Yes

For peer review only

Measures for model calibration are described E.g. calibration plot, calibration slope or intercept, ca

D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
V - No
D - No
D - No
D - No
D - No
V - Yes
D - No
D - No
D+V - No
D - No
D - No
D - No

For peer review only

Other performance measures are described E.g. R2, Brier score, predictive values, sensitivity, speci

D - No
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
D - No
D - Yes
V - Yes
D - No
D - Yes
D - No
D - Yes
V - No
D - Yes
D - Yes
D+V - No
D - Yes
D - No
D - Yes

For peer review only

10d - Statistical analysis methods - Score D, V, IV, D+V: Score 1 if element 10di and 10dii are scored
0
0
0
1
0
0
0
1
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0
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1
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0

For peer review only

A description of model-updating is given. E.g. Intercept recalibration, regression coefficient recalibr

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA
V - NA
D - NA
D - NA
D+V - NA
D - NA
D - NA
D - NA

For peer review only

10e - Statistical analysis methods - Score: V, IV, D+V: Score 1 if element is scores as 'Y'; score Not a

Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable

0

Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable

For peer review only

If risk groups were created, risk group boundaries (risk thresholds) are specified Score this item sep

D - NA
D - NA
D - NA
D - Yes
D - Yes
D - NA
D - Yes
D - Yes
D - Yes
D - NA
D - Yes
D - Yes
D - NA
D - NA
D - Yes
D - NA
D - Yes
D - NA
V - NA
D - NA
D - NA
D - NA
D - NA
V - NA
D - Yes
D - NA
D+V - NA
D - Yes
D - NA
D - Yes

For peer review only

11 - Risk groups - Score: Score 1 if element is scores as 'Y'; score Not Applicable if element is scorec

Not applicable
Not applicable
Not applicable
1
1
Not applicable
1
1
1
Not applicable
1
1
Not applicable
Not applicable
1
Not applicable
1
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
1
Not applicable
Not applicable
1
Not applicable
1

For peer review only

Differences or similarities in definitions with the development study are described. Mentioning of a

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D+V - Yes
D - NA
D - NA
D - NA

For peer review only

12 - Development vs. validation V, D+V: Score 1 if element is scored as 'Y' IV: Score 1 if element is
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
1
Not applicable
Not applicable
Not applicable
Not applicable
0
Not applicable
Not applicable
1
Not applicable
Not applicable
Not applicable

For peer review only

The flow of participants is reported

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

The number of participants with and without the outcome are reported If outcomes are continuou

D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - No
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

A summary of follow-up time is presented This notably applies to prognosis studies and diagnostic

D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
V - Yes
D - No
D - No
D - No
D - Yes
V - Yes
D - Yes
D - No
D+V - No
D - No
D - No
D - No

For peer review only

13a - Results - Participants: D, V, IV, D+V: Score 1 if all elements are scored as 'Y' or 'NA'

0
0
0
0
0
0
0
1
1
0
0
0
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1
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1
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1
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1
0
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0
0

For peer review only

Basic demographics are reported

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - No

For peer review only

Summary information is provided for all predictors included in the final developed/validated mode

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - No

For peer review only

The number of participants with missing data for predictors is reported

D - Yes
D - No
D - Yes
D - No
D - No
D - No
D - No
D - Yes
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
V - No
D - No
D - No
D - No
D - Yes
V - Yes
D - No
D - Yes
D+V - No
D - Yes
D - No
D - No

For peer review only

The number of participants with missing data for the outcome is reported

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - No
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - No
V - No
D - No
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

13b - Results - Participants: Score D, V, IV, D+V: Score 1 if all elements are scored as 'Y'

1
0
1
0
0
0
0
1
0
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0
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For peer review only

Demographic characteristics (at least age and gender) of the validation study participants are repor

D - NA
D - NA
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D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - Yes
D - NA
D - NA
D - NA

For peer review only

Distributions of predictors in the model of the validation study participants are reported along with

D - NA
D - NA
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D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - No
D - NA
D - NA
D - NA

For peer review only

Outcomes of the validation study participants are reported along with those of the original develop

D - NA
D - NA
D - NA
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D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - Yes
D - NA
D - NA
D - NA

For peer review only

13c - Results - Participants: Score V, D+V: Score 1 if all elements are scored as 'Y' IV: Score 1 if all e

Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
Not applicable
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Not applicable

For peer review only

The number of participants in each analysis (e.g. in the analysis of each model if more than one mo

D - No
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - NA
D - No
D - Yes
D - No
D - Yes
V - NA
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

The number of outcome events in each analysis is specified (e.g. in the analysis of each model if mc

D - No
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - Yes
D - Yes
D - Yes
D - Yes
V - NA
D - Yes
D - Yes
D - Yes
D - Yes
V - NA
D - No
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

14a - Model development: Score	D, IV, D+V: Score 1 if both elements are scored as 'Y' or 'NA'	V: Sc
		0
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Not applicable		
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		1
Not applicable		
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For peer review only

The unadjusted associations between each predictor and outcome are reported. If any univariable

D - NA
D - NA
D - Yes
D - Yes
D - NA
D - NA
D - NA
D - No
D - Yes
D - NA
D - NA
D - Yes
D - No
D - NA
D - Yes
D - No
D - NA
D - No
V - NA
D - Yes
D - NA
D - NA
D - NA
V - NA
D - NA
D - NA
D+V - NA
D - NA
D - NA
D - NA

For peer review only

14b - Model development - Score: D, IV, D+V: Score 1 if element i scored as 'Y'; score Not applicabl	
Not applicable	
Not applicable	
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Not applicable	
Not applicable	
Not applicable	
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Not applicable	
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Not applicable	
Not applicable	

For peer review only

The regression coefficient (or a derivative such as hazard ratio, odds ratio, risk ratio) for each predi

D - No
D - No
D - Yes
D - Yes
D - No
D - Yes
D - No
D - Yes
D - Yes
D - No
D - Yes
D - No
D - Yes
D - No
D - Yes
D - No
D - Yes
D - No
V - NA
D - No
D - Yes
D - Yes
D - No
V - NA
D - No
D - Yes
D+V - Yes
D - No
D - Yes
D - No

For peer review only

The intercept or the cumulative baseline hazard (or baseline survival) for at least one time point is

D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
V - NA
D - No
D - No
D - Yes
D - No
V - NA
D - No
D - No
D+V - Yes
D - No
D - Yes
D - No

For peer review only

15a - Model specification: Score D, IV, D+V: Score 1 if both elements are scored as 'Y' V: Score Not

0
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Not applicable

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Not applicable

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For peer review only

An explanation (e.g. a simplified scoring rule, chart, nomogram of the model, reference to online ca

D - No
D - No
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
D - No
D - Yes
D - No
D - Yes
D - No
V - NA
D - Yes
D - No
D - Yes
D - No
V - NA
D - Yes
D - Yes
D+V - No
D - Yes
D - No
D - Yes

For peer review only

15b - Model specification: Score D, IV, D+V: Score 1 if element is scored as 'Y' V: Score Not applica	
	0
	0
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	1
	1
	0
	1
	1
	0
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	1
	0
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Not applicable	
	1
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Not applicable	
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For peer review only

A discrimination measure is presented. E.g. C-index / area under the ROC curve

D - Yes
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
V - Yes
D - Yes
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - No
D - No
D - Yes

For peer review only

The confidence interval (or standard error) of the discrimination measure is presented

D - Yes
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
V - Yes
D - Yes
D - No
D - Yes
D - Yes
V - Yes
D - Yes
D - No
D+V - No
D - No
D - No
D - No

For peer review only

Measures for model calibration are described. E.g. calibration plot, calibration slope or intercept, c

D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
V - No
D - No
D - No
D - No
D - No
V - Yes
D - No
D - No
D+V - No
D - No
D - Yes
D - No

For peer review only

Other model performance measures are presented. E.g. R2, Brier score, predictive values, sensitivit

D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - No
D - No
D - Yes
V - Yes
D - No
D - Yes
D - No
D - Yes
V - No
D - Yes
D - Yes
D+V - No
D - Yes
D - Yes
D - Yes

For peer review only

16 - Model performance: Score D, V, IV, D+V: Score 1 if elements 16i, 16ii, and 16iii are scored as 'Y	
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For peer review only

The updated regression coefficients for each predictor in the model are reported If model updating

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA

D - NA
D - NA

D - NA
D - NA
D - NA

For peer review only

The updated intercept or cumulative baseline hazard or baseline survival (for at least one time poir

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA

D - NA
D - NA

D - NA
D - NA
D - NA

For peer review only

The discrimination of the updated model is reported

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D - NA
D - NA

D - NA
D - NA

D - NA
D - NA
D - NA

For peer review only

The confidence interval (or standard error) of the discrimination measure of the updated model is r

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D - NA
D - NA

D - NA
D - NA

D - NA
D - NA
D - NA

For peer review only

The calibration of the updated model is reported

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA

D - NA
D - NA

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D - NA
D - NA

For peer review only

17 - Model updating: Score If updating was not done, score this TRIPOD item as 'Not applicable'. V

Not applicable
Not applicable
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Not applicable
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Not applicable

For peer review only

Limitations of the study are discussed Stating any limitation is sufficient.

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

18 - Limitations: Score D, V, IV, D+V: Score 1 if element is scored as 'Y'

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Comparison of results to reported performance in development studies and/or other validation stu

D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
D - NA
V - No
D - NA
D - NA
D - NA
D - NA
V - Yes
D - NA
D - NA
D+V - Yes
D - NA
D - NA
D - NA

For peer review only

19a - Interpretation: Score V, IV, D+V: Score 1 if elementis scored as 'Y' D: Score Not applicable

Not applicable	
Not applicable	
Not applicable	
Not applicable	
Not applicable	
Not applicable	
Not applicable	
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Not applicable	
Not applicable	

For peer review only

An overall interpretation of the results is given

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

19b - Interpretation: Score D, V, IV, D+V: Score 1 if element is scored as 'Y'

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The potential clinical use is discussed. E.g. an explicit description of the context in which the predic

D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

Implications for future research are discussed. E.g. a description of what the next stage of investiga

D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D - No
D - Yes
V - Yes
D - No
D - No
D+V - Yes
D - Yes
D - No
D - Yes

For peer review only

20 - Implications: Score D, V, IV, D+V: Score 1 if both elements are scored as 'Y'

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For peer review only

Information about supplementary resources is provided

D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - Yes
D - No
D - No
D - Yes
V - No
D - No
D - No
D - Yes
D - No
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - No
D - No

For peer review only

The source of funding is reported or there is explicit mention that there was no external funding in

D - Yes
D - No
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - Yes
D - No
D - Yes
D - Yes
D - Yes
V - No
D - No
D - Yes
D - Yes
D - Yes
V - Yes
D - Yes
D - Yes
D+V - Yes
D - Yes
D - Yes
D - Yes

For peer review only

The role of funders is reported or there is explicit mention that there was no external funding

D - No
D - No
D - No
D - No
D - No
D - No
D - No
D - Yes
D - No
D - No
D - No
D - Yes
D - No
D - Yes
D - No
D - Yes
D - Yes
D - No
V - No
D - No
D - Yes
D - No
D - Yes
V - No
D - No
D - Yes
D+V - Yes
D - No
D - No
D - No

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22 - Funding: Score D, V, IV, D+V: Score 1 if both elements are scored as 'Y'

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Overall TRIPOD adherence score: Developmental studies

4,074,074,074,074,070
37,037,037,037,037
5,714,285,714,285,710
6,206,896,551,724,130
5,714,285,714,285,710
40,740,740,740,741
44,827,586,206,896,496
7,586,206,896,551,720
44,827,586,206,897
44,444,444,444,444
6,071,428,571,428,570
55,172,413,793,103
3,448,275,862,068,960
3,703,703,703,703,700
70
4,137,931,034,482,750
6,071,428,571,428,570
35,714,285,714,285,700
85,714,285,714,286
50
35,714,285,714,285,700
48,148,148,148,148,096
55,555,555,555,556
9,629,629,629,629,620
4,642,857,142,857,140
5,185,185,185,185,180
48,148,148,148,148,096
67,857,142,857,143
4,444,444,444,444,440
5,357,142,857,142,850

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Overall TRIPOD adherence score: Validation studies

7,307,692,307,692,300
65,384,615,384,615
8,076,923,076,923,070
7,407,407,407,407,400
7,777,777,777,777,770
65,384,615,384,615
6,428,571,428,571,420
9,629,629,629,629,620
66,666,666,666,667
73,076,923,076,923
7,777,777,777,777,770
74,074,074,074,074
6,296,296,296,296,290
65,384,615,384,615
9,285,714,285,714,280
7,037,037,037,037,030
7,777,777,777,777,770
6,538,461,538,461,530
51,724,137,931,034
7,307,692,307,692,300
6,296,296,296,296,290
6,538,461,538,461,530
80,769,230,769,231
7,307,692,307,692,300
7,037,037,037,037,030
7,307,692,307,692,300
5,384,615,384,615,380
7,777,777,777,777,770
6,923,076,923,076,920
7,777,777,777,777,770

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Overall TRIPOD adherence score: Incremental value studies

6,129,032,258,064,510
58,064,516,129,032
75
7,878,787,878,787,870
75
61,290,322,580,645
6,363,636,363,636,360
909,090,909,090,909
63,636,363,636,364
64,516,129,032,258
78,125
72,727,272,727,273
5,454,545,454,545,450
5,806,451,612,903,220
8,529,411,764,705,880
6,060,606,060,606,060
78,125

56.25

75,757,575,757,576

68.75

56.25

6,774,193,548,387,090
74,193,548,387,097
9,354,838,709,677,410
65,625
7,096,774,193,548,380
5,161,290,322,580,640

81.25

6,451,612,903,225,800
71,875

For peer review only

Overall TRIPOD adherence score: Developmental studies with external validation	Complete?
65,625	Complete
62.5	Complete
7,878,787,878,787,870	Complete
8,235,294,117,647,050	Complete
7,878,787,878,787,870	Complete
65,625	Complete
6,764,705,882,352,940	Complete
9,411,764,705,882,350	Complete
67,647,058,823,529	Complete
68.75	Complete
8,181,818,181,818,180	Complete
76,470,588,235,294	Complete
5,882,352,941,176,470	Complete
62.5	Complete
8,857,142,857,142,850	Complete
6,470,588,235,294,110	Complete
8,181,818,181,818,180	Complete
6,060,606,060,606,060	Complete
73,529,411,764,706	Complete
7,272,727,272,727,270	Complete
6,060,606,060,606,060	Complete
71,875	Complete
78,125	Complete
96,875	Complete
696,969,696,969,697	Complete
75	Complete
56.25	Complete
84,848,484,848,485	Complete
68.75	Complete
7,575,757,575,757,570	Complete

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Final number of candidate predictors:

71
32
34
35
32
23
5
38
22
4
42
28
28
37
22
6
31
23
7
9
96
15
7
27
3
28
580
7
17
32

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Section/topic	#	Checklist item	Reported on page #
TITLE			
Title	1	Identify the report as a systematic review, meta-analysis, or both.	#2
ABSTRACT			
Structured summary	2	Provide a structured summary including, as applicable: background; objectives; data sources; study eligibility criteria, participants, and interventions; study appraisal and synthesis methods; results; limitations; conclusions and implications of key findings; systematic review registration number.	#4
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of what is already known.	#6
Objectives	4	Provide an explicit statement of questions being addressed with reference to participants, interventions, comparisons, outcomes, and study design (PICOS).	#6
METHODS			
Protocol and registration	5	Indicate if a review protocol exists, if and where it can be accessed (e.g., Web address), and, if available, provide registration information including registration number.	#6
Eligibility criteria	6	Specify study characteristics (e.g., PICOS, length of follow-up) and report characteristics (e.g., years considered, language, publication status) used as criteria for eligibility, giving rationale.	#6-8
Information sources	7	Describe all information sources (e.g., databases with dates of coverage, contact with study authors to identify additional studies) in the search and date last searched.	#8
Search	8	Present full electronic search strategy for at least one database, including any limits used, such that it could be repeated.	#8 + Appendix 3
Study selection	9	State the process for selecting studies (i.e., screening, eligibility, included in systematic review, and, if applicable, included in the meta-analysis).	#8-9 + Appendix 4
Data collection process	10	Describe method of data extraction from reports (e.g., piloted forms, independently, in duplicate) and any processes for obtaining and confirming data from investigators.	#9
Data items	11	List and define all variables for which data were sought (e.g., PICOS, funding sources) and any assumptions and simplifications made.	#9 + Appendix 5
Risk of bias in individual studies	12	Describe methods used for assessing risk of bias of individual studies (including specification of whether this was done at the study or outcome level), and how this information is to be used in any data synthesis.	#9-10
Summary measures	13	State the principal summary measures (e.g., risk ratio, difference in means).	#10

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Synthesis of results	14	Describe the methods of handling data and combining results of studies, if done, including measures of consistency (e.g., I^2) for each meta-analysis.	**NA**

Section/topic	#	Checklist item	Reported on page #
Risk of bias across studies	15	Specify any assessment of risk of bias that may affect the cumulative evidence (e.g., publication bias, selective reporting within studies).	#10
Additional analyses	16	Describe methods of additional analyses (e.g., sensitivity or subgroup analyses, meta-regression), if done, indicating which were pre-specified.	**NA**
RESULTS			
Study selection	17	Give numbers of studies screened, assessed for eligibility, and included in the review, with reasons for exclusions at each stage, ideally with a flow diagram.	#10 + Fig 1 + Appendix 6-8
Study characteristics	18	For each study, present characteristics for which data were extracted (e.g., study size, PICOS, follow-up period) and provide the citations.	#10-12 + Fig 2 + Appendix 9-10
Risk of bias within studies	19	Present data on risk of bias of each study and, if available, any outcome level assessment (see item 12).	#12 + Table 1
Results of individual studies	20	For all outcomes considered (benefits or harms), present, for each study: (a) simple summary data for each intervention group (b) effect estimates and confidence intervals, ideally with a forest plot.	#12
Synthesis of results	21	Present results of each meta-analysis done, including confidence intervals and measures of consistency.	**NA**
Risk of bias across studies	22	Present results of any assessment of risk of bias across studies (see Item 15).	#12
Additional analysis	23	Give results of additional analyses, if done (e.g., sensitivity or subgroup analyses, meta-regression [see Item 16]).	**NA**
DISCUSSION			
Summary of evidence	24	Summarize the main findings including the strength of evidence for each main outcome; consider their relevance to key groups (e.g., healthcare providers, users, and policy makers).	#12-13
Limitations	25	Discuss limitations at study and outcome level (e.g., risk of bias), and at review-level (e.g., incomplete retrieval of identified research, reporting bias).	#13
Conclusions	26	Provide a general interpretation of the results in the context of other evidence, and implications for future research.	#13-15
FUNDING			

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Funding	27	Describe sources of funding for the systematic review and other support (e.g., supply of data); role of funders for the systematic review.	#10

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BMJ Open

Predicting falls in community-dwelling older adults: a systematic review of prognostic models

Journal:	*BMJ Open*
Manuscript ID	bmjopen-2020-044170.R1
Article Type:	Original research
Date Submitted by the Author:	24-Feb-2021
Complete List of Authors:	Gade, Gustav; Aalborg University Hospital, Department of Geriatric Medicine; Aalborg University, Department of Clinical Medicine Jørgensen, Martin; Aalborg University Hospital, Department of Geriatric Medicine Ryg, Jesper; Odense University Hospital, Department of Geriatric Medicine; University of Southern Denmark, Department of Clinical Research Riis, Johannes; Aalborg University Hospital, Department of Geriatric Medicine; Aalborg University, Department of Clinical Medicine Thomsen , Katja ; Odense University Hospital, Department of Geriatric Medicine; University of Southern Denmark, Department of Clinical Research Masud, Tahir ; Nottingham University Hospitals NHS Trust, Department of Healthcare for Older People Andersen, Stig; Aalborg University Hospital, Department of Geriatric Medicine; Aalborg University, Department of Clinical Medicine
Primary Subject Heading:	Geriatric medicine
Secondary Subject Heading:	Public health
Keywords:	GERIATRIC MEDICINE, PUBLIC HEALTH, STATISTICS & RESEARCH METHODS

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Title page

Title: Predicting falls in community-dwelling older adults: a systematic review of prognostic models

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Keywords:
- Accidental falls
- Aged
- Models, Theoretical
- Multivariate analysis
- Prognosis

Word count: 4,996.

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Abstract

Objective

To systematically review and critically appraise prognostic models for falls in community-dwelling older adults.

Eligibility criteria

Prospective cohort studies with any follow-up period. Studies had to develop or validate multifactorial prognostic models for falls in community-dwelling older adults (60+ years). Models had to be applicable for screening in a general population setting.

Information source

MEDLINE, EMBASE, CINAHL, The Cochrane Library, PsycINFO, and Web of Science for studies published in English, Danish, Norwegian, or Swedish until January 2020. Sources also included trial registries, clinical guidelines, reference lists of included papers, along with contacting clinical experts to locate published studies.

Data extraction and risk of bias

Two authors performed all review stages independently. Data extraction followed the CHARMS (Critical appraisal and data extraction for systematic reviews of prediction modelling studies) checklist. Risk of bias assessments on participants, predictors, outcomes, and analysis methods followed PROBAST (Prediction model risk of bias assessment tool).

Results

After screening 11,789 studies, 30 were eligible for inclusion (n = 86,369 participants). Median age of participants ranged from 67.5-83.0 years. Falls incidences varied from 5.9-59%. Included studies reported 69 developed and three validated prediction models. Most frequent falls predictors were prior falls, age, sex, measures of gait, balance and strength, along with vision and disability. The area under the curve was available for 40 (55.6%) models, ranging from 0.49-0.87. Validated models' AUCs ranged from 0.62-0.69. All models had a high risk of bias, mostly due to limitations in statistical methods, outcome assessments, and restrictive eligibility criteria.

Conclusions

An abundance of prognostic models on falls risk have been developed, but with a wide range in discriminatory performance. All models exhibited a high risk of bias rendering them unreliable for prediction in clinical practice. Future prognostic prediction models should comply with recent recommendations such as TRIPOD.

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PROSPERO registration number: CRD42019124021

Strengths and limitations of this study

- This systematic review is the first to summarise all prediction models on falls in community-dwelling older adults of the general population.
- The extensive search strategy supports identifying all available prospective cohort studies predicting falls in community-dwelling older adults (60+ years).
- Guidelines on prediction modelling reviews were strictly followed for search strings, data extraction (CHARMS), risk of bias assessment (PROBAST), along with development (PRISMA-P) and transparent reporting of the review (PRISMA).
- All review stages were performed independently and in duplicate.
- The exclusion of non-English language studies constitutes a risk of selection bias.

Background

The propensity to fall is a serious and common health issue among older adults with one-third of community-dwelling adults ≥65 years and half of those ≥80 years falling annually.[1] Consequences of falls are considerable with loss of independence, increased morbidity, and mortality.[2–4] Furthermore, the healthcare costs of falls increase substantially with age.[5] Therefore, as the prevalence of older fallers is predicted to increase with changes in demography, preventing falls is of utmost importance.[6]

Falls interventions have proven effective when aimed at older adults with a high risk of falling.[7] However, identifying these high-risk individuals is not straight-forward since falling is multifactorial. Prognostic models combine risk factors to estimate the individual's risk of a future outcome.[8] Thus, a prognostic model may be a valuable tool to discriminate between older adults at high versus low risk of falling. To prevent the consequences of falls, healthcare professionals could perform screening in the general population using prognostic models.[9] However, no systematic review has addressed prognostic models on falls for community-dwelling older adults.

This systematic review aims to provide an updated overview of available models to be used by healthcare professionals and for researchers to improve upon. The primary objective was to describe the discriminatory performance of prognostic models for falls in prospective cohort studies on community-dwelling older adults. Secondary objectives were to describe the study and model characteristics of these models.

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Methods

A protocol was preregistered at the International Register of Systematic Reviews (PROSPERO) before commencing the review process (CRD: CRD42019124021)[10] and is available in data supplements (Appendix 1). The review and its protocol followed the Preferred Reporting Items for Systematic reviews and Meta-Analyses statement (PRISMA)[11] and PRISMA-Protocols[12], respectively. A completed PRISMA Checklist is available in data supplements (Appendix 2). During the review process, we found the protocol unclear in terms of eligibility criteria for the study designs, participants, models, outcomes, and settings, for which reason we have further described these. Rationales for the changes are given in the protocol. Box 1 provides an introduction to commonly used prediction modelling terms.

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Box 1: Commonly used prediction modelling terms with examples related to falls

Prognostic factor

A prognostic factor, also called a predictor, is any measure that, among people with a given health condition, is associated with a subsequent clinical outcome such as falls.[13]

Prognostic prediction model

A prognostic prediction model is a statistical combination of multiple predictors from which risks of a longitudinal outcome, e.g. falls, can be calculated for individuals.[8]

Development and validation studies

A prediction model development study aims to develop a prediction model by combining essential predictors from a data set into a model and testing its predictive performance within the same development data set.[14] A model validation study aims to assess the predictive performance of a developed prediction model using new data not used in the development of the model.[14]

Model performance, overfitting, and internal validation

A model's predictive performance is termed model performance. This term encompasses several measures with the two most important being discrimination and calibration.[15] Estimates of model performance derived directly from a data set used for developing the model is termed the apparent performance.[14] Since the model is fitted explicitly to the development data set, predictions on new data, i.e. new older adults with different characteristics, may yield poorer model performance estimates, i.e. poor generalisability. Hence, clinicians would typically find the apparent performance optimistic in terms of predicting a fall in their population which has not been used for developing the model. In consequence, fall preventive interventions could end up being provided to those not needing it and not offered to those actually in need hereof. The optimism in apparent performance is due to the model fitting too well to its data, a term known as overfitting. In such situations, predictions would be biased when the model is used on older adults with different characteristics, i.e. frequency distributions of predictors.[15] Estimating the amount of optimism in the development study's model can be done using internal validation techniques such as bootstrap validation. However, since the population of older adults is heterogeneous, generalising a model's performance to the entire population would be more clinically relevant. Here, internal validation procedures fail, and the model should instead be tested in a validation study.

Model discrimination

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Model discrimination is a performance measure referring to the models' ability to correctly predict if an individual will experience a fall or not. Therefore, as an example, it can be used by healthcare professionals to assess how confident a model assigns individuals to a high-risk group and guides the clinician when allocating fall preventive interventions.[16] A perfectly discriminating model assigns a higher risk of falling to all older adults experiencing a fall. Likewise, a lower risk is appointed to those not suffering a fall. Usually, discrimination is reported as a concordance index (c-index) or an area under the curve (AUC), but other measures are also available. Here, a value of 1 equals perfect discrimination, and 0.5 indicates that the model discriminates no better than chance. If a model shows poor discriminative performance, it could predict low-risk older adults to fall and high-risk older adults to not fall.

Model calibration

Model calibration is a performance measure used to examine whether a model over- or underestimates the predicted risks in a sample. More specifically, it is the agreement between predictions made by the model and the frequency of the outcome to be predicted.[14] Healthcare professionals can utilise this information to assess how confident the model predicts the specific risk of having a fall for the individual. In brief, it is crucial when counselling older adults on their fall risk that the risk estimate is as accurate as possible.[16] If the model predicts a person to have a 10% risk of falling within one year, the observed frequency of people falling with such a predicted risk should be 10 out of 100 for the model to have good calibration. However, should the frequency be only 5 of 100 people, the model overestimates the risk. Calibration is typically assessed graphically using calibration plots. In development studies, models are usually calibrated well to the data from which they are developed and therefore yield limited information.[4] Thus, it is more relevant how well the model is calibrated when introduced to a new sample of older adults used to validate the model. This information would enable healthcare professionals to evaluate whether the model over- or underestimates the risk of falling when used in their population of community-dwelling older adults. If the model is not correctly calibrated, it could predict low-risk older adults to have a higher risk and vice versa, or systematically over- or underestimate all predictions.

Eligibility criteria

Participants and setting

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All participants had to be community-dwelling, 60 years of age or older, and be recruited from a general population setting. For that reason, we excluded models intended for hospitals, general practitioners, and nursing homes. Studies restricted to participants with pre-specified diseases, conditions, or symptoms such as Parkinson's disease or stroke were excluded to raise external validity. However, we included studies that excluded certain types of community-dwelling older adults, such as those with known neurological, spinal, or cognitive disorders.

Index (Model)

Studies had to present a final multifactorial prognostic model defined by the inclusion of two prognostic factors or more. This definition was chosen as causes of falls are multifactorial and coexisting.[1] Thus, prognostic factor studies investigating the association between predictors and prospective falls were excluded. Both development studies and validation studies with and without model updating were included.

Outcome

We included studies defining falls as "an unexpected event in which the participants come to rest on the ground, floor, or lower-level".[17] However, studies without an outcome definition were also included since this would not rule out the definition mentioned above. We excluded studies using fall definitions excluding certain types of falls presumed to be due to a specific cause, e.g. external forces or acute medical events. This approach was chosen since post-fall classification methods may introduce recall bias.[18] Finally, we did not include studies predicting only injurious falls in older adults since risk factors for these are different from those experiencing non-injurious falls.[19] No restrictions were made on method or timing of outcome assessment other than it had to be prospectively recorded.

Study designs

We only included prospective cohort studies since this study design allows optimal control when measuring predictors and outcomes. Thus, it is the recommended study design for prognostic modelling studies.[20] We excluded randomised controlled trials since these can have different limitations incorporated within their design. Typically, strict eligibility criteria are used that generate a highly selected sample of participants. This narrows predictors' distribution and hence reduces the discriminatory performance in the prognostic models.[15] Also, strict criteria may compromise generalisability to the target population.[21] Lastly, interventions in the study may also influence the discriminatory performance of the models.[21] Retrospective cohort studies were also

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excluded due to issues of missing data and restrictions on which predictors to apply since data is already collected.[21]

Timing

No restrictions on follow-up or predictive horizon were made since we found it clinically relevant to include models both able to predict falls within short and long periods ahead in time.

Language and publication year

Due to the composition of the study group, we only included published studies reported in English, Danish, Norwegian, or Swedish languages. No restrictions on publication year were made.

Information sources

We searched electronic databases, trial registries, and clinical guidelines. Furthermore, we consulted with additional clinical experts. Lastly, we screened conference abstracts along with reference lists of both the included studies and systematic reviews found during the search. Databases included Pubmed.gov (PubMed interface), EMBASE (Embase.com), CINAHL (EBSCOhost interface), The Cochrane Library (Wiley interface), PsycINFO (APA PsycNET interface), and Web of Science (Web of Science Core Collection). All databases were searched from inception dates to the 3rd of January 2020. Trial registries included PROSPERO, Clinicaltrials.gov, WHO International Clinical Trials Registry Platform, and Open Grey. Guidelines included Guidelines International Network (GIN), the National Institute for Health and Care Excellence (NICE), Centre for Reviews and Dissemination and to Health Technology Assessments (CRD/HTA), and Scottish Intercollegiate Guidelines Network (SIGN). Conference abstracts and studies in trial registries were used to obtain full-text papers through contact with authors. Letters to the editor were excluded.

Search

We used a validated search string for prediction models.[22] With the help of a research librarian in health science, we added the following terms to the search string: independent living, aged, and accidental falls. Details on the search string are available in data supplements (Appendix 3). No search filters were applied. We included "Aged" as a search term in the search string. Since this would restrict the number of search hits and thus the sensitivity of the search string, we pre-tested the search string without "Aged" in all databases before commencing the review. From this, the first 3,000 hits were screened independently and in duplicate, and we did not find studies not identified by the final search string. Thus, we believe this had a limited influence on the sensitivity of the search string.

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Study selection

Duplicates were removed using EndNote (EndNote X9, Clarivate Analytics, Philadelphia, USA). Two reviewers independently screened titles and abstracts (GVG and JRI) and full-text papers (GVG and KT) according to the inclusion criteria. We contacted authors for clarification when information on this review's eligibility criteria was missing. Disagreement among reviewers was resolved by consensus for one study by including a third reviewer (MGJ). For screening of titles and abstracts along with full-text reading, we used Covidence (Covidence systematic review software, Veritas Health Innovation, Melbourne, Australia. Available at www.covidence.org). Exclusion of studies after full-text reading was performed using a prioritised list of reasons (Appendix 4). Reviewers were not blinded to author names, institutions, or journal titles.

Data collection process

We developed a standardised data collection form using REDCap[23], a research electronic data capture software, following the Critical Appraisal and Data Extraction for Systematic Reviews of Prediction Modelling Studies (CHARMS) checklist.[21] Data extraction was performed in duplicate and independently by two reviewers (GVG and JRI). Independence between reviewers was ensured using a double data entry module in REDCap, thereby denying access to each other's responses. Disagreements among the reviewers were discussed, and the third reviewer (MGJ) was not consulted during data collection since consensus was reached in all studies. We contacted all study authors for retrieval of information on data items not reported. None of the included studies were published more than once.

Data items

We extracted data on the following items: country, publication year, authors, inclusion criteria, exclusion criteria, age, outcome definition, number of falls and fallers, candidate predictors, missing data, choice of statistical analysis, C-statistic and Area-Under the receiver-operating-characteristic Curve (AUC), internal and external validation procedures, final model presentation, and sources of funding. If available, 51 data items were extracted from each paper as detailed in data supplements (Appendix 5).

Risk of bias and reporting transparency

To follow current recommendations[24], the Prediction study Risk Of Bias Assessment Tool (PROBAST)[25] was used for the risk of bias assessment in individual studies. The tool comprises 20 signalling questions in four domains: participants, predictors, outcomes, and analysis. The tool also includes an evaluation of each model's applicability for the intended population, predictors and

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outcome of the review. Reporting transparency was assessed using the Transparent reporting of a multivariable prediction model for individual prognosis or diagnosis (TRIPOD) adherence assessment form.[26] The bias, applicability, and reporting assessments were performed in duplicate and independently by two reviewers (GVS and JRI). Independence between reviewers was ensured using a double data entry module in REDCap, thereby denying access to each other's responses. Disagreements among the reviewers were discussed, and consensus was reached for all studies. Thus, a third reviewer was not consulted for a final decision. The reviewers were not blinded to study authors, institutions, or journal titles. The results of the risk of bias assessments of all included studies were incorporated into the qualitative synthesis. We sought to investigate outcome reporting bias by comparing the study papers to their pertaining protocols to examine whether outcomes were pre-specified and not differing from the published paper.

Summary measures and planned method of analysis

The principal summary measure of this systematic review was the discriminatory performance measured either in a C-index or AUC. In the pre-specified protocol, we decided not to perform meta-analyses due to the presumed heterogeneity of the prognostic models. This assumption was confirmed after the review was complete. Furthermore, we summarised the study and model characteristics using ranges and percentage proportions when appropriate. When data were available, we summarised continuous measures using medians and interquartile ranges.

Patient public involvement

We did not involve patients or the public in the research.

Results

Study selection

The search yielded 19,612 publications with 11,789 remaining after removal of duplicates. Screening titles and abstracts led to the exclusion of 11,611 publications leaving 178 for full-text reading. Of these, 148 were excluded due to: wrong outcome (n = 45), wrong study design (n = 45), not being a prediction model (n = 25), no full-text paper published (n = 14), wrong population (n = 8), not multifactorial (n = 8), or wrong setting (n = 3). Thirty studies met the eligibility criteria and were included.[27-56] Figure 1 displays the PRISMA flow diagram for the study selection. Details on excluded papers can be found as data supplements (Appendix 6-8).

Characteristics of included studies

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A summary of included studies along with models' performance can be found in Supplementary Table 1 (Appendix 9). All studies were published in English from 1994 to 2019. Seventy-two prognostic models were reported, of which 69 models were developed, and three were validated.

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Table 1. Reporting, risk of bias, and applicability ratings of individual studies

Study	Reporting	Risk of bias				Applicability			Overall	
	TRIPOD	Participants	Predictors	Outcome	Analysis	Participants	Predictors	Outcome	ROB	Applicability
Yamashita et al.[48], 2011	34%	-	+	-	-	-	+	?	-	-
Weiss et al.[52], 2013	36%	+	+	-	-	-	+	+	-	-
Muhaidat et al.[27], 2014	36%	+	+	?	-	+	+	?	-	?
Brauer et al.[25], 2000	37%	-	+	?	-	-	-	+	-	-
Panzer et al.[50], 2011	37%	-	+	-	-	-	+	?	-	?
Maki et al.[24], 1994	41%	+	+	-	-	+	+	+	-	+
Boulgarides et al.[45], 2003	41%	+	+	?	-	+	+	?	-	?
Viccaro et al.[51], 2011	41%	+	+	-	-	?	+	+	-	?
Lindemann et al.[44], 2008	44%	-	+	-	-	-	+	+	-	-
Singh et al.[37], 2019	44%	-	+	?	-	-	+	+	-	-
Nandy et al.[46], 2004	45%	+	+	-	-	?	+	?	-	?
Okochi et al.[43], 2006	45%	+	+	-	-	?	+	+	-	?
Rodriguez-Molinero et al.[33], 2017	46%	-	+	-	-	+	+	+	-	+
Gadkaree et al.[29], 2015	48%	+	+	-	-	+	+	+	-	+
de Vries et al.[26], 2013	50%	+	+	-	-	+	+	+	-	+
Hnizdo et al.[53], 2013	52%	+	+	?	-	-	+	+	-	-
Lohman et al.[34], 2017	52%	+	+	-	-	+	+	-	-	-
Gillain et al.[38], 2019	54%	-	+	-	-	-	-	+	-	-
Delbaere et al.[47], 2010	55%	+	+	-	-	+	+	-	-	-
Cawthon et al.[30], 2015	56%	+	+	-	-	-	+	+	-	-
Kim et al.[35], 2017	56%	-	+	-	-	-	+	?	-	-
Tromp et al.[28], 2001	57%	+	+	-	-	+	+	+	-	+
Stel et al.[41], 2003	57%	+	+	-	-	+	+	+	-	+
Lamb et al.[36], 2008	61%	+	+	-	-	-	+	?	-	-
Yamashita et al.[49], 2012	61%	+	+	-	-	+	+	-	-	-
Stalenhoef et al.[40], 2002	62%	?	+	-	-	-	+	+	-	-
Palumbo et al.[32], 2018	68%	+	+	-	-	?	+	?	-	?
Bongue et al.[39], 2011	70%	-	+	?	-	-	+	+	-	-
Palumbo et al.[31], 2016	73%	+	+	-	-	+	+	?	-	?
Pluijm et al.[42], 2006	76%	+	+	-	-	+	+	+	-	+

Note: + indicates low risk of bias /low concern regarding applicability; − indicates high risk of bias/high concern regarding applicability; ? indicates unclear risk of bias/unclear concern regarding applicability; ROB indicates risk of bias; TRIPOD indicates the percentage proportion of reported items recommended in reporting guidelines.

Participants

Studies were conducted in Australia, Belgium, Canada, France, Germany, Israel, Italy, Japan, the Netherlands, Malaysia, Spain, the United Kingdom, and the USA. Sample sizes ranged from 65 to 23,417 participants with median ages from 67.5-83 years. Studies used primarily a probability sampling method (n = 16), followed by convenience sampling (n = 8), and consecutive sampling (n = 2). Four studies did not report their sampling methods.

Index/Model

The median (IQR) number of predictors in the final models were five (3-9) and ranged from two to 96 predictors. Figure 2 shows the number of studies including a specific predictor. The most frequently applied predictors were prior falls, age, sex, measures of gait, balance, and strength, along with vision and disability. Predictors were measured in homes (n = 19), research centres (n = 19), or both (n = 12). Locations for measuring predictors were not reported for 22 models.

Outcomes and timing

The percentage of fallers ranged from 5.9-59%, and the number of recurrent fallers (≥ 2 falls) ranged from 6.3-54.1%. Models primarily predicted any falls (n = 34), i.e. single and recurrent falls, and recurrent falls only (n = 34). Two models predicted first-time falls[34, 48] and two predicted time to a fall.[37, 49] Participants were followed for a median (IQR) time of 12 (9.75-12) months. Individual study data items extracted are available in the data supplement (Appendix 10).

Model performance

Discriminatory measures were reported for 40 (55.6%) models. AUCs were 0.49-0.87 and 0.62-0.69 for developed (n = 37) and validated (n = 3) models respectively. Corresponding confidence intervals were reported for 27 (37.5%) models. Calibration measures were available for seven (9.7%) models. For validated models (n = 1), calibration was imperfect due to over- and underestimated predicted risks of falling for high- and low-risk participants, respectively.[42] Regarding developed models (n = 6), calibration was found acceptable, but studies did not assess model calibration in new participants.[44, 47, 50, 52]

Risk of bias, reporting transparency, and applicability within studies

Risk of bias

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Table 1 summarises ratings on risk of bias, applicability, and reporting transparency for the individual studies. All studies had a high risk of bias mainly due to methods of analysis and outcome assessment along with restrictive eligibility criteria. Regarding analysis methods, missing data were excluded in 13 out of 30 studies, and no internal validation methods were applied. As to the outcome, only four studies recorded falls daily with monthly notifications.[28, 30, 35, 38] Also, the majority of studies did not report the outcome definition used or whether outcomes assessors were blinded. Eligibility criteria were found restrictive for the majority of studies due to the exclusion of individuals with falls-risk-increasing conditions. These selective criteria limit the usability of models for the target population of community-dwellers. Overall, risk of bias and applicability assessments were complicated by studies only reporting, on average, 50% of all items recommended in reporting guidelines. Furthermore, this was complicated by a low response rate with four out of 30 study authors responding when contacted for clarification on study characteristics and data extraction items. Finally, outcome reporting bias assessments were not possible due to studies not referring to a preregistered protocol for their prognostic modelling study.

Applicability

Seven (23%) studies, with a total of 21 models, had low applicability concerns for the review question. Regarding participants, 17 (56.7%) studies were rated as having high or unclear applicability concerns for the review question. This concern was primarily due to restrictive eligibility criteria impeding generalisation to the general population of community dwellers. Restrictions were made by excluding participants with specific diseases or conditions that could increase the risk of falling, such as disability or impaired mobility. Furthermore, studies rated as having unclear applicability concerns did not sufficiently report whether the participants were community-dwellers or whether the setting was the general population rather than, e.g. primary or secondary care. Regarding predictors, 28 (93.3%) studies had no concerns. The remaining two studies used specific laboratory measures which may be challenging to apply in a general population setting.[28, 48] Regarding applicability concerns for the outcome, 18 (60%) studies had no concerns since they reported using the falls definition of the review or similar.

Discussion

The current systematic review found 72 prognostic models on falls risk with the area under the curve ranging from 0.49 to 0.87. All models had a high risk of bias mostly due to limitations in statistical methods and outcome assessments, combined with restrictive eligibility criteria. Thus,

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using the models in clinical practice would entail unreliable predictions. This review provides an extensive overview of prediction models for falls and information for future study methodology.

Strengths

The current review followed guidelines on prediction modelling reviews strictly for search strings[22], data extraction[21], risk of bias assessment[25], along with development[12, 21] and transparent reporting of the review.[11]

Limitations

Review level

We excluded potentially eligible studies without full-text (n = 14) or published in other languages (n = 7) during screening of titles and abstracts. These studies are listed in the data supplement. Furthermore, we excluded randomised controlled trials and retrospective cohort studies. Consequently, we were only able to include 0.25 percent (30/11,789) of studies screened even though other models, based on other study designs, may had been available. As prespecified in the study protocol, this exclusion criterion was chosen due to limitations with generalisability and missing data when developing or validating prediction models using these designs. Thus, this systematic review only provides an overview of models based on a specific study design, but we consider this exclusion of the other studies to be justified.

Study level

Limitations were found in the studies with a high risk of bias, poor quality of reporting, and finally, a low response rate when contacting authors for retrieval of missing data extraction items.

Risk of bias

We found a high risk of bias within all studies. Hence, the predictive performance may be low, and predictions unreliable when models are used in clinical practice. The bias ratings were primarily based on eligibility criteria, methods of outcome assessments, and statistical analysis. Building prediction models on selected subgroups of the target population can yield biased performance estimates when used in clinical practice on a different population.[15] Thus, study eligibility criteria should be aligned with the research questions, i.e. broad and with as few exclusion criteria as possible. In terms of outcome assessments, we found the definition of falls missing for one-third of studies along with varying falls recording methods. These findings are similar to results of a previous review on methodology in falls prevention trials, where only half of the studies provided a falls definition and recording methods varied highly.[18] The problem with not defining a fall is that the notion of falls is taken for granted. As seen in our review, the prevalence of falls differed

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markedly between studies, which could be due to different understandings of the fall's definition. Consequently, falls become harder to predict[15] while at the same time, comparing and combining studies in systematic reviews with meta-analyses becomes complicated. To address these issues, a common outcome data set on falls trials is available along with a falls definition and recommendations for falls recording methods.[17] Finally, statistical analysis methods raised concerns for risk of bias. Primarily, this was due to the handling of missing data with most of the studies applying a complete-case analysis method. Significant limitations can arise from the exclusion of participants due to missing data, e.g. on a single predictor among many, since otherwise useful predictors on each participant are lost. Consequently, this can lead to low sample sizes and biased model performances. In such cases, imputation methods have proved useful when dealing with missing data.[15] Furthermore, for the majority of developed models, no internal validation procedures were applied. This shortcoming typically causes models' predictive performance to be optimistic.[14] Finally, the critical appraisal was compromised due to incomplete reporting. We believe that future studies and systematic reviews would benefit from adhering to reporting guidelines for prediction modelling studies.[20]

Implications for clinical practice

Only seven studies could address the review question appropriately, and all of these had a high risk of bias. Consequently, the evidence available to inform healthcare professionals is limited and, as mentioned, possibly biased. Thus, no model can currently be recommended for clinical practice.

Implications for research

We recognise that most studies (n = 23/30) were conducted before the publishing of prediction modelling guidelines.[20, 25] Thus, with the benefit of hindsight, studies would be expected to have different shortcomings within their methods and reporting. On the other hand, this also supports the reason for publishing guidelines in the first place. Despite this, the included studies provide valuable information on future candidate predictors. Thus, selecting predictors for prediction models on non-statistical grounds, i.e. based on literature and clinical knowledge, is commonly used to avoid predictor selection bias.[57] Therefore, future development studies may include the most frequently applied predictors found in this review. Lastly, it is essential to test the generalisability of developed models by performing validation studies to determine which models provide stable predictions across different populations.[58]

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Conclusions

There are several studies on falls prognostic models intended for a general population setting, but only a few are fully applicable to the heterogeneous population of community-dwelling older adults. Thus, the evidence available to address this is limited. From all included studies, we found an abundance of falls prognostic models available. However, the discriminatory performance of these varied and was only reported for half of the models. Each model had concerns regarding risk of bias mainly due to restrictive eligibility criteria along with methods of statistical analysis and outcome assessments. Consequently, this could give rise to unreliable predictions should the models be used in clinical practice. Future prognostic prediction models should comply with TRIPOD.

Acknowledgements

The authors wish to thank health science librarian Pernille Skou Gaardsted for assisting with the development of the search strategy.

Contributors:

GVG is the guarantor. GVG drafted the manuscript for the protocol and performed preliminary searches and search strategy. GVG, JRY, MGJ, TM, KT, and SA developed selection criteria. JRI assisted GVG in screening titles, abstracts, and reference lists of papers included after full text reading along with data extraction, assessing risk of bias, presence of meta-bias along with adherence to reporting guidelines. KT assisted GVG in full-text reading, and MGJ was arbitrator if agreement could not be reached between reviewers. GVG drafted the manuscript for the paper, while MGJ, JRY, SA, TM, JRI, and KT assisted in the interpretation of results, read, provided feedback, and approved the final manuscript of the paper. The corresponding author attests that all listed authors meet authorship criteria and that no others meeting the criteria have been omitted.

Funding:

This research received no specific grant from any funding agency in the public, commercial or not-for-profit sectors.

Transparency statement:

The lead author affirms that the manuscript is an honest, accurate, and transparent account of the study being reported; that no important aspects of the study have been omitted; and that any discrepancies from the study as originally planned have been explained.

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Competing interests:

All authors have completed the ICMJE uniform disclosure form at www.icmje.org/coi_disclosure.pdf and declare: no support from any organisation for the submitted work; no financial relationships with any organisations that might have an interest in the submitted work in the previous three years; no other relationships or activities that could appear to have influenced the submitted work.

Ethical approval:

Not required.

Dissemination to participants and related patient and public communities:

The study did not involve participants for whom the study results can be disseminated. The authors intend to distribute the results of the study by engaging with local healthcare providers for the general population setting during 2020 and 2021, along with social media to reach a broader audience.

Data availability statement:

The study protocol is available online at https://www.crd.york.ac.uk/prospero/display_record.php?RecordID=124021. All information extracted for the included studies in the review is available as a data supplement.

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Figure legends:

Figure 1 PRISMA (preferred reporting items for systematic reviews and meta-analyses) diagram of the study selection process.

Figure 2 Number of studies using a specific predictor

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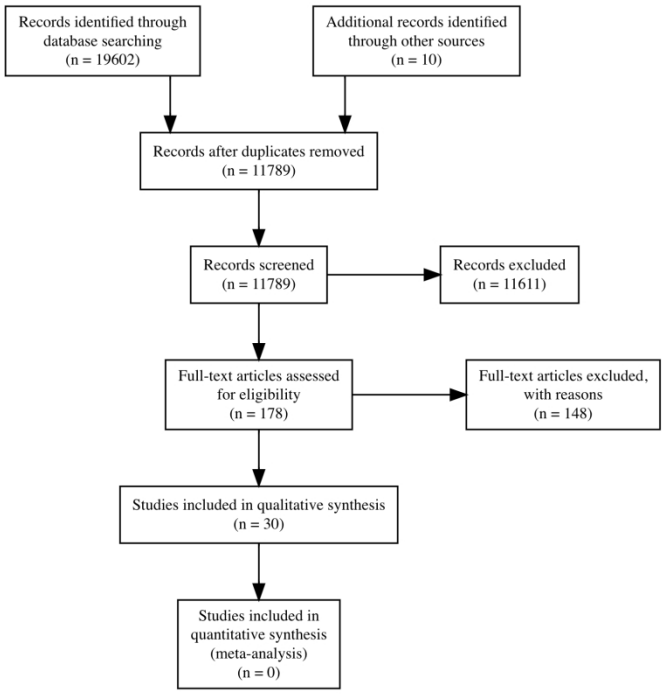

Figure 1 PRISMA (preferred reporting items for systematic reviews and meta-analyses) diagram of the study selection process.

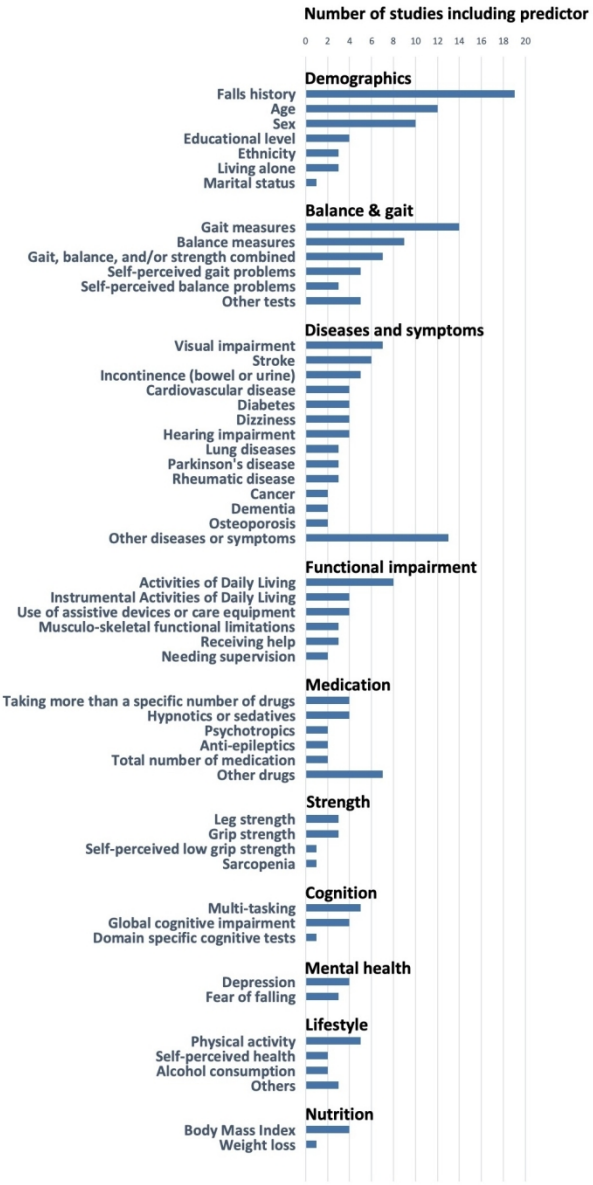

Figure 2 Number of studies using a specific predictor

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Appendix 1: protocol for systematic review

Administrative information

Title:

Systematic review of prognostic models for predicting falls in community-dwelling older adults

Registration:

This protocol was registered with the International Prospective Register of Systematic Reviews (PROSPERO) on (1st February 2019).

Registration number: CRD42019124021

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Contributions:
GS is the guarantor. GS drafted the manuscript for the protocol and performed preliminary searches and search strategy. GS, JR, MGJ and SA developed selection criteria. JRI will assist GS in screening titles, abstracts and reference lists of papers included after full-text reading along with data extraction, assessing risk of bias, presence of meta-bias along with adherence to reporting guidelines. KT will assist GS in full-text reading. MGJ will be arbitrator if agreement cannot be reached between reviewers. GS will draft the manuscript for the paper. MGJ, JRY, SA, TM, JRI and KT will assist in interpretation of results, read, provide feedback and approve the final manuscript of the paper.

Amendments:
In the event of protocol amendments, this section will describe the date, changes and rationale of each amendment. Changes will be incorporated into the protocol sections. All authors will be responsible for approving the amendments. Also, GS will be responsible for documentation and implementation of these.

Current version of the protocol: 3.

- 7th of August 2019
    - **Change #1:**
        - Setting:

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- We further specified which setting the review is, and is not, intended for. We changed the wording "community setting" to "general population setting". Also, we specified that prediction models intended for a primary care would also be excluded.

  - Rationale:
    - Change in wording: To apply the same terminology of the CHARMS checklist.
    - Primary care exclusion: To increase transparency and homogeneity in settings.

- **Change #2:**
  - Risk of bias:
    - As a supplement to the risk of bias assessments, the newly published TRIPOD adherence tool will be used.
  - Rationale
    - To assess adherence to reporting guidelines for prediction modelling studies.

- 17th of June 2019
  - **Change #1**
    - Study design:
      - We further specified which study designs would not be included. Thus, randomised controlled trials and retrospective cohort studies will not be included.
    - Rationale:
      - To increase transparency.
  - **Change #2:**
    - Participants: Age
      - We further specified the inclusion criterium regarding age. Thus, studies with total age ranges extending below 60 years will be excluded. Exclusion will also be made if mean age subtracted by two standard deviations extends below 60 years of age unless inclusion criterium in studies specifically states a lower age limit of 60 years or above.

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- Rationale:
  - To increase transparency.

- **Change #3:**
  - Participants: Community-dwelling
    - We further specified which studies would be included. Thus, we will include studies excluding certain types of community-dwelling individuals, e.g. with known neurological, spinal or cognitive disorders.
  - Rationale:
    - These samples may also contribute with relevant information about the target population.
- **Change #4:**
  - Index (Model):
    - We further specified which studies to include based on the model presented. Thus, we will also include studies with:
      - Two or more prognostic factors combined into a scale giving an individual score used to assess the predictive performance on future falls.
      - Two or more prognostic factors included in a test instrument from which a prediction model would be generated.
  - Rationale:
    - To increase transparency.
- **Change #5:**
  - Outcome (and rationale):
    - We further specified which studies to include based on the outcome. Thus, studies without an outcome definition will also be included since this will not rule out the outcome definition of this review. We will exclude studies using falls definitions excluding certain types of falls presumed to be due to a specific cause e.g. acute medical events or external forces. This post fall classification method may introduce assessor-bias.

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Support:
The Department of Geriatric Medicine, Aalborg University Hospital, Aalborg, Denmark and the Department of Clinical Medicine, Aalborg University, Aalborg, Denmark will fund and sponsor this research.

Introduction:

Rationale of the review

Falling over in community-dwelling older adults is a frequent problem with an annual prevalence of 30 % in 65+ year olds and 50 % in 80+ year olds.[1] Total number of falls are expected to increase significantly in the future due to the ageing population.[2,3] For instance, in 2017, the global population of 65+ year olds was estimated to be 962 million and is estimated to increase towards 1.4 and 2.1 billion in 2030 and 2050 respectively.[2] This frequent and escalating problem of fall accidents is a major concern globally due to their associations with elevated morbidity, mortality, poorer physical functioning and early admission to long-term care facilities which leads to elevated financial costs to society[4–6].

Fall prevention is therefore highly relevant to society, next of kin and to the individual. Unfortunately, more than 400 risk factors for falling have been identified thereby making it a complex area/problem.[7] In addition, the risk factors spread across different domains including socio-demographics, the environment, medical conditions and medications, physical performance, psychology and cognition[8]. In consequence, secondary multifactorial fall risk prevention has been recommended[3,9]. On the other hand, if individuals at high risk of falling could be identified before their first fall, primary preventive interventions could be instituted, which would be even more beneficial. Therefore, individual assessments of fall risk using multifactorial prognostic prediction models are imperative. Few reviews on the ability of prognostic prediction model studies to discriminate fallers from non-fallers in community-dwelling older adults exist[10–12]. However, in these reviews, methodologies were varied with no review protocols being reported[10–12], outcome definitions not following the current consensus definition[10–13], and search strategies being restricted[10,12] or based on search filters for diagnostic studies[11,12]. Also, risk-of-bias assessments were done using tools designed for diagnostic studies[11,12] and reporting of data extraction items and -methods were inconsistent[10,12].

Nonetheless, in recent years, prognostic research methods have evolved. Thus, new guidelines have been published to encourage researchers to transparently report prediction modelling studies[14], systematic reviews[15] and their respective protocols[16]. Also, within the field of prediction modelling reviews, literature search strategies[17] along with guidance papers on data extraction items[18] and risk

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of bias tools[19] have been developed. However, no reviews on fall prediction studies have applied the abovementioned guidelines as yet.

The scope of this review is to provide a systematic update on current model performance, and other characteristics, on developmental and validation studies within the field of fall accidents on multifactorial prognostic prediction models in community-dwelling older adults.

Objective:

The aim of this systematic review is to describe model performance along with other model characteristics (predictors along with methods of model development, -evaluation and -presentation) of existing multifactorial prognostic prediction models on falls in 60+ year old non-institutionalised, community-dwelling older adults.

Methods:

This protocol follows the guidelines of the Preferred Reporting Items for Systematic reviews and Meta-analyses Protocols (PRISMA-P)[16]. The protocol is registered in the PROSPERO database.

Eligibility Criteria

The following criteria outlined below will be used to select studies for the review.

Study designs:

We will only include prospective cohort studies since this is the preferred design for prognostic prediction modelling studies[14]. We will include both developmental and validation studies with and without model updating. Randomised controlled trials will not be included since these tend to have narrow predictor distributions resulting in poor discriminatory performance. This may also be influenced by treatment effects in the design[18]. Furthermore, generalisation to the target population may be compromised due to strict eligibility criteria[20]. Retrospective cohort studies will be excluded since the predictive performance may be substantially limited by missing data, and only predictors available in the data set collected can be applied[18].

Participants:

Only studies with all participants aged 60 years or older will be included. This cut-off was chosen in order to encompass studies using different age cut-offs for being an older adult. Thus, studies with total age ranges extending below 60 years will be excluded. Also, mean age subtracted by 2 standard deviations must not extend below 60 years unless inclusion criteria specifically stated a lower age limit of 60 years or above.

Participants should be community-dwelling and not institutionalised, i.e. living independently and not in nursing homes or short term-care where the risk of falling is substantially different from

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the general population[21]. Studies restricted to participants with pre-specified diseases, conditions or symptoms such as Parkinsonism or stroke will be excluded so that generalisation to the overall community population is not compromised. We will include studies excluding certain types of community-dwelling individuals, e.g. with known neurological, spinal or cognitive disorders since these samples may contribute with relevant information about the target population of the review.

Index (Model):

Multifactorial prognostic prediction models, i.e. including 2 prognostic factors or more due the multifactorial aetiology of falls[3,22]. Thus, explanatory studies investigating the association between a predictor and prospective falls were excluded. To broaden the search, we will include the following studies with:

- Two or more prognostic factors measuring on the same domain will be included.
- Two or more prognostic factors combined into a scale giving an individual score used to assess the predictive performance on future falls.
- Two or more prognostic factors included in a test instrument from which a prediction model would be generated.

Comparator:

None.

Outcome:

Primary outcome in the included prospective cohort studies will be falls defined by "an unexpected event in which the participants come to rest on the ground, floor, or lower level".[13] Both single and recurrent falls, i.e. >1 fall, will be included. Studies without an outcome definition were also included since this would not rule out the abovementioned definition. We excluded studies using falls definitions excluding certain types of falls presumed to be due to a specific cause e.g. acute medical events or external forces. This post fall classification method may introduce bias in the outcome assessment due to the subjective judgements involved[23].

Timing:

No restrictions on follow-up on falls will be made.

Setting:

The models should be used to screen for risk of prospective falls in a general population setting, and we will exclude models intended for primary care, hospitals and nursing homes.

Language:

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Only studies reported in an English, Danish, Norwegian or Swedish language will be included. This was chosen due to resource limitations. However, a list of possibly relevant studies in other languages found during the literature search will be included in an appendix.

Publication year:

No restrictions on publication year will be made.

Information sources

Studies will be collected from the following databases: Pubmed.gov (PubMed interface, inception date to date of search), EMBASE (Embase.com, inception date to date of search), CINAHL (EBSCOhost interface, inception date to date of search), The Cochrane Library (Wiley interface, inception date to date of search), PsycINFO (APA PsycNET interface, inception date to date of search) and Web of Science (Web of Science Core Collection, inception date to date of search). Both controlled terms (i.e. MeSH or EMTREE terms) and simple phrase terms will be used to search the databases when appropriate. Also, hand searches from the reference lists of the included studies will be performed. Conference abstracts found during the literature search will only be used for obtaining their respective full-text articles. If not found elsewhere, we will try to contact the respective authors for this. If the full-text articles are not obtainable, the study will be excluded. However, a list of these possibly relevant studies found during the literature search will be included in an appendix. Primary literature within prior systematic reviews on fall prediction models found during the literature search will be screened. Finally, two experts in the field of falls research will be consulted to enquire for knowledge on additional studies fulfilling the eligibility criteria of this systematic review.

Grey literature

PROSPERO will be searched for completed reviews with this focus. Also, Clinicaltrials.gov, WHO International Clinical Trials Registry Platform, Open Grey, GIN, NICE, CRD/HTA, SIGN will be searched for relevant studies using key-terms from the main search (falling AND elderly OR Older adults). If not found elsewhere, we will try to contact authors of these relevant studies for retrieving of the full-text.

Search strategy

The search strategy follows current Cochrane recommendations for systematic reviews on prediction models[17,24]. Also, to accommodate the search strategy to our eligibility criteria, the search string was further developed by GS in collaboration with a Health Sciences Librarian at the Medical Library of Aalborg University Hospital, Denmark. The search strategies of the selected databases are included in Appendix 1. The final search strategy will be approved by a second reviewer (KT).

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Study records:

Data management:

Duplicates will be removed using EndNote (EndNote X9, Clarivate Analytics, Philadelphia, USA). The results of the literature search will be uploaded to Covidence (Covidence systematic review software, Veritas Health Innovation, Melbourne, Australia. Available at www.covidence.org) to ease the collaboration between reviewers on titles and abstracts screening along with full-text reading. Risk of bias assessments and extraction of data will be performed using a standardised form in REDCap using a double data-entry module[25]. If several articles report results from the same trial, the "primary publication" will be prioritized; i.e. typically defined as the first full-text publication reporting on the primary outcome.

Selection process:

Screening titles and abstracts:

Two reviewers (GS and JRI) will independently screen titles and abstracts from the inclusion criteria. The screening process of titles and abstracts will undergo pilot testing. Reviewers (GS and JRI) will meet and discuss the inclusion of the first 50 articles found by the search strategy during screening of titles and abstracts. If agreement cannot be reached, a third author (MGJ) will be consulted for arbitration. If needed afterwards, refinement of inclusion criteria will be performed.

Full-text reading

Full-text reports will be obtained on eligible studies, and studies where uncertainty exists regarding eligibility based on titles and abstracts. GS and KT will independently screen the full-text reports for a final decision on eligibility. Disagreement among the reviewers will be discussed. If agreement cannot be reached, a third author (MGJ) will be consulted for arbitration. Reasons for excluding studies after full-text reading will be provided.

Data collection process:

Two reviewers (GS and JRI) will independently extract data from a pre-specified form (see Data items) in REDCap from each study found eligible for inclusion after full-text reading. If data reporting is incomplete, we will try to contact authors of the relevant studies to obtain data. A maximum of two attempts will be done to contact the authors by e-mail. If e-mails are not responded within 1 month from the first e-mail sent, the data field will be labelled as having no information. If the authors do not gain access to data, these will be extracted from figures and graphs if possible. If companion studies (multiple reports of the same study) with the same outcome of falls is found, data will be

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extracted from the primary publication of the study. Disagreement among the reviewers will be discussed. If agreement cannot be reached, a third author (MGJ) will be consulted for arbitration. The total number of times arbitration by a third author was required will be given.

Data items:

Data extraction will comply with Critical Appraisal and Data Extraction for Systematic Reviews of Prediction Modelling Studies (CHARMS) guidelines.[18] The following data will be extracted if possible:

- General study information:
  - Authors.
  - Year of publication.
  - Study design.
  - Type of prediction modelling study:
    - Developmental without external validation.
    - Developmental with external validation.
    - External model validation without model updating.
    - External model validation with model updating.
    - Others
  - Country of origin.
  - Setting where candidate predictors were measured.
  - Number of study centres.
  - Inclusion criteria.
  - Exclusion criteria.
  - Sample size.
  - Methods for participant recruitment/sampling:
    - Consecutive sampling.
    - Convenience sampling.
    - Probability sampling.
    - Others
  - Dates of participant recruitment.
  - Duration of follow-up.
  - How many participants completed follow-up percentage-wise?
- Participants:

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- Gender.
- Age.
- Fall history.

- Outcome:
  - Outcome definition.
    - Was it pre-specified?
  - Type of fall recording/method of outcome measurement.
  - Was the same outcome definition and recording method used in all participants (Yes/No/Not Available)?
  - Was the outcome assessor blinded towards predictors (Yes/No/Not Available)?
  - Number of falls.
  - Number of fallers.
  - Number of non-fallers.
  - Number of frequent fallers.
  - Fall-rate per person per year.
  - Summary of follow-up period.
- Candidate predictors:
  - Number of candidate predictors studied.
  - Definitions of candidate predictors.
  - Methods for measuring candidate predictors.
  - Number of outcomes (falls) in relation to number of candidate predictors (events per variable (EPV)).
- Missing data:
  - Number of participants with missing data (both predictors and outcomes) in total.
  - Did participants with missing data differ from those without missing data (Yes/No/Not Available)?
  - Number of participants with missing data in total for each predictor
  - Method of handling missing data.
    - Single imputation
    - Multiple imputation
    - Participants with missing data were excluded from the analysis (complete case analysis)

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- Others (comment).

- Were participants with missing data included in the model development, validation or updating (Yes/No/Not Available)?

- Model development (not relevant if the prediction modelling study does not include model development):
  - Type of model:
    - Linear regression
    - Logistic regression.
    - Survival analysis.
    - Others (comment).
  - Were assumptions for the model checked (Yes/No/Not available)?
  - Were assumptions for the model satisfied (Yes/No/Not available)?
  - Predictor selection methods for inclusion into the multivariable analysis:
    - All predictors were predetermined to be included in the analysis
    - Predictors were selected for inclusion based on univariate associations with the outcome
    - Others
  - Did any statistical transformation of candidate predictors occur (i.e. dichotomising a continuous or categorical variable) prior to inclusion in the multivariate modelling process (Yes/No/Not Available)?
    - If YES, what transformation procedure was applied?
  - Predictor selection methods during the multivariable modelling:
    - Full model approach (all predictors were predefined for the final model and no predictors were omitted).
    - Forward selection (candidate predictors were selected based on pre-specified criteria).
      - If forward selection was applied, which criteria/significance level were used?
        - P-value
        - Akaike's Information Criteria
        - R^2
        - Others

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- Backward elimination (all candidate predictors started in the model and were removed or kept based on a pre-specified criterion)
    - If backward elimination was applied, which criteria/ significance level were used?
        - P-value
        - Akaike's Information Criteria
        - R^2
        - Others
- Were shrinkage techniques applied (Yes/No/Not available)?
    - If YES, which procedure was applied?

- Model performance:
    - Overall measures of model performance
        - R^2
        - Brier Score
    - Discrimination:
        - Area Under Curve/*c*-statistic
        - D-statistic
        - Others
    - Calibration:
        - Calibration plot
        - Calibration intercept and slope
        - Calibration table
        - Hosmer-Lemeshow test
        - Observed:Expected Ratio
        - Others.
    - Classification:
        - Sensitivity
        - Specificity
        - Positive Predictive Value
        - Negative Predictive Value
        - Net reclassification index
        - Others

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- Was the cut-point:
  - Predefined/made a priori?
  - Derived from the dataset?

- Model evaluation:
  - **External validation:** Were the model performance measures based on separate external data (Y/N/NA)?
    - If YES, how was the dataset different from the developmental dataset:
      - Temporal/differed in time
      - Different geographical location
      - Different setting
      - Different investigator
      - Others
  - **Internal validation:** Were the model performance measures based on the developmental dataset (Y/N/NA)?
    - Which approach was chosen to evaluate model performance?
      - Split-sample validation
        - What was the percentage-wise allocation of participants?
        - Was the split random to model development and validation(Y/N/NA)?
      - Cross-validation
        - How many subsets were chosen?
      - Bootstrap validation
      - Jack-knife resampling
      - Others
      - None

- Model presentation:
  - What format did the study present their model in to permit calculations of individual risks?
    - Regression formula (comment)
    - Rounded scoring rules (comment)
    - Predefined risk groups (comment)

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- Were the risk-groups:
  - Predefined/made a priori?
  - Derived from the dataset?
- Nomogram
- Score chart
- Others
- None

- Sources of funding in the individual study.

Outcomes and prioritisation

Main outcomes:

The main outcome of this systematic review is to describe model performance. Secondary outcomes are to describe the following characteristics: Study setup, participants, final model predictors, outcomes together with model development, -evaluation and -presentation.

Risk of bias

The Prediction study Risk Of Bias Assessment Tool (PROBAST)[19] will be applied for risk of bias assessment. Two reviewers (GS and JRI both with no prior experience in risk of bias assessments) will independently assess for risk of bias. These will not be blinded to study titles or authors. If reporting is incomplete in order to make a complete assessment, we will try to contact authors of the concerned study. A maximum of two attempts will be done to contact the authors by e-mail. If e-mails are not responded within 1 month from the first e-mail sent, the data field will be labelled as "Unclear". Disagreement among the reviewers will be discussed. If agreement still is not reached, a third author (MGJ) will be consulted for arbitration. The total number of times arbitration by a third author was required will be given. Each domain rating will be reported instead of a summative score of all domains. Previously, one review on fall prediction models assessed reporting in included studies to be poor[12]. However, no standardised method of evaluating reporting in studies was reported. Thus, as a supplement to the risk of bias assessments, the newly published TRIPOD adherence tool[26] will be used to assess adherence to reporting guidelines for prediction modelling studies.

Data synthesis

Meta-analysis will not be considered due to this systematic review merely being descriptive. In the qualitative synthesis, information will be presented in text, figures, and tables of the included studies. Reporting of studies will be presented in tables by their publication year. Final model predictors will be presented in main categories in a figure.

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Meta-bias

Presence of outcome reporting bias will be investigated by comparing the studies with their respective protocol if available. The following aspects will be considered:

- Was publication of the protocol done before recruitment of patients?
- Was the intended outcome in the protocol the same in the published study?

Confidence in cumulative estimate

Assessment of strength of evidence will be made using the PROBAST tool.

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Appendix 2: PRISMA checklist

See tables on the next page. Fields pertaining to meta-analyses have been labelled as not available (NA) since meta-analyses was not performed.

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Section/topic	#	Checklist item	Reported on page #
TITLE			
Title	1	Identify the report as a systematic review, meta-analysis, or both.	#2
ABSTRACT			
Structured summary	2	Provide a structured summary including, as applicable: background; objectives; data sources; study eligibility criteria, participants, and interventions; study appraisal and synthesis methods; results; limitations; conclusions and implications of key findings; systematic review registration number.	#4
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of what is already known.	#6
Objectives	4	Provide an explicit statement of questions being addressed with reference to participants, interventions, comparisons, outcomes, and study design (PICOS).	#6
METHODS			
Protocol and registration	5	Indicate if a review protocol exists, if and where it can be accessed (e.g., Web address), and, if available, provide registration information including registration number.	#6
Eligibility criteria	6	Specify study characteristics (e.g., PICOS, length of follow-up) and report characteristics (e.g., years considered, language, publication status) used as criteria for eligibility, giving rationale.	#6-8
Information sources	7	Describe all information sources (e.g., databases with dates of coverage, contact with study authors to identify additional studies) in the search and date last searched.	#8
Search	8	Present full electronic search strategy for at least one database, including any limits used, such that it could be repeated.	#8 + Appendix 3
Study selection	9	State the process for selecting studies (i.e., screening, eligibility, included in systematic review, and, if applicable, included in the meta-analysis).	#8-9 + Appendix 4
Data collection process	10	Describe method of data extraction from reports (e.g., piloted forms, independently, in duplicate) and any processes for obtaining and confirming data from investigators.	#9
Data items	11	List and define all variables for which data were sought (e.g., PICOS, funding sources) and any assumptions and simplifications made.	#9 + Appendix 5
Risk of bias in individual studies	12	Describe methods used for assessing risk of bias of individual studies (including specification of whether this was done at the study or outcome level), and how this information is to be used in any data synthesis.	#9-10
Summary measures	13	State the principal summary measures (e.g., risk ratio, difference in means).	#10

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Synthesis of results	14	Describe the methods of handling data and combining results of studies, if done, including measures of consistency (e.g., I^2) for each meta-analysis.	**NA**

Section/topic	#	Checklist item	Reported on page #
Risk of bias across studies	15	Specify any assessment of risk of bias that may affect the cumulative evidence (e.g., publication bias, selective reporting within studies).	#10
Additional analyses	16	Describe methods of additional analyses (e.g., sensitivity or subgroup analyses, meta-regression), if done, indicating which were pre-specified.	**NA**
RESULTS			
Study selection	17	Give numbers of studies screened, assessed for eligibility, and included in the review, with reasons for exclusions at each stage, ideally with a flow diagram.	#10 + Fig 1 + Appendix 6-8
Study characteristics	18	For each study, present characteristics for which data were extracted (e.g., study size, PICOS, follow-up period) and provide the citations.	#10-12 + Fig 2 + Appendix 9-10
Risk of bias within studies	19	Present data on risk of bias of each study and, if available, any outcome level assessment (see item 12).	#12 + Table 1
Results of individual studies	20	For all outcomes considered (benefits or harms), present, for each study: (a) simple summary data for each intervention group (b) effect estimates and confidence intervals, ideally with a forest plot.	#12
Synthesis of results	21	Present results of each meta-analysis done, including confidence intervals and measures of consistency.	**NA**
Risk of bias across studies	22	Present results of any assessment of risk of bias across studies (see Item 15).	#12
Additional analysis	23	Give results of additional analyses, if done (e.g., sensitivity or subgroup analyses, meta-regression [see Item 16]).	**NA**
DISCUSSION			
Summary of evidence	24	Summarize the main findings including the strength of evidence for each main outcome; consider their relevance to key groups (e.g., healthcare providers, users, and policy makers).	#12-13
Limitations	25	Discuss limitations at study and outcome level (e.g., risk of bias), and at review-level (e.g., incomplete retrieval of identified research, reporting bias).	#13
Conclusions	26	Provide a general interpretation of the results in the context of other evidence, and implications for future research.	#13-15
FUNDING			

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Funding	27	Describe sources of funding for the systematic review and other support (e.g., supply of data); role of funders for the systematic review.	#10

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Appendix 3: search strategy

Pubmed.gov

((((((home-dwell*[tw] OR "Independent Living"[Mesh] OR Independent*[tw] OR community-dwell*[tw] OR home-based*[tw] OR community-living*[tw]))

AND ("Aged"[Mesh] OR aged[tw] OR senior*[tw] OR elder*[tw] OR old[tw] OR older[tw]))) AND ("Accidental Falls"[Mesh] OR fall*[Text Word])))

AND ((((Validat*[tw] OR Predict*[ti] OR Rule*[tw]) OR (Predict*[tw] AND (Outcome*[tw] OR Risk*[tw] OR Model*[tw])) OR ((History[tw] OR Variable*[tw] OR Criteria [tw]OR Scor*[tw] OR Characteristic*[tw] OR Finding*[tw] OR Factor*[tw]) AND (Predict*[tw] OR Model*[tw] OR Decision*[tw] OR Identif*[tw] OR Prognos*[tw])) OR (Decision*[tw] AND (Model*[tw] OR Clinical*[tw] OR "Logistic Models"[MESH])) OR (Prognostic[tw] AND (History[tw] OR Variable*[tw] OR Criteria[tw] OR Scor*[tw] OR Characteristic*[tw] OR Finding*[tw] OR Factor*[tw] OR Model*[tw])))) OR ((((((((((("ROC Curve"[Mesh]) OR stratificat*[tw]) OR discriminat*[tw]) OR c statistic*[tw]) OR Area under the curve*[tw]) OR AUC[tw]) OR Calibrat*[tw]) OR Indices[tw]) OR Algorithm*[tw]) OR Multivariable*[tw])) OR ((Predict*[tw] OR Predictive value of tests[mh] OR Scor*[tw] OR Observ*[tw] OR Observer variation[mh]))))

Embase:

('falling'/exp OR fall*:ti,ab,kw)

AND ('aged'/exp OR aged:ti,ab,kw OR senior*:ti,ab,kw OR elder*:ti,ab,kw OR old:ti,ab,kw OR older:ti,ab,kw)

AND (validat* OR rule* OR (predict* AND (outcome* OR risk* OR model*)) OR ((history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor*)

AND (predict* OR model* OR decision* OR identif* OR prognos*)) OR (decision* AND (model* OR clinical* OR 'statistical model'/exp)) OR (prognostic AND (history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor* OR model*)) OR 'receiver operating characteristic'/exp OR stratificat* OR discriminat* OR 'c statistic*' OR 'area under the curve*' OR auc OR calibrat* OR indices OR algorithm* OR multivariable* OR predict* OR 'predictive value'/exp OR scor* OR observ* OR 'observer variation'/exp) AND ('community living'/exp OR 'at home':ti,ab,kw OR (((community OR home OR independent*) NEAR/3 (dwell* OR based OR live OR living)):ti,ab,kw) OR 'home accident'/exp OR 'community dwelling person'/exp)

CINAHL:

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((MH "Community Living") OR (MH "Assisted Living") OR ((community OR home OR independent*) N3 (dwell* OR based OR live OR living)) OR (MH "Accidents, Home") OR at home) AND ((MH "Accidental Falls") OR fall*)

AND ((MH "Aged") OR (MH "Aged, 80 and Over") OR (MH "Frail Elderly") OR aged OR senior* OR elder* OR old OR older)

AND (validat* OR rule* OR (predict* AND (outcome* OR risk* OR model*)) OR ((history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor*) AND (predict* OR model* OR decision* OR identif* OR prognos*)) OR (decision* AND (model* OR clinical* OR (MH "Models, Statistical"))) OR (prognostic AND (history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor* OR model*)) OR (MH "ROC Curve") OR stratificat* OR discriminat* OR 'c statistic*' OR 'area under the curve*' OR auc OR calibrat* OR indices OR algorithm* OR multivariable* OR predict* OR (MH "Predictive Value of Tests") OR scor* OR observ*)

Psycinfo
http://psycnet.apa.org.auh.aub.aau.dk/permalink/19512998-9a97-e90a-722c-bd157326fa55

((Any Field: (validat*) OR Any Field: (rule*) OR (Any Field: (predict*) AND (Any Field: (outcome*) OR Any Field: (risk*) OR Any Field: (model*))) OR ((Any Field: (history) OR Any Field: (variable*) OR Any Field: (criteria) OR Any Field: (scor*) OR Any Field: (characteristic*) OR Any Field: (finding*) OR Any Field: (factor*)) AND (Any Field: (predict*) OR Any Field: (model*) OR Any Field: (decision*) OR Any Field: (identif*) OR Any Field: (prognos*))) OR (Any Field: (decision*) AND (Any Field: (model*) OR Any Field: (clinical*))) OR (Any Field: (prognostic) AND (Any Field: (history) OR Any Field: (variable*) OR Any Field: (criteria) OR Any Field: (scor*) OR Any Field: (characteristic*) OR Any Field: (finding*) OR Any Field: (factor*) OR Any Field: (model*))) OR Any Field: (stratificat*) OR Any Field: (discriminat*) OR Any Field: ('c statistic*') OR Any Field: ('area under the curve*') OR Any Field: (ROC) OR Any Field: (auc) OR Any Field: (calibrat*) OR Any Field: (indices) OR Any Field: (algorithm*) OR Any Field: (multivariable*) OR Any Field: (predict*) OR Any Field: (scor*) OR Any Field: (observ*))))

AND ((((((Any Field: ('at home'))) OR ((((Any Field: (community))) OR ((Any Field: (home))) OR ((Any Field: (independent*)))) NEAR/3 (((Any Field: (dwell*))) OR ((Any Field: (based))) OR ((Any Field: (live))) OR ((Any Field: (living)))))) OR ((((IndexTermsFilt: ("Home Accidents"))))))

AND (((Any Field: (aged))) OR ((Any Field: (elder*))) OR ((Any Field: (old))) OR ((Any Field: (older))) OR ((Any Field: (senior*))))

AND (((IndexTermsFilt: ("Falls"))) OR ((Any Field: (fall*)))))

Cochrane Library:
ID Search

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#1 MeSH descriptor: [Accidental Falls] explode all trees
#2 fall*:ti,ab,kw
#3 #1 OR #2
#4 MeSH descriptor: [Aged] explode all trees
#5 aged:ti,ab,kw
#6 senior*:ti,ab,kw
#7 elder*:ti,ab,kw
#8 old:ti,ab,kw
#9 older:ti,ab,kw
#10 #4 OR #5 OR #6 OR #7 OR #8 OR #9
#11 MeSH descriptor: [Independent Living] this term only
#12 "at home":ti,ab,kw
#13 MeSH descriptor: [Accidents, Home] explode all trees
#14 #3 or #13
#15 (((community OR home OR independent*) NEAR/3 (dwell* OR based OR live OR living)):ti,ab,kw)
#16 #11 OR #12 OR #13 OR #15
#17 #14 AND #16 AND #10
#18 MeSH descriptor: [Logistic Models] explode all trees
#19 MeSH descriptor: [ROC Curve] explode all trees
#20 MeSH descriptor: [Predictive Value of Tests] explode all trees
#21 MeSH descriptor: [Observer Variation] explode all trees
#22 ((((Validat* OR Predict*:ti OR Rule*) OR (Predict* AND (Outcome* OR Risk* OR Model*)) OR ((History OR Variable* OR Criteria OR Scor* OR Characteristic* OR Finding* OR Factor*) AND (Predict* OR Model* OR Decision* OR Identif* OR Prognos*)) OR (Decision* AND (Model* OR Clinical* OR #18)) OR (Prognostic AND (History OR Variable* OR Criteria OR Scor* OR Characteristic* OR Finding* OR Factor* OR Model*)))) OR ((((((((((#19) OR stratificat*) OR discriminat*) OR c statistic*) OR Area under the curve*) OR AUC) OR Calibrat*) OR Indices) OR Algorithm*) OR Multivariable*)) OR ((Predict* OR #20 OR Scor* OR Observ* OR #21))))
#23 #17 AND #22

Web of Science:
#1 ts=fall*
#2 ts=(aged OR senior* OR elder* OR old OR older)
#3 ts=((validat* OR rule* OR (predict* AND (outcome* OR risk* OR model*)) OR ((history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR factor*) AND (predict* OR model* OR decision* OR identif* OR prognos*)) OR (decision* AND (model* OR clinical* OR "statistical model")) OR (prognostic AND (history OR variable* OR criteria OR scor* OR characteristic* OR finding* OR

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factor* OR model*)) OR ("receiver operating characteristic" OR stratificat* OR discriminat* OR "c statistic" OR "area under the curve*" OR auc OR calibrat* OR indices OR algorithm* OR multivariable* OR predict* OR "predictive value" OR scor* OR observ* OR "observer variation")))
#4 ts=("community living" OR "at home" OR ((community OR home OR independent*) NEAR/3 (dwell* OR based OR live OR living)) OR "home accident" OR "community dwelling person")
#4 AND #3 AND #2 AND #1

PROSPERO:
https://www.crd.york.ac.uk/prospero

Clinicaltrials.gov:
https://clinicaltrials.gov/ct2/results?cond=falling+and+elderly+or+older+adults&term=&cntry=&state=&city=&dist=

WHO International Clinical Trials Registry Platform:
http://apps.who.int/trialsearch/

Open Grey:
http://www.opengrey.eu/search/request?q=Falling+AND+Elderly+OR+Older+adults

GIN:
https://www.g-i-n.net/library/international-guidelines-library/international-guidelines-library/@@guideline_search_results?basic-searchable-text=falling+and+elderly+or+older+adults&type=basic&action=Search&advanced-authors=&diseases=&meshterm=&search=

NICE:
https://www.nice.org.uk/Search?q=Falling+AND+Elderly+OR+Older+adults

CRD/HTA:
https://www.crd.york.ac.uk/CRDWeb/

SIGN:
https://www.sign.ac.uk/our-guidelines.html

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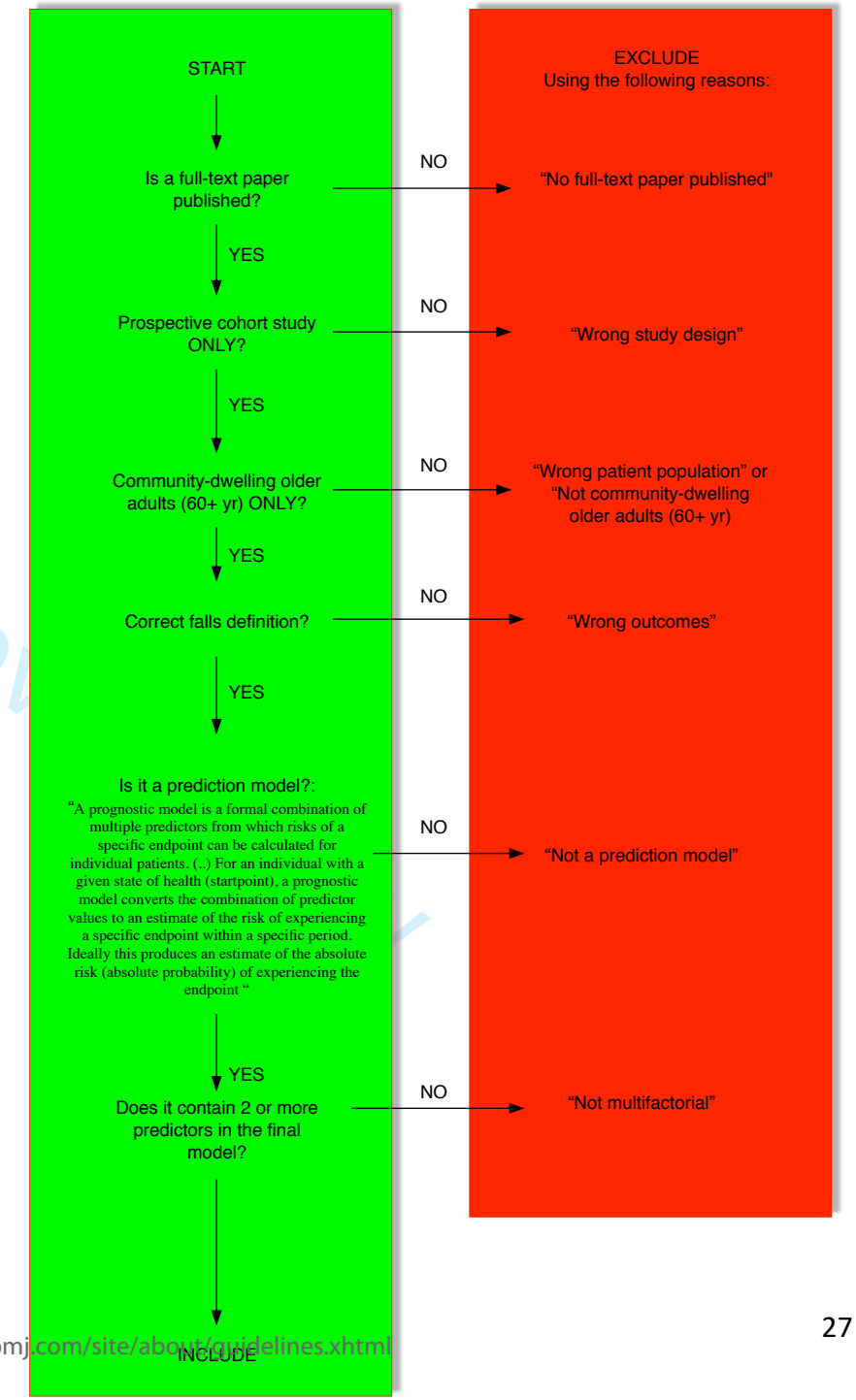
Appendix 4: prioritised list for exclusion reasons



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Appendix 5: data extraction items

- General study information:
    - Authors
    - Year of publication
    - Study design
    - Type of prediction modelling study:
        - Developmental without external validation
        - Developmental with external validation
        - External model validation without model updating
        - External model validation with model updating
        - Others
    - Country of origin
    - Setting where candidate predictors were measured
    - Number of study centres
    - Inclusion criteria
    - Exclusion criteria
    - Sample size
    - Methods for participant recruitment/sampling:
        - Consecutive sampling
        - Convenience sampling
        - Probability sampling
        - Others

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- Dates of participant recruitment
- Duration of follow-up
- How many participants completed follow-up percentage-wise?

- Participants:
  - Gender
  - Age
  - Fall history
- Outcome:
  - Outcome definition
    - Was it pre-specified?
  - Type of fall recording/method of outcome measurement
  - Was the same outcome definition and recording method used in all participants (Yes/No/Not Available)?
  - Was the outcome assessor blinded towards predictors (Yes/No/Not Available)?
  - Number of falls
  - Number of fallers
  - Number of non-fallers
  - Number of frequent fallers
  - Fall-rate per person per year
  - Summary of follow-up period
- Candidate predictors:
  - Number of candidate predictors studied
  - Definitions of candidate predictors
  - Methods for measuring candidate predictors

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- Number of outcomes (falls) in relation to number of candidate predictors (events per variable (EPV))

- Missing data:
  - Number of participants with missing data (both predictors and outcomes) in total
  - Did participants with missing data differ from those without missing data (Yes/No/Not Available)?
  - Number of participants with missing data in total for each predictor
  - Method of handling missing data
    - Single imputation
    - Multiple imputation
    - Participants with missing data were excluded from the analysis (complete case analysis)
    - Others (comment)
  - Were participants with missing data included in the model development, validation or updating (Yes/No/Not Available)?
- Model development (not relevant if the prediction modelling study does not include model development):
  - Type of model:
    - Linear regression
    - Logistic regression
    - Survival analysis
    - Others (comment)
  - Were assumptions for the model checked (Yes/No/Not available)?
  - Were assumptions for the model satisfied (Yes/No/Not available)?
  - Predictor selection methods for inclusion into the multivariable analysis:
    - All predictors were predetermined to be included in the analysis
    - Predictors were selected for inclusion based on univariate associations with the outcome
    - Others

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- Did any statistical transformation of candidate predictors occur (i.e. dichotomising a continuous or categorical variable) prior to inclusion in the multivariate modelling process (Yes/No/Not Available)?
  - If YES, what transformation procedure was applied?
- Predictor selection methods during the multivariable modelling:
  - Full model approach (all predictors were predefined for the final model and no predictors were omitted)
  - Forward selection (candidate predictors were selected based on pre-specified criteria)
    - If forward selection was applied, which criteria/significance level were used?
      - P-value
      - Akaike's Information Criteria
      - R^2
      - Others
  - Backward elimination (all candidate predictors started in the model and were removed or kept based on a pre-specified criterion)
    - If backward elimination was applied, which criteria/ significance level were used?
      - P-value
      - Akaike's Information Criteria
      - R^2
      - Others
- Were shrinkage techniques applied (Yes/No/Not available)?
  - If YES, which procedure was applied?

- Model performance:
  - Overall measures of model performance
    - R^2

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        - Brier Score
    - Discrimination:
        - Area Under Curve/*c*-statistic
        - D-statistic
        - Others
    - Calibration:
        - Calibration plot
        - Calibration intercept and slope
        - Calibration table
        - Hosmer-Lemeshow test
        - Observed:Expected Ratio
        - Others
    - Classification:
        - Sensitivity
        - Specificity
        - Positive Predictive Value
        - Negative Predictive Value
        - Net reclassification index
        - Others
        - Was the cut-point:
            - Predefined/made a priori?
            - Derived from the dataset?
- Model evaluation:

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- **External validation:** Were the model performance measures based on separate external data (Y/N/NA)?
  - If YES, how was the dataset different from the developmental dataset:
    - Temporal/differed in time
    - Different geographical location
    - Different setting
    - Different investigator
    - Others
- **Internal validation:** Were the model performance measures based on the developmental dataset (Y/N/NA)?
  - Which approach was chosen to evaluate model performance?
    - Split-sample validation
      - What was the percentage-wise allocation of participants?
      - Was the split random to model development and validation(Y/N/NA)?
    - Cross-validation
      - How many subsets were chosen?
    - Bootstrap validation
    - Jack-knife resampling
    - Others
    - None

- Model presentation:
  - What format did the study present their model in to permit calculations of individual risks?
    - Regression formula (comment)
    - Rounded scoring rules (comment)
    - Predefined risk groups (comment)

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- Were the risk-groups:
    - Predefined/made a priori?
    - Derived from the dataset?
- Nomogram
- Score chart
- Others
- None

- Sources of funding in the individual study

We used the following definitions of developmental studies and validation studies:

- Developmental study: “Model development studies aim to derive a prediction model by selecting predictors and combining them into a multivariable model”[1].
- Validation study: A fully specified existing prognostic model including both predictors and their coefficients [2].
    - Studies with prespecified predictors, but without any coefficients were classified as developmental studies.

1 Collins GS, Reitsma JB, Altman DG, *et al.* Transparent reporting of a multivariable prediction model for individual prognosis or diagnosis (TRIPOD): the TRIPOD Statement. *BMC Med* 2015;**13**:1. doi:10.1186/s12916-014-0241-z

2 Altman DG, Vergouwe Y, Royston P, *et al.* Prognosis and prognostic research: Validating a prognostic model. *BMJ* 2009;**338**:1432–5. doi:10.1136/bmj.b605

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Appendix 6: studies excluded during screening of titles and abstracts:

See separate PDF file: "Appendix 6".

Appendix 7: possibly relevant studies in other languages excluded during screening of titles and abstract:

Title	Author	Year	Language:	Reference:
Fall Prediction Model for Community-dwelling Elders based on Gender	Eun Suk, Yun	2012	Korean	J Korean Acad Nurs. 2012;42(6):810-818. doi:10.4040/jkan.2012.42.6.810
Fall risk and fracture. Aging and fall/fracture	Kozaki, K.	2013	Japanese	Clin Calcium. 2013;23(5):653-660
Fall risk assessment in regular exercising elderly women	Kikuchi, R.; Kozaki, K.; Kawashima, Y.; Iwata, A.; Hasegawa, H.; Igata, A.; Toba, K.	2008	Japanese	Nihon Ronen Igakkai Zasshi. 2008;45(5):526-531. doi:10.3143/geriatrics.45.526
Risk profiles and preventive measures of falls in elderly persons	Tromp, E	2002	Dutch	Tijdschr Gerontol Geriatr. 2002;33(1):21-25.
Fall-related factors in a cohort of elderly community residents	Rodriguez Perracini, M.; Ramos, L. R.	2002	Spanish	Rev Saude Publica. 2002;36(6):709-716. doi:10.1590/s0034-89102002000700008
Identifying the elderly at risk for falling and accompanying protocols	Galinsky, D.; Fried, V.; Biderman, A.; Cwikel, J.; Ben Moshe, Y.	2000	Hebrew	Harefuah. 2000;138(3):189-271.
Impact of fall risk and fear of falling on mobility of independently living senior citizens transitioning to frailty: Screening results concerning fallprevention in the community	Anders, J.; Dapp, U.; Laub, S.; Von Renteln-Kruse, W.	2007	German	Z Gerontol Geriatr. 2007;40(4):255-267. doi:10.1007/s00391-007-0473-z

Appendix 8: studies excluded during full-text screening.

See separate PDF file: "Appendix 8".

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Appendix 9: supplementary table 1 with characteristics of included studies

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Supplementary Table 1 Characteristics of included studies

First author, year, country, study type	Study characteristics (sample size, age distribution, % female, % of sample with prior falls)	Outcomes (n; %) and length of follow-up	Statistical model and validation technique	Final models and their presentation (model no. (outcome): predictors with/without weights)	Model performance
Maki et al. [1], 1994, Canada Development study study	n = 100 Age, mean (SD): 83 (6) years Female: 83% Prior falls: 46.8%	Any falls (59; 59%) Recurrent falls (23; 23%) Follow-up: 12 months	Logistic regression Cross-validation (n-1)	**Model** 1 (any falls): Spontaneous mediolateral sway (root-mean-square) with eyes blindfolded; induced sway in anterior-posterior direction with eyes open (mean COP location / length of base-of-support) **Model** 2 (recurrent falls): Induced sway in mediolateral direction with eyes blindfolded (mean coherence of the input-output model); induced sway in mediolateral direction with eyes blindfolded (Relative COP overshoot in the predicted transient response)	**Model** 1: AUC (SE): 0.76 (0.05) Calibration: no information SN: 78% (43/55) SP: 50% (18/36) **Model** 2: AUC (SE): 0.87 (0.05) Calibration: no information SN: 53% (9/17) SP: 89% (31/35)
Brauer et al. [2], 2000, Australia Developmental study	n = 100 Age, mean (SD): 71 (5) years Female: 100% Prior falls: 35%	Any falls (35; 35%) Follow-up: 6 months	Logistic regression Cross-validation (no information on subsets)	**Model** 1 (any falls): Gluteus medius onset time; Movement time in a high preparation step task; Step time in a high preparation step task; Maximum COP excursion when moving to the right LOS; COP maximum mediolateral velocity; Total distance moved in quiet stance **Model** 2 (any falls): Movement time in a high preparation step task; step time in a high preparation step task; total time in a high preparation step task; movement time in a neutral preparation step task; step time in a neutral preparation step task; total time in a neutral preparation step task **Model** 3 (any falls): Gluteus medius in a neutral preparation step task; tensor fascia latae in a neutral preparation step task; hip adductors in a neutral preparation step task; gastrocnemius onset times in a neutral preparation step task; gluteus medius in a high preparation step task; tensor fascia latae in a high preparation step task; hip adductors in a high	**Model** 1: AUC: no information Calibration: no information SN: 51% SP: 91% Total predictive ability: 77% **Model** 2: AUC: no information Calibration: no information SN: 34% SP: 89% Total predictive ability: 70% **Model** 3: AUC: no information Calibration: no information SN: 23% SP: 88% Total predictive ability: 65%

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				preparation step task; gastrocnemius onset times in a high preparation step task **Model** 4 (any falls): COP maximum mediolateral velocity with eyes open; COP maximum mediolateral velocity with eyes closed; COP position in quiet stance with eyes open; COP position in quiet stance with eyes closed; COP total distance moved in quiet stance with eyes open; COP total distance moved in quiet stance with eyes closed **Model** 5 (any falls): COP maximum excursion when moving to the right LOS; COP maximum excursion when moving to the left LOS; COP maximum excursion when moving to the anterior LOS; COP maximum excursion when moving to the posterior LOS **Model** 6 (any falls): Left Functional Reach; Right Functional Reach; Right Lateral Reach; Left Lateral Reach; Step-Up number; Berg Balance Scale score **Model** 7 (any falls): Left Functional Reach; Right Functional Reach; Right Lateral Reach; Left Lateral Reach; Step-Up number; Berg Balance Scale score; Gluteus medius onset time; Movement time in a high preparation step task; Step time in a high preparation step task; Maximum COP excursion when moving to the right LOS; COP maximum mediolateral velocity; Total distance moved in quiet stance	**Model** 4: AUC: no information Calibration: no information SN: 29% SP: 88% Total predictive ability: 67% **Model** 5: AUC: no information Calibration: no information SN: 6% SP: 97% Total predictive ability: 65% **Model** 6: AUC: no information Calibration: no information SN: 12% SP: 95% Total predictive ability: 66% **Model** 7: AUC: no information Calibration: no information SN: 59% SP: 86% Total predictive ability: 77%
Tromp et al. [3], 2001, The Netherlands, Development study	n = 1,374 Age, mean (SD): 75.2 (6.5) years Age, range: 64.8-88.6 years Of the 1,285 with complete follow-up, female: 51%	Any falls (428; 31.1%) Recurrent falls (147; 10.7%) Follow-up: 12 months	Logistic regression No information on model validation	**Model** 1 (any falls): Regression table with regression coefficients: constant: no information; previous falls: 0.90; urinary incontinence: 0.46; visual impairment: 0.44; use of benzodiazepines: 0.44 Score chart - previous falls: 5; urinary incontinence: 2; visual impairment: 2; use of benzodiazepines: 2	**Model** 1: AUC: 0.65 Calibration: no information

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	Prior falls: 31%			**Model** 2 (recurrent falls): Regression table with regression coefficients: constant: no information; previous falls: 0.99; urinary incontinence: 0.53; visual impairment: 0.82; use of benzodiazepines: 0.54 Score chart: previous falls: 5; urinary incontinence: 3; visual impairment: 4; use of benzodiazepines: 3	**Model** 2: AUC: 0.71 Calibration: no information Cut-off: 7 points SN: 54% SP: 79% PPV (CI): 24.9% (22.5-27.3%) NPV (CI): 93% (91.6-94.4%)
Stalenhoef et al. [4], 2002, The Netherlands, Development study	n = 302 Of the 287 with complete follow-up, age, mean (SD): - Female: 78.5 (5.2) years - Male: 77.2 (4.9) years Of the 287 with complete follow-up, female: 60% Prior falls: 66%	Recurrent falls (46; 15.2%) Follow-up: 9 months	Logistic regression No information on model validation	**Model** 1 (recurrent falls): Regression table including regression coefficients with SE: constant: -2.28; female gender: -0.39 (0.4); age >= 80 years: 0.04 (0.39); falls >= 2 in previous year: 1.14 (0.39); depression - SCL90 >= 22: 0.78 (0.37); hand dynamometry (men <= 22 kg or women: <= 12 kg: 1.14 (0.38); postural sway abnormal: 1.36 (0.58) Rounded scoring rule - men: age >= 80 years: was not included due to low impact; falls >= 2 in previous year: 6; depression - SCL90 >= 22 4; hand dynamometry (Men <= 22 kg or Women: <= 12 kg): 6; postural sway abnormal: 7 Rounded scoring rule – women: age >= 80 years: Was not included due to low impact; falls >= 2 in previous year: 4; depression - SCL90 >= 22: 2; hand dynamometry (men <= 22 kg or women: <= 12 kg): 4; postural sway abnormal: 5	**Model** 1: AUC: 0.79 Calibration: "The comparison of the percentages predicted probability with the percentage of observed recurrent fallers showed a general agreement. The predicted values of the model, calculated according to the Hosmer Lemeshow goodness of fit, showed good fit." Cut-off: 0.30 SN: 59% SP: 87% PPV: 52% NPV: 90%
Stel et al. [5], 2003, The Netherlands, Development study	n = 1,365 Age, mean (SD): 75.3 (6.4) years Age, range: 64.8-88.6 years Female: 51% Prior falls: 31%	Recurrent falls (337; 24.7%) Follow-up: - Primary length of follow-up: 3 years - Secondary length of follow-up: 1 year.	Tree-structured survival analysis No information on model validation	**Model** 1 (recurrent falls): Classification tree with a follow-up of 3 years: fall history, function limitations, dizziness, performance test score, grip strength, alcohol consumption, pain, educational level, and physical activity **Model** 2 (recurrent falls): Classification tree with a follow-up of 1 year: fall history, function limitations, and grip strength	**Model** 1: AUC: no information Calibration: no information **Model** 2: AUC: no information Calibration: no information
Boulgarides et al. [6], 2003, USA,	n = 106 Age, mean (SD):	Recurrent falls (20; 18.9%)	Logistic regression	**Model** 1 (recurrent falls): Regression table with coefficients and SE:	**Model** 1: AUC: no information Calibration: no information

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Development study	74.02 (5.64) years Age range: 65-90 years Of 99 participants included in analysis: female: 61% Prior falls: 50.5%	Follow-up: 12 months	No information on model validation	constant: no information; postural sway while standing on a firm surface with eyes closed: 1.786 (1.332); age: 0.072 (0.048); sex: 0.822 (0.540)	% Correct prediction: 80.8%, though only predicted 2/20 of multiple fallers SN: 10% SP: 98.7%
Nandy et al. [7], 2004, UK, Development study	n = 510 Of the 345 with complete follow-up, age, mean (SD): 74.4 (6.4) years Of the 345 with complete follow-up, female: 55% Prior falls: 25%	Any falls (no information) Follow-up: 6 months	Only SN, SP, PPV, NPV, and Youden's index were calculated No information on model validation	**Model** 1 (any falls): Three or more of the following risk factors: history of falling in the previous year, taking four or more prescribed medications, history of stroke or Parkinson's disease, and reported problems with balance and loss of proximal muscle strength	**Model** 1: AUC: no information Calibration: no information SN: 0.42 (0.32-0.54) SP: 0.92 (0.88-0.94) PPV: 0.57 (0.43-0.69) NPV: 0.86 (0.83-0.89) Youden's Index: 0.339 (0.185-0.493)
Pluijm et al. [8], 2006, The Netherlands, Development study	n = 1,365 Age, mean (SD): 75.3 (6.4) years Age, range: 64.8-88.6 years Female: 51.1% Prior falls: 14.2%	Recurrent falls (457; 33.5%) Follow-up: - Primary: 3 years - Secondary: 1 year	Logistic regression No information on model validation	**Model** 1 (recurrent falls within 3 years): Regression table with regression coefficients: constant: - 2.19; two or more previous falls: 0.71; dizziness: 0.77; functional limitations: 0.53; weak grip strength: 0.55; low body weight: 0.37; fear of falling: 0.34; the presence of dogs/cats in the household: 0.40; a high education level: 0.21; drinking of 18 or more alcoholic consumptions per week: 0.11; interaction term (high education × 18 or more alcohol consumptions per week): 0.86; interaction term (two or more previous falls × fear of falling): 0.83 Score chart: two or more previous falls: 4; dizziness: 4; functional limitations: 3; weak grip strength: 3; low body weight: 2; fear of falling: 2; the presence of dogs/cats in the household: 2; a high education level: 1; drinking 18 or more alcoholic consumptions per week: 1; interaction term (high education × 18 or more alcohol consumptions per week): 4; interaction term (two or more previous falls × fear of falling): 4 **Model** 2 (recurrent falling within 1 year): Regression table with regression coefficients: constant: - 3.13; two or more previous falls: 0.64; dizziness: 0.52; functional limitations: 0.39; weak grip strength: 0.65; low body weight: 0.32;	**Model** 1: AUC (CI): 0.71 (0.67-0.74) Calibration: The Hosmer-Lemeshow goodness-of-fit test for the multiple logistic regression was not significant (p=0.56), indicating that the model fits the data well Cut-off: 5 points SN: 59% SP: 71.4% PPV: 38.6% NPV: 85.1% **Model** 2: AUC: (CI): 0.72 (0.67- 0.77) Calibration: The Hosmer-Lemeshow goodness-of-fit test for the multiple logistic regression

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				fear of falling: 0.09; the presence of dogs/cats in the household: 0.81; a high educational level: 0.08; drinking of 18 or more alcoholic consumptions per week: - 0.15; interaction term (high education × 18 or more alcohol consumptions per week): 0.87; interaction term (two or more previous falls × fear of falling): 1.15	was not significant (p=0.94), indicating that the model fits the data well
Okochi et al. [9], 2006, Japan, Development study	n = 1,734 Age, mean (SD): 75.8 (6.8) years Female: no information Prior falls: 16%	Any falls (208; 12.0%) Follow-up: 6 months	Logistic regression Split-sample validation (random split, 50%/50%)	**Model** 1 (any falls): Scoring system from 0-13 based on the odds ratio at an integer level from logistic regression: history of falls - probable score 0/5; do you feel your walking speed has declined recently - probable score 0/2; do you use cane when you walk - probable score 0/2; is your back bended: probable score 0/2; do you take more than five kinds of prescription medicines - probable score 0/2	**Model** 1: AUC (95% CI): 0.74 (0.69-0.79) Calibration: no information Cut-off: 6 SN: 68% SP: 70% PPV: 27.9% Negative predictive power: 93%
Lindemann et al. [10], 2008, Germany, Development study	n = 65 Age, mean (SD): 67.7 (6.0) years Female: 57% Prior falls: 45%	Any falls (30; 46.2%) Follow-up: 12 months	The cut-off values for differentiating between persons, who fell and persons who did not, were defined for each parameter as the point on the Receiver Operating Characteristic curve with the minimal Euclidian distance to the point (0/1) No information on model validation	**Model** 1 (any falls): Adjusted mean valid step length (cut-off: 64% of body height), and at least one fall in previous year **Model** 2: (any falls): Adjusted max. valid step length (cut-off: 66% of body height), and at least one fall in previous year	**Model** 1: AUC and calibration: author response: "The information in the paper is the only we can provide. New calculations are not possible" SN (CI): 93% (86.7-100) SP (CI): 54% (40.5-67.1) PPV (CI): 70% (57.8-82.2) NPV (CI): 88% (78.7-96.3) +LR: 2.0 -LR: 0.1 **Model** 2: AUC: no information Calibration: no information SN (CI): 90% (82-98) SP (CI): 58% (44.5-70.9) PPV (CI): 71% (58.9-83.2) NPV (CI): 83% (73.4-93.3) +LR: 2.1 -LR: 0.2
Lamb et al. [11], 2008, USA, Development study	n = 1,002 Age, mean (SD): 78 (8.1) years	Any falls (346; 34.5%) Follow-up: 12 months	Tree-based classification Cross-validation (20 subsets)	**Model** 1 (any falls): Decision tree with self-report algorithm: how many falls have you had in the last year?; how often do you have problems balancing while	**Model** 1: AUC: no information Calibration: no information Fall probability threshold:

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	Of the 885 included, female: 100% Of 830 included, prior falls: 34%			walking?; how much difficulty do you have with activities of daily living? **Model** 2 (any falls): Decision tree with self-report and performance item algorithm: how many falls have you had in the last year?; how often do you have problems balancing while walking?; knee extensor strength test; 4-metre usual gait speed; Body Mass Index	>= 0.34; SN: 0.59; SP: 0.64; +LR: 1.64; -LR: 0.64; Diagnostic Odds Ratio: 2.56 Fall probability threshold: >= 0.44; SN: 0.46; SP: 0.77; +LR: 2.00; -LR: 0.70; Diagnostic Odds Ratio: 2.85 Fall probability threshold: >= 0.55; SN: 0.32; SP: 0.87; +LR: 2.46; -LR: 0.78; Diagnostic Odds Ratio: 3.15 Fall probability threshold: >= 0.62; SN: 0.09; SP: 0.96; +LR: 2.25; -LR: 0.95; Diagnostic Odds Ratio: 2.37 **Model** 2: AUC: no information Calibration: no information Fall probability threshold: >= 0.25; SN: 0.78; SP: 0.46; +LR: 1.44; -LR: 0.48; Diagnostic Odds Ratio: 3.02 Fall probability threshold: >= 0.33; SN: 0.73; SP: 0.56; +LR: 1.66; -LR: 0.48; Diagnostic Odds Ratio: 3.44 Fall probability threshold: >= 0.42; SN: 0.54; SP: 0.74 +LR: 2.08; -LR: 0.62; Diagnostic Odds Ratio: 3.34 Fall probability threshold: >= 0.46; SN: 0.47; SP: 0.80; +LR: 2.35; -LR: 0.66; Diagnostic Odds Ratio: 3.54 Fall probability threshold: >= 0.56; SN: 0.33; SP: 0.90

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					+LR: 3.30; -LR: 0.74; Diagnostic Odds Ratio: 4.43 Fall probability threshold: >= 0.69; SN: 0.16; SP: 0.97 +LR: 5.33; -LR: 0.87; Diagnostic Odds Ratio: 6.15
Delbaere et al. [12], 2010, Australia, Development study	n = 500 Age, mean (SD): 77.9 (4.1) years Female: 54% Prior falls: 29.6%	Recurrent falls (94; 18.8%) Follow-up: 12 months	Classification and Regression Tree (CRT) No information on model validation	**Model** 1 (recurrent falls): Risk groups: - Low risk: Physiological fall risk (Physiological Profile Assessment) <0.60. Subgroups were made from the Disability score >0. If Disability score >0, further subdivision was made using Incidental and Planned Exercise Questionnaire < 4hrs/week - High risk group: Physiological fall risk (Physiological Profile Assessment) >=0.60. Subgroups were made from Trail-Making-Test time <50. If Trail-Making-Test > 50, further subdivision was made using poor coordinated stability (error score >= 15). If score >= 15, further subdivision was done using Incidental and Planned Exercise Questionnaire > 0	**Model** 1: AUC and calibration: "We didn't calculate an AUC or related measure for our tree and our sample was not big enough to run a calibration analysis as well. No other classification measures were calculated " (author response)
Yamashita et al. [13], 2011, USA, Development study	n = 23,417 Age, mean (SD): 76.1 (8.94) years Female: 79% Prior falls: 3%	Any falls (approx. 1,400 (5.9%) Follow-up: between one day and 18 months due to the study using an open cohort design	Logistic regression No information on model validation	**Model** 1 (any falls): Regression table with coefficients: constant: no information; fall history: 0.997; female: 0.133; age: 0.013; blacks: -0.334; others: -0.363; married: -0.137; Alzheimer's disease: 0.055; cancer: -0.307; cataracts: -0.21; dementia: -0.135; depression: 0.334; diabetes: -0.034; emphysema: -0.171; glaucoma: 0.118; incontinence: 0.189; Parkinson's disease: 0.261; stroke: 0.103; vertigo: 0.085; total number of medications: 0.029; activities of daily living score: -0.07; instrumental activities of daily living score: 0.01; gait-shuffling: 0.027; gait-unsteady: 0.178; grasp-tremors: 0.426; grasp-weakness: -0.451; joint pain: 0.129; recent weight loss: 0.332; medication administration: 0.119; need for supervision: -0.265	**Model** 1: AUC: 0.61 Calibration: no information
Panzer et al. [14], 2011, USA, Development study	n = 74 Non-fallers: - Age, range: 65-87 years - Age, mean (SD): 75.1 (6.5) years	Recurrent falls (40; 54.1%) Follow-up: 12 months	Only sensitivity and specificity were calculated	**Model** 1 (recurrent falls): Multiple falls, gait velocity, turn time, turn number of steps, down 3 stairs, and step in tub **Model** 2 (recurrent falls):	**Model** 1: AUC: no information Calibration: no information Extracted from figure 2: SN: 52%; SP: 55%

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	Fallers: - Age, range: 70-94 years - Age, mean (SD): 80.1 (6.2) years Female: no information Prior falls: 63.5%		No information on model validation	Multiple falls, gait velocity, turn time, turn number of steps, and down 3 stairs **Model** 3 (recurrent falls): Multiple falls, gait velocity, turn time, and turn number of steps **Model** 4 (recurrent falls): Multiple falls, quiet standing, maximal leaning, sway area, and medial-lateral excursion **Model** 5 (recurrent falls): Multiple falls, quiet standing, maximal leaning	**Model** 2: AUC: no information Calibration: no information Extracted from figure 2: SN: 55%; SP: 55% **Model** 3: AUC: no information Calibration: no information Extracted from figure 2: SN: 55%; SP: 52% **Model** 4: AUC: no information Calibration: no information Extracted from figure 2: SN: 78%; SP: 55% **Model** 5: AUC: no information Calibration: no information Extracted from figure 2: SN: 68%; SP: 55%
Bongue et al. [15], 2011, France, Development study	n = 1,759 Age, mean (SD): 70.7 (4.6) years Female: 51% Prior falls: 26%	Any falls (563; 32%) Follow-up: 12 months	Cox regression No information on model validation	**Model** 1 (time to any falls): Regression table (coefficient): baseline hazard: no information; women 0.506; living alone: in couple: 0, family: 0.438, alone: 0.315; self-reported osteoarthritis: 0.22; history of falls - 1 year: 0 falls: 0, 1 fall: 0.616, 2 falls: 0.907, 3 or more falls: 1.42; psychoactive drug use: 0.217; balance impairment: 0.270 Scoring rule (points): women (2); living alone (1); self-reported osteoarthritis (1); history of falls - 1 year: 1 fall (2), 2 falls (4), 3 or more falls (6); psychoactive drug use (1); balance impairment (1) Cut-off: 7 Low risk: Score: 0-3; frequencies: 55.3; OR: 1 Moderate risk: Score: 4-6; frequencies: 34.9; OR 2.4 (2.2-3.4) High risk: Score: 7-12; frequencies: 9.8; OR: 7.8 (5.5-11.1)	**Model** 1: AUC (CI): 0.70 (0.67-0.73) Calibration: no information Youden index = 3, for this: SN: 70.2% SP: 60.3% PPV: 45.5% NPV: 81.1% Evolution of PPV and NPV cut-off = 7, for this: SN: 19.2% SP: 96.5% PPV: 72% NPV: 72.7%
Viccaro et al. [16], 2011, USA,	n = 492	First time falls (83; 19.5%)	Logistic regression	**Model** 1 (first time fall):	**Model** 1: AUC: 0.60

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Development study	Age, mean (SD): 74 (5.7) years Of the 457 with complete follow-up (except n = 18 who died during follow-up): female: 43.5% Prior falls: 29.7%	Any falls (155; 36.5%) Recurrent falls (58; 13.9%) Follow-up: 12 months	No information on model validation	Timed Up & Go test, 4 m gait speed test, age, and fall history **Model** 2 (any fall): Timed Up & Go test, 4 m gait speed test, age, and fall history **Model** 3 (recurrent falls): Timed Up & Go test, 4 m gait speed test, age, and fall history	Calibration: no information **Model** 2: AUC: 0.729 Calibration: no information **Model** 3: AUC: 0.786 Calibration: no information
Yamashita et al. [17], 2012, USA, Development study	n = 9,661 Age, mean (SD): 74.2 (7.16) years Of the 9,592 included in the analyses: female: 57.8% Prior falls: 31%	Any falls (3,299; 34%) Follow-up: 24 months	Logistic Tree with Unbiased Selection (LOTUS)/ Logistic Regression Tree Method Cross-validation (10 subsets)	**Model** 1 (any falls): Fall history, age, difficulty with knees, activities of daily living, cognitive impairment, self-rated health, instrumental activities of daily living, prescription drugs, and stroke	**Model** 1: AUC: 0.71 Calibration: no information
Weiss et al. [18], 2013, Israel, Development study	n = 71 Age, mean (SD): 78.36 (4.71) years Female: 65% Prior falls: 45%	Recurrent falls (12; 16.9%) Follow-up: 6 months	Logistic regression No information on model validation	**Model** 1 (recurrent falls): Four Square Step Test, total activity duration (Accelerometer), anterior-posterior acceleration range (Accelerometer), anterior-posterior width (Accelerometer), and age	**Model** 1: AUC: no information Calibration: no information SN: 75% SP: 100% "94.7% of the subjects were successfully identified as future fallers and non-fallers"
Hnizdo et al. [19], 2013, USA, Validation	n = 113 Age, mean: 79.8 years Female: 34.6% Prior falls: 49.5%	Any falls (33; 29.2%) Follow-up: participants were followed until discharged from home health services	Validation study	**Model** 1 (any falls): Age, fall history, elimination problems, high risk medications, use of patient care equipment, limited mobility, and altered cognition	**Model** 1: AUC (95% CI): 0.66 (0.55-0.78) Calibration: no information Cut-off: 14 points SN: 72.5% SP: 52.2% PPV: 39.6% NPV: 81.4%
de Vries et al. [20], 2013, The Netherlands, Development study	n = 1,509 Age, median (range): 75.6 (64.8-88.8) years Female: 51.8% Prior falls: 32.2%	Recurrent falls, ≥ 2 falls (174; 11.5%) Recurrent falls, ≥ 3 falls (91; 6%) Any falls	Cox regression Logistic regression No information on model validation	**Model** 1 (time to second fall): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24 **Model** 2 (any falls): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low	**Model** 1: AUC:0.58 (0.53-0.62) Calibration: no information **Model** 2: AUC: 0.51 (0.346-0.56) Calibration: no information **Model** 3:

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		(468; 31.0%) Follow-up: 12 months		visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24 **Model** 3 (recurrent falls, ≥ 2 falls): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24 **Model** 4 (recurrent falls, ≥ 3 falls): Low mastery, depression, urinary incontinence, hearing impairment, low physical activity, low visual acuity, body mass index ≤23, low peak flow, Mini-Mental State Examination ≤ 24	AUC: 0.50 (0.42-0.57) Calibration: no information **Model** 4: AUC: 0.49 (0.39-0.59) Calibration: no information
Muhaidat et al. [21], 2014, United Kingdom, Development study	n = 66 Non-fallers: Age, mean (SD): 75 (11.5) years Fallers: Age, mean (SD): 82 (12) years Female: 66% Prior falls: 45%	Any fall (13; 19.7%) Follow-up: 6 months	Random Forrest Classification Training set 67% of the sample Test set: 33% of the sample	**Model** 1 (any falls): Table of predictors with corresponding Mean Decrease in Accuracy, and Mean Decrease in Gini: time required to complete triple task, time required avoiding a moving obstacle and cup, time required for TUG and cup, time required for single-task avoiding a moving obstacle, absolute difference in time between single-task TUG and dual-task TUG	**Model** 1: AUC: no information Calibration: no information Out-of-bag error rate: 27.4% Correct classification: 72.6%
Gadkaree et al. [22], 2015, USA, Development study	n = 7,609 Age groups, % (CI): - 65-69: 27.9% (27.0-29.0) - 70-74: 25.0% (24.1-25.8) - 75-79: 19.1% (18.2-19.9) - 80-84: 14.7% (14.0-15.4) - 85-89: 9.1% (8.5-9.8) - 90+: 4.3% (3.8-4.7) Female: 56.6% Prior falls: 30.5%	Any falls (2,028; 26.7%) Recurrent falls (957; 12.6%) Follow-up: 12 months	Logistic regression Split-sample validation (random split; 66.6%/33.3%) Cross-validation (no information on subsets)	**Model** 1 (any falls): Age, gender, and race **Model** 2 (recurrent falls): Age, gender, and race **Model** 3 (any falls): y = -1.44 + ((Age 70-74 years)* -0.33) + ((Age 75-59 years)*0.07) + ((Age 80-84 years)*0.17) + ((Age 85-89 years)*0.37) + ((Age 90+ years)*0.26) + Female*0.12 + (Black ethnicity * -0.27) + (Other ethnicity * -0.52) + (Hispanic ethnicity * 0.07) + Self-reported balance problems*0.69 + Fall history*1.15 **Model** 4 (recurrent falls)	**Model** 1: AUC (95% CI): 0.57 (0.54-0.60) Calibration: no information **Model** 2: AUC (95% CI): 0.59 (0.56-0.61) Calibration: no information **Model** 3: AUC (95% CI): 0.69 (0.67-0.71) Calibration: no information Performance in validation set: AUC (95% CI): 0.70 (0.67-0.73) **Model** 4: AUC (95% CI): 0.77 (0.74-0.79)

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				y = 2.67 + ((Age 70-74 years)* -0.66) + ((Age 75-59 years)*-0.08) + ((Age 80-84 years)*0.11) + ((Age 85-89 years)*0.49) + ((Age 90+ years)*0.47) + Female*-0.22 + (Black ethnicity * -0.27) + (Other ethnicity * -0.99) + (Hispanic ethnicity * 0.02) + Self-reported balance problems*1.11 + Fall history*1.46 **Model** 5 (any falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, and hearing impairment **Model** 6 (recurrent falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, and hearing impairment **Model** 7 (any falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, hearing impairment, and Short Physical Performance Battery **Model** 8 (recurrent falls): Age, gender, race, self-reported balance problems, history of falls, heart attack, heart disease, stroke, hypertension, diabetes, osteoporosis, vision impairment, hearing impairment, and Short Physical Performance Battery	Calibration: no information Performance in validation set: AUC (95% CI): 0.76 (0.73-0.80) **Model** 5: AUC (95% CI): 0.71 (0.69-0.73) Calibration: no information **Model** 6: AUC (95% CI): 0.78 (0.76-0.81) Calibration: no information **Model** 7: AUC (95% CI): 0.72 (0.70-0.73) Calibration: no information **Model** 8: AUC (95% CI): 0.79 (0.76-0.81) Calibration: no information
Cawthon et al. [23], 2015, USA, Development study	n = 5,994 Age, mean: 74 years (based on other studies on the same cohort)	Recurrent falls (694; 11.6%) Follow-up: 12 months	Logistic regression No information on model validation	**Model** 1 (recurrent falls): Age and Baumgartner Sarcopenia Definition **Model** 2 (recurrent falls): Age and Newman Sarcopenia Definition	**Model** 1: Change in AUC compared to age alone (AUC: 0.577): 0.000 (-0.002; 0.003) Calibration: "We did not generate calibration plots for these

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	Female: 0% Prior falls: 21% (based on other studies on the same cohort)			**Model** 3 (recurrent falls): Age and IWG Sarcopenia Definition **Model** 4 (recurrent falls): Age and EWGSOP Sarcopenia Definition **Model** 5 (recurrent falls): Age and FNIH Definition#1 **Model** 6 (recurrent falls): Age and FNIH Definition#2	analyses, just the discrimination and the C statistic information" (author response) NRI events: 0.12 (0.08, 0.16) NRI non-events: -0.12 (-0.14, -0.11) **Model** 2: Change in AUC compared to age alone (AUC: 0.577): 0.001 (-0.002; 0.003) Calibration: no information NRI events: 0.07 (0.04, 0.11) NRI non-events: -0.08 (-0.09, -0.06) **Model** 3: Change in AUC compared to age alone (AUC: 0.577): 0.010 (0.002; 0.018) Calibration: no information NRI events: -0.33 (-0.38, -0.28) NRI non-events: 0.34 (0.32, 0.35) **Model** 4: Change in AUC compared to age alone (AUC: 0.577): 0.009 (0.002; 0.015) Calibration: no information NRI events: -0.33 (-0.38, -0.28) NRI non-events: 0.35 (0.34, 0.36) **Model** 5: Change in AUC compared to age alone (AUC: 0.577): 0.004 (-0.001; 0.008) Calibration: no information NRI events: -0.11 (-0.14, -0.08) NRI non-events: 0.07 (0.06, 0.08) **Model** 6:

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					Change in AUC compared to age alone (AUC: 0.577): 0.001 (-0.001; 0.003) Calibration: no information NRI events: -0.05(-0.06, -0.03) NRI non-events: 0.03(0.02, 0.03)
Palumbo et al. [24], 2016, Italy, Germany, Ireland, and England, Validation	ActiFE: n = 1,416 ELSA: n = 3,303 InCHIANTI: n = 892 TILDA: n = 2,101 Age, mean (SD): ActiFE: 75.7 (6.76) years ELSA: 74.56 (7.31) years InCHIANTI: 73.78 (6.62) years TILDA: 72.79 (5.22) years Female: ActiFE: 56.8% ELSA: 56.7% InCHIANTI: 56.2% TILDA: 53.5% Prior falls: ActiFE: 36.1% ELSA: 22.7% InCHIANTI: 20.8% TILDA: 22.8%	Any falls ActiFE (466; 32.9%) ELSA (730; 22.1% 1 years adjusted) InCHIANTI (203; 22.8%) TILDA (569; 27.1% 2 years adjusted) Follow-up: ActiFE: 12 months ELSA: 2 years InCHIANTI: 1 year TILDA: 2 years	Validation study	**Model** 1 (any falls): Age, cognitive impairment, depression, diabetes, comorbidity, dizziness and vertigo, fear of falling, female sex, gait problems, hearing impairment, history of falls, history of stroke, instrumental disability, living alone, number of medications, pain, Parkinson's disease, physical activity limitation, physical disability, poor self-perceived health status, rheumatic disease, urinary incontinence, use of antiepileptics, use of antihypertensives, use of sedatives, vision impairment, and walking aid use	**Model** 1: ActiFE: AUC (95% CI): 0.562 (0.530 - 0.594) ELSA: AUC (95% CI): 0.699 (0.680 - 0.718) InCHIANTI: AUC (95% CI): 0.636 (0.594 - 0.681) TILDA: AUC (95% CI): 0.685 (0.660 - 0.709) Calibration: calibration plots were displayed for all four cohorts. For ActiFE and InCHIANTI, participants with low (high) risk scores, experienced more (respectively, less) falls than expected. For ELSA and TILDA, the model overestimated the risk consistently across strata
Rodriguez-Molinero et al. [25], 2017, Spain, Development study	n = 772 Of participants completing the first follow-up period, age, median (SD): 80.7 (0.1) years Of participants completing the first follow-up period, female: 62.5% Prior falls: 26.4%	Recurrent falls (43; 9.9%) Follow-up: 12 months	No regression analyses were performed. Sensitivity, specificity and area under the ROC curve (AUC) were calculated as well as the Odds Ratio (OR) and Relative Risk (RR) associated to positive responses	**Model** 1(recurrent falls): Score chart (range 1-6) on the questions: - Have you ever fallen in the last 6 months? - What is the probability that you fall in the next few months?	**Model** 1: AUC (95% CI): 0.74 (0.66-0.82) Calibration: after contacting study authors, a calibration plot was provided showing acceptable calibration. Calibration slopes were not considered. Cut-off: 3 points SN (95% CI): 70% (56%-84%) SP (95% CI): 72% (68%-76%)

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			No information on model validation		
Lohman et al. [26], 2017, USA, Development study	n = 7,609 Age groups, n (%): 65-69 years: 2,099 (28.4%) 70-74 years: 1,863 (25.2%) 75-79 years: 1,427 (19.3%) 80-84 years: 1,079 (14.6%) 85-89 years: 636 (8.6%) 90+ years: 288 (3.9%) Female: 56.4% Prior falls: no information	Any falls (3,903; 51.3%) Recurrent falls (2,181; 28.7%) Follow-up: 48 months	Logistic regression No information on model validation	**Model** 1 (any falls): STEADI algorithm: - Low risk, all present: no falls in past year, no worrying about falling, no unsafe/unsteady feeling while walking - Moderate risk, all present: yes, to one of the above-mentioned questions the low risk group, >4 chair stands in 30 sec., completion of all balance stages in 4 stage balance test. If NO to one of tests then both succeeding questions need to be answered as follows: no multiple falls in the past year, and no previous hip fracture since the age of 50 - High risk, all present: the same as in the moderate risk group except one of the succeeding questions are answered as follows: yes, to multiple falls in the past year or yes to previous hip fracture since the age of 50 **Model** 2 (any falls): Covariates: age, race, gender, education, living alone, smokings status, body mass index, vision impairment, hearing impairment, medical burden, functional Impairment, and frailty **Model** 3 (any falls): Model 1 and 2 combined	**Model** 1: AUC: 0.641 ("No CI was calculated" – author response) Calibration: no information SN: 65% SP: 65% PPV: 62% NPV 68% **Model** 2: AUC: 0.575 Calibration: no information **Model** 3: AUC: Calibration: no information
Kim et al. [27], 2017, USA, Development study + Validation study	Development: n = 5,593 (3,960 at follow-up) Validation: n = 4,424 (3,273 at follow-up) Age, median (IQR): Development: 77 (71, 83) Validation: 78 (71, 83) Female: Development: 58.4% Validation: 57.4% Prior falls: no information	Recurrent falls - Development: (834; 14.9%) - Validation: (514; 11.6%) Follow-up: No exact length of follow-up was reported. However, they reported that they wanted to predict the outcomes in the following year from baseline	Lasso regression model Bootstrap validation	**Model** 1 (recurrent falls - development): Age, sex, combined comorbidity index, 52 International Classification of Diseases 9-codes, 25 Current Procedural Terminology codes, and 16 Healthcare Common Procedure Coding System level II codes **Model** 2 (recurrent falls - validation): Age, sex, combined comorbidity index, 52 International Classification of Diseases 9-codes, 25 Current Procedural Terminology codes, and 16 Healthcare Common Procedure Coding System level II codes	**Model** 1: C-statistic: 0.62-0.66 Calibration: no information **Model** 2: C-statistic: consistent with C-statistic in Development study sample Calibration: no information

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Palumbo et al. [28], 2018, Italy, Development study	n = 541 Age, mean (SD): 82.4 (6.5) years Of the 438 participants with a complete data set, female: 60.7% Prior falls: 27%	Recurrent falls (34; 6.3%) Any falls (87; 16.1%) Follow-up: 12 months	No regression analysis was performed. The predictive accuracy was quantified from TP, TN, FP and FN in terms of SN, SP, PPV, NPV, and accuracy No information on model validation	**Model** 1 (any fall): A table with classification measures for the model with 3 different cut-off values for the TUG-test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Timed up and go test (cut-off: >12 s, >13.5s, and >15s) **Model** 2 (any fall): A table with classification measures for the model with 2 different cut-off values for the SPPB. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Short Physical Performance Battery (cut-off: < 9, and < 11 point) **Model** 3 (any falls): A table with classification measures for the model with 2 different cut-off values for the 7m gait speed test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and 7m gait speed test (cut-off: <0.8 m/s, and < 1m/s) **Model** 4 (recurrent falls): A table with classification measures for the model with 3 different cut-off values for the TUG-test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Timed up and go test (cut-off: >12 s, >13.5s, and >15s) **Model** 5 (recurrent falls): A table with classification measures for the model with 2 different cut-off values for the SPPB. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and Short Physical Performance Battery (cut-off: < 9, and < 11 point)	**Model** 1: AUC: no information Calibration: no information TUG > 12s: SN: 36.5% (22.3%-54.5%) SP: 82.5% (76.9%-87.1%) PPV: 25.5% (16.8%-37.6%) NPV: 88.8% (83%-93.6%) Accuracy: 76% (70.2%-81.4%) TUG > 13.5s: SN: 35.8% (23.2%-52.7%) SP: 84% (79.3%-88.4%) PPV: 26.9% (17.3%-38.8%) NPV: 88.8% (83.9%-93.7%) Accuracy: 77.2% (72.4%-82.3%) TUG > 15s: SN: 35.1% (22.7%-52.6%) SP: 84.1% (79.3%-88.4%) PPV: 26.7% (17.5%-38.7%) NPV: 88.7% (83.2%-93.1%) Accuracy: 77.2% (71.7%-82.4%) **Model** 2: AUC: no information Calibration: no information SPPB < 9: SN: 37.2% (24.1%-54.1%) SP: 83.4% (78.7%-87.7%) PPV: 27% (17.8%-37.6%) NPV: 89% (83.3%-94%) Accuracy: 76.9% (71%-81.8%) SPPB < 11: SN: 43.3% (28.4%-62.7%) SP: 79% (72.7%-84.5%) PPV: 25.4% (16.4%-35.8%) NPV: 89.4% (83.9%-94.3%) Accuracy: 74% (67.8%-79.9%) **Model** 3: AUC: no information Calibration: no information

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				Model 6 (recurrent falls): A table with classification measures for the model with 2 different cut-off values for the 7m gait speed test. Predictors were two or more falls in the past 12 months, presents with acute fall, difficulty with walking or balance, single fall in the past 12 months, and 7m gait speed test (cut-off: <0.8 m/s, and < 1m/s)	Gait speed < 0.8 m/s: SN: 35.1% (22.6%-52.5%) SP: 84.3% (78.8%-88.6%) PPV: 26.9% (17.7%-39.1%) NPV: 88.8% (82.9%-93.4%) Accuracy: 77.4% (71%-82.5%) Gait speed < 1 m/s: SN: 35.8% (22.4%-54.4%) SP: 82.4% (76.9%-87.3%) PPV: 25.1% (15.9%-36.5%) NPV: 88.6% (83%-93.3%) Accuracy: 75.8% (69.8%-81.5%) **Model** 4: AUC: no information Calibration: no information TUG > 12 s: SN: 56.2% (32.2%-92.8%) SP: 82.1% (76.9%-86.6%) PPV: 16.8% (8.9%-27.8%) NPV: 96.7% (92.9%-99.7%) Accuracy: 80.5% (75%-85.3%) TUG > 13.5s SN 56.2% (27.6%-89.8%) SP: 83.6% (79.4%-87.6%) PPV: 18.1% (9.7%-29.2%) NPV: 96.7% (92.3%-99.6%) Accuracy: 81.9% (76.9%-86.4%) TUG > 15 s SN: 56.2% (30.3%-92.6%) SP: 83.8% (79.2%-87.7%) PPV: 18.3% (10%-28.6%) NPV: 96.7% (93.1%-99.7%) Accuracy: 82.1% (76.9%-86.8%) **Model** 5: AUC: no information Calibration: no information SPPB < 9: SN: 56.2% (32.1%-93.2%)

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					SP: 82.9% (78.1%-87%) PPV: 17.5% (9.3%-28.4%) NPV: 96.7% (93%-99.7%) Accuracy: 81.3% (76%-85.6%) SPPB < 11: SN: 59% (32.3%-97.4%) SP: 78.1% (72.4%-83.3%) PPV: 14.8% (8.1%-24.3%) NPV: 96.7% (92.7%-99.9%) Accuracy: 76.9% (71%-82.6%) **Model** 6: AUC: no information Calibration: no information Gait speed < 0.8 m/s: SN: 56.2% (30.6%-91.7%) SP: 84% (79.3%-88.2%) PPV: 18.4% (9.9%-29.9%) NPV: 96.7% (93.1%-99.7%) Accuracy: 82.3% (77.2%-87.2%) Gait speed < 1 m/s: SN: 56.2% (30.8%-92%) SP: 82.1% (77.2%-86.5%) PPV: 16.8% (8.9%-27.4%) NPV: 96.7% (92.9%-99.6%) Accuracy: 80.5% (74.9%-85.3%)
Singh et al. [29], 2019, Malaysia, Development study	n = 325 Age, mean (SD): 67.55 (5.5) years Of the n = 305 analysed, female: 56.1% Prior falls: 16.7%	Any falls (81; 24.9%) Follow-up: 6 months	Logistic regression No information on model validation	**Model** 1 (any falls): constant: -5.03, age: -0.003, gender: 0.19, medication: -0.24, primary education: -0.27, secondary education: -0.85, history of falls: 0.67, Walk While Talking Test: 0.68, gait speed: 0.25, instrumental activities of daily living: -0.01, Timed up and go test: 0.14, and Physiological Profile Assessment: 1.16 **Model** 2 (any falls): constant: -5.06, age: 0.05, gender: 0.46, medication: -0.14, primary education: 0.07, secondary education: 0.85, history of falls: 0.12, Walk While Talking Test: 0.23, gait speed: -	**Model** 1: Cox-Snell R^2: 0.21 Nagelkerke R^2: 0.31 AUC: no information Calibration: "Hosmer-Lemeshow test result confirmed that the model was a good fit for the data $\chi 2(df = 8, N = 305) = 10.80, P = .21$" Accuracy: 76.6% **Model** 2: Cox-Snell R^2: 0.07 Nagelkerke R^2: 0.95 AUC: no information Calibration: "Hosmer-Lemeshow test results confirmed that the

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				0.07, instrumental activities of daily living: -0.03, and Timed up and go test: 0.16	model was a good fit for the data $\chi 2$ (df = 8, N =305) = 4.77, P = 0.78" Accuracy: 74.1%
Gillain et al. [30], 2019, Belgium, Development study	n = 105 Age, mean (SD): 71.3 (5.4) years Age, range: 65-89 years Of the 96 analysed, female: 50% Prior falls: 0%	First time falls (35; 33.3%) Follow-up: 24 months	Classification tree Split-sample validation (no information on allocation) Cross-validation (10 subsets)	**Model** 1 (first time falls): Classification tree: symmetry dual task walking condition cost, fast walking condition stride length, stiffness, comfortable walking condition mean minimum toe clearance, dual task walking condition coefficient of variation of minimum toe clearance cost, fast walking condition variance of minimum toe clearance values, fast walking condition mean minimum toe clearance value, dual task walking condition delta1 minimum toe clearance, and gender	**Model** 1: AUC: 0.84 Calibration: no information SN: 80% SP: 87% PPV: 78% NPV: 88%

Note: AUC = Area Under the Curve; Any falls = both single a CI = confidence interval; COP = centre of pressure; FN = False negative; FP = False positive; kg = kilograms; LOS = limits of stability; +LR = positive likelihood ratio; -LR = negative likelihood ratio; PPV = positive predictive value; N = no; NPV = negative predictive value; n = number; SD = standard deviation; SE = standard error; SN = sensitivity; SP = specificity; TN = True negative; TP = True positive; Y = yes; % = percentage proportion

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Appendix 10: extracted data

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APPENDIX 6: REFERENCES EXCLUDED DURING SCREENING OF TITLES AND ABSTRACTS:

Title	Authors	Published Year	Journal	Volume	Issue	Pages	DOI
Activity Recommendation or Intervention Reporting Index (aCRIN) a Novel Work-Related Activity Intervention Classification Framework	Kang, H., Hidajat, L. H. Kaplan, A. et al.		Int J Environ Res Public Health	18	3	1-12	10.3390/ijerph18031165
Active Recruitment, Informed Consent and Retention of Low-Income Children and Families Enrolled in a Clinical Trial Offering Financial Incentives	Finan, S., Madden, M., Gordon, M., Hughes, R., Wilson, K. et al.		Contemp Clin Trials Commun	16		1-6	10.1016/j.conctc.2019.100471
Adapting Evidence-Based Interventions for Community Use: Lessons Learned From Implementing a Home Visiting Program	Duggan, A., Fiene, R., Szapocznik, M., Campbell, K. et al.		Child Youth Serv Rev	30	2	1-12	10.1016/j.childyouth.2007.07.009
Adaptation of Interventions for Online Delivery and Evaluation Through Interviews With Health Leaders: Deductive Thematic Analysis Using the Conceptual Framework of Normalization Process Theory	Murray, E., Treweek, S., Pope, C., MacFarlane, A., Ballini, L. et al.		J Med Internet Res	12	1	1-12	10.2196/jmir.1178
Adaptive Designs for Clinical Trials	Chow, S., Chang, M.		Annu Rev Stat Appl	8		1-29	10.1146/annurev-statistics-030718-105251
Adaptive Randomization of Neurosurgical Trials	Antonijevic, Z., Kim-Tenser, M., Pierce, J., Sauer, C., Sanossian, N. et al.		J Stroke Cerebrovasc Dis	26	12	2799-2802	10.1016/j.jstrokecerebrovasdis.2017.07.014
Adaptive Trial Designs	Bhatt, D., Mehta, C.		Annu Rev Pharmacol Toxicol	56		541-556	10.1146/annurev-pharmtox-032112-135235
Addressing Attrition in Longitudinal Studies: How Useful Are Strategies for Improving Participation and Reducing Bias?	Ribisl, K., Walton, M., Mowbray, C., Luke, D., Davidson, W. et al.		J Community Psychol	24	1	1-19	10.1002/(SICI)1520-6629(199601)24:1<1::AID-JCOP1>3.0.CO;2-C
Addressing Barriers to Online Mindfulness: Investigating the Acceptability of a Text Messaging Delivered Intervention to Increase Uptake of Mindfulness-Based Programs	Bostock, S., Crosswell, A., Prather, A., Steptoe, A. et al.		Int J Behav Med	26	2	157-167	10.1007/s12529-018-9758-x
Addressing Participant Attrition in a Dyadic Research Study With Cognitively Impaired Older Adults	Wackerbarth, S., Johnson, M.		Qual Health Res	11	5	589-603	10.1177/104973201129119352
Adherence and Retention in Behavioral Weight Loss Trials: Identifying Causes, Consequences, and Future Directions	Wing, R., Phelan, S.		Ann Behav Med	25	2	132-137	10.1207/S15324796ABM2502_07
Adherence to Internet-Delivered Cognitive Behavioural Therapy Intervention for Social Anxiety Disorder: Clinical Correlates and Treatment Outcome	Andersson, G., Carlbring, P., Furmark, T., Carlbring, P., Hofmann, S. et al.		Behav Change	25	4	235-246	10.1375/howc.25.4.235

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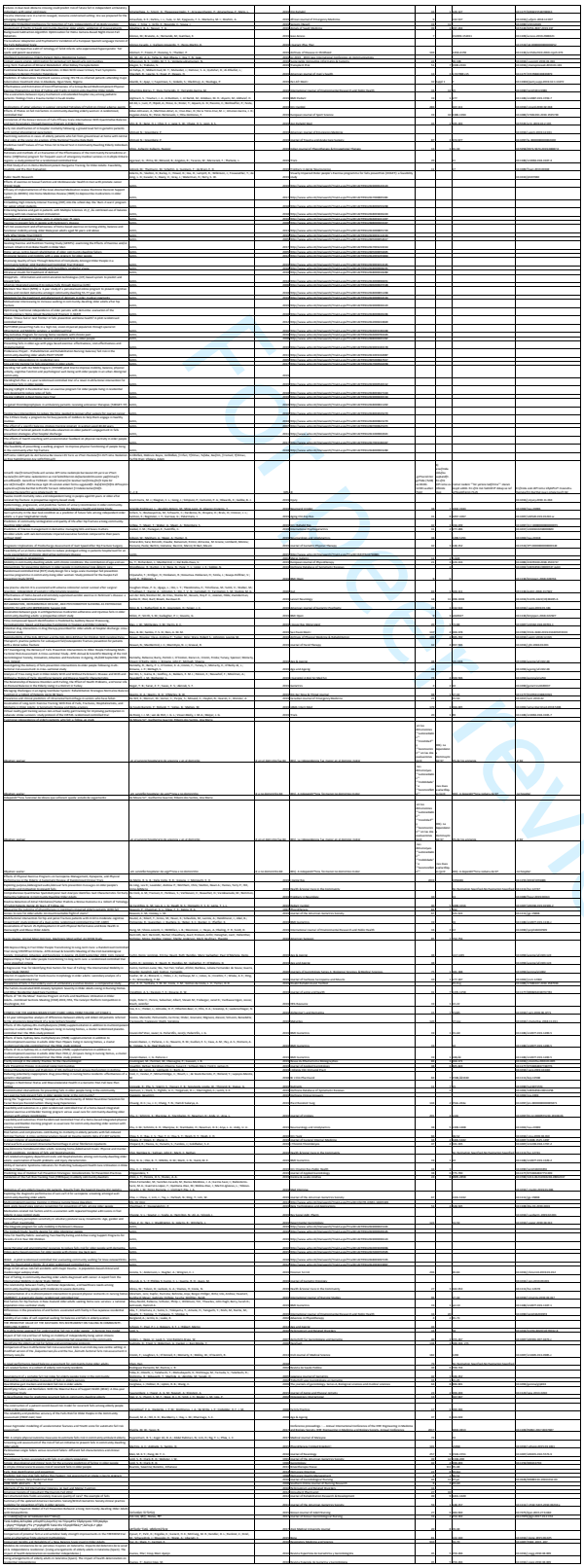

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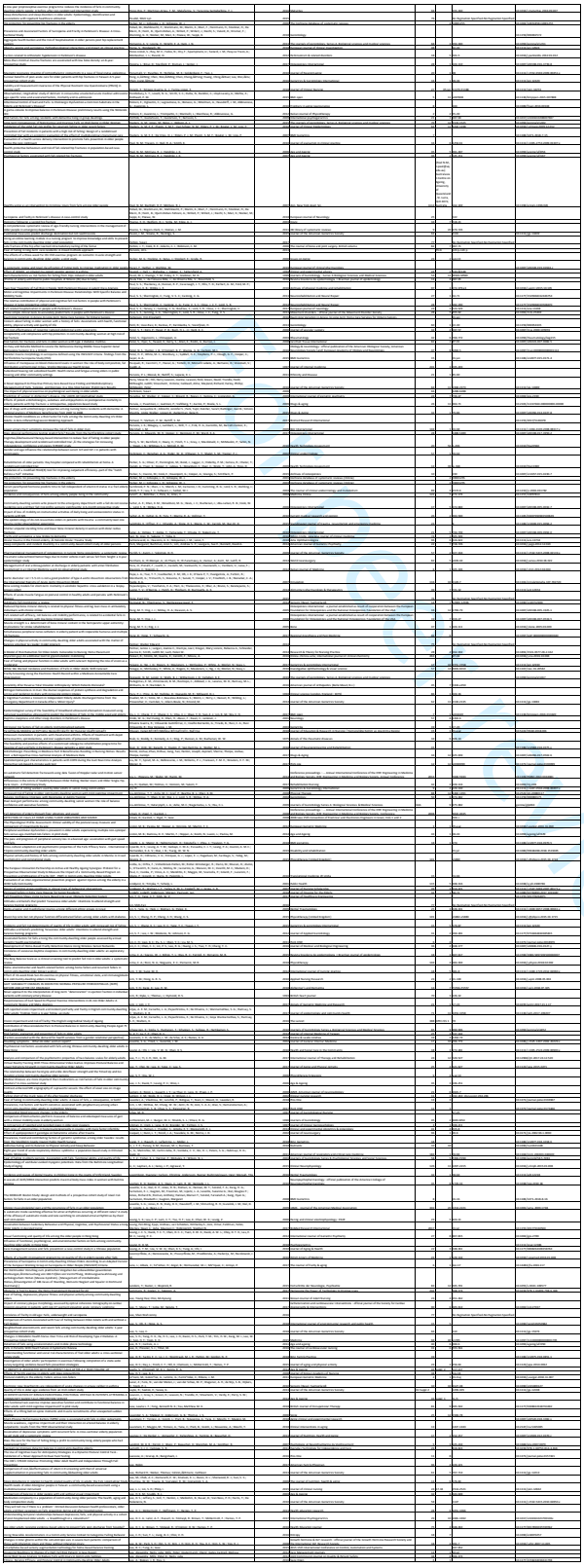

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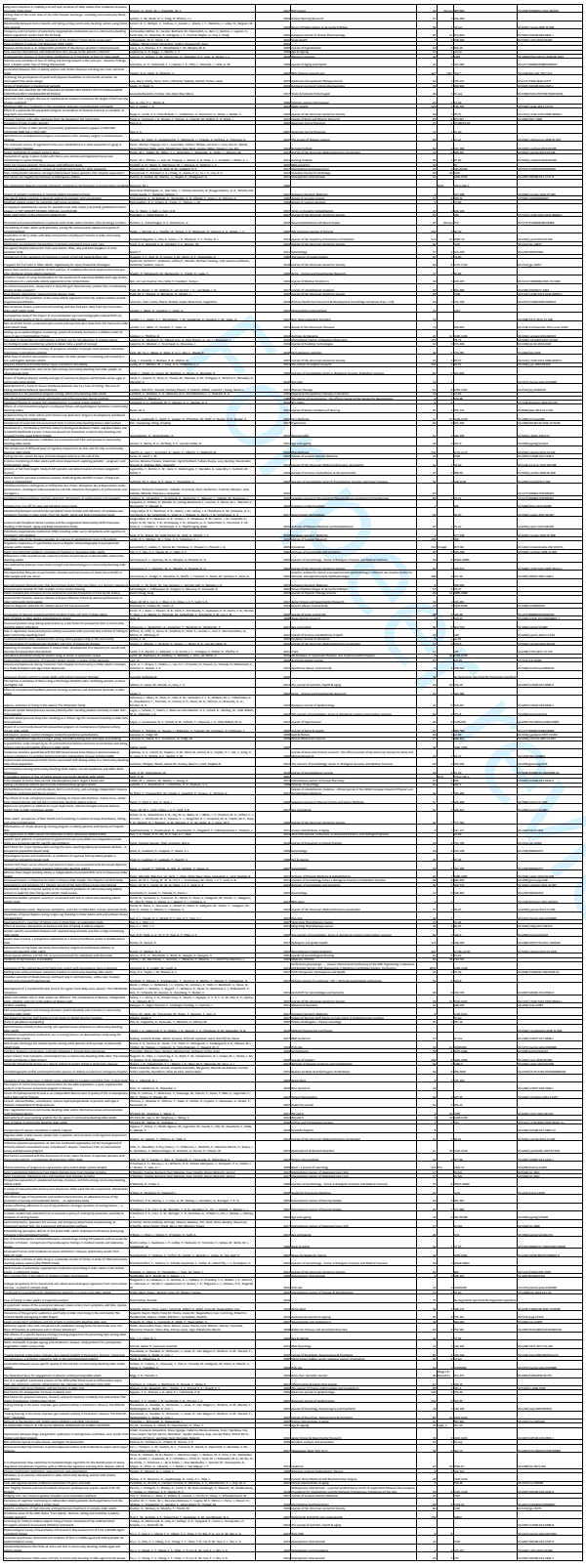

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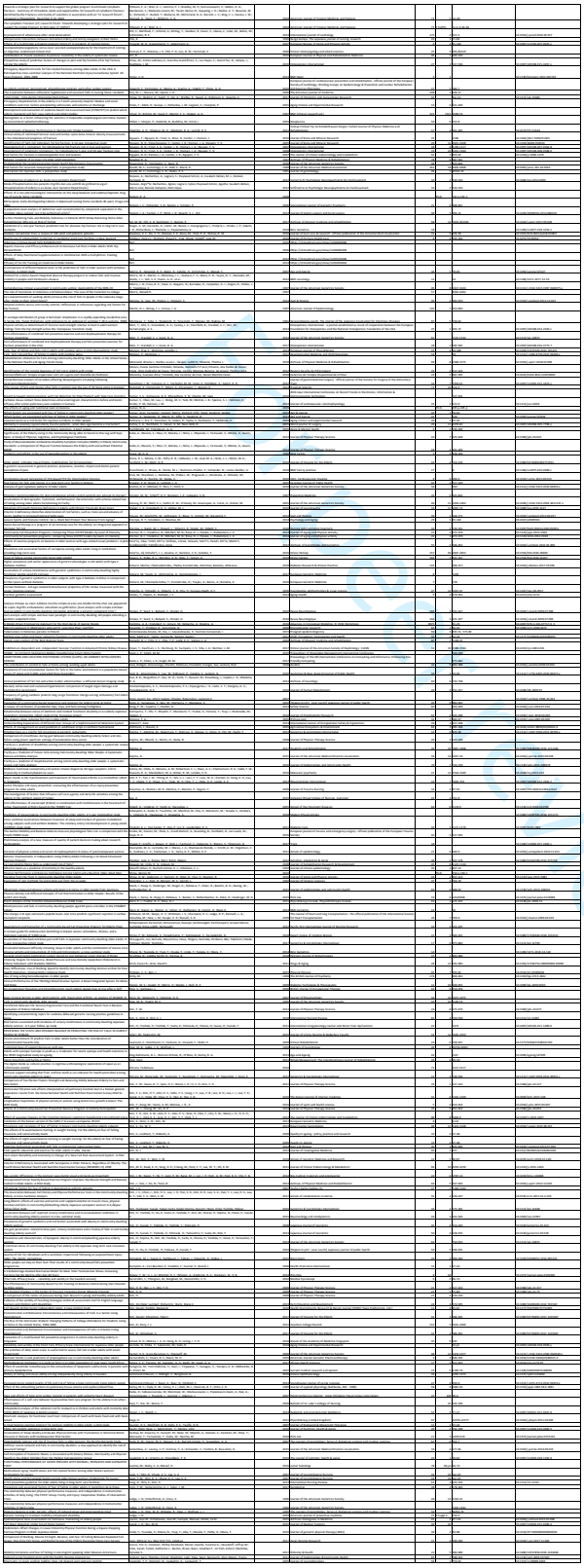

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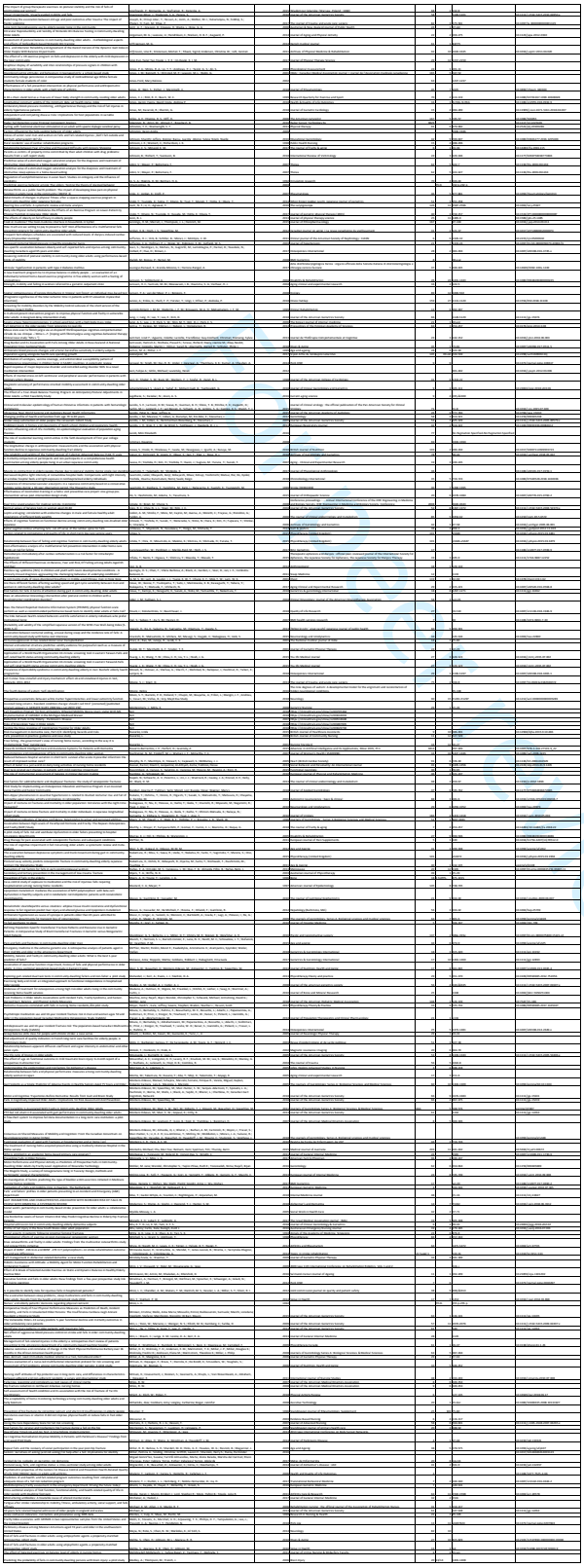

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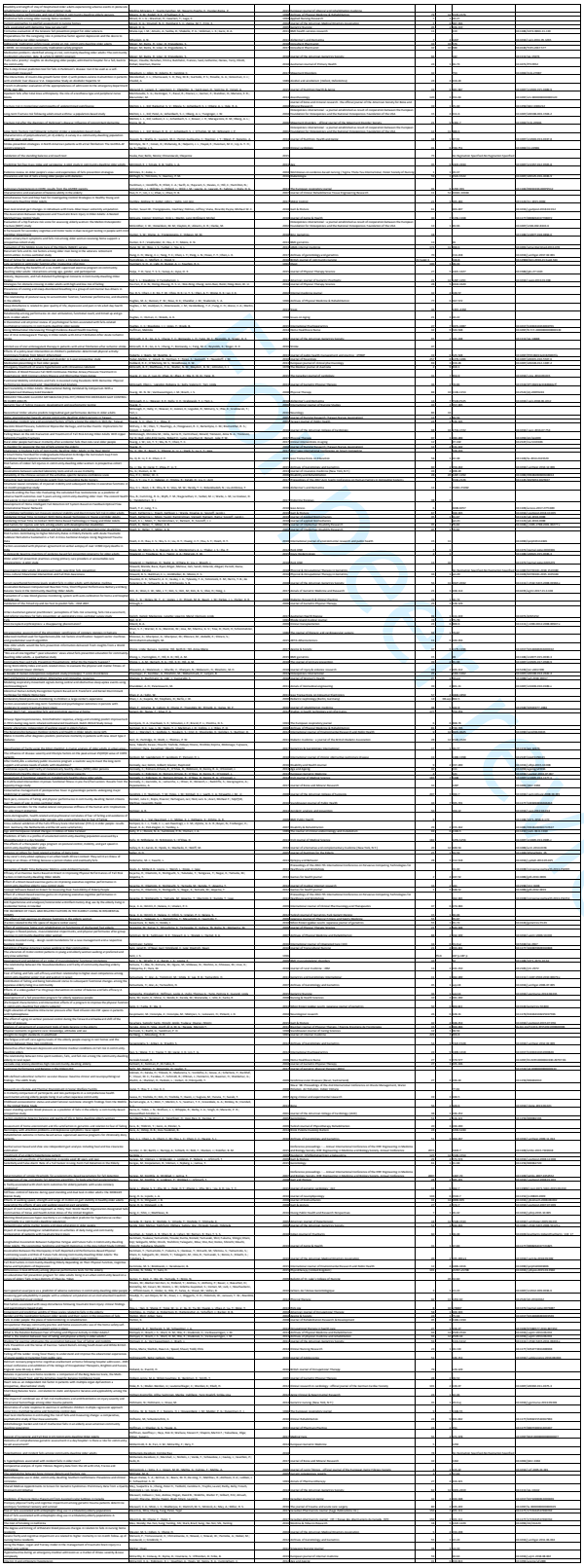

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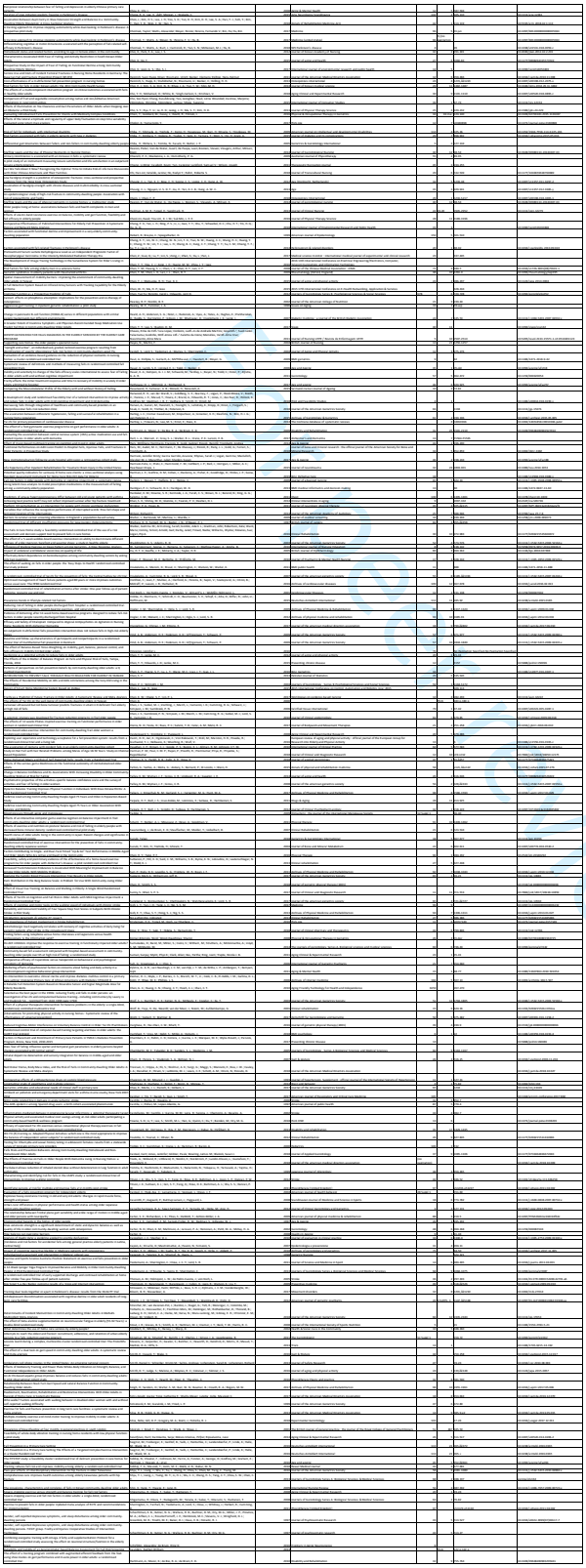

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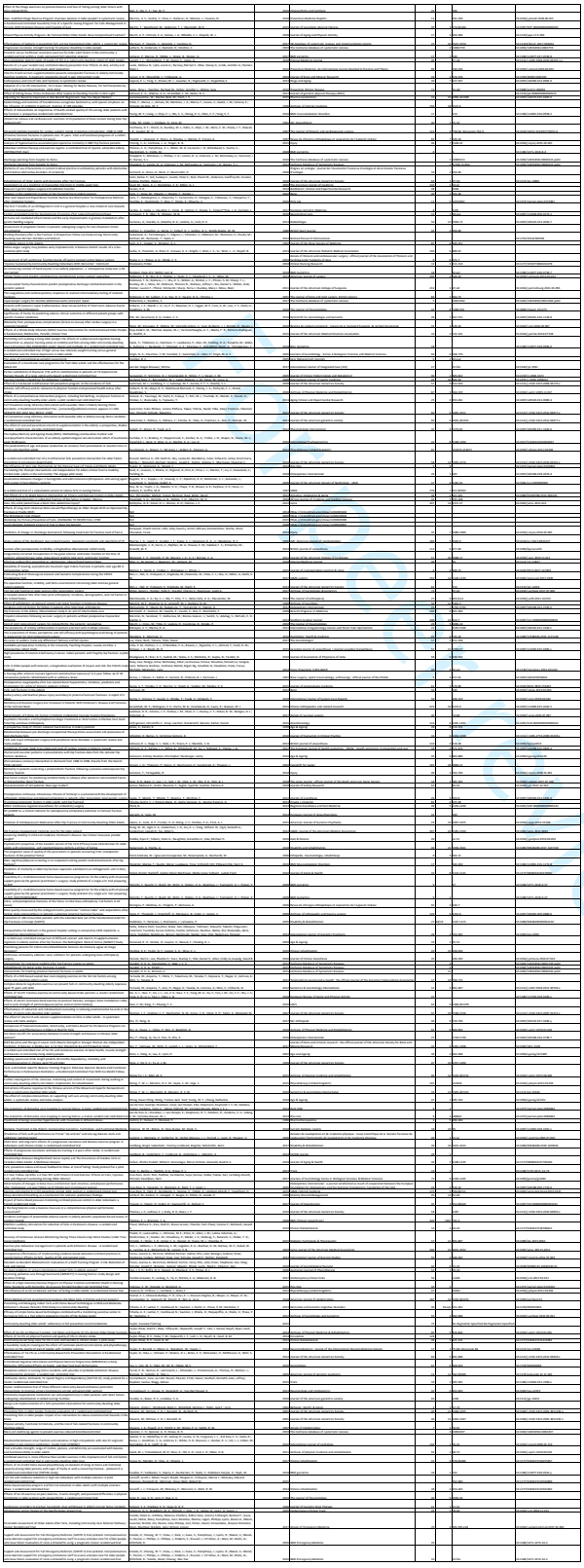

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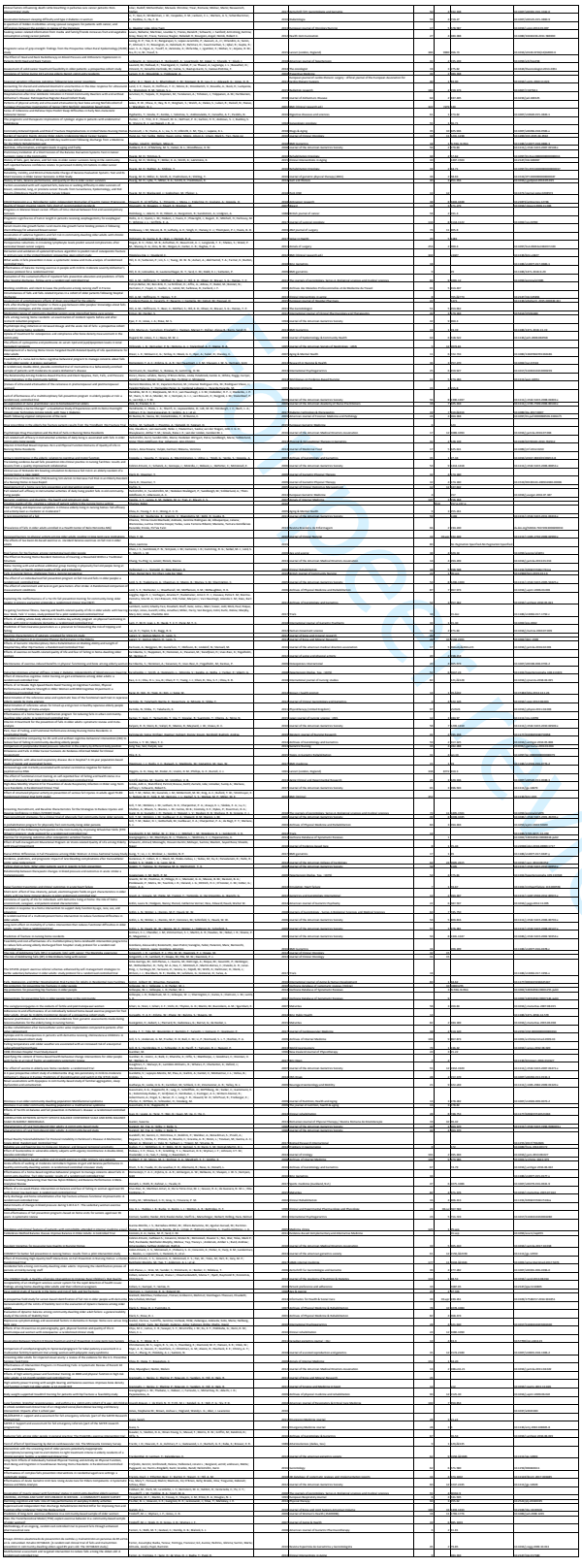

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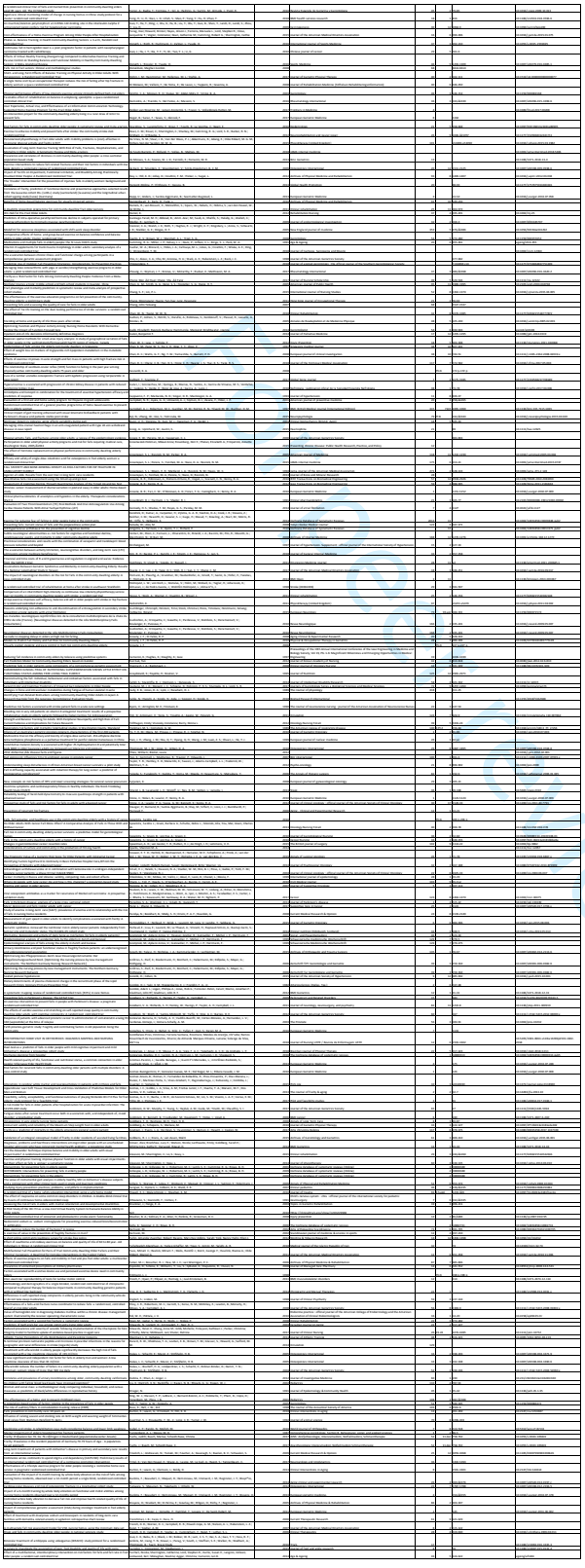

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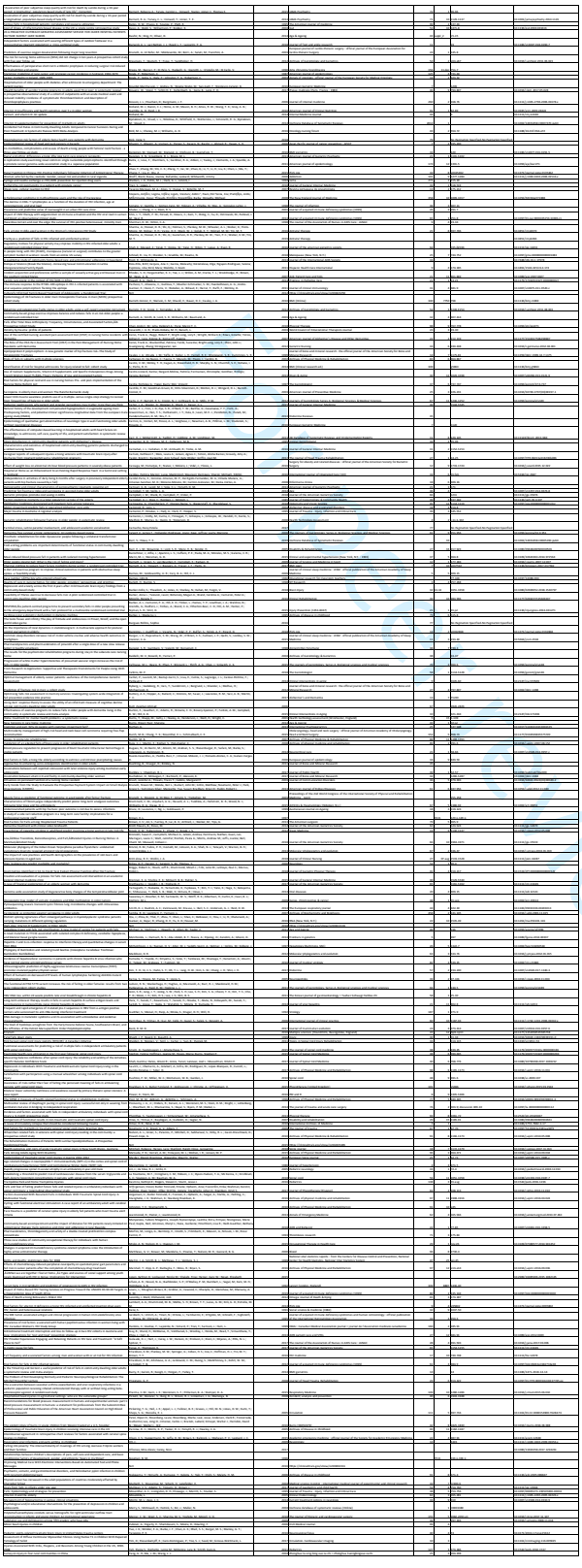

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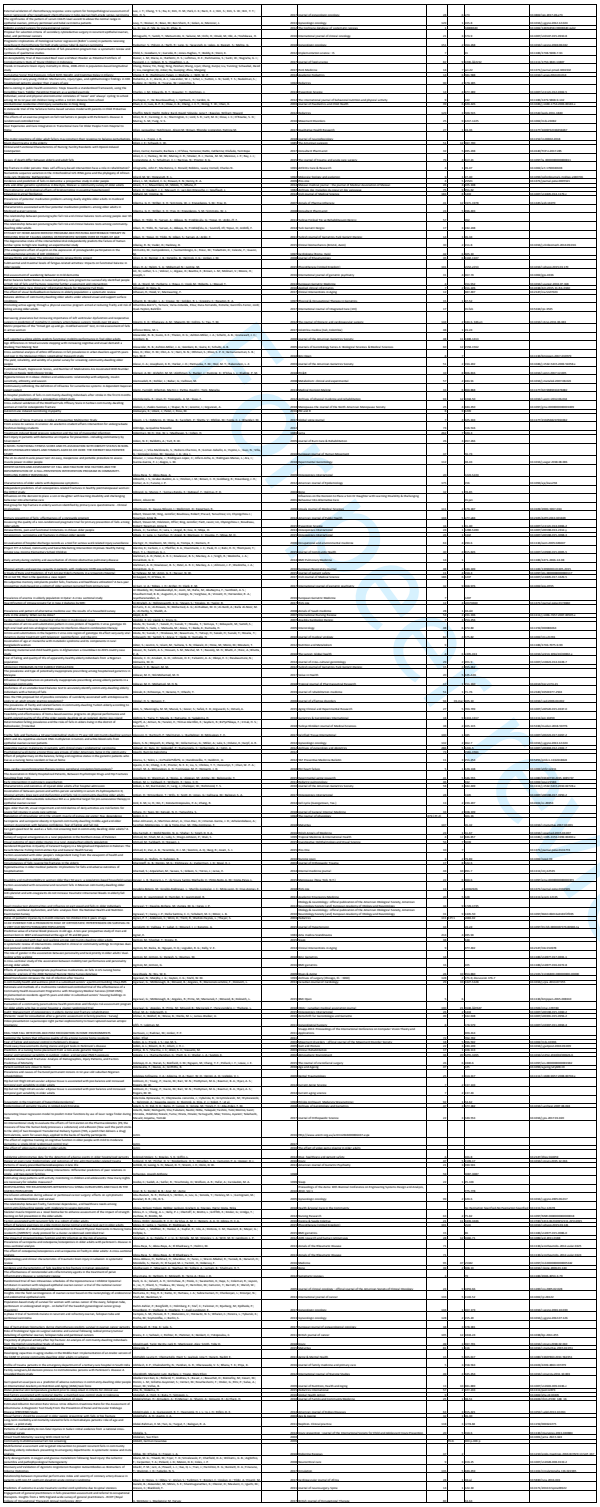

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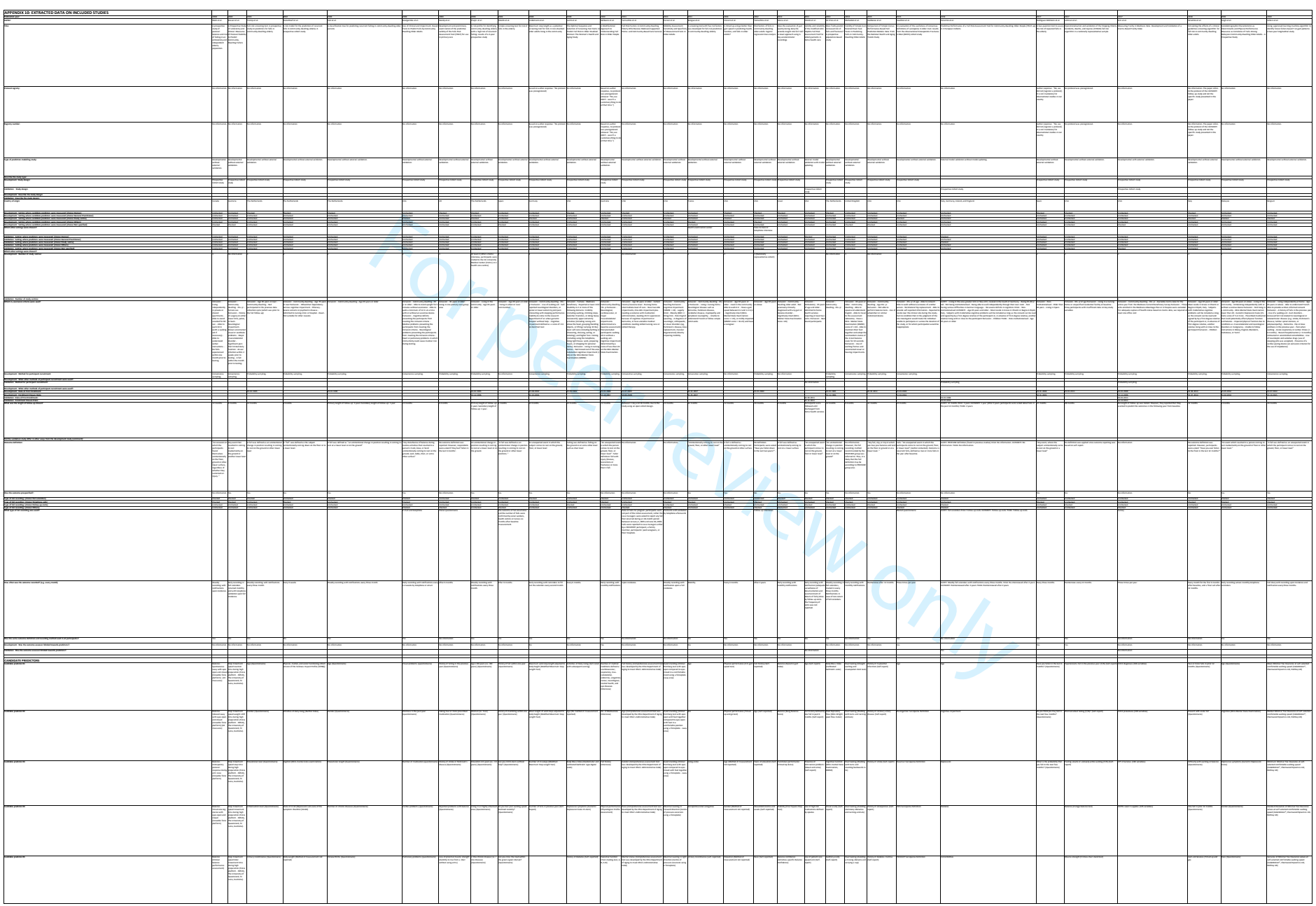
APPENDIX 8: REFERENCES EXCLUDED DURING FULL-TEXT READING:

Title	Authors	Published Year	Journal	Volume	Issue
Accuracy of clinical-functional tools to identify risk of falls among community-dwelling older adults	Sampaio, Raphaela Xavier. Santos Abreu, Amanda Maria. Almeida Nagata, Cristiane. Acevedo Garcia, Patricia	2019	Fisioterapia em Movimento	32	1
Do voluntary step reactions in dual task conditions have an added value over single task for fall prediction? A prospective study	Melzer, Itshaki; Kurz, Ilan; Shahar, Dan(; Oddsson, Lars I. E	2010	Aging Clinical & Experimental Research	22	05 jun
Use of the BESTest and the Mini-BESTest for Fall Risk Prediction in Community-Dwelling Older Adults Between 60 and 102 Years of Age	Magnani, P. E.; Genovez, M. B.; Porto, J. M.; Zanellato, N. F. G.; Alvarenga, I. C.; Freire, R. C., Jr.; de Abreu, D. C. C.	2019	J Geriatr Phys Ther		
Psychometric comparisons of the timed up and go, one-leg stand, functional reach, and Tinetti balance measures in community-dwelling older people	Lin, M.; Hwang, H.; Hu, M.; Wu, H. I.; Wang, Y.; Huang, F.	2004	Journal of the American Geriatrics Society	52	8
Comparison of 2 Frailty Indexes for Prediction of Falls, Disability, Fractures, and Death in Older Women	Ensrud, Kristine E; Ewing, Susan K.; Taylor, Brent C.; Fink, Howard A.; Cawthon, Peggy M.; Stone, Katie L.; Hillier, Teresa A.; Cauley, Jane A.; Hoc	2008	Archives of Internal Medicine	168	4
A comparison of frailty indexes for the prediction of falls, disability, fractures, and mortality in older men	Ensrud, Kristine E; Ewing, Susan K; Cawthon, Peggy M; Fink, Howard A; Taylor, Brent C; Cauley, Jane A; Dam, Thuy-Tien; Marshall, Lynn M; Orwo	2008	Journal of the American Geriatrics Society	57	3
Predicting falls in community dwelling older adults using the Activities-specific Balance Confidence Scale	Cleary, Kimberly; Skomsvkov, Elena	2017	Archives of Gerontology & Geriatrics	72	
Predicting falls in older adults using the four square step test	Cleary, K.; Skomsvkov, E.	2017	Physiotherapy Theory and practice	33	10
Fusion of Clinical, Self-Reported, and Multisensor Data for Predicting Falls	Silva, J. R.; Sousa, I.; Cardoso, J. S.	2019	IEEE J Biomed Health Inform		
Using Machine Learning on Home Health Care Assessments to Predict Fall Risk...The 17th World Congress of Medical and Health Informatics, 25-30 August 2019, Lyon, France	Yancy, Lo; Lynch, Selah F.; Urbanowicz, Ryan J.; Olson, Randal S.; Ritter, Ashley Z.; Whitehouse, Christina R.; O'Connor, Melissa; Keim, Susan K.	2019	Studies in Health Technology & Informatics	264	
Agreement and predictive power of six fall risk assessment methods in community-dwelling older adults	Menezes, M.; de Mello Mariel Filho, N. A.; Aquje, C. S.; Lemos, T.; Ferreira, A. S.	2019	Braz J Phys Ther	97	
The Clinical Potential of Frailty Indicators on Identifying Recurrent Fallers in the Community: The Mr. Os and Ms. OS Cohort Study in Hong Kong	Lam, F. M. H.; Leung, J. C. S.; Kwok, T. C. Y.	2019	J Am Med Dir Assoc	20	10
Development and validation of an algorithm to assess risk of first-time falling among home care clients	Kuspinar, Ayse; Hirdes, John P.; Berg, Katherine; McArthur, Caitlin; Morris, John N.	2019	BMC Geriatrics	19	1
A frailty profile for use in the community: Inclusion of psychological variables improves prediction of care needs and falls	Holland, C.; Garner, I.; O'Donnell, J.; Gwyther, H.	2019	Age and Ageing	48	
A model of falls risk in older adults	Genna, Gregory Joseph	2019	ProQuest (Dissertation)	80	
A decision model to predict the risk of the first fall onset	Deschamps, T.; Le Goff, C. G.; Benut, G.; Cornu, C.; Mignardot, J. B.	2019	Experimental gerontology	121	
How steady is the STEADI? Inferential analysis of the CDC fall risk toolkit	Nithman, R. W.; Vincenzo, J. L.	2019	Arch Gerontol Geriatr	83	
The Aachen Falls Prevention Scale: Multi-Study Evaluation and Comparison	Raeche, P.; Nitsch, V.; Rantaniemiet, L.; Coburn, M.; Buecking, B.; Bliemel, C.; Bollheimer, L. C.; Pajna, H. C.; Knobe, M.	2019	JMIR Aging	2	1
Accuracy of modified 30-s chair-stand test for predicting falls in older adults	Roongpisuthipong, C.; Siriphorn, A.	2019	Ann Phys Rehabil Med		
When will my patient fall? Sensor-based in-home walking speed identifies future falls in older adults	Pieu, A.; Mathek, N.; Crissey, R.; Beattie, Z.; Dodge, H.; Kaye, J.	2019	J Gerontol A Biol Sci Med Sci		
Reliability, validity, and clinical utility of a self-reported screening tool in the prediction of fall incidence in older adults	Lai, F. H. Y.; Yan, E. W. H.; Mackenzie, L.; Fong, K. N. K.; Kranz, G. S.; Ho, E. C. W.; Fan, S. H. U.; Lee, A. T. K.	2019	Disabil Rehabil		
Predicting incident falls: Relationship between postural sway and limits of stability in older adults	Johansson, J.; Jaencka, E.; Westling, G.; Nordstrom, A.; Nordstrom, P.	2019	Hum Mov Sci	66	
Automatic measurement of fall risk indicators in timed up and go test	Dubois, A.; Bihl, T.; Bresciani, J. P.	2019	Informatics for Health & Social Care	44	3
Prevalence, Severity, and Risk of Future Falls in Community-Dwelling Older Adults Living in a Rural Community: The Alshuaiba Project	Del Brutto, O. H.; Mera, R. M.; Peñado, C. D.; Sedler, M. J.	2019	Journal of Community Health	44	3
Assessment of Balance and Risk for Falls in a Sample of Community-Dwelling Adults Aged 65 and Older	Chojar, Harshita; Kaur, Gurpreet	2019	Indian Journal of Physiotherapy & Occupational Therapy	13	2
Reliability and construct validity of the stepping-forward affordance perception test for fall risk assessment in community-dwelling older adults	Almeida, G., Bravo, J.; Folgado, H.; Rosado, H.; Mendes, F.; Pereira, C.	2018	PLoS ONE	14	11
The predictive value of gait speed and maximum walking speed for falls in community-dwelling older persons	Bongers, K. T. J.; Schoon, Y.; Dronkers, M. J.; Schers, H. J.; Meis, R. J.; Rikkert, Mgmo	2015	Age and Ageing	44	2
A Simple Algorithm to Predict Falls in Primary Care Patients Aged 65 to 74 Years: The International Mobility in Aging Study	Gomez, F.; Wu, Y. Y.; Ayala, M.; Vallee, A.; Zunzunegui, M. V.	2017	Journal of the American Medical Directors Association	18	9
Evaluation of falls risk in community-dwelling older adults using body-worn sensors	Greene, B. R.; Doheny, E. P.; Walsh, C.; Cunningham, C.; Crosby, L.; Kenny, R. A.	2012	Gerontology	58	5
Activities-specific balance confidence scale for predicting future falls in Indian older adults	Moiz, J. A.; Bansal, N.; Nouha, M. M.; Gaur, S. N.; Hussain, M. E.; Anwar, S.; Alghadir, A.	2017	Clinical Interventions in Aging	12	
Gait disturbances as specific predictive markers of the first fall onset in elderly people: A two-year prospective observational study	Mignardot, J. B.; Deschamps, T.; Barney, E.; Auvinet, B.; Benut, G.; Cornu, C.; Constans, T.; De Decker, L.	2014	Frontiers in Aging Neuroscience	6	FEB
Proposed Prediction of Falls in Community-Dwelling Older Adults Through Phase-Dependent Entropy of Daily-Life Walking	Ihlen, E. A. F.; van Schooten, K. S.; Bruijn, S. M.; van Dieen, J. H.; Vereijken, B.; Helbostad, J. L.; Pijnappels, M.	2018	Frontiers in Aging Neuroscience	10	
The harmonic ratio of trunk acceleration predicts falling among older people: results of a 1-year prospective study	Doi, T.; Hirata, S.; Ono, R.; Tsutsumimoto, K.; Misu, S.; Ando, H.	2013	Journal of neuroengineering and rehabilitation	10	
History and Mobility Exam Index to Identify Community-Dwelling Elderly Persons at Risk of Falling	Covinsky, Kenneth E.; Kahana, Eva; Kahana, Boaz; Kercher, Kyle; Schumacher, John G.; Justice, Amy C.	2001	Journals of Gerontology Series A: Biological Sciences & Medical Sciences	56	4
Functional gait assessment: concurrent, discriminative, and predictive validity in community-dwelling older adults	Wrisley, D. M.; Kumar, N. A.	2010	Physical Therapy	90	5
Development of a simple scoring tool in the primary care setting for prediction of recurrent falls in men and women aged 65 years and over living in the community	Woo, J.; Leung, J.; Wong, S.; Kwok, T.; Lee, J.; Lynn, H.	2008	Journal of Clinical Nursing	18	7
Performance and impairment-based assessments among community dwelling elderly: sensitivity and specificity	Trueblood, P. R.; Hodson-Chennault, N.; McCubbin, A.; Youngclarke, D.	2001	Issues on Aging	24	1
Predictors for falls and fractures in the longitudinal aging study Amsterdam	Temp, A. M.; Smit, J. H.; Deeg, D. J. H.; Bouter, L. M.; Lips, P.	1998	Journal of Bone and Mineral Research	13	10
Predictive validity of the functional independence and difficulty scale in community-dwelling Japanese older adults	Saito, T.; Mitsui, N.; Watanabe, S.	2017	Journal of physical therapy science	29	5
Fall Risk Assessment Tools for Elderly Living in the Community: Can We Do Better?	Palumbo, P.; Palmerini, L.; Bandinelli, S.; Chiari, L.	2015	PloS one	10	12
Mobility, balance and frailty in community-dwelling older adults: at is the best 1-year predictor of falls?	Mulasso, A.; Roppolo, M.; Gobbens, R. J.; Rabaglietti, E.	2017	Geriatrics & Gerontology International	17	10
Validation of an accelerometer-based fall prediction model.	Liu, Y.; Redmond, S. J.; Sharry, T.; Woodgar, J.; Narayanan, M. R.; Lord, S. R.; Lovell, N. H.	2014	Conference proceedings : ... Annual International Conference of the IEEE Engineering in Medicine and Biolo	2014	
FRAT-up, a Web-based fall-risk assessment tool for elderly people living in the community	Cattelani, L.; Palumbo, P.; Palmerini, L.; Bandinelli, S.; Becker, C.; Chesani, F.; Chiari, L.	2017	Journal of medical Internet research	17	2
Depression and falls among community dwelling elderly people: a search for common risk factors	Bilderman, A.; Cwilkel, J.; Fried, A. V.; Galinsky, D.	2002	Journal of Epidemiology & Community Health	56	9
Local dynamic stability during gait for predicting falls in elderly people: A one-year prospective study	Bizovska, L.; Svoboda, Z.; Janura, M.; Bisi, M. C.; Vuillerme, N.	2018	Plos One	13	5
Frailty in older adults: evidence for a phenotype	Fried, L. P.; Tangen, C. M.; Walston, J.; Newman, A. B.; Hirsch, C.; Gottdiener, J.; Seeman, T.; Tracy, R.; Kop, W. J.; Burke, G.; McBurnie, M. A.	2001	Journals of Gerontology Series A: Biological Sciences & Medical Sciences	56	3
Twenty-one-item Fall Risk Index predicts falls in elderly community-dwelling Japanese	Wada, T.; Ishimoto, Y.; Hirosaki, M.; Konno, A.; Kasahara, Y.; Kimura, Y.; Nakatsuka, H.; Sakamoto, R.; Ishina, M.; Okumiya, K.; Fujisawa, M.; Ohe	2008	Journal of the American Geriatrics Society	57	12
Screening older adults at risk of falling with the Tinetti balance scale	Raiche, M.; Hebert, R.; Prince, F.; Corriveau, H.	2000	Lancet	356	9234
Quantitative gait assessment as a predictor of prospective and retrospective falls in community-dwelling older women	Fahrner, M. E.; Macrae, P. G.; McNitt-Gray, J. L.	1994	Archives of Physical Medicine & Rehabilitation	75	4
A modified fall risk assessment tool that is specific to physical function predicts falls in community-dwelling elderly people	Hirase, T.; Inokuchi, S.; Matsusaka, N.; Nakahara, K.; Okita, M.	2014	Journal of geriatric physical therapy (2001)	37	4
Frailty as a risk factor for predicting falls and fractures in community-dwelling older women	Berlooch, D.; Malmgren, L.; Bladbrine, D.; McGuigan, F.; Akesson, K.	2018	Osteoporosis International	29	1
A Quantitative Frailty Index Predicts Falls and Fractures in 75y Old Community-Dwelling Women - A 10 Year Longitudinal Study	Akesson, K.; Bartosch, P.; Malmgren, L.; Buchebner, D.; Fiona, E.	2017	Journal of Bone and Mineral Research	33	
Cognitive dual-task Timed-Up-and-Go test as a predictor of falls in the community-dwelling elderly	Tong, Y.; Tan, X.; Wang, Y.; Han, J.; Waddington, G.; Adams, R.	2018	Journal of Science and Medicine in Sport	21	
Validation of a fall-risk screening test, the Elderly Fall Screening Test (EFST), for community-dwelling elderly	Cwilkel, J. G.; Fried, A. V.; Bilderman, A.; Galinsky, D.	1998	Disability & Rehabilitation	20	9
Predictive validity and cut-off scores in four diagnostic tests for falls--A study in frail older people at home	U. Olsson Möller, J. Kristensson, P. Midlöv, C. Ekdahl & U. Jakobsson	2012	Physical & Occupational Therapy In Geriatrics	30	
Two-Year Follow-Up of Fall Prediction Among Older Adults in an Independent-Living Community	Zur, O.; Berner, Y.; Oral, Y.; Carmeli, E.	2018	Advances in experimental medicine and biology	1046	
Morse fall scale predicts falls in home-bound frail elderly population	Millard, J.; Thomas, G. M.	2011	Journal of the American Geriatrics Society	59	
Predictive performance of four frailty measures in an older Australian population	Widagdo, Imsiira S.; Pratt, Nicole; Russell, Mary; Roughead, Elizabeth E.	2015	Age & Ageing	44	6
Daily-life gait quality as predictor of falls in older people: A 1-year prospective cohort study	Van Schooten, K. S.; Pijnappels, M.; Rispens, S. M.; Elders, P. J. M.; Lips, P.; Daffertshofer, A.; Beek, P. J.; Van Dieën, J. H.	2016	PLoS ONE	11	7
The falls risk assessment test (That-FRAT) developed for community-dwelling thai elderly	Thiemwong, L.; Thamarpirat, J.; Maneesriwongul, W.; Jitapunkul, S.	2008	Journal of the Medical Association of Thailand	91	10
60 THE PREDICTIVE VALUE OF THE EASYCARE-TOS INSTRUMENT ON FALLING IN COMMUNITY-DWELLING ELDERLY. ,British Geriatrics Society Communications to the Spring Meeting 23 – 25 A	Schoon, Y.; Post, P. I. J.; Bongers, K. T. J.; Olde Rikkert, M. G. M.	2004	Age & Ageing	43	suppl_2
Risk factors for falling in older Mexican Americans	Reyes-Ortiz, C. A.; Al Snih, S.; Loera, J.; Ray, L. A.; Markides, K.	2004	Ethnicity & disease	14	3
Predictive Value of Frailty Indices for Adverse Outcomes in Older Adults	Perez-Zepeda, M. U.; Cesari, M.; Garcia-Pena, C.	2016	Revista de investigacion clinica; organo del Hospital de Enfermedades de la Nutricion	68	2
Application of a fall screening algorithm stratified fall risk but missed preventive opportunities in community-dwelling older adults: a prospective study	Muir, Susan W.; Berg, Katherine; Chesworth, Bert; Klar, Neil; Speechley, Mark	2010	Journal of Geriatric Physical Therapy	33	4
Modifiable risk factors identify people who transition from non-fallers to fallers in community-dwelling older adults: a prospective study	Muir, S. W.; Berg, K.; Chesworth, B. M.; Klar, N.; Speechley, M.	2010	Physiotherapy Canada	62	4
Strength measures are better than muscle mass measures in predicting health-related outcomes in older people: time to abandon the term sarcopenia?	Menant, J. C.; Weber, F.; Lo, J.; Sturnieks, D. L.; Close, J. C.; Sachdev, P. S.; Brodaty, H.; Lord, S. R.	2017	Osteoporosis International	28	1
Does the short physical performance battery (SPPB) predict falls in older community-dwelling people?	Lundin-Olsson, L.; Nordin, E.	2011	Physiotherapy (United Kingdom)	97	
Fall risk in an active elderly population - Can it be assessed?	Jessica, U.; Hoos, H. C.; Simonsen, O.; Sinkjaer, T.; Voigt, M.	2007	Journal of Negative Results in BioMedicine	6	
Validity of a new fall risk screen	Killough, J.	2005	Journal of Geriatric Physical Therapy	28	3
Validation and comparison of two frailty indexes: the MOBILIZE Boston Study	Kiely, D. K.; Cupples, L. A.; Lipsitz, L. A.	2009	Journal of the American Geriatrics Society	57	9
Predicting Value of Nine-Item Berg Balance Scale Among the Aged: A 3-Year Prospective Follow-Up Study	Hohtari-Kivimäki, U.; Salminen, M.; Vahlberg, T.; Kivelä, S. L.	2018	Experimental aging research	42	2
Falls in the elderly: A prospective study of risk factors and risk profiles	Graafmans, W. C.; Ooms, M. E.; Hofstee, H. M. W.; Bezemer, P. D.; Bouter, L. M.; Lips, P.	1996	American Journal of Epidemiology	143	11
Clinical prediction based on biomechanical parameters and adverse outcomes in community-dwelling older people: a 12-month study	Gardner, P.; Ringsberg, K. A.; Akesson, K.; Obrant, K. J.	2000	Journal of clinical epidemiology	53	3
Encore presentation predicting falls in a senior living community using balance and lower extremity muscle mass	Gaines, J.; Marx, K. A.; Burke, K. L.; Parish, J. M.	2011	Journal of the American Geriatrics Society	59	
Kinect-based choice reaching and stepping reaction time tests for clinical and in-home assessment of fall risk in older people: a prospective study	Gujski, A.; Gschwind, Y. J.; Brodie, M.; Ziegler, W. L.; Lord, S. R.; Delbaere, K.	2016	European Review of Aging and Physical Activity	13	
The Physical Performance Test as a predictor of frequent fallers: a prospective community-based cohort study	Delbaere, K.; Van den Noortgate, N.; Bourgois, J.; Vanderstraeten, G.; Tina, W.; Cambier, D.	2005	Clinical Rehabilitation	20	1
Fear-related avoidance of activities, falls and physical frailty. A prospective community-based cohort study	Delbaere, K.; Crombez, G.; Vanderstraeten, G.; Willems, T.; Cambier, D.	2004	Age & Ageing	33	4
Reliability and validity of a steadiness score	Clark, D. O.; Callahan, M.; Counsell, S. R.	2005	Journal of the American Geriatrics Society	53	9
Comparison of Fallers and Nonfallers on Four Physical Performance Tests: A Prospective Cohort Study of Community-Dwelling Older Indigenous Taiwanese Women	Chan, J. C.; Liang, C. C.; Chang, Q. X.	2018	International Journal of Gerontology	12	1
Special feature: the Home Assessment Profile -- a reliable and valid assessment tool	Chandler, J. M.; Duncan, P. W.; Weiner, D. K.; Studenski, S. A.	2001	Topics in Geriatric Rehabilitation	16	3
Reactive stepping behaviour in response to forward loss of balance predicts future falls in community-dwelling older adults	Carty, C. P.; Cronin, N. J.; Nicholson, D.; Lichtwark, G. A.; Mills, P. M.; Kerr, G.; Cresswell, A. G.; Barrett, R. S.	2015	Age and ageing	44	1
Parkinsonian signs and incident falls in older persons without Parkinson's disease	Buracchio, Teresa; Arvanitakis, Zoe; Leurgans, Sue; Bennett, David A.	2010	Journal of the American Geriatrics Society	58	1
Use of the Berg Balance Test to predict falls in elderly persons	Bogle Thorbahn, L. D.; Newton, R. A.	1996	Physical therapy	76	6
Comparative performance of current definitions of sarcopenia against the prospective incidence of falls among community-dwelling seniors age 65 and older	Bijlsdorff-Ferrari, H. A.; Dev, J. E.; Kanis, J. A.; Rizzoli, R.; Schlogl, M.; Staehelin, H. B.; Willett, W. C.; Dawson-Hughes, B.	2015	Osteoporosis International	26	12
Self-reported walking, balance testing and risk of fall	Bergland, A.; Jarnlo, G. B.; Wyller, T. B.	2006	Tidsskrift for den Norske Laegeforening	126	2
Correlates of transient falling in independent community-dwelling older adults	Berg, W. P.; Alessio, H. M.; Mills, E. M.; Tong, C.	1997	Journal of Motor Behavior	29	3
New approach to risk determination: development of risk profile for new falls among community-dwelling older people by use of a Genetic Algorithm Neural Network (GANN)	Bath, P. A.; Pendleton, N.; Morgan, K.; Clague, J. E.; Horan, M. A.; Lucas, S. B.	2000	The journals of gerontology. Series A, Biological sciences and medical sciences	55	1
Can FRAX predict falls in older women?	Atkinson, E.; Achenbach, S.; Crenshaw, J.; Kaufman, K.; Khosla, S.; Melton, L. J.	2015	Journal of Bone and Mineral Research	30	
The role of postural instability/gait difficulty and fear of falling in predicting falls in non-demented older adults	Allali, Gilles; Ayers, Emmeline I.; Holtzer, Roee; Verghese, Joe	2017	Archives of Gerontology & Geriatrics	69	
Multiple modes of assessment of gait are better than one to predict incident falls	Allali, G.; Ayers, E. I.; Verghese, J.	2015	Archives of gerontology and geriatrics	60	3
Clinical parameters associated with falls in an elderly population	Clark, R. D.; Lord, S. R.; Webster, I. W.	1995	Gerontology	39	2
Force Platform Balance Measures as Predictors of Indoor and Outdoor Falls in Community-Dwelling Women Aged 63-76 Years	Pajala, Sirkku; Era, Pertti; Koskenvuo, Markku; Kaprio, Jaakko; Tormakangas, Timo; Rantanen, Taina	2008	Journals of Gerontology Series A: Biological Sciences & Medical Sciences	63	2
Changes in step-width during dual-task walking predicts falls	Nordin, E.; Moe-Nilssen, R.; Ramnemark, A.; Lundin-Olsson, L.	2010	Gait and Posture	32	
Clinically meaningful cutpoints of instrumented physical functioning: obability, falls, and hospitalizations in older adults	Ni, M.; Ward, R.; Beem, J.	2017	Journal of Physical Medicine and Rehabilitation	98	10
Use of the Berg Balance Scale for predicting multiple falls in community-dwelling elderly people: a prospective study	Muir, S. W.; Berg, K.; Chesworth, B.; Speechley, M.	2008	Physical Therapy	88	4
Comparison of the validity of four fall-related psychological measures in a community-based falls risk screening	Moore, D. S.; Ellis, R.; Kosma, M.; Fabre, J. M.; McCarter, K. S.; Wood, R. H.	2011	Research Quarterly for Exercise and Sport	82	3
Is it possible to predict falls in older adults using gait biomarkers?	Manjundi, Nise Ribeiro; Spinoso, Deborah Helding; Cardoso, Bruna Carvalho; Moreno, Vinicius Christianini; Kuroda, Marina Hiromi; Nawiga, Marcelo	2018	Clinical Biomechanics	59	
Risk factors for falling among community-based seniors using home care services	Fletcher, P. C.; Hirdes, J. P.	2002	Journals of Gerontology - Series A Biological Sciences and Medical Sciences	57	8
Falls and fear of falling: which comes first? A longitudinal prediction model suggests strategies for primary and secondary prevention	Friedman, S. M.; Munoz, B.; West, S. K.; Rubin, G. S.; Fried, L. P.	2002	Journal of the American Geriatrics Society	50	8
Development of a new fall risk assessment index for older adults	Yamada, M.; Aoyama, T.; Nagai, K.; Tanaka, B.; Uehara, T.; Aoyama, T.	2010	International Journal of Gerontology	4	3
Can the Ordered Multi-Stepping over Hoop test be useful for predicting fallers among older people? A preliminary 1 year cohort study	Taulsumimoto, K.; Doi, T.; Misu, S.; Ono, R.; Hirata, S.	2013	Aging Clinical and Experimental Research	25	4
'Fall risk index' helps clinicians identify high-risk individuals	Toba, K.; Kikuchi, R.; Iwata, A.; Kozaki, K.	2009	Japan Medical Association Journal	52	4
Risk assessment for falls in the elderly population	Tobis, E.	2008	Geriatrics & Gerontology International	8	
The comparative ability of eight functional mobility tests for predicting falls in community-dwelling older people	Tiedemann, A.; Shimada, H.; Sherrington, C.; Murray, S.; Lord, S.	2008	Age & Ageing	37	4
Fall prediction based on biomechanics equilibrium using Kinect	Tao, X.; Yun, Z.	2017	International Journal of Distributed Sensor Networks	13	4
Which neuromuscular or cognitive test is the optimal screening tool to predict falls in frail community-dwelling older people?	Shimada, H.; Suzukawa, M.; Tiedemann, A.; Kobayashi, K.; Yoshida, H.; Suzuki, T.	2009	Gerontology	55	3
Multifactorial screening for fall risk in community-dwelling older adults in the primary care office: Development and validation of the Fall Risk Assessment & Screening Tool (FRAST)	Rancho, Mindy Osman	2012	ProQuest (Dissertation)	72	
Quantitative trunk sway and prediction of incident falls in older adults	Mahboobin, Arsalan; Loughlin, P.; Oh-Park, Moonyoo; Ayers, Emmeline; Verghese, Joe	2017	Gait & Posture	58	
Choice stepping reaction time: a composite measure of falls risk in older people	Lord, S. R.; Fitzpatrick, R. C.	2001	The journals of gerontology. Series A, Biological sciences and medical sciences	56	10
Visual risk factors for falls in older people	Lord, S. R.; Dayhew, J.	2001	Journal of the American Geriatrics Society	49	5
Risk factors for falling among community-dwelling seniors using home-care services: An extended hazards model with time-dependent covariates and multiple events	Leclerc, B. S.; Begin, C.; Cadieux, E.; Gouliet, L.; Leduc, N.; Kergoat, M. J.; Lebel, P.	2008	Chronic Diseases in Canada	29	4
Conceptualizing a Dynamic Fall Risk Model Including Intrinsic Risks and Exposures	Klenk, J.; Becker, C.; Palumbo, P.; Schwickert, L.; Rapp, K.; Helbostad, J. L.; Todd, C.; Lord, S. R.; Kerse, N.	2012	J Am Med Dir Assoc	18	11
Postural stiffness, distractions, and prospective falls: A multiple-boston study	Kang, H. G.; Quach, L.; Li, W.; Leipzig,	2010	Movement Disorders and Related Disorders		
Risk factors for falls in older adults in a South African Urban Community	Kalula, S. Z.; Ferreira, M.; Swingler, G. H.; Badri, M.	2016	BMC geriatrics	16	
Short Berg Balance Scale, BBS-9, as a predictor of fall risk among the aged: A prospective 12-month follow-up study	Hohtari-Kivimäki, U.; Salminen, M.; Vahlberg, T.; Kivelä, S. L.	2013	Aging Clinical and Experimental Research	25	6
Falls among healthy, community-dwelling, older women: a prospective study of frequency, circumstances, consequences and prediction accuracy	Hill, K.; Schwarz, J.; Flicker, L.; Carroll, S.	1999	Australian and New Zealand Journal of Public Health	23	1
Balance self-efficacy predicts risk factors for side falls and frequent falls in community-dwelling elderly	Gunter, K. B.; De Costa, J.; White, K. N.; Hooker, K.; Hayes, W. C.; Snow, C. M.	2003	Journal of Aging & Physical Activity	11	1
Fall Risk Assessment Through Automatic Combination of Clinical Fall Risk Factors and Body-Worn Sensor Data	Greene, B. R.; Redmond, S. J.; Caulfield, B.	2017	IEEE Journal of Biomedical and Health Informatics	21	3
Comparison of regression models for the analysis of fall risk factors in older veterans	Gill, D. P.; Zou, G. Y.; Jones, G. R.; Speechley, M.	2009	Annals of Epidemiology	19	4
A Multinomial Regression Model of Risk for Falls (RFF) Factors Among Filipino Elderly in a Community Setting	de Guzman, Allan B.; Garcia, Jan Michael G.; Garcia, Juan Paulo S. P.; Garcia, Monica B.; Geman, Rosa T.; Gerong, Martha Sabrina C.; Grajo, Alm	2015	Educational Gerontology	39	9
Falls among older adults: Characteristics of falls, predictors of falls, and the impact of falls on health care and long-term care utilization	Chui, In Hea	2009	ProQuest (Dissertation)	69	
Pilot study: Predictive measures of self-reported confidence levels and physical performance related to fall risk in community dwelling older adults	Bright, B.; Staley, J.; Trey, L.; Vrchota, S.; Watson, K.	2016	Archives of Physical Medicine and Rehabilitation	97	10
The Standardization of Frailty Phenotype Criteria Improves Its Predictive Ability: The Toledo Study for Healthy Aging	Bouzon, C. A.; Carricaro, J. A.; Turin, J. G.; Garcia-Garcia, F. J.; Estebán, A.; Rodriguez-Manas, L.	2017	Journal of the American Medical Directors Association	18	5
Differential risk factor profiles for indoor and outdoor falls in older people living at home in Nottingham, UK	Bath, P. A.; Morgan, K.	1999	European Journal of Epidemiology	15	1
An 8-year prospective study of the relationship between cognitive performance and falling in very old adults	Anstey, K. J.; von Sanden, C.; Luszcz, M. A.	2006	Journal of the American Geriatrics Society	54	8
Discriminative and Predictive Ability of Physical Performance Measures in Identifying Fall Risk among Older Adults	Abu Samah, Z.; Singh, D. K. A.; Murukesu, R. R.; Shahar, S.; Nordin, N. A. M.; Omar, M. A.; Ai-Vyrn, C.	2018	Sains Malaysiana	47	11
Performance-Oriented Assessment of Mobility Problems in Elderly Patients	Mill, Tinetti	1986	Journal of the American Geriatrics Society	34	2
Predicting the probability of falls in community-dwelling elderly individuals using the trail-walking test	Yamada, M.; Ichihashi, N.	2010	Environmental Health and Preventive Medicine	15	6
The development and validation of a brief performance-based fall risk assessment tool for use in primary care	Tiedemann, A.; Lord, S. R.; Sherrington, C.	2010	Journals of Gerontology Series A: Biological Sciences & Medical Sciences	65A	8
Falls prediction in elderly people: A 1-year prospective study	Swanenburg, J.; de Bruin, E. D.; Uebelhart, D.; Mulder, T.	2010	Gait and Posture	31	3
Predicting falls: the role of mobility and nonphysical factors.	Studenski S; Duncan PW; Chandler J; Samsa G; Prescott B.; Hogue C; Beavon LB.	1994	Journal of the American Geriatrics Society	42	3
Fall risk in Chinese community-dwelling older adults: A physiological profile assessment study	Siong, Kar-Hai Kwan, Jacinda Mun-Sen; Lord, Stephen R.; Lam, Andrew Kwok-Cheung; Tsang, William Wai-Nam; Cheong, Allen Ming-Yan	2018	Geriatrics & Gerontology International	18	2
Development and Evaluation of an Online Fall-Risk Questionnaire for Nonfrail Community-Dwelling Elderly Persons: A Pilot Study	Olviet, S.; Rogan, S.; Hillker, R.	2018	Current gerontology and geriatrics research	2018	
A novel infrared laser device that measures multilateral parameters of stepping performance for assessment of fall risk in elderly individuals [corrected]	Nishiguchi, S.; Yamada, M.; Uemura, K.; Matsumura, T.; Takahashi, M.; Moriguchi, T.; Aoyama, T.	2013	Aging clinical and experimental research	25	3
Screening for falls in community-dwelling elderly	Murphy, M. A.; Olson, S. L.; Protas, E. J.; Overby, A. R.	2003	Journal of Aging & Physical Activity	11	1
Spatiotemporal gait parameters and recurrent falls in community-dwelling elderly women: a prospective study	Morelo, Bruno S.; Sampaio, Rosana F.; Kirkwood, Renata N.	2015	Brazilian Journal of Physical Therapy / Revista Brasileira de Fisioterapia	19	1
Understanding Falls Risk Screening Practices and Potential for Electronic Health Record Data-Driven Falls Risk Identification in Select West Virginia Primary Care Centers	Mace, Adam	2018	ProQuest (Dissertation)	78	
Physiological factors associated with falls in older community-dwelling women	Lord, S. R.; Ward, J. A.; Williams, P.; Anstey, K. J.	1994	Journal of the American Geriatrics Society	42	10
A classification and regression tree for predicting recurrent falling among community-dwelling seniors using home-care services	Leclerc, B. S.; Begin, C.; Cadieux, E.; Gouliet, L.; Allaire, J. F.; Matocha, J.; Leduc, N.; Kergoat, M. J.	2009	Canadian journal of public health = Revue canadienne de sante publique	100	4
Validation of the Saskatoon Falls Prevention Consortium's Falls Screening and Referral Algorithm	Lawson, S. N.; Zaluski, N.; Petta, A.; Arnold, C.; Basran, J.; Dal Bello-Haas, V.	2013	Physiotherapy Canada. Physiotherapie Canada	65	1
Timed up and go test can predict recurrent falls: A longitudinal study of the community-dwelling elderly in China	Kang, L.; Han, P.; Wang, J.; Ma, Y.; Jia, L.; Fu, L.; Yu, H.; Chen, X.; Niu, K.; Guo, Q.	2017	Clinical Interventions in Aging	12	
A Screening Tool Using Five Risk Factors Was Developed for Fall-Risk Prediction in Chinese Community-Dwelling Elderly Individuals	Kang, L.; Chen, X. Y.; Han, P. P.; Ma, Y. X.; Jia, L. Y.; Fu, L. Y.; Yu, H. R.; Wang, L.; Hou, L.; Yu, X.; An, Z. Y.; Wang, X. T.; Li, L.; Zhang, Y. Y.; Zh	2018	Rejuvenation Research	21	5
Falling in the elderly: Do statistical models matter for performance criteria of fall prediction? Results from two large population-based studies	Kabeshova, A.; Launay, C. P.; Gromov, V. A.; Fantino, B.; Levinoff, E. J.; Allali, G.; Beauchet, O.	2016	European Journal of Internal Medicine	27	
Comparison of the Berg Balance Scale and Fullerton Advanced Balance Scale to predict falls in community-dwelling adults	Jeon, Y. J.; Kim, G. M.	2017	Journal of physical therapy science	29	3
Lateral balance factors predict future falls in community-living older adults	Hilliard, M. J.; Martinez, K. M.; Janssen, I.; Edwards, B.; Mille, M.; Zhang, Y.; Rogers, M. W.	2008	Archives of Physical Medicine & Rehabilitation	89	9
An analysis of risk assessment tools



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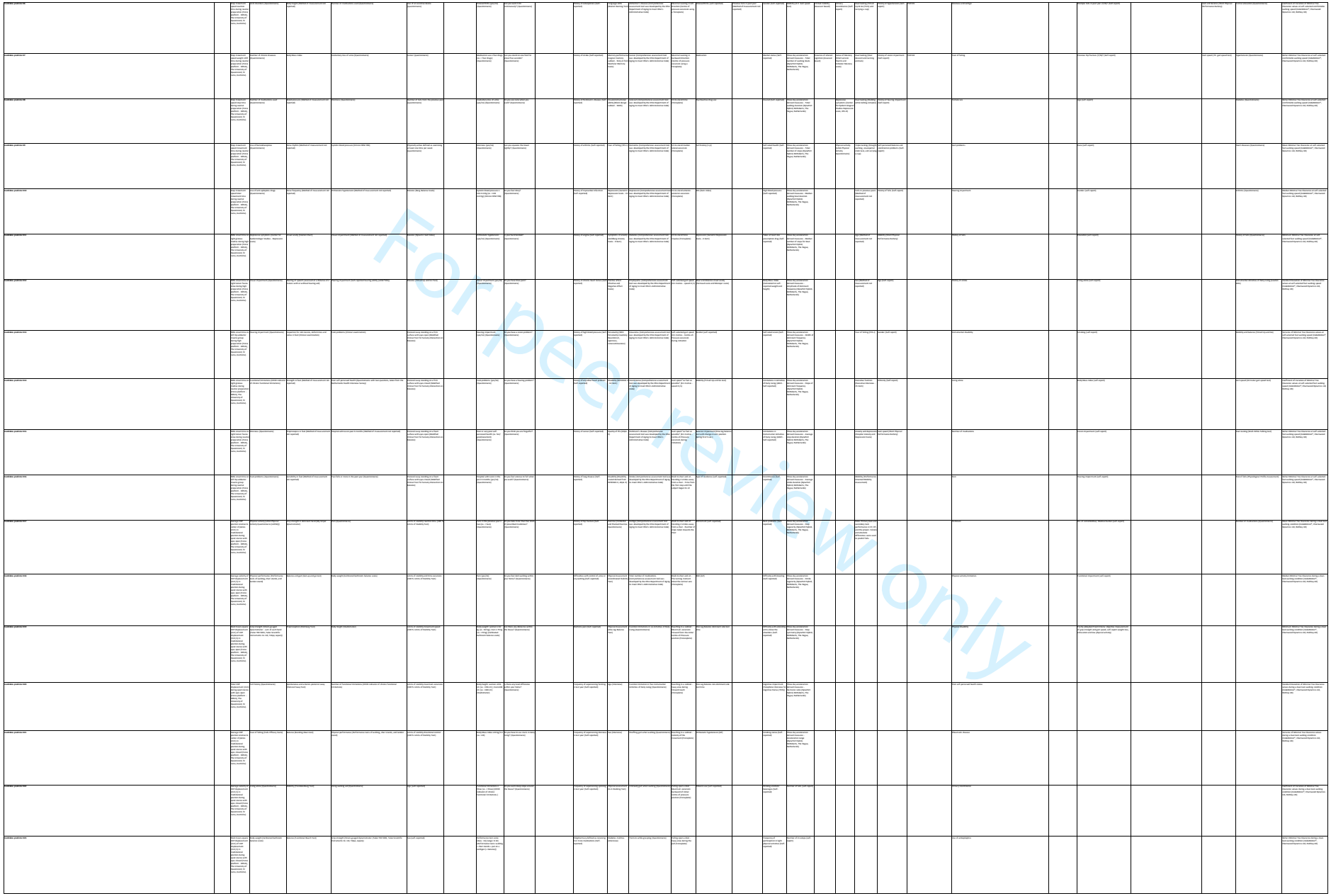

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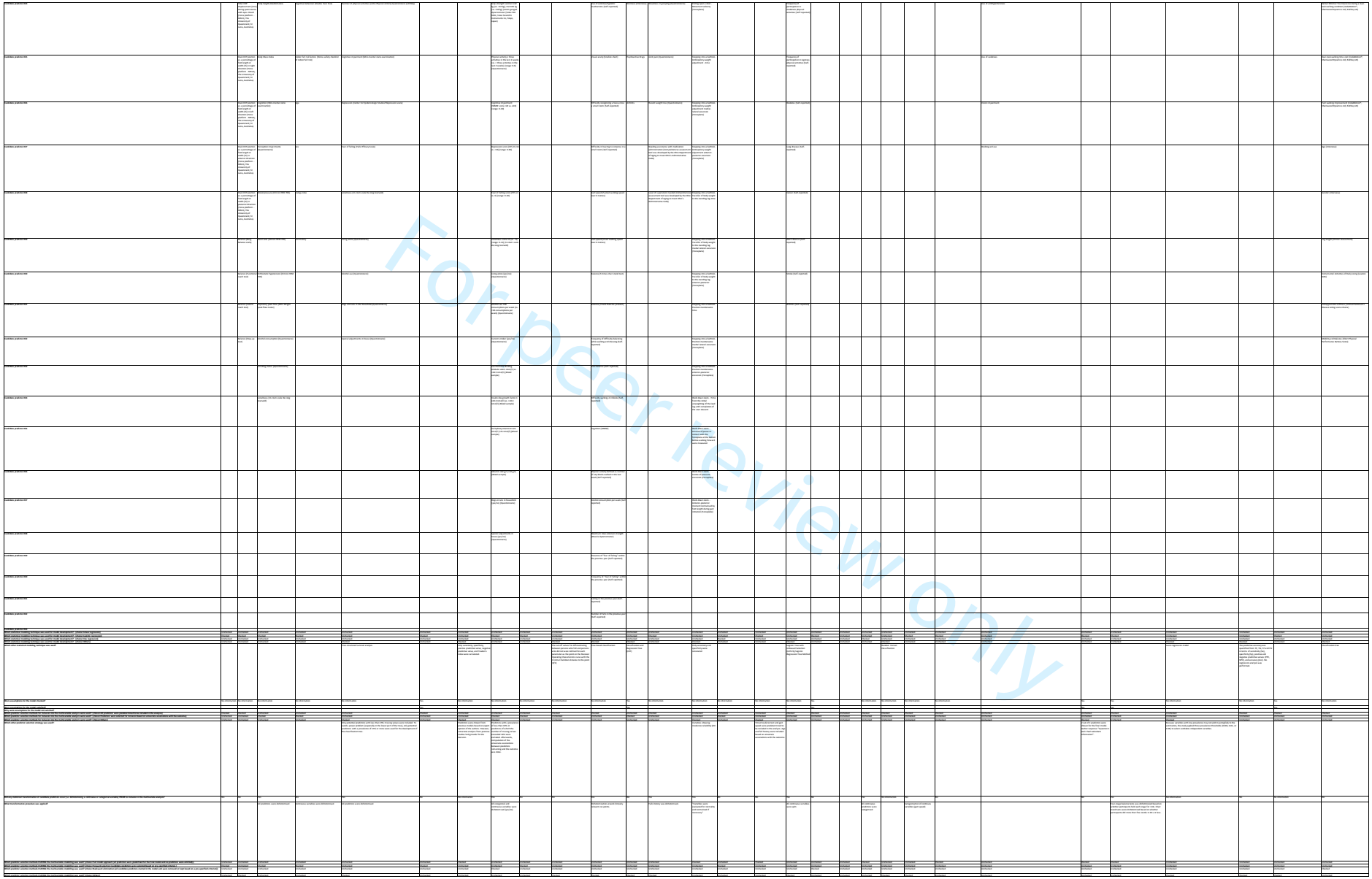

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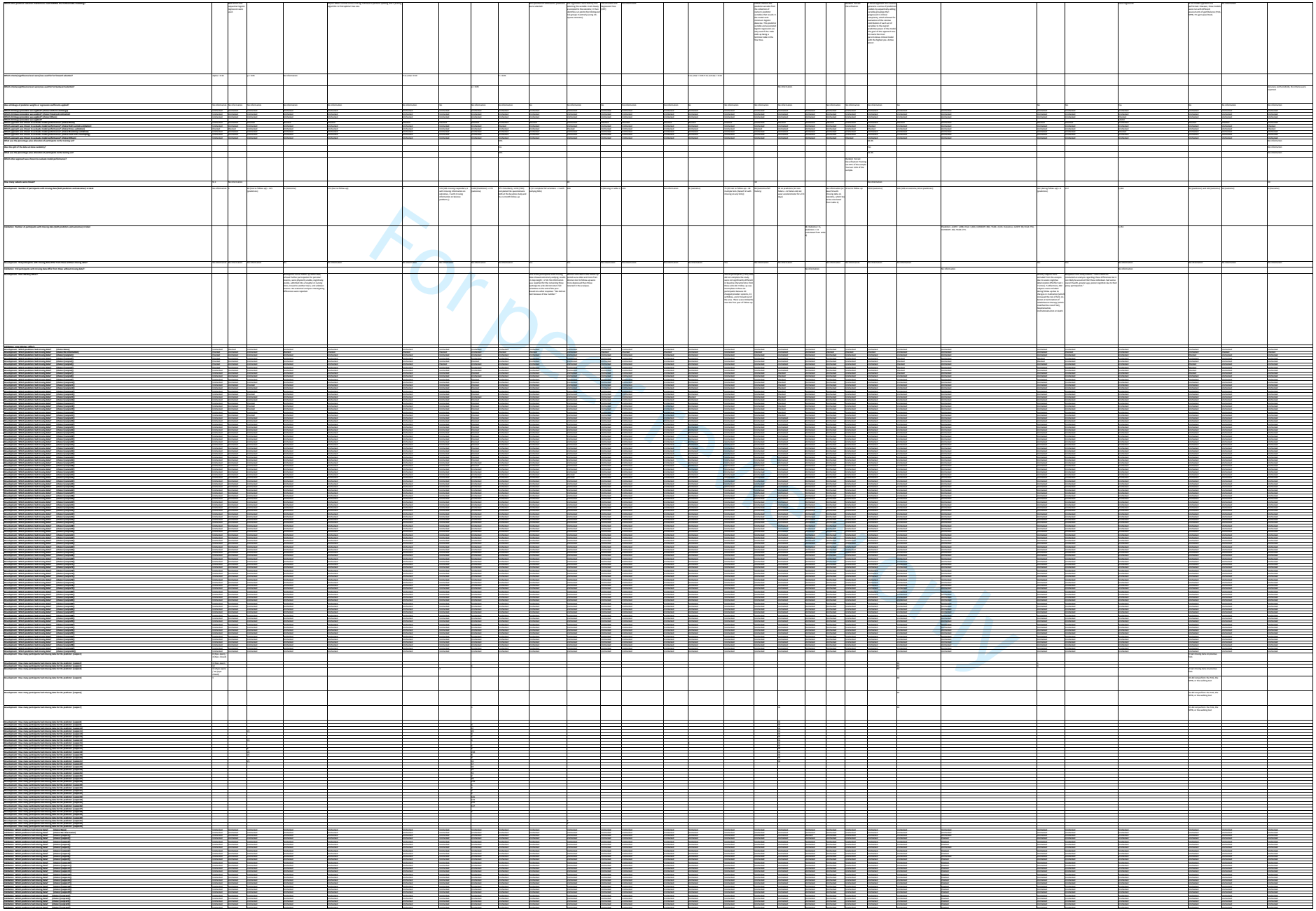

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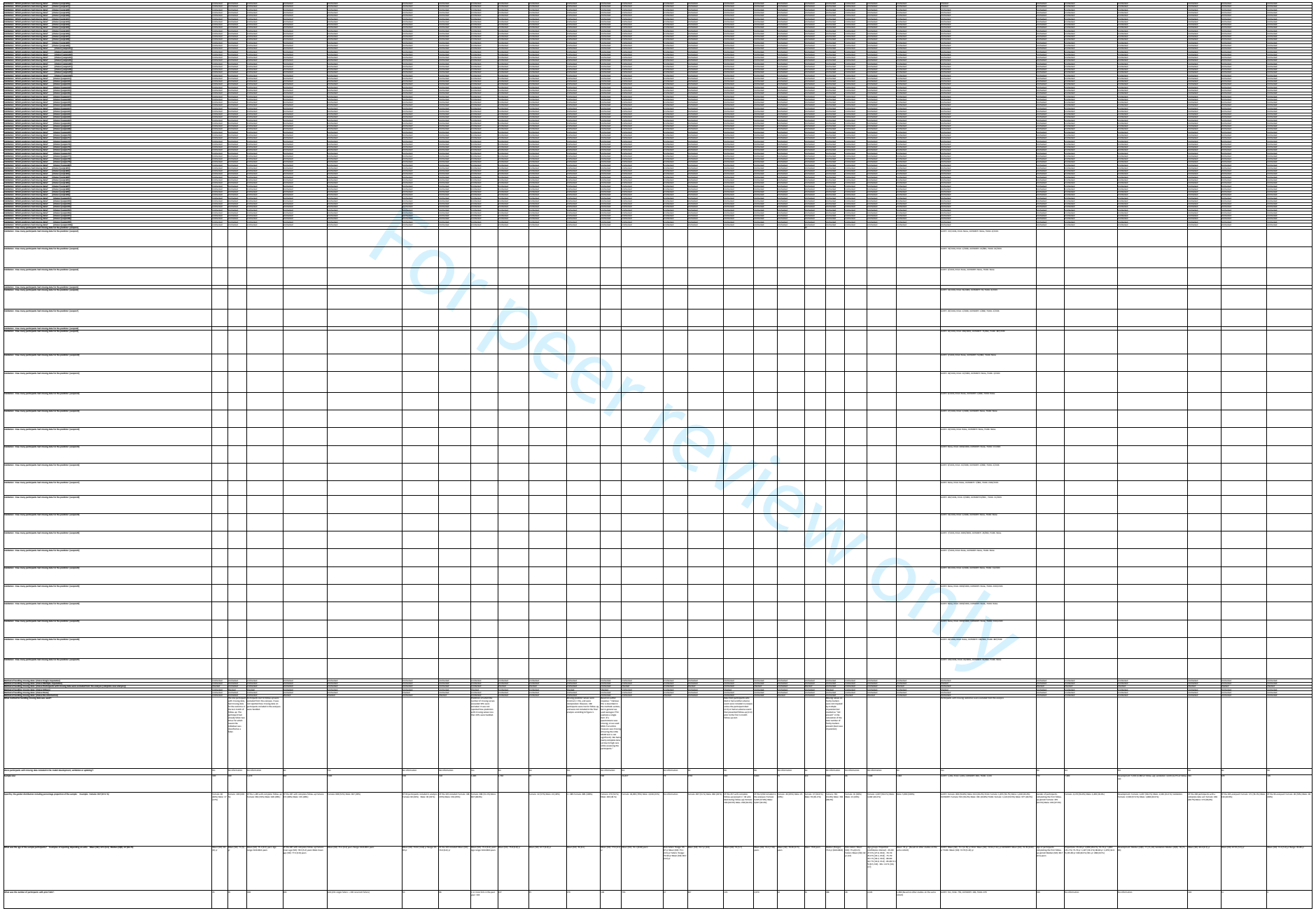

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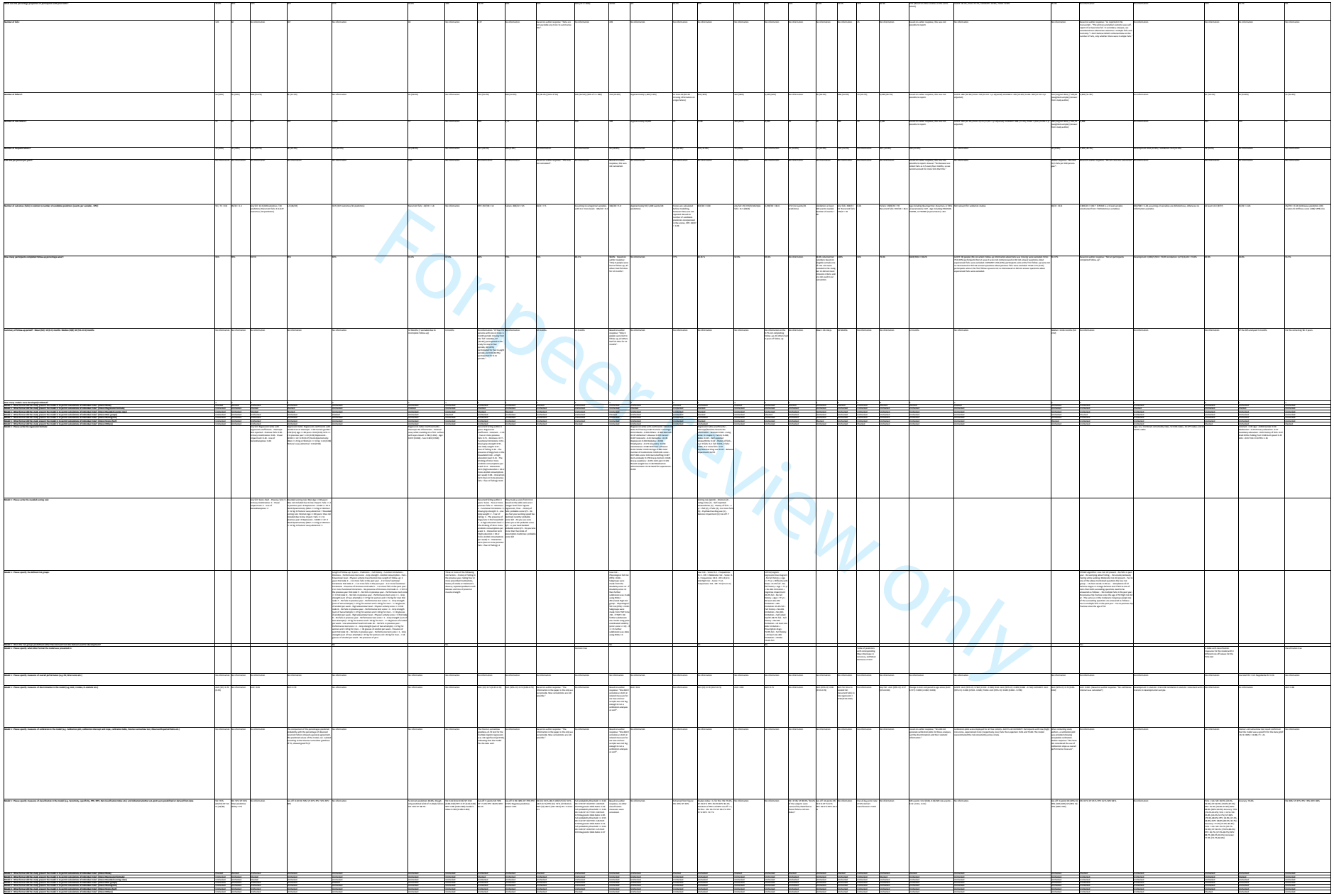

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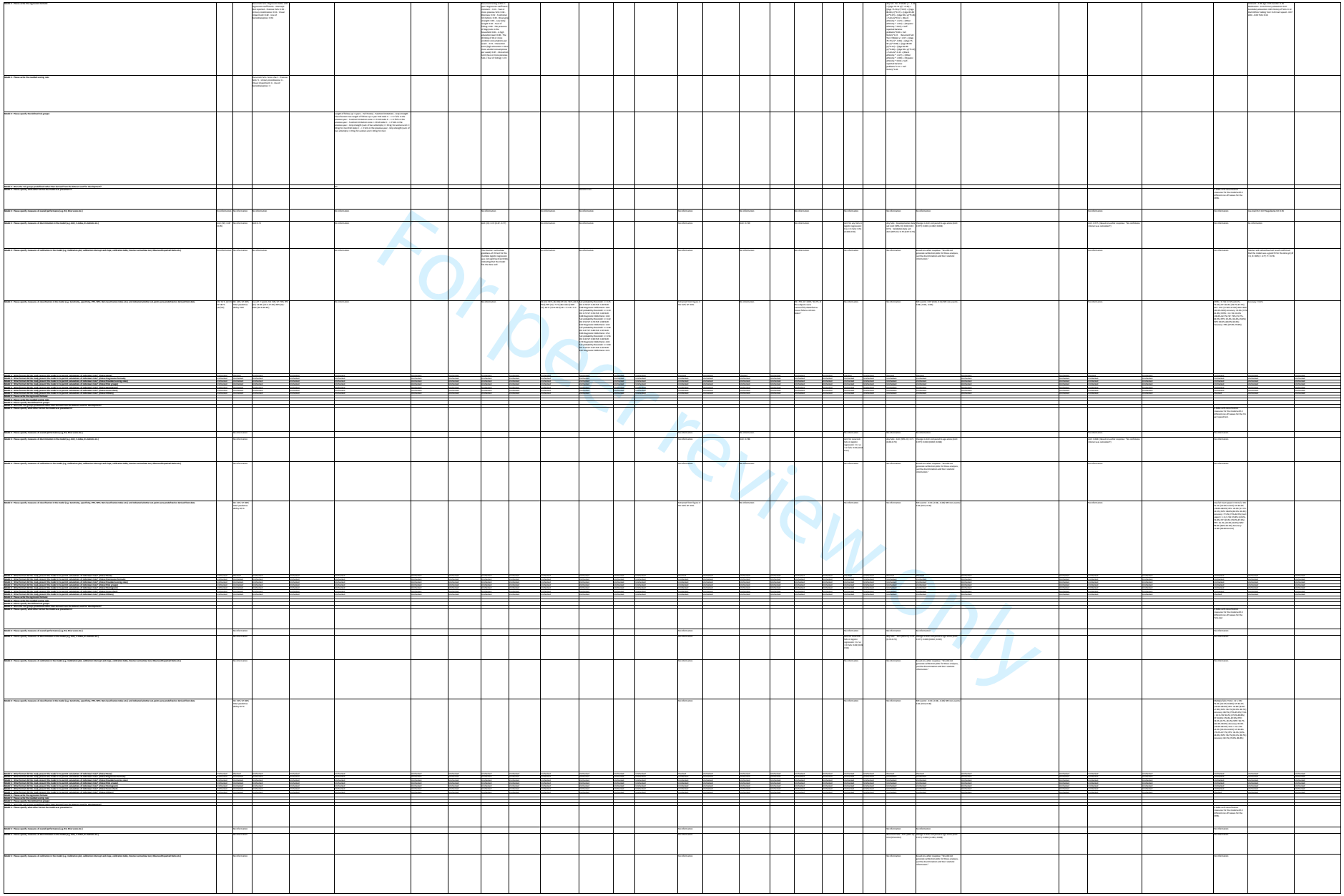

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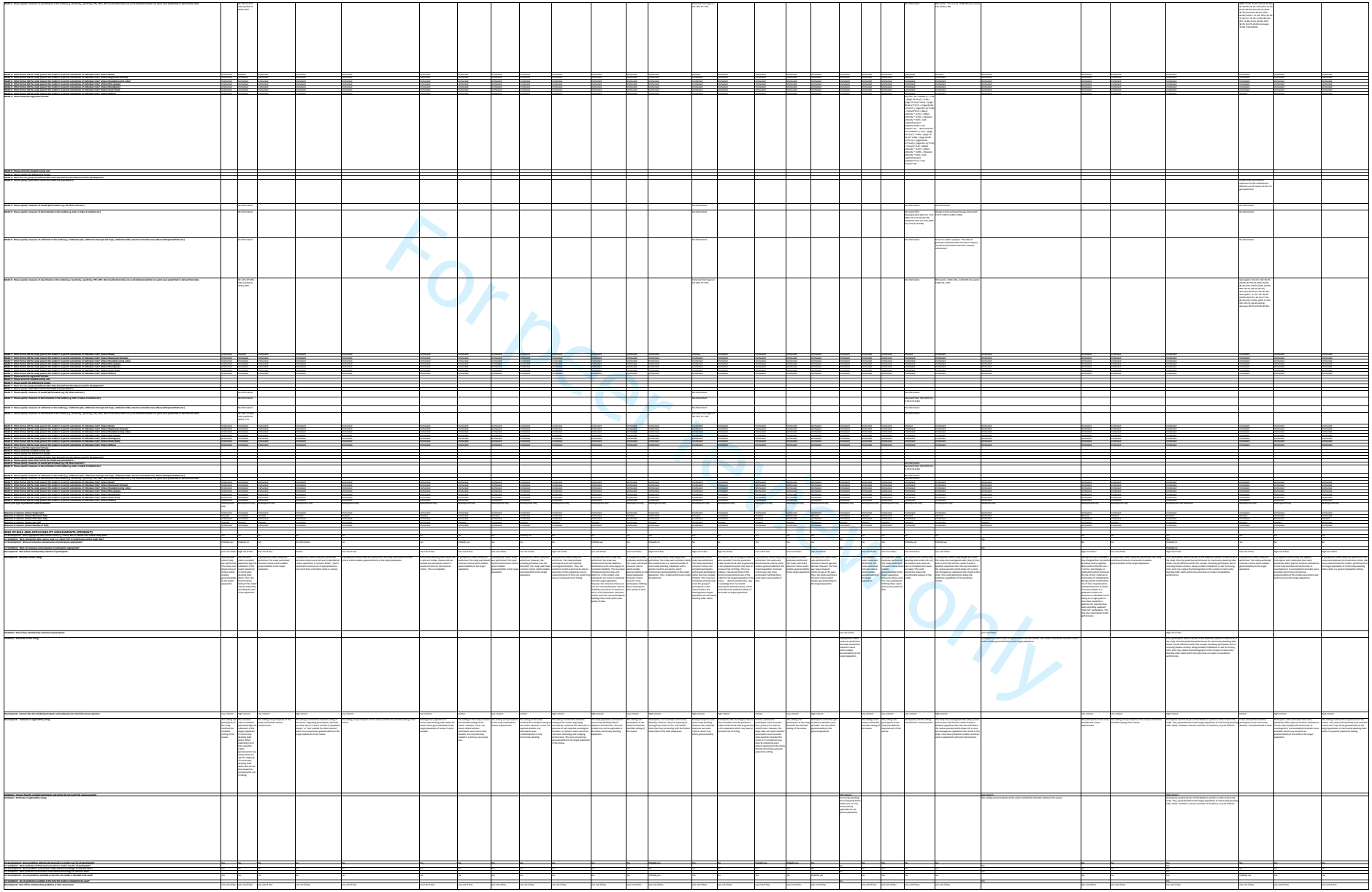

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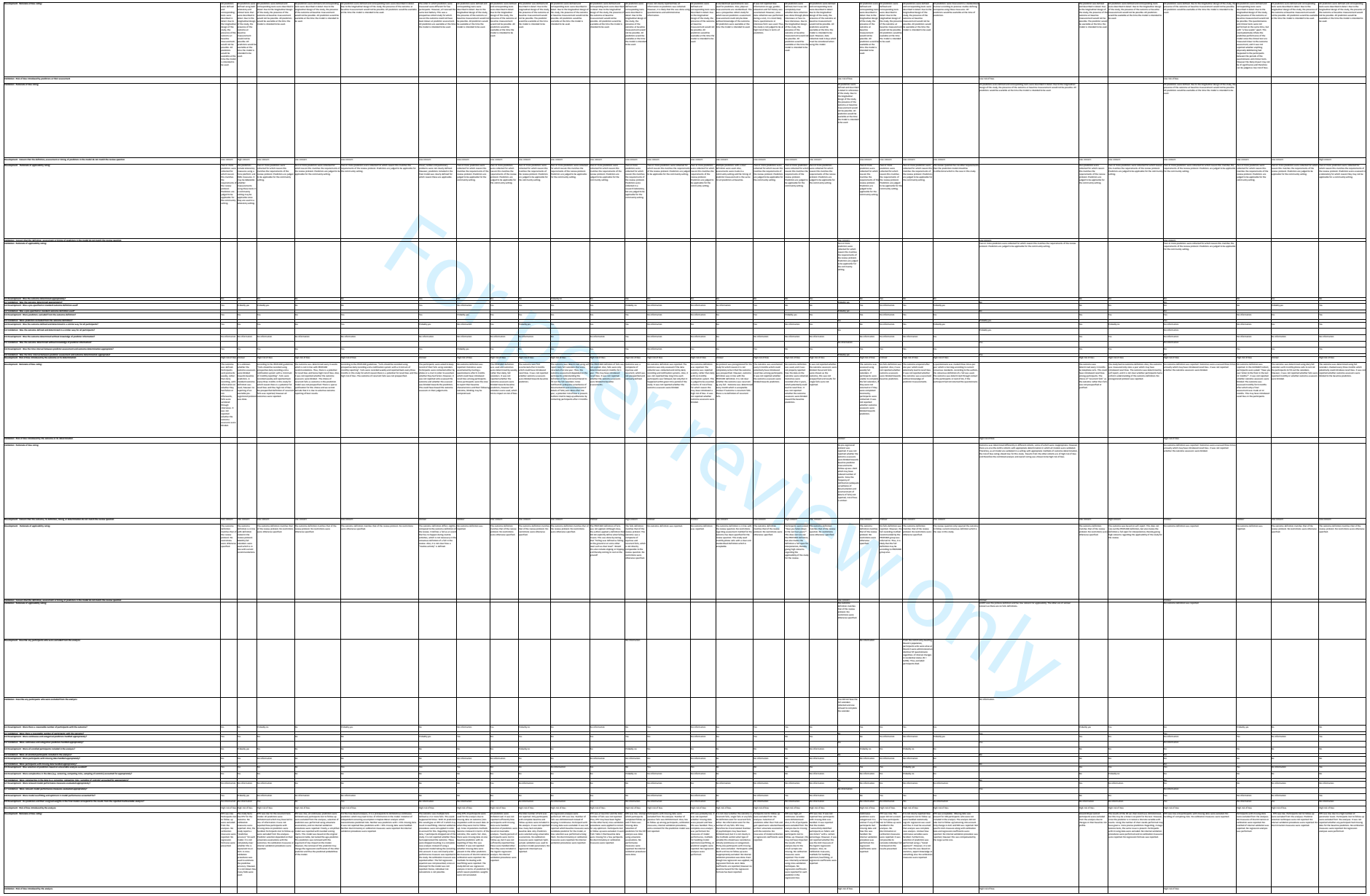

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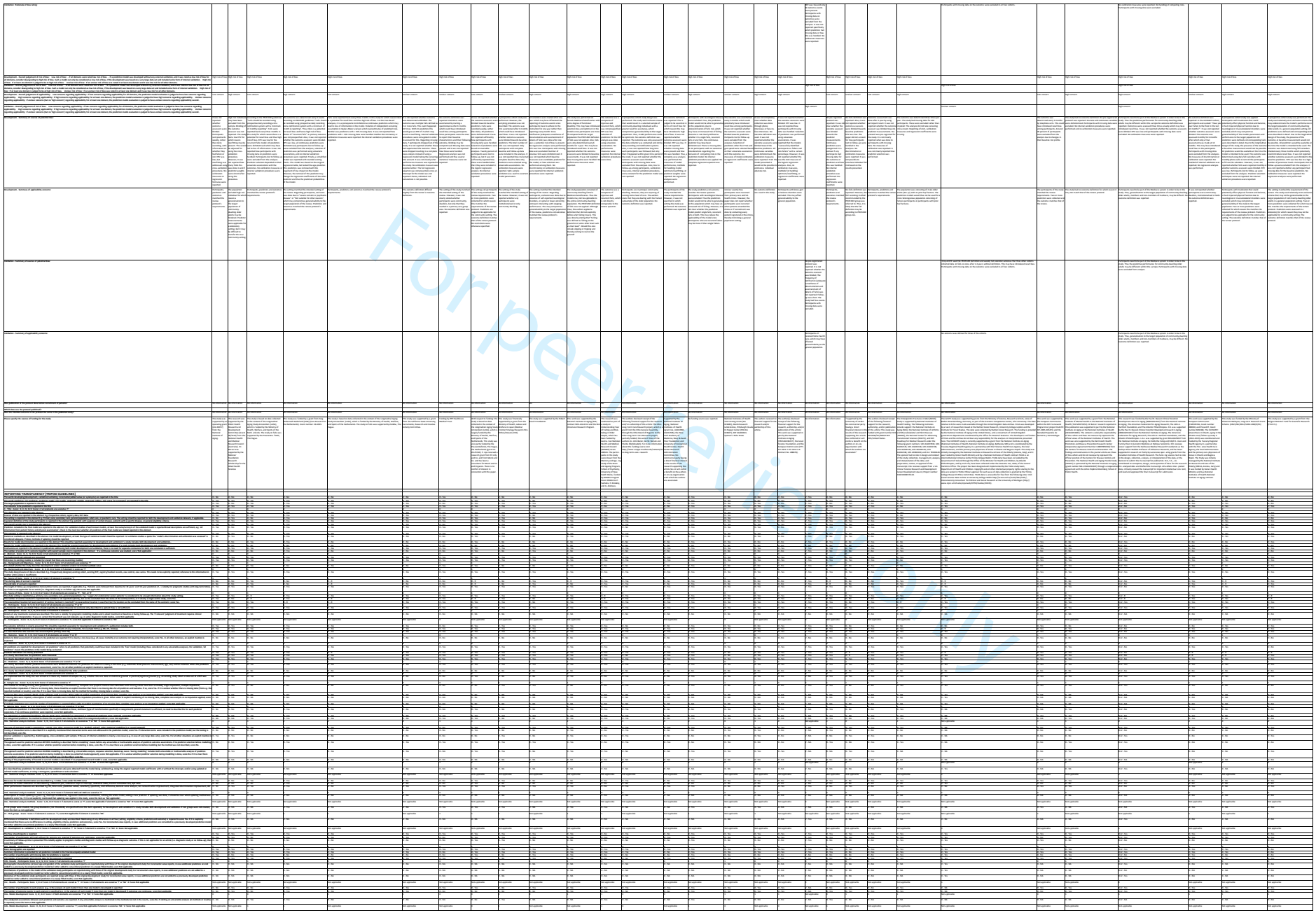

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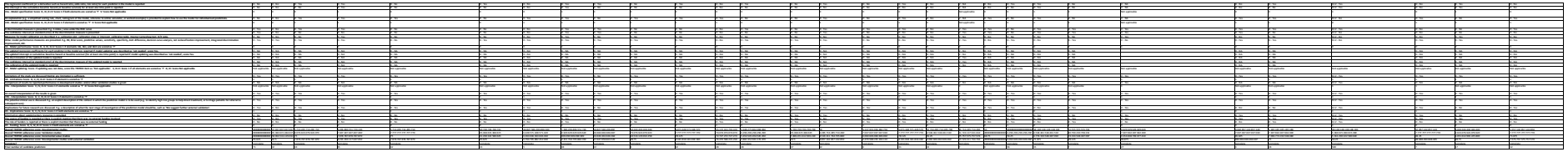

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Section/topic	#	Checklist item	Reported on page #
TITLE			
Title	1	Identify the report as a systematic review, meta-analysis, or both.	#2
ABSTRACT			
Structured summary	2	Provide a structured summary including, as applicable: background; objectives; data sources; study eligibility criteria, participants, and interventions; study appraisal and synthesis methods; results; limitations; conclusions and implications of key findings; systematic review registration number.	#4
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of what is already known.	#6
Objectives	4	Provide an explicit statement of questions being addressed with reference to participants, interventions, comparisons, outcomes, and study design (PICOS).	#6
METHODS			
Protocol and registration	5	Indicate if a review protocol exists, if and where it can be accessed (e.g., Web address), and, if available, provide registration information including registration number.	#6
Eligibility criteria	6	Specify study characteristics (e.g., PICOS, length of follow-up) and report characteristics (e.g., years considered, language, publication status) used as criteria for eligibility, giving rationale.	#6-8
Information sources	7	Describe all information sources (e.g., databases with dates of coverage, contact with study authors to identify additional studies) in the search and date last searched.	#8
Search	8	Present full electronic search strategy for at least one database, including any limits used, such that it could be repeated.	#8 + Appendix 3
Study selection	9	State the process for selecting studies (i.e., screening, eligibility, included in systematic review, and, if applicable, included in the meta-analysis).	#8-9 + Appendix 4
Data collection process	10	Describe method of data extraction from reports (e.g., piloted forms, independently, in duplicate) and any processes for obtaining and confirming data from investigators.	#9
Data items	11	List and define all variables for which data were sought (e.g., PICOS, funding sources) and any assumptions and simplifications made.	#9 + Appendix 5
Risk of bias in individual studies	12	Describe methods used for assessing risk of bias of individual studies (including specification of whether this was done at the study or outcome level), and how this information is to be used in any data synthesis.	#9-10
Summary measures	13	State the principal summary measures (e.g., risk ratio, difference in means).	#10

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Synthesis of results	14	Describe the methods of handling data and combining results of studies, if done, including measures of consistency (e.g., I^2) for each meta-analysis.	**NA**

Section/topic	#	Checklist item	Reported on page #
Risk of bias across studies	15	Specify any assessment of risk of bias that may affect the cumulative evidence (e.g., publication bias, selective reporting within studies).	#10
Additional analyses	16	Describe methods of additional analyses (e.g., sensitivity or subgroup analyses, meta-regression), if done, indicating which were pre-specified.	**NA**
RESULTS			
Study selection	17	Give numbers of studies screened, assessed for eligibility, and included in the review, with reasons for exclusions at each stage, ideally with a flow diagram.	#10 + Fig 1 + Appendix 6-8
Study characteristics	18	For each study, present characteristics for which data were extracted (e.g., study size, PICOS, follow-up period) and provide the citations.	#10-12 + Fig 2 + Appendix 9-10
Risk of bias within studies	19	Present data on risk of bias of each study and, if available, any outcome level assessment (see item 12).	#12 + Table 1
Results of individual studies	20	For all outcomes considered (benefits or harms), present, for each study: (a) simple summary data for each intervention group (b) effect estimates and confidence intervals, ideally with a forest plot.	#12
Synthesis of results	21	Present results of each meta-analysis done, including confidence intervals and measures of consistency.	**NA**
Risk of bias across studies	22	Present results of any assessment of risk of bias across studies (see Item 15).	#12
Additional analysis	23	Give results of additional analyses, if done (e.g., sensitivity or subgroup analyses, meta-regression [see Item 16]).	**NA**
DISCUSSION			
Summary of evidence	24	Summarize the main findings including the strength of evidence for each main outcome; consider their relevance to key groups (e.g., healthcare providers, users, and policy makers).	#12-13
Limitations	25	Discuss limitations at study and outcome level (e.g., risk of bias), and at review-level (e.g., incomplete retrieval of identified research, reporting bias).	#13
Conclusions	26	Provide a general interpretation of the results in the context of other evidence, and implications for future research.	#13-15
FUNDING			

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Funding	27	Describe sources of funding for the systematic review and other support (e.g., supply of data); role of funders for the systematic review.	#10

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